II

COMPANION TO THE INTEGRATED NOTEBOOK

The Question Bank Dental Sleep Medicine

An IFDSM examination revision companion · Author edition

1,000 single-best-answer questions across ten IFDSM competencies

Integrated patient scenarios and clinical decision-making

Five options, one best answer, with an explanatory rationale

Syllabus-section references to the main revision notebook

A structured question bank supporting Notes In Dental Sleep Medicine: focused recall, clinical application, and examination practice mapped to the integrated revision notebook.

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An independent educational question bank. It is not an official IFDSM examination paper and has not been endorsed by an examining or professional body.

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The question bank

Exam-style edition — how to use Volume II

This edition is a complete reworking of 1,000 five-option single-best-answer questions mapped to the ten competencies in the accompanying *Notes in Dental Sleep Medicine*. It emphasises clinical interpretation and patient-centred decision-making, with a single defensible best answer.

I. Read the complete stem

Identify the relevant clinical details, the decision required and any safety or scope-of-practice issue. All five options must be considered before selecting one.

II. Choose one best answer

Each question has five mutually exclusive choices, A–E. All-of-the-above and none-of-the-above choices have been removed. There is one intended best answer per item.

III. Explain and repair

Read the justification, then use the competency number and syllabus subheading to return to the integrated notebook. Explain why alternative actions or interpretations are less appropriate.

IV. Simulate the examination

Attempt blocks of 25–100 questions without answers. The accompanying website provides saved progress, scored mock papers, topic practice and missed-question review.

Question composition

Clinical decision-making SBA: **160** questions

Applied clinical SBA: **424** questions

Clinical interpretation SBA: **369** questions

Focused knowledge SBA: **47** questions

Editorial note

These questions are independently authored study material derived from the supplied revision notebook, not actual or official IFDSM examination items. The answers and clinical statements have not undergone independent expert peer review. Local scope, reimbursement and treatment pathways must be verified for the relevant jurisdiction.

Coverage map

All ten syllabus competencies and all 78 learning-outcome subheadings are represented. Each question keeps a stable number to support bookmarking and cross-referencing.

01 Normal sleep, sleep disorders and pathophysiology of SDB (130)

1.1 Benefits and functions of normal sleep — 8 questions

1.2 Sleep physiology and architecture — 14 questions

1.3 Anatomical/non-anatomical SDB mechanisms — 15 questions

1.4 Prevalence and comorbidities — 8 questions

1.5 Age, sex and other risk factors — 9 questions

1.6 Signs and symptoms — 3 questions

1.7 Impact on the individual — 3 questions

1.8 Impact on society — 2 questions

1.9 Insomnia — 16 questions

1.10 Other respiratory sleep disorders — 24 questions

1.11 Other non-respiratory sleep disorders — 7 questions

1.12 Medications and sleep — 21 questions

02 Diagnostic sleep testing and interpretation (115)

2.1 Normal PSG architecture and parameters — 18 questions

2.2 Abnormal respiratory and sleep indices — 25 questions

2.3 PSG interpretation and treatment choices — 9 questions

2.4 Assessment of therapeutic response — 9 questions

2.5 Attended vs unattended studies — 19 questions

2.6 Vigilance testing: MSLT and MWT — 12 questions

2.7 Screening and dental findings — 23 questions

03 Oral appliance therapy and other treatments (125)

3.1 History of OAT — 16 questions

3.2 Mechanism of OAT — 2 questions

3.3 Indications and contraindications — 9 questions

3.4 PAP therapies — 19 questions

3.5 Surgery — 7 questions

3.6 Positional and behavioural therapies — 11 questions

3.7 Emerging therapies — 12 questions

3.8 Combination therapies — 13 questions

3.9 Comparative efficacy — 9 questions

3.10 Mean disease alleviation — 10 questions

3.11 Guidelines and evidence-based care — 17 questions

04 History, dental examination, imaging and patient selection (100)

4.1 History, objectives and examination — 19 questions

4.2 Medical and lifestyle modifiers — 10 questions

4.3 Other sleep disorders — 17 questions

4.4 Medical comorbidities — 5 questions

4.5 Ethics and patient consent — 5 questions

4.6 Multidisciplinary communication — 10 questions

4.7 Sleep bruxism — 18 questions

4.8 Pain, TMD and sleep — 16 questions

05 Appliance design, registration, manufacture and fitting (130)

5.1 Clinical guideline and diagnosis — 11 questions

5.2 Upper-airway mechanisms and predictors — 21 questions

5.3 Indications and contraindications — 5 questions

5.4 Device designs and materials — 22 questions

5.5 Individual device selection — 16 questions

5.6 Construction bite registration — 22 questions

5.7 Impression accuracy — 3 questions

5.8 Laboratory prescription — 4 questions

5.9 Appliance delivery — 6 questions

5.10 Home care and maintenance — 20 questions

06 Titration, treatment effectiveness and adherence (80)

6.1 Chief complaint and adjustment — 13 questions

6.2 Symptoms and side-effects monitoring — 14 questions

6.3 Referral for objective assessment — 9 questions

6.4 Clinical and instrumented titration — 16 questions

6.5 Efficacy and effectiveness — 14 questions

6.6 Adherence measurement — 14 questions

07 Long-term oral appliance follow-up (60)

7.1 Age, weight, alcohol, drugs and sleep — 12 questions

7.2 Longitudinal symptom documentation — 9 questions

7.3 Treatment modification with progression — 9 questions

7.4 Annual device and clinical review — 30 questions

08 Paediatric sleep-disordered breathing (100)

8.1 Paediatric prevalence — 6 questions

8.2 Aetiology and pathophysiology — 4 questions

8.3 Signs and symptoms — 5 questions

8.4 Insufficient sleep and development — 5 questions

8.5 Adolescent sleep and performance — 14 questions

8.6 Paediatric screening — 14 questions

8.7 Treatment approaches — 15 questions

8.8 Differences from adult SDB — 11 questions

8.9 Underlying syndromes and conditions — 13 questions

8.10 Referral and diagnostic pathways — 13 questions

09 Best-practice, documentation and ethical responsibilities (60)

9.1 Physician diagnosis and portable testing — 13 questions

9.2 Clinical records and communication — 16 questions

9.3 Billing and payer codes — 14 questions

9.4 Ethics and professional competence — 17 questions

10 Informed consent and management of adverse effects (100)

10.1 Valid informed consent — 5 questions

10.2 Short/long-term side effects — 18 questions

10.3 Craniofacial and TMJ effects — 8 questions

10.4 Morning guide and exercises — 20 questions

10.5 Managing side effects — 27 questions

10.6 Suspending or abandoning treatment — 22 questions

01 Normal sleep, sleep disorders and pathophysiology of SDB

QUESTION 0001 · §1.6 · Clinical decision-making SBA

An adult describes loud snoring, witnessed pauses, gasping awakenings and morning headache. Which interpretation is most appropriate?

A. A definitive indication for immediate surgery

B. Only an insomnia presentation

C. A pattern of nocturnal and daytime features warranting OSA assessment

D. A confirmed OSA diagnosis from history alone

E. Sufficient proof that no sleep study is needed

CORRECT ANSWER C

The notebook lists these as common OSA features. They increase suspicion, but physician-led testing and clinical review are still required.

REFERENCE Integrated notebook, Topic 1, §1.6

QUESTION 0002 · §1.6 · Clinical decision-making SBA

A 52-year-old taxi driver reports loud nightly snoring, witnessed breathing pauses and episodes of nearly falling asleep while driving. He asks for a snoring splint. What is the most appropriate immediate response?

A. Supply a non-adjustable snoring guard and review in 12 months

B. Address driving safety urgently and refer for physician-led sleep assessment

C. Assure him that snoring alone is harmless

D. Use the Epworth score as the sole diagnostic test

E. Arrange a CBCT scan to confirm the severity of OSA

CORRECT ANSWER B

Safety-critical sleepiness requires urgent action and appropriate driving advice. A dentist screens and refers; objective diagnostic assessment is arranged and interpreted by the appropriate physician. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 1, §1.6

QUESTION 0003 · §1.2 · Clinical decision-making SBA

A shift worker sleeps for eight hours before a night shift but becomes sleepy at 04:00. Which mechanism best explains the sleepiness?

A. A sudden failure of glymphatic clearance

B. Circadian alerting trough despite adequate prior sleep

C. Normal REM latency always less than 20 minutes

D. Progressive reduction in the homeostatic sleep drive while awake

E. Complete loss of orexin neurons in all night workers

CORRECT ANSWER B

Circadian Process C can produce a low-alertness interval near the biological night even after adequate prior sleep; Process S is sleep pressure linked to wakefulness. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 1, §1.2

QUESTION 0004 · §1.9 · Clinical decision-making SBA

A 44-year-old reports difficulty initiating sleep four nights per week for five months, with daytime impairment despite adequate opportunity to sleep. Which diagnosis is best supported?

A. Chronic insomnia disorder

B. Normal short sleep

C. Acute insomnia only

D. Narcolepsy type 1

E. Circadian phase advance disorder

CORRECT ANSWER A

Chronic insomnia requires sleep difficulty and daytime impairment despite opportunity, typically at least three nights weekly for three months or more. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 1, §1.9

QUESTION 0005 · §1.3 · Clinical decision-making SBA

A patient with OSA has a relatively collapsible pharynx and frequent arousals before upper-airway dilator recruitment. Which physiological trait most directly explains the premature awakening?

A. Low arousal threshold

B. Low loop gain

C. Increased REM atonia

D. Reduced airway resistance during sleep

E. High pharyngeal closing pressure alone

CORRECT ANSWER A

A low respiratory arousal threshold promotes premature waking before pharyngeal dilator recruitment. Pcrit reflects airway anatomy, whereas loop gain reflects ventilatory control stability. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 1, §1.3

QUESTION 0006 · §1.6 · Clinical decision-making SBA

A patient denies sleepiness yet a partner witnesses recurrent apnoeas. What is the best conclusion?

A. Witnessed pauses prove the AHI is below five

B. A normal ESS excludes moderate OSA

C. OSA requires daytime sleepiness in every case

D. The absence of sleepiness does not exclude clinically important OSA

E. The dentist may independently diagnose and treat without referral

CORRECT ANSWER D

OSA may be minimally symptomatic or asymptomatic; screening and physician assessment should incorporate witnessed nocturnal events.

REFERENCE Integrated notebook, Topic 1, §1.6

QUESTION 0007 · §1.2 · Applied clinical SBA

Which of the following statements about REM atonia applies to the pharyngeal dilators too is most accurate in dental sleep medicine?

A. GABA and galanin from the ventrolateral preoptic area promote NREM sleep.

B. Loss of pharyngeal dilator tone during REM sleep contributes to REM-predominant obstructive events.

C. The suprachiasmatic nucleus regulates circadian timing and is entrained mainly by light.

D. Homeostatic sleep pressure increases with time awake and dissipates during sleep.

E. Wakefulness is supported by orexin, monoaminergic pathways and other arousal-promoting transmitters.

CORRECT ANSWER B

Loss of pharyngeal dilator tone during REM sleep contributes to REM-predominant obstructive events. Additional revision detail: This is why REM-predominant OSA exists and why it responds relatively poorly to mandibular advancement. The other options describe different mechanisms, investigations or findings rather than rem atonia applies to the pharyngeal dilators too. Consult Integrated notebook, page 20, §1.2 for the full context.

REFERENCE Integrated notebook, page 20, §1.2

QUESTION 0008 · §1.2 · Clinical interpretation SBA

A 48-year-old with snoring and unrefreshing sleep asks about the underlying sleep physiology. The clinical team records the following finding: This is why REM-predominant OSA exists and why it responds relatively poorly to mandibular advancement . Which concept best explains this observation?

A. N3 (old stages 3+4)

B. Process C (circadian)

C. REM atonia applies to the pharyngeal dilators too

D. Wake-promoting

E. NREM-promoting

CORRECT ANSWER C

This finding most directly describes REM atonia applies to the pharyngeal dilators too: This is why REM-predominant OSA exists and why it responds relatively poorly to mandibular advancement (Topic 7). Distractor distinctions: A refers instead to ~13–23%, 20–40 min first cycle. High-voltage slow delta; highest arousal threshold of all NREM stages; concentrated in the fi; B refers instead to endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract; D refers instead to orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), a; E refers instead to GABA and galanin from the ventrolateral preoptic area.

REFERENCE Integrated notebook, page 20, §1.2

QUESTION 0009 · §1.8 · Clinical decision-making SBA

A bus driver with untreated sleepiness asks why the condition matters outside personal health. Which societal consequence is most immediate?

A. Greater cognitive speed during night shift

B. Increased risk of transport and workplace accidents

C. Reduced healthcare use in every patient

D. Improved alertness at work

E. Elimination of public safety risk by snoring control alone

CORRECT ANSWER B

Excessive sleepiness impairs vigilance and decision-making, creating community and occupational harm.

REFERENCE Integrated notebook, Topic 1, §1.8

QUESTION 0010 · §1.3 · Applied clinical SBA

Which of the following statements about awake upper airway collapsibility index is most accurate in dental sleep medicine?

A. Peripharyngeal adipose tissue increases the mechanical load on the upper airway during sleep.

B. An observational study linked at least two OSA features with a 73% higher incidence of first-onset TMD.

C. The awake upper-airway collapsibility index uses choanal and epiglottic pressure measurements.

D. CPAP dial-down includes CPAP withdrawn for 5 breaths, repeated 5–10×.

E. Enlargement of the tongue, soft palate or tonsils can narrow the pharyngeal airway.

CORRECT ANSWER C

The awake upper-airway collapsibility index uses choanal and epiglottic pressure measurements. Additional revision detail: choanal and epiglottic pressure catheters during negative-pressure inspiration; % collapsibility = (choanal − epiglottic)/choanal. The other options describe different mechanisms, investigations or findings rather than awake upper airway collapsibility index. Consult Integrated notebook, page 11, §1.3 for the full context.

REFERENCE Integrated notebook, page 11, §1.3

QUESTION 0011 · §1.3 · Clinical interpretation SBA

An adult with irregular shifts reports fatigue and reduced daytime concentration. The clinical team records the following finding: choanal and epiglottic pressure catheters during negative-pressure inspiration; % collapsibility = (choanal − epiglottic)/choanal. Which concept best explains this observation?

A. Obesity

B. Soft tissue

C. Craniofacial

D. Awake upper airway collapsibility index

E. CPAP dial-down

CORRECT ANSWER D

This finding most directly describes Awake upper airway collapsibility index: choanal and epiglottic pressure catheters during negative-pressure inspiration; % collapsibility = (choanal − epiglottic)/choanal. Distractor distinctions: A refers instead to peripharyngeal fat mechanically loads the airway, offsetting dilator activity most markedly in sleep; B refers instead to macroglossia, elongated soft palate, enlarged uvula, adenotonsillar hypertrophy (dominant in children), parapharyngeal fat pa; C refers instead to retrognathia, micrognathia, maxillary constriction with high-arched/narrow palate, steep mandibular plane angle, excessive ve; E refers instead to CPAP withdrawn for 5 breaths, repeated 5–10×. Peak inspiratory flow as % of CPAP-supported baseline; PIF ≥30% = only mildly c.

REFERENCE Integrated notebook, page 11, §1.3

QUESTION 0012 · §1.8 · Clinical decision-making SBA

An employer reports absenteeism and reduced work performance in staff with untreated sleep disorders. Which domain is being illustrated?

A. Diagnostic specificity of STOP-BANG

B. Evidence that OAT never helps quality of life

C. A mandatory indication for UPPP

D. An isolated dental occlusion problem

E. The economic and productivity impact of disrupted sleep

CORRECT ANSWER E

The notes include lost productivity, sick leave, disability and greater healthcare use among societal consequences.

REFERENCE Integrated notebook, Topic 1, §1.8

QUESTION 0013 · §1.4 · Applied clinical SBA

Which of the following statements about gender is most accurate in dental sleep medicine?

A. OSA is more common in men, although the sex difference narrows after menopause.

B. OSA is associated with obesity, hypertension, cardiovascular disease and type 2 diabetes.

C. Adenotonsillar enlargement predominates in younger children; excess weight becomes more important in adolescence.

D. Women with OSA may present with fatigue, insomnia or mood symptoms rather than classic witnessed apnoeas.

E. The original Wisconsin cohort reported AHI above 5/h in 9% of women and 24% of men.

CORRECT ANSWER A

OSA is more common in men, although the sex difference narrows after menopause. Additional revision detail: ~2:1–3:1 male predominance; the gap narrows with age and after the menopause — loss of oestrogen's protective effect plus post-menopausal weight gain and central fat redistribution. The other options describe different mechanisms, investigations or findings rather than gender. Consult Integrated notebook, page 11, §1.4 for the full context.

REFERENCE Integrated notebook, page 11, §1.4

QUESTION 0014 · §1.4 · Focused knowledge SBA

Which of the following statements about gender is most accurate in dental sleep medicine?

A. OSA is more common in men, although the sex difference narrows after menopause.

B. The original Wisconsin cohort reported AHI above 5/h in 9% of women and 24% of men.

C. OSA is associated with obesity, hypertension, cardiovascular disease and type 2 diabetes.

D. Adenotonsillar enlargement predominates in younger children; excess weight becomes more important in adolescence.

E. Women with OSA may present with fatigue, insomnia or mood symptoms rather than classic witnessed apnoeas.

CORRECT ANSWER A

OSA is more common in men, although the sex difference narrows after menopause. Additional revision detail: ~2:1–3:1 male predominance; the gap narrows with age and after the menopause — loss of oestrogen's protective effect plus post-menopausal weight gain and central fat redistribution. The other options describe different mechanisms, investigations or findings rather than gender. Consult Integrated notebook, page 11, §1.4 for the full context.

REFERENCE Integrated notebook, page 11, §1.4

QUESTION 0015 · §1.2 · Clinical decision-making SBA

An adult's overnight study shows REM periods lengthening towards morning. What is the best physiological explanation?

A. REM disappears in the latter half

B. N1 replaces REM during the last cycle

C. Normal REM episodes become longer in later sleep cycles

D. Sleep becomes exclusively N2 after midnight

E. N3 dominates the final third

CORRECT ANSWER C

Normal sleep is cyclical, with slow-wave N3 concentrated earlier and progressively longer REM periods later; REM-related OSA may therefore be prominent before waking.

REFERENCE Integrated notebook, Topic 1, §1.2

QUESTION 0016 · §1.5 · Applied clinical SBA

Which of the following statements about women more often present atypically is most accurate in dental sleep medicine?

A. Substantially more prevalent in men is roughly a 2:1 to 3:1 ratio at any given threshold.

B. Women with OSA may present with fatigue, insomnia or mood symptoms rather than classic witnessed apnoeas.

C. OSA is more common in men, although the sex difference narrows after menopause.

D. Smoking includes chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS.

E. Adenotonsillar enlargement predominates in younger children; excess weight becomes more important in adolescence.

CORRECT ANSWER B

Women with OSA may present with fatigue, insomnia or mood symptoms rather than classic witnessed apnoeas. Additional revision detail: insomnia, fatigue, mood disturbance rather than classic witnessed apnoea — and are therefore under-diagnosed. The other options describe different mechanisms, investigations or findings rather than women more often present atypically. Consult Integrated notebook, page 24, §1.5 for the full context.

REFERENCE Integrated notebook, page 24, §1.5

QUESTION 0017 · §1.5 · Clinical interpretation SBA

A patient referred from a dental clinic reports fragmented sleep and morning fatigue. The clinical team records the following finding: insomnia, fatigue, mood disturbance rather than classic witnessed apnoea — and are therefore under-diagnosed. Which concept best explains this observation?

A. Smoking

B. In children the dominant risk factor shifts with age

C. SDB is substantially more prevalent in men

D. Women more often present atypically

E. Central disorders of hypersomnolence

CORRECT ANSWER D

This finding most directly describes Women more often present atypically: insomnia, fatigue, mood disturbance rather than classic witnessed apnoea — and are therefore under-diagnosed. This is a good point to raise in a viva about screening. Other risk factors. Distractor distinctions: A refers instead to chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS, neurological disease; B refers instead to adenotonsillar hypertrophy predominates in younger children (peak 2–8 years, when tonsillar tissue is proportionally largest); C refers instead to roughly a 2:1 to 3:1 ratio at any given threshold; E refers instead to Narcolepsy type 1 — hypocretin/orexin deficiency with cataplexy; type 2 without.

REFERENCE Integrated notebook, page 24, §1.5

QUESTION 0018 · §1.3 · Clinical decision-making SBA

A patient has severe anatomic collapsibility despite adequate tongue-muscle recruitment. Which PALM component primarily describes the problem?

A. A: low arousal threshold

B. M: poor muscle responsiveness

C. C: circadian phase delay

D. P: passive anatomy / critical closing pressure

E. L: high loop gain

CORRECT ANSWER D

Pcrit describes the pressure at which the upper airway collapses. A more positive Pcrit indicates a more collapsible airway; A, L and M describe non-anatomical traits.

REFERENCE Integrated notebook, Topic 1, §1.3

QUESTION 0019 · §1.7 · Applied clinical SBA

Which of the following statements about public safety is most accurate in dental sleep medicine?

A. Daytime sleepiness can impair vigilance and increase the risk of road and workplace accidents.

B. Low positive airway pressure may suppress uncomplicated snoring, but physician assessment is still needed.

C. High altitude can destabilise ventilatory control and cause central periodic breathing.

D. Insomnia risk is increased by age, comorbidity and disrupted work or sleep schedules.

E. Nicotine stimulates wakefulness, while overnight withdrawal can fragment sleep.

CORRECT ANSWER A

Daytime sleepiness can impair vigilance and increase the risk of road and workplace accidents. Additional revision detail: impaired reflexes and judgement → higher risk of serious and fatal crashes; workplace accidents. The other options describe different mechanisms, investigations or findings rather than public safety. Consult Integrated notebook, page 12, §1.7 for the full context.

REFERENCE Integrated notebook, page 12, §1.7

QUESTION 0020 · §1.7 · Clinical interpretation SBA

A patient referred from a dental clinic reports fragmented sleep and morning fatigue. The clinical team records the following finding: impaired reflexes and judgement → higher risk of serious and fatal crashes; workplace accidents. Which concept best explains this observation?

A. Public safety

B. Complementary

C. CPAP dial-down

D. Nasal surgery

E. Complications

CORRECT ANSWER A

This finding most directly describes Public safety: impaired reflexes and judgement → higher risk of serious and fatal crashes; workplace accidents. Distractor distinctions: B refers instead to nasal valve dilators, lubricating sprays, herbal remedies; C refers instead to CPAP withdrawn for 5 breaths, repeated 5–10×. Peak inspiratory flow as % of CPAP-supported baseline; PIF ≥30% = only mildly c; D refers instead to septoplasty with turbinate reduction only where there are both symptoms and objective nasendoscopic evidence of obstruction; E refers instead to frequent nocturnal awakenings, excessive daytime sleepiness and increased risk of adverse cardiovascular outcomes.

REFERENCE Integrated notebook, page 12, §1.7

QUESTION 0021 · §1.3 · Clinical decision-making SBA

A patient awakens repeatedly before upper-airway dilator muscles can recruit adequately. Which endotype is most implicated?

A. Reduced loop gain alone

B. High threshold for arousal

C. Severe nasal septal deviation alone

D. Low respiratory arousal threshold

E. Increased REM atonia during wake

CORRECT ANSWER D

A low arousal threshold precipitates waking before recruitment of compensatory dilator activity, perpetuating fragmentation and ventilatory instability.

REFERENCE Integrated notebook, Topic 1, §1.3

QUESTION 0022 · §1.9 · Applied clinical SBA

Which of the following statements about short-term insomnia disorder is most accurate in dental sleep medicine?

A. Short-term insomnia produces clinically significant distress but does not meet chronic-duration criteria.

B. CBT for insomnia can improve sleep latency, wakefulness after sleep onset and sleep efficiency.

C. Insomnia symptoms are reported by roughly 33–50% of adults, with lower rates of chronic disorder.

D. Chronic insomnia involves difficulty initiating or maintaining sleep with daytime impairment over an extended period.

E. Insomnia risk is increased by age, comorbidity and disrupted work or sleep schedules.

CORRECT ANSWER A

Short-term insomnia produces clinically significant distress but does not meet chronic-duration criteria. Additional revision detail: clinically significant dissatisfaction or impairment failing those frequency/duration criteria. Other insomnia disorder — rare cases not meeting short-term criteria but warranting attention. The other options describe different mechanisms, investigations or findings rather than short-term insomnia disorder. Consult Integrated notebook, page 12, §1.9 for the full context.

REFERENCE Integrated notebook, page 12, §1.9

QUESTION 0023 · §1.9 · Clinical interpretation SBA

A patient referred from a dental clinic reports fragmented sleep and morning fatigue. The clinical team records the following finding: clinically significant dissatisfaction or impairment failing those frequency/duration criteria. Other insomnia disorder — rare cases not meeting short-term criteria but warranting attention. Which concept best explains this observation?

A. Insomnia symptoms

B. Risk factors

C. Chronic insomnia disorder

D. Short-term insomnia disorder

E. Effect on outcomes

CORRECT ANSWER D

This finding most directly describes Short-term insomnia disorder: clinically significant dissatisfaction or impairment failing those frequency/duration criteria. Other insomnia disorder — rare cases not meeting short-term criteria but warranting attention. Distractor distinctions: A refers instead to 33–50% of the adult population; about one third report dissatisfaction with sleep; B refers instead to increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status; C refers instead to sleep-onset and/or sleep-maintenance complaints with associated daytime impairment, exceeding minimal frequency and duration; E refers instead to marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over.

REFERENCE Integrated notebook, page 12, §1.9

QUESTION 0024 · §1.3 · Clinical decision-making SBA

An obese adult has recurrent obstructive events with ventilatory overshoot and undershoot despite moderate anatomy. Which non-anatomical mechanism is implicated?

A. Reduced closing pressure

B. Excessive jaw protrusion

C. High loop gain

D. Normal chemoreflex gain

E. Isolated macroglossia

CORRECT ANSWER C

High loop gain reflects unstable ventilatory control with overcorrection, hypocapnia and subsequent undershoot. It can contribute to incomplete oral-appliance response.

REFERENCE Integrated notebook, Topic 1, §1.3

QUESTION 0025 · §1.10 · Applied clinical SBA

A 48-year-old with snoring and unrefreshing sleep asks about the underlying sleep physiology. Which of the following statements about complementary is most accurate?

A. Hypoventilation syndromes includes insufficient alveolar ventilation causing CO2 retention and hypoxaemia.

B. Complementary includes nasal valve dilators, lubricating sprays, herbal remedies.

C. Low positive airway pressure may suppress uncomplicated snoring, but physician assessment is still needed.

D. Sleep-related hypoxaemia produces sustained oxygen desaturation without an equivalent rise in carbon dioxide.

E. Catathrenia presents as prolonged, often monotone groaning during expiration in sleep.

CORRECT ANSWER B

Complementary includes nasal valve dilators, lubricating sprays, herbal remedies. Additional revision detail: nasal valve dilators, lubricating sprays, herbal remedies. The other options describe different mechanisms, investigations or findings rather than complementary. Consult Integrated notebook, page 14, §1.10 for the full context.

REFERENCE Integrated notebook, page 14, §1.10

QUESTION 0026 · §1.11 · Clinical interpretation SBA

A patient referred from a dental clinic reports fragmented sleep and morning fatigue. The clinical team records the following finding: Delayed phase (adolescents), advanced phase (older adults), irregular and non-24-hour rhythms, shift work disorder, jet lag. Which concept best explains this observation?

A. Circadian rhythm sleep–wake disorders

B. Sleep-related movement disorders

C. Central disorders of hypersomnolence

D. Process C (circadian)

E. Sleep-related hypoxaemia disorder

CORRECT ANSWER A

This finding most directly describes Circadian rhythm sleep–wake disorders: Delayed phase (adolescents), advanced phase (older adults), irregular and non-24-hour rhythms, shift work disorder, jet lag. Distractor distinctions: B refers instead to Restless legs syndrome (Willis–Ekbom) — urge to move with creeping/crawling sensations, worse in the evening and at rest, rel; C refers instead to Narcolepsy type 1 — hypocretin/orexin deficiency with cataplexy; type 2 without; D refers instead to endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract; E refers instead to sustained desaturation during sleep without a comparable rise in CO2.

REFERENCE Integrated notebook, page 14, §1.11

QUESTION 0027 · §1.9 · Clinical decision-making SBA

An adult complains of difficulty sleeping four nights per week for four months with impaired daytime function. How should the duration criterion be classified?

A. Insomnia cannot coexist with OSA

B. The pattern meets the chronic-insomnia duration and frequency criteria

C. A PSG AHI of 15 is necessary for diagnosis

D. It is acute because it lasts less than a year

E. Chronic insomnia requires nightly symptoms for six months

CORRECT ANSWER B

The notebook specifies chronic insomnia at least three nights weekly for at least three months; daytime consequences also require clinical consideration.

REFERENCE Integrated notebook, Topic 1, §1.9

QUESTION 0028 · §1.11 · Applied clinical SBA

Which of the following statements about circadian rhythm sleep–wake disorders is most accurate in dental sleep medicine?

A. Circadian rhythm disorders include delayed and advanced sleep phase and shift-work disturbance.

B. CBT for insomnia can improve sleep latency, wakefulness after sleep onset and sleep efficiency.

C. Central disorders of hypersomnolence includes narcolepsy type 1 — hypocretin/orexin deficiency with cataplexy; type 2 without.

D. Smoking includes chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS.

E. Restless legs syndrome causes an urge to move the limbs, especially during evening rest.

CORRECT ANSWER A

Circadian rhythm disorders include delayed and advanced sleep phase and shift-work disturbance. Additional revision detail: Delayed phase (adolescents), advanced phase (older adults), irregular and non-24-hour rhythms, shift work disorder, jet lag. The other options describe different mechanisms, investigations or findings rather than circadian rhythm sleep–wake disorders. Consult Integrated notebook, page 14, §1.11 for the full context.

REFERENCE Integrated notebook, page 14, §1.11

QUESTION 0029 · §1.12 · Focused knowledge SBA

Which of the following statements about alcohol is most accurate in dental sleep medicine?

A. An observational study linked at least two OSA features with a 73% higher incidence of first-onset TMD.

B. Historical population data reported AHI above 5/h in about 9% of women and 24% of men.

C. Caffeine blocks adenosine receptors and has an elimination half-life of roughly five hours.

D. Alcohol may shorten sleep onset but subsequently fragments sleep and worsens upper-airway obstruction.

E. Approximately 70% of people with OSA have at least one non-anatomical physiological impairment.

CORRECT ANSWER D

Alcohol may shorten sleep onset but subsequently fragments sleep and worsens upper-airway obstruction. Additional revision detail: reduces sleep latency but suppresses REM in the first half of the night with rebound in the second; fragments sleep; reduces pharyngeal dilator muscle tone and blunts arousal responses, worsening OSA and snoring. The other options describe different mechanisms, investigations or findings rather than alcohol. Consult Integrated notebook, page 32, §1.12 for the full context.

REFERENCE Integrated notebook, page 32, §1.12

QUESTION 0030 · §1.5 · Clinical decision-making SBA

A post-menopausal woman reports fatigue and insomnia rather than loud snoring. Which approach best avoids under-recognition of OSA?

A. Exclude OSA because snoring is absent

B. Use male-only screening thresholds

C. Treat with a mandibular device before medical review

D. Diagnose depression without assessment

E. Screen for OSA despite an atypical symptom presentation

CORRECT ANSWER E

Women may present with fatigue, insomnia and mood complaints. Apparent absence of classic symptoms does not safely rule out OSA.

REFERENCE Integrated notebook, Topic 1, §1.5

QUESTION 0031 · §1.12 · Applied clinical SBA

Which of the following statements about alcohol is most accurate in dental sleep medicine?

A. Caffeine blocks adenosine receptors and has an elimination half-life of roughly five hours.

B. Alcohol may shorten sleep onset but subsequently fragments sleep and worsens upper-airway obstruction.

C. Approximately 70% of people with OSA have at least one non-anatomical physiological impairment.

D. An observational study linked at least two OSA features with a 73% higher incidence of first-onset TMD.

E. Historical population data reported AHI above 5/h in about 9% of women and 24% of men.

CORRECT ANSWER B

Alcohol can shorten sleep latency but disrupts later sleep and reduces upper-airway dilator muscle tone. It may therefore worsen snoring and obstructive events, especially when combined with sedating drugs. Caffeine acts primarily on adenosine receptors, whereas the other choices concern epidemiology or disease associations rather than alcohol.

REFERENCE Integrated notebook, page 32, §1.12

QUESTION 0032 · §1.1 · Clinical interpretation SBA

A 51-year-old with witnessed respiratory pauses has a consultation with a sleep specialist. The clinical team records the following finding: declarative in NREM (sleep spindles), procedural and emotional in REM. Which concept best explains this observation?

A. Biochemical refreshment

B. Hormone regulation

C. Emotional regulation

D. Physical restoration and repair

E. Memory consolidation

CORRECT ANSWER E

This finding most directly describes Memory consolidation: declarative in NREM (sleep spindles), procedural and emotional in REM. Distractor distinctions: A refers instead to synaptic and neuronal function; glial-mediated glymphatic clearance of metabolites; glycogen restoration; neurogenesis; B refers instead to growth hormone in early SWS; prolactin, testosterone, PTH; thyroid hormone late evening; melatonin under light–dark control; C refers instead to REM-dependent processing of affect; D refers instead to protein synthesis, cell and tissue repair, anabolic dominance.

REFERENCE Integrated notebook, page 9, §1.1

QUESTION 0033 · §1.11 · Clinical decision-making SBA

A sleepy patient suddenly loses muscle tone when laughing and enters REM unusually quickly. Which disorder must be considered by a sleep physician?

A. Primary snoring

B. Positional OSA

C. Narcolepsy

D. Idiopathic tooth grinding

E. Sleep-related hypoventilation alone

CORRECT ANSWER C

Cataplexy and abnormal REM onset are characteristic clues to narcolepsy, a central hypersomnolence disorder requiring medical assessment.

REFERENCE Integrated notebook, Topic 1, §1.11

QUESTION 0034 · §1.1 · Applied clinical SBA

A 48-year-old with snoring and unrefreshing sleep asks about the underlying sleep physiology. Which of the following statements about memory consolidation is most accurate?

A. Growth hormone release increases during early-night slow-wave sleep.

B. Emotional regulation includes rEM-dependent processing of affect.

C. Sleep supports synaptic regulation and glymphatic clearance of metabolic by-products.

D. Sleep supports protein synthesis, tissue repair and restorative anabolic processes.

E. Memory consolidation includes declarative in NREM (sleep spindles), procedural and emotional in REM.

CORRECT ANSWER E

Memory consolidation includes declarative in NREM (sleep spindles), procedural and emotional in REM. Additional revision detail: declarative in NREM (sleep spindles), procedural and emotional in REM. The other options describe different mechanisms, investigations or findings rather than memory consolidation. Consult Integrated notebook, page 9, §1.1 for the full context.

REFERENCE Integrated notebook, page 9, §1.1

QUESTION 0035 · §1.2 · Clinical interpretation SBA

A patient referred from a dental clinic reports fragmented sleep and morning fatigue. The clinical team records the following finding: homeostatic sleep drive. Sleep pressure accumulates with time awake (adenosine accumulation is the principal candidate mediator) and dissipates during sleep. Which concept best explains this observation?

A. Wake-promoting

B. N3 (old stages 3+4)

C. Process S

D. NREM-promoting

E. Process C (circadian)

CORRECT ANSWER C

This finding most directly describes Process S: homeostatic sleep drive. Sleep pressure accumulates with time awake (adenosine accumulation is the principal candidate mediator) and dissipates during sleep. Distractor distinctions: A refers instead to orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), a; B refers instead to ~13–23%, 20–40 min first cycle. High-voltage slow delta; highest arousal threshold of all NREM stages; concentrated in the fi; D refers instead to GABA and galanin from the ventrolateral preoptic area; E refers instead to endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract.

REFERENCE Integrated notebook, page 18, §1.2

QUESTION 0036 · §1.12 · Clinical decision-making SBA

A patient with suspected OSA takes an opioid at bedtime and develops more complex events. What is the key concern?

A. Opioids are first-line OSA treatment

B. Opioids can depress ventilatory drive and exacerbate sleep-disordered breathing

C. Opioids reliably restore REM sleep

D. Opioids abolish central apnoea

E. Opioids increase pharyngeal muscle tone

CORRECT ANSWER B

Opioids suppress respiration and can worsen sleep-related breathing disturbances, including central events; medication review is necessary.

REFERENCE Integrated notebook, Topic 1, §1.12

QUESTION 0037 · §1.2 · Applied clinical SBA

Which of the following statements about process s is most accurate in dental sleep medicine?

A. Loss of pharyngeal dilator tone during REM sleep contributes to REM-predominant obstructive events.

B. The suprachiasmatic nucleus regulates circadian timing and is entrained mainly by light.

C. N3 (old stages 3+4): ~13–23%, 20–40 min first cycle.

D. Wakefulness is supported by orexin, monoaminergic pathways and other arousal-promoting transmitters.

E. Homeostatic sleep pressure increases with time awake and dissipates during sleep.

CORRECT ANSWER E

Homeostatic sleep pressure increases with time awake and dissipates during sleep. Additional revision detail: homeostatic sleep drive. Sleep pressure accumulates with time awake (adenosine accumulation is the principal candidate mediator) and dissipates during sleep. The other options describe different mechanisms, investigations or findings rather than process s. Consult Integrated notebook, page 18, §1.2 for the full context.

REFERENCE Integrated notebook, page 18, §1.2

QUESTION 0038 · §1.3 · Clinical interpretation SBA

A 63-year-old with recurrent nocturnal awakenings is being assessed for disordered breathing. The clinical team records the following finding: retrognathia, micrognathia, maxillary constriction with high-arched/narrow palate, steep mandibular plane angle, excessive vertical facial growth, inferiorly positioned hyoid, reduced maxillary/mandibular…. Which concept best explains this observation?

A. Obesity

B. CPAP dial-down

C. Soft tissue

D. Craniofacial

E. Awake upper airway collapsibility index

CORRECT ANSWER D

This finding most directly describes Craniofacial: retrognathia, micrognathia, maxillary constriction with high-arched/narrow palate, steep mandibular plane angle, excessive vertical facial growth, inferiorly positioned hyoid, reduced maxillary/mandibular…. Distractor distinctions: A refers instead to peripharyngeal fat mechanically loads the airway, offsetting dilator activity most markedly in sleep; B refers instead to CPAP withdrawn for 5 breaths, repeated 5–10×. Peak inspiratory flow as % of CPAP-supported baseline; PIF ≥30% = only mildly c; C refers instead to macroglossia, elongated soft palate, enlarged uvula, adenotonsillar hypertrophy (dominant in children), parapharyngeal fat pa; E refers instead to choanal and epiglottic pressure catheters during negative-pressure inspiration; % collapsibility = (choanal − epiglottic)/cho.

REFERENCE Integrated notebook, page 10, §1.3

QUESTION 0039 · §1.7 · Clinical decision-making SBA

A professional driver with suspected OSA reports dozing at traffic lights. What is the most important immediate concern?

A. Rely on the absence of witnessed apnoeas

B. Delay all action until appliance fabrication

C. Urgent assessment of driving safety and medical referral

D. Advise further caffeine without assessment

E. Treat snoring only

CORRECT ANSWER C

Untreated sleepiness creates an immediate public-safety risk. The notebook emphasises explicit driving advice and prompt specialist evaluation for safety-critical workers.

REFERENCE Integrated notebook, Topic 1, §1.7

QUESTION 0040 · §1.3 · Applied clinical SBA

Which of the following statements about craniofacial is most accurate in dental sleep medicine?

A. Peripharyngeal adipose tissue increases the mechanical load on the upper airway during sleep.

B. CPAP dial-down includes CPAP withdrawn for 5 breaths, repeated 5–10×.

C. Enlargement of the tongue, soft palate or tonsils can narrow the pharyngeal airway.

D. Retrognathia, micrognathia and maxillary constriction can increase anatomical susceptibility to OSA.

E. The awake upper-airway collapsibility index uses choanal and epiglottic pressure measurements.

CORRECT ANSWER D

Retrognathia, micrognathia and maxillary constriction can increase anatomical susceptibility to OSA. Additional revision detail: retrognathia, micrognathia, maxillary constriction, a high and narrow palate, steep mandibular plane angle, inferior hyoid position, and reduced jaw length. The other options describe different mechanisms, investigations or findings rather than craniofacial. Consult Integrated notebook, page 10, §1.3 for the full context.

REFERENCE Integrated notebook, page 10, §1.3

QUESTION 0041 · §1.4 · Clinical interpretation SBA

A 48-year-old with snoring and unrefreshing sleep asks about the underlying sleep physiology. The clinical team records the following finding: obesity, hypertension, cardiovascular disease (especially ischaemic heart disease), type 2 diabetes. Long-term: neurocognitive impairment, metabolic syndrome, systemic hypertension, cardiovascular disease. Which concept best explains this observation?

A. Biochemical refreshment

B. Gender

C. Prevalence

D. Others

E. Predictive comorbidities

CORRECT ANSWER E

This finding most directly describes Predictive comorbidities: obesity, hypertension, cardiovascular disease (especially ischaemic heart disease), type 2 diabetes. Long-term: neurocognitive impairment, metabolic syndrome, systemic hypertension, cardiovascular disease. Distractor distinctions: A refers instead to synaptic and neuronal function; glial-mediated glymphatic clearance of metabolites; glycogen restoration; neurogenesis; B refers instead to ~2:1–3:1 male predominance; the gap narrows with age and after the menopause — loss of oestrogen's protective effect plus pos; C refers instead to symptoms 33–50%; ~⅓ report dissatisfaction with sleep; with distress/impairment 10–15%; chronic 5–15%; with medical/psychiatr; D refers instead to narrowed airway, craniofacial abnormality, large neck circumference, large tongue, family history, smoking, alcohol/ sedative.

REFERENCE Integrated notebook, page 11, §1.4

QUESTION 0042 · §1.3 · Clinical decision-making SBA

A patient with severe pharyngeal collapsibility and high loop gain does not normalise on maximal comfortable mandibular advancement. Why might further protrusion fail?

A. A device directly raises arousal threshold

B. Protrusion always corrects loop gain

C. The non-anatomical ventilatory instability remains uncorrected

D. A negative Pcrit necessarily indicates severe anatomy

E. Mandibular advancement treats all OSA phenotypes equally

CORRECT ANSWER C

Oral appliances primarily improve airway anatomy; they do not systematically correct unstable ventilatory control, explaining selected non-responders and combined treatment approaches.

REFERENCE Integrated notebook, Topic 1, §1.3

QUESTION 0043 · §1.4 · Applied clinical SBA

Which of the following statements about predictive comorbidities is most accurate in dental sleep medicine?

A. Short-term insomnia produces clinically significant distress but does not meet chronic-duration criteria.

B. OSA is more common in men, although the sex difference narrows after menopause.

C. Heavy snoring may be associated with local carotid vascular injury, although causation is uncertain.

D. The original Wisconsin cohort reported AHI above 5/h in 9% of women and 24% of men.

E. OSA is associated with obesity, hypertension, cardiovascular disease and type 2 diabetes.

CORRECT ANSWER E

OSA is associated with obesity, hypertension, cardiovascular disease and type 2 diabetes. Additional revision detail: obesity, hypertension, cardiovascular disease (especially ischaemic heart disease), type 2 diabetes. Long-term: neurocognitive impairment, metabolic syndrome, systemic hypertension, cardiovascular disease. The other options describe different mechanisms, investigations or findings rather than predictive comorbidities. Consult Integrated notebook, page 11, §1.4 for the full context.

REFERENCE Integrated notebook, page 11, §1.4

QUESTION 0044 · §1.5 · Clinical interpretation SBA

An adult with irregular shifts reports fatigue and reduced daytime concentration. The clinical team records the following finding: roughly a 2:1 to 3:1 ratio at any given threshold. Which concept best explains this observation?

A. Women more often present atypically

B. In children the dominant risk factor shifts with age

C. CPAP in those already committed or willing

D. SDB is substantially more prevalent in men

E. Smoking

CORRECT ANSWER D

This finding most directly describes SDB is substantially more prevalent in men: roughly a 2:1 to 3:1 ratio at any given threshold. Distractor distinctions: A refers instead to insomnia, fatigue, mood disturbance rather than classic witnessed apnoea — and are therefore under-diagnosed. This is a good; B refers instead to adenotonsillar hypertrophy predominates in younger children (peak 2–8 years, when tonsillar tissue is proportionally largest); C refers instead to effective and safe, compliance highly variable; 4–6 cmH2O eliminates snoring; E refers instead to chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS, neurological disease.

REFERENCE Integrated notebook, page 24, §1.5

QUESTION 0045 · §1.5 · Applied clinical SBA

A 48-year-old with snoring and unrefreshing sleep asks about the underlying sleep physiology. Which of the following statements about SDB is substantially more prevalent in men is most accurate?

A. GABA and galanin from the ventrolateral preoptic area promote NREM sleep.

B. Smoking includes chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS.

C. Adenotonsillar enlargement predominates in younger children; excess weight becomes more important in adolescence.

D. Substantially more prevalent in men is roughly a 2:1 to 3:1 ratio at any given threshold.

E. Women with OSA may present with fatigue, insomnia or mood symptoms rather than classic witnessed apnoeas.

CORRECT ANSWER D

Substantially more prevalent in men is roughly a 2:1 to 3:1 ratio at any given threshold. Additional revision detail: roughly a 2:1 to 3:1 ratio at any given threshold. The other options describe different mechanisms, investigations or findings rather than sdb is substantially more prevalent in men. Consult Integrated notebook, page 24, §1.5 for the full context.

REFERENCE Integrated notebook, page 24, §1.5

QUESTION 0046 · §1.9 · Clinical interpretation SBA

A 48-year-old with snoring and unrefreshing sleep asks about the underlying sleep physiology. The clinical team records the following finding: sleep-onset and/or sleep-maintenance complaints with associated daytime impairment, exceeding minimal frequency and duration thresholds shown to be associated with clinically significant morbidity. Which concept best explains this observation?

A. Insomnia symptoms

B. Risk factors

C. Effect on outcomes

D. Chronic insomnia disorder

E. Short-term insomnia disorder

CORRECT ANSWER D

This finding most directly describes Chronic insomnia disorder: sleep-onset and/or sleep-maintenance complaints with associated daytime impairment, exceeding minimal frequency and duration thresholds shown to be associated with clinically significant morbidity. Distractor distinctions: A refers instead to 33–50% of the adult population; about one third report dissatisfaction with sleep; B refers instead to increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status; C refers instead to marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over; E refers instead to clinically significant dissatisfaction or impairment failing those frequency/duration criteria. Other insomnia disorder — rar.

REFERENCE Integrated notebook, page 26, §1.9

QUESTION 0047 · §1.9 · Applied clinical SBA

Which of the following statements about chronic insomnia disorder is most accurate in dental sleep medicine?

A. CBT for insomnia can improve sleep latency, wakefulness after sleep onset and sleep efficiency.

B. Chronic insomnia involves difficulty initiating or maintaining sleep with daytime impairment over an extended period.

C. Insomnia risk is increased by age, comorbidity and disrupted work or sleep schedules.

D. Short-term insomnia produces clinically significant distress but does not meet chronic-duration criteria.

E. Insomnia symptoms are reported by roughly 33–50% of adults, with lower rates of chronic disorder.

CORRECT ANSWER B

Chronic insomnia involves difficulty initiating or maintaining sleep with daytime impairment over an extended period. Additional revision detail: sleep-onset and/or sleep-maintenance complaints with associated daytime impairment, exceeding minimal frequency and duration thresholds shown to be associated with clinically significant morbidity. The other options describe different mechanisms, investigations or findings rather than chronic insomnia disorder. Consult Integrated notebook, page 26, §1.9 for the full context.

REFERENCE Integrated notebook, page 26, §1.9

QUESTION 0048 · §1.11 · Clinical interpretation SBA

An adult with irregular shifts reports fatigue and reduced daytime concentration. The clinical team records the following finding: Narcolepsy type 1 — hypocretin/orexin deficiency with cataplexy; type 2 without. Which concept best explains this observation?

A. Sleep-related movement disorders

B. Central disorders of hypersomnolence

C. Awake upper airway collapsibility index

D. Circadian rhythm sleep–wake disorders

E. Women more often present atypically

CORRECT ANSWER B

This finding most directly describes Central disorders of hypersomnolence: Narcolepsy type 1 — hypocretin/orexin deficiency with cataplexy; type 2 without. Distractor distinctions: A refers instead to Restless legs syndrome (Willis–Ekbom) — urge to move with creeping/crawling sensations, worse in the evening and at rest, rel; C refers instead to choanal and epiglottic pressure catheters during negative-pressure inspiration; % collapsibility = (choanal − epiglottic)/cho; D refers instead to Delayed phase (adolescents), advanced phase (older adults), irregular and non-24-hour rhythms, shift work disorder, jet lag; E refers instead to insomnia, fatigue, mood disturbance rather than classic witnessed apnoea — and are therefore under-diagnosed. This is a good.

REFERENCE Integrated notebook, page 14, §1.11

QUESTION 0049 · §1.10 · Applied clinical SBA

Which of the following statements about sleep-related hypoxaemia disorder is most accurate in dental sleep medicine?

A. Catathrenia presents as prolonged, often monotone groaning during expiration in sleep.

B. Hypoventilation syndromes includes insufficient alveolar ventilation causing CO2 retention and hypoxaemia.

C. Sleep-related hypoxaemia produces sustained oxygen desaturation without an equivalent rise in carbon dioxide.

D. Low positive airway pressure may suppress uncomplicated snoring, but physician assessment is still needed.

E. Complementary includes nasal valve dilators, lubricating sprays, herbal remedies.

CORRECT ANSWER C

Sleep-related hypoxaemia produces sustained oxygen desaturation without an equivalent rise in carbon dioxide. Additional revision detail: sustained desaturation during sleep without a comparable rise in CO2. The other options describe different mechanisms, investigations or findings rather than sleep-related hypoxaemia disorder. Consult Integrated notebook, page 29, §1.10 for the full context.

REFERENCE Integrated notebook, page 29, §1.10

QUESTION 0050 · §1.12 · Clinical interpretation SBA

A patient referred from a dental clinic reports fragmented sleep and morning fatigue. The clinical team records the following finding: an adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure) with a half-life of roughly five hours. Which concept best explains this observation?

A. Caffeine

B. Insomnia

C. Alcohol

D. OSA–TMD

E. Nicotine

CORRECT ANSWER A

This finding most directly describes Caffeine: an adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure) with a half-life of roughly five hours. Distractor distinctions: B refers instead to symptoms 33–50% · disorder 10–15% · chronic 5–15% · comorbid 50–75% · chronic = ≥3 nights/week × ≥3 months; C refers instead to reduces sleep latency but suppresses REM in the first half of the night with rebound in the second; fragments sleep; reduces; D refers instead to ≥2 signs/symptoms → 73% greater first-onset TMD incidence; chronic TMD >3× more frequent; E refers instead to stimulant plus nocturnal withdrawal arousals; smoking also causes upper airway inflammation.

REFERENCE Integrated notebook, page 32, §1.12

QUESTION 0051 · §1.11 · Applied clinical SBA

Which of the following statements about central disorders of hypersomnolence is most accurate in dental sleep medicine?

A. Restless legs syndrome causes an urge to move the limbs, especially during evening rest.

B. Circadian rhythm disorders include delayed and advanced sleep phase and shift-work disturbance.

C. Catathrenia presents as prolonged, often monotone groaning during expiration in sleep.

D. Central disorders of hypersomnolence includes narcolepsy type 1 — hypocretin/orexin deficiency with cataplexy; type 2 without.

E. Cognitive behavioural therapy for insomnia is first-line treatment for chronic insomnia.

CORRECT ANSWER D

Central disorders of hypersomnolence includes narcolepsy type 1 — hypocretin/orexin deficiency with cataplexy; type 2 without. Additional revision detail: Narcolepsy type 1 — hypocretin/orexin deficiency with cataplexy; type 2 without. The other options describe different mechanisms, investigations or findings rather than central disorders of hypersomnolence. Consult Integrated notebook, page 14, §1.11 for the full context.

REFERENCE Integrated notebook, page 14, §1.11

QUESTION 0052 · §1.1 · Clinical interpretation SBA

A 51-year-old with witnessed respiratory pauses has a consultation with a sleep specialist. The clinical team records the following finding: synaptic and neuronal function; glial-mediated glymphatic clearance of metabolites; glycogen restoration; neurogenesis. Which concept best explains this observation?

A. Memory consolidation

B. Hormone regulation

C. Emotional regulation

D. Biochemical refreshment

E. Physical restoration and repair

CORRECT ANSWER D

This finding most directly describes Biochemical refreshment: synaptic and neuronal function; glial-mediated glymphatic clearance of metabolites; glycogen restoration; neurogenesis. Distractor distinctions: A refers instead to declarative in NREM (sleep spindles), procedural and emotional in REM; B refers instead to growth hormone in early SWS; prolactin, testosterone, PTH; thyroid hormone late evening; melatonin under light–dark control; C refers instead to REM-dependent processing of affect; E refers instead to protein synthesis, cell and tissue repair, anabolic dominance.

REFERENCE Integrated notebook, page 9, §1.1

QUESTION 0053 · §1.12 · Applied clinical SBA

A 48-year-old with snoring and unrefreshing sleep asks about the underlying sleep physiology. Which of the following statements about caffeine is most accurate?

A. Historical population data reported AHI above 5/h in about 9% of women and 24% of men.

B. Caffeine blocks adenosine receptors and has an elimination half-life of roughly five hours.

C. Approximately 70% of people with OSA have at least one non-anatomical physiological impairment.

D. An observational study linked at least two OSA features with a 73% higher incidence of first-onset TMD.

E. Opioids can suppress respiratory drive and cause central apnoeas and irregular breathing.

CORRECT ANSWER B

Caffeine blocks adenosine receptors and has an elimination half-life of roughly five hours. Additional revision detail: an adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure) with a half-life of roughly five hours. The other options describe different mechanisms, investigations or findings rather than caffeine. Consult Integrated notebook, page 32, §1.12 for the full context.

REFERENCE Integrated notebook, page 32, §1.12

QUESTION 0054 · §1.2 · Clinical interpretation SBA

A 63-year-old with recurrent nocturnal awakenings is being assessed for disordered breathing. The clinical team records the following finding: GABA and galanin from the ventrolateral preoptic area. Which concept best explains this observation?

A. Process S

B. Wake-promoting

C. NREM-promoting

D. Process C (circadian)

E. N3 (old stages 3+4)

CORRECT ANSWER C

This finding most directly describes NREM-promoting: GABA and galanin from the ventrolateral preoptic area. Distractor distinctions: A refers instead to homeostatic sleep drive. Sleep pressure accumulates with time awake (adenosine accumulation is the principal candidate mediat; B refers instead to orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), a; D refers instead to endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract; E refers instead to ~13–23%, 20–40 min first cycle. High-voltage slow delta; highest arousal threshold of all NREM stages; concentrated in the fi.

REFERENCE Integrated notebook, page 9, §1.2

QUESTION 0055 · §1.1 · Applied clinical SBA

Which of the following statements about biochemical refreshment is most accurate in dental sleep medicine?

A. Sleep supports synaptic regulation and glymphatic clearance of metabolic by-products.

B. Memory consolidation includes declarative in NREM (sleep spindles), procedural and emotional in REM.

C. Growth hormone release increases during early-night slow-wave sleep.

D. Sleep supports protein synthesis, tissue repair and restorative anabolic processes.

E. Emotional regulation includes rEM-dependent processing of affect.

CORRECT ANSWER A

Sleep supports synaptic regulation and glymphatic clearance of metabolic by-products. Additional revision detail: synaptic and neuronal function; glial-mediated glymphatic clearance of metabolites; glycogen restoration; neurogenesis. The other options describe different mechanisms, investigations or findings rather than biochemical refreshment. Consult Integrated notebook, page 9, §1.1 for the full context.

REFERENCE Integrated notebook, page 9, §1.1

QUESTION 0056 · §1.3 · Clinical interpretation SBA

An adult with irregular shifts reports fatigue and reduced daytime concentration. The clinical team records the following finding: CPAP withdrawn for 5 breaths, repeated 5–10×. Peak inspiratory flow as % of CPAP-supported baseline; PIF ≥30% = only mildly collapsible airway; higher PIF = more negative Pcrit. Which concept best explains this observation?

A. Obesity

B. CPAP dial-down

C. Soft tissue

D. Craniofacial

E. Awake upper airway collapsibility index

CORRECT ANSWER B

This finding most directly describes CPAP dial-down: CPAP withdrawn for 5 breaths, repeated 5–10×. Peak inspiratory flow as % of CPAP-supported baseline; PIF ≥30% = only mildly collapsible airway; higher PIF = more negative Pcrit. Distractor distinctions: A refers instead to peripharyngeal fat mechanically loads the airway, offsetting dilator activity most markedly in sleep; C refers instead to macroglossia, elongated soft palate, enlarged uvula, adenotonsillar hypertrophy (dominant in children), parapharyngeal fat pa; D refers instead to retrognathia, micrognathia, maxillary constriction with high-arched/narrow palate, steep mandibular plane angle, excessive ve; E refers instead to choanal and epiglottic pressure catheters during negative-pressure inspiration; % collapsibility = (choanal − epiglottic)/cho.

REFERENCE Integrated notebook, page 11, §1.3

QUESTION 0057 · §1.2 · Applied clinical SBA

Which of the following statements about NREM-promoting is most accurate in dental sleep medicine?

A. N3 (old stages 3+4): ~13–23%, 20–40 min first cycle.

B. Loss of pharyngeal dilator tone during REM sleep contributes to REM-predominant obstructive events.

C. The suprachiasmatic nucleus regulates circadian timing and is entrained mainly by light.

D. Wakefulness is supported by orexin, monoaminergic pathways and other arousal-promoting transmitters.

E. GABA and galanin from the ventrolateral preoptic area promote NREM sleep.

CORRECT ANSWER E

GABA and galanin from the ventrolateral preoptic area promote NREM sleep. Additional revision detail: GABA and galanin from the ventrolateral preoptic area. The other options describe different mechanisms, investigations or findings rather than nrem-promoting. Consult Integrated notebook, page 9, §1.2 for the full context.

REFERENCE Integrated notebook, page 9, §1.2

QUESTION 0058 · §1.4 · Clinical interpretation SBA

A patient referred from a dental clinic reports fragmented sleep and morning fatigue. The clinical team records the following finding: AHI>5 — 9% women / 24% men; AHI>5 with sleepiness — 2% / 4%; AHI>15 — 4% / 9% (Wisconsin 1993). Which concept best explains this observation?

A. Others

B. Nasal EPAP

C. Predictive comorbidities

D. Prevalence

E. Gender

CORRECT ANSWER D

This finding most directly describes Prevalence: AHI>5 — 9% women / 24% men; AHI>5 with sleepiness — 2% / 4%; AHI>15 — 4% / 9% (Wisconsin 1993). Distractor distinctions: A refers instead to narrowed airway, craniofacial abnormality, large neck circumference, large tongue, family history, smoking, alcohol/ sedative; B refers instead to one-way resistor valves creating positive end-expiratory pressure to stabilise the airway; overall improvement but less than; C refers instead to obesity, hypertension, cardiovascular disease (especially ischaemic heart disease), type 2 diabetes. Long-term: neurocognitiv; E refers instead to ~2:1–3:1 male predominance; the gap narrows with age and after the menopause — loss of oestrogen's protective effect plus pos.

REFERENCE Integrated notebook, page 11, §1.4

QUESTION 0059 · §1.3 · Applied clinical SBA

Which of the following statements about CPAP dial-down is most accurate in dental sleep medicine?

A. CPAP dial-down includes CPAP withdrawn for 5 breaths, repeated 5–10×.

B. Enlargement of the tongue, soft palate or tonsils can narrow the pharyngeal airway.

C. The awake upper-airway collapsibility index uses choanal and epiglottic pressure measurements.

D. Adenotonsillar enlargement predominates in younger children; excess weight becomes more important in adolescence.

E. Peripharyngeal adipose tissue increases the mechanical load on the upper airway during sleep.

CORRECT ANSWER A

CPAP dial-down includes CPAP withdrawn for 5 breaths, repeated 5–10×. Additional revision detail: CPAP withdrawn for 5 breaths, repeated 5–10×. Peak inspiratory flow as % of CPAP-supported baseline; PIF ≥30% = only mildly collapsible airway; higher PIF = more negative Pcrit. The other options describe different mechanisms, investigations or findings rather than cpap dial-down. Consult Integrated notebook, page 11, §1.3 for the full context.

REFERENCE Integrated notebook, page 11, §1.3

QUESTION 0060 · §1.5 · Clinical interpretation SBA

A 48-year-old with snoring and unrefreshing sleep asks about the underlying sleep physiology. The clinical team records the following finding: adenotonsillar hypertrophy predominates in younger children (peak 2–8 years, when tonsillar tissue is proportionally largest), while overweight becomes the major factor in ad- olescence. Gender. Which concept best explains this observation?

A. Positional therapy in supine-dominant snorers

B. Women more often present atypically

C. SDB is substantially more prevalent in men

D. Smoking

E. In children the dominant risk factor shifts with age

CORRECT ANSWER E

This finding most directly describes In children the dominant risk factor shifts with age: adenotonsillar hypertrophy predominates in younger children (peak 2–8 years, when tonsillar tissue is proportionally largest), while overweight becomes the major factor in ad- olescence. Gender. Distractor distinctions: A refers instead to physical barrier or a position-sensing vibratory device. Reduces snoring frequency and rate in most non-apnoeic snorers (AHI<; B refers instead to insomnia, fatigue, mood disturbance rather than classic witnessed apnoea — and are therefore under-diagnosed. This is a good; C refers instead to roughly a 2:1 to 3:1 ratio at any given threshold; D refers instead to chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS, neurological disease.

REFERENCE Integrated notebook, page 24, §1.5

QUESTION 0061 · §1.4 · Applied clinical SBA

A 48-year-old with snoring and unrefreshing sleep asks about the underlying sleep physiology. Which of the following statements about Prevalence is most accurate?

A. Central disorders of hypersomnolence includes narcolepsy type 1 — hypocretin/orexin deficiency with cataplexy; type 2 without.

B. The original Wisconsin cohort reported AHI above 5/h in 9% of women and 24% of men.

C. Opioids can suppress respiratory drive and cause central apnoeas and irregular breathing.

D. OSA is associated with obesity, hypertension, cardiovascular disease and type 2 diabetes.

E. OSA is more common in men, although the sex difference narrows after menopause.

CORRECT ANSWER B

The original Wisconsin cohort reported AHI above 5/h in 9% of women and 24% of men. Additional revision detail: AHI>5 — 9% women / 24% men; AHI>5 with sleepiness — 2% / 4%; AHI>15 — 4% / 9% (Wisconsin 1993). The other options describe different mechanisms, investigations or findings rather than prevalence. Consult Integrated notebook, page 11, §1.4 for the full context.

REFERENCE Integrated notebook, page 11, §1.4

QUESTION 0062 · §1.9 · Clinical interpretation SBA

A 63-year-old with recurrent nocturnal awakenings is being assessed for disordered breathing. The clinical team records the following finding: 33–50% of the adult population; about one third report dissatisfaction with sleep. Which concept best explains this observation?

A. Risk factors

B. Short-term insomnia disorder

C. Insomnia symptoms

D. Effect on outcomes

E. Chronic insomnia disorder

CORRECT ANSWER C

This finding most directly describes Insomnia symptoms: 33–50% of the adult population; about one third report dissatisfaction with sleep. Distractor distinctions: A refers instead to increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status; B refers instead to clinically significant dissatisfaction or impairment failing those frequency/duration criteria. Other insomnia disorder — rar; D refers instead to marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over; E refers instead to sleep-onset and/or sleep-maintenance complaints with associated daytime impairment, exceeding minimal frequency and duration.

REFERENCE Integrated notebook, page 27, §1.9

QUESTION 0063 · §1.5 · Applied clinical SBA

Which of the following statements about in children the dominant risk factor shifts with age is most accurate in dental sleep medicine?

A. Enlargement of the tongue, soft palate or tonsils can narrow the pharyngeal airway.

B. Substantially more prevalent in men is roughly a 2:1 to 3:1 ratio at any given threshold.

C. Women with OSA may present with fatigue, insomnia or mood symptoms rather than classic witnessed apnoeas.

D. Adenotonsillar enlargement predominates in younger children; excess weight becomes more important in adolescence.

E. Smoking includes chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS.

CORRECT ANSWER D

Adenotonsillar enlargement predominates in younger children; excess weight becomes more important in adolescence. Additional revision detail: adenotonsillar hypertrophy predominates in younger children (peak 2–8 years, when tonsillar tissue is proportionally largest), while overweight becomes the major factor in ad- olescence. Gender. The other options describe different mechanisms, investigations or findings rather than in children the dominant risk factor shifts with age. Consult Integrated notebook, page 24, §1.5 for the full context.

REFERENCE Integrated notebook, page 24, §1.5

QUESTION 0064 · §1.11 · Clinical interpretation SBA

A 48-year-old with snoring and unrefreshing sleep asks about the underlying sleep physiology. The clinical team records the following finding: Restless legs syndrome (Willis–Ekbom) — urge to move with creeping/crawling sensations, worse in the evening and at rest, relieved by movement; associated with iron deficiency and dopaminergic dysfunction. Which concept best explains this observation?

A. Central disorders of hypersomnolence

B. Physical restoration and repair

C. Circadian rhythm sleep–wake disorders

D. Women more often present atypically

E. Sleep-related movement disorders

CORRECT ANSWER E

This finding most directly describes Sleep-related movement disorders: Restless legs syndrome (Willis–Ekbom) — urge to move with creeping/crawling sensations, worse in the evening and at rest, relieved by movement; associated with iron deficiency and dopaminergic dysfunction. Distractor distinctions: A refers instead to Narcolepsy type 1 — hypocretin/orexin deficiency with cataplexy; type 2 without; B refers instead to protein synthesis, cell and tissue repair, anabolic dominance; C refers instead to Delayed phase (adolescents), advanced phase (older adults), irregular and non-24-hour rhythms, shift work disorder, jet lag; D refers instead to insomnia, fatigue, mood disturbance rather than classic witnessed apnoea — and are therefore under-diagnosed. This is a good.

REFERENCE Integrated notebook, page 14, §1.11

QUESTION 0065 · §1.9 · Applied clinical SBA

Which of the following statements about insomnia symptoms is most accurate in dental sleep medicine?

A. Insomnia symptoms are reported by roughly 33–50% of adults, with lower rates of chronic disorder.

B. Insomnia symptoms: 33–50% of the adult population; about one third report dissatisfaction with sleep.

C. Cognitive behavioural therapy for insomnia is first-line treatment for chronic insomnia.

D. CBT for insomnia can improve sleep latency, wakefulness after sleep onset and sleep efficiency.

E. Insomnia risk is increased by age, comorbidity and disrupted work or sleep schedules.

CORRECT ANSWER B

Insomnia symptoms: 33–50% of the adult population; about one third report dissatisfaction with sleep. Additional revision detail: 33–50% of the adult population; about one third report dissatisfaction with sleep. The other options describe different mechanisms, investigations or findings rather than insomnia symptoms. Consult Integrated notebook, page 27, §1.9 for the full context.

REFERENCE Integrated notebook, page 27, §1.9

QUESTION 0066 · §1.12 · Clinical interpretation SBA

A 51-year-old with witnessed respiratory pauses has a consultation with a sleep specialist. The clinical team records the following finding: ~70% have ≥1; loop gain ~36%, low arousal threshold ~37%, poor muscle responsiveness ~36%; Pcrit distribution ~19% / 58% / 23%. Which concept best explains this observation?

A. Caffeine

B. OSA prevalence

C. OSA and TMD

D. Non-anatomical traits

E. Insomnia

CORRECT ANSWER D

This finding most directly describes Non-anatomical traits: ~70% have ≥1; loop gain ~36%, low arousal threshold ~37%, poor muscle responsiveness ~36%; Pcrit distribution ~19% / 58% / 23%. Distractor distinctions: A refers instead to an adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure) with a half-life of roughly five hours; B refers instead to AHI>5 — 9% F / 24% M (Wisconsin 1993); moderate–severe — 6% F / 13% M (2007–2010); global 936 million / 425 million moderate–; C refers instead to ≥2 OSA signs/symptoms → 73% greater incidence of first-onset TMD; chronic TMD >3× more frequent in high-OSA-likelihood adults; E refers instead to symptoms 33–50% · disorder 10–15% · chronic 5–15% · comorbid 50–75% · chronic = ≥3 nights/week × ≥3 months.

REFERENCE Integrated notebook, page 33, §1.12

QUESTION 0067 · §1.10 · Applied clinical SBA

Which of the following statements about aetiologies is most accurate in dental sleep medicine?

A. Complications includes frequent nocturnal awakenings, excessive daytime sleepiness and increased risk of adverse cardiovascular.

B. Hypoventilation syndromes includes insufficient alveolar ventilation causing CO2 retention and hypoxaemia.

C. Low positive airway pressure may suppress uncomplicated snoring, but physician assessment is still needed.

D. High altitude can destabilise ventilatory control and cause central periodic breathing.

E. Catathrenia presents as prolonged, often monotone groaning during expiration in sleep.

CORRECT ANSWER D

High altitude can destabilise ventilatory control and cause central periodic breathing. Additional revision detail: high-altitude periodic breathing (which most healthy people develop given sufficient alveolar hypoxia). The other options describe different mechanisms, investigations or findings rather than aetiologies. Consult Integrated notebook, page 13, §1.10 for the full context.

REFERENCE Integrated notebook, page 13, §1.10

QUESTION 0068 · §1.1 · Clinical interpretation SBA

An adult with irregular shifts reports fatigue and reduced daytime concentration. The clinical team records the following finding: growth hormone in early SWS; prolactin, testosterone, PTH; thyroid hormone late evening; melatonin under light–dark control. Which concept best explains this observation?

A. Hormone regulation

B. Physical restoration and repair

C. Memory consolidation

D. Biochemical refreshment

E. Emotional regulation

CORRECT ANSWER A

This finding most directly describes Hormone regulation: growth hormone in early SWS; prolactin, testosterone, PTH; thyroid hormone late evening; melatonin under light–dark control. Distractor distinctions: B refers instead to protein synthesis, cell and tissue repair, anabolic dominance; C refers instead to declarative in NREM (sleep spindles), procedural and emotional in REM; D refers instead to synaptic and neuronal function; glial-mediated glymphatic clearance of metabolites; glycogen restoration; neurogenesis; E refers instead to REM-dependent processing of affect.

REFERENCE Integrated notebook, page 9, §1.1

QUESTION 0069 · §1.11 · Applied clinical SBA

A 48-year-old with snoring and unrefreshing sleep asks about the underlying sleep physiology. Which of the following statements about sleep-related movement disorders is most accurate?

A. Chronic insomnia involves difficulty initiating or maintaining sleep with daytime impairment over an extended period.

B. Approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5).

C. Central disorders of hypersomnolence includes narcolepsy type 1 — hypocretin/orexin deficiency with cataplexy; type 2 without.

D. Circadian rhythm disorders include delayed and advanced sleep phase and shift-work disturbance.

E. Restless legs syndrome causes an urge to move the limbs, especially during evening rest.

CORRECT ANSWER E

Restless legs syndrome causes an urge to move the limbs, especially during evening rest. Additional revision detail: Restless legs syndrome (Willis–Ekbom) — urge to move with creeping/crawling sensations, worse in the evening and at rest, relieved by movement; associated with iron deficiency and dopaminergic dysfunction. The other options describe different mechanisms, investigations or findings rather than sleep-related movement disorders. Consult Integrated notebook, page 14, §1.11 for the full context.

REFERENCE Integrated notebook, page 14, §1.11

QUESTION 0070 · §1.3 · Focused knowledge SBA

Which of the following statements about obesity is most accurate in dental sleep medicine?

A. High altitude can destabilise ventilatory control and cause central periodic breathing.

B. Enlargement of the tongue, soft palate or tonsils can narrow the pharyngeal airway.

C. CPAP dial-down includes CPAP withdrawn for 5 breaths, repeated 5–10×.

D. Peripharyngeal adipose tissue increases the mechanical load on the upper airway during sleep.

E. The awake upper-airway collapsibility index uses choanal and epiglottic pressure measurements.

CORRECT ANSWER D

Peripharyngeal adipose tissue increases the mechanical load on the upper airway during sleep. Additional revision detail: peripharyngeal fat mechanically loads the airway, offsetting dilator activity most markedly in sleep. The other options describe different mechanisms, investigations or findings rather than obesity. Consult Integrated notebook, page 11, §1.3 for the full context.

REFERENCE Integrated notebook, page 11, §1.3

QUESTION 0071 · §1.3 · Applied clinical SBA

Which of the following statements about non-anatomical traits is most accurate in dental sleep medicine?

A. Caffeine blocks adenosine receptors and has an elimination half-life of roughly five hours.

B. Insomnia symptoms affect approximately one-third to one-half of adults; chronic insomnia is less common.

C. An observational study linked at least two OSA features with a 73% higher incidence of first-onset TMD.

D. Historical population data reported AHI above 5/h in about 9% of women and 24% of men.

E. Approximately 70% of people with OSA have at least one non-anatomical physiological impairment.

CORRECT ANSWER E

The best answer is the presence of at least one non-anatomical impairment in approximately 70% of people with OSA. The source notebook describes high loop gain (~36%), a low respiratory arousal threshold (~37%) and poor pharyngeal muscle responsiveness (~36%) as key non-anatomical traits. Pcrit instead assesses passive upper-airway collapsibility and is the anatomical element of the PALM framework. The other statements concern unrelated drug effects, epidemiology and TMD, so they do not explain the OSA endotype finding.

REFERENCE Integrated notebook, Topic 1, §1.3 (page 11)

QUESTION 0072 · §1.4 · Focused knowledge SBA

Which of the following statements about additional predisposing factors is most accurate in dental sleep medicine?

A. OSA is associated with obesity, hypertension, cardiovascular disease and type 2 diabetes.

B. Alcohol may shorten sleep onset but subsequently fragments sleep and worsens upper-airway obstruction.

C. OSA is more common in men, although the sex difference narrows after menopause.

D. The original Wisconsin cohort reported AHI above 5/h in 9% of women and 24% of men.

E. OSA risk factors include a narrow airway, craniofacial restriction, large neck circumference and sedative exposure.

CORRECT ANSWER E

OSA risk factors include a narrow airway, craniofacial restriction, large neck circumference and sedative exposure. Additional revision detail: narrow upper airway, abnormal craniofacial structure, large neck and tongue, family history, tobacco, alcohol, sedatives or opioids, nasal obstruction, supine posture, and relevant endocrine or cardiometabolic disease. The other options describe different mechanisms, investigations or findings rather than others. Consult Integrated notebook, page 11, §1.4 for the full context.

REFERENCE Integrated notebook, page 11, §1.4

QUESTION 0073 · §1.1 · Applied clinical SBA

Which of the following statements about hormone regulation is most accurate in dental sleep medicine?

A. Sleep supports protein synthesis, tissue repair and restorative anabolic processes.

B. Growth hormone release increases during early-night slow-wave sleep.

C. Memory consolidation includes declarative in NREM (sleep spindles), procedural and emotional in REM.

D. Emotional regulation includes rEM-dependent processing of affect.

E. Sleep supports synaptic regulation and glymphatic clearance of metabolic by-products.

CORRECT ANSWER B

Growth hormone release increases during early-night slow-wave sleep. Additional revision detail: growth hormone in early SWS; prolactin, testosterone, PTH; thyroid hormone late evening; melatonin under light–dark control. The other options describe different mechanisms, investigations or findings rather than hormone regulation. Consult Integrated notebook, page 9, §1.1 for the full context.

REFERENCE Integrated notebook, page 9, §1.1

QUESTION 0074 · §1.5 · Focused knowledge SBA

Which of the following statements about smoking is most accurate in dental sleep medicine?

A. Adenotonsillar enlargement predominates in younger children; excess weight becomes more important in adolescence.

B. Smoking includes chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS.

C. Women with OSA may present with fatigue, insomnia or mood symptoms rather than classic witnessed apnoeas.

D. Septoplasty with turbinate reduction only where there are both symptoms and objective nasendoscopic evidence of obstruction.

E. Substantially more prevalent in men is roughly a 2:1 to 3:1 ratio at any given threshold.

CORRECT ANSWER B

Smoking includes chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS. Additional revision detail: chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS, neurological disease. The other options describe different mechanisms, investigations or findings rather than smoking. Consult Integrated notebook, page 24, §1.5 for the full context.

REFERENCE Integrated notebook, page 24, §1.5

QUESTION 0075 · §1.3 · Applied clinical SBA

Which of the following statements about obesity is most accurate in dental sleep medicine?

A. Peripharyngeal adipose tissue increases the mechanical load on the upper airway during sleep.

B. Enlargement of the tongue, soft palate or tonsils can narrow the pharyngeal airway.

C. High altitude can destabilise ventilatory control and cause central periodic breathing.

D. CPAP dial-down includes CPAP withdrawn for 5 breaths, repeated 5–10×.

E. The awake upper-airway collapsibility index uses choanal and epiglottic pressure measurements.

CORRECT ANSWER A

Peripharyngeal adipose tissue increases the mechanical load on the upper airway during sleep. Additional revision detail: peripharyngeal fat mechanically loads the airway, offsetting dilator activity most markedly in sleep. The other options describe different mechanisms, investigations or findings rather than obesity. Consult Integrated notebook, page 11, §1.3 for the full context.

REFERENCE Integrated notebook, page 11, §1.3

QUESTION 0076 · §1.12 · Clinical interpretation SBA

A 51-year-old with witnessed respiratory pauses has a consultation with a sleep specialist. The clinical team records the following finding: symptoms 33–50% · disorder 10–15% · chronic 5–15% · comorbid 50–75% · chronic = ≥3 nights/week × ≥3 months. Which concept best explains this observation?

A. Nicotine

B. OSA–TMD

C. Alcohol

D. Insomnia

E. Caffeine

CORRECT ANSWER D

This finding most directly describes Insomnia: symptoms 33–50% · disorder 10–15% · chronic 5–15% · comorbid 50–75% · chronic = ≥3 nights/week × ≥3 months. Distractor distinctions: A refers instead to stimulant plus nocturnal withdrawal arousals; smoking also causes upper airway inflammation; B refers instead to ≥2 signs/symptoms → 73% greater first-onset TMD incidence; chronic TMD >3× more frequent; C refers instead to reduces sleep latency but suppresses REM in the first half of the night with rebound in the second; fragments sleep; reduces; E refers instead to an adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure) with a half-life of roughly five hours.

REFERENCE Integrated notebook, page 16, §1.12

QUESTION 0077 · §1.4 · Applied clinical SBA

Which of the following statements about additional predisposing factors is most accurate in dental sleep medicine?

A. Alcohol may shorten sleep onset but subsequently fragments sleep and worsens upper-airway obstruction.

B. OSA is associated with obesity, hypertension, cardiovascular disease and type 2 diabetes.

C. OSA is more common in men, although the sex difference narrows after menopause.

D. The original Wisconsin cohort reported AHI above 5/h in 9% of women and 24% of men.

E. OSA risk factors include a narrow airway, craniofacial restriction, large neck circumference and sedative exposure.

CORRECT ANSWER E

OSA risk factors include a narrow airway, craniofacial restriction, large neck circumference and sedative exposure. Additional revision detail: narrow upper airway, abnormal craniofacial structure, large neck and tongue, family history, tobacco, alcohol, sedatives or opioids, nasal obstruction, supine posture, and relevant endocrine or cardiometabolic disease. The other options describe different mechanisms, investigations or findings rather than others. Consult Integrated notebook, page 11, §1.4 for the full context.

REFERENCE Integrated notebook, page 11, §1.4

QUESTION 0078 · §1.1 · Clinical interpretation SBA

An adult with irregular shifts reports fatigue and reduced daytime concentration. The clinical team records the following finding: REM-dependent processing of affect. Which concept best explains this observation?

A. Physical restoration and repair

B. Biochemical refreshment

C. Emotional regulation

D. Hormone regulation

E. Memory consolidation

CORRECT ANSWER C

This finding most directly describes Emotional regulation: REM-dependent processing of affect. Distractor distinctions: A refers instead to protein synthesis, cell and tissue repair, anabolic dominance; B refers instead to synaptic and neuronal function; glial-mediated glymphatic clearance of metabolites; glycogen restoration; neurogenesis; D refers instead to growth hormone in early SWS; prolactin, testosterone, PTH; thyroid hormone late evening; melatonin under light–dark control; E refers instead to declarative in NREM (sleep spindles), procedural and emotional in REM.

REFERENCE Integrated notebook, page 9, §1.1

QUESTION 0079 · §1.5 · Applied clinical SBA

Which of the following statements about smoking is most accurate in dental sleep medicine?

A. Septoplasty with turbinate reduction only where there are both symptoms and objective nasendoscopic evidence of obstruction.

B. Women with OSA may present with fatigue, insomnia or mood symptoms rather than classic witnessed apnoeas.

C. Smoking includes chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS.

D. Substantially more prevalent in men is roughly a 2:1 to 3:1 ratio at any given threshold.

E. Adenotonsillar enlargement predominates in younger children; excess weight becomes more important in adolescence.

CORRECT ANSWER C

Smoking includes chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS. Additional revision detail: chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS, neurological disease. The other options describe different mechanisms, investigations or findings rather than smoking. Consult Integrated notebook, page 24, §1.5 for the full context.

REFERENCE Integrated notebook, page 24, §1.5

QUESTION 0080 · §1.2 · Clinical interpretation SBA

A patient referred from a dental clinic reports fragmented sleep and morning fatigue. The clinical team records the following finding: orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), acetylcholine, dopamine. Which concept best explains this observation?

A. Wake-promoting

B. N3 (old stages 3+4)

C. Process C (circadian)

D. Process S

E. NREM-promoting

CORRECT ANSWER A

This finding most directly describes Wake-promoting: orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), acetylcholine, dopamine. Distractor distinctions: B refers instead to ~13–23%, 20–40 min first cycle. High-voltage slow delta; highest arousal threshold of all NREM stages; concentrated in the fi; C refers instead to endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract; D refers instead to homeostatic sleep drive. Sleep pressure accumulates with time awake (adenosine accumulation is the principal candidate mediat; E refers instead to GABA and galanin from the ventrolateral preoptic area.

REFERENCE Integrated notebook, page 9, §1.2

QUESTION 0081 · §1.10 · Applied clinical SBA

Which of the following statements about CPAP in those already committed or willing is most accurate in dental sleep medicine?

A. Sleep-related hypoxaemia produces sustained oxygen desaturation without an equivalent rise in carbon dioxide.

B. Low positive airway pressure may suppress uncomplicated snoring, but physician assessment is still needed.

C. Catathrenia presents as prolonged, often monotone groaning during expiration in sleep.

D. Hypoventilation syndromes includes insufficient alveolar ventilation causing CO2 retention and hypoxaemia.

E. Complementary includes nasal valve dilators, lubricating sprays, herbal remedies.

CORRECT ANSWER B

Low positive airway pressure may suppress uncomplicated snoring, but physician assessment is still needed. Additional revision detail: effective and safe, compliance highly variable; 4–6 cmH2O eliminates snoring. The other options describe different mechanisms, investigations or findings rather than cpap in those already committed or willing. Consult Integrated notebook, page 14, §1.10 for the full context.

REFERENCE Integrated notebook, page 14, §1.10

QUESTION 0082 · §1.3 · Clinical interpretation SBA

A 48-year-old with snoring and unrefreshing sleep asks about the underlying sleep physiology. The clinical team records the following finding: macroglossia, elongated soft palate, enlarged uvula, adenotonsillar hypertrophy (dominant in children), parapharyngeal fat pads, large mandibular tori, lateral pharyngeal wall thickening. Which concept best explains this observation?

A. CPAP dial-down

B. Craniofacial

C. Obesity

D. Soft tissue

E. Awake upper airway collapsibility index

CORRECT ANSWER D

This finding most directly describes Soft tissue: macroglossia, elongated soft palate, enlarged uvula, adenotonsillar hypertrophy (dominant in children), parapharyngeal fat pads, large mandibular tori, lateral pharyngeal wall thickening. Distractor distinctions: A refers instead to CPAP withdrawn for 5 breaths, repeated 5–10×. Peak inspiratory flow as % of CPAP-supported baseline; PIF ≥30% = only mildly c; B refers instead to retrognathia, micrognathia, maxillary constriction with high-arched/narrow palate, steep mandibular plane angle, excessive ve; C refers instead to peripharyngeal fat mechanically loads the airway, offsetting dilator activity most markedly in sleep; E refers instead to choanal and epiglottic pressure catheters during negative-pressure inspiration; % collapsibility = (choanal − epiglottic)/cho.

REFERENCE Integrated notebook, page 10, §1.3

QUESTION 0083 · §1.12 · Applied clinical SBA

A 48-year-old with snoring and unrefreshing sleep asks about the underlying sleep physiology. Which of the following statements about insomnia is most accurate?

A. An observational study linked at least two OSA features with a 73% higher incidence of first-onset TMD.

B. Opioids can suppress respiratory drive and cause central apnoeas and irregular breathing.

C. Insomnia symptoms affect approximately one-third to one-half of adults; chronic insomnia is less common.

D. At least two OSA symptoms were associated with a 73% higher incidence of first-onset TMD.

E. Caffeine blocks adenosine receptors and has an elimination half-life of roughly five hours.

CORRECT ANSWER C

Insomnia symptoms affect approximately one-third to one-half of adults; chronic insomnia is less common. Additional revision detail: symptoms 33–50% · disorder 10–15% · chronic 5–15% · comorbid 50–75% · chronic = ≥3 nights/week × ≥3 months. The other options describe different mechanisms, investigations or findings rather than insomnia. Consult Integrated notebook, page 16, §1.12 for the full context.

REFERENCE Integrated notebook, page 16, §1.12

QUESTION 0084 · §1.9 · Clinical interpretation SBA

A 51-year-old with witnessed respiratory pauses has a consultation with a sleep specialist. The clinical team records the following finding: marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over time. Which concept best explains this observation?

A. Risk factors

B. Insomnia symptoms

C. Prevalence

D. Effect on outcomes

E. Chronic insomnia disorder

CORRECT ANSWER D

This finding most directly describes Effect on outcomes: marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over time. Distractor distinctions: A refers instead to increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status; B refers instead to 33–50% of the adult population; about one third report dissatisfaction with sleep; C refers instead to symptoms 33–50%; ~⅓ report dissatisfaction with sleep; with distress/impairment 10–15%; chronic 5–15%; with medical/psychiatr; E refers instead to sleep-onset and/or sleep-maintenance complaints with associated daytime impairment, exceeding minimal frequency and duration.

REFERENCE Integrated notebook, page 13, §1.9

QUESTION 0085 · §1.1 · Applied clinical SBA

Which of the following statements about emotional regulation is most accurate in dental sleep medicine?

A. Sleep supports protein synthesis, tissue repair and restorative anabolic processes.

B. Growth hormone release increases during early-night slow-wave sleep.

C. Sleep supports synaptic regulation and glymphatic clearance of metabolic by-products.

D. Memory consolidation includes declarative in NREM (sleep spindles), procedural and emotional in REM.

E. Emotional regulation includes rEM-dependent processing of affect.

CORRECT ANSWER E

Emotional regulation includes rEM-dependent processing of affect. Additional revision detail: REM-dependent processing of affect. The other options describe different mechanisms, investigations or findings rather than emotional regulation. Consult Integrated notebook, page 9, §1.1 for the full context.

REFERENCE Integrated notebook, page 9, §1.1

QUESTION 0086 · §1.10 · Clinical interpretation SBA

A 63-year-old with recurrent nocturnal awakenings is being assessed for disordered breathing. The clinical team records the following finding: heavy snoring (>50% of sleep time) may induce endothelial injury at the carotid bifurcation given its proximity to the hypopharyngeal walls → increased intima-media thickness, carotid atherosclerosis, metabolic syndrome. Which concept best explains this observation?

A. Aetiologies

B. Not benign

C. Complementary

D. Nasal EPAP

E. Catathrenia

CORRECT ANSWER B

This finding most directly describes Not benign: heavy snoring (>50% of sleep time) may induce endothelial injury at the carotid bifurcation given its proximity to the hypopharyngeal walls → increased intima-media thickness, carotid atherosclerosis, metabolic syndrome. Distractor distinctions: A refers instead to high-altitude periodic breathing (which most healthy people develop given sufficient alveolar hypoxia); C refers instead to nasal valve dilators, lubricating sprays, herbal remedies; D refers instead to one-way resistor valves creating positive end-expiratory pressure to stabilise the airway; overall improvement but less than; E refers instead to slow, monotone, groaning expiratory vocalisation; benign, often mistaken for a parasomnia.

REFERENCE Integrated notebook, page 14, §1.10

QUESTION 0087 · §1.2 · Applied clinical SBA

Which of the following statements about wake-promoting is most accurate in dental sleep medicine?

A. Wakefulness is supported by orexin, monoaminergic pathways and other arousal-promoting transmitters.

B. Loss of pharyngeal dilator tone during REM sleep contributes to REM-predominant obstructive events.

C. Homeostatic sleep pressure increases with time awake and dissipates during sleep.

D. The suprachiasmatic nucleus regulates circadian timing and is entrained mainly by light.

E. N3 (old stages 3+4): ~13–23%, 20–40 min first cycle.

CORRECT ANSWER A

Wakefulness is supported by orexin, monoaminergic pathways and other arousal-promoting transmitters. Additional revision detail: orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), acetylcholine, dopamine. The other options describe different mechanisms, investigations or findings rather than wake-promoting. Consult Integrated notebook, page 9, §1.2 for the full context.

REFERENCE Integrated notebook, page 9, §1.2

QUESTION 0088 · §1.12 · Focused knowledge SBA

A 48-year-old with snoring and unrefreshing sleep asks about the underlying sleep physiology. Which of the following statements about OSA–TMD is most accurate?

A. An observational study linked at least two OSA features with a 73% higher incidence of first-onset TMD.

B. Opioids can suppress respiratory drive and cause central apnoeas and irregular breathing.

C. Insomnia symptoms affect approximately one-third to one-half of adults; chronic insomnia is less common.

D. At least two OSA symptoms were associated with a 73% higher incidence of first-onset TMD.

E. Nicotine stimulates wakefulness, while overnight withdrawal can fragment sleep.

CORRECT ANSWER D

At least two OSA symptoms were associated with a 73% higher incidence of first-onset TMD. Additional revision detail: ≥2 signs/symptoms → 73% greater first-onset TMD incidence; chronic TMD >3× more frequent. The other options describe different mechanisms, investigations or findings rather than osa–tmd. Consult Integrated notebook, page 16, §1.12 for the full context.

REFERENCE Integrated notebook, page 16, §1.12

QUESTION 0089 · §1.3 · Applied clinical SBA

Which of the following statements about soft tissue is most accurate in dental sleep medicine?

A. Adenotonsillar enlargement predominates in younger children; excess weight becomes more important in adolescence.

B. CPAP dial-down includes CPAP withdrawn for 5 breaths, repeated 5–10×.

C. Peripharyngeal adipose tissue increases the mechanical load on the upper airway during sleep.

D. Enlargement of the tongue, soft palate or tonsils can narrow the pharyngeal airway.

E. The awake upper-airway collapsibility index uses choanal and epiglottic pressure measurements.

CORRECT ANSWER D

Enlargement of the tongue, soft palate or tonsils can narrow the pharyngeal airway. Additional revision detail: macroglossia, elongated soft palate, enlarged uvula, adenotonsillar hypertrophy (dominant in children), parapharyngeal fat pads, large mandibular tori, lateral pharyngeal wall thickening. The other options describe different mechanisms, investigations or findings rather than soft tissue. Consult Integrated notebook, page 10, §1.3 for the full context.

REFERENCE Integrated notebook, page 10, §1.3

QUESTION 0090 · §1.2 · Clinical interpretation SBA

A patient referred from a dental clinic reports fragmented sleep and morning fatigue. The clinical team records the following finding: ~13–23%, 20–40 min first cycle. High-voltage slow delta; highest arousal threshold of all NREM stages; concentrated in the first third of the night; GH pulse here; disorders of arousal arise here. Which concept best explains this observation?

A. Process C (circadian)

B. Wake-promoting

C. NREM-promoting

D. N3 (old stages 3+4)

E. Process S

CORRECT ANSWER D

This finding most directly describes N3 (old stages 3+4): ~13–23%, 20–40 min first cycle. High-voltage slow delta; highest arousal threshold of all NREM stages; concentrated in the first third of the night; GH pulse here; disorders of arousal arise here. Distractor distinctions: A refers instead to endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract; B refers instead to orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), a; C refers instead to GABA and galanin from the ventrolateral preoptic area; E refers instead to homeostatic sleep drive. Sleep pressure accumulates with time awake (adenosine accumulation is the principal candidate mediat.

REFERENCE Integrated notebook, page 9, §1.2

QUESTION 0091 · §1.9 · Applied clinical SBA

A 48-year-old with snoring and unrefreshing sleep asks about the underlying sleep physiology. Which of the following statements about effect on outcomes is most accurate?

A. Insomnia symptoms are reported by roughly 33–50% of adults, with lower rates of chronic disorder.

B. CBT for insomnia can improve sleep latency, wakefulness after sleep onset and sleep efficiency.

C. Insomnia risk is increased by age, comorbidity and disrupted work or sleep schedules.

D. Short-term insomnia produces clinically significant distress but does not meet chronic-duration criteria.

E. Cognitive behavioural therapy for insomnia is first-line treatment for chronic insomnia.

CORRECT ANSWER B

CBT for insomnia can improve sleep latency, wakefulness after sleep onset and sleep efficiency. Additional revision detail: marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over time. The other options describe different mechanisms, investigations or findings rather than effect on outcomes. Consult Integrated notebook, page 13, §1.9 for the full context.

REFERENCE Integrated notebook, page 13, §1.9

QUESTION 0092 · §1.9 · Focused knowledge SBA

Which of the following statements about treatment is most accurate in dental sleep medicine?

A. Cognitive behavioural therapy for insomnia is first-line treatment for chronic insomnia.

B. Insomnia risk is increased by age, comorbidity and disrupted work or sleep schedules.

C. Insomnia symptoms are reported by roughly 33–50% of adults, with lower rates of chronic disorder.

D. CBT for insomnia can improve sleep latency, wakefulness after sleep onset and sleep efficiency.

E. Insomnia symptoms: 33–50% of the adult population; about one third report dissatisfaction with sleep.

CORRECT ANSWER A

Cognitive behavioural therapy for insomnia is first-line treatment for chronic insomnia. Additional revision detail: CBT-I is first line, without tolerance or the adverse effects of pharmacotherapy. The other options describe different mechanisms, investigations or findings rather than treatment. Consult Integrated notebook, page 13, §1.9 for the full context.

REFERENCE Integrated notebook, page 13, §1.9

QUESTION 0093 · §1.10 · Applied clinical SBA

Which of the following statements about not benign is most accurate in dental sleep medicine?

A. Nasal EPAP includes one-way resistor valves creating positive end-expiratory pressure to stabilise the airway.

B. Heavy snoring may be associated with local carotid vascular injury, although causation is uncertain.

C. Hypercapnic central apnoea may reflect impaired brainstem respiratory drive or motor output.

D. Positional therapy in supine-dominant snorers includes physical barrier or a position-sensing vibratory device.

E. Approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5).

CORRECT ANSWER B

Heavy snoring may be associated with local carotid vascular injury, although causation is uncertain. Additional revision detail: heavy snoring (>50% of sleep time) may induce endothelial injury at the carotid bifurcation given its proximity to the hypopharyngeal walls → increased intima-media thickness, carotid atherosclerosis, metabolic syndrome. The other options describe different mechanisms, investigations or findings rather than not benign. Consult Integrated notebook, page 14, §1.10 for the full context.

REFERENCE Integrated notebook, page 14, §1.10

QUESTION 0094 · §1.10 · Clinical interpretation SBA

An adult with irregular shifts reports fatigue and reduced daytime concentration. The clinical team records the following finding: septoplasty with turbinate reduction only where there are both symptoms and objective nasendoscopic evidence of obstruction. Which concept best explains this observation?

A. Nasal surgery

B. Complementary

C. Complications

D. Nasal EPAP

E. Aetiologies

CORRECT ANSWER A

This finding most directly describes Nasal surgery: septoplasty with turbinate reduction only where there are both symptoms and objective nasendoscopic evidence of obstruction. Distractor distinctions: B refers instead to nasal valve dilators, lubricating sprays, herbal remedies; C refers instead to frequent nocturnal awakenings, excessive daytime sleepiness and increased risk of adverse cardiovascular outcomes; D refers instead to one-way resistor valves creating positive end-expiratory pressure to stabilise the airway; overall improvement but less than; E refers instead to high-altitude periodic breathing (which most healthy people develop given sufficient alveolar hypoxia).

REFERENCE Integrated notebook, page 14, §1.10

QUESTION 0095 · §1.12 · Applied clinical SBA

Which of the following statements about OSA–TMD is most accurate in dental sleep medicine?

A. Nicotine stimulates wakefulness, while overnight withdrawal can fragment sleep.

B. Insomnia symptoms affect approximately one-third to one-half of adults; chronic insomnia is less common.

C. An observational study linked at least two OSA features with a 73% higher incidence of first-onset TMD.

D. Opioids can suppress respiratory drive and cause central apnoeas and irregular breathing.

E. At least two OSA symptoms were associated with a 73% higher incidence of first-onset TMD.

CORRECT ANSWER E

At least two OSA symptoms were associated with a 73% higher incidence of first-onset TMD. Additional revision detail: ≥2 signs/symptoms → 73% greater first-onset TMD incidence; chronic TMD >3× more frequent. The other options describe different mechanisms, investigations or findings rather than osa–tmd. Consult Integrated notebook, page 16, §1.12 for the full context.

REFERENCE Integrated notebook, page 16, §1.12

QUESTION 0096 · §1.12 · Focused knowledge SBA

Which of the following statements about opioids is most accurate in dental sleep medicine?

A. An observational study linked at least two OSA features with a 73% higher incidence of first-onset TMD.

B. Opioids can suppress respiratory drive and cause central apnoeas and irregular breathing.

C. Insomnia symptoms affect approximately one-third to one-half of adults; chronic insomnia is less common.

D. Caffeine blocks adenosine receptors and has an elimination half-life of roughly five hours.

E. Nicotine stimulates wakefulness, while overnight withdrawal can fragment sleep.

CORRECT ANSWER B

Opioids can suppress respiratory drive and cause central apnoeas and irregular breathing. Additional revision detail: dose-dependent respiratory depression with central apnoeas and ataxic breathing; a recognised cause of CSA. The other options describe different mechanisms, investigations or findings rather than opioids. Consult Integrated notebook, page 15, §1.12 for the full context.

REFERENCE Integrated notebook, page 15, §1.12

QUESTION 0097 · §1.2 · Applied clinical SBA

Which of the following statements about n3 (old stages 3+4) is most accurate in dental sleep medicine?

A. Loss of pharyngeal dilator tone during REM sleep contributes to REM-predominant obstructive events.

B. Wakefulness is supported by orexin, monoaminergic pathways and other arousal-promoting transmitters.

C. The suprachiasmatic nucleus regulates circadian timing and is entrained mainly by light.

D. N3 (old stages 3+4): ~13–23%, 20–40 min first cycle.

E. Homeostatic sleep pressure increases with time awake and dissipates during sleep.

CORRECT ANSWER D

N3 (old stages 3+4): ~13–23%, 20–40 min first cycle. Additional revision detail: ~13–23%, 20–40 min first cycle. High-voltage slow delta; highest arousal threshold of all NREM stages; concentrated in the first third of the night; GH pulse here; disorders of arousal arise here. The other options describe different mechanisms, investigations or findings rather than n3 (old stages 3+4). Consult Integrated notebook, page 9, §1.2 for the full context.

REFERENCE Integrated notebook, page 9, §1.2

QUESTION 0098 · §1.2 · Clinical interpretation SBA

A 48-year-old with snoring and unrefreshing sleep asks about the underlying sleep physiology. The clinical team records the following finding: endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract. Which concept best explains this observation?

A. Process C (circadian)

B. NREM-promoting

C. Process S

D. Wake-promoting

E. N3 (old stages 3+4)

CORRECT ANSWER A

This finding most directly describes Process C (circadian): endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract. Distractor distinctions: B refers instead to GABA and galanin from the ventrolateral preoptic area; C refers instead to homeostatic sleep drive. Sleep pressure accumulates with time awake (adenosine accumulation is the principal candidate mediat; D refers instead to orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), a; E refers instead to ~13–23%, 20–40 min first cycle. High-voltage slow delta; highest arousal threshold of all NREM stages; concentrated in the fi.

REFERENCE Integrated notebook, page 9, §1.2

QUESTION 0099 · §1.9 · Applied clinical SBA

Which of the following statements about treatment is most accurate in dental sleep medicine?

A. Cognitive behavioural therapy for insomnia is first-line treatment for chronic insomnia.

B. Insomnia symptoms: 33–50% of the adult population; about one third report dissatisfaction with sleep.

C. CBT for insomnia can improve sleep latency, wakefulness after sleep onset and sleep efficiency.

D. Insomnia symptoms are reported by roughly 33–50% of adults, with lower rates of chronic disorder.

E. Insomnia risk is increased by age, comorbidity and disrupted work or sleep schedules.

CORRECT ANSWER A

Cognitive behavioural therapy for insomnia is first-line treatment for chronic insomnia. Additional revision detail: CBT-I is first line, without tolerance or the adverse effects of pharmacotherapy. The other options describe different mechanisms, investigations or findings rather than treatment. Consult Integrated notebook, page 13, §1.9 for the full context.

REFERENCE Integrated notebook, page 13, §1.9

QUESTION 0100 · §1.10 · Clinical interpretation SBA

A 48-year-old with snoring and unrefreshing sleep asks about the underlying sleep physiology. The clinical team records the following finding: pre-phase + adjunctive, adjunctive + mainstream, or multiple mainstream. Higher efficacy, limited evidence. 1.11 Other non-respiratory sleep disorders. Which concept best explains this observation?

A. Relationship to OSA

B. Complications

C. Combination therapy

D. Positional therapy in supine-dominant snorers

E. Complementary

CORRECT ANSWER C

This finding most directly describes Combination therapy: pre-phase + adjunctive, adjunctive + mainstream, or multiple mainstream. Higher efficacy, limited evidence. 1.11 Other non-respiratory sleep disorders. Distractor distinctions: A refers instead to approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5). Snoring there; B refers instead to frequent nocturnal awakenings, excessive daytime sleepiness and increased risk of adverse cardiovascular outcomes; D refers instead to physical barrier or a position-sensing vibratory device. Reduces snoring frequency and rate in most non-apnoeic snorers (AHI<; E refers instead to nasal valve dilators, lubricating sprays, herbal remedies.

REFERENCE Integrated notebook, page 14, §1.10

QUESTION 0101 · §1.10 · Applied clinical SBA

Which of the following statements about nasal surgery is most accurate in dental sleep medicine?

A. Catathrenia presents as prolonged, often monotone groaning during expiration in sleep.

B. Combination OSA treatment can involve sequential or concurrent adjunctive and primary therapies.

C. High altitude can destabilise ventilatory control and cause central periodic breathing.

D. Complications includes frequent nocturnal awakenings, excessive daytime sleepiness and increased risk of adverse cardiovascular.

E. Septoplasty with turbinate reduction only where there are both symptoms and objective nasendoscopic evidence of obstruction.

CORRECT ANSWER E

Septoplasty with turbinate reduction only where there are both symptoms and objective nasendoscopic evidence of obstruction. The other options describe different mechanisms, investigations or findings rather than nasal surgery. Consult Integrated notebook, page 14, §1.10 for the full context.

REFERENCE Integrated notebook, page 14, §1.10

QUESTION 0102 · §1.9 · Focused knowledge SBA

Which of the following statements about risk factors is most accurate in dental sleep medicine?

A. Cognitive behavioural therapy for insomnia is first-line treatment for chronic insomnia.

B. Insomnia risk is increased by age, comorbidity and disrupted work or sleep schedules.

C. Insomnia symptoms are reported by roughly 33–50% of adults, with lower rates of chronic disorder.

D. Insomnia symptoms: 33–50% of the adult population; about one third report dissatisfaction with sleep.

E. CBT for insomnia can improve sleep latency, wakefulness after sleep onset and sleep efficiency.

CORRECT ANSWER B

Insomnia risk is increased by age, comorbidity and disrupted work or sleep schedules. Additional revision detail: increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status. The other options describe different mechanisms, investigations or findings rather than risk factors. Consult Integrated notebook, page 27, §1.9 for the full context.

REFERENCE Integrated notebook, page 27, §1.9

QUESTION 0103 · §1.12 · Applied clinical SBA

A 48-year-old with snoring and unrefreshing sleep asks about the underlying sleep physiology. Which of the following statements about opioids is most accurate?

A. Nicotine stimulates wakefulness, while overnight withdrawal can fragment sleep.

B. Opioids can suppress respiratory drive and cause central apnoeas and irregular breathing.

C. Caffeine blocks adenosine receptors and has an elimination half-life of roughly five hours.

D. Insomnia symptoms affect approximately one-third to one-half of adults; chronic insomnia is less common.

E. An observational study linked at least two OSA features with a 73% higher incidence of first-onset TMD.

CORRECT ANSWER B

Opioids can suppress respiratory drive and cause central apnoeas and irregular breathing. Additional revision detail: dose-dependent respiratory depression with central apnoeas and ataxic breathing; a recognised cause of CSA. The other options describe different mechanisms, investigations or findings rather than opioids. Consult Integrated notebook, page 15, §1.12 for the full context.

REFERENCE Integrated notebook, page 15, §1.12

QUESTION 0104 · §1.12 · Focused knowledge SBA

Which of the following statements about weight is most accurate in dental sleep medicine?

A. Nicotine stimulates wakefulness, while overnight withdrawal can fragment sleep.

B. At least two OSA symptoms were associated with a 73% higher incidence of first-onset TMD.

C. Insomnia symptoms affect approximately one-third to one-half of adults; chronic insomnia is less common.

D. Opioids can suppress respiratory drive and cause central apnoeas and irregular breathing.

E. A 10% weight gain is associated with an approximately 32% rise in AHI.

CORRECT ANSWER E

A 10% weight gain is associated with an approximately 32% rise in AHI. Additional revision detail: 10% gain → +32% AHI; 10% loss → −26% AHI. The other options describe different mechanisms, investigations or findings rather than weight. Consult Integrated notebook, page 16, §1.12 for the full context.

REFERENCE Integrated notebook, page 16, §1.12

QUESTION 0105 · §1.2 · Applied clinical SBA

Which of the following statements about process c (circadian) is most accurate in dental sleep medicine?

A. The suprachiasmatic nucleus regulates circadian timing and is entrained mainly by light.

B. GABA and galanin from the ventrolateral preoptic area promote NREM sleep.

C. Loss of pharyngeal dilator tone during REM sleep contributes to REM-predominant obstructive events.

D. Wakefulness is supported by orexin, monoaminergic pathways and other arousal-promoting transmitters.

E. Homeostatic sleep pressure increases with time awake and dissipates during sleep.

CORRECT ANSWER A

The suprachiasmatic nucleus regulates circadian timing and is entrained mainly by light. Additional revision detail: endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract. The other options describe different mechanisms, investigations or findings rather than process c (circadian). Consult Integrated notebook, page 9, §1.2 for the full context.

REFERENCE Integrated notebook, page 9, §1.2

QUESTION 0106 · §1.9 · Clinical interpretation SBA

A 48-year-old with snoring and unrefreshing sleep asks about the underlying sleep physiology. The clinical team records the following finding: symptoms 33–50%; ~⅓ report dissatisfaction with sleep; with distress/impairment 10–15%; chronic 5–15%; with medical/psychiatric comorbidity 50–75%. Which concept best explains this observation?

A. Treatment

B. Risk factors

C. Effect on outcomes

D. Insomnia symptoms

E. Prevalence

CORRECT ANSWER E

This finding most directly describes Prevalence: symptoms 33–50%; ~⅓ report dissatisfaction with sleep; with distress/impairment 10–15%; chronic 5–15%; with medical/psychiatric comorbidity 50–75%. Distractor distinctions: A refers instead to CBT-I is first line, without tolerance or the adverse effects of pharmacotherapy; B refers instead to increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status; C refers instead to marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over; D refers instead to 33–50% of the adult population; about one third report dissatisfaction with sleep.

REFERENCE Integrated notebook, page 13, §1.9

QUESTION 0107 · §1.10 · Applied clinical SBA

Which of the following statements about combination therapy is most accurate in dental sleep medicine?

A. Hypercapnic central apnoea may reflect impaired brainstem respiratory drive or motor output.

B. Septoplasty with turbinate reduction only where there are both symptoms and objective nasendoscopic evidence of obstruction.

C. Complications includes frequent nocturnal awakenings, excessive daytime sleepiness and increased risk of adverse cardiovascular.

D. Combination OSA treatment can involve sequential or concurrent adjunctive and primary therapies.

E. Nasal EPAP includes one-way resistor valves creating positive end-expiratory pressure to stabilise the airway.

CORRECT ANSWER D

Combination OSA treatment can involve sequential or concurrent adjunctive and primary therapies. Additional revision detail: pre-phase + adjunctive, adjunctive + mainstream, or multiple mainstream. Higher efficacy, limited evidence. 1.11 Other non-respiratory sleep disorders. The other options describe different mechanisms, investigations or findings rather than combination therapy. Consult Integrated notebook, page 14, §1.10 for the full context.

REFERENCE Integrated notebook, page 14, §1.10

QUESTION 0108 · §1.10 · Clinical interpretation SBA

A 51-year-old with witnessed respiratory pauses has a consultation with a sleep specialist. The clinical team records the following finding: slow, monotone, groaning expiratory vocalisation; benign, often mistaken for a parasomnia. Which concept best explains this observation?

A. Not benign

B. Complementary

C. Catathrenia

D. Nasal EPAP

E. Aetiologies

CORRECT ANSWER C

This finding most directly describes Catathrenia: slow, monotone, groaning expiratory vocalisation; benign, often mistaken for a parasomnia. Distractor distinctions: A refers instead to heavy snoring (>50% of sleep time) may induce endothelial injury at the carotid bifurcation given its proximity to the hypoph; B refers instead to nasal valve dilators, lubricating sprays, herbal remedies; D refers instead to one-way resistor valves creating positive end-expiratory pressure to stabilise the airway; overall improvement but less than; E refers instead to high-altitude periodic breathing (which most healthy people develop given sufficient alveolar hypoxia).

REFERENCE Integrated notebook, page 13, §1.10

QUESTION 0109 · §1.9 · Applied clinical SBA

Which of the following statements about risk factors is most accurate in dental sleep medicine?

A. CBT for insomnia can improve sleep latency, wakefulness after sleep onset and sleep efficiency.

B. Insomnia risk is increased by age, comorbidity and disrupted work or sleep schedules.

C. Insomnia symptoms: 33–50% of the adult population; about one third report dissatisfaction with sleep.

D. Insomnia symptoms are reported by roughly 33–50% of adults, with lower rates of chronic disorder.

E. Cognitive behavioural therapy for insomnia is first-line treatment for chronic insomnia.

CORRECT ANSWER B

Insomnia risk is increased by age, comorbidity and disrupted work or sleep schedules. Additional revision detail: increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status. The other options describe different mechanisms, investigations or findings rather than risk factors. Consult Integrated notebook, page 27, §1.9 for the full context.

REFERENCE Integrated notebook, page 27, §1.9

QUESTION 0110 · §1.12 · Clinical interpretation SBA

A patient referred from a dental clinic reports fragmented sleep and morning fatigue. The clinical team records the following finding: AHI>5 — 9% F / 24% M (Wisconsin 1993); moderate–severe — 6% F / 13% M (2007–2010); global 936 million / 425 million moderate–severe. Which concept best explains this observation?

A. Insomnia

B. Caffeine

C. OSA–TMD

D. OSA and TMD

E. OSA prevalence

CORRECT ANSWER E

This finding most directly describes OSA prevalence: AHI>5 — 9% F / 24% M (Wisconsin 1993); moderate–severe — 6% F / 13% M (2007–2010); global 936 million / 425 million moderate–severe. Distractor distinctions: A refers instead to symptoms 33–50% · disorder 10–15% · chronic 5–15% · comorbid 50–75% · chronic = ≥3 nights/week × ≥3 months; B refers instead to an adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure) with a half-life of roughly five hours; C refers instead to ≥2 signs/symptoms → 73% greater first-onset TMD incidence; chronic TMD >3× more frequent; D refers instead to ≥2 OSA signs/symptoms → 73% greater incidence of first-onset TMD; chronic TMD >3× more frequent in high-OSA-likelihood adults.

REFERENCE Integrated notebook, page 33, §1.12

QUESTION 0111 · §1.12 · Applied clinical SBA

Which of the following statements about weight is most accurate in dental sleep medicine?

A. Insomnia symptoms affect approximately one-third to one-half of adults; chronic insomnia is less common.

B. Opioids can suppress respiratory drive and cause central apnoeas and irregular breathing.

C. At least two OSA symptoms were associated with a 73% higher incidence of first-onset TMD.

D. A 10% weight gain is associated with an approximately 32% rise in AHI.

E. Nicotine stimulates wakefulness, while overnight withdrawal can fragment sleep.

CORRECT ANSWER D

A 10% weight gain is associated with an approximately 32% rise in AHI. Additional revision detail: 10% gain → +32% AHI; 10% loss → −26% AHI. The other options describe different mechanisms, investigations or findings rather than weight. Consult Integrated notebook, page 16, §1.12 for the full context.

REFERENCE Integrated notebook, page 16, §1.12

QUESTION 0112 · §1.10 · Clinical interpretation SBA

A patient referred from a dental clinic reports fragmented sleep and morning fatigue. The clinical team records the following finding: hypercapnic CSA, reflecting either abnormal central pattern generator output ("won't breathe") or impairment of respiratory motor output caudal to the pattern generator ("can't breathe"). Which concept best explains this observation?

A. Hypoventilation syndromes

B. Sleep-related hypoxaemia disorder

C. Classification by awake CO2

D. Relationship to OSA

E. Combination therapy

CORRECT ANSWER C

This finding most directly describes Classification by awake CO2: hypercapnic CSA, reflecting either abnormal central pattern generator output ("won't breathe") or impairment of respiratory motor output caudal to the pattern generator ("can't breathe"). Distractor distinctions: A refers instead to insufficient alveolar ventilation causing CO2 retention and hypoxaemia; B refers instead to sustained desaturation during sleep without a comparable rise in CO2; D refers instead to approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5). Snoring there; E refers instead to pre-phase + adjunctive, adjunctive + mainstream, or multiple mainstream. Higher efficacy, limited evidence. 1.11 Other non-re.

REFERENCE Integrated notebook, page 28, §1.10

QUESTION 0113 · §1.9 · Applied clinical SBA

Which of the following statements about Prevalence is most accurate in dental sleep medicine?

A. Insomnia symptoms: 33–50% of the adult population; about one third report dissatisfaction with sleep.

B. Insomnia symptoms are reported by roughly 33–50% of adults, with lower rates of chronic disorder.

C. Chronic insomnia involves difficulty initiating or maintaining sleep with daytime impairment over an extended period.

D. CBT for insomnia can improve sleep latency, wakefulness after sleep onset and sleep efficiency.

E. Short-term insomnia produces clinically significant distress but does not meet chronic-duration criteria.

CORRECT ANSWER B

Insomnia symptoms are reported by roughly 33–50% of adults, with lower rates of chronic disorder. Additional revision detail: symptoms 33–50%; ~⅓ report dissatisfaction with sleep; with distress/impairment 10–15%; chronic 5–15%; with medical/psychiatric comorbidity 50–75%. The other options describe different mechanisms, investigations or findings rather than prevalence. Consult Integrated notebook, page 13, §1.9 for the full context.

REFERENCE Integrated notebook, page 13, §1.9

QUESTION 0114 · §1.12 · Clinical interpretation SBA

A 63-year-old with recurrent nocturnal awakenings is being assessed for disordered breathing. The clinical team records the following finding: stimulant plus nocturnal withdrawal arousals; smoking also causes upper airway inflammation. Which concept best explains this observation?

A. OSA–TMD

B. Caffeine

C. Insomnia

D. Nicotine

E. Alcohol

CORRECT ANSWER D

This finding most directly describes Nicotine: stimulant plus nocturnal withdrawal arousals; smoking also causes upper airway inflammation. Distractor distinctions: A refers instead to ≥2 signs/symptoms → 73% greater first-onset TMD incidence; chronic TMD >3× more frequent; B refers instead to an adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure) with a half-life of roughly five hours; C refers instead to symptoms 33–50% · disorder 10–15% · chronic 5–15% · comorbid 50–75% · chronic = ≥3 nights/week × ≥3 months; E refers instead to reduces sleep latency but suppresses REM in the first half of the night with rebound in the second; fragments sleep; reduces.

REFERENCE Integrated notebook, page 15, §1.12

QUESTION 0115 · §1.10 · Applied clinical SBA

A 48-year-old with snoring and unrefreshing sleep asks about the underlying sleep physiology. Which of the following statements about catathrenia is most accurate?

A. High altitude can destabilise ventilatory control and cause central periodic breathing.

B. Catathrenia presents as prolonged, often monotone groaning during expiration in sleep.

C. Hypoventilation syndromes includes insufficient alveolar ventilation causing CO2 retention and hypoxaemia.

D. Low positive airway pressure may suppress uncomplicated snoring, but physician assessment is still needed.

E. Sleep-related hypoxaemia produces sustained oxygen desaturation without an equivalent rise in carbon dioxide.

CORRECT ANSWER B

Catathrenia presents as prolonged, often monotone groaning during expiration in sleep. Additional revision detail: slow, monotone, groaning expiratory vocalisation; benign, often mistaken for a parasomnia. The other options describe different mechanisms, investigations or findings rather than catathrenia. Consult Integrated notebook, page 13, §1.10 for the full context.

REFERENCE Integrated notebook, page 13, §1.10

QUESTION 0116 · §1.10 · Focused knowledge SBA

Which of the following statements about complications is most accurate in dental sleep medicine?

A. Sleep-related hypoxaemia produces sustained oxygen desaturation without an equivalent rise in carbon dioxide.

B. High altitude can destabilise ventilatory control and cause central periodic breathing.

C. Catathrenia presents as prolonged, often monotone groaning during expiration in sleep.

D. Septoplasty with turbinate reduction only where there are both symptoms and objective nasendoscopic evidence of obstruction.

E. Complications includes frequent nocturnal awakenings, excessive daytime sleepiness and increased risk of adverse cardiovascular.

CORRECT ANSWER E

Complications includes frequent nocturnal awakenings, excessive daytime sleepiness and increased risk of adverse cardiovascular. Additional revision detail: frequent nocturnal awakenings, excessive daytime sleepiness and increased risk of adverse cardiovascular outcomes. The other options describe different mechanisms, investigations or findings rather than complications. Consult Integrated notebook, page 28, §1.10 for the full context.

REFERENCE Integrated notebook, page 28, §1.10

QUESTION 0117 · §1.12 · Applied clinical SBA

Which of the following statements about OSA prevalence is most accurate in dental sleep medicine?

A. An observational study linked at least two OSA features with a 73% higher incidence of first-onset TMD.

B. Caffeine blocks adenosine receptors and has an elimination half-life of roughly five hours.

C. Opioids can suppress respiratory drive and cause central apnoeas and irregular breathing.

D. Approximately 70% of people with OSA have at least one non-anatomical physiological impairment.

E. Historical population data reported AHI above 5/h in about 9% of women and 24% of men.

CORRECT ANSWER E

Historical population data reported AHI above 5/h in about 9% of women and 24% of men. Additional revision detail: AHI>5 — 9% F / 24% M (Wisconsin 1993); moderate–severe — 6% F / 13% M (2007–2010); global 936 million / 425 million moderate–severe. The other options describe different mechanisms, investigations or findings rather than osa prevalence. Consult Integrated notebook, page 33, §1.12 for the full context.

REFERENCE Integrated notebook, page 33, §1.12

QUESTION 0118 · §1.12 · Clinical interpretation SBA

A 48-year-old with snoring and unrefreshing sleep asks about the underlying sleep physiology. The clinical team records the following finding: ≥2 OSA signs/symptoms → 73% greater incidence of first-onset TMD; chronic TMD >3× more frequent in high-OSA-likelihood adults. Which concept best explains this observation?

A. OSA prevalence

B. Insomnia

C. OSA–TMD

D. OSA and TMD

E. Caffeine

CORRECT ANSWER D

This finding most directly describes OSA and TMD: ≥2 OSA signs/symptoms → 73% greater incidence of first-onset TMD; chronic TMD >3× more frequent in high-OSA-likelihood adults. Distractor distinctions: A refers instead to AHI>5 — 9% F / 24% M (Wisconsin 1993); moderate–severe — 6% F / 13% M (2007–2010); global 936 million / 425 million moderate–; B refers instead to symptoms 33–50% · disorder 10–15% · chronic 5–15% · comorbid 50–75% · chronic = ≥3 nights/week × ≥3 months; C refers instead to ≥2 signs/symptoms → 73% greater first-onset TMD incidence; chronic TMD >3× more frequent; E refers instead to an adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure) with a half-life of roughly five hours.

REFERENCE Integrated notebook, page 33, §1.12

QUESTION 0119 · §1.10 · Applied clinical SBA

Which of the following statements about classification by awake co2 is most accurate in dental sleep medicine?

A. Hypercapnic central apnoea may reflect impaired brainstem respiratory drive or motor output.

B. Nasal EPAP includes one-way resistor valves creating positive end-expiratory pressure to stabilise the airway.

C. Heavy snoring may be associated with local carotid vascular injury, although causation is uncertain.

D. Approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5).

E. Positional therapy in supine-dominant snorers includes physical barrier or a position-sensing vibratory device.

CORRECT ANSWER A

Hypercapnic central apnoea may reflect impaired brainstem respiratory drive or motor output. Additional revision detail: hypercapnic CSA, reflecting either abnormal central pattern generator output ("won't breathe") or impairment of respiratory motor output caudal to the pattern generator ("can't breathe"). The other options describe different mechanisms, investigations or findings rather than classification by awake co2. Consult Integrated notebook, page 28, §1.10 for the full context.

REFERENCE Integrated notebook, page 28, §1.10

QUESTION 0120 · §1.10 · Clinical interpretation SBA

A 48-year-old with snoring and unrefreshing sleep asks about the underlying sleep physiology. The clinical team records the following finding: insufficient alveolar ventilation causing CO2 retention and hypoxaemia. Which concept best explains this observation?

A. Relationship to OSA

B. Hypoventilation syndromes

C. Sleep-related hypoxaemia disorder

D. Combination therapy

E. Classification by awake CO2

CORRECT ANSWER B

This finding most directly describes Hypoventilation syndromes: insufficient alveolar ventilation causing CO2 retention and hypoxaemia. Distractor distinctions: A refers instead to approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5). Snoring there; C refers instead to sustained desaturation during sleep without a comparable rise in CO2; D refers instead to pre-phase + adjunctive, adjunctive + mainstream, or multiple mainstream. Higher efficacy, limited evidence. 1.11 Other non-re; E refers instead to hypercapnic CSA, reflecting either abnormal central pattern generator output ("won't breathe") or impairment of respiratory m.

REFERENCE Integrated notebook, page 13, §1.10

QUESTION 0121 · §1.12 · Applied clinical SBA

Which of the following statements about nicotine is most accurate in dental sleep medicine?

A. Nicotine stimulates wakefulness, while overnight withdrawal can fragment sleep.

B. Approximately 70% of people with OSA have at least one non-anatomical physiological impairment.

C. Insomnia symptoms affect approximately one-third to one-half of adults; chronic insomnia is less common.

D. Opioids can suppress respiratory drive and cause central apnoeas and irregular breathing.

E. At least two OSA symptoms were associated with a 73% higher incidence of first-onset TMD.

CORRECT ANSWER A

Nicotine stimulates wakefulness, while overnight withdrawal can fragment sleep. Additional revision detail: stimulant plus nocturnal withdrawal arousals; smoking also causes upper airway inflammation. The other options describe different mechanisms, investigations or findings rather than nicotine. Consult Integrated notebook, page 15, §1.12 for the full context.

REFERENCE Integrated notebook, page 15, §1.12

QUESTION 0122 · §1.10 · Clinical interpretation SBA

A patient referred from a dental clinic reports fragmented sleep and morning fatigue. The clinical team records the following finding: one-way resistor valves creating positive end-expiratory pressure to stabilise the airway; overall improvement but less than CPAP; minimally harmful, costly long term. Which concept best explains this observation?

A. Nasal EPAP

B. Aetiologies

C. Not benign

D. Catathrenia

E. Nasal surgery

CORRECT ANSWER A

This finding most directly describes Nasal EPAP: one-way resistor valves creating positive end-expiratory pressure to stabilise the airway; overall improvement but less than CPAP; minimally harmful, costly long term. Distractor distinctions: B refers instead to high-altitude periodic breathing (which most healthy people develop given sufficient alveolar hypoxia); C refers instead to heavy snoring (>50% of sleep time) may induce endothelial injury at the carotid bifurcation given its proximity to the hypoph; D refers instead to slow, monotone, groaning expiratory vocalisation; benign, often mistaken for a parasomnia; E refers instead to septoplasty with turbinate reduction only where there are both symptoms and objective nasendoscopic evidence of obstruction.

REFERENCE Integrated notebook, page 14, §1.10

QUESTION 0123 · §1.10 · Applied clinical SBA

Which of the following statements about complications is most accurate in dental sleep medicine?

A. Septoplasty with turbinate reduction only where there are both symptoms and objective nasendoscopic evidence of obstruction.

B. Sleep-related hypoxaemia produces sustained oxygen desaturation without an equivalent rise in carbon dioxide.

C. Catathrenia presents as prolonged, often monotone groaning during expiration in sleep.

D. High altitude can destabilise ventilatory control and cause central periodic breathing.

E. Complications includes frequent nocturnal awakenings, excessive daytime sleepiness and increased risk of adverse cardiovascular.

CORRECT ANSWER E

Complications includes frequent nocturnal awakenings, excessive daytime sleepiness and increased risk of adverse cardiovascular. Additional revision detail: frequent nocturnal awakenings, excessive daytime sleepiness and increased risk of adverse cardiovascular outcomes. The other options describe different mechanisms, investigations or findings rather than complications. Consult Integrated notebook, page 28, §1.10 for the full context.

REFERENCE Integrated notebook, page 28, §1.10

QUESTION 0124 · §1.10 · Clinical interpretation SBA

An adult with irregular shifts reports fatigue and reduced daytime concentration. The clinical team records the following finding: physical barrier or a position-sensing vibratory device. Reduces snoring frequency and rate in most non-apnoeic snorers (AHI<5) but not in those with OSA; older devices had poor long-term adherence. Which concept best explains this observation?

A. Classification by awake CO2

B. Positional therapy in supine-dominant snorers

C. CPAP in those already committed or willing

D. Sleep-related hypoxaemia disorder

E. Combination therapy

CORRECT ANSWER B

This finding most directly describes Positional therapy in supine-dominant snorers: physical barrier or a position-sensing vibratory device. Reduces snoring frequency and rate in most non-apnoeic snorers (AHI<5) but not in those with OSA; older devices had poor long-term adherence. Distractor distinctions: A refers instead to hypercapnic CSA, reflecting either abnormal central pattern generator output ("won't breathe") or impairment of respiratory m; C refers instead to effective and safe, compliance highly variable; 4–6 cmH2O eliminates snoring; D refers instead to sustained desaturation during sleep without a comparable rise in CO2; E refers instead to pre-phase + adjunctive, adjunctive + mainstream, or multiple mainstream. Higher efficacy, limited evidence. 1.11 Other non-re.

REFERENCE Integrated notebook, page 14, §1.10

QUESTION 0125 · §1.12 · Applied clinical SBA

Which of the following statements about OSA and TMD is most accurate in dental sleep medicine?

A. Historical population data reported AHI above 5/h in about 9% of women and 24% of men.

B. Insomnia symptoms affect approximately one-third to one-half of adults; chronic insomnia is less common.

C. An observational study linked at least two OSA features with a 73% higher incidence of first-onset TMD.

D. Caffeine blocks adenosine receptors and has an elimination half-life of roughly five hours.

E. At least two OSA symptoms were associated with a 73% higher incidence of first-onset TMD.

CORRECT ANSWER C

An observational study linked at least two OSA features with a 73% higher incidence of first-onset TMD. Additional revision detail: ≥2 OSA signs/symptoms → 73% greater incidence of first-onset TMD; chronic TMD >3× more frequent in high-OSA-likelihood adults. The other options describe different mechanisms, investigations or findings rather than osa and tmd. Consult Integrated notebook, page 33, §1.12 for the full context.

REFERENCE Integrated notebook, page 33, §1.12

QUESTION 0126 · §1.10 · Clinical interpretation SBA

A 63-year-old with recurrent nocturnal awakenings is being assessed for disordered breathing. The clinical team records the following finding: approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5). Snoring therefore neither rules OSA in nor out — the single most useful sentence on this subject. Which concept best explains this observation?

A. Combination therapy

B. Hypoventilation syndromes

C. Complications

D. Relationship to OSA

E. Complementary

CORRECT ANSWER D

This finding most directly describes Relationship to OSA: approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5). Snoring therefore neither rules OSA in nor out — the single most useful sentence on this subject. Distractor distinctions: A refers instead to pre-phase + adjunctive, adjunctive + mainstream, or multiple mainstream. Higher efficacy, limited evidence. 1.11 Other non-re; B refers instead to insufficient alveolar ventilation causing CO2 retention and hypoxaemia; C refers instead to frequent nocturnal awakenings, excessive daytime sleepiness and increased risk of adverse cardiovascular outcomes; E refers instead to nasal valve dilators, lubricating sprays, herbal remedies.

REFERENCE Integrated notebook, page 29, §1.10

QUESTION 0127 · §1.10 · Applied clinical SBA

Which of the following statements about hypoventilation syndromes is most accurate in dental sleep medicine?

A. Hypoventilation syndromes includes insufficient alveolar ventilation causing CO2 retention and hypoxaemia.

B. Complementary includes nasal valve dilators, lubricating sprays, herbal remedies.

C. Sleep-related hypoxaemia produces sustained oxygen desaturation without an equivalent rise in carbon dioxide.

D. Catathrenia presents as prolonged, often monotone groaning during expiration in sleep.

E. Low positive airway pressure may suppress uncomplicated snoring, but physician assessment is still needed.

CORRECT ANSWER A

Hypoventilation syndromes includes insufficient alveolar ventilation causing CO2 retention and hypoxaemia. Additional revision detail: insufficient alveolar ventilation causing CO2 retention and hypoxaemia. The other options describe different mechanisms, investigations or findings rather than hypoventilation syndromes. Consult Integrated notebook, page 13, §1.10 for the full context.

REFERENCE Integrated notebook, page 13, §1.10

QUESTION 0128 · §1.10 · Applied clinical SBA

Which of the following statements about nasal EPAP is most accurate in dental sleep medicine?

A. Septoplasty with turbinate reduction only where there are both symptoms and objective nasendoscopic evidence of obstruction.

B. Combination OSA treatment can involve sequential or concurrent adjunctive and primary therapies.

C. Hypercapnic central apnoea may reflect impaired brainstem respiratory drive or motor output.

D. Nasal EPAP includes one-way resistor valves creating positive end-expiratory pressure to stabilise the airway.

E. Positional therapy in supine-dominant snorers includes physical barrier or a position-sensing vibratory device.

CORRECT ANSWER D

Nasal EPAP includes one-way resistor valves creating positive end-expiratory pressure to stabilise the airway. Additional revision detail: one-way resistor valves creating positive end-expiratory pressure to stabilise the airway; overall improvement but less than CPAP; minimally harmful, costly long term. The other options describe different mechanisms, investigations or findings rather than nasal epap. Consult Integrated notebook, page 14, §1.10 for the full context.

REFERENCE Integrated notebook, page 14, §1.10

QUESTION 0129 · §1.10 · Applied clinical SBA

Which of the following statements about positional therapy in supine-dominant snorers is most accurate in dental sleep medicine?

A. Nasal EPAP includes one-way resistor valves creating positive end-expiratory pressure to stabilise the airway.

B. Heavy snoring may be associated with local carotid vascular injury, although causation is uncertain.

C. Approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5).

D. Hypercapnic central apnoea may reflect impaired brainstem respiratory drive or motor output.

E. Positional therapy in supine-dominant snorers includes physical barrier or a position-sensing vibratory device.

CORRECT ANSWER E

Positional therapy in supine-dominant snorers includes physical barrier or a position-sensing vibratory device. Additional revision detail: physical barrier or a position-sensing vibratory device. Reduces snoring frequency and rate in most non-apnoeic snorers (AHI<5) but not in those with OSA; older devices had poor long-term adherence. The other options describe different mechanisms, investigations or findings rather than positional therapy in supine-dominant snorers. Consult Integrated notebook, page 14, §1.10 for the full context.

REFERENCE Integrated notebook, page 14, §1.10

QUESTION 0130 · §1.10 · Applied clinical SBA

A 48-year-old with snoring and unrefreshing sleep asks about the underlying sleep physiology. Which of the following statements about relationship to OSA is most accurate?

A. Nasal EPAP includes one-way resistor valves creating positive end-expiratory pressure to stabilise the airway.

B. Hypercapnic central apnoea may reflect impaired brainstem respiratory drive or motor output.

C. Positional therapy in supine-dominant snorers includes physical barrier or a position-sensing vibratory device.

D. Approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5).

E. Heavy snoring may be associated with local carotid vascular injury, although causation is uncertain.

CORRECT ANSWER D

Approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5). Additional revision detail: approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5). Snoring therefore neither rules OSA in nor out — the single most useful sentence on this subject. The other options describe different mechanisms, investigations or findings rather than relationship to osa. Consult Integrated notebook, page 29, §1.10 for the full context.

REFERENCE Integrated notebook, page 29, §1.10

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02 Diagnostic sleep testing and interpretation

QUESTION 0131 · §2.2 · Clinical decision-making SBA

A respiratory event shows a 95% airflow drop lasting 14 seconds while thoracoabdominal effort continues. Which classification is most appropriate?

A. Obstructive apnoea

B. RERA

C. Central apnoea

D. Hypopnoea

E. Mixed apnoea without effort

CORRECT ANSWER A

An apnoea involves at least 90% airflow reduction for at least 10 seconds; persistent effort indicates obstruction.

REFERENCE Integrated notebook, Topic 2, §2.2

QUESTION 0132 · §2.2 · Clinical decision-making SBA

During polysomnography, airflow falls by 95% for 13 seconds while thoracic and abdominal respiratory effort continues. How should this respiratory event be classified?

A. Central apnoea

B. Obstructive apnoea

C. Mixed apnoea beginning centrally

D. Hypopnoea

E. Respiratory effort-related arousal

CORRECT ANSWER B

An apnoea requires at least a 90% airflow fall for at least ten seconds. Preserved breathing effort makes this an obstructive rather than central event; a hypopnoea uses a smaller reduction. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 2, §2.2

QUESTION 0133 · §2.2 · Clinical decision-making SBA

A study records a 40% airflow reduction for 15 seconds accompanied by an EEG arousal but no 4% oxygen desaturation. How should the event be approached under the recommended AASM hypopnoea rule?

A. Exclude it because a 4% fall is always required

B. Score a hypopnoea if airflow and arousal criteria are met

C. Classify it automatically as central apnoea

D. Score an obstructive apnoea because it lasts over ten seconds

E. Interpret it as N3 sleep

CORRECT ANSWER B

The recommended hypopnoea rule accepts a ≥30% reduction lasting ≥10 s with a ≥3% desaturation or arousal; some payer rules differ. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 2, §2.2

QUESTION 0134 · §2.6 · Clinical decision-making SBA

A patient with suspected narcolepsy has excessive sleepiness despite adequate sleep and an otherwise unremarkable respiratory assessment. Which test best evaluates sleep propensity and sleep-onset REM periods?

A. STOP-BANG

B. MSLT performed under an appropriate sleep-laboratory protocol

C. Single-night home pulse oximetry

D. Dental panoramic radiography

E. Berlin questionnaire

CORRECT ANSWER B

The multiple sleep latency test measures sleep propensity and sleep-onset REM periods, used with appropriate preceding sleep assessment in suspected central hypersomnolence. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 2, §2.6

QUESTION 0135 · §2.5 · Clinical decision-making SBA

A patient with suspected OSA also has significant heart failure and possible central apnoeas. Which diagnostic investigation is generally most appropriate?

A. Overnight pulse oximetry alone

B. Screening questionnaire alone

C. Unattended type 4 sleep test

D. Attended laboratory polysomnography

E. Dental CBCT airway-volume measurement

CORRECT ANSWER D

Significant cardiorespiratory comorbidity and suspected central events favour attended laboratory polysomnography, which provides sleep staging and a broader set of physiological signals. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 2, §2.5

QUESTION 0136 · §2.2 · Clinical decision-making SBA

A 12-second airflow reduction of 40% is associated with a cortical arousal but no 4% oxygen fall. How is it scored under the recommended AASM rule?

A. Central apnoea automatically

B. Only a RERA can be scored

C. Normal breathing

D. An apnoea requiring a 90% fall

E. Hypopnoea

CORRECT ANSWER E

The recommended hypopnoea rule requires at least a 30% airflow reduction for at least 10 seconds plus 3% desaturation or arousal; 4% is not universally necessary.

REFERENCE Integrated notebook, Topic 2, §2.2

QUESTION 0137 · §2.2 · Applied clinical SBA

A patient on treatment asks whether a favourable symptom report proves normal breathing during sleep. Which of the following statements about chronic insomnia on PSG is most accurate?

A. In adults, an AHI below 5/h is normal; severe OSA begins at AHI 30/h or more.

B. Untreated OSA often increases sleep fragmentation and reduces consolidated slow-wave and REM sleep.

C. A respiratory effort-related arousal follows increasing respiratory effort without meeting apnoea or hypopnoea criteria.

D. An adult hypopnoea requires an airflow reduction of at least 30% for 10 s with an arousal or qualifying desaturation.

E. Sleep efficiency can fall when sleep is fragmented by repeated respiratory events.

CORRECT ANSWER E

Sleep efficiency can fall when sleep is fragmented by repeated respiratory events. Additional revision detail: increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age. The other options describe different mechanisms, investigations or findings rather than chronic insomnia on psg. Consult Integrated notebook, page 37, §2.2 for the full context.

REFERENCE Integrated notebook, page 37, §2.2

QUESTION 0138 · §2.2 · Clinical interpretation SBA

A 57-year-old with witnessed apnoeas undergoes diagnostic polysomnography. The clinical team records the following finding: increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age. Which concept best explains this observation?

A. Fragmentation

B. Chronic insomnia on PSG

C. Respiratory disturbance index

D. Irregular or absent stages

E. Apnoea-hypopnoea index

CORRECT ANSWER B

This finding most directly describes Chronic insomnia on PSG: increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age. Distractor distinctions: A refers instead to increased arousal index, increased stage shifts, increased N1, reduced slow-wave and REM sleep: the signature of untreated OS; C refers instead to AHI plus respiratory effort-related arousals per hour of sleep; D refers instead to irregular cycling or missing stages point to specific disorders — for example, a patient with narcolepsy may enter sleep dire; E refers instead to Total scored apnoeas and hypopnoeas divided by hours of sleep.

REFERENCE Integrated notebook, page 37, §2.2

QUESTION 0139 · §2.2 · Clinical decision-making SBA

An adult has a PSG AHI of 22 events per hour. Which severity category applies?

A. Moderate OSA

B. Severe OSA

C. Not classifiable without ESS

D. Mild OSA

E. Normal

CORRECT ANSWER A

For adults, 5–<15 is mild, 15–<30 moderate and at least 30 severe; AHI 22 is moderate.

REFERENCE Integrated notebook, Topic 2, §2.2

QUESTION 0140 · §2.3 · Applied clinical SBA

A patient with cardiopulmonary comorbidity is being considered for objective sleep testing. Which of the following statements about surgery is most accurate?

A. Medication may treat comorbid sleep disorders or residual sleepiness but is not primary OSA therapy.

B. Successful OSA treatment may produce REM rebound and reduced sleep fragmentation.

C. Positional therapy where the study shows supine dependence; weight loss where obesity is present.

D. Surgery may be considered in refractory severe OSA when correctable anatomy is present.

E. Oral appliances may be offered for mild-to-moderate OSA or when CPAP is unsuitable in selected severe cases.

CORRECT ANSWER D

Surgery may be considered in refractory severe OSA when correctable anatomy is present. Additional revision detail: considered for severe disease where CPAP and appliances are not tolerated or effective, particularly where a specific correctable anatomical obstruction exists. The other options describe different mechanisms, investigations or findings rather than surgery. Consult Integrated notebook, page 48, §2.3 for the full context.

REFERENCE Integrated notebook, page 48, §2.3

QUESTION 0141 · §2.3 · Focused knowledge SBA

A patient on treatment asks whether a favourable symptom report proves normal breathing during sleep. Which of the following statements about surgery is most accurate?

A. Medication may treat comorbid sleep disorders or residual sleepiness but is not primary OSA therapy.

B. Surgery may be considered in refractory severe OSA when correctable anatomy is present.

C. Positional therapy where the study shows supine dependence; weight loss where obesity is present.

D. Oral appliances may be offered for mild-to-moderate OSA or when CPAP is unsuitable in selected severe cases.

E. Successful OSA treatment may produce REM rebound and reduced sleep fragmentation.

CORRECT ANSWER B

Surgery may be considered in refractory severe OSA when correctable anatomy is present. Additional revision detail: considered for severe disease where CPAP and appliances are not tolerated or effective, particularly where a specific correctable anatomical obstruction exists. The other options describe different mechanisms, investigations or findings rather than surgery. Consult Integrated notebook, page 48, §2.3 for the full context.

REFERENCE Integrated notebook, page 48, §2.3

QUESTION 0142 · §2.5 · Clinical decision-making SBA

A patient has significant heart failure, possible central apnoea and severe daytime sleepiness. Which diagnostic test is most appropriate?

A. Attended in-laboratory polysomnography under physician care

B. A trial appliance without diagnostic testing

C. A symptom questionnaire as definitive testing

D. A dentist-interpreted portable monitor for diagnosis

E. Unsupervised single-channel oximetry

CORRECT ANSWER A

Comorbid cardiopulmonary disease and suspected non-obstructive events limit uncomplicated home testing; attended PSG is the appropriate comprehensive test.

REFERENCE Integrated notebook, Topic 2, §2.5

QUESTION 0143 · §2.4 · Applied clinical SBA

A 60-year-old with suspected OSA asks how a sleep-study result should be interpreted. Which of the following statements about treatment failure is most accurate?

A. A partial oral appliance response includes symptom improvement and at least a 50% reduction in AHI.

B. A type 4 home device usually records only one or two variables, such as oximetry or airflow.

C. Successful OSA treatment may produce REM rebound and reduced sleep fragmentation.

D. Persistent symptoms or an insufficient objective reduction in AHI indicate treatment failure.

E. Complete response includes resolution of OSA symptoms and treatment AHI <5/h.

CORRECT ANSWER D

Persistent symptoms or an insufficient objective reduction in AHI indicate treatment failure. Additional revision detail: ongoing clinical symptoms, with or without <50% AHI reduction. The other options describe different mechanisms, investigations or findings rather than treatment failure. Consult Integrated notebook, page 38, §2.4 for the full context.

REFERENCE Integrated notebook, page 38, §2.4

QUESTION 0144 · §2.4 · Clinical interpretation SBA

A patient with cardiopulmonary comorbidity is being considered for objective sleep testing. The clinical team records the following finding: ongoing clinical symptoms, with or without <50% AHI reduction. Which concept best explains this observation?

A. Partial response

B. Complete response

C. Signature of a good outcome

D. Type 1 advantages

E. Treatment failure

CORRECT ANSWER E

This finding most directly describes Treatment failure: ongoing clinical symptoms, with or without <50% AHI reduction. Distractor distinctions: A refers instead to improved symptoms and ≥50% AHI reduction, but AHI remains ≥5/h; B refers instead to resolution of OSA symptoms and treatment AHI <5/h; C refers instead to Sleep: REM increases, NREM decreases; TST typically unchanged — significant redistribution to more REM (REM rebound), not mor; D refers instead to most accurate and reliable reference standard; low failure rate including for non-OSA SDB; accurate staging and event quantif.

REFERENCE Integrated notebook, page 38, §2.4

QUESTION 0145 · §2.5 · Clinical decision-making SBA

A type 3 home sleep study reports REI 12 with long wake periods. What limitation should the dentist explain?

A. Recording time rather than verified sleep time can underestimate event frequency

B. REI always exceeds true AHI

C. Type 3 always includes EEG staging

D. Home testing is mandatory in suspected narcolepsy

E. A type 3 test diagnoses insomnia

CORRECT ANSWER A

Without EEG staging, home devices use monitoring time as the denominator, which can underestimate sleep event burden.

REFERENCE Integrated notebook, Topic 2, §2.5

QUESTION 0146 · §2.5 · Applied clinical SBA

A sleep physician is reviewing a home sleep apnoea study with a patient who snores. Which of the following statements about type 1 is most accurate?

A. Unattended type 2 PSG is most appropriate in carefully selected patients without complicating disorders.

B. Type 1 indications includes low probability of moderate–severe OSA.

C. A type 3 home sleep test monitors respiratory variables with approximately four to seven sensors but does not stage sleep.

D. A type 1 polysomnogram is attended in a laboratory and records sleep stages and respiratory events.

E. Attended laboratory polysomnography remains the reference standard for characterising complex sleep disorders.

CORRECT ANSWER D

A type 1 polysomnogram is attended in a laboratory and records sleep stages and respiratory events. Additional revision detail: attended, in-laboratory; 15–20 sensors, ≥7 channels. Full montage: EEG, EOG, EMG, ECG, oximetry, airflow, effort, position, limb movements. Measures sleep stage and quantifies respiratory events accurately. The other options describe different mechanisms, investigations or findings rather than type 1. Consult Integrated notebook, page 39, §2.5 for the full context.

REFERENCE Integrated notebook, page 39, §2.5

QUESTION 0147 · §2.5 · Focused knowledge SBA

A 60-year-old with suspected OSA asks how a sleep-study result should be interpreted. Which of the following statements about type 1 is most accurate?

A. A type 3 home sleep test monitors respiratory variables with approximately four to seven sensors but does not stage sleep.

B. Type 1 indications includes low probability of moderate–severe OSA.

C. A type 1 polysomnogram is attended in a laboratory and records sleep stages and respiratory events.

D. Attended laboratory polysomnography remains the reference standard for characterising complex sleep disorders.

E. Unattended type 2 PSG is most appropriate in carefully selected patients without complicating disorders.

CORRECT ANSWER C

A type 1 polysomnogram is attended in a laboratory and records sleep stages and respiratory events. Additional revision detail: attended, in-laboratory; 15–20 sensors, ≥7 channels. Full montage: EEG, EOG, EMG, ECG, oximetry, airflow, effort, position, limb movements. Measures sleep stage and quantifies respiratory events accurately. The other options describe different mechanisms, investigations or findings rather than type 1. Consult Integrated notebook, page 39, §2.5 for the full context.

REFERENCE Integrated notebook, page 39, §2.5

QUESTION 0148 · §2.6 · Clinical decision-making SBA

A patient is being investigated for narcolepsy after an overnight PSG. Which next-day test quantifies propensity to fall asleep and SOREMPs?

A. Overnight oximetry alone

B. NoSAS

C. Multiple Sleep Latency Test

D. Maintenance of Wakefulness Test

E. STOP-BANG

CORRECT ANSWER C

The MSLT quantifies mean sleep latency and sleep-onset REM periods; MWT primarily examines ability to remain awake.

REFERENCE Integrated notebook, Topic 2, §2.6

QUESTION 0149 · §2.6 · Applied clinical SBA

A 57-year-old with witnessed apnoeas undergoes diagnostic polysomnography. Which of the following statements about MWT interpretation is most accurate?

A. MSLT indications includes diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia.

B. MSLT method includes follows an overnight PSG to confirm adequate prior sleep.

C. A longer maintenance-of-wakefulness test latency indicates better ability to remain awake.

D. During MWT, the patient sits quietly in dim light and is instructed to remain awake.

E. Short mean sleep latency on MSLT indicates greater physiological sleepiness.

CORRECT ANSWER C

A longer maintenance-of-wakefulness test latency indicates better ability to remain awake. Additional revision detail: a longer mean sleep latency indicates greater ability to maintain wakefulness; considered a more accurate reflection of the ability to function and maintain alertness in common situations of inactivity. The other options describe different mechanisms, investigations or findings rather than mwt interpretation. Consult Integrated notebook, page 40, §2.6 for the full context.

REFERENCE Integrated notebook, page 40, §2.6

QUESTION 0150 · §2.6 · Clinical interpretation SBA

A sleep physician is reviewing a home sleep apnoea study with a patient who snores. The clinical team records the following finding: a longer mean sleep latency indicates greater ability to maintain wakefulness; considered a more accurate reflection of the ability to function and maintain alertness in common situations of inactivity. Which concept best explains this observation?

A. MWT interpretation

B. Type 1 indications

C. MSLT indications

D. MSLT method

E. MWT method

CORRECT ANSWER A

This finding most directly describes MWT interpretation: a longer mean sleep latency indicates greater ability to maintain wakefulness; considered a more accurate reflection of the ability to function and maintain alertness in common situations of inactivity. Distractor distinctions: B refers instead to low probability of moderate–severe OSA; unsuitable for a type 2 study; failed type 2 study. Also the standard for narcolepsy,; C refers instead to diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia; D refers instead to follows an overnight PSG to confirm adequate prior sleep; E refers instead to the patient sits up in a chair in a quiet dimly lit room, instructed to stay awake; no vocalisation or movement.

REFERENCE Integrated notebook, page 40, §2.6

QUESTION 0151 · §2.7 · Clinical decision-making SBA

A STOP-BANG score of five is recorded at dental assessment. How should it be used?

A. As a replacement for objective testing

B. As a screen prompting physician evaluation, not as a diagnosis

C. As proof of severe OSA

D. As proof an appliance will succeed

E. As a measurement of treatment AHI

CORRECT ANSWER B

STOP-BANG is a screening questionnaire. Positive risk does not establish the diagnosis, severity or treatment efficacy.

REFERENCE Integrated notebook, Topic 2, §2.7

QUESTION 0152 · §2.7 · Applied clinical SBA

A patient on treatment asks whether a favourable symptom report proves normal breathing during sleep. Which of the following statements about macroglossia is most accurate?

A. OSA may meet diagnostic criteria at RDI above 15/h, or above 5/h with relevant symptoms.

B. Scalloping (crenations) of the lateral tongue borders suggests the tongue is too wide for the arch, causing oropharyngeal crowding.

C. Maxillary constriction reduces oral cavity space.

D. STOP-BANG combines symptoms with blood pressure and anthropometric risk factors.

E. Mouth breathing with dry mouth includes compensation for nasal obstruction.

CORRECT ANSWER B

Scalloping (crenations) of the lateral tongue borders suggests the tongue is too wide for the arch, causing oropharyngeal crowding. Additional revision detail: scalloping (crenations) of the lateral tongue borders suggests the tongue is too wide for the arch, causing oropharyngeal crowding; often with inferior-posterior hyoid position placing tongue muscles at mechanical disadvantage. The other options describe different mechanisms, investigations or findings rather than macroglossia. Consult Integrated notebook, page 42, §2.7 for the full context.

REFERENCE Integrated notebook, page 42, §2.7

QUESTION 0153 · §2.7 · Focused knowledge SBA

A 57-year-old with witnessed apnoeas undergoes diagnostic polysomnography. Which of the following statements about macroglossia is most accurate?

A. Mouth breathing with dry mouth includes compensation for nasal obstruction.

B. OSA may meet diagnostic criteria at RDI above 15/h, or above 5/h with relevant symptoms.

C. Maxillary constriction reduces oral cavity space.

D. STOP-BANG combines symptoms with blood pressure and anthropometric risk factors.

E. Scalloping (crenations) of the lateral tongue borders suggests the tongue is too wide for the arch, causing oropharyngeal crowding.

CORRECT ANSWER E

Scalloping (crenations) of the lateral tongue borders suggests the tongue is too wide for the arch, causing oropharyngeal crowding. Additional revision detail: scalloping (crenations) of the lateral tongue borders suggests the tongue is too wide for the arch, causing oropharyngeal crowding; often with inferior-posterior hyoid position placing tongue muscles at mechanical disadvantage. The other options describe different mechanisms, investigations or findings rather than macroglossia. Consult Integrated notebook, page 42, §2.7 for the full context.

REFERENCE Integrated notebook, page 42, §2.7

QUESTION 0154 · §2.4 · Clinical decision-making SBA

Baseline AHI is 32; with the appliance it is 8 and symptoms improve. Which label is most appropriate in the notebook?

A. Complete response

B. Partial treatment response

C. No change

D. Treatment failure by definition

E. Central sleep apnoea

CORRECT ANSWER B

The AHI fell by 75% but remains at least 5; improved symptoms plus at least 50% AHI reduction is a partial response, not complete normalisation.

REFERENCE Integrated notebook, Topic 2, §2.4

QUESTION 0155 · §2.1 · Applied clinical SBA

A sleep physician is reviewing a home sleep apnoea study with a patient who snores. Which of the following statements about eeg + eog + emg together determine sleep stage is most accurate?

A. Body position includes positional dependence; directly relevant to appliance and positional therapy decisions.

B. The minimum oxygen level contributes information on hypoxaemic severity.

C. Sleep staging requires EEG, electro-oculography and electromyography signals.

D. An oronasal thermal sensor helps identify apnoea, while nasal pressure measures hypopnoea.

E. Snoring can be measured acoustically using a sound-level meter with standardised positioning.

CORRECT ANSWER C

Sleep staging requires EEG, electro-oculography and electromyography signals. Additional revision detail: No staging is possible without them — the fundamental limitation of level 3 and 4 studies. The other options describe different mechanisms, investigations or findings rather than eeg + eog + emg together determine sleep stage. Consult Integrated notebook, page 36, §2.1 for the full context.

REFERENCE Integrated notebook, page 36, §2.1

QUESTION 0156 · §2.1 · Clinical interpretation SBA

A 60-year-old with suspected OSA asks how a sleep-study result should be interpreted. The clinical team records the following finding: No staging is possible without them — the fundamental limitation of level 3 and 4 studies. Which concept best explains this observation?

A. End-tidal or transcutaneous CO2

B. NREM 75–80% dominating the first half, REM 20–25% dominating the second

C. Oxygen saturation nadir

D. EEG (scalp)

E. EEG + EOG + EMG together determine sleep stage

CORRECT ANSWER E

This finding most directly describes EEG + EOG + EMG together determine sleep stage: No staging is possible without them — the fundamental limitation of level 3 and 4 studies. Distractor distinctions: A refers instead to hypoventilation; a standard measurement in paediatric PSG. Video/audio — parasomnia, seizure, positional confirmation; B refers instead to Arousal index low, scored by the ASDA 3-second rule; C refers instead to The minimum oxygen level contributes information on hypoxaemic severity; D refers instead to sleep staging and arousals. EOG (periorbital) — eye movements, identifies REM and sleep onset. EMG (submental, tibial) — musc.

REFERENCE Integrated notebook, page 36, §2.1

QUESTION 0157 · §2.7 · Clinical decision-making SBA

A patient has a high ESS but a low respiratory event index. What is the best interpretation?

A. Low AHI proves adequate alertness

B. Sleepiness requires assessment; ESS alone neither diagnoses nor grades OSA

C. The dentist should raise the appliance to maximum immediately

D. Sleepiness excludes narcolepsy

E. An ESS above ten proves severe OSA

CORRECT ANSWER B

ESS reflects subjective sleep propensity and correlates poorly with AHI. Discordance warrants evaluation of other causes and objective findings.

REFERENCE Integrated notebook, Topic 2, §2.7

QUESTION 0158 · §2.2 · Applied clinical SBA

A 57-year-old with witnessed apnoeas undergoes diagnostic polysomnography. Which of the following statements about respiratory effort-related arousal (RERA) is most accurate?

A. Respiratory disturbance index includes aHI plus respiratory effort-related arousals per hour of sleep.

B. An adult hypopnoea requires an airflow reduction of at least 30% for 10 s with an arousal or qualifying desaturation.

C. A respiratory effort-related arousal follows increasing respiratory effort without meeting apnoea or hypopnoea criteria.

D. Untreated OSA often increases sleep fragmentation and reduces consolidated slow-wave and REM sleep.

E. Sleep efficiency can fall when sleep is fragmented by repeated respiratory events.

CORRECT ANSWER C

A respiratory effort-related arousal follows increasing respiratory effort without meeting apnoea or hypopnoea criteria. Additional revision detail: Arousal following increasing effort or flattened airflow that does not meet apnoea or hypopnoea criteria. The other options describe different mechanisms, investigations or findings rather than respiratory effort-related arousal (rera). Consult Integrated notebook, page 37, §2.2 for the full context.

REFERENCE Integrated notebook, page 37, §2.2

QUESTION 0159 · §2.2 · Clinical interpretation SBA

A sleep physician is reviewing a home sleep apnoea study with a patient who snores. The clinical team records the following finding: Arousal following increasing effort or flattened airflow that does not meet apnoea or hypopnoea criteria. Which concept best explains this observation?

A. Irregular or absent stages point to specific disorders

B. Respiratory effort-related arousal (RERA)

C. Irregular or absent stages

D. Chronic insomnia on PSG

E. Respiratory disturbance index

CORRECT ANSWER B

This finding most directly describes Respiratory effort-related arousal (RERA): Arousal following increasing effort or flattened airflow that does not meet apnoea or hypopnoea criteria. Distractor distinctions: A refers instead to narcolepsy patients may enter sleep directly into REM; C refers instead to irregular cycling or missing stages point to specific disorders — for example, a patient with narcolepsy may enter sleep dire; D refers instead to increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age; E refers instead to AHI plus respiratory effort-related arousals per hour of sleep.

REFERENCE Integrated notebook, page 37, §2.2

QUESTION 0160 · §2.3 · Clinical decision-making SBA

A dentist reviews a study revealing substantial central apnoeas. What is the next step?

A. Discuss with the sleep physician before choosing an OSA appliance

B. Use a snoring questionnaire to replace the PSG

C. Diagnose central apnoea independently

D. Treat all events by protrusion alone

E. Ignore central events if dentition is sound

CORRECT ANSWER A

Event type changes management: central apnoeas need physician-led interpretation, and the dentist must not extrapolate from OSA appliance efficacy.

REFERENCE Integrated notebook, Topic 2, §2.3

QUESTION 0161 · §2.3 · Applied clinical SBA

A patient on treatment asks whether a favourable symptom report proves normal breathing during sleep. Which of the following statements about oral appliance therapy is most accurate?

A. Surgery may be considered in refractory severe OSA when correctable anatomy is present.

B. Medication may treat comorbid sleep disorders or residual sleepiness but is not primary OSA therapy.

C. Positional therapy where the study shows supine dependence; weight loss where obesity is present.

D. Oral appliances may be offered for mild-to-moderate OSA or when CPAP is unsuitable in selected severe cases.

E. Type 1 indications includes low probability of moderate–severe OSA.

CORRECT ANSWER D

Oral appliances may be offered for mild-to-moderate OSA or when CPAP is unsuitable in selected severe cases. Additional revision detail: for mild to moderate OSA, or severe OSA where CPAP is refused, not tolerated or ineffective. Positional dependence increases the likelihood of success. The other options describe different mechanisms, investigations or findings rather than oral appliance therapy. Consult Integrated notebook, page 48, §2.3 for the full context.

REFERENCE Integrated notebook, page 48, §2.3

QUESTION 0162 · §2.3 · Clinical interpretation SBA

A 57-year-old with witnessed apnoeas undergoes diagnostic polysomnography. The clinical team records the following finding: for mild to moderate OSA, or severe OSA where CPAP is refused, not tolerated or ineffective. Positional dependence increases the likelihood of success. Which concept best explains this observation?

A. Surgery

B. Oral appliance therapy

C. Apnoea-hypopnoea index

D. Behavioural therapy

E. Pharmacotherapy

CORRECT ANSWER B

This finding most directly describes Oral appliance therapy: for mild to moderate OSA, or severe OSA where CPAP is refused, not tolerated or ineffective. Positional dependence increases the likelihood of success. Distractor distinctions: A refers instead to considered for severe disease where CPAP and appliances are not tolerated or effective, particularly where a specific correct; C refers instead to Total scored apnoeas and hypopnoeas divided by hours of sleep; D refers instead to positional therapy where the study shows supine dependence; weight loss where obesity is present; E refers instead to used for other disorders diagnosed on the study (e.g. RBD) or for residual sleepiness; it is not a primary treatment for OSA.

REFERENCE Integrated notebook, page 48, §2.3

QUESTION 0163 · §2.4 · Applied clinical SBA

A 60-year-old with suspected OSA asks how a sleep-study result should be interpreted. Which of the following statements about partial response is most accurate?

A. Persistent symptoms or an insufficient objective reduction in AHI indicate treatment failure.

B. Complete response includes resolution of OSA symptoms and treatment AHI <5/h.

C. Successful OSA treatment may produce REM rebound and reduced sleep fragmentation.

D. Apnoea-hypopnoea index includes total scored apnoeas and hypopnoeas divided by hours of sleep.

E. A partial oral appliance response includes symptom improvement and at least a 50% reduction in AHI.

CORRECT ANSWER E

A partial oral appliance response includes symptom improvement and at least a 50% reduction in AHI. Additional revision detail: improved symptoms and ≥50% AHI reduction, but AHI remains ≥5/h. The other options describe different mechanisms, investigations or findings rather than partial response. Consult Integrated notebook, page 38, §2.4 for the full context.

REFERENCE Integrated notebook, page 38, §2.4

QUESTION 0164 · §2.4 · Clinical interpretation SBA

A patient with cardiopulmonary comorbidity is being considered for objective sleep testing. The clinical team records the following finding: improved symptoms and ≥50% AHI reduction, but AHI remains ≥5/h. Which concept best explains this observation?

A. Treatment failure

B. MSLT indications

C. Complete response

D. Signature of a good outcome

E. Partial response

CORRECT ANSWER E

This finding most directly describes Partial response: improved symptoms and ≥50% AHI reduction, but AHI remains ≥5/h. Distractor distinctions: A refers instead to ongoing clinical symptoms, with or without <50% AHI reduction; B refers instead to diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia; C refers instead to resolution of OSA symptoms and treatment AHI <5/h; D refers instead to Sleep: REM increases, NREM decreases; TST typically unchanged — significant redistribution to more REM (REM rebound), not mor.

REFERENCE Integrated notebook, page 38, §2.4

QUESTION 0165 · §2.5 · Applied clinical SBA

A 57-year-old with witnessed apnoeas undergoes diagnostic polysomnography. Which of the following statements about type 3 is most accurate?

A. Attended laboratory polysomnography remains the reference standard for characterising complex sleep disorders.

B. Unattended type 2 PSG is most appropriate in carefully selected patients without complicating disorders.

C. Type 1 indications includes low probability of moderate–severe OSA.

D. A type 3 home sleep test monitors respiratory variables with approximately four to seven sensors but does not stage sleep.

E. A type 1 polysomnogram is attended in a laboratory and records sleep stages and respiratory events.

CORRECT ANSWER D

A type 3 home sleep test monitors respiratory variables with approximately four to seven sensors but does not stage sleep. Additional revision detail: home; 4–7 sensors. Limited channel monitoring of breathing parameters without sleep assessment: airflow (≥2 channels of respiratory movement, or movement and airflow), effort, oximetry, heart rate/ECG. The other options describe different mechanisms, investigations or findings rather than type 3. Consult Integrated notebook, page 39, §2.5 for the full context.

REFERENCE Integrated notebook, page 39, §2.5

QUESTION 0166 · §2.5 · Focused knowledge SBA

A sleep physician is reviewing a home sleep apnoea study with a patient who snores. Which of the following statements about type 3 is most accurate?

A. Attended laboratory polysomnography remains the reference standard for characterising complex sleep disorders.

B. Unattended type 2 PSG is most appropriate in carefully selected patients without complicating disorders.

C. Type 1 indications includes low probability of moderate–severe OSA.

D. A type 3 home sleep test monitors respiratory variables with approximately four to seven sensors but does not stage sleep.

E. A type 1 polysomnogram is attended in a laboratory and records sleep stages and respiratory events.

CORRECT ANSWER D

A type 3 home sleep test monitors respiratory variables with approximately four to seven sensors but does not stage sleep. Additional revision detail: home; 4–7 sensors. Limited channel monitoring of breathing parameters without sleep assessment: airflow (≥2 channels of respiratory movement, or movement and airflow), effort, oximetry, heart rate/ECG. The other options describe different mechanisms, investigations or findings rather than type 3. Consult Integrated notebook, page 39, §2.5 for the full context.

REFERENCE Integrated notebook, page 39, §2.5

QUESTION 0167 · §2.6 · Applied clinical SBA

A patient with cardiopulmonary comorbidity is being considered for objective sleep testing. Which of the following statements about MSLT indications is most accurate?

A. MSLT indications includes diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia.

B. During MWT, the patient sits quietly in dim light and is instructed to remain awake.

C. A longer maintenance-of-wakefulness test latency indicates better ability to remain awake.

D. Short mean sleep latency on MSLT indicates greater physiological sleepiness.

E. MSLT method includes follows an overnight PSG to confirm adequate prior sleep.

CORRECT ANSWER A

MSLT indications includes diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia. Additional revision detail: diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia. The other options describe different mechanisms, investigations or findings rather than mslt indications. Consult Integrated notebook, page 40, §2.6 for the full context.

REFERENCE Integrated notebook, page 40, §2.6

QUESTION 0168 · §2.6 · Clinical interpretation SBA

A patient on treatment asks whether a favourable symptom report proves normal breathing during sleep. The clinical team records the following finding: diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia. Which concept best explains this observation?

A. MSLT method

B. MWT method

C. MSLT indications

D. MSLT interpretation

E. MWT interpretation

CORRECT ANSWER C

This finding most directly describes MSLT indications: diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia. Distractor distinctions: A refers instead to follows an overnight PSG to confirm adequate prior sleep; B refers instead to the patient sits up in a chair in a quiet dimly lit room, instructed to stay awake; no vocalisation or movement; D refers instead to mean sleep latency averaged across naps — shorter = more severe sleepiness; E refers instead to a longer mean sleep latency indicates greater ability to maintain wakefulness; considered a more accurate reflection of the a.

REFERENCE Integrated notebook, page 40, §2.6

QUESTION 0169 · §2.7 · Applied clinical SBA

A sleep physician is reviewing a home sleep apnoea study with a patient who snores. Which of the following statements about modified mallampati is most accurate?

A. Mouth breathing with dry mouth includes compensation for nasal obstruction.

B. Maxillary constriction reduces oral cavity space.

C. STOP-BANG combines symptoms with blood pressure and anthropometric risk factors.

D. Modified Mallampati classification estimates visible oropharyngeal crowding during oral examination.

E. Scalloping (crenations) of the lateral tongue borders suggests the tongue is too wide for the arch, causing oropharyngeal crowding.

CORRECT ANSWER D

Modified Mallampati classification estimates visible oropharyngeal crowding during oral examination. Additional revision detail: the modified Mallampati classification assesses visible oropharyngeal structures with the patient seated, mouth open and tongue protruded without phonation. The other options describe different mechanisms, investigations or findings rather than modified mallampati. Consult Integrated notebook, page 42, §2.7 for the full context.

REFERENCE Integrated notebook, page 42, §2.7

QUESTION 0170 · §2.7 · Clinical interpretation SBA

A 60-year-old with suspected OSA asks how a sleep-study result should be interpreted. The clinical team records the following finding: describes oral airway patency. Technique: patient sits upright, opens widely, protrudes the tongue maximally without phonation (the "without phonation" distinguishes the modified from the original anaesthetic…. Which concept best explains this observation?

A. Macroglossia

B. Modified Mallampati

C. OSA diagnosis

D. Screening thresholds

E. Australian direct referral

CORRECT ANSWER B

This finding most directly describes Modified Mallampati: describes oral airway patency. Technique: patient sits upright, opens widely, protrudes the tongue maximally without phonation (the "without phonation" distinguishes the modified from the original anaesthetic…. Distractor distinctions: A refers instead to scalloping (crenations) of the lateral tongue borders suggests the tongue is too wide for the arch, causing oropharyngeal cro; C refers instead to RDI >15/h, or >5/h with symptoms. AHI ≥15 → material increase in metabolic and vascular risk; D refers instead to ESS >10 · STOP-BANG ≥3 · OSA50 ≥5 · Berlin ≥2 categories · NoSAS ≥8 · PSQ >0.33; E refers instead to positive questionnaire (Berlin / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8.

REFERENCE Integrated notebook, page 42, §2.7

QUESTION 0171 · §2.1 · Applied clinical SBA

A 60-year-old with suspected OSA asks how a sleep-study result should be interpreted. Which of the following statements about effort is most accurate?

A. Thoracic and abdominal respiratory effort belts help distinguish obstructive from central apnoeas.

B. The minimum oxygen level contributes information on hypoxaemic severity.

C. An oronasal thermal sensor helps identify apnoea, while nasal pressure measures hypopnoea.

D. Carbon dioxide monitoring helps detect hypoventilation during paediatric polysomnography.

E. Eeg (scalp) includes sleep staging and arousals.

CORRECT ANSWER A

Thoracic and abdominal respiratory effort belts help distinguish obstructive from central apnoeas. Additional revision detail: thoracic and abdominal RIP belts, diaphragm EMG. Distinguishes obstructive from central; paradoxical movement indicates obstruction. The other options describe different mechanisms, investigations or findings rather than effort. Consult Integrated notebook, page 36, §2.1 for the full context.

REFERENCE Integrated notebook, page 36, §2.1

QUESTION 0172 · §2.1 · Focused knowledge SBA

A patient with cardiopulmonary comorbidity is being considered for objective sleep testing. Which of the following statements about effort is most accurate?

A. An oronasal thermal sensor helps identify apnoea, while nasal pressure measures hypopnoea.

B. The minimum oxygen level contributes information on hypoxaemic severity.

C. Carbon dioxide monitoring helps detect hypoventilation during paediatric polysomnography.

D. Thoracic and abdominal respiratory effort belts help distinguish obstructive from central apnoeas.

E. Eeg (scalp) includes sleep staging and arousals.

CORRECT ANSWER D

Thoracic and abdominal respiratory effort belts help distinguish obstructive from central apnoeas. Additional revision detail: thoracic and abdominal RIP belts, diaphragm EMG. Distinguishes obstructive from central; paradoxical movement indicates obstruction. The other options describe different mechanisms, investigations or findings rather than effort. Consult Integrated notebook, page 36, §2.1 for the full context.

REFERENCE Integrated notebook, page 36, §2.1

QUESTION 0173 · §2.2 · Applied clinical SBA

A 57-year-old with witnessed apnoeas undergoes diagnostic polysomnography. Which of the following statements about fragmentation is most accurate?

A. A respiratory effort-related arousal follows increasing respiratory effort without meeting apnoea or hypopnoea criteria.

B. Sleep efficiency can fall when sleep is fragmented by repeated respiratory events.

C. Irregular cycling or missing stages point to specific disorders, for example.

D. Untreated OSA often increases sleep fragmentation and reduces consolidated slow-wave and REM sleep.

E. An adult hypopnoea requires an airflow reduction of at least 30% for 10 s with an arousal or qualifying desaturation.

CORRECT ANSWER D

Untreated OSA often increases sleep fragmentation and reduces consolidated slow-wave and REM sleep. Additional revision detail: increased arousal index, increased stage shifts, increased N1, reduced slow-wave and REM sleep: the signature of untreated OSA. The other options describe different mechanisms, investigations or findings rather than fragmentation. Consult Integrated notebook, page 37, §2.2 for the full context.

REFERENCE Integrated notebook, page 37, §2.2

QUESTION 0174 · §2.2 · Clinical interpretation SBA

A sleep physician is reviewing a home sleep apnoea study with a patient who snores. The clinical team records the following finding: increased arousal index, increased stage shifts, increased N1, reduced slow-wave and REM sleep: the signature of untreated OSA. Which concept best explains this observation?

A. Apnoea

B. Hypopnoea

C. Apnoea-hypopnoea index

D. Severity

E. Fragmentation

CORRECT ANSWER E

This finding most directly describes Fragmentation: increased arousal index, increased stage shifts, increased N1, reduced slow-wave and REM sleep: the signature of untreated OSA. Distractor distinctions: A refers instead to cessation of airflow (≥90% reduction) for ≥10 s. Obstructive if effort continues; central if effort absent; mixed if it begin; B refers instead to reduction in airflow and/or thoracoabdominal movement of ≥30% of baseline for ≥10 s, with ≥3% desaturation and/or an arousal; C refers instead to Total scored apnoeas and hypopnoeas divided by hours of sleep; D refers instead to adult: normal <5 · mild 5–<15 · moderate 15–<30 · severe ≥30. Paediatric: <1 · 1–<5 · 5–<10 · ≥10.

REFERENCE Integrated notebook, page 37, §2.2

QUESTION 0175 · §2.3 · Applied clinical SBA

A patient with cardiopulmonary comorbidity is being considered for objective sleep testing. Which of the following statements about pharmacotherapy is most accurate?

A. Medication may treat comorbid sleep disorders or residual sleepiness but is not primary OSA therapy.

B. Untreated OSA often increases sleep fragmentation and reduces consolidated slow-wave and REM sleep.

C. Surgery may be considered in refractory severe OSA when correctable anatomy is present.

D. Positional therapy where the study shows supine dependence; weight loss where obesity is present.

E. Oral appliances may be offered for mild-to-moderate OSA or when CPAP is unsuitable in selected severe cases.

CORRECT ANSWER A

Medication may treat comorbid sleep disorders or residual sleepiness but is not primary OSA therapy. Additional revision detail: used for other disorders diagnosed on the study (e.g. RBD) or for residual sleepiness; it is not a primary treatment for OSA itself. The other options describe different mechanisms, investigations or findings rather than pharmacotherapy. Consult Integrated notebook, page 48, §2.3 for the full context.

REFERENCE Integrated notebook, page 48, §2.3

QUESTION 0176 · §2.3 · Clinical interpretation SBA

A patient on treatment asks whether a favourable symptom report proves normal breathing during sleep. The clinical team records the following finding: used for other disorders diagnosed on the study (e.g. RBD) or for residual sleepiness; it is not a primary treatment for OSA itself. Which concept best explains this observation?

A. Surgery

B. Oral appliance therapy

C. Pharmacotherapy

D. Behavioural therapy

E. MSLT indications

CORRECT ANSWER C

This finding most directly describes Pharmacotherapy: used for other disorders diagnosed on the study (e.g. RBD) or for residual sleepiness; it is not a primary treatment for OSA itself. Distractor distinctions: A refers instead to considered for severe disease where CPAP and appliances are not tolerated or effective, particularly where a specific correct; B refers instead to for mild to moderate OSA, or severe OSA where CPAP is refused, not tolerated or ineffective. Positional dependence increases; D refers instead to positional therapy where the study shows supine dependence; weight loss where obesity is present; E refers instead to diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia.

REFERENCE Integrated notebook, page 48, §2.3

QUESTION 0177 · §2.4 · Applied clinical SBA

A sleep physician is reviewing a home sleep apnoea study with a patient who snores. Which of the following statements about signature of a good outcome is most accurate?

A. Persistent symptoms or an insufficient objective reduction in AHI indicate treatment failure.

B. Surgery may be considered in refractory severe OSA when correctable anatomy is present.

C. A partial oral appliance response includes symptom improvement and at least a 50% reduction in AHI.

D. Complete response includes resolution of OSA symptoms and treatment AHI <5/h.

E. Successful OSA treatment may produce REM rebound and reduced sleep fragmentation.

CORRECT ANSWER E

Successful OSA treatment may produce REM rebound and reduced sleep fragmentation. Additional revision detail: Sleep: REM increases, NREM decreases; TST typically unchanged — significant redistribution to more REM (REM rebound), not more sleep; arousal index decreases. The other options describe different mechanisms, investigations or findings rather than signature of a good outcome. Consult Integrated notebook, page 38, §2.4 for the full context.

REFERENCE Integrated notebook, page 38, §2.4

QUESTION 0178 · §2.4 · Clinical interpretation SBA

A 60-year-old with suspected OSA asks how a sleep-study result should be interpreted. The clinical team records the following finding: Sleep: REM increases, NREM decreases; TST typically unchanged — significant redistribution to more REM (REM rebound), not more sleep; arousal index decreases. Which concept best explains this observation?

A. Partial response

B. Australian direct referral

C. Signature of a good outcome

D. Complete response

E. Treatment failure

CORRECT ANSWER C

This finding most directly describes Signature of a good outcome: Sleep: REM increases, NREM decreases; TST typically unchanged — significant redistribution to more REM (REM rebound), not more sleep; arousal index decreases. Distractor distinctions: A refers instead to improved symptoms and ≥50% AHI reduction, but AHI remains ≥5/h; B refers instead to positive questionnaire (Berlin / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8; D refers instead to resolution of OSA symptoms and treatment AHI <5/h; E refers instead to ongoing clinical symptoms, with or without <50% AHI reduction.

REFERENCE Integrated notebook, page 38, §2.4

QUESTION 0179 · §2.5 · Applied clinical SBA

A patient on treatment asks whether a favourable symptom report proves normal breathing during sleep. Which of the following statements about type 2 indications is most accurate?

A. Unattended type 2 PSG is most appropriate in carefully selected patients without complicating disorders.

B. The physician should consider attended polysomnography when clinical suspicion remains high.

C. A type 2 study uses a full PSG-type montage but is usually performed unattended outside the laboratory.

D. A type 4 home device usually records only one or two variables, such as oximetry or airflow.

E. Type 1 indications includes low probability of moderate–severe OSA.

CORRECT ANSWER A

Unattended type 2 PSG is most appropriate in carefully selected patients without complicating disorders. Additional revision detail: high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder. The other options describe different mechanisms, investigations or findings rather than type 2 indications. Consult Integrated notebook, page 39, §2.5 for the full context.

REFERENCE Integrated notebook, page 39, §2.5

QUESTION 0180 · §2.5 · Clinical interpretation SBA

A 57-year-old with witnessed apnoeas undergoes diagnostic polysomnography. The clinical team records the following finding: high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder. Which concept best explains this observation?

A. Type 2 indications

B. Type 2

C. Type 1

D. Type 1 advantages

E. Type 3

CORRECT ANSWER A

This finding most directly describes Type 2 indications: high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder. Distractor distinctions: B refers instead to unattended, home (occasionally laboratory-applied then home); 10–15 sensors, ≥7 channels; C refers instead to attended, in-laboratory; 15–20 sensors, ≥7 channels. Full montage: EEG, EOG, EMG, ECG, oximetry, airflow, effort, position, l; D refers instead to most accurate and reliable reference standard; low failure rate including for non-OSA SDB; accurate staging and event quantif; E refers instead to home; 4–7 sensors. Limited channel monitoring of breathing parameters without sleep assessment: airflow (≥2 channels of respi.

REFERENCE Integrated notebook, page 39, §2.5

QUESTION 0181 · §2.6 · Applied clinical SBA

A 60-year-old with suspected OSA asks how a sleep-study result should be interpreted. Which of the following statements about MSLT method is most accurate?

A. Short mean sleep latency on MSLT indicates greater physiological sleepiness.

B. MSLT method includes follows an overnight PSG to confirm adequate prior sleep.

C. MSLT indications includes diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia.

D. A longer maintenance-of-wakefulness test latency indicates better ability to remain awake.

E. During MWT, the patient sits quietly in dim light and is instructed to remain awake.

CORRECT ANSWER B

MSLT method includes follows an overnight PSG to confirm adequate prior sleep. Additional revision detail: follows an overnight PSG to confirm adequate prior sleep. The other options describe different mechanisms, investigations or findings rather than mslt method. Consult Integrated notebook, page 40, §2.6 for the full context.

REFERENCE Integrated notebook, page 40, §2.6

QUESTION 0182 · §2.6 · Clinical interpretation SBA

A patient with cardiopulmonary comorbidity is being considered for objective sleep testing. The clinical team records the following finding: follows an overnight PSG to confirm adequate prior sleep. Which concept best explains this observation?

A. MSLT indications

B. EEG (scalp)

C. MWT interpretation

D. MSLT method

E. MSLT interpretation

CORRECT ANSWER D

This finding most directly describes MSLT method: follows an overnight PSG to confirm adequate prior sleep. Distractor distinctions: A refers instead to diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia; B refers instead to sleep staging and arousals. EOG (periorbital) — eye movements, identifies REM and sleep onset. EMG (submental, tibial) — musc; C refers instead to a longer mean sleep latency indicates greater ability to maintain wakefulness; considered a more accurate reflection of the a; E refers instead to mean sleep latency averaged across naps — shorter = more severe sleepiness.

REFERENCE Integrated notebook, page 40, §2.6

QUESTION 0183 · §2.7 · Applied clinical SBA

A 57-year-old with witnessed apnoeas undergoes diagnostic polysomnography. Which of the following statements about sensors is most accurate?

A. An Epworth Sleepiness Scale score above 10/24 suggests excessive daytime sleepiness.

B. OSA may meet diagnostic criteria at RDI above 15/h, or above 5/h with relevant symptoms.

C. An Epworth Sleepiness Scale score greater than 10 out of 24 suggests increased daytime sleepiness.

D. A type 1 sleep study uses approximately 15–20 sensors, whereas a type 4 study uses only 1–2.

E. STOP-BANG combines symptoms with blood pressure and anthropometric risk factors.

CORRECT ANSWER D

A type 1 sleep study uses approximately 15–20 sensors, whereas a type 4 study uses only 1–2. Additional revision detail: type 1 = 15–20 · 2 = 10–15 · 3 = 4–7 · 4 = 1–2. Type 2 failure ~6.6–7%; underestimates AHI ~10%. The other options describe different mechanisms, investigations or findings rather than sensors. Consult Integrated notebook, page 43, §2.7 for the full context.

REFERENCE Integrated notebook, page 43, §2.7

QUESTION 0184 · §2.7 · Focused knowledge SBA

A sleep physician is reviewing a home sleep apnoea study with a patient who snores. Which of the following statements about sensors is most accurate?

A. An Epworth Sleepiness Scale score above 10/24 suggests excessive daytime sleepiness.

B. A type 1 sleep study uses approximately 15–20 sensors, whereas a type 4 study uses only 1–2.

C. OSA may meet diagnostic criteria at RDI above 15/h, or above 5/h with relevant symptoms.

D. An Epworth Sleepiness Scale score greater than 10 out of 24 suggests increased daytime sleepiness.

E. STOP-BANG combines symptoms with blood pressure and anthropometric risk factors.

CORRECT ANSWER B

A type 1 sleep study uses approximately 15–20 sensors, whereas a type 4 study uses only 1–2. Additional revision detail: type 1 = 15–20 · 2 = 10–15 · 3 = 4–7 · 4 = 1–2. Type 2 failure ~6.6–7%; underestimates AHI ~10%. The other options describe different mechanisms, investigations or findings rather than sensors. Consult Integrated notebook, page 43, §2.7 for the full context.

REFERENCE Integrated notebook, page 43, §2.7

QUESTION 0185 · §2.1 · Applied clinical SBA

A sleep physician is reviewing a home sleep apnoea study with a patient who snores. Which of the following statements about end-tidal or transcutaneous co2 is most accurate?

A. Body position includes positional dependence; directly relevant to appliance and positional therapy decisions.

B. An oronasal thermal sensor helps identify apnoea, while nasal pressure measures hypopnoea.

C. Thoracic and abdominal respiratory effort belts help distinguish obstructive from central apnoeas.

D. Carbon dioxide monitoring helps detect hypoventilation during paediatric polysomnography.

E. Sleep staging requires EEG, electro-oculography and electromyography signals.

CORRECT ANSWER D

Carbon dioxide monitoring helps detect hypoventilation during paediatric polysomnography. Additional revision detail: hypoventilation; a standard measurement in paediatric PSG. Video/audio — parasomnia, seizure, positional confirmation. The other options describe different mechanisms, investigations or findings rather than end-tidal or transcutaneous co2. Consult Integrated notebook, page 36, §2.1 for the full context.

REFERENCE Integrated notebook, page 36, §2.1

QUESTION 0186 · §2.1 · Clinical interpretation SBA

A 60-year-old with suspected OSA asks how a sleep-study result should be interpreted. The clinical team records the following finding: hypoventilation; a standard measurement in paediatric PSG. Video/audio — parasomnia, seizure, positional confirmation. Which concept best explains this observation?

A. RIP mechanism

B. Body position

C. Oxygen saturation nadir

D. End-tidal or transcutaneous CO2

E. EEG + EOG + EMG together determine sleep stage

CORRECT ANSWER D

This finding most directly describes End-tidal or transcutaneous CO2: hypoventilation; a standard measurement in paediatric PSG. Video/audio — parasomnia, seizure, positional confirmation. Distractor distinctions: A refers instead to thoracic and abdominal bands each contain a conductor forming a wire loop; respiration changes circumference, so cross-sectio; B refers instead to positional dependence; directly relevant to appliance and positional therapy decisions; C refers instead to The minimum oxygen level contributes information on hypoxaemic severity; E refers instead to No staging is possible without them — the fundamental limitation of level 3 and 4 studies.

REFERENCE Integrated notebook, page 36, §2.1

QUESTION 0187 · §2.2 · Applied clinical SBA

A patient on treatment asks whether a favourable symptom report proves normal breathing during sleep. Which of the following statements about respiratory disturbance index is most accurate?

A. Respiratory disturbance index includes aHI plus respiratory effort-related arousals per hour of sleep.

B. In adults, an AHI below 5/h is normal; severe OSA begins at AHI 30/h or more.

C. A respiratory effort-related arousal follows increasing respiratory effort without meeting apnoea or hypopnoea criteria.

D. Apnoea-hypopnoea index includes total scored apnoeas and hypopnoeas divided by hours of sleep.

E. Narcolepsy patients may enter sleep directly into REM.

CORRECT ANSWER A

Respiratory disturbance index includes aHI plus respiratory effort-related arousals per hour of sleep. Additional revision detail: AHI plus respiratory effort-related arousals per hour of sleep. The other options describe different mechanisms, investigations or findings rather than respiratory disturbance index. Consult Integrated notebook, page 37, §2.2 for the full context.

REFERENCE Integrated notebook, page 37, §2.2

QUESTION 0188 · §2.2 · Clinical interpretation SBA

A 57-year-old with witnessed apnoeas undergoes diagnostic polysomnography. The clinical team records the following finding: AHI plus respiratory effort-related arousals per hour of sleep. Which concept best explains this observation?

A. Apnoea-hypopnoea index

B. Chronic insomnia on PSG

C. Respiratory effort-related arousal (RERA)

D. Irregular or absent stages

E. Respiratory disturbance index

CORRECT ANSWER E

This finding most directly describes Respiratory disturbance index: AHI plus respiratory effort-related arousals per hour of sleep. Distractor distinctions: A refers instead to Total scored apnoeas and hypopnoeas divided by hours of sleep; B refers instead to increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age; C refers instead to Arousal following increasing effort or flattened airflow that does not meet apnoea or hypopnoea criteria; D refers instead to irregular cycling or missing stages point to specific disorders — for example, a patient with narcolepsy may enter sleep dire.

REFERENCE Integrated notebook, page 37, §2.2

QUESTION 0189 · §2.3 · Applied clinical SBA

A 60-year-old with suspected OSA asks how a sleep-study result should be interpreted. Which of the following statements about behavioural therapy is most accurate?

A. Surgery may be considered in refractory severe OSA when correctable anatomy is present.

B. Positional therapy where the study shows supine dependence; weight loss where obesity is present.

C. Snoring can be measured acoustically using a sound-level meter with standardised positioning.

D. Oral appliances may be offered for mild-to-moderate OSA or when CPAP is unsuitable in selected severe cases.

E. Medication may treat comorbid sleep disorders or residual sleepiness but is not primary OSA therapy.

CORRECT ANSWER B

Positional therapy where the study shows supine dependence; weight loss where obesity is present. The other options describe different mechanisms, investigations or findings rather than behavioural therapy. Consult Integrated notebook, page 48, §2.3 for the full context.

REFERENCE Integrated notebook, page 48, §2.3

QUESTION 0190 · §2.3 · Clinical interpretation SBA

A patient with cardiopulmonary comorbidity is being considered for objective sleep testing. The clinical team records the following finding: positional therapy where the study shows supine dependence; weight loss where obesity is present. Which concept best explains this observation?

A. Surgery

B. Behavioural therapy

C. Oral appliance therapy

D. MSLT interpretation

E. Pharmacotherapy

CORRECT ANSWER B

This finding most directly describes Behavioural therapy: positional therapy where the study shows supine dependence; weight loss where obesity is present. Distractor distinctions: A refers instead to considered for severe disease where CPAP and appliances are not tolerated or effective, particularly where a specific correct; C refers instead to for mild to moderate OSA, or severe OSA where CPAP is refused, not tolerated or ineffective. Positional dependence increases; D refers instead to mean sleep latency averaged across naps — shorter = more severe sleepiness; E refers instead to used for other disorders diagnosed on the study (e.g. RBD) or for residual sleepiness; it is not a primary treatment for OSA.

REFERENCE Integrated notebook, page 48, §2.3

QUESTION 0191 · §2.4 · Applied clinical SBA

A 57-year-old with witnessed apnoeas undergoes diagnostic polysomnography. Which of the following statements about complete response is most accurate?

A. Relevant findings include arousal index low, scored by the ASDA 3-second rule.

B. Complete response includes resolution of OSA symptoms and treatment AHI <5/h.

C. A partial oral appliance response includes symptom improvement and at least a 50% reduction in AHI.

D. Persistent symptoms or an insufficient objective reduction in AHI indicate treatment failure.

E. Successful OSA treatment may produce REM rebound and reduced sleep fragmentation.

CORRECT ANSWER B

Complete response includes resolution of OSA symptoms and treatment AHI <5/h. Additional revision detail: resolution of OSA symptoms and treatment AHI <5/h. The other options describe different mechanisms, investigations or findings rather than complete response. Consult Integrated notebook, page 38, §2.4 for the full context.

REFERENCE Integrated notebook, page 38, §2.4

QUESTION 0192 · §2.4 · Clinical interpretation SBA

A sleep physician is reviewing a home sleep apnoea study with a patient who snores. The clinical team records the following finding: resolution of OSA symptoms and treatment AHI <5/h. Which concept best explains this observation?

A. Type 1 advantages

B. Partial response

C. Treatment failure

D. Signature of a good outcome

E. Complete response

CORRECT ANSWER E

This finding most directly describes Complete response: resolution of OSA symptoms and treatment AHI <5/h. Distractor distinctions: A refers instead to most accurate and reliable reference standard; low failure rate including for non-OSA SDB; accurate staging and event quantif; B refers instead to improved symptoms and ≥50% AHI reduction, but AHI remains ≥5/h; C refers instead to ongoing clinical symptoms, with or without <50% AHI reduction; D refers instead to Sleep: REM increases, NREM decreases; TST typically unchanged — significant redistribution to more REM (REM rebound), not mor.

REFERENCE Integrated notebook, page 38, §2.4

QUESTION 0193 · §2.5 · Applied clinical SBA

A patient with cardiopulmonary comorbidity is being considered for objective sleep testing. Which of the following statements about type 2 is most accurate?

A. Unattended type 2 PSG is most appropriate in carefully selected patients without complicating disorders.

B. The physician should consider attended polysomnography when clinical suspicion remains high.

C. A type 1 polysomnogram is attended in a laboratory and records sleep stages and respiratory events.

D. A type 2 study uses a full PSG-type montage but is usually performed unattended outside the laboratory.

E. A type 4 home device usually records only one or two variables, such as oximetry or airflow.

CORRECT ANSWER D

A type 2 study uses a full PSG-type montage but is usually performed unattended outside the laboratory. Additional revision detail: unattended, home (occasionally laboratory-applied then home); 10–15 sensors, ≥7 channels. The other options describe different mechanisms, investigations or findings rather than type 2. Consult Integrated notebook, page 39, §2.5 for the full context.

REFERENCE Integrated notebook, page 39, §2.5

QUESTION 0194 · §2.5 · Focused knowledge SBA

A patient on treatment asks whether a favourable symptom report proves normal breathing during sleep. Which of the following statements about type 2 is most accurate?

A. Unattended type 2 PSG is most appropriate in carefully selected patients without complicating disorders.

B. The physician should consider attended polysomnography when clinical suspicion remains high.

C. A type 1 polysomnogram is attended in a laboratory and records sleep stages and respiratory events.

D. A type 4 home device usually records only one or two variables, such as oximetry or airflow.

E. A type 2 study uses a full PSG-type montage but is usually performed unattended outside the laboratory.

CORRECT ANSWER E

A type 2 study uses a full PSG-type montage but is usually performed unattended outside the laboratory. Additional revision detail: unattended, home (occasionally laboratory-applied then home); 10–15 sensors, ≥7 channels. The other options describe different mechanisms, investigations or findings rather than type 2. Consult Integrated notebook, page 39, §2.5 for the full context.

REFERENCE Integrated notebook, page 39, §2.5

QUESTION 0195 · §2.6 · Applied clinical SBA

A sleep physician is reviewing a home sleep apnoea study with a patient who snores. Which of the following statements about MWT method is most accurate?

A. A longer maintenance-of-wakefulness test latency indicates better ability to remain awake.

B. Short mean sleep latency on MSLT indicates greater physiological sleepiness.

C. MSLT method includes follows an overnight PSG to confirm adequate prior sleep.

D. MSLT indications includes diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia.

E. During MWT, the patient sits quietly in dim light and is instructed to remain awake.

CORRECT ANSWER E

During MWT, the patient sits quietly in dim light and is instructed to remain awake. Additional revision detail: the patient sits up in a chair in a quiet dimly lit room, instructed to stay awake; no vocalisation or movement. The other options describe different mechanisms, investigations or findings rather than mwt method. Consult Integrated notebook, page 40, §2.6 for the full context.

REFERENCE Integrated notebook, page 40, §2.6

QUESTION 0196 · §2.6 · Clinical interpretation SBA

A 60-year-old with suspected OSA asks how a sleep-study result should be interpreted. The clinical team records the following finding: the patient sits up in a chair in a quiet dimly lit room, instructed to stay awake; no vocalisation or movement. Which concept best explains this observation?

A. MSLT interpretation

B. MSLT indications

C. EEG (scalp)

D. MWT interpretation

E. MWT method

CORRECT ANSWER E

This finding most directly describes MWT method: the patient sits up in a chair in a quiet dimly lit room, instructed to stay awake; no vocalisation or movement. Distractor distinctions: A refers instead to mean sleep latency averaged across naps — shorter = more severe sleepiness; B refers instead to diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia; C refers instead to sleep staging and arousals. EOG (periorbital) — eye movements, identifies REM and sleep onset. EMG (submental, tibial) — musc; D refers instead to a longer mean sleep latency indicates greater ability to maintain wakefulness; considered a more accurate reflection of the a.

REFERENCE Integrated notebook, page 40, §2.6

QUESTION 0197 · §2.7 · Applied clinical SBA

A patient on treatment asks whether a favourable symptom report proves normal breathing during sleep. Which of the following statements about australian direct-referral pathway is most accurate?

A. OSA may meet diagnostic criteria at RDI above 15/h, or above 5/h with relevant symptoms.

B. An Epworth Sleepiness Scale score above 10/24 suggests excessive daytime sleepiness.

C. Selected referral pathways combine a positive OSA-risk screen with a raised Epworth score.

D. A type 1 sleep study uses approximately 15–20 sensors, whereas a type 4 study uses only 1–2.

E. An Epworth Sleepiness Scale score greater than 10 out of 24 suggests increased daytime sleepiness.

CORRECT ANSWER C

Selected referral pathways combine a positive OSA-risk screen with a raised Epworth score. Additional revision detail: positive questionnaire (Berlin positive / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8. The other options describe different mechanisms, investigations or findings rather than australian direct-referral pathway. Consult Integrated notebook, page 59, §2.7 for the full context.

REFERENCE Integrated notebook, page 59, §2.7

QUESTION 0198 · §2.7 · Clinical interpretation SBA

A 57-year-old with witnessed apnoeas undergoes diagnostic polysomnography. The clinical team records the following finding: positive questionnaire (Berlin positive / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8. Which concept best explains this observation?

A. Australian direct-referral pathway

B. Mouth breathing with dry mouth

C. Epworth Sleepiness Scale threshold

D. High palatal vault / narrow arch

E. Screening thresholds

CORRECT ANSWER A

This finding most directly describes Australian direct-referral pathway: positive questionnaire (Berlin positive / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8. Distractor distinctions: B refers instead to compensation for nasal obstruction. Bruxism — common in disrupted sleep and airway problems. Dental erosion — may indicate GO; C refers instead to More than ten points of the possible twenty-four suggests excessive daytime sleepiness; D refers instead to maxillary constriction reduces oral cavity space; narrow tapered V-shaped maxillary arch, possibly with posterior crossbites; E refers instead to ESS >10 · STOP-BANG ≥3 · OSA50 ≥5 · Berlin ≥2 categories · NoSAS ≥8 · PSQ >0.33.

REFERENCE Integrated notebook, page 59, §2.7

QUESTION 0199 · §2.1 · Applied clinical SBA

A 57-year-old with witnessed apnoeas undergoes diagnostic polysomnography. Which of the following statements about oxygen saturation nadir is most accurate?

A. Sleep staging requires EEG, electro-oculography and electromyography signals.

B. Body position includes positional dependence; directly relevant to appliance and positional therapy decisions.

C. Snoring can be measured acoustically using a sound-level meter with standardised positioning.

D. The minimum oxygen level contributes information on hypoxaemic severity.

E. An oronasal thermal sensor helps identify apnoea, while nasal pressure measures hypopnoea.

CORRECT ANSWER D

The minimum oxygen level contributes information on hypoxaemic severity. The other options describe different mechanisms, investigations or findings rather than oxygen saturation nadir. Consult Integrated notebook, page 36, §2.1 for the full context.

REFERENCE Integrated notebook, page 36, §2.1

QUESTION 0200 · §2.1 · Clinical interpretation SBA

A sleep physician is reviewing a home sleep apnoea study with a patient who snores. The clinical team records the following finding: The minimum oxygen level contributes information on hypoxaemic severity. Which concept best explains this observation?

A. Oxygen saturation nadir

B. Body position

C. End-tidal or transcutaneous CO2

D. EEG (scalp)

E. RIP mechanism

CORRECT ANSWER A

This finding most directly describes Oxygen saturation nadir: The minimum oxygen level contributes information on hypoxaemic severity. Distractor distinctions: B refers instead to positional dependence; directly relevant to appliance and positional therapy decisions; C refers instead to hypoventilation; a standard measurement in paediatric PSG. Video/audio — parasomnia, seizure, positional confirmation; D refers instead to sleep staging and arousals. EOG (periorbital) — eye movements, identifies REM and sleep onset. EMG (submental, tibial) — musc; E refers instead to thoracic and abdominal bands each contain a conductor forming a wire loop; respiration changes circumference, so cross-sectio.

REFERENCE Integrated notebook, page 36, §2.1

QUESTION 0201 · §2.2 · Applied clinical SBA

A patient with cardiopulmonary comorbidity is being considered for objective sleep testing. Which of the following statements about apnoea-hypopnoea index is most accurate?

A. Apnoea-hypopnoea index includes total scored apnoeas and hypopnoeas divided by hours of sleep.

B. A respiratory effort-related arousal follows increasing respiratory effort without meeting apnoea or hypopnoea criteria.

C. Narcolepsy patients may enter sleep directly into REM.

D. Respiratory disturbance index includes aHI plus respiratory effort-related arousals per hour of sleep.

E. In adults, an AHI below 5/h is normal; severe OSA begins at AHI 30/h or more.

CORRECT ANSWER A

Apnoea-hypopnoea index includes total scored apnoeas and hypopnoeas divided by hours of sleep. Additional revision detail: Total scored apnoeas and hypopnoeas divided by hours of sleep. The other options describe different mechanisms, investigations or findings rather than apnoea-hypopnoea index. Consult Integrated notebook, page 37, §2.2 for the full context.

REFERENCE Integrated notebook, page 37, §2.2

QUESTION 0202 · §2.2 · Clinical interpretation SBA

A patient on treatment asks whether a favourable symptom report proves normal breathing during sleep. The clinical team records the following finding: Total scored apnoeas and hypopnoeas divided by hours of sleep. Which concept best explains this observation?

A. Chronic insomnia on PSG

B. Hypopnoea

C. Apnoea

D. Apnoea-hypopnoea index

E. Respiratory disturbance index

CORRECT ANSWER D

This finding most directly describes Apnoea-hypopnoea index: Total scored apnoeas and hypopnoeas divided by hours of sleep. Distractor distinctions: A refers instead to increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age; B refers instead to reduction in airflow and/or thoracoabdominal movement of ≥30% of baseline for ≥10 s, with ≥3% desaturation and/or an arousal; C refers instead to cessation of airflow (≥90% reduction) for ≥10 s. Obstructive if effort continues; central if effort absent; mixed if it begin; E refers instead to AHI plus respiratory effort-related arousals per hour of sleep.

REFERENCE Integrated notebook, page 37, §2.2

QUESTION 0203 · §2.5 · Applied clinical SBA

A patient with cardiopulmonary comorbidity is being considered for objective sleep testing. Which of the following statements about type 4 is most accurate?

A. Unattended type 2 PSG is most appropriate in carefully selected patients without complicating disorders.

B. A type 4 home device usually records only one or two variables, such as oximetry or airflow.

C. A type 3 home sleep test monitors respiratory variables with approximately four to seven sensors but does not stage sleep.

D. The physician should consider attended polysomnography when clinical suspicion remains high.

E. A type 2 study uses a full PSG-type montage but is usually performed unattended outside the laboratory.

CORRECT ANSWER B

A type 4 home device usually records only one or two variables, such as oximetry or airflow. Additional revision detail: home; 1–2 bioparameters, typically oximetry ± airflow or heart rate. The other options describe different mechanisms, investigations or findings rather than type 4. Consult Integrated notebook, page 39, §2.5 for the full context.

REFERENCE Integrated notebook, page 39, §2.5

QUESTION 0204 · §2.5 · Focused knowledge SBA

A patient on treatment asks whether a favourable symptom report proves normal breathing during sleep. Which of the following statements about type 4 is most accurate?

A. Unattended type 2 PSG is most appropriate in carefully selected patients without complicating disorders.

B. A type 3 home sleep test monitors respiratory variables with approximately four to seven sensors but does not stage sleep.

C. A type 2 study uses a full PSG-type montage but is usually performed unattended outside the laboratory.

D. The physician should consider attended polysomnography when clinical suspicion remains high.

E. A type 4 home device usually records only one or two variables, such as oximetry or airflow.

CORRECT ANSWER E

A type 4 home device usually records only one or two variables, such as oximetry or airflow. Additional revision detail: home; 1–2 bioparameters, typically oximetry ± airflow or heart rate. The other options describe different mechanisms, investigations or findings rather than type 4. Consult Integrated notebook, page 39, §2.5 for the full context.

REFERENCE Integrated notebook, page 39, §2.5

QUESTION 0205 · §2.6 · Applied clinical SBA

A sleep physician is reviewing a home sleep apnoea study with a patient who snores. Which of the following statements about MSLT interpretation is most accurate?

A. MSLT method includes follows an overnight PSG to confirm adequate prior sleep.

B. During MWT, the patient sits quietly in dim light and is instructed to remain awake.

C. Short mean sleep latency on MSLT indicates greater physiological sleepiness.

D. A longer maintenance-of-wakefulness test latency indicates better ability to remain awake.

E. MSLT indications includes diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia.

CORRECT ANSWER C

Short mean sleep latency on MSLT indicates greater physiological sleepiness. Additional revision detail: mean sleep latency averaged across naps — shorter = more severe sleepiness. The other options describe different mechanisms, investigations or findings rather than mslt interpretation. Consult Integrated notebook, page 40, §2.6 for the full context.

REFERENCE Integrated notebook, page 40, §2.6

QUESTION 0206 · §2.6 · Clinical interpretation SBA

A 60-year-old with suspected OSA asks how a sleep-study result should be interpreted. The clinical team records the following finding: mean sleep latency averaged across naps — shorter = more severe sleepiness. Which concept best explains this observation?

A. MSLT interpretation

B. MWT method

C. MSLT indications

D. Behavioural therapy

E. MSLT method

CORRECT ANSWER A

This finding most directly describes MSLT interpretation: mean sleep latency averaged across naps — shorter = more severe sleepiness. Distractor distinctions: B refers instead to the patient sits up in a chair in a quiet dimly lit room, instructed to stay awake; no vocalisation or movement; C refers instead to diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia; D refers instead to positional therapy where the study shows supine dependence; weight loss where obesity is present; E refers instead to follows an overnight PSG to confirm adequate prior sleep.

REFERENCE Integrated notebook, page 40, §2.6

QUESTION 0207 · §2.7 · Applied clinical SBA

A patient on treatment asks whether a favourable symptom report proves normal breathing during sleep. Which of the following statements about mouth breathing with dry mouth is most accurate?

A. An Epworth Sleepiness Scale score greater than 10 out of 24 suggests increased daytime sleepiness.

B. Mouth breathing with dry mouth includes compensation for nasal obstruction.

C. STOP-BANG combines symptoms with blood pressure and anthropometric risk factors.

D. Maxillary constriction reduces oral cavity space.

E. OSA may meet diagnostic criteria at RDI above 15/h, or above 5/h with relevant symptoms.

CORRECT ANSWER B

Mouth breathing with dry mouth includes compensation for nasal obstruction. Additional revision detail: compensation for nasal obstruction. Bruxism — common in disrupted sleep and airway problems. Dental erosion — may indicate GORD, which frequently coexists. The other options describe different mechanisms, investigations or findings rather than mouth breathing with dry mouth. Consult Integrated notebook, page 42, §2.7 for the full context.

REFERENCE Integrated notebook, page 42, §2.7

QUESTION 0208 · §2.7 · Clinical interpretation SBA

A 57-year-old with witnessed apnoeas undergoes diagnostic polysomnography. The clinical team records the following finding: compensation for nasal obstruction. Bruxism — common in disrupted sleep and airway problems. Dental erosion — may indicate GORD, which frequently coexists. Which concept best explains this observation?

A. Australian direct referral

B. High palatal vault / narrow arch

C. Epworth Sleepiness Scale threshold

D. Mouth breathing with dry mouth

E. Australian direct-referral pathway

CORRECT ANSWER D

This finding most directly describes Mouth breathing with dry mouth: compensation for nasal obstruction. Bruxism — common in disrupted sleep and airway problems. Dental erosion — may indicate GORD, which frequently coexists. Distractor distinctions: A refers instead to positive questionnaire (Berlin / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8; B refers instead to maxillary constriction reduces oral cavity space; narrow tapered V-shaped maxillary arch, possibly with posterior crossbites; C refers instead to More than ten points of the possible twenty-four suggests excessive daytime sleepiness; E refers instead to positive questionnaire (Berlin positive / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8.

REFERENCE Integrated notebook, page 42, §2.7

QUESTION 0209 · §2.1 · Applied clinical SBA

A 57-year-old with witnessed apnoeas undergoes diagnostic polysomnography. Which of the following statements about body position is most accurate?

A. Sleep staging requires EEG, electro-oculography and electromyography signals.

B. Body position includes positional dependence; directly relevant to appliance and positional therapy decisions.

C. Snoring can be measured acoustically using a sound-level meter with standardised positioning.

D. Relevant findings include arousal index low, scored by the ASDA 3-second rule.

E. An oronasal thermal sensor helps identify apnoea, while nasal pressure measures hypopnoea.

CORRECT ANSWER B

Body position includes positional dependence; directly relevant to appliance and positional therapy decisions. Additional revision detail: positional dependence; directly relevant to appliance and positional therapy decisions. The other options describe different mechanisms, investigations or findings rather than body position. Consult Integrated notebook, page 36, §2.1 for the full context.

REFERENCE Integrated notebook, page 36, §2.1

QUESTION 0210 · §2.1 · Clinical interpretation SBA

A sleep physician is reviewing a home sleep apnoea study with a patient who snores. The clinical team records the following finding: positional dependence; directly relevant to appliance and positional therapy decisions. Which concept best explains this observation?

A. Body position

B. EEG (scalp)

C. RIP mechanism

D. Airflow

E. Snoring

CORRECT ANSWER A

This finding most directly describes Body position: positional dependence; directly relevant to appliance and positional therapy decisions. Distractor distinctions: B refers instead to sleep staging and arousals. EOG (periorbital) — eye movements, identifies REM and sleep onset. EMG (submental, tibial) — musc; C refers instead to thoracic and abdominal bands each contain a conductor forming a wire loop; respiration changes circumference, so cross-sectio; D refers instead to oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both; E refers instead to microphone/sound level meter 1 m above the head; frequency per hour and intensity in dB.

REFERENCE Integrated notebook, page 36, §2.1

QUESTION 0211 · §2.5 · Applied clinical SBA

A sleep physician is reviewing a home sleep apnoea study with a patient who snores. Which of the following statements about high-risk patient with negative hsat is most accurate?

A. Unattended type 2 PSG is most appropriate in carefully selected patients without complicating disorders.

B. The physician should consider attended polysomnography when clinical suspicion remains high.

C. A type 4 home device usually records only one or two variables, such as oximetry or airflow.

D. A type 2 study uses a full PSG-type montage but is usually performed unattended outside the laboratory.

E. Type 1 indications includes low probability of moderate–severe OSA.

CORRECT ANSWER B

The physician should consider attended polysomnography when clinical suspicion remains high. The other options describe different mechanisms, investigations or findings rather than high-risk patient with negative hsat. Consult Integrated notebook, page 39, §2.5 for the full context.

REFERENCE Integrated notebook, page 39, §2.5

QUESTION 0212 · §2.5 · Clinical interpretation SBA

A 60-year-old with suspected OSA asks how a sleep-study result should be interpreted. The clinical team records the following finding: The physician should consider attended polysomnography when clinical suspicion remains high. Which concept best explains this observation?

A. Type 1

B. Type 1 advantages

C. Type 1 indications

D. Type 2 indications

E. High-risk patient with negative HSAT

CORRECT ANSWER E

This finding most directly describes High-risk patient with negative HSAT: The physician should consider attended polysomnography when clinical suspicion remains high. Distractor distinctions: A refers instead to attended, in-laboratory; 15–20 sensors, ≥7 channels. Full montage: EEG, EOG, EMG, ECG, oximetry, airflow, effort, position, l; B refers instead to most accurate and reliable reference standard; low failure rate including for non-OSA SDB; accurate staging and event quantif; C refers instead to low probability of moderate–severe OSA; unsuitable for a type 2 study; failed type 2 study. Also the standard for narcolepsy,; D refers instead to high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder.

REFERENCE Integrated notebook, page 39, §2.5

QUESTION 0213 · §2.7 · Applied clinical SBA

A 57-year-old with witnessed apnoeas undergoes diagnostic polysomnography. Which of the following statements about australian direct referral is most accurate?

A. OSA may meet diagnostic criteria at RDI above 15/h, or above 5/h with relevant symptoms.

B. Australian direct referral includes positive questionnaire (Berlin / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8.

C. A type 1 sleep study uses approximately 15–20 sensors, whereas a type 4 study uses only 1–2.

D. An Epworth Sleepiness Scale score above 10/24 suggests excessive daytime sleepiness.

E. An Epworth Sleepiness Scale score greater than 10 out of 24 suggests increased daytime sleepiness.

CORRECT ANSWER B

Australian direct referral includes positive questionnaire (Berlin / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8. Additional revision detail: positive questionnaire (Berlin / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8. The other options describe different mechanisms, investigations or findings rather than australian direct referral. Consult Integrated notebook, page 43, §2.7 for the full context.

REFERENCE Integrated notebook, page 43, §2.7

QUESTION 0214 · §2.7 · Clinical interpretation SBA

A sleep physician is reviewing a home sleep apnoea study with a patient who snores. The clinical team records the following finding: positive questionnaire (Berlin / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8. Which concept best explains this observation?

A. High palatal vault / narrow arch

B. Modified Mallampati

C. Australian direct referral

D. Mouth breathing with dry mouth

E. Screening thresholds

CORRECT ANSWER C

This finding most directly describes Australian direct referral: positive questionnaire (Berlin / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8. Distractor distinctions: A refers instead to maxillary constriction reduces oral cavity space; narrow tapered V-shaped maxillary arch, possibly with posterior crossbites; B refers instead to describes oral airway patency. Technique: patient sits upright, opens widely, protrudes the tongue maximally without phonatio; D refers instead to compensation for nasal obstruction. Bruxism — common in disrupted sleep and airway problems. Dental erosion — may indicate GO; E refers instead to ESS >10 · STOP-BANG ≥3 · OSA50 ≥5 · Berlin ≥2 categories · NoSAS ≥8 · PSQ >0.33.

REFERENCE Integrated notebook, page 43, §2.7

QUESTION 0215 · §2.1 · Applied clinical SBA

A sleep physician is reviewing a home sleep apnoea study with a patient who snores. Which of the following statements about airflow is most accurate?

A. An oronasal thermal sensor helps identify apnoea, while nasal pressure measures hypopnoea.

B. Body position includes positional dependence; directly relevant to appliance and positional therapy decisions.

C. Sleep staging requires EEG, electro-oculography and electromyography signals.

D. The minimum oxygen level contributes information on hypoxaemic severity.

E. Snoring can be measured acoustically using a sound-level meter with standardised positioning.

CORRECT ANSWER A

An oronasal thermal sensor helps identify apnoea, while nasal pressure measures hypopnoea. Additional revision detail: oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both. The other options describe different mechanisms, investigations or findings rather than airflow. Consult Integrated notebook, page 36, §2.1 for the full context.

REFERENCE Integrated notebook, page 36, §2.1

QUESTION 0216 · §2.1 · Focused knowledge SBA

A 60-year-old with suspected OSA asks how a sleep-study result should be interpreted. Which of the following statements about airflow is most accurate?

A. The minimum oxygen level contributes information on hypoxaemic severity.

B. Body position includes positional dependence; directly relevant to appliance and positional therapy decisions.

C. Sleep staging requires EEG, electro-oculography and electromyography signals.

D. Snoring can be measured acoustically using a sound-level meter with standardised positioning.

E. An oronasal thermal sensor helps identify apnoea, while nasal pressure measures hypopnoea.

CORRECT ANSWER E

An oronasal thermal sensor helps identify apnoea, while nasal pressure measures hypopnoea. Additional revision detail: oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both. The other options describe different mechanisms, investigations or findings rather than airflow. Consult Integrated notebook, page 36, §2.1 for the full context.

REFERENCE Integrated notebook, page 36, §2.1

QUESTION 0217 · §2.2 · Applied clinical SBA

A patient on treatment asks whether a favourable symptom report proves normal breathing during sleep. Which of the following statements about severity is most accurate?

A. In adults, an AHI below 5/h is normal; severe OSA begins at AHI 30/h or more.

B. Respiratory disturbance index includes aHI plus respiratory effort-related arousals per hour of sleep.

C. Sleep efficiency can fall when sleep is fragmented by repeated respiratory events.

D. An adult hypopnoea requires an airflow reduction of at least 30% for 10 s with an arousal or qualifying desaturation.

E. A respiratory effort-related arousal follows increasing respiratory effort without meeting apnoea or hypopnoea criteria.

CORRECT ANSWER A

In adults, an AHI below 5/h is normal; severe OSA begins at AHI 30/h or more. Additional revision detail: adult: normal <5 · mild 5–<15 · moderate 15–<30 · severe ≥30. Paediatric: <1 · 1–<5 · 5–<10 · ≥10. The other options describe different mechanisms, investigations or findings rather than severity. Consult Integrated notebook, page 37, §2.2 for the full context.

REFERENCE Integrated notebook, page 37, §2.2

QUESTION 0218 · §2.2 · Clinical interpretation SBA

A 57-year-old with witnessed apnoeas undergoes diagnostic polysomnography. The clinical team records the following finding: adult: normal <5 · mild 5–<15 · moderate 15–<30 · severe ≥30. Paediatric: <1 · 1–<5 · 5–<10 · ≥10. Which concept best explains this observation?

A. Fragmentation

B. Apnoea-hypopnoea index

C. Apnoea

D. Severity

E. Hypopnoea

CORRECT ANSWER D

This finding most directly describes Severity: adult: normal <5 · mild 5–<15 · moderate 15–<30 · severe ≥30. Paediatric: <1 · 1–<5 · 5–<10 · ≥10. Distractor distinctions: A refers instead to increased arousal index, increased stage shifts, increased N1, reduced slow-wave and REM sleep: the signature of untreated OS; B refers instead to Total scored apnoeas and hypopnoeas divided by hours of sleep; C refers instead to cessation of airflow (≥90% reduction) for ≥10 s. Obstructive if effort continues; central if effort absent; mixed if it begin; E refers instead to reduction in airflow and/or thoracoabdominal movement of ≥30% of baseline for ≥10 s, with ≥3% desaturation and/or an arousal.

REFERENCE Integrated notebook, page 37, §2.2

QUESTION 0219 · §2.5 · Applied clinical SBA

A patient on treatment asks whether a favourable symptom report proves normal breathing during sleep. Which of the following statements about type 1 indications is most accurate?

A. Type 1 indications includes low probability of moderate–severe OSA.

B. A type 1 polysomnogram is attended in a laboratory and records sleep stages and respiratory events.

C. Unattended type 2 PSG is most appropriate in carefully selected patients without complicating disorders.

D. A type 3 home sleep test monitors respiratory variables with approximately four to seven sensors but does not stage sleep.

E. Attended laboratory polysomnography remains the reference standard for characterising complex sleep disorders.

CORRECT ANSWER A

Type 1 indications includes low probability of moderate–severe OSA. Additional revision detail: low probability of moderate–severe OSA; unsuitable for a type 2 study; failed type 2 study. Also the standard for narcolepsy, central sleep apnoea, parasomnias and movement disorders. The other options describe different mechanisms, investigations or findings rather than type 1 indications. Consult Integrated notebook, page 39, §2.5 for the full context.

REFERENCE Integrated notebook, page 39, §2.5

QUESTION 0220 · §2.5 · Clinical interpretation SBA

A 57-year-old with witnessed apnoeas undergoes diagnostic polysomnography. The clinical team records the following finding: low probability of moderate–severe OSA; unsuitable for a type 2 study; failed type 2 study. Also the standard for narcolepsy, central sleep apnoea, parasomnias and movement disorders. Which concept best explains this observation?

A. Type 1

B. Type 2

C. Type 1 advantages

D. Type 1 indications

E. Type 3

CORRECT ANSWER D

This finding most directly describes Type 1 indications: low probability of moderate–severe OSA; unsuitable for a type 2 study; failed type 2 study. Also the standard for narcolepsy, central sleep apnoea, parasomnias and movement disorders. Distractor distinctions: A refers instead to attended, in-laboratory; 15–20 sensors, ≥7 channels. Full montage: EEG, EOG, EMG, ECG, oximetry, airflow, effort, position, l; B refers instead to unattended, home (occasionally laboratory-applied then home); 10–15 sensors, ≥7 channels; C refers instead to most accurate and reliable reference standard; low failure rate including for non-OSA SDB; accurate staging and event quantif; E refers instead to home; 4–7 sensors. Limited channel monitoring of breathing parameters without sleep assessment: airflow (≥2 channels of respi.

REFERENCE Integrated notebook, page 39, §2.5

QUESTION 0221 · §2.7 · Applied clinical SBA

A patient with cardiopulmonary comorbidity is being considered for objective sleep testing. Which of the following statements about screening thresholds is most accurate?

A. An Epworth Sleepiness Scale score greater than 10 out of 24 suggests increased daytime sleepiness.

B. OSA may meet diagnostic criteria at RDI above 15/h, or above 5/h with relevant symptoms.

C. An Epworth Sleepiness Scale score above 10/24 suggests excessive daytime sleepiness.

D. Selected referral pathways combine a positive OSA-risk screen with a raised Epworth score.

E. A type 1 sleep study uses approximately 15–20 sensors, whereas a type 4 study uses only 1–2.

CORRECT ANSWER C

An Epworth Sleepiness Scale score above 10/24 suggests excessive daytime sleepiness. Additional revision detail: ESS >10 · STOP-BANG ≥3 · OSA50 ≥5 · Berlin ≥2 categories · NoSAS ≥8 · PSQ >0.33. The other options describe different mechanisms, investigations or findings rather than screening thresholds. Consult Integrated notebook, page 59, §2.7 for the full context.

REFERENCE Integrated notebook, page 59, §2.7

QUESTION 0222 · §2.7 · Clinical interpretation SBA

A patient on treatment asks whether a favourable symptom report proves normal breathing during sleep. The clinical team records the following finding: ESS >10 · STOP-BANG ≥3 · OSA50 ≥5 · Berlin ≥2 categories · NoSAS ≥8 · PSQ >0.33. Which concept best explains this observation?

A. Macroglossia

B. Australian direct referral

C. Screening thresholds

D. Modified Mallampati

E. OSA diagnosis

CORRECT ANSWER C

This finding most directly describes Screening thresholds: ESS >10 · STOP-BANG ≥3 · OSA50 ≥5 · Berlin ≥2 categories · NoSAS ≥8 · PSQ >0.33. Distractor distinctions: A refers instead to scalloping (crenations) of the lateral tongue borders suggests the tongue is too wide for the arch, causing oropharyngeal cro; B refers instead to positive questionnaire (Berlin / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8; D refers instead to describes oral airway patency. Technique: patient sits upright, opens widely, protrudes the tongue maximally without phonatio; E refers instead to RDI >15/h, or >5/h with symptoms. AHI ≥15 → material increase in metabolic and vascular risk.

REFERENCE Integrated notebook, page 59, §2.7

QUESTION 0223 · §2.1 · Applied clinical SBA

A patient on treatment asks whether a favourable symptom report proves normal breathing during sleep. Which of the following statements about eeg (scalp) is most accurate?

A. Respiratory inductance plethysmography detects breathing effort through changes in thoracoabdominal band inductance.

B. Thoracic and abdominal respiratory effort belts help distinguish obstructive from central apnoeas.

C. Eeg (scalp) includes sleep staging and arousals.

D. Sleep staging requires EEG, electro-oculography and electromyography signals.

E. Carbon dioxide monitoring helps detect hypoventilation during paediatric polysomnography.

CORRECT ANSWER C

Eeg (scalp) includes sleep staging and arousals. Additional revision detail: sleep staging and arousals. EOG (periorbital) — eye movements, identifies REM and sleep onset. EMG (submental, tibial) — muscle tone/REM atonia, leg movements; masseter EMG for bruxism. The other options describe different mechanisms, investigations or findings rather than eeg (scalp). Consult Integrated notebook, page 36, §2.1 for the full context.

REFERENCE Integrated notebook, page 36, §2.1

QUESTION 0224 · §2.1 · Clinical interpretation SBA

A 57-year-old with witnessed apnoeas undergoes diagnostic polysomnography. The clinical team records the following finding: sleep staging and arousals. EOG (periorbital) — eye movements, identifies REM and sleep onset. EMG (submental, tibial) — muscle tone/REM atonia, leg movements; masseter EMG for bruxism. Which concept best explains this observation?

A. EEG (scalp)

B. Airflow

C. RIP mechanism

D. Body position

E. EEG + EOG + EMG together determine sleep stage

CORRECT ANSWER A

This finding most directly describes EEG (scalp): sleep staging and arousals. EOG (periorbital) — eye movements, identifies REM and sleep onset. EMG (submental, tibial) — muscle tone/REM atonia, leg movements; masseter EMG for bruxism. Distractor distinctions: B refers instead to oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both; C refers instead to thoracic and abdominal bands each contain a conductor forming a wire loop; respiration changes circumference, so cross-sectio; D refers instead to positional dependence; directly relevant to appliance and positional therapy decisions; E refers instead to No staging is possible without them — the fundamental limitation of level 3 and 4 studies.

REFERENCE Integrated notebook, page 36, §2.1

QUESTION 0225 · §2.2 · Applied clinical SBA

A 60-year-old with suspected OSA asks how a sleep-study result should be interpreted. Which of the following statements about apnoea is most accurate?

A. Irregular cycling or missing stages point to specific disorders, for example.

B. An adult hypopnoea requires an airflow reduction of at least 30% for 10 s with an arousal or qualifying desaturation.

C. Untreated OSA often increases sleep fragmentation and reduces consolidated slow-wave and REM sleep.

D. A respiratory effort-related arousal follows increasing respiratory effort without meeting apnoea or hypopnoea criteria.

E. An apnoea is scored when airflow decreases by at least 90% for at least 10 seconds.

CORRECT ANSWER E

An apnoea is scored when airflow decreases by at least 90% for at least 10 seconds. Additional revision detail: cessation of airflow (≥90% reduction) for ≥10 s. Obstructive if effort continues; central if effort absent; mixed if it begins central and ends obstructive. The other options describe different mechanisms, investigations or findings rather than apnoea. Consult Integrated notebook, page 37, §2.2 for the full context.

REFERENCE Integrated notebook, page 37, §2.2

QUESTION 0226 · §2.2 · Focused knowledge SBA

A patient with cardiopulmonary comorbidity is being considered for objective sleep testing. Which of the following statements about apnoea is most accurate?

A. Untreated OSA often increases sleep fragmentation and reduces consolidated slow-wave and REM sleep.

B. A respiratory effort-related arousal follows increasing respiratory effort without meeting apnoea or hypopnoea criteria.

C. An adult hypopnoea requires an airflow reduction of at least 30% for 10 s with an arousal or qualifying desaturation.

D. Irregular cycling or missing stages point to specific disorders, for example.

E. An apnoea is scored when airflow decreases by at least 90% for at least 10 seconds.

CORRECT ANSWER E

An apnoea is scored when airflow decreases by at least 90% for at least 10 seconds. Additional revision detail: cessation of airflow (≥90% reduction) for ≥10 s. Obstructive if effort continues; central if effort absent; mixed if it begins central and ends obstructive. The other options describe different mechanisms, investigations or findings rather than apnoea. Consult Integrated notebook, page 37, §2.2 for the full context.

REFERENCE Integrated notebook, page 37, §2.2

QUESTION 0227 · §2.5 · Applied clinical SBA

A 60-year-old with suspected OSA asks how a sleep-study result should be interpreted. Which of the following statements about type 1 advantages is most accurate?

A. Unattended type 2 PSG is most appropriate in carefully selected patients without complicating disorders.

B. Attended laboratory polysomnography remains the reference standard for characterising complex sleep disorders.

C. A type 3 home sleep test monitors respiratory variables with approximately four to seven sensors but does not stage sleep.

D. Type 1 indications includes low probability of moderate–severe OSA.

E. A type 1 polysomnogram is attended in a laboratory and records sleep stages and respiratory events.

CORRECT ANSWER B

Attended laboratory polysomnography remains the reference standard for characterising complex sleep disorders. Additional revision detail: most accurate and reliable reference standard; low failure rate including for non-OSA SDB; accurate staging and event quantification; expert supervision permitting real-time sensor troubleshooting and immediate comfort intervention. The other options describe different mechanisms, investigations or findings rather than type 1 advantages. Consult Integrated notebook, page 39, §2.5 for the full context.

REFERENCE Integrated notebook, page 39, §2.5

QUESTION 0228 · §2.5 · Focused knowledge SBA

A patient with cardiopulmonary comorbidity is being considered for objective sleep testing. Which of the following statements about type 1 advantages is most accurate?

A. Type 1 indications includes low probability of moderate–severe OSA.

B. A type 3 home sleep test monitors respiratory variables with approximately four to seven sensors but does not stage sleep.

C. Unattended type 2 PSG is most appropriate in carefully selected patients without complicating disorders.

D. A type 1 polysomnogram is attended in a laboratory and records sleep stages and respiratory events.

E. Attended laboratory polysomnography remains the reference standard for characterising complex sleep disorders.

CORRECT ANSWER E

Attended laboratory polysomnography remains the reference standard for characterising complex sleep disorders. Additional revision detail: most accurate and reliable reference standard; low failure rate including for non-OSA SDB; accurate staging and event quantification; expert supervision permitting real-time sensor troubleshooting and immediate comfort intervention. The other options describe different mechanisms, investigations or findings rather than type 1 advantages. Consult Integrated notebook, page 39, §2.5 for the full context.

REFERENCE Integrated notebook, page 39, §2.5

QUESTION 0229 · §2.7 · Applied clinical SBA

A sleep physician is reviewing a home sleep apnoea study with a patient who snores. Which of the following statements about high palatal vault / narrow arch is most accurate?

A. Mouth breathing with dry mouth includes compensation for nasal obstruction.

B. An Epworth Sleepiness Scale score greater than 10 out of 24 suggests increased daytime sleepiness.

C. OSA may meet diagnostic criteria at RDI above 15/h, or above 5/h with relevant symptoms.

D. Maxillary constriction reduces oral cavity space.

E. Scalloping (crenations) of the lateral tongue borders suggests the tongue is too wide for the arch, causing oropharyngeal crowding.

CORRECT ANSWER D

Maxillary constriction reduces oral cavity space. Additional revision detail: maxillary constriction reduces oral cavity space; narrow tapered V-shaped maxillary arch, possibly with posterior crossbites or compensatory mandibular constriction. The other options describe different mechanisms, investigations or findings rather than high palatal vault / narrow arch. Consult Integrated notebook, page 42, §2.7 for the full context.

REFERENCE Integrated notebook, page 42, §2.7

QUESTION 0230 · §2.7 · Clinical interpretation SBA

A 60-year-old with suspected OSA asks how a sleep-study result should be interpreted. The clinical team records the following finding: maxillary constriction reduces oral cavity space; narrow tapered V-shaped maxillary arch, possibly with posterior crossbites or compensatory mandibular constriction. Which concept best explains this observation?

A. High palatal vault / narrow arch

B. Mouth breathing with dry mouth

C. Epworth Sleepiness Scale threshold

D. Australian direct-referral pathway

E. Australian direct referral

CORRECT ANSWER A

This finding most directly describes High palatal vault / narrow arch: maxillary constriction reduces oral cavity space; narrow tapered V-shaped maxillary arch, possibly with posterior crossbites or compensatory mandibular constriction. Distractor distinctions: B refers instead to compensation for nasal obstruction. Bruxism — common in disrupted sleep and airway problems. Dental erosion — may indicate GO; C refers instead to More than ten points of the possible twenty-four suggests excessive daytime sleepiness; D refers instead to positive questionnaire (Berlin positive / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8; E refers instead to positive questionnaire (Berlin / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8.

REFERENCE Integrated notebook, page 42, §2.7

QUESTION 0231 · §2.1 · Applied clinical SBA

A 60-year-old with suspected OSA asks how a sleep-study result should be interpreted. Which of the following statements about RIP mechanism is most accurate?

A. Thoracic and abdominal respiratory effort belts help distinguish obstructive from central apnoeas.

B. Respiratory inductance plethysmography detects breathing effort through changes in thoracoabdominal band inductance.

C. Snoring can be measured acoustically using a sound-level meter with standardised positioning.

D. Carbon dioxide monitoring helps detect hypoventilation during paediatric polysomnography.

E. Eeg (scalp) includes sleep staging and arousals.

CORRECT ANSWER B

Respiratory inductance plethysmography detects breathing effort through changes in thoracoabdominal band inductance. Additional revision detail: thoracic and abdominal bands each contain a conductor forming a wire loop; respiration changes circumference, so cross-sectional area and hence inductance change; a sensor converts this to a voltage reflecting thoracoabdominal volume. The other options describe different mechanisms, investigations or findings rather than rip mechanism. Consult Integrated notebook, page 36, §2.1 for the full context.

REFERENCE Integrated notebook, page 36, §2.1

QUESTION 0232 · §2.1 · Focused knowledge SBA

A patient with cardiopulmonary comorbidity is being considered for objective sleep testing. Which of the following statements about RIP mechanism is most accurate?

A. Respiratory inductance plethysmography detects breathing effort through changes in thoracoabdominal band inductance.

B. Snoring can be measured acoustically using a sound-level meter with standardised positioning.

C. Carbon dioxide monitoring helps detect hypoventilation during paediatric polysomnography.

D. Eeg (scalp) includes sleep staging and arousals.

E. Thoracic and abdominal respiratory effort belts help distinguish obstructive from central apnoeas.

CORRECT ANSWER A

Respiratory inductance plethysmography detects breathing effort through changes in thoracoabdominal band inductance. Additional revision detail: thoracic and abdominal bands each contain a conductor forming a wire loop; respiration changes circumference, so cross-sectional area and hence inductance change; a sensor converts this to a voltage reflecting thoracoabdominal volume. The other options describe different mechanisms, investigations or findings rather than rip mechanism. Consult Integrated notebook, page 36, §2.1 for the full context.

REFERENCE Integrated notebook, page 36, §2.1

QUESTION 0233 · §2.2 · Applied clinical SBA

A 57-year-old with witnessed apnoeas undergoes diagnostic polysomnography. Which of the following statements about irregular or absent stages is most accurate?

A. A respiratory effort-related arousal follows increasing respiratory effort without meeting apnoea or hypopnoea criteria.

B. Untreated OSA often increases sleep fragmentation and reduces consolidated slow-wave and REM sleep.

C. An adult hypopnoea requires an airflow reduction of at least 30% for 10 s with an arousal or qualifying desaturation.

D. Irregular cycling or missing stages point to specific disorders, for example.

E. An apnoea is scored when airflow decreases by at least 90% for at least 10 seconds.

CORRECT ANSWER D

Irregular cycling or missing stages point to specific disorders, for example. Additional revision detail: irregular cycling or missing stages point to specific disorders — for example, a patient with narcolepsy may enter sleep directly into REM (sleep-onset REM period). The other options describe different mechanisms, investigations or findings rather than irregular or absent stages. Consult Integrated notebook, page 47, §2.2 for the full context.

REFERENCE Integrated notebook, page 47, §2.2

QUESTION 0234 · §2.2 · Clinical interpretation SBA

A sleep physician is reviewing a home sleep apnoea study with a patient who snores. The clinical team records the following finding: irregular cycling or missing stages point to specific disorders — for example, a patient with narcolepsy may enter sleep directly into REM (sleep-onset REM period). Which concept best explains this observation?

A. Apnoea-hypopnoea index

B. Irregular or absent stages point to specific disorders

C. Chronic insomnia on PSG

D. Respiratory disturbance index

E. Irregular or absent stages

CORRECT ANSWER E

This finding most directly describes Irregular or absent stages: irregular cycling or missing stages point to specific disorders — for example, a patient with narcolepsy may enter sleep directly into REM (sleep-onset REM period). Distractor distinctions: A refers instead to Total scored apnoeas and hypopnoeas divided by hours of sleep; B refers instead to narcolepsy patients may enter sleep directly into REM; C refers instead to increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age; D refers instead to AHI plus respiratory effort-related arousals per hour of sleep.

REFERENCE Integrated notebook, page 47, §2.2

QUESTION 0235 · §2.7 · Applied clinical SBA

A 60-year-old with suspected OSA asks how a sleep-study result should be interpreted. Which of the following statements about OSA diagnosis is most accurate?

A. An Epworth Sleepiness Scale score greater than 10 out of 24 suggests increased daytime sleepiness.

B. Selected referral pathways combine a positive OSA-risk screen with a raised Epworth score.

C. An Epworth Sleepiness Scale score above 10/24 suggests excessive daytime sleepiness.

D. OSA may meet diagnostic criteria at RDI above 15/h, or above 5/h with relevant symptoms.

E. A type 1 sleep study uses approximately 15–20 sensors, whereas a type 4 study uses only 1–2.

CORRECT ANSWER D

OSA may meet diagnostic criteria at RDI above 15/h, or above 5/h with relevant symptoms. Additional revision detail: RDI >15/h, or >5/h with symptoms. AHI ≥15 → material increase in metabolic and vascular risk. The other options describe different mechanisms, investigations or findings rather than osa diagnosis. Consult Integrated notebook, page 59, §2.7 for the full context.

REFERENCE Integrated notebook, page 59, §2.7

QUESTION 0236 · §2.7 · Clinical interpretation SBA

A patient with cardiopulmonary comorbidity is being considered for objective sleep testing. The clinical team records the following finding: RDI >15/h, or >5/h with symptoms. AHI ≥15 → material increase in metabolic and vascular risk. Which concept best explains this observation?

A. STOP-BANG

B. OSA diagnosis

C. Sensors

D. Modified Mallampati

E. Macroglossia

CORRECT ANSWER B

This finding most directly describes OSA diagnosis: RDI >15/h, or >5/h with symptoms. AHI ≥15 → material increase in metabolic and vascular risk. Distractor distinctions: A refers instead to Snoring, Tiredness, Observed apnoea, Pressure (hypertension), BMI >35, Age >50, Neck >40 cm, Gender male; C refers instead to type 1 = 15–20 · 2 = 10–15 · 3 = 4–7 · 4 = 1–2. Type 2 failure ~6.6–7%; underestimates AHI ~10%; D refers instead to describes oral airway patency. Technique: patient sits upright, opens widely, protrudes the tongue maximally without phonatio; E refers instead to scalloping (crenations) of the lateral tongue borders suggests the tongue is too wide for the arch, causing oropharyngeal cro.

REFERENCE Integrated notebook, page 59, §2.7

QUESTION 0237 · §2.1 · Applied clinical SBA

A patient with cardiopulmonary comorbidity is being considered for objective sleep testing. Which of the following statements about NREM 75–80% dominating the first half, REM 20–25% dominating the second is most accurate?

A. Relevant findings include arousal index low, scored by the ASDA 3-second rule.

B. Sleep staging requires EEG, electro-oculography and electromyography signals.

C. Body position includes positional dependence; directly relevant to appliance and positional therapy decisions.

D. Snoring can be measured acoustically using a sound-level meter with standardised positioning.

E. The minimum oxygen level contributes information on hypoxaemic severity.

CORRECT ANSWER A

Relevant findings include arousal index low, scored by the ASDA 3-second rule. Additional revision detail: Arousal index low, scored by the ASDA 3-second rule. The other options describe different mechanisms, investigations or findings rather than nrem 75–80% dominating the first half, rem 20–25% dominating the second. Consult Integrated notebook, page 37, §2.1 for the full context.

REFERENCE Integrated notebook, page 37, §2.1

QUESTION 0238 · §2.1 · Clinical interpretation SBA

A patient on treatment asks whether a favourable symptom report proves normal breathing during sleep. The clinical team records the following finding: Arousal index low, scored by the ASDA 3-second rule. Which concept best explains this observation?

A. Oxygen saturation nadir

B. Body position

C. End-tidal or transcutaneous CO2

D. NREM 75–80% dominating the first half, REM 20–25% dominating the second

E. EEG + EOG + EMG together determine sleep stage

CORRECT ANSWER D

This finding most directly describes NREM 75–80% dominating the first half, REM 20–25% dominating the second: Arousal index low, scored by the ASDA 3-second rule. Distractor distinctions: A refers instead to The minimum oxygen level contributes information on hypoxaemic severity; B refers instead to positional dependence; directly relevant to appliance and positional therapy decisions; C refers instead to hypoventilation; a standard measurement in paediatric PSG. Video/audio — parasomnia, seizure, positional confirmation; E refers instead to No staging is possible without them — the fundamental limitation of level 3 and 4 studies.

REFERENCE Integrated notebook, page 37, §2.1

QUESTION 0239 · §2.2 · Applied clinical SBA

A sleep physician is reviewing a home sleep apnoea study with a patient who snores. Which of the following statements about hypopnoea is most accurate?

A. In adults, an AHI below 5/h is normal; severe OSA begins at AHI 30/h or more.

B. Sleep efficiency can fall when sleep is fragmented by repeated respiratory events.

C. An adult hypopnoea requires an airflow reduction of at least 30% for 10 s with an arousal or qualifying desaturation.

D. A respiratory effort-related arousal follows increasing respiratory effort without meeting apnoea or hypopnoea criteria.

E. Untreated OSA often increases sleep fragmentation and reduces consolidated slow-wave and REM sleep.

CORRECT ANSWER C

An adult hypopnoea requires an airflow reduction of at least 30% for 10 s with an arousal or qualifying desaturation. Additional revision detail: reduction in airflow and/or thoracoabdominal movement of ≥30% of baseline for ≥10 s, with ≥3% desaturation and/or an arousal. The other options describe different mechanisms, investigations or findings rather than hypopnoea. Consult Integrated notebook, page 37, §2.2 for the full context.

REFERENCE Integrated notebook, page 37, §2.2

QUESTION 0240 · §2.2 · Clinical interpretation SBA

A 60-year-old with suspected OSA asks how a sleep-study result should be interpreted. The clinical team records the following finding: reduction in airflow and/or thoracoabdominal movement of ≥30% of baseline for ≥10 s, with ≥3% desaturation and/or an arousal. Which concept best explains this observation?

A. Severity

B. Apnoea-hypopnoea index

C. Fragmentation

D. Hypopnoea

E. Apnoea

CORRECT ANSWER D

This finding most directly describes Hypopnoea: reduction in airflow and/or thoracoabdominal movement of ≥30% of baseline for ≥10 s, with ≥3% desaturation and/or an arousal. Distractor distinctions: A refers instead to adult: normal <5 · mild 5–<15 · moderate 15–<30 · severe ≥30. Paediatric: <1 · 1–<5 · 5–<10 · ≥10; B refers instead to Total scored apnoeas and hypopnoeas divided by hours of sleep; C refers instead to increased arousal index, increased stage shifts, increased N1, reduced slow-wave and REM sleep: the signature of untreated OS; E refers instead to cessation of airflow (≥90% reduction) for ≥10 s. Obstructive if effort continues; central if effort absent; mixed if it begin.

REFERENCE Integrated notebook, page 37, §2.2

QUESTION 0241 · §2.7 · Applied clinical SBA

A patient with cardiopulmonary comorbidity is being considered for objective sleep testing. Which of the following statements about epworth sleepiness scale threshold is most accurate?

A. An Epworth Sleepiness Scale score above 10/24 suggests excessive daytime sleepiness.

B. An Epworth Sleepiness Scale score greater than 10 out of 24 suggests increased daytime sleepiness.

C. Selected referral pathways combine a positive OSA-risk screen with a raised Epworth score.

D. OSA may meet diagnostic criteria at RDI above 15/h, or above 5/h with relevant symptoms.

E. A type 1 sleep study uses approximately 15–20 sensors, whereas a type 4 study uses only 1–2.

CORRECT ANSWER B

An Epworth Sleepiness Scale score greater than 10 out of 24 suggests increased daytime sleepiness. Additional revision detail: More than ten points of the possible twenty-four suggests excessive daytime sleepiness. The other options describe different mechanisms, investigations or findings rather than epworth sleepiness scale threshold. Consult Integrated notebook, page 40, §2.7 for the full context.

REFERENCE Integrated notebook, page 40, §2.7

QUESTION 0242 · §2.7 · Clinical interpretation SBA

A patient on treatment asks whether a favourable symptom report proves normal breathing during sleep. The clinical team records the following finding: More than ten points of the possible twenty-four suggests excessive daytime sleepiness. Which concept best explains this observation?

A. Epworth Sleepiness Scale threshold

B. High palatal vault / narrow arch

C. Australian direct referral

D. Australian direct-referral pathway

E. Mouth breathing with dry mouth

CORRECT ANSWER A

This finding most directly describes Epworth Sleepiness Scale threshold: More than ten points of the possible twenty-four suggests excessive daytime sleepiness. Distractor distinctions: B refers instead to maxillary constriction reduces oral cavity space; narrow tapered V-shaped maxillary arch, possibly with posterior crossbites; C refers instead to positive questionnaire (Berlin / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8; D refers instead to positive questionnaire (Berlin positive / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8; E refers instead to compensation for nasal obstruction. Bruxism — common in disrupted sleep and airway problems. Dental erosion — may indicate GO.

REFERENCE Integrated notebook, page 40, §2.7

QUESTION 0243 · §2.2 · Applied clinical SBA

A 57-year-old with witnessed apnoeas undergoes diagnostic polysomnography. Which of the following statements about irregular or absent stages point to specific disorders is most accurate?

A. In adults, an AHI below 5/h is normal; severe OSA begins at AHI 30/h or more.

B. Sleep efficiency can fall when sleep is fragmented by repeated respiratory events.

C. Narcolepsy patients may enter sleep directly into REM.

D. Respiratory disturbance index includes aHI plus respiratory effort-related arousals per hour of sleep.

E. Apnoea-hypopnoea index includes total scored apnoeas and hypopnoeas divided by hours of sleep.

CORRECT ANSWER C

Narcolepsy patients may enter sleep directly into REM. The other options describe different mechanisms, investigations or findings rather than irregular or absent stages point to specific disorders. Consult Integrated notebook, page 37, §2.2 for the full context.

REFERENCE Integrated notebook, page 37, §2.2

QUESTION 0244 · §2.2 · Clinical interpretation SBA

A sleep physician is reviewing a home sleep apnoea study with a patient who snores. The clinical team records the following finding: narcolepsy patients may enter sleep directly into REM. Which concept best explains this observation?

A. Respiratory effort-related arousal (RERA)

B. Chronic insomnia on PSG

C. Irregular or absent stages point to specific disorders

D. Respiratory disturbance index

E. Irregular or absent stages

CORRECT ANSWER C

This finding most directly describes Irregular or absent stages point to specific disorders: narcolepsy patients may enter sleep directly into REM. Distractor distinctions: A refers instead to Arousal following increasing effort or flattened airflow that does not meet apnoea or hypopnoea criteria; B refers instead to increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age; D refers instead to AHI plus respiratory effort-related arousals per hour of sleep; E refers instead to irregular cycling or missing stages point to specific disorders — for example, a patient with narcolepsy may enter sleep dire.

REFERENCE Integrated notebook, page 37, §2.2

QUESTION 0245 · §2.7 · Clinical interpretation SBA

A 60-year-old with suspected OSA asks how a sleep-study result should be interpreted. The clinical team records the following finding: Snoring, Tiredness, Observed apnoea, Pressure (hypertension), BMI >35, Age >50, Neck >40 cm, Gender male. Which concept best explains this observation?

A. Macroglossia

B. OSA diagnosis

C. Sensors

D. STOP-BANG

E. Modified Mallampati

CORRECT ANSWER D

This finding most directly describes STOP-BANG: Snoring, Tiredness, Observed apnoea, Pressure (hypertension), BMI >35, Age >50, Neck >40 cm, Gender male. Distractor distinctions: A refers instead to scalloping (crenations) of the lateral tongue borders suggests the tongue is too wide for the arch, causing oropharyngeal cro; B refers instead to RDI >15/h, or >5/h with symptoms. AHI ≥15 → material increase in metabolic and vascular risk; C refers instead to type 1 = 15–20 · 2 = 10–15 · 3 = 4–7 · 4 = 1–2. Type 2 failure ~6.6–7%; underestimates AHI ~10%; E refers instead to describes oral airway patency. Technique: patient sits upright, opens widely, protrudes the tongue maximally without phonatio.

REFERENCE Integrated notebook, page 41, §2.7

03 Oral appliance therapy and other treatments

QUESTION 0246 · §3.2 · Clinical decision-making SBA

A patient asks why a protrusive device can reduce retropalatal obstruction although it does not directly contact the soft palate. What is the best explanation?

A. It prevents all arousal during REM

B. Mandibular advancement alters tongue position and tensions connected palatal and pharyngeal tissues

C. It abolishes loop gain directly

D. It delivers positive airway pressure via an oral pump

E. It reverses all adenotonsillar hypertrophy

CORRECT ANSWER B

Mandibular advancement can enlarge the velopharynx through connected palatoglossal/palatopharyngeal soft-tissue structures and improve passive anatomy.

REFERENCE Integrated notebook, Topic 3, §3.2

QUESTION 0247 · §3.3 · Clinical decision-making SBA

A 48-year-old with physician-diagnosed mild OSA prefers mandibular advancement therapy. There is sufficient dental support and no active TMJ pain. Which treatment approach is most appropriate?

A. A custom titratable appliance with follow-up and physician verification of efficacy

B. An irreversible occlusal adjustment before appliance delivery

C. A non-adjustable stock mouthguard without medical follow-up

D. Appliance treatment only after proving that CPAP has failed in every case

E. A flat-plane bruxism splint used as the sole OSA treatment

CORRECT ANSWER A

A custom titratable oral appliance is a supported option after diagnosis and assessment. Dental monitoring and objective physician-led confirmation of therapeutic response remain necessary. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 3, §3.3

QUESTION 0248 · §3.4 · Clinical decision-making SBA

A patient with confirmed OSA on CPAP complains of nasal dryness and congestion. Which is a reasonable first-line supportive intervention?

A. Immediate upper airway surgery without assessment

B. Heated humidification and troubleshooting of the interface

C. Permanent discontinuation of CPAP without an alternative

D. Conversion to fixed non-titratable splint without medical review

E. Routine doubling of CPAP pressure

CORRECT ANSWER B

Humidification can reduce nasal and oral dryness and congestion; interface fit and pressure management should be reviewed with the PAP team. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 3, §3.4

QUESTION 0249 · §3.6 · Clinical decision-making SBA

A patient has predominantly supine OSA with substantially fewer events when sleeping laterally. Which adjunctive strategy is most directly relevant?

A. Positional therapy

B. Uvulopalatopharyngoplasty for every patient

C. A high vertical opening MAD alone

D. Routine oxygen instead of disease-directed treatment

E. Mandibular surgery as the only initial option

CORRECT ANSWER A

Reducing time spent supine may be helpful for position-dependent disease, usually as part of an individualised treatment plan with objective assessment. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 3, §3.6

QUESTION 0250 · §3.10 · Clinical decision-making SBA

In a comparative trial, CPAP produces a larger reduction in AHI than a mandibular advancement device, but patients wear the oral appliance for longer. Which concept best reconciles these outcomes?

A. Pcrit alone

B. Mean disease alleviation, incorporating efficacy and adherence

C. The Epworth Sleepiness Scale alone

D. Hypoxic burden as a substitute for adherence

E. The initial protrusion percentage alone

CORRECT ANSWER B

Mean disease alleviation incorporates both physiological efficacy and actual use. A device can be more efficacious when worn yet have a similar real-world treatment effect because adherence differs. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 3, §3.10

QUESTION 0251 · §3.2 · Clinical decision-making SBA

Why can OAT fail in a patient with severe upper-airway collapsibility and high loop gain despite excellent nightly wear?

A. It always normalises upper-airway muscle responsiveness and loop gain

B. It raises the arousal threshold in every patient

C. It improves anatomy more than non-anatomical ventilatory instability

D. All effective devices must have maximal vertical opening

E. High loop gain can be measured from tooth wear

CORRECT ANSWER C

The appliance advances jaw-related structures and lowers collapsibility but does not reliably normalise respiratory control traits.

REFERENCE Integrated notebook, Topic 3, §3.2

QUESTION 0252 · §3.3 · Applied clinical SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. Which of the following statements about advantages, including over CPAP is most accurate?

A. Effective oral appliance (aadsm 2019) includes advancement in increments of 1 mm or less; protrusive adjustment range ≥5 mm.

B. Advantages, including over CPAP Reduces snoring and the harmful effects of OSA; less obtrusive.

C. OSA must first be excluded by a physician before oral appliance therapy for primary snoring.

D. Indications includes primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA.

E. The physician diagnoses OSA and verifies treatment efficacy, while the dentist provides and monitors the appliance.

CORRECT ANSWER B

Advantages, including over CPAP Reduces snoring and the harmful effects of OSA; less obtrusive. Additional revision detail: Reduces snoring and the harmful effects of OSA; less obtrusive. The other options describe different mechanisms, investigations or findings rather than advantages, including over cpap. Consult Integrated notebook, page 62, §3.3 for the full context.

REFERENCE Integrated notebook, page 62, §3.3

QUESTION 0253 · §3.5 · Clinical interpretation SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. The clinical team records the following finding: genioglossus advancement; midline glossectomy; lingual tonsillectomy; transoral robotic surgery; tongue coblation for retroglossal prolapse or hypertrophic lingual tonsil. Which concept best explains this observation?

A. Indications

B. Nasal (usually adjunctive/pre-phase)

C. UPPP success convention

D. Tongue base

E. Advantages

CORRECT ANSWER D

This finding most directly describes Tongue base: genioglossus advancement; midline glossectomy; lingual tonsillectomy; transoral robotic surgery; tongue coblation for retroglossal prolapse or hypertrophic lingual tonsil. Distractor distinctions: A refers instead to Primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA; snoring cannot be dist; B refers instead to septoplasty; inferior turbinectomy or turbinoplasty; nasal valve surgery for lateral wall collapse; endoscopic sinus surgery;; C refers instead to More than fifty percent reduction in AHI with postoperative AHI below twenty; E refers instead to more efficacious than a MAD for AHI, arousal index, ODI and oxygen saturation.

REFERENCE Integrated notebook, page 63, §3.5

QUESTION 0254 · §3.3 · Clinical decision-making SBA

A 52-year-old with diagnosed moderate OSA cannot tolerate CPAP and has stable dentition. Which treatment is guideline-consistent?

A. A dentist-fitted custom titratable mandibular advancement device in shared care

B. No treatment because CPAP failed

C. Mandibular advancement without further physician involvement

D. A diagnosis made from snoring alone

E. A stock mouthguard as first choice

CORRECT ANSWER A

The guideline supports custom titratable oral appliance therapy for adults intolerant of CPAP or preferring an alternative, with physician verification of efficacy.

REFERENCE Integrated notebook, Topic 3, §3.3

QUESTION 0255 · §3.4 · Applied clinical SBA

A patient returning to the sleep clinic asks about the relative effects of different OSA treatments. Which of the following statements about monitoring is most accurate?

A. CPAP usually reduces AHI and oxygen desaturation more than mandibular advancement.

B. CPAP and APAP are both accepted ongoing PAP modalities for appropriately selected adults with OSA.

C. Heated humidification can relieve upper-airway dryness and nasal irritation during PAP treatment.

D. Monitoring includes suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual.

E. PAP titration seeks the lowest pressure that adequately controls respiratory events.

CORRECT ANSWER D

Monitoring includes suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual. Additional revision detail: suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual severity, unintentional leak and settings, triggering early intervention. The other options describe different mechanisms, investigations or findings rather than monitoring. Consult Integrated notebook, page 63, §3.4 for the full context.

REFERENCE Integrated notebook, page 63, §3.4

QUESTION 0256 · §3.6 · Clinical interpretation SBA

A sleep physician and dentist are discussing the most suitable treatment pathway. The clinical team records the following finding: positional data from the PSG establish the indication. Which concept best explains this observation?

A. Sleep hygiene and behavioural therapy

B. Methods

C. Principle

D. Indication

E. Advantages

CORRECT ANSWER D

This finding most directly describes Indication: positional data from the PSG establish the indication. Distractor distinctions: A refers instead to Sleep behavioural therapy = sleep hygiene, sleeping position, weight control, avoidance of sedatives including alcohol; B refers instead to older techniques — tennis or golf ball in a sock or a bra worn backwards, body pillows, special shirts and vests — with poor; C refers instead to promotes side sleeping instead of supine sleeping. Snoring severity and AHI are greater supine; in children the lowest AHI is; E refers instead to more efficacious than a MAD for AHI, arousal index, ODI and oxygen saturation.

REFERENCE Integrated notebook, page 78, §3.6

QUESTION 0257 · §3.4 · Clinical decision-making SBA

A patient has moderate-to-severe OSA and asks which treatment suppresses AHI most during use. Which is the strongest answer?

A. Positional therapy is superior in all phenotypes

B. Adherence and physiologic efficacy are identical

C. A mandibular device always eliminates more events

D. UPPP invariably outperforms CPAP

E. CPAP is generally more efficacious than an oral appliance

CORRECT ANSWER E

CPAP generally produces a larger AHI reduction when used; adherence and patient acceptance affect real-world effectiveness.

REFERENCE Integrated notebook, Topic 3, §3.4

QUESTION 0258 · §3.5 · Applied clinical SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. Which of the following statements about tongue base is most accurate?

A. First suggested breathing affects sleep quality: nasal rather than mouth breathing gives more restful, better quality sleep.

B. More than fifty percent reduction in AHI with postoperative AHI below twenty.

C. Nasal operations may improve obstruction and PAP tolerance but are usually adjunctive OSA treatments.

D. Tongue-base surgery can include genioglossus advancement or lingual tissue reduction in selected cases.

E. Efficacy describes a treatment's maximal benefit under controlled conditions.

CORRECT ANSWER D

Tongue-base surgery can include genioglossus advancement or lingual tissue reduction in selected cases. Additional revision detail: genioglossus advancement; midline glossectomy; lingual tonsillectomy; transoral robotic surgery; tongue coblation for retroglossal prolapse or hypertrophic lingual tonsil. The other options describe different mechanisms, investigations or findings rather than tongue base. Consult Integrated notebook, page 63, §3.5 for the full context.

REFERENCE Integrated notebook, page 63, §3.5

QUESTION 0259 · §3.7 · Clinical interpretation SBA

A patient who finds continuous positive airway pressure difficult to tolerate asks about alternatives. The clinical team records the following finding: Electrical stimulation of the hypoglossal nerve recruits tongue dilator function. Which concept best explains this observation?

A. Acetazolamide

B. Modafinil / armodafinil

C. Orofacial myofunctional therapy

D. GLP-1 receptor agonists (e.g. tirzepatide)

E. Hypoglossal stimulation anatomy

CORRECT ANSWER E

This finding most directly describes Hypoglossal stimulation anatomy: Electrical stimulation of the hypoglossal nerve recruits tongue dilator function. Distractor distinctions: A refers instead to carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype; B refers instead to recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifi; C refers instead to targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct; D refers instead to reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most signi.

REFERENCE Integrated notebook, page 66, §3.7

QUESTION 0260 · §3.10 · Clinical decision-making SBA

An appliance reduces untreated disease burden by 65% and is worn 80% of prescribed sleep time. What is the mean disease alleviation?

A. About 65%

B. About 52%

C. About 20%

D. About 80%

E. About 145%

CORRECT ANSWER B

Mean disease alleviation approximates efficacy × adherence: 0.65 × 0.80 = 0.52, demonstrating why effective use matters.

REFERENCE Integrated notebook, Topic 3, §3.10

QUESTION 0261 · §3.6 · Applied clinical SBA

A sleep physician and dentist are discussing the most suitable treatment pathway. Which of the following statements about indication is most accurate?

A. Minimum protrusive distance for MAD: 6 mm (limited protrusion <6 mm is a contraindication).

B. A positional treatment approach should be considered when sleep testing confirms positional dependence.

C. Positional therapy aims to reduce supine sleep in patients with position-dependent obstruction.

D. Behavioural management addresses sleep habits, weight, posture and sedative exposure.

E. Mechanical anti-supine devices can reduce positional sleep but frequently have poor long-term adherence.

CORRECT ANSWER B

A positional treatment approach should be considered when sleep testing confirms positional dependence. Additional revision detail: positional data from the PSG establish the indication. The other options describe different mechanisms, investigations or findings rather than indication. Consult Integrated notebook, page 78, §3.6 for the full context.

REFERENCE Integrated notebook, page 78, §3.6

QUESTION 0262 · §3.8 · Clinical interpretation SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. The clinical team records the following finding: for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting a lower, more comfortable pressure. Which concept best explains this observation?

A. OAT + nasal or oral EPAP

B. CPAP + weight loss

C. OAT + positional therapy

D. OAT + CPAP (connected or not)

E. OAT and CPAP used interchangeably

CORRECT ANSWER D

This finding most directly describes OAT + CPAP (connected or not): for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting a lower, more comfortable pressure. Distractor distinctions: A refers instead to significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency; B refers instead to incremental reduction in oxybutynin; acetazolamide plus eszopiclone; C refers instead to CPAP + weight loss — incremental reduction in insulin resistance, triglycerides and blood pressure versus either alone; E refers instead to improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of app.

REFERENCE Integrated notebook, page 65, §3.8

QUESTION 0263 · §3.6 · Clinical decision-making SBA

A patient with positional OSA has events almost exclusively when supine. What adjunct is most anatomically relevant?

A. A positional treatment that reduces supine sleep

B. Insomnia medication as a definitive OSA cure

C. No reassessment after weight gain

D. Routine opioid at bedtime

E. Increased supine sleeping

CORRECT ANSWER A

Positional treatment targets the sleep posture contributing to the events and may be combined with other therapy where appropriate.

REFERENCE Integrated notebook, Topic 3, §3.6

QUESTION 0264 · §3.7 · Applied clinical SBA

A patient who finds continuous positive airway pressure difficult to tolerate asks about alternatives. Which of the following statements about hypoglossal stimulation anatomy is most accurate?

A. Targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct.

B. Hypoglossal stimulation anatomy includes electrical stimulation of the hypoglossal nerve recruits tongue dilator function.

C. Acetazolamide includes carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype.

D. GLP-1 receptor agonist therapies can reduce weight and AHI in selected patients with obesity-related OSA.

E. Relevant findings include target poor muscle responsiveness.

CORRECT ANSWER B

Hypoglossal stimulation anatomy includes electrical stimulation of the hypoglossal nerve recruits tongue dilator function. Additional revision detail: Electrical stimulation of the hypoglossal nerve recruits tongue dilator function. The other options describe different mechanisms, investigations or findings rather than hypoglossal stimulation anatomy. Consult Integrated notebook, page 66, §3.7 for the full context.

REFERENCE Integrated notebook, page 66, §3.7

QUESTION 0265 · §3.9 · Clinical interpretation SBA

A patient returning to the sleep clinic asks about the relative effects of different OSA treatments. The clinical team records the following finding: CPAP lower · OAT higher · positional therapy similar to OAT. Which concept best explains this observation?

A. Custom OA

B. AHI reduction

C. PAP versus oral appliance efficacy

D. Adherence

E. Versus positional therapy

CORRECT ANSWER D

This finding most directly describes Adherence: CPAP lower · OAT higher · positional therapy similar to OAT. Distractor distinctions: A refers instead to made from digital or physical impressions/models; not prefabricated; biocompatible material; engages both arches. Non-custom; B refers instead to CPAP ~75% · OA ~42% · UPPP ~30%. MDA: OA ~51% vs CPAP ~50%. MAD 6.5 vs CPAP 5.2 h/night; C refers instead to PAP typically has higher physiological suppression of AHI, while OAT can be better tolerated; E refers instead to OAT better for sleepiness; other measures similar in supine-dependent OSA.

REFERENCE Integrated notebook, page 66, §3.9

QUESTION 0266 · §3.5 · Clinical decision-making SBA

Drug-induced sleep endoscopy identifies complete concentric palatal collapse in a patient considered for hypoglossal stimulation. What is the implication?

A. It guarantees successful stimulation

B. It proves a mandibular appliance cannot be used

C. This is an exclusion for the selected stimulation pathway described in the notebook

D. It rules out all surgical options

E. It is equivalent to a negative sleep study

CORRECT ANSWER C

The notebook lists complete concentric retropalatal collapse as an exclusion for the described HNS selection approach.

REFERENCE Integrated notebook, Topic 3, §3.5

QUESTION 0267 · §3.8 · Applied clinical SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. Which of the following statements about oat + CPAP (connected or not) is most accurate?

A. Combining nasal EPAP with oral appliances may lower residual AHI in some incomplete responders.

B. Alternating PAP and oral appliances may improve overall treatment exposure in selected patients.

C. Combining CPAP with weight reduction may improve insulin resistance and blood pressure beyond either alone.

D. Combining oral appliances and PAP may increase overall treatment exposure in selected patients.

E. For severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone.

CORRECT ANSWER E

For severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone. Additional revision detail: for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting a lower, more comfortable pressure. The other options describe different mechanisms, investigations or findings rather than oat + cpap (connected or not). Consult Integrated notebook, page 65, §3.8 for the full context.

REFERENCE Integrated notebook, page 65, §3.8

QUESTION 0268 · §3.10 · Clinical interpretation SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. The clinical team records the following finding: the overall percentage reduction in disease severity achieved in realworld conditions. Reflects true effectiveness. Which concept best explains this observation?

A. Efficacy

B. Compliance

C. PAP versus oral appliance efficacy

D. Mean disease alleviation = efficacy × compliance

E. Mean disease alleviation calculation

CORRECT ANSWER D

This finding most directly describes Mean disease alleviation = efficacy × compliance: the overall percentage reduction in disease severity achieved in real- world conditions. Reflects true effectiveness. Distractor distinctions: A refers instead to the maximum potential benefit of a treatment, typically measured in controlled trials as the reduction in a disease marker su; B refers instead to the extent to which patients correctly follow medical advice, often quantified as the percentage of prescribed therapy actual; C refers instead to PAP typically has higher physiological suppression of AHI, while OAT can be better tolerated; E refers instead to Physiological efficacy multiplied by the proportion of treatment actually used.

REFERENCE Integrated notebook, page 66, §3.10

QUESTION 0269 · §3.8 · Clinical decision-making SBA

A patient requires high CPAP pressure and reports mask intolerance but can tolerate a mandibular appliance. What combined strategy may reduce pressure needs?

A. Use an untitratable boil-and-bite mouthguard only

B. Increase vertical opening without limitation

C. Combine mandibular advancement with PAP under specialist supervision

D. Stop all medical follow-up

E. Use the appliance to diagnose treatment severity

CORRECT ANSWER C

OAT and CPAP can be combined in selected patients to improve airway patency and reduce required PAP pressure; efficacy should be confirmed medically.

REFERENCE Integrated notebook, Topic 3, §3.8

QUESTION 0270 · §3.9 · Applied clinical SBA

A patient returning to the sleep clinic asks about the relative effects of different OSA treatments. Which of the following statements about adherence is most accurate?

A. OAT better for sleepiness; other measures similar in supine-dependent OSA.

B. CPAP generally reduces AHI more than oral appliance treatment in indirect comparisons.

C. A positional treatment approach should be considered when sleep testing confirms positional dependence.

D. PAP typically has higher physiological suppression of AHI, while OAT can be better tolerated.

E. Oral appliances may achieve greater nightly use than CPAP, although adherence varies between patients.

CORRECT ANSWER E

Oral appliances may achieve greater nightly use than CPAP, although adherence varies between patients. Additional revision detail: CPAP lower · OAT higher · positional therapy similar to OAT. The other options describe different mechanisms, investigations or findings rather than adherence. Consult Integrated notebook, page 66, §3.9 for the full context.

REFERENCE Integrated notebook, page 66, §3.9

QUESTION 0271 · §3.11 · Clinical interpretation SBA

A sleep physician and dentist are discussing the most suitable treatment pathway. The clinical team records the following finding: increments ≤1 mm · range ≥5 mm · reversible · verifiable. Which concept best explains this observation?

A. Effective OA definition

B. Effective mandibular appliance adjustment

C. UPPP success

D. AHI reduction

E. Effective OA

CORRECT ANSWER E

This finding most directly describes Effective OA: increments ≤1 mm · range ≥5 mm · reversible · verifiable. Distractor distinctions: A refers instead to increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting; B refers instead to At most one-millimetre increments across a range of at least five millimetres; C refers instead to >50% AHI reduction and AHI <20; Friedman stage I best, III mostly unsuccessful; ~4% surgical adverse event rate; D refers instead to CPAP ~75% · OA ~42% · UPPP ~30%. MDA: OA ~51% vs CPAP ~50%. MAD 6.5 vs CPAP 5.2 h/night.

REFERENCE Integrated notebook, page 68, §3.11

QUESTION 0272 · §3.6 · Clinical decision-making SBA

An obese adult asks whether losing 10% of body weight is expected to cure OSA. What is the best counselling answer?

A. Weight is unrelated to upper-airway collapsibility

B. Only surgery can alter OSA severity

C. Every 10% loss guarantees AHI zero

D. Diet is contraindicated with oral appliances

E. Weight reduction often lowers AHI but does not guarantee cure or remove the need for objective reassessment

CORRECT ANSWER E

Weight loss reduces OSA burden on average, but disease may persist; treatment decisions still depend on symptoms and repeat objective assessment.

REFERENCE Integrated notebook, Topic 3, §3.6

QUESTION 0273 · §3.10 · Applied clinical SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. Which of the following statements about mean disease alleviation = efficacy × compliance is most accurate?

A. Efficacy describes a treatment's maximal benefit under controlled conditions.

B. The overall percentage reduction in disease severity achieved in realworld conditions. Reflects true effectiveness.

C. Adherence describes the extent to which a patient follows the prescribed treatment regimen.

D. Mean disease alleviation combines physiological efficacy with actual treatment adherence.

E. Behavioural management addresses sleep habits, weight, posture and sedative exposure.

CORRECT ANSWER B

The overall percentage reduction in disease severity achieved in realworld conditions. Reflects true effectiveness. The other options describe different mechanisms, investigations or findings rather than mean disease alleviation = efficacy × compliance. Consult Integrated notebook, page 66, §3.10 for the full context.

REFERENCE Integrated notebook, page 66, §3.10

QUESTION 0274 · §3.1 · Clinical interpretation SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. The clinical team records the following finding: first suggested breathing affects sleep quality: nasal rather than mouth breathing gives more restful, better quality sleep, better daytime function and better general health. Which concept best explains this observation?

A. Pierre Robin, 1903

B. Cartwright & Samelson, 1982

C. Meier-Ewert et al., 1985

D. Oral appliance

E. George Catlin (19th c.)

CORRECT ANSWER E

This finding most directly describes George Catlin (19th c.): first suggested breathing affects sleep quality: nasal rather than mouth breathing gives more restful, better quality sleep, better daytime function and better general health. Distractor distinctions: A refers instead to the monobloc, for glossoptosis. ~30 years later used to reposition the mandible; B refers instead to the tongue retaining device, holding the tongue forward by suction; historically the second most common type; C refers instead to an intraoral protraction device; the 1980s work was the catalyst for the modern field; D refers instead to a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep.

REFERENCE Integrated notebook, page 61, §3.1

QUESTION 0275 · §3.9 · Clinical decision-making SBA

A patient has superior nightly use of a MAD despite a smaller per-hour AHI reduction than CPAP. Which concept best reconciles comparable outcomes?

A. Oral devices cure central sleep apnoea

B. Adherence cannot be measured

C. AHI has no relation to disease burden

D. CPAP always produces more nightly use

E. Real-world effectiveness depends on both efficacy and adherence

CORRECT ANSWER E

Mean disease alleviation combines the amount of disease reduced while worn with the amount of time the device is actually used.

REFERENCE Integrated notebook, Topic 3, §3.9

QUESTION 0276 · §3.11 · Applied clinical SBA

A sleep physician and dentist are discussing the most suitable treatment pathway. Which of the following statements about effective oa is most accurate?

A. A 10% weight gain is associated with an approximately 32% rise in AHI.

B. CPAP produces a larger average AHI reduction than oral appliance therapy in indirect comparisons.

C. The device should provide adjustment increments no larger than 1 mm across at least 5 mm of travel.

D. A titratable appliance should permit verifiable incremental advancement of 1 mm or less over at least 5 mm.

E. Minimum protrusive distance for MAD: 6 mm (limited protrusion <6 mm is a contraindication).

CORRECT ANSWER D

A titratable appliance should permit verifiable incremental advancement of 1 mm or less over at least 5 mm. Additional revision detail: increments ≤1 mm · range ≥5 mm · reversible · verifiable. The other options describe different mechanisms, investigations or findings rather than effective oa. Consult Integrated notebook, page 68, §3.11 for the full context.

REFERENCE Integrated notebook, page 68, §3.11

QUESTION 0277 · §3.3 · Focused knowledge SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. Which of the following statements about indications is most accurate?

A. Indications includes primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA.

B. CPAP and APAP are both accepted ongoing PAP modalities for appropriately selected adults with OSA.

C. OSA must first be excluded by a physician before oral appliance therapy for primary snoring.

D. Advantages, including over CPAP Reduces snoring and the harmful effects of OSA; less obtrusive.

E. The physician diagnoses OSA and verifies treatment efficacy, while the dentist provides and monitors the appliance.

CORRECT ANSWER A

Indications includes primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA. Additional revision detail: Primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA; snoring cannot be distinguished from OSA clinically, so patients who snore must not be treated without a diagnosis. The other options describe different mechanisms, investigations or findings rather than indications. Consult Integrated notebook, page 61, §3.3 for the full context.

REFERENCE Integrated notebook, page 61, §3.3

QUESTION 0278 · §3.11 · Clinical decision-making SBA

A new device has appealing advertising but no independently published efficacy data. Which principle should guide selection?

A. Price is the only relevant criterion

B. Device choice can ignore dental conditions

C. Evidence-based appraisal and patient-specific decision-making

D. Manufacturer claims alone determine efficacy

E. Patient preference eliminates the need for evidence

CORRECT ANSWER C

Evidence-based dental sleep medicine integrates valid research, clinical expertise and patient values rather than commercial claims alone.

REFERENCE Integrated notebook, Topic 3, §3.11

QUESTION 0279 · §3.1 · Applied clinical SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. Which of the following statements about george catlin (19th c.) is most accurate?

A. A tongue-retaining device uses suction to maintain an anterior tongue position.

B. First suggested breathing affects sleep quality: nasal rather than mouth breathing gives more restful, better quality sleep.

C. Modern era includes focus on mandibular advancement appliances; case reports gave way to RCTs confirming efficacy in mild–.

D. Custom oa includes made from digital or physical impressions/models.

E. A titratable oral appliance permits progressive mandibular advancement, unlike a fixed-position device.

CORRECT ANSWER B

First suggested breathing affects sleep quality: nasal rather than mouth breathing gives more restful, better quality sleep. Additional revision detail: first suggested breathing affects sleep quality: nasal rather than mouth breathing gives more restful, better quality sleep, better daytime function and better general health. The other options describe different mechanisms, investigations or findings rather than george catlin (19th c.). Consult Integrated notebook, page 61, §3.1 for the full context.

REFERENCE Integrated notebook, page 61, §3.1

QUESTION 0280 · §3.4 · Clinical interpretation SBA

A patient returning to the sleep clinic asks about the relative effects of different OSA treatments. The clinical team records the following finding: treatment must be based on a diagnosis established by objective sleep apnoea testing. Which concept best explains this observation?

A. Disadvantages

B. Advantages

C. Good practice

D. Titration

E. Monitoring

CORRECT ANSWER C

This finding most directly describes Good practice: treatment must be based on a diagnosis established by objective sleep apnoea testing. Distractor distinctions: A refers instead to lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%; B refers instead to more efficacious than a MAD for AHI, arousal index, ODI and oxygen saturation; D refers instead to establish the minimum pressure at which snoring ceases and apnoeic events are absent. Start 4 cmH2O, titrate to the lowest ef; E refers instead to suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual severity, uninten.

REFERENCE Integrated notebook, page 62, §3.4

QUESTION 0281 · §3.3 · Applied clinical SBA

A patient returning to the sleep clinic asks about the relative effects of different OSA treatments. Which of the following statements about indications is most accurate?

A. The physician diagnoses OSA and verifies treatment efficacy, while the dentist provides and monitors the appliance.

B. Advantages, including over CPAP Reduces snoring and the harmful effects of OSA; less obtrusive.

C. CPAP and APAP are both accepted ongoing PAP modalities for appropriately selected adults with OSA.

D. OSA must first be excluded by a physician before oral appliance therapy for primary snoring.

E. Indications includes primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA.

CORRECT ANSWER E

Indications includes primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA. Additional revision detail: Primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA; snoring cannot be distinguished from OSA clinically, so patients who snore must not be treated without a diagnosis. The other options describe different mechanisms, investigations or findings rather than indications. Consult Integrated notebook, page 61, §3.3 for the full context.

REFERENCE Integrated notebook, page 61, §3.3

QUESTION 0282 · §3.5 · Clinical interpretation SBA

A sleep physician and dentist are discussing the most suitable treatment pathway. The clinical team records the following finding: More than fifty percent reduction in AHI with postoperative AHI below twenty. Which concept best explains this observation?

A. Nasal (usually adjunctive/pre-phase)

B. Effective OA definition

C. Tongue base

D. UPPP success

E. UPPP success convention

CORRECT ANSWER E

This finding most directly describes UPPP success convention: More than fifty percent reduction in AHI with postoperative AHI below twenty. Distractor distinctions: A refers instead to septoplasty; inferior turbinectomy or turbinoplasty; nasal valve surgery for lateral wall collapse; endoscopic sinus surgery;; B refers instead to increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting; C refers instead to genioglossus advancement; midline glossectomy; lingual tonsillectomy; transoral robotic surgery; tongue coblation for retrogl; D refers instead to >50% AHI reduction and AHI <20; Friedman stage I best, III mostly unsuccessful; ~4% surgical adverse event rate.

REFERENCE Integrated notebook, page 66, §3.5

QUESTION 0283 · §3.4 · Applied clinical SBA

A sleep physician and dentist are discussing the most suitable treatment pathway. Which of the following statements about good practice is most accurate?

A. Oral appliance treatment for OSA requires an objective physician-established diagnosis.

B. Disadvantages includes lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%.

C. Heated humidification can relieve upper-airway dryness and nasal irritation during PAP treatment.

D. CPAP usually reduces AHI and oxygen desaturation more than mandibular advancement.

E. Initial adult pap titration pressure includes around four centimetres of water pressure.

CORRECT ANSWER A

Oral appliance treatment for OSA requires an objective physician-established diagnosis. Additional revision detail: treatment must be based on a diagnosis established by objective sleep apnoea testing. The other options describe different mechanisms, investigations or findings rather than good practice. Consult Integrated notebook, page 62, §3.4 for the full context.

REFERENCE Integrated notebook, page 62, §3.4

QUESTION 0284 · §3.6 · Clinical interpretation SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. The clinical team records the following finding: Sleep behavioural therapy = sleep hygiene, sleeping position, weight control, avoidance of sedatives including alcohol. Which concept best explains this observation?

A. Principle

B. OSA and primary snoring distinction

C. Sleep hygiene and behavioural therapy

D. Methods

E. Indication

CORRECT ANSWER C

This finding most directly describes Sleep hygiene and behavioural therapy: Sleep behavioural therapy = sleep hygiene, sleeping position, weight control, avoidance of sedatives including alcohol. Distractor distinctions: A refers instead to promotes side sleeping instead of supine sleeping. Snoring severity and AHI are greater supine; in children the lowest AHI is; B refers instead to OSA must first be excluded by a physician before oral appliance therapy for primary snoring; D refers instead to older techniques — tennis or golf ball in a sock or a bra worn backwards, body pillows, special shirts and vests — with poor; E refers instead to positional data from the PSG establish the indication.

REFERENCE Integrated notebook, page 65, §3.6

QUESTION 0285 · §3.5 · Applied clinical SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. Which of the following statements about uppp success convention is most accurate?

A. Effective oral appliance (aadsm 2019) includes advancement in increments of 1 mm or less; protrusive adjustment range ≥5 mm.

B. CPAP usually reduces AHI and oxygen desaturation more than mandibular advancement.

C. Nasal operations may improve obstruction and PAP tolerance but are usually adjunctive OSA treatments.

D. More than fifty percent reduction in AHI with postoperative AHI below twenty.

E. Tongue-base surgery can include genioglossus advancement or lingual tissue reduction in selected cases.

CORRECT ANSWER D

More than fifty percent reduction in AHI with postoperative AHI below twenty. The other options describe different mechanisms, investigations or findings rather than uppp success convention. Consult Integrated notebook, page 66, §3.5 for the full context.

REFERENCE Integrated notebook, page 66, §3.5

QUESTION 0286 · §3.7 · Clinical interpretation SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. The clinical team records the following finding: recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifiable cause. Not a treatment for the OSA. Which concept best explains this observation?

A. Modafinil / armodafinil

B. Orofacial myofunctional therapy

C. Hypoglossal stimulation anatomy

D. GLP-1 receptor agonists (e.g. tirzepatide)

E. Acetazolamide

CORRECT ANSWER A

This finding most directly describes Modafinil / armodafinil: recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifiable cause. Not a treatment for the OSA. Distractor distinctions: B refers instead to targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct; C refers instead to Electrical stimulation of the hypoglossal nerve recruits tongue dilator function; D refers instead to reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most signi; E refers instead to carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype.

REFERENCE Integrated notebook, page 65, §3.7

QUESTION 0287 · §3.6 · Applied clinical SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. Which of the following statements about sleep hygiene and behavioural therapy is most accurate?

A. Mechanical anti-supine devices can reduce positional sleep but frequently have poor long-term adherence.

B. Behavioural management addresses sleep habits, weight, posture and sedative exposure.

C. A titratable oral appliance permits progressive mandibular advancement, unlike a fixed-position device.

D. A positional treatment approach should be considered when sleep testing confirms positional dependence.

E. Positional therapy aims to reduce supine sleep in patients with position-dependent obstruction.

CORRECT ANSWER B

Behavioural management addresses sleep habits, weight, posture and sedative exposure. Additional revision detail: Sleep behavioural therapy = sleep hygiene, sleeping position, weight control, avoidance of sedatives including alcohol. The other options describe different mechanisms, investigations or findings rather than sleep hygiene and behavioural therapy. Consult Integrated notebook, page 65, §3.6 for the full context.

REFERENCE Integrated notebook, page 65, §3.6

QUESTION 0288 · §3.8 · Clinical interpretation SBA

A patient who finds continuous positive airway pressure difficult to tolerate asks about alternatives. The clinical team records the following finding: significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency is lower with the combination. Which concept best explains this observation?

A. OAT + CPAP (connected or not)

B. OAT + CPAP, connected or unconnected

C. OAT + positional therapy

D. OAT + nasal or oral EPAP

E. OAT and CPAP used interchangeably

CORRECT ANSWER D

This finding most directly describes OAT + nasal or oral EPAP: significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency is lower with the combination. Distractor distinctions: A refers instead to for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting; B refers instead to for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage; C refers instead to CPAP + weight loss — incremental reduction in insulin resistance, triglycerides and blood pressure versus either alone; E refers instead to improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of app.

REFERENCE Integrated notebook, page 65, §3.8

QUESTION 0289 · §3.7 · Applied clinical SBA

A patient who finds continuous positive airway pressure difficult to tolerate asks about alternatives. Which of the following statements about modafinil / armodafinil is most accurate?

A. Wake-promoting medication may be considered for persistent sleepiness despite optimised OSA therapy.

B. Acetazolamide includes carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype.

C. GLP-1 receptor agonist therapies can reduce weight and AHI in selected patients with obesity-related OSA.

D. Targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct.

E. Relevant findings include target poor muscle responsiveness.

CORRECT ANSWER A

Wake-promoting medication may be considered for persistent sleepiness despite optimised OSA therapy. Additional revision detail: recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifiable cause. Not a treatment for the OSA. The other options describe different mechanisms, investigations or findings rather than modafinil / armodafinil. Consult Integrated notebook, page 65, §3.7 for the full context.

REFERENCE Integrated notebook, page 65, §3.7

QUESTION 0290 · §3.9 · Clinical interpretation SBA

A patient returning to the sleep clinic asks about the relative effects of different OSA treatments. The clinical team records the following finding: CPAP highest (~75%) · OAT moderate (~42%) · UPPP variable (~30%) · exercise and weight loss similar to MAD. Which concept best explains this observation?

A. Versus positional therapy

B. PAP versus oral appliance efficacy

C. Adherence

D. AHI reduction

E. Acetazolamide

CORRECT ANSWER D

This finding most directly describes AHI reduction: CPAP highest (~75%) · OAT moderate (~42%) · UPPP variable (~30%) · exercise and weight loss similar to MAD. Distractor distinctions: A refers instead to OAT better for sleepiness; other measures similar in supine-dependent OSA; B refers instead to PAP typically has higher physiological suppression of AHI, while OAT can be better tolerated; C refers instead to CPAP lower · OAT higher · positional therapy similar to OAT; E refers instead to carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype.

REFERENCE Integrated notebook, page 66, §3.9

QUESTION 0291 · §3.8 · Applied clinical SBA

A patient returning to the sleep clinic asks about the relative effects of different OSA treatments. Which of the following statements about oat + nasal or oral EPAP is most accurate?

A. For severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone.

B. Combining oral appliances and PAP may increase overall treatment exposure in selected patients.

C. Combining nasal EPAP with oral appliances may lower residual AHI in some incomplete responders.

D. Alternating PAP and oral appliances may improve overall treatment exposure in selected patients.

E. Combining CPAP with weight reduction may improve insulin resistance and blood pressure beyond either alone.

CORRECT ANSWER C

Combining nasal EPAP with oral appliances may lower residual AHI in some incomplete responders. Additional revision detail: significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency is lower with the combination. The other options describe different mechanisms, investigations or findings rather than oat + nasal or oral epap. Consult Integrated notebook, page 65, §3.8 for the full context.

REFERENCE Integrated notebook, page 65, §3.8

QUESTION 0292 · §3.10 · Focused knowledge SBA

A sleep physician and dentist are discussing the most suitable treatment pathway. Which of the following statements about efficacy is most accurate?

A. The overall percentage reduction in disease severity achieved in realworld conditions. Reflects true effectiveness.

B. Adherence describes the extent to which a patient follows the prescribed treatment regimen.

C. Mean disease alleviation combines physiological efficacy with actual treatment adherence.

D. Relevant findings include target poor muscle responsiveness.

E. Efficacy describes a treatment's maximal benefit under controlled conditions.

CORRECT ANSWER E

Efficacy describes a treatment's maximal benefit under controlled conditions. Additional revision detail: the maximum potential benefit of a treatment, typically measured in controlled trials as the reduction in a disease marker such as AHI. Sets the upper limit of benefit. The other options describe different mechanisms, investigations or findings rather than efficacy. Consult Integrated notebook, page 66, §3.10 for the full context.

REFERENCE Integrated notebook, page 66, §3.10

QUESTION 0293 · §3.9 · Applied clinical SBA

A sleep physician and dentist are discussing the most suitable treatment pathway. Which of the following statements about AHI reduction is most accurate?

A. CPAP generally reduces AHI more than oral appliance treatment in indirect comparisons.

B. CPAP produces a larger average AHI reduction than oral appliance therapy in indirect comparisons.

C. OAT better for sleepiness; other measures similar in supine-dependent OSA.

D. Oral appliances may achieve greater nightly use than CPAP, although adherence varies between patients.

E. PAP typically has higher physiological suppression of AHI, while OAT can be better tolerated.

CORRECT ANSWER A

CPAP generally reduces AHI more than oral appliance treatment in indirect comparisons. Additional revision detail: CPAP highest (~75%) · OAT moderate (~42%) · UPPP variable (~30%) · exercise and weight loss similar to MAD. The other options describe different mechanisms, investigations or findings rather than ahi reduction. Consult Integrated notebook, page 66, §3.9 for the full context.

REFERENCE Integrated notebook, page 66, §3.9

QUESTION 0294 · §3.11 · Clinical interpretation SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. The clinical team records the following finding: >50% AHI reduction and AHI <20; Friedman stage I best, III mostly unsuccessful; ~4% surgical adverse event rate. Which concept best explains this observation?

A. UPPP success

B. Purpose of EBD

C. AHI reduction

D. Weight

E. Effective OA

CORRECT ANSWER A

This finding most directly describes UPPP success: >50% AHI reduction and AHI <20; Friedman stage I best, III mostly unsuccessful; ~4% surgical adverse event rate. Distractor distinctions: B refers instead to provide appropriate care, reduce inappropriate care, increase effectiveness of care, by promoting best available evidence in; C refers instead to CPAP ~75% · OA ~42% · UPPP ~30%. MDA: OA ~51% vs CPAP ~50%. MAD 6.5 vs CPAP 5.2 h/night; D refers instead to 10% gain → +32% AHI; 10% loss → −26%; BMI target <30; bariatric BMI ≥35; E refers instead to increments ≤1 mm · range ≥5 mm · reversible · verifiable.

REFERENCE Integrated notebook, page 82, §3.11

QUESTION 0295 · §3.10 · Applied clinical SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. Which of the following statements about efficacy is most accurate?

A. Efficacy describes a treatment's maximal benefit under controlled conditions.

B. Adherence describes the extent to which a patient follows the prescribed treatment regimen.

C. Mean disease alleviation combines physiological efficacy with actual treatment adherence.

D. Relevant findings include target poor muscle responsiveness.

E. The overall percentage reduction in disease severity achieved in realworld conditions. Reflects true effectiveness.

CORRECT ANSWER A

Efficacy describes a treatment's maximal benefit under controlled conditions. Additional revision detail: the maximum potential benefit of a treatment, typically measured in controlled trials as the reduction in a disease marker such as AHI. Sets the upper limit of benefit. The other options describe different mechanisms, investigations or findings rather than efficacy. Consult Integrated notebook, page 66, §3.10 for the full context.

REFERENCE Integrated notebook, page 66, §3.10

QUESTION 0296 · §3.1 · Clinical interpretation SBA

A sleep physician and dentist are discussing the most suitable treatment pathway. The clinical team records the following finding: the monobloc, for glossoptosis. ~30 years later used to reposition the mandible. Which concept best explains this observation?

A. Oral appliance

B. Meier-Ewert et al., 1985

C. Pierre Robin, 1903

D. Modern era

E. George Catlin (19th c.)

CORRECT ANSWER C

This finding most directly describes Pierre Robin, 1903: the monobloc, for glossoptosis. ~30 years later used to reposition the mandible. Distractor distinctions: A refers instead to a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep; B refers instead to an intraoral protraction device; the 1980s work was the catalyst for the modern field; D refers instead to focus on mandibular advancement appliances; case reports gave way to RCTs confirming efficacy in mild– moderate and more rece; E refers instead to first suggested breathing affects sleep quality: nasal rather than mouth breathing gives more restful, better quality sleep,.

REFERENCE Integrated notebook, page 61, §3.1

QUESTION 0297 · §3.11 · Applied clinical SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. Which of the following statements about uppp success is most accurate?

A. An effective titratable appliance permits reversible adjustments of 1 mm or less over a range of at least 5 mm.

B. A 10% weight gain is associated with an approximately 32% rise in AHI.

C. The clinician should provide appropriate care, reduce inappropriate care, increase effectiveness of care, by promoting best.

D. The device should provide adjustment increments no larger than 1 mm across at least 5 mm of travel.

E. UPPP success: >50% AHI reduction and AHI <20; Friedman stage I best, III mostly unsuccessful; ~4% surgical adverse event rate.

CORRECT ANSWER E

UPPP success: >50% AHI reduction and AHI <20; Friedman stage I best, III mostly unsuccessful; ~4% surgical adverse event rate. Additional revision detail: >50% AHI reduction and AHI <20; Friedman stage I best, III mostly unsuccessful; ~4% surgical adverse event rate. The other options describe different mechanisms, investigations or findings rather than uppp success. Consult Integrated notebook, page 82, §3.11 for the full context.

REFERENCE Integrated notebook, page 82, §3.11

QUESTION 0298 · §3.3 · Clinical interpretation SBA

A patient who finds continuous positive airway pressure difficult to tolerate asks about alternatives. The clinical team records the following finding: OSA must first be excluded by a physician before oral appliance therapy for primary snoring. Which concept best explains this observation?

A. Indications

B. Advantages, including over CPAP

C. Side effects are usually well tolerated once symptomatic relief is achieved

D. OSA and primary snoring distinction

E. OAT and CPAP used interchangeably

CORRECT ANSWER D

This finding most directly describes OSA and primary snoring distinction: OSA must first be excluded by a physician before oral appliance therapy for primary snoring. Distractor distinctions: A refers instead to Primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA; snoring cannot be dist; B refers instead to Reduces snoring and the harmful effects of OSA; less obtrusive; C refers instead to Division of responsibility: diagnosis, prescription and monitoring of effectiveness rest with the medical practitioner; E refers instead to improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of app.

REFERENCE Integrated notebook, page 63, §3.3

QUESTION 0299 · §3.1 · Applied clinical SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. Which of the following statements about pierre robin, 1903 is most accurate?

A. An intraoral protraction device; the 1980s work was the catalyst for the modern field.

B. Pierre Robin described a monobloc appliance for managing glossoptosis.

C. Effective oral appliance (aadsm 2019) includes advancement in increments of 1 mm or less; protrusive adjustment range ≥5 mm.

D. Oral appliance includes a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep.

E. A titratable oral appliance permits progressive mandibular advancement, unlike a fixed-position device.

CORRECT ANSWER B

Pierre Robin described a monobloc appliance for managing glossoptosis. Additional revision detail: the monobloc, for glossoptosis. ~30 years later used to reposition the mandible. The other options describe different mechanisms, investigations or findings rather than pierre robin, 1903. Consult Integrated notebook, page 61, §3.1 for the full context.

REFERENCE Integrated notebook, page 61, §3.1

QUESTION 0300 · §3.4 · Clinical interpretation SBA

A patient returning to the sleep clinic asks about the relative effects of different OSA treatments. The clinical team records the following finding: dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice and reduced smell. Which concept best explains this observation?

A. Disadvantages

B. Heated humidification is suggested to reduce side effects

C. Heated humidification suggested

D. Good practice

E. Initial adult PAP titration pressure

CORRECT ANSWER B

This finding most directly describes Heated humidification is suggested to reduce side effects: dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice and reduced smell. Distractor distinctions: A refers instead to lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%; C refers instead to reduces dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse; D refers instead to treatment must be based on a diagnosis established by objective sleep apnoea testing; E refers instead to Around four centimetres of water pressure.

REFERENCE Integrated notebook, page 73, §3.4

QUESTION 0301 · §3.3 · Applied clinical SBA

A patient returning to the sleep clinic asks about the relative effects of different OSA treatments. Which of the following statements about OSA and primary snoring distinction is most accurate?

A. Advantages, including over CPAP Reduces snoring and the harmful effects of OSA; less obtrusive.

B. PAP typically has higher physiological suppression of AHI, while OAT can be better tolerated.

C. Indications includes primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA.

D. OSA must first be excluded by a physician before oral appliance therapy for primary snoring.

E. The physician diagnoses OSA and verifies treatment efficacy, while the dentist provides and monitors the appliance.

CORRECT ANSWER D

OSA must first be excluded by a physician before oral appliance therapy for primary snoring. The other options describe different mechanisms, investigations or findings rather than osa and primary snoring distinction. Consult Integrated notebook, page 63, §3.3 for the full context.

REFERENCE Integrated notebook, page 63, §3.3

QUESTION 0302 · §3.5 · Clinical interpretation SBA

A sleep physician and dentist are discussing the most suitable treatment pathway. The clinical team records the following finding: septoplasty; inferior turbinectomy or turbinoplasty; nasal valve surgery for lateral wall collapse; endoscopic sinus surgery; polypectomy. Which concept best explains this observation?

A. Tongue base

B. OAT + nasal or oral EPAP

C. Side effects are usually well tolerated once symptomatic relief is achieved

D. Nasal (usually adjunctive/pre-phase)

E. UPPP success convention

CORRECT ANSWER D

This finding most directly describes Nasal (usually adjunctive/pre-phase): septoplasty; inferior turbinectomy or turbinoplasty; nasal valve surgery for lateral wall collapse; endoscopic sinus surgery; polypectomy. Distractor distinctions: A refers instead to genioglossus advancement; midline glossectomy; lingual tonsillectomy; transoral robotic surgery; tongue coblation for retrogl; B refers instead to significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency; C refers instead to Division of responsibility: diagnosis, prescription and monitoring of effectiveness rest with the medical practitioner; E refers instead to More than fifty percent reduction in AHI with postoperative AHI below twenty.

REFERENCE Integrated notebook, page 63, §3.5

QUESTION 0303 · §3.4 · Applied clinical SBA

A sleep physician and dentist are discussing the most suitable treatment pathway. Which of the following statements about heated humidification is suggested to reduce side effects is most accurate?

A. Heated humidification can relieve upper-airway dryness and nasal irritation during PAP treatment.

B. PAP titration seeks the lowest pressure that adequately controls respiratory events.

C. Monitoring includes suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual.

D. Oral appliance treatment for OSA requires an objective physician-established diagnosis.

E. Disadvantages includes lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%.

CORRECT ANSWER A

Heated humidification can relieve upper-airway dryness and nasal irritation during PAP treatment. Additional revision detail: dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice and reduced smell. The other options describe different mechanisms, investigations or findings rather than heated humidification is suggested to reduce side effects. Consult Integrated notebook, page 73, §3.4 for the full context.

REFERENCE Integrated notebook, page 73, §3.4

QUESTION 0304 · §3.6 · Clinical interpretation SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. The clinical team records the following finding: promotes side sleeping instead of supine sleeping. Snoring severity and AHI are greater supine; in children the lowest AHI is often prone. Which concept best explains this observation?

A. Methods

B. Indication

C. Sleep hygiene and behavioural therapy

D. Adherence

E. Principle

CORRECT ANSWER E

This finding most directly describes Principle: promotes side sleeping instead of supine sleeping. Snoring severity and AHI are greater supine; in children the lowest AHI is often prone. Distractor distinctions: A refers instead to older techniques — tennis or golf ball in a sock or a bra worn backwards, body pillows, special shirts and vests — with poor; B refers instead to positional data from the PSG establish the indication; C refers instead to Sleep behavioural therapy = sleep hygiene, sleeping position, weight control, avoidance of sedatives including alcohol; D refers instead to CPAP lower · OAT higher · positional therapy similar to OAT.

REFERENCE Integrated notebook, page 78, §3.6

QUESTION 0305 · §3.5 · Applied clinical SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. Which of the following statements about nasal (usually adjunctive/pre-phase) is most accurate?

A. Nasal operations may improve obstruction and PAP tolerance but are usually adjunctive OSA treatments.

B. Positional therapy aims to reduce supine sleep in patients with position-dependent obstruction.

C. Tongue-base surgery can include genioglossus advancement or lingual tissue reduction in selected cases.

D. More than fifty percent reduction in AHI with postoperative AHI below twenty.

E. Heated humidification can relieve upper-airway dryness and nasal irritation during PAP treatment.

CORRECT ANSWER A

Nasal operations may improve obstruction and PAP tolerance but are usually adjunctive OSA treatments. Additional revision detail: septoplasty; inferior turbinectomy or turbinoplasty; nasal valve surgery for lateral wall collapse; endoscopic sinus surgery; polypectomy. The other options describe different mechanisms, investigations or findings rather than nasal (usually adjunctive/pre-phase). Consult Integrated notebook, page 63, §3.5 for the full context.

REFERENCE Integrated notebook, page 63, §3.5

QUESTION 0306 · §3.7 · Clinical interpretation SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. The clinical team records the following finding: carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype. Which concept best explains this observation?

A. Hypoglossal stimulation anatomy

B. Modafinil / armodafinil

C. GLP-1 receptor agonists (e.g. tirzepatide)

D. Acetazolamide

E. Orofacial myofunctional therapy

CORRECT ANSWER D

This finding most directly describes Acetazolamide: carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype. Distractor distinctions: A refers instead to Electrical stimulation of the hypoglossal nerve recruits tongue dilator function; B refers instead to recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifi; C refers instead to reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most signi; E refers instead to targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct.

REFERENCE Integrated notebook, page 65, §3.7

QUESTION 0307 · §3.6 · Applied clinical SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. Which of the following statements about principle is most accurate?

A. Positional therapy aims to reduce supine sleep in patients with position-dependent obstruction.

B. Behavioural management addresses sleep habits, weight, posture and sedative exposure.

C. A positional treatment approach should be considered when sleep testing confirms positional dependence.

D. Mechanical anti-supine devices can reduce positional sleep but frequently have poor long-term adherence.

E. Targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct.

CORRECT ANSWER A

Positional therapy aims to reduce supine sleep in patients with position-dependent obstruction. Additional revision detail: promotes side sleeping instead of supine sleeping. Snoring severity and AHI are greater supine; in children the lowest AHI is often prone. The other options describe different mechanisms, investigations or findings rather than principle. Consult Integrated notebook, page 78, §3.6 for the full context.

REFERENCE Integrated notebook, page 78, §3.6

QUESTION 0308 · §3.8 · Clinical interpretation SBA

A patient who finds continuous positive airway pressure difficult to tolerate asks about alternatives. The clinical team records the following finding: improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of appliance side effects. Which concept best explains this observation?

A. OAT + CPAP, connected or unconnected

B. OAT and CPAP used interchangeably

C. OAT + positional therapy

D. OAT + nasal or oral EPAP

E. OAT + CPAP (connected or not)

CORRECT ANSWER B

This finding most directly describes OAT and CPAP used interchangeably: improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of appliance side effects. Distractor distinctions: A refers instead to for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage; C refers instead to CPAP + weight loss — incremental reduction in insulin resistance, triglycerides and blood pressure versus either alone; D refers instead to significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency; E refers instead to for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting.

REFERENCE Integrated notebook, page 65, §3.8

QUESTION 0309 · §3.7 · Applied clinical SBA

A patient who finds continuous positive airway pressure difficult to tolerate asks about alternatives. Which of the following statements about acetazolamide is most accurate?

A. Targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct.

B. GLP-1 receptor agonist therapies can reduce weight and AHI in selected patients with obesity-related OSA.

C. Hypoglossal stimulation anatomy includes electrical stimulation of the hypoglossal nerve recruits tongue dilator function.

D. Acetazolamide includes carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype.

E. Relevant findings include target poor muscle responsiveness.

CORRECT ANSWER D

Acetazolamide includes carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype. Additional revision detail: carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype. The other options describe different mechanisms, investigations or findings rather than acetazolamide. Consult Integrated notebook, page 65, §3.7 for the full context.

REFERENCE Integrated notebook, page 65, §3.7

QUESTION 0310 · §3.9 · Clinical interpretation SBA

A patient returning to the sleep clinic asks about the relative effects of different OSA treatments. The clinical team records the following finding: PAP typically has higher physiological suppression of AHI, while OAT can be better tolerated. Which concept best explains this observation?

A. AHI reduction

B. Versus positional therapy

C. PAP versus oral appliance efficacy

D. Effective oral appliance (AADSM 2019)

E. Adherence

CORRECT ANSWER C

This finding most directly describes PAP versus oral appliance efficacy: PAP typically has higher physiological suppression of AHI, while OAT can be better tolerated. Distractor distinctions: A refers instead to CPAP ~75% · OA ~42% · UPPP ~30%. MDA: OA ~51% vs CPAP ~50%. MAD 6.5 vs CPAP 5.2 h/night; B refers instead to OAT better for sleepiness; other measures similar in supine-dependent OSA; D refers instead to advancement in increments of 1 mm or less; protrusive adjustment range ≥5 mm; E refers instead to CPAP lower · OAT higher · positional therapy similar to OAT.

REFERENCE Integrated notebook, page 67, §3.9

QUESTION 0311 · §3.8 · Applied clinical SBA

A patient returning to the sleep clinic asks about the relative effects of different OSA treatments. Which of the following statements about oat and CPAP used interchangeably is most accurate?

A. For severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone.

B. Combining nasal EPAP with oral appliances may lower residual AHI in some incomplete responders.

C. Combining CPAP with weight reduction may improve insulin resistance and blood pressure beyond either alone.

D. Combining oral appliances and PAP may increase overall treatment exposure in selected patients.

E. Alternating PAP and oral appliances may improve overall treatment exposure in selected patients.

CORRECT ANSWER E

Alternating PAP and oral appliances may improve overall treatment exposure in selected patients. Additional revision detail: improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of appliance side effects. The other options describe different mechanisms, investigations or findings rather than oat and cpap used interchangeably. Consult Integrated notebook, page 65, §3.8 for the full context.

REFERENCE Integrated notebook, page 65, §3.8

QUESTION 0312 · §3.10 · Clinical interpretation SBA

A sleep physician and dentist are discussing the most suitable treatment pathway. The clinical team records the following finding: the extent to which patients correctly follow medical advice, often quantified as the percentage of prescribed therapy actually used. Determines how much of the efficacy is realised. Which concept best explains this observation?

A. Mean disease alleviation = efficacy × compliance

B. Mean disease alleviation calculation

C. Efficacy

D. Compliance

E. Advantages

CORRECT ANSWER D

This finding most directly describes Compliance: the extent to which patients correctly follow medical advice, often quantified as the percentage of prescribed therapy actually used. Determines how much of the efficacy is realised. Distractor distinctions: A refers instead to the overall percentage reduction in disease severity achieved in real- world conditions. Reflects true effectiveness; B refers instead to Physiological efficacy multiplied by the proportion of treatment actually used; C refers instead to the maximum potential benefit of a treatment, typically measured in controlled trials as the reduction in a disease marker su; E refers instead to more efficacious than a MAD for AHI, arousal index, ODI and oxygen saturation.

REFERENCE Integrated notebook, page 66, §3.10

QUESTION 0313 · §3.9 · Applied clinical SBA

A sleep physician and dentist are discussing the most suitable treatment pathway. Which of the following statements about pap versus oral appliance efficacy is most accurate?

A. CPAP generally reduces AHI more than oral appliance treatment in indirect comparisons.

B. PAP typically has higher physiological suppression of AHI, while OAT can be better tolerated.

C. Mean disease alleviation combines physiological efficacy with actual treatment adherence.

D. OAT better for sleepiness; other measures similar in supine-dependent OSA.

E. Oral appliances may achieve greater nightly use than CPAP, although adherence varies between patients.

CORRECT ANSWER B

PAP typically has higher physiological suppression of AHI, while OAT can be better tolerated. The other options describe different mechanisms, investigations or findings rather than pap versus oral appliance efficacy. Consult Integrated notebook, page 67, §3.9 for the full context.

REFERENCE Integrated notebook, page 67, §3.9

QUESTION 0314 · §3.11 · Focused knowledge SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. Which of the following statements about weight is most accurate?

A. The device should provide adjustment increments no larger than 1 mm across at least 5 mm of travel.

B. An effective titratable appliance permits reversible adjustments of 1 mm or less over a range of at least 5 mm.

C. Minimum protrusive distance for MAD: 6 mm (limited protrusion <6 mm is a contraindication).

D. CPAP produces a larger average AHI reduction than oral appliance therapy in indirect comparisons.

E. A 10% weight gain is associated with an approximately 32% rise in AHI.

CORRECT ANSWER E

A 10% weight gain is associated with an approximately 32% rise in AHI. Additional revision detail: 10% gain → +32% AHI; 10% loss → −26%; BMI target <30; bariatric BMI ≥35. The other options describe different mechanisms, investigations or findings rather than weight. Consult Integrated notebook, page 68, §3.11 for the full context.

REFERENCE Integrated notebook, page 68, §3.11

QUESTION 0315 · §3.10 · Applied clinical SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. Which of the following statements about compliance is most accurate?

A. Adherence describes the extent to which a patient follows the prescribed treatment regimen.

B. The overall percentage reduction in disease severity achieved in realworld conditions. Reflects true effectiveness.

C. Efficacy describes a treatment's maximal benefit under controlled conditions.

D. Mechanical anti-supine devices can reduce positional sleep but frequently have poor long-term adherence.

E. Mean disease alleviation combines physiological efficacy with actual treatment adherence.

CORRECT ANSWER A

Adherence describes the extent to which a patient follows the prescribed treatment regimen. Additional revision detail: the extent to which patients correctly follow medical advice, often quantified as the percentage of prescribed therapy actually used. Determines how much of the efficacy is realised. The other options describe different mechanisms, investigations or findings rather than compliance. Consult Integrated notebook, page 66, §3.10 for the full context.

REFERENCE Integrated notebook, page 66, §3.10

QUESTION 0316 · §3.1 · Clinical interpretation SBA

A sleep physician and dentist are discussing the most suitable treatment pathway. The clinical team records the following finding: the tongue retaining device, holding the tongue forward by suction; historically the second most common type. Which concept best explains this observation?

A. Meier-Ewert et al., 1985

B. Cartwright & Samelson, 1982

C. Pierre Robin, 1903

D. George Catlin (19th c.)

E. Effective oral appliance (AADSM 2019)

CORRECT ANSWER B

This finding most directly describes Cartwright & Samelson, 1982: the tongue retaining device, holding the tongue forward by suction; historically the second most common type. Distractor distinctions: A refers instead to an intraoral protraction device; the 1980s work was the catalyst for the modern field; C refers instead to the monobloc, for glossoptosis. ~30 years later used to reposition the mandible; D refers instead to first suggested breathing affects sleep quality: nasal rather than mouth breathing gives more restful, better quality sleep,; E refers instead to advancement in increments of 1 mm or less; protrusive adjustment range ≥5 mm.

REFERENCE Integrated notebook, page 61, §3.1

QUESTION 0317 · §3.11 · Applied clinical SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. Which of the following statements about weight is most accurate?

A. The device should provide adjustment increments no larger than 1 mm across at least 5 mm of travel.

B. CPAP produces a larger average AHI reduction than oral appliance therapy in indirect comparisons.

C. An effective titratable appliance permits reversible adjustments of 1 mm or less over a range of at least 5 mm.

D. A 10% weight gain is associated with an approximately 32% rise in AHI.

E. Minimum protrusive distance for MAD: 6 mm (limited protrusion <6 mm is a contraindication).

CORRECT ANSWER D

A 10% weight gain is associated with an approximately 32% rise in AHI. Additional revision detail: 10% gain → +32% AHI; 10% loss → −26%; BMI target <30; bariatric BMI ≥35. The other options describe different mechanisms, investigations or findings rather than weight. Consult Integrated notebook, page 68, §3.11 for the full context.

REFERENCE Integrated notebook, page 68, §3.11

QUESTION 0318 · §3.3 · Clinical interpretation SBA

A patient who finds continuous positive airway pressure difficult to tolerate asks about alternatives. The clinical team records the following finding: Division of responsibility: diagnosis, prescription and monitoring of effectiveness rest with the medical practitioner. Which concept best explains this observation?

A. OSA and primary snoring distinction

B. Side effects are usually well tolerated once symptomatic relief is achieved

C. Heated humidification is suggested to reduce side effects

D. Advantages, including over CPAP

E. Indications

CORRECT ANSWER B

This finding most directly describes Side effects are usually well tolerated once symptomatic relief is achieved: Division of responsibility: diagnosis, prescription and monitoring of effectiveness rest with the medical practitioner. Distractor distinctions: A refers instead to OSA must first be excluded by a physician before oral appliance therapy for primary snoring; C refers instead to dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice a; D refers instead to Reduces snoring and the harmful effects of OSA; less obtrusive; E refers instead to Primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA; snoring cannot be dist.

REFERENCE Integrated notebook, page 62, §3.3

QUESTION 0319 · §3.1 · Applied clinical SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. Which of the following statements about cartwright & samelson, 1982 is most accurate?

A. Oral appliance includes a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep.

B. An intraoral protraction device; the 1980s work was the catalyst for the modern field.

C. Pierre Robin described a monobloc appliance for managing glossoptosis.

D. A titratable oral appliance permits progressive mandibular advancement, unlike a fixed-position device.

E. A tongue-retaining device uses suction to maintain an anterior tongue position.

CORRECT ANSWER E

A tongue-retaining device uses suction to maintain an anterior tongue position. Additional revision detail: the tongue retaining device, holding the tongue forward by suction; historically the second most common type. The other options describe different mechanisms, investigations or findings rather than cartwright & samelson, 1982. Consult Integrated notebook, page 61, §3.1 for the full context.

REFERENCE Integrated notebook, page 61, §3.1

QUESTION 0320 · §3.4 · Clinical interpretation SBA

A patient returning to the sleep clinic asks about the relative effects of different OSA treatments. The clinical team records the following finding: Around four centimetres of water pressure. Which concept best explains this observation?

A. Heated humidification is suggested to reduce side effects

B. Initial adult PAP titration pressure

C. Heated humidification suggested

D. Disadvantages

E. Titration

CORRECT ANSWER B

This finding most directly describes Initial adult PAP titration pressure: Around four centimetres of water pressure. Distractor distinctions: A refers instead to dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice a; C refers instead to reduces dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse; D refers instead to lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%; E refers instead to establish the minimum pressure at which snoring ceases and apnoeic events are absent. Start 4 cmH2O, titrate to the lowest ef.

REFERENCE Integrated notebook, page 64, §3.4

QUESTION 0321 · §3.3 · Applied clinical SBA

A patient returning to the sleep clinic asks about the relative effects of different OSA treatments. Which of the following statements about side effects are usually well tolerated once symptomatic relief is achieved is most accurate?

A. Advantages, including over CPAP Reduces snoring and the harmful effects of OSA; less obtrusive.

B. Indications includes primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA.

C. OSA must first be excluded by a physician before oral appliance therapy for primary snoring.

D. The physician diagnoses OSA and verifies treatment efficacy, while the dentist provides and monitors the appliance.

E. Combining CPAP with weight reduction may improve insulin resistance and blood pressure beyond either alone.

CORRECT ANSWER D

The physician diagnoses OSA and verifies treatment efficacy, while the dentist provides and monitors the appliance. Additional revision detail: Division of responsibility: diagnosis, prescription and monitoring of effectiveness rest with the medical practitioner. The other options describe different mechanisms, investigations or findings rather than side effects are usually well tolerated once symptomatic relief is achieved. Consult Integrated notebook, page 62, §3.3 for the full context.

REFERENCE Integrated notebook, page 62, §3.3

QUESTION 0322 · §3.6 · Focused knowledge SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. Which of the following statements about methods is most accurate?

A. Mechanical anti-supine devices can reduce positional sleep but frequently have poor long-term adherence.

B. Positional therapy aims to reduce supine sleep in patients with position-dependent obstruction.

C. Monitoring includes suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual.

D. Behavioural management addresses sleep habits, weight, posture and sedative exposure.

E. A positional treatment approach should be considered when sleep testing confirms positional dependence.

CORRECT ANSWER A

Mechanical anti-supine devices can reduce positional sleep but frequently have poor long-term adherence. Additional revision detail: older techniques — tennis or golf ball in a sock or a bra worn backwards, body pillows, special shirts and vests — with poor long-term adherence due to discomfort and ineffectiveness. The other options describe different mechanisms, investigations or findings rather than methods. Consult Integrated notebook, page 78, §3.6 for the full context.

REFERENCE Integrated notebook, page 78, §3.6

QUESTION 0323 · §3.4 · Applied clinical SBA

A sleep physician and dentist are discussing the most suitable treatment pathway. Which of the following statements about initial adult pap titration pressure is most accurate?

A. CPAP usually reduces AHI and oxygen desaturation more than mandibular advancement.

B. Initial adult pap titration pressure includes around four centimetres of water pressure.

C. CPAP and APAP are both accepted ongoing PAP modalities for appropriately selected adults with OSA.

D. Oral appliance treatment for OSA requires an objective physician-established diagnosis.

E. Disadvantages includes lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%.

CORRECT ANSWER B

Initial adult pap titration pressure includes around four centimetres of water pressure. Additional revision detail: Around four centimetres of water pressure. The other options describe different mechanisms, investigations or findings rather than initial adult pap titration pressure. Consult Integrated notebook, page 64, §3.4 for the full context.

REFERENCE Integrated notebook, page 64, §3.4

QUESTION 0324 · §3.7 · Clinical interpretation SBA

A patient who finds continuous positive airway pressure difficult to tolerate asks about alternatives. The clinical team records the following finding: reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most significant recent pharmacological development. Which concept best explains this observation?

A. GLP-1 receptor agonists (e.g. tirzepatide)

B. Modafinil / armodafinil

C. Noradrenergic–antimuscarinic combinations (atomoxetine + oxybutynin)

D. Hypoglossal stimulation anatomy

E. Orofacial myofunctional therapy

CORRECT ANSWER A

This finding most directly describes GLP-1 receptor agonists (e.g. tirzepatide): reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most significant recent pharmacological development. Distractor distinctions: B refers instead to recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifi; C refers instead to target poor muscle responsiveness; significant AHI reduction and improved sleep quality, particularly in combination; long-te; D refers instead to Electrical stimulation of the hypoglossal nerve recruits tongue dilator function; E refers instead to targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct.

REFERENCE Integrated notebook, page 65, §3.7

QUESTION 0325 · §3.6 · Applied clinical SBA

A patient who finds continuous positive airway pressure difficult to tolerate asks about alternatives. Which of the following statements about methods is most accurate?

A. Positional therapy aims to reduce supine sleep in patients with position-dependent obstruction.

B. A positional treatment approach should be considered when sleep testing confirms positional dependence.

C. Mechanical anti-supine devices can reduce positional sleep but frequently have poor long-term adherence.

D. Behavioural management addresses sleep habits, weight, posture and sedative exposure.

E. Monitoring includes suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual.

CORRECT ANSWER C

Mechanical anti-supine devices can reduce positional sleep but frequently have poor long-term adherence. Additional revision detail: older techniques — tennis or golf ball in a sock or a bra worn backwards, body pillows, special shirts and vests — with poor long-term adherence due to discomfort and ineffectiveness. The other options describe different mechanisms, investigations or findings rather than methods. Consult Integrated notebook, page 78, §3.6 for the full context.

REFERENCE Integrated notebook, page 78, §3.6

QUESTION 0326 · §3.8 · Clinical interpretation SBA

A patient returning to the sleep clinic asks about the relative effects of different OSA treatments. The clinical team records the following finding: incremental reduction in oxybutynin; acetazolamide plus eszopiclone. Which concept best explains this observation?

A. OAT + nasal or oral EPAP

B. OAT + CPAP (connected or not)

C. OAT + CPAP, connected or unconnected

D. OAT and CPAP used interchangeably

E. CPAP + weight loss

CORRECT ANSWER E

This finding most directly describes CPAP + weight loss: incremental reduction in oxybutynin; acetazolamide plus eszopiclone. Distractor distinctions: A refers instead to significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency; B refers instead to for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting; C refers instead to for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage; D refers instead to improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of app.

REFERENCE Integrated notebook, page 80, §3.8

QUESTION 0327 · §3.7 · Applied clinical SBA

A patient returning to the sleep clinic asks about the relative effects of different OSA treatments. Which of the following statements about glp-1 receptor agonists (e.g. tirzepatide) is most accurate?

A. Targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct.

B. Acetazolamide includes carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype.

C. GLP-1 receptor agonist therapies can reduce weight and AHI in selected patients with obesity-related OSA.

D. Relevant findings include target poor muscle responsiveness.

E. Wake-promoting medication may be considered for persistent sleepiness despite optimised OSA therapy.

CORRECT ANSWER C

GLP-1 receptor agonist therapies can reduce weight and AHI in selected patients with obesity-related OSA. Additional revision detail: reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most significant recent pharmacological development. The other options describe different mechanisms, investigations or findings rather than glp-1 receptor agonists (e.g. Consult Integrated notebook, page 65, §3.7 for the full context.

REFERENCE Integrated notebook, page 65, §3.7

QUESTION 0328 · §3.9 · Clinical interpretation SBA

A sleep physician and dentist are discussing the most suitable treatment pathway. The clinical team records the following finding: OAT better for sleepiness; other measures similar in supine-dependent OSA. Which concept best explains this observation?

A. Versus positional therapy

B. Orofacial myofunctional therapy

C. PAP versus oral appliance efficacy

D. Adherence

E. AHI reduction

CORRECT ANSWER A

This finding most directly describes Versus positional therapy: OAT better for sleepiness; other measures similar in supine-dependent OSA. Distractor distinctions: B refers instead to targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct; C refers instead to PAP typically has higher physiological suppression of AHI, while OAT can be better tolerated; D refers instead to CPAP lower · OAT higher · positional therapy similar to OAT; E refers instead to CPAP ~75% · OA ~42% · UPPP ~30%. MDA: OA ~51% vs CPAP ~50%. MAD 6.5 vs CPAP 5.2 h/night.

REFERENCE Integrated notebook, page 66, §3.9

QUESTION 0329 · §3.8 · Applied clinical SBA

A sleep physician and dentist are discussing the most suitable treatment pathway. Which of the following statements about CPAP + weight loss is most accurate?

A. Combining nasal EPAP with oral appliances may lower residual AHI in some incomplete responders.

B. Combining oral appliances and PAP may increase overall treatment exposure in selected patients.

C. CPAP + weight loss includes incremental reduction in oxybutynin; acetazolamide plus eszopiclone.

D. Alternating PAP and oral appliances may improve overall treatment exposure in selected patients.

E. Combining CPAP with weight reduction may improve insulin resistance and blood pressure beyond either alone.

CORRECT ANSWER C

CPAP + weight loss includes incremental reduction in oxybutynin; acetazolamide plus eszopiclone. Additional revision detail: incremental reduction in oxybutynin; acetazolamide plus eszopiclone. The other options describe different mechanisms, investigations or findings rather than cpap + weight loss. Consult Integrated notebook, page 80, §3.8 for the full context.

REFERENCE Integrated notebook, page 80, §3.8

QUESTION 0330 · §3.10 · Clinical interpretation SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. The clinical team records the following finding: Physiological efficacy multiplied by the proportion of treatment actually used. Which concept best explains this observation?

A. Mean disease alleviation = efficacy × compliance

B. Efficacy

C. Initial adult PAP titration pressure

D. Mean disease alleviation calculation

E. Compliance

CORRECT ANSWER D

This finding most directly describes Mean disease alleviation calculation: Physiological efficacy multiplied by the proportion of treatment actually used. Distractor distinctions: A refers instead to the overall percentage reduction in disease severity achieved in real- world conditions. Reflects true effectiveness; B refers instead to the maximum potential benefit of a treatment, typically measured in controlled trials as the reduction in a disease marker su; C refers instead to Around four centimetres of water pressure; E refers instead to the extent to which patients correctly follow medical advice, often quantified as the percentage of prescribed therapy actual.

REFERENCE Integrated notebook, page 67, §3.10

QUESTION 0331 · §3.9 · Applied clinical SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. Which of the following statements about versus positional therapy is most accurate?

A. CPAP generally reduces AHI more than oral appliance treatment in indirect comparisons.

B. OAT better for sleepiness; other measures similar in supine-dependent OSA.

C. PAP typically has higher physiological suppression of AHI, while OAT can be better tolerated.

D. Mean disease alleviation combines physiological efficacy with actual treatment adherence.

E. Oral appliances may achieve greater nightly use than CPAP, although adherence varies between patients.

CORRECT ANSWER B

OAT better for sleepiness; other measures similar in supine-dependent OSA. The other options describe different mechanisms, investigations or findings rather than versus positional therapy. Consult Integrated notebook, page 66, §3.9 for the full context.

REFERENCE Integrated notebook, page 66, §3.9

QUESTION 0332 · §3.11 · Clinical interpretation SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. The clinical team records the following finding: provide appropriate care, reduce inappropriate care, increase effectiveness of care, by promoting best available evidence in decision-making. Which concept best explains this observation?

A. Effective OA

B. UPPP success

C. AHI reduction

D. Weight

E. Purpose of EBD

CORRECT ANSWER E

This finding most directly describes Purpose of EBD: provide appropriate care, reduce inappropriate care, increase effectiveness of care, by promoting best available evidence in decision-making. Distractor distinctions: A refers instead to increments ≤1 mm · range ≥5 mm · reversible · verifiable; B refers instead to >50% AHI reduction and AHI <20; Friedman stage I best, III mostly unsuccessful; ~4% surgical adverse event rate; C refers instead to CPAP ~75% · OA ~42% · UPPP ~30%. MDA: OA ~51% vs CPAP ~50%. MAD 6.5 vs CPAP 5.2 h/night; D refers instead to 10% gain → +32% AHI; 10% loss → −26%; BMI target <30; bariatric BMI ≥35.

REFERENCE Integrated notebook, page 67, §3.11

QUESTION 0333 · §3.10 · Applied clinical SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. Which of the following statements about mean disease alleviation calculation is most accurate?

A. Efficacy describes a treatment's maximal benefit under controlled conditions.

B. Hypoglossal stimulation anatomy includes electrical stimulation of the hypoglossal nerve recruits tongue dilator function.

C. Mean disease alleviation combines physiological efficacy with actual treatment adherence.

D. Adherence describes the extent to which a patient follows the prescribed treatment regimen.

E. The overall percentage reduction in disease severity achieved in realworld conditions. Reflects true effectiveness.

CORRECT ANSWER C

Mean disease alleviation combines physiological efficacy with actual treatment adherence. Additional revision detail: Physiological efficacy multiplied by the proportion of treatment actually used. The other options describe different mechanisms, investigations or findings rather than mean disease alleviation calculation. Consult Integrated notebook, page 67, §3.10 for the full context.

REFERENCE Integrated notebook, page 67, §3.10

QUESTION 0334 · §3.1 · Clinical interpretation SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. The clinical team records the following finding: made from digital or physical impressions/models; not prefabricated; biocompatible material; engages both arches. Non-custom ("boil-and-bite") — prefabricated, only partially modified. Which concept best explains this observation?

A. Oral appliance

B. Modern era

C. Custom OA

D. Titratable

E. Pierre Robin, 1903

CORRECT ANSWER C

This finding most directly describes Custom OA: made from digital or physical impressions/models; not prefabricated; biocompatible material; engages both arches. Non-custom ("boil-and-bite") — prefabricated, only partially modified. Distractor distinctions: A refers instead to a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep; B refers instead to focus on mandibular advancement appliances; case reports gave way to RCTs confirming efficacy in mild– moderate and more rece; D refers instead to allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible; E refers instead to the monobloc, for glossoptosis. ~30 years later used to reposition the mandible.

REFERENCE Integrated notebook, page 61, §3.1

QUESTION 0335 · §3.11 · Applied clinical SBA

A patient who finds continuous positive airway pressure difficult to tolerate asks about alternatives. Which of the following statements about purpose of ebd is most accurate?

A. The device should provide adjustment increments no larger than 1 mm across at least 5 mm of travel.

B. UPPP success: >50% AHI reduction and AHI <20; Friedman stage I best, III mostly unsuccessful; ~4% surgical adverse event rate.

C. The clinician should provide appropriate care, reduce inappropriate care, increase effectiveness of care, by promoting best.

D. An effective titratable appliance permits reversible adjustments of 1 mm or less over a range of at least 5 mm.

E. CPAP produces a larger average AHI reduction than oral appliance therapy in indirect comparisons.

CORRECT ANSWER C

The clinician should provide appropriate care, reduce inappropriate care, increase effectiveness of care, by promoting best. Additional revision detail: provide appropriate care, reduce inappropriate care, increase effectiveness of care, by promoting best available evidence in decision-making. The other options describe different mechanisms, investigations or findings rather than purpose of ebd. Consult Integrated notebook, page 67, §3.11 for the full context.

REFERENCE Integrated notebook, page 67, §3.11

QUESTION 0336 · §3.4 · Clinical interpretation SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. The clinical team records the following finding: establish the minimum pressure at which snoring ceases and apnoeic events are absent. Start 4 cmH2O, titrate to the lowest effective pressure, maximum ~20 cmH2O. Which concept best explains this observation?

A. Titration

B. Advantages

C. Initial adult PAP titration pressure

D. Monitoring

E. Modality

CORRECT ANSWER A

This finding most directly describes Titration: establish the minimum pressure at which snoring ceases and apnoeic events are absent. Start 4 cmH2O, titrate to the lowest effective pressure, maximum ~20 cmH2O. Distractor distinctions: B refers instead to more efficacious than a MAD for AHI, arousal index, ODI and oxygen saturation; C refers instead to Around four centimetres of water pressure; D refers instead to suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual severity, uninten; E refers instead to recommend either CPAP or APAP for ongoing treatment (no difference in adherence, sleepiness or quality of life; APAP's advant.

REFERENCE Integrated notebook, page 62, §3.4

QUESTION 0337 · §3.1 · Applied clinical SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. Which of the following statements about custom oa is most accurate?

A. Custom oa includes made from digital or physical impressions/models.

B. A tongue-retaining device uses suction to maintain an anterior tongue position.

C. Modern era includes focus on mandibular advancement appliances; case reports gave way to RCTs confirming efficacy in mild–.

D. First suggested breathing affects sleep quality: nasal rather than mouth breathing gives more restful, better quality sleep.

E. A titratable oral appliance permits progressive mandibular advancement, unlike a fixed-position device.

CORRECT ANSWER A

Custom oa includes made from digital or physical impressions/models. Additional revision detail: made from digital or physical impressions/models; not prefabricated; biocompatible material; engages both arches. Non-custom ("boil-and-bite") — prefabricated, only partially modified. The other options describe different mechanisms, investigations or findings rather than custom oa. Consult Integrated notebook, page 61, §3.1 for the full context.

REFERENCE Integrated notebook, page 61, §3.1

QUESTION 0338 · §3.7 · Clinical interpretation SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. The clinical team records the following finding: targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct. Which concept best explains this observation?

A. Hypoglossal stimulation anatomy

B. GLP-1 receptor agonists (e.g. tirzepatide)

C. Acetazolamide

D. Modafinil / armodafinil

E. Orofacial myofunctional therapy

CORRECT ANSWER E

This finding most directly describes Orofacial myofunctional therapy: targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct. Distractor distinctions: A refers instead to Electrical stimulation of the hypoglossal nerve recruits tongue dilator function; B refers instead to reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most signi; C refers instead to carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype; D refers instead to recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifi.

REFERENCE Integrated notebook, page 65, §3.7

QUESTION 0339 · §3.4 · Applied clinical SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. Which of the following statements about titration is most accurate?

A. Heated humidification can relieve upper-airway dryness and nasal irritation during PAP treatment.

B. Monitoring includes suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual.

C. PAP titration seeks the lowest pressure that adequately controls respiratory events.

D. CPAP and APAP are both accepted ongoing PAP modalities for appropriately selected adults with OSA.

E. Disadvantages includes lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%.

CORRECT ANSWER C

PAP titration seeks the lowest pressure that adequately controls respiratory events. Additional revision detail: establish the minimum pressure at which snoring ceases and apnoeic events are absent. Start 4 cmH2O, titrate to the lowest effective pressure, maximum ~20 cmH2O. The other options describe different mechanisms, investigations or findings rather than titration. Consult Integrated notebook, page 62, §3.4 for the full context.

REFERENCE Integrated notebook, page 62, §3.4

QUESTION 0340 · §3.8 · Clinical interpretation SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. The clinical team records the following finding: for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage of time on treatment, while. Which concept best explains this observation?

A. CPAP + weight loss

B. OAT + CPAP, connected or unconnected

C. OAT and CPAP used interchangeably

D. OAT + nasal or oral EPAP

E. OAT + positional therapy

CORRECT ANSWER B

This finding most directly describes OAT + CPAP, connected or unconnected: for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage of time on treatment, while. Distractor distinctions: A refers instead to incremental reduction in oxybutynin; acetazolamide plus eszopiclone; C refers instead to improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of app; D refers instead to significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency; E refers instead to CPAP + weight loss — incremental reduction in insulin resistance, triglycerides and blood pressure versus either alone.

REFERENCE Integrated notebook, page 79, §3.8

QUESTION 0341 · §3.7 · Applied clinical SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. Which of the following statements about orofacial myofunctional therapy is most accurate?

A. Targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct.

B. Relevant findings include target poor muscle responsiveness.

C. Acetazolamide includes carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype.

D. Wake-promoting medication may be considered for persistent sleepiness despite optimised OSA therapy.

E. Hypoglossal stimulation anatomy includes electrical stimulation of the hypoglossal nerve recruits tongue dilator function.

CORRECT ANSWER A

Targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct. The other options describe different mechanisms, investigations or findings rather than orofacial myofunctional therapy. Consult Integrated notebook, page 65, §3.7 for the full context.

REFERENCE Integrated notebook, page 65, §3.7

QUESTION 0342 · §3.11 · Clinical interpretation SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. The clinical team records the following finding: CPAP ~75% · OA ~42% · UPPP ~30%. MDA: OA ~51% vs CPAP ~50%. MAD 6.5 vs CPAP 5.2 h/night. Which concept best explains this observation?

A. Purpose of EBD

B. UPPP success

C. Effective OA

D. Weight

E. AHI reduction

CORRECT ANSWER E

This finding most directly describes AHI reduction: CPAP ~75% · OA ~42% · UPPP ~30%. MDA: OA ~51% vs CPAP ~50%. MAD 6.5 vs CPAP 5.2 h/night. Distractor distinctions: A refers instead to provide appropriate care, reduce inappropriate care, increase effectiveness of care, by promoting best available evidence in; B refers instead to >50% AHI reduction and AHI <20; Friedman stage I best, III mostly unsuccessful; ~4% surgical adverse event rate; C refers instead to increments ≤1 mm · range ≥5 mm · reversible · verifiable; D refers instead to 10% gain → +32% AHI; 10% loss → −26%; BMI target <30; bariatric BMI ≥35.

REFERENCE Integrated notebook, page 68, §3.11

QUESTION 0343 · §3.8 · Applied clinical SBA

A patient who finds continuous positive airway pressure difficult to tolerate asks about alternatives. Which of the following statements about oat + CPAP, connected or unconnected is most accurate?

A. Alternating PAP and oral appliances may improve overall treatment exposure in selected patients.

B. Combining CPAP with weight reduction may improve insulin resistance and blood pressure beyond either alone.

C. Combining oral appliances and PAP may increase overall treatment exposure in selected patients.

D. Combining nasal EPAP with oral appliances may lower residual AHI in some incomplete responders.

E. For severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone.

CORRECT ANSWER C

Combining oral appliances and PAP may increase overall treatment exposure in selected patients. Additional revision detail: for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage of time on treatment, while. The other options describe different mechanisms, investigations or findings rather than oat + cpap, connected or unconnected. Consult Integrated notebook, page 79, §3.8 for the full context.

REFERENCE Integrated notebook, page 79, §3.8

QUESTION 0344 · §3.1 · Clinical interpretation SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. The clinical team records the following finding: allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible. Which concept best explains this observation?

A. Pierre Robin, 1903

B. Oral appliance

C. Modern era

D. Titratable

E. Custom OA

CORRECT ANSWER D

This finding most directly describes Titratable: allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible. Distractor distinctions: A refers instead to the monobloc, for glossoptosis. ~30 years later used to reposition the mandible; B refers instead to a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep; C refers instead to focus on mandibular advancement appliances; case reports gave way to RCTs confirming efficacy in mild– moderate and more rece; E refers instead to made from digital or physical impressions/models; not prefabricated; biocompatible material; engages both arches. Non-custom.

REFERENCE Integrated notebook, page 61, §3.1

QUESTION 0345 · §3.11 · Applied clinical SBA

A patient who finds continuous positive airway pressure difficult to tolerate asks about alternatives. Which of the following statements about AHI reduction is most accurate?

A. Minimum protrusive distance for MAD: 6 mm (limited protrusion <6 mm is a contraindication).

B. The device should provide adjustment increments no larger than 1 mm across at least 5 mm of travel.

C. CPAP produces a larger average AHI reduction than oral appliance therapy in indirect comparisons.

D. An effective titratable appliance permits reversible adjustments of 1 mm or less over a range of at least 5 mm.

E. A 10% weight gain is associated with an approximately 32% rise in AHI.

CORRECT ANSWER C

CPAP produces a larger average AHI reduction than oral appliance therapy in indirect comparisons. Additional revision detail: CPAP ~75% · OA ~42% · UPPP ~30%. MDA: OA ~51% vs CPAP ~50%. MAD 6.5 vs CPAP 5.2 h/night. The other options describe different mechanisms, investigations or findings rather than ahi reduction. Consult Integrated notebook, page 68, §3.11 for the full context.

REFERENCE Integrated notebook, page 68, §3.11

QUESTION 0346 · §3.4 · Clinical interpretation SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. The clinical team records the following finding: reduces dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice, reduced smell. Trade-offs: "rain-out", cost, maintenance. Which concept best explains this observation?

A. Heated humidification suggested

B. Disadvantages

C. Good practice

D. Heated humidification is suggested to reduce side effects

E. Initial adult PAP titration pressure

CORRECT ANSWER A

This finding most directly describes Heated humidification suggested: reduces dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice, reduced smell. Trade-offs: "rain-out", cost, maintenance. Distractor distinctions: B refers instead to lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%; C refers instead to treatment must be based on a diagnosis established by objective sleep apnoea testing; D refers instead to dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice a; E refers instead to Around four centimetres of water pressure.

REFERENCE Integrated notebook, page 62, §3.4

QUESTION 0347 · §3.1 · Applied clinical SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. Which of the following statements about titratable is most accurate?

A. A titratable oral appliance permits progressive mandibular advancement, unlike a fixed-position device.

B. Pierre Robin described a monobloc appliance for managing glossoptosis.

C. An intraoral protraction device; the 1980s work was the catalyst for the modern field.

D. Oral appliance includes a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep.

E. A tongue-retaining device uses suction to maintain an anterior tongue position.

CORRECT ANSWER A

A titratable oral appliance permits progressive mandibular advancement, unlike a fixed-position device. Additional revision detail: allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible. The other options describe different mechanisms, investigations or findings rather than titratable. Consult Integrated notebook, page 61, §3.1 for the full context.

REFERENCE Integrated notebook, page 61, §3.1

QUESTION 0348 · §3.7 · Clinical interpretation SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. The clinical team records the following finding: target poor muscle responsiveness; significant AHI reduction and improved sleep quality, particularly in combination; long-term safety and selection under investigation. Which concept best explains this observation?

A. GLP-1 receptor agonists (e.g. tirzepatide)

B. Noradrenergic–antimuscarinic combinations (atomoxetine + oxybutynin)

C. Orofacial myofunctional therapy

D. Modafinil / armodafinil

E. Hypoglossal stimulation anatomy

CORRECT ANSWER B

This finding most directly describes Noradrenergic–antimuscarinic combinations (atomoxetine + oxybutynin): target poor muscle responsiveness; significant AHI reduction and improved sleep quality, particularly in combination; long-term safety and selection under investigation. Distractor distinctions: A refers instead to reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most signi; C refers instead to targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct; D refers instead to recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifi; E refers instead to Electrical stimulation of the hypoglossal nerve recruits tongue dilator function.

REFERENCE Integrated notebook, page 65, §3.7

QUESTION 0349 · §3.4 · Applied clinical SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. Which of the following statements about heated humidification suggested is most accurate?

A. PAP titration seeks the lowest pressure that adequately controls respiratory events.

B. Disadvantages includes lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%.

C. CPAP and APAP are both accepted ongoing PAP modalities for appropriately selected adults with OSA.

D. Heated humidification can relieve upper-airway dryness and nasal irritation during PAP treatment.

E. Monitoring includes suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual.

CORRECT ANSWER D

Heated humidification can relieve upper-airway dryness and nasal irritation during PAP treatment. Additional revision detail: reduces dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice, reduced smell. Trade-offs: "rain-out", cost, maintenance. The other options describe different mechanisms, investigations or findings rather than heated humidification suggested. Consult Integrated notebook, page 62, §3.4 for the full context.

REFERENCE Integrated notebook, page 62, §3.4

QUESTION 0350 · §3.8 · Clinical interpretation SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. The clinical team records the following finding: CPAP + weight loss — incremental reduction in insulin resistance, triglycerides and blood pressure versus either alone. Which concept best explains this observation?

A. OAT + nasal or oral EPAP

B. OAT + CPAP (connected or not)

C. OAT and CPAP used interchangeably

D. OAT + CPAP, connected or unconnected

E. OAT + positional therapy

CORRECT ANSWER E

This finding most directly describes OAT + positional therapy: CPAP + weight loss — incremental reduction in insulin resistance, triglycerides and blood pressure versus either alone. Distractor distinctions: A refers instead to significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency; B refers instead to for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting; C refers instead to improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of app; D refers instead to for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage.

REFERENCE Integrated notebook, page 66, §3.8

QUESTION 0351 · §3.7 · Applied clinical SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. Which of the following statements about noradrenergic–antimuscarinic combinations (atomoxetine + oxybutynin) is most accurate?

A. GLP-1 receptor agonist therapies can reduce weight and AHI in selected patients with obesity-related OSA.

B. Acetazolamide includes carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype.

C. Wake-promoting medication may be considered for persistent sleepiness despite optimised OSA therapy.

D. Relevant findings include target poor muscle responsiveness.

E. Targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct.

CORRECT ANSWER D

Relevant findings include target poor muscle responsiveness. Additional revision detail: target poor muscle responsiveness; significant AHI reduction and improved sleep quality, particularly in combination; long-term safety and selection under investigation. The other options describe different mechanisms, investigations or findings rather than noradrenergic–antimuscarinic combinations (atomoxetine + oxybutynin). Consult Integrated notebook, page 65, §3.7 for the full context.

REFERENCE Integrated notebook, page 65, §3.7

QUESTION 0352 · §3.11 · Clinical interpretation SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. The clinical team records the following finding: increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting. Which concept best explains this observation?

A. Effective mandibular appliance adjustment

B. Effective OA

C. Purpose of EBD

D. Effective OA definition

E. AHI reduction

CORRECT ANSWER D

This finding most directly describes Effective OA definition: increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting. Distractor distinctions: A refers instead to At most one-millimetre increments across a range of at least five millimetres; B refers instead to increments ≤1 mm · range ≥5 mm · reversible · verifiable; C refers instead to provide appropriate care, reduce inappropriate care, increase effectiveness of care, by promoting best available evidence in; E refers instead to CPAP ~75% · OA ~42% · UPPP ~30%. MDA: OA ~51% vs CPAP ~50%. MAD 6.5 vs CPAP 5.2 h/night.

REFERENCE Integrated notebook, page 82, §3.11

QUESTION 0353 · §3.8 · Applied clinical SBA

A patient who finds continuous positive airway pressure difficult to tolerate asks about alternatives. Which of the following statements about oat + positional therapy is most accurate?

A. For severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone.

B. Combining CPAP with weight reduction may improve insulin resistance and blood pressure beyond either alone.

C. Alternating PAP and oral appliances may improve overall treatment exposure in selected patients.

D. Combining oral appliances and PAP may increase overall treatment exposure in selected patients.

E. CPAP + weight loss includes incremental reduction in oxybutynin; acetazolamide plus eszopiclone.

CORRECT ANSWER B

Combining CPAP with weight reduction may improve insulin resistance and blood pressure beyond either alone. Additional revision detail: CPAP + weight loss — incremental reduction in insulin resistance, triglycerides and blood pressure versus either alone. The other options describe different mechanisms, investigations or findings rather than oat + positional therapy. Consult Integrated notebook, page 66, §3.8 for the full context.

REFERENCE Integrated notebook, page 66, §3.8

QUESTION 0354 · §3.1 · Clinical interpretation SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. The clinical team records the following finding: a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep. Which concept best explains this observation?

A. Oral appliance

B. Modern era

C. Effective oral appliance (AADSM 2019)

D. Pierre Robin, 1903

E. Titratable

CORRECT ANSWER A

This finding most directly describes Oral appliance: a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep. Distractor distinctions: B refers instead to focus on mandibular advancement appliances; case reports gave way to RCTs confirming efficacy in mild– moderate and more rece; C refers instead to advancement in increments of 1 mm or less; protrusive adjustment range ≥5 mm; D refers instead to the monobloc, for glossoptosis. ~30 years later used to reposition the mandible; E refers instead to allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible.

REFERENCE Integrated notebook, page 61, §3.1

QUESTION 0355 · §3.11 · Applied clinical SBA

A patient who finds continuous positive airway pressure difficult to tolerate asks about alternatives. Which of the following statements about effective oa definition is most accurate?

A. CPAP produces a larger average AHI reduction than oral appliance therapy in indirect comparisons.

B. The device should provide adjustment increments no larger than 1 mm across at least 5 mm of travel.

C. A 10% weight gain is associated with an approximately 32% rise in AHI.

D. Minimum protrusive distance for MAD: 6 mm (limited protrusion <6 mm is a contraindication).

E. An effective titratable appliance permits reversible adjustments of 1 mm or less over a range of at least 5 mm.

CORRECT ANSWER E

An effective titratable appliance permits reversible adjustments of 1 mm or less over a range of at least 5 mm. Additional revision detail: increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting. The other options describe different mechanisms, investigations or findings rather than effective oa definition. Consult Integrated notebook, page 82, §3.11 for the full context.

REFERENCE Integrated notebook, page 82, §3.11

QUESTION 0356 · §3.4 · Clinical interpretation SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. The clinical team records the following finding: recommend either CPAP or APAP for ongoing treatment (no difference in adherence, sleepiness or quality of life; APAP's advantage is automatic adjustment for alcohol, position and weight change). Which concept best explains this observation?

A. Advantages

B. Disadvantages

C. Monitoring

D. Titration

E. Modality

CORRECT ANSWER E

This finding most directly describes Modality: recommend either CPAP or APAP for ongoing treatment (no difference in adherence, sleepiness or quality of life; APAP's advantage is automatic adjustment for alcohol, position and weight change). Distractor distinctions: A refers instead to more efficacious than a MAD for AHI, arousal index, ODI and oxygen saturation; B refers instead to lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%; C refers instead to suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual severity, uninten; D refers instead to establish the minimum pressure at which snoring ceases and apnoeic events are absent. Start 4 cmH2O, titrate to the lowest ef.

REFERENCE Integrated notebook, page 63, §3.4

QUESTION 0357 · §3.1 · Applied clinical SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. Which of the following statements about oral appliance is most accurate?

A. Effective oral appliance (aadsm 2019) includes advancement in increments of 1 mm or less; protrusive adjustment range ≥5 mm.

B. A tongue-retaining device uses suction to maintain an anterior tongue position.

C. Oral appliance includes a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep.

D. A titratable oral appliance permits progressive mandibular advancement, unlike a fixed-position device.

E. An intraoral protraction device; the 1980s work was the catalyst for the modern field.

CORRECT ANSWER C

Oral appliance includes a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep. Additional revision detail: a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep. The other options describe different mechanisms, investigations or findings rather than oral appliance. Consult Integrated notebook, page 61, §3.1 for the full context.

REFERENCE Integrated notebook, page 61, §3.1

QUESTION 0358 · §3.11 · Clinical interpretation SBA

A patient returning to the sleep clinic asks about the relative effects of different OSA treatments. The clinical team records the following finding: 6 mm (limited protrusion <6 mm is a contraindication). Which concept best explains this observation?

A. Minimum protrusive distance for MAD

B. Effective mandibular appliance adjustment

C. AHI reduction

D. Purpose of EBD

E. Effective OA definition

CORRECT ANSWER A

This finding most directly describes Minimum protrusive distance for MAD: 6 mm (limited protrusion <6 mm is a contraindication). Distractor distinctions: B refers instead to At most one-millimetre increments across a range of at least five millimetres; C refers instead to CPAP ~75% · OA ~42% · UPPP ~30%. MDA: OA ~51% vs CPAP ~50%. MAD 6.5 vs CPAP 5.2 h/night; D refers instead to provide appropriate care, reduce inappropriate care, increase effectiveness of care, by promoting best available evidence in; E refers instead to increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting.

REFERENCE Integrated notebook, page 82, §3.11

QUESTION 0359 · §3.4 · Applied clinical SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. Which of the following statements about modality is most accurate?

A. Oral appliance treatment for OSA requires an objective physician-established diagnosis.

B. Monitoring includes suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual.

C. PAP titration seeks the lowest pressure that adequately controls respiratory events.

D. Heated humidification can relieve upper-airway dryness and nasal irritation during PAP treatment.

E. CPAP and APAP are both accepted ongoing PAP modalities for appropriately selected adults with OSA.

CORRECT ANSWER E

CPAP and APAP are both accepted ongoing PAP modalities for appropriately selected adults with OSA. Additional revision detail: recommend either CPAP or APAP for ongoing treatment (no difference in adherence, sleepiness or quality of life; APAP's advantage is automatic adjustment for alcohol, position and weight change). The other options describe different mechanisms, investigations or findings rather than modality. Consult Integrated notebook, page 63, §3.4 for the full context.

REFERENCE Integrated notebook, page 63, §3.4

QUESTION 0360 · §3.1 · Clinical interpretation SBA

A patient who finds continuous positive airway pressure difficult to tolerate asks about alternatives. The clinical team records the following finding: focus on mandibular advancement appliances; case reports gave way to RCTs confirming efficacy in mild– moderate and more recently severe OSA. Which concept best explains this observation?

A. Custom OA

B. Oral appliance

C. Modern era

D. Titratable

E. Pierre Robin, 1903

CORRECT ANSWER C

This finding most directly describes Modern era: focus on mandibular advancement appliances; case reports gave way to RCTs confirming efficacy in mild– moderate and more recently severe OSA. Distractor distinctions: A refers instead to made from digital or physical impressions/models; not prefabricated; biocompatible material; engages both arches. Non-custom; B refers instead to a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep; D refers instead to allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible; E refers instead to the monobloc, for glossoptosis. ~30 years later used to reposition the mandible.

REFERENCE Integrated notebook, page 61, §3.1

QUESTION 0361 · §3.11 · Applied clinical SBA

A sleep physician and dentist are discussing the most suitable treatment pathway. Which of the following statements about minimum protrusive distance for MAD is most accurate?

A. A 10% weight gain is associated with an approximately 32% rise in AHI.

B. The device should provide adjustment increments no larger than 1 mm across at least 5 mm of travel.

C. A titratable appliance should permit verifiable incremental advancement of 1 mm or less over at least 5 mm.

D. CPAP produces a larger average AHI reduction than oral appliance therapy in indirect comparisons.

E. Minimum protrusive distance for MAD: 6 mm (limited protrusion <6 mm is a contraindication).

CORRECT ANSWER E

Minimum protrusive distance for MAD: 6 mm (limited protrusion <6 mm is a contraindication). Additional revision detail: 6 mm (limited protrusion <6 mm is a contraindication). The other options describe different mechanisms, investigations or findings rather than minimum protrusive distance for mad. Consult Integrated notebook, page 82, §3.11 for the full context.

REFERENCE Integrated notebook, page 82, §3.11

QUESTION 0362 · §3.4 · Clinical interpretation SBA

A patient who finds continuous positive airway pressure difficult to tolerate asks about alternatives. The clinical team records the following finding: lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%. Which concept best explains this observation?

A. Titration

B. Disadvantages

C. Monitoring

D. Good practice

E. Modality

CORRECT ANSWER B

This finding most directly describes Disadvantages: lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%. Distractor distinctions: A refers instead to establish the minimum pressure at which snoring ceases and apnoeic events are absent. Start 4 cmH2O, titrate to the lowest ef; C refers instead to suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual severity, uninten; D refers instead to treatment must be based on a diagnosis established by objective sleep apnoea testing; E refers instead to recommend either CPAP or APAP for ongoing treatment (no difference in adherence, sleepiness or quality of life; APAP's advant.

REFERENCE Integrated notebook, page 62, §3.4

QUESTION 0363 · §3.1 · Applied clinical SBA

A patient returning to the sleep clinic asks about the relative effects of different OSA treatments. Which of the following statements about modern era is most accurate?

A. A tongue-retaining device uses suction to maintain an anterior tongue position.

B. A titratable oral appliance permits progressive mandibular advancement, unlike a fixed-position device.

C. Modern era includes focus on mandibular advancement appliances; case reports gave way to RCTs confirming efficacy in mild–.

D. First suggested breathing affects sleep quality: nasal rather than mouth breathing gives more restful, better quality sleep.

E. Oral appliance includes a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep.

CORRECT ANSWER C

Modern era includes focus on mandibular advancement appliances; case reports gave way to RCTs confirming efficacy in mild–. Additional revision detail: focus on mandibular advancement appliances; case reports gave way to RCTs confirming efficacy in mild– moderate and more recently severe OSA. The other options describe different mechanisms, investigations or findings rather than modern era. Consult Integrated notebook, page 61, §3.1 for the full context.

REFERENCE Integrated notebook, page 61, §3.1

QUESTION 0364 · §3.1 · Clinical interpretation SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. The clinical team records the following finding: advancement in increments of 1 mm or less; protrusive adjustment range ≥5 mm. Which concept best explains this observation?

A. Cartwright & Samelson, 1982

B. Oral appliance

C. Meier-Ewert et al., 1985

D. Effective oral appliance (AADSM 2019)

E. George Catlin (19th c.)

CORRECT ANSWER D

This finding most directly describes Effective oral appliance (AADSM 2019): advancement in increments of 1 mm or less; protrusive adjustment range ≥5 mm. Distractor distinctions: A refers instead to the tongue retaining device, holding the tongue forward by suction; historically the second most common type; B refers instead to a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep; C refers instead to an intraoral protraction device; the 1980s work was the catalyst for the modern field; E refers instead to first suggested breathing affects sleep quality: nasal rather than mouth breathing gives more restful, better quality sleep,.

REFERENCE Integrated notebook, page 61, §3.1

QUESTION 0365 · §3.4 · Applied clinical SBA

A patient returning to the sleep clinic asks about the relative effects of different OSA treatments. Which of the following statements about disadvantages is most accurate?

A. Disadvantages includes lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%.

B. Oral appliance treatment for OSA requires an objective physician-established diagnosis.

C. CPAP usually reduces AHI and oxygen desaturation more than mandibular advancement.

D. Heated humidification can relieve upper-airway dryness and nasal irritation during PAP treatment.

E. PAP titration seeks the lowest pressure that adequately controls respiratory events.

CORRECT ANSWER A

Disadvantages includes lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%. Additional revision detail: lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%. The other options describe different mechanisms, investigations or findings rather than disadvantages. Consult Integrated notebook, page 62, §3.4 for the full context.

REFERENCE Integrated notebook, page 62, §3.4

QUESTION 0366 · §3.4 · Focused knowledge SBA

A 54-year-old with physician-confirmed moderate OSA is weighing treatment options. Which of the following statements about advantages is most accurate?

A. Oral appliance treatment for OSA requires an objective physician-established diagnosis.

B. Heated humidification can relieve upper-airway dryness and nasal irritation during PAP treatment.

C. Initial adult pap titration pressure includes around four centimetres of water pressure.

D. CPAP usually reduces AHI and oxygen desaturation more than mandibular advancement.

E. Disadvantages includes lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%.

CORRECT ANSWER D

CPAP usually reduces AHI and oxygen desaturation more than mandibular advancement. Additional revision detail: more efficacious than a MAD for AHI, arousal index, ODI and oxygen saturation. The other options describe different mechanisms, investigations or findings rather than advantages. Consult Integrated notebook, page 62, §3.4 for the full context.

REFERENCE Integrated notebook, page 62, §3.4

QUESTION 0367 · §3.1 · Applied clinical SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. Which of the following statements about effective oral appliance (aadsm 2019) is most accurate?

A. A titratable oral appliance permits progressive mandibular advancement, unlike a fixed-position device.

B. Oral appliance includes a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep.

C. Effective oral appliance (aadsm 2019) includes advancement in increments of 1 mm or less; protrusive adjustment range ≥5 mm.

D. Pierre Robin described a monobloc appliance for managing glossoptosis.

E. An intraoral protraction device; the 1980s work was the catalyst for the modern field.

CORRECT ANSWER C

Effective oral appliance (aadsm 2019) includes advancement in increments of 1 mm or less; protrusive adjustment range ≥5 mm. Additional revision detail: advancement in increments of 1 mm or less; protrusive adjustment range ≥5 mm. The other options describe different mechanisms, investigations or findings rather than effective oral appliance (aadsm 2019). Consult Integrated notebook, page 61, §3.1 for the full context.

REFERENCE Integrated notebook, page 61, §3.1

QUESTION 0368 · §3.11 · Clinical interpretation SBA

A patient returning to the sleep clinic asks about the relative effects of different OSA treatments. The clinical team records the following finding: At most one-millimetre increments across a range of at least five millimetres. Which concept best explains this observation?

A. Effective OA

B. Effective OA definition

C. Purpose of EBD

D. Minimum protrusive distance for MAD

E. Effective mandibular appliance adjustment

CORRECT ANSWER E

This finding most directly describes Effective mandibular appliance adjustment: At most one-millimetre increments across a range of at least five millimetres. Distractor distinctions: A refers instead to increments ≤1 mm · range ≥5 mm · reversible · verifiable; B refers instead to increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting; C refers instead to provide appropriate care, reduce inappropriate care, increase effectiveness of care, by promoting best available evidence in; D refers instead to 6 mm (limited protrusion <6 mm is a contraindication).

REFERENCE Integrated notebook, page 62, §3.11

QUESTION 0369 · §3.4 · Applied clinical SBA

An adult with hypertension and confirmed OSA requests an explanation of available therapies. Which of the following statements about advantages is most accurate?

A. Initial adult pap titration pressure includes around four centimetres of water pressure.

B. Disadvantages includes lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%.

C. Heated humidification can relieve upper-airway dryness and nasal irritation during PAP treatment.

D. CPAP usually reduces AHI and oxygen desaturation more than mandibular advancement.

E. Oral appliance treatment for OSA requires an objective physician-established diagnosis.

CORRECT ANSWER D

CPAP usually reduces AHI and oxygen desaturation more than mandibular advancement. Additional revision detail: more efficacious than a MAD for AHI, arousal index, ODI and oxygen saturation. The other options describe different mechanisms, investigations or findings rather than advantages. Consult Integrated notebook, page 62, §3.4 for the full context.

REFERENCE Integrated notebook, page 62, §3.4

QUESTION 0370 · §3.11 · Applied clinical SBA

A patient who finds continuous positive airway pressure difficult to tolerate asks about alternatives. Which of the following statements about effective mandibular appliance adjustment is most accurate?

A. CPAP produces a larger average AHI reduction than oral appliance therapy in indirect comparisons.

B. A 10% weight gain is associated with an approximately 32% rise in AHI.

C. The device should provide adjustment increments no larger than 1 mm across at least 5 mm of travel.

D. An effective titratable appliance permits reversible adjustments of 1 mm or less over a range of at least 5 mm.

E. Minimum protrusive distance for MAD: 6 mm (limited protrusion <6 mm is a contraindication).

CORRECT ANSWER C

The device should provide adjustment increments no larger than 1 mm across at least 5 mm of travel. Additional revision detail: At most one-millimetre increments across a range of at least five millimetres. The other options describe different mechanisms, investigations or findings rather than effective mandibular appliance adjustment. Consult Integrated notebook, page 62, §3.11 for the full context.

REFERENCE Integrated notebook, page 62, §3.11

04 History, dental examination, imaging and patient selection

QUESTION 0371 · §4.4 · Clinical decision-making SBA

A patient with OSA has resistant hypertension and type 2 diabetes. Which interpretation is most appropriate?

A. They establish the AHI without testing

B. They are unrelated to OSA burden

C. These are recognised comorbidities relevant to risk and medical coordination

D. They prove that oral appliance treatment is contraindicated

E. They remove the need to assess cardiovascular risk

CORRECT ANSWER C

OSA is associated with cardiovascular and metabolic disease. Documentation and communication with the medical team matter.

REFERENCE Integrated notebook, Topic 4, §4.4

QUESTION 0372 · §4.7 · Clinical decision-making SBA

A patient with significant tooth wear reports frequent grinding, snoring and witnessed respiratory pauses. The patient asks for an occlusal splint. What should the dentist do first?

A. Presume all bruxism is caused by OSA

B. Fit a flat-plane splint without evaluating sleep symptoms

C. Screen for possible OSA and arrange appropriate medical referral if indicated

D. Diagnose OSA from tooth wear alone

E. Perform irreversible equilibration to stop the snoring

CORRECT ANSWER C

Bruxism and OSA can coexist, but neither proves the other. Snoring and witnessed pauses require appropriate OSA screening and referral before a splint is chosen. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 4, §4.7

QUESTION 0373 · §4.1 · Clinical decision-making SBA

An older patient has six heavily restored teeth in each arch, mobile posterior teeth and untreated periodontitis. Which assessment is most important before designing a MAD?

A. Tooth colour matching

B. Retention and periodontal/abutment prognosis before appliance selection

C. Choice of a laboratory based solely on price

D. Protruding to the end of range immediately

E. Estimation of OSA severity from the number of restorations

CORRECT ANSWER B

A MAD needs stable anchorage, adequate retention and oral disease control. Mobile or compromised teeth may change device selection or delay treatment. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 4, §4.1

QUESTION 0374 · §4.6 · Clinical decision-making SBA

A patient asks whether the dentist alone can confirm that a new MAD has eliminated their severe OSA. What is the best team-based reply?

A. A successful seating check confirms medical efficacy

B. The dentist monitors oral therapy and requests physician-directed objective verification

C. A normal daytime blood pressure replaces a sleep study

D. A reduction in snoring is sufficient to issue a cure certificate

E. Routine CBCT confirms treatment efficacy

CORRECT ANSWER B

A dentist manages dental treatment and adverse effects, while the physician remains responsible for diagnosis and objective efficacy confirmation in the shared-care pathway described. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 4, §4.6

QUESTION 0375 · §4.8 · Clinical decision-making SBA

A patient with controlled, mild TMD and physician-confirmed OSA is being considered for a custom MAD. What is the most appropriate conclusion?

A. Any past TMD is an absolute contraindication

B. TMD requires immediate TMJ surgery before OAT

C. TMD status is irrelevant to appliance selection

D. Assess current pain, function and joint symptoms; TMD is a relative consideration

E. Advance immediately to maximum protrusion to prevent pain

CORRECT ANSWER D

TMD is generally a relative rather than absolute contraindication. Active or severe symptoms should be assessed and managed, and advancement should be individualised and monitored. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 4, §4.8

QUESTION 0376 · §4.1 · Clinical decision-making SBA

A patient requests a mouthguard for severe tooth wear and also reports gasping during sleep. What should be addressed before choosing the splint?

A. Prescribe a flat-plane splint without considering breathing

B. Advise that occlusal protection cures OSA

C. Assume all grinding is due to OSA

D. Screen for OSA and refer for physician evaluation when indicated

E. Diagnose sleep apnoea from tooth wear alone

CORRECT ANSWER D

Sleep bruxism and OSA can coexist but causation is unproven; some stabilisation splints may worsen airway obstruction, so breathing symptoms require assessment.

REFERENCE Integrated notebook, Topic 4, §4.1

QUESTION 0377 · §4.2 · Applied clinical SBA

A patient with facial pain and unrefreshing sleep presents for multidisciplinary assessment. Which of the following statements about obesity is most accurate?

A. Drugs and medications includes anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function.

B. Smoking contributes to upper-airway inflammation and may increase sleep-disordered breathing risk.

C. Rhinosinusitis, allergic rhinitis and obstructive lung disease may complicate OSA treatment.

D. Alcohol can impair pharyngeal muscle tone and worsen obstruction, including during appliance use.

E. Obesity increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure.

CORRECT ANSWER E

Obesity increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure. Additional revision detail: increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure; requires tailored perioperative management. The other options describe different mechanisms, investigations or findings rather than obesity. Consult Integrated notebook, page 87, §4.2 for the full context.

REFERENCE Integrated notebook, page 87, §4.2

QUESTION 0378 · §4.3 · Focused knowledge SBA

A patient with tooth wear, jaw clicking and disruptive snoring attends an initial dental assessment. Which of the following statements about the relationship appears mutual rather than cause-and-effect is most accurate?

A. OSA and other sleep disorders may coexist or be associated; shared risk factors and sleep fragmentation prevent a simple causal interpretation.

B. SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia.

C. Sleep-disordered breathing and limb-movement disorders may coexist and compound sleep fragmentation.

D. SDB can exacerbate daytime dysfunction in people with insufficient sleep or shift-work disorder.

E. Coexisting sleep disorders can compound daytime sleepiness and complicate clinical diagnosis.

CORRECT ANSWER A

OSA and other sleep disorders may coexist or be associated; shared risk factors and sleep fragmentation prevent a simple causal interpretation. The other options describe different mechanisms, investigations or findings rather than the relationship appears mutual rather than cause-and-effect. Consult Integrated notebook, page 95, §4.3 for the full context.

REFERENCE Integrated notebook, page 95, §4.3

QUESTION 0379 · §4.1 · Clinical decision-making SBA

Before a MAD is delivered, several posterior teeth show mobility and untreated periodontitis. What is the priority?

A. Use no baseline dental records

B. Treat periodontal disease and assess whether retention is safe

C. Measure only neck circumference

D. Ignore mobility once PSG is positive

E. Advance to maximum because loose teeth increase comfort

CORRECT ANSWER B

A device loads supporting teeth: periodontal health, mobility and retention are central suitability criteria.

REFERENCE Integrated notebook, Topic 4, §4.1

QUESTION 0380 · §4.3 · Applied clinical SBA

A 59-year-old with dental restorations and OSA asks about eligibility for mandibular advancement. Which of the following statements about the relationship appears mutual rather than cause-and-effect is most accurate?

A. Coexisting sleep disorders can compound daytime sleepiness and complicate clinical diagnosis.

B. OSA and other sleep disorders may coexist or be associated; shared risk factors and sleep fragmentation prevent a simple causal interpretation.

C. SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia.

D. Sleep-disordered breathing and limb-movement disorders may coexist and compound sleep fragmentation.

E. SDB can exacerbate daytime dysfunction in people with insufficient sleep or shift-work disorder.

CORRECT ANSWER B

OSA and other sleep disorders may coexist or be associated; shared risk factors and sleep fragmentation prevent a simple causal interpretation. The other options describe different mechanisms, investigations or findings rather than the relationship appears mutual rather than cause-and-effect. Consult Integrated notebook, page 95, §4.3 for the full context.

REFERENCE Integrated notebook, page 95, §4.3

QUESTION 0381 · §4.4 · Clinical interpretation SBA

A 46-year-old with OSA is referred for dental suitability assessment before oral appliance therapy. The clinical team records the following finding: Higher rates particularly in women and post-menopausal populations. Neurocognitive decline — executive function, attention, memory — in adults and children, even at mild disease levels. Which concept best explains this observation?

A. Depression, anxiety, neurocognitive decline

B. Metabolic syndrome and diabetes

C. Symptoms of OSA and depression overlap

D. Insufficient sleep and shift work disorder

E. CPAP's effect on depression is not conclusive

CORRECT ANSWER A

This finding most directly describes Depression, anxiety, neurocognitive decline: Higher rates particularly in women and post-menopausal populations. Neurocognitive decline — executive function, attention, memory — in adults and children, even at mild disease levels. Distractor distinctions: B refers instead to Hypertension, diabetes and metabolic syndrome are independently associated with moderate– severe SDB, and coexistence with ob; C refers instead to tiredness, fatigue, sleepiness, poor motivation and poor concentration — complicating the diagnosis and treatment of each; D refers instead to SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are; E refers instead to evidence is limited; some studies report benefit for both sleepiness and depression, others no effect. 4.4 Medical comorbidit.

REFERENCE Integrated notebook, page 88, §4.4

QUESTION 0382 · §4.1 · Clinical decision-making SBA

A clinician proposes CBCT solely to calculate a static airway volume before a MAD. Which response best matches the notes?

A. Cephalometry replaces physician diagnosis

B. Imaging has no radiation considerations

C. CBCT is mandatory for every OSA appliance

D. A static airway slice proves disease severity

E. Routine CBCT is not justified by airway-volume measurement alone

CORRECT ANSWER E

Upper-airway obstruction is dynamic; static CBCT airway measurements are limited and radiation must be justified by clinical need.

REFERENCE Integrated notebook, Topic 4, §4.1

QUESTION 0383 · §4.4 · Applied clinical SBA

A dentist is reviewing the medical and dental history of a new oral appliance candidate. Which of the following statements about depression, anxiety, neurocognitive decline is most accurate?

A. SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia.

B. Hypertension, diabetes and metabolic syndrome are independently associated with moderate– severe SDB.

C. Dental radiographs should be justified by clinical indication rather than ordered routinely without purpose.

D. SDB can exacerbate daytime dysfunction in people with insufficient sleep or shift-work disorder.

E. Depression, anxiety, neurocognitive decline includes higher rates particularly in women and post-menopausal populations.

CORRECT ANSWER E

Depression, anxiety, neurocognitive decline includes higher rates particularly in women and post-menopausal populations. Additional revision detail: Higher rates particularly in women and post-menopausal populations. Neurocognitive decline — executive function, attention, memory — in adults and children, even at mild disease levels. The other options describe different mechanisms, investigations or findings rather than depression, anxiety, neurocognitive decline. Consult Integrated notebook, page 88, §4.4 for the full context.

REFERENCE Integrated notebook, page 88, §4.4

QUESTION 0384 · §4.5 · Clinical interpretation SBA

A patient with facial pain and unrefreshing sleep presents for multidisciplinary assessment. The clinical team records the following finding: Explain risks, benefits, alternatives including no treatment, and expected objective verification. Which concept best explains this observation?

A. Informed dental treatment choice

B. Medical–dental history

C. Successful treatment outcome

D. Patient-specific goals

E. Metabolic syndrome and diabetes

CORRECT ANSWER A

This finding most directly describes Informed dental treatment choice: Explain risks, benefits, alternatives including no treatment, and expected objective verification. Distractor distinctions: B refers instead to age, weight, height; neck and belt circumference as indicated; C refers instead to AHI/RDI/REI 50% reduction; D refers instead to Align the outcome with the patient and the physician, including symptom and objective disease endpoints; E refers instead to Hypertension, diabetes and metabolic syndrome are independently associated with moderate– severe SDB, and coexistence with ob.

REFERENCE Integrated notebook, page 89, §4.5

QUESTION 0385 · §4.6 · Clinical decision-making SBA

A patient presents with a diagnostic PSG but no physician treatment communication. What should the dentist obtain?

A. Medical report, treatment recommendation and a shared-care plan

B. A verbal diagnosis from the patient alone

C. Only a panoramic radiograph

D. An appliance prescription generated by the receptionist

E. Permission to bypass efficacy testing

CORRECT ANSWER A

Shared care requires documented physician diagnosis, treatment context, subsequent objective verification and two-way communication.

REFERENCE Integrated notebook, Topic 4, §4.6

QUESTION 0386 · §4.5 · Applied clinical SBA

A patient with tooth wear, jaw clicking and disruptive snoring attends an initial dental assessment. Which of the following statements about informed dental treatment choice is most accurate?

A. Patient-specific goals includes align the outcome with the patient and the physician, including symptom and objective disease endpoints.

B. The clinician should explain risks, benefits, alternatives including no treatment, and expected objective verification.

C. Rhinosinusitis, allergic rhinitis and obstructive lung disease may complicate OSA treatment.

D. Fibromyalgia (acr 1990) includes pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%.

E. Shared clinical features includes both commoner supine; both involve oropharyngeal and masticatory muscle activation.

CORRECT ANSWER B

The clinician should explain risks, benefits, alternatives including no treatment, and expected objective verification. Additional revision detail: Explain risks, benefits, alternatives including no treatment, and expected objective verification. The other options describe different mechanisms, investigations or findings rather than informed dental treatment choice. Consult Integrated notebook, page 89, §4.5 for the full context.

REFERENCE Integrated notebook, page 89, §4.5

QUESTION 0387 · §4.6 · Clinical interpretation SBA

A 59-year-old with dental restorations and OSA asks about eligibility for mandibular advancement. The clinical team records the following finding: Adult mild 5–14 · moderate 15–30 · severe >30. Which concept best explains this observation?

A. Shared

B. Severity and outcome in the shared-care setting

C. The relationship appears mutual rather than cause-and-effect

D. Borderline cases

E. Portable monitor levels

CORRECT ANSWER B

This finding most directly describes Severity and outcome in the shared-care setting: Adult mild 5–14 · moderate 15–30 · severe >30. Distractor distinctions: A refers instead to written professional communication between GP, sleep physician and dentist; C refers instead to The pathophysiological relationship is not fully understood: it may be coincidental, or due to sleep fragmentation or repeate; D refers instead to Primary snoring without sleepiness → still refer to exclude OSA before providing an appliance; E refers instead to Level 3 = ≥4 channels including airflow (≥2 channels of respiratory movement, or movement and airflow), heart rate/ECG, oxyge.

REFERENCE Integrated notebook, page 89, §4.6

QUESTION 0388 · §4.7 · Clinical decision-making SBA

A patient with both sleep bruxism and OSA asks whether one causes the other. What is the best answer?

A. Both are defined by the same PSG event

B. They are associated, but a causal relationship has not been established

C. The conditions cannot coexist

D. Bruxism invariably causes OSA

E. OSA invariably causes all bruxism

CORRECT ANSWER B

The notebook stresses distinguishing association from causation; arousal-related mechanisms are proposed but not proof of directionality.

REFERENCE Integrated notebook, Topic 4, §4.7

QUESTION 0389 · §4.6 · Applied clinical SBA

A 46-year-old with OSA is referred for dental suitability assessment before oral appliance therapy. Which of the following statements about severity and outcome in the shared-care setting is most accurate?

A. Adult OSA is conventionally classified as mild at AHI 5–14.9/h, moderate at 15–29.9/h and severe at 30/h or more.

B. Shared includes written professional communication between GP, sleep physician and dentist.

C. A type 3 sleep test measures several respiratory channels without EEG-based sleep staging.

D. Dental changes, discomfort — may occur.

E. Patients with snoring still require physician assessment to exclude OSA before oral appliance treatment.

CORRECT ANSWER A

Adult OSA is conventionally classified as mild at AHI 5–14.9/h, moderate at 15–29.9/h and severe at 30/h or more. Additional revision detail: Adult mild 5–14 · moderate 15–30 · severe >30. The other options describe different mechanisms, investigations or findings rather than severity and outcome in the shared-care setting. Consult Integrated notebook, page 89, §4.6 for the full context.

REFERENCE Integrated notebook, page 89, §4.6

QUESTION 0390 · §4.7 · Clinical interpretation SBA

A dentist is reviewing the medical and dental history of a new oral appliance candidate. The clinical team records the following finding: 6–91%, depending on assessment method. Which concept best explains this observation?

A. Sleep bruxism

B. Assessment grading

C. Wide reported range

D. Self-reported sleep bruxism

E. Shared clinical features

CORRECT ANSWER C

This finding most directly describes Wide reported range: 6–91%, depending on assessment method. Distractor distinctions: A refers instead to a masticatory muscle activity during sleep, characterised as rhythmic (phasic — grinding) or non-rhythmic (tonic — clenching); B refers instead to possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental ass; D refers instead to ~8% of the adult population; E refers instead to both commoner supine; both involve oropharyngeal and masticatory muscle activation.

REFERENCE Integrated notebook, page 99, §4.7

QUESTION 0391 · §4.8 · Clinical decision-making SBA

A patient with TMD is being considered for oral appliance therapy. Which assessment is essential?

A. Assume successful AHI reduction eliminates TMD

B. Record baseline pain, joint function, movements and clinical findings

C. Avoid asking about pain

D. Skip occlusal examination

E. Treat all previous disc displacements as absolute exclusions

CORRECT ANSWER B

TMD is not invariably an absolute contraindication, but active symptoms and baseline function influence risk, counselling and monitoring.

REFERENCE Integrated notebook, Topic 4, §4.8

QUESTION 0392 · §4.7 · Applied clinical SBA

A patient with facial pain and unrefreshing sleep presents for multidisciplinary assessment. Which of the following statements about wide reported range is most accurate?

A. Self-reported sleep bruxism: ~8% of the adult population.

B. SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences.

C. Sleep bruxism may be classified as possible, probable or definite according to assessment methods.

D. Shared clinical features includes both commoner supine; both involve oropharyngeal and masticatory muscle activation.

E. Wide reported range: 6–91%, depending on assessment method.

CORRECT ANSWER E

Wide reported range: 6–91%, depending on assessment method. Additional revision detail: 6–91%, depending on assessment method. The other options describe different mechanisms, investigations or findings rather than wide reported range. Consult Integrated notebook, page 99, §4.7 for the full context.

REFERENCE Integrated notebook, page 99, §4.7

QUESTION 0393 · §4.8 · Clinical interpretation SBA

A patient with tooth wear, jaw clicking and disruptive snoring attends an initial dental assessment. The clinical team records the following finding: pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%. Which concept best explains this observation?

A. Successful outcome

B. Successful treatment outcome

C. Implications

D. SB prevalence

E. Fibromyalgia (ACR 1990)

CORRECT ANSWER E

This finding most directly describes Fibromyalgia (ACR 1990): pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%. Distractor distinctions: A refers instead to AHI/RDI/REI 50% reduction; B refers instead to AHI/RDI/REI 50% reduction; C refers instead to chronic pain management must address sleep simultaneously, usually multidisciplinary; psychological input for the mood and an; D refers instead to reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients.

REFERENCE Integrated notebook, page 91, §4.8

QUESTION 0394 · §4.2 · Clinical decision-making SBA

A sleep patient takes sedatives, drinks alcohol at bedtime and has a BMI of 36. Which medical-history interpretation is best?

A. Alcohol reduces pharyngeal collapse

B. BMI does not matter after fitting

C. These factors guarantee oral appliance success

D. Multiple modifiable factors may increase airway collapsibility and reduce treatment response

E. The dentist should independently stop prescribed medicines

CORRECT ANSWER D

These are relevant risk modifiers to document and address in coordination with the prescriber and sleep physician.

REFERENCE Integrated notebook, Topic 4, §4.2

QUESTION 0395 · §4.8 · Applied clinical SBA

A 59-year-old with dental restorations and OSA asks about eligibility for mandibular advancement. Which of the following statements about fibromyalgia (acr 1990) is most accurate?

A. Mechanisms includes dysregulated serotonin, noradrenaline and dopamine (regulating both pain and sleep).

B. Published estimates of sleep bruxism prevalence vary widely because diagnostic definitions differ.

C. Fibromyalgia (acr 1990) includes pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%.

D. Hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation.

E. Successful outcome includes aHI/RDI/REI 50% reduction.

CORRECT ANSWER C

Fibromyalgia (acr 1990) includes pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%. Additional revision detail: pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%. The other options describe different mechanisms, investigations or findings rather than fibromyalgia (acr 1990). Consult Integrated notebook, page 91, §4.8 for the full context.

REFERENCE Integrated notebook, page 91, §4.8

QUESTION 0396 · §4.1 · Clinical interpretation SBA

A dentist is reviewing the medical and dental history of a new oral appliance candidate. The clinical team records the following finding: attachment loss, mobility, prognosis of the teeth that will bear the appliance. Which concept best explains this observation?

A. Medical–dental history

B. Periodontal evaluation

C. Teeth and restorations

D. Soft tissue/intraoral

E. Reason for attendance

CORRECT ANSWER B

This finding most directly describes Periodontal evaluation: attachment loss, mobility, prognosis of the teeth that will bear the appliance. Distractor distinctions: A refers instead to age, weight, height; neck and belt circumference as indicated; C refers instead to number of functional teeth, mobility, spacing, curve of Spee, heavily restored teeth, anticipated restorative needs; D refers instead to modified Mallampati, tonsil grade, tongue size and scalloping, palatal vault, posterior pharyngeal crowding, signs of mouth b; E refers instead to snoring, suspected or diagnosed OSA, sleep bruxism, or orofacial pain related to sleep disorders. Ask about sleep symptoms, p.

REFERENCE Integrated notebook, page 86, §4.1

QUESTION 0397 · §4.3 · Clinical decision-making SBA

A person treated for OSA continues to have unrefreshing sleep and an urge to move the legs at night. What should be considered?

A. Leg movements diagnose OSA

B. ESS cannot be used in follow-up

C. All sleep disorders are corrected by mandibular advancement

D. Coexisting restless legs syndrome needs its own assessment

E. Persisting symptoms prove appliance fracture

CORRECT ANSWER D

SDB may coexist with RLS/PLMD and other sleep disorders; symptom persistence should not automatically trigger further protrusion.

REFERENCE Integrated notebook, Topic 4, §4.3

QUESTION 0398 · §4.1 · Applied clinical SBA

A patient with facial pain and unrefreshing sleep presents for multidisciplinary assessment. Which of the following statements about periodontal evaluation is most accurate?

A. A screening assessment should document age, weight, height and relevant circumferences.

B. Occlusion — examine and record the existing condition; this is your medicolegal baseline.

C. Relevant findings include CPAP, surgery, positional therapy.

D. Attachment loss, mobility, prognosis of the teeth that will bear the appliance.

E. Dental suitability assessment considers functional teeth, mobility, periodontal status and restorative needs.

CORRECT ANSWER D

Attachment loss, mobility, prognosis of the teeth that will bear the appliance. The other options describe different mechanisms, investigations or findings rather than periodontal evaluation. Consult Integrated notebook, page 86, §4.1 for the full context.

REFERENCE Integrated notebook, page 86, §4.1

QUESTION 0399 · §4.2 · Clinical interpretation SBA

A patient with tooth wear, jaw clicking and disruptive snoring attends an initial dental assessment. The clinical team records the following finding: anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function, increasing obstruction and respiratory failure risk, particularly perioperatively. Which concept best explains this observation?

A. Smoking

B. Drugs and medications

C. Alcohol

D. Obesity

E. Other sleep disorders and comorbidities

CORRECT ANSWER B

This finding most directly describes Drugs and medications: anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function, increasing obstruction and respiratory failure risk, particularly perioperatively. Distractor distinctions: A refers instead to chronic inflammation, upper airway pathology, susceptibility to infection and cancer, higher morbidity and mortality; C refers instead to impairs airway muscle tone and increases obstruction risk, especially with sedatives; even moderate intake may cause transien; D refers instead to increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure; requires ta; E refers instead to chronic rhinosinusitis, allergic rhinitis, asthma or COPD worsen airway function and complicate management.

REFERENCE Integrated notebook, page 87, §4.2

QUESTION 0400 · §4.5 · Clinical decision-making SBA

A patient values minimal dental side effects over complete numerical normalisation. What is the most appropriate planning approach?

A. Promise AHI zero in every case

B. Ignore the patient's goals

C. Shared goal setting with clear discussion of risks and physician-agreed success criteria

D. Define success exclusively by loudness of snoring

E. Avoid describing alternatives

CORRECT ANSWER C

Treatment aims should reflect realistic efficacy, patient values, tolerability and medical verification of remaining disease.

REFERENCE Integrated notebook, Topic 4, §4.5

QUESTION 0401 · §4.2 · Applied clinical SBA

A 59-year-old with dental restorations and OSA asks about eligibility for mandibular advancement. Which of the following statements about drugs and medications is most accurate?

A. Rhinosinusitis, allergic rhinitis and obstructive lung disease may complicate OSA treatment.

B. Drugs and medications includes anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function.

C. Obesity increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure.

D. Alcohol can impair pharyngeal muscle tone and worsen obstruction, including during appliance use.

E. Smoking contributes to upper-airway inflammation and may increase sleep-disordered breathing risk.

CORRECT ANSWER B

Drugs and medications includes anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function. Additional revision detail: anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function, increasing obstruction and respiratory failure risk, particularly perioperatively. The other options describe different mechanisms, investigations or findings rather than drugs and medications. Consult Integrated notebook, page 87, §4.2 for the full context.

REFERENCE Integrated notebook, page 87, §4.2

QUESTION 0402 · §4.3 · Clinical interpretation SBA

A 46-year-old with OSA is referred for dental suitability assessment before oral appliance therapy. The clinical team records the following finding: May coexist, compounding sleepiness and cognitive impairment; symptom overlap makes clinical differentiation unreliable, so careful sleep studies are needed. Which concept best explains this observation?

A. Narcolepsy

B. Mood disorders

C. Symptoms of OSA and depression overlap

D. RLS and PLMD

E. Insomnia

CORRECT ANSWER A

This finding most directly describes Narcolepsy: May coexist, compounding sleepiness and cognitive impairment; symptom overlap makes clinical differentiation unreliable, so careful sleep studies are needed. Distractor distinctions: B refers instead to SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia, creating a cyc; C refers instead to tiredness, fatigue, sleepiness, poor motivation and poor concentration — complicating the diagnosis and treatment of each; D refers instead to Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute; E refers instead to COMISA. Highly prevalent in SDB; the overlap produces more severe fragmentation, increased sleepiness and poorer quality of l.

REFERENCE Integrated notebook, page 87, §4.3

QUESTION 0403 · §4.3 · Applied clinical SBA

A patient with tooth wear, jaw clicking and disruptive snoring attends an initial dental assessment. Which of the following statements about narcolepsy is most accurate?

A. The evidence for improved depression after CPAP is mixed and does not establish universal benefit.

B. SDB can exacerbate daytime dysfunction in people with insufficient sleep or shift-work disorder.

C. Coexisting sleep disorders can compound daytime sleepiness and complicate clinical diagnosis.

D. Sleep-disordered breathing and limb-movement disorders may coexist and compound sleep fragmentation.

E. OSA and depression share fatigue, impaired concentration and sleep-related symptoms.

CORRECT ANSWER C

Coexisting sleep disorders can compound daytime sleepiness and complicate clinical diagnosis. Additional revision detail: May coexist, compounding sleepiness and cognitive impairment; symptom overlap makes clinical differentiation unreliable, so careful sleep studies are needed. The other options describe different mechanisms, investigations or findings rather than narcolepsy. Consult Integrated notebook, page 87, §4.3 for the full context.

REFERENCE Integrated notebook, page 87, §4.3

QUESTION 0404 · §4.4 · Clinical interpretation SBA

A 59-year-old with dental restorations and OSA asks about eligibility for mandibular advancement. The clinical team records the following finding: Hypertension, diabetes and metabolic syndrome are independently associated with moderate– severe SDB, and coexistence with obesity further increases risk. Which concept best explains this observation?

A. Symptoms of OSA and depression overlap

B. Results vary, and some side effects

C. Depression, anxiety, neurocognitive decline

D. Other sleep disorders and comorbidities

E. Metabolic syndrome and diabetes

CORRECT ANSWER E

This finding most directly describes Metabolic syndrome and diabetes: Hypertension, diabetes and metabolic syndrome are independently associated with moderate– severe SDB, and coexistence with obesity further increases risk. Distractor distinctions: A refers instead to tiredness, fatigue, sleepiness, poor motivation and poor concentration — complicating the diagnosis and treatment of each; B refers instead to dental changes, discomfort — may occur; C refers instead to Higher rates particularly in women and post-menopausal populations. Neurocognitive decline — executive function, attention, m; D refers instead to chronic rhinosinusitis, allergic rhinitis, asthma or COPD worsen airway function and complicate management.

REFERENCE Integrated notebook, page 87, §4.4

QUESTION 0405 · §4.4 · Applied clinical SBA

A patient with facial pain and unrefreshing sleep presents for multidisciplinary assessment. Which of the following statements about metabolic syndrome and diabetes is most accurate?

A. Sleep bruxism may be classified as possible, probable or definite according to assessment methods.

B. Mechanisms includes dysregulated serotonin, noradrenaline and dopamine (regulating both pain and sleep).

C. Dental radiographs should be justified by clinical indication rather than ordered routinely without purpose.

D. Depression, anxiety, neurocognitive decline includes higher rates particularly in women and post-menopausal populations.

E. Hypertension, diabetes and metabolic syndrome are independently associated with moderate– severe SDB.

CORRECT ANSWER E

Hypertension, diabetes and metabolic syndrome are independently associated with moderate– severe SDB. Additional revision detail: Hypertension, diabetes and metabolic syndrome are independently associated with moderate– severe SDB, and coexistence with obesity further increases risk. The other options describe different mechanisms, investigations or findings rather than metabolic syndrome and diabetes. Consult Integrated notebook, page 87, §4.4 for the full context.

REFERENCE Integrated notebook, page 87, §4.4

QUESTION 0406 · §4.5 · Clinical interpretation SBA

A patient with tooth wear, jaw clicking and disruptive snoring attends an initial dental assessment. The clinical team records the following finding: Align the outcome with the patient and the physician, including symptom and objective disease endpoints. Which concept best explains this observation?

A. Medical–dental history

B. Informed dental treatment choice

C. Teeth and restorations

D. Patient-specific goals

E. Periodontal evaluation

CORRECT ANSWER D

This finding most directly describes Patient-specific goals: Align the outcome with the patient and the physician, including symptom and objective disease endpoints. Distractor distinctions: A refers instead to age, weight, height; neck and belt circumference as indicated; B refers instead to Explain risks, benefits, alternatives including no treatment, and expected objective verification; C refers instead to number of functional teeth, mobility, spacing, curve of Spee, heavily restored teeth, anticipated restorative needs; E refers instead to attachment loss, mobility, prognosis of the teeth that will bear the appliance.

REFERENCE Integrated notebook, page 89, §4.5

QUESTION 0407 · §4.5 · Applied clinical SBA

A dentist is reviewing the medical and dental history of a new oral appliance candidate. Which of the following statements about patient-specific goals is most accurate?

A. Occlusion — examine and record the existing condition; this is your medicolegal baseline.

B. Rhinosinusitis, allergic rhinitis and obstructive lung disease may complicate OSA treatment.

C. The clinician should explain risks, benefits, alternatives including no treatment, and expected objective verification.

D. Published estimates of sleep bruxism prevalence vary widely because diagnostic definitions differ.

E. Patient-specific goals includes align the outcome with the patient and the physician, including symptom and objective disease endpoints.

CORRECT ANSWER E

Patient-specific goals includes align the outcome with the patient and the physician, including symptom and objective disease endpoints. Additional revision detail: Align the outcome with the patient and the physician, including symptom and objective disease endpoints. The other options describe different mechanisms, investigations or findings rather than patient-specific goals. Consult Integrated notebook, page 89, §4.5 for the full context.

REFERENCE Integrated notebook, page 89, §4.5

QUESTION 0408 · §4.6 · Clinical interpretation SBA

A patient with facial pain and unrefreshing sleep presents for multidisciplinary assessment. The clinical team records the following finding: Level 3 = ≥4 channels including airflow (≥2 channels of respiratory movement, or movement and airflow), heart rate/ECG, oxygen saturation. Which concept best explains this observation?

A. Borderline cases

B. Shared

C. Fibromyalgia (ACR 1990)

D. Portable monitor levels

E. Severity and outcome in the shared-care setting

CORRECT ANSWER D

This finding most directly describes Portable monitor levels: Level 3 = ≥4 channels including airflow (≥2 channels of respiratory movement, or movement and airflow), heart rate/ECG, oxygen saturation. Distractor distinctions: A refers instead to Primary snoring without sleepiness → still refer to exclude OSA before providing an appliance; B refers instead to written professional communication between GP, sleep physician and dentist; C refers instead to pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%; E refers instead to Adult mild 5–14 · moderate 15–30 · severe >30.

REFERENCE Integrated notebook, page 88, §4.6

QUESTION 0409 · §4.6 · Applied clinical SBA

A 46-year-old with OSA is referred for dental suitability assessment before oral appliance therapy. Which of the following statements about portable monitor levels is most accurate?

A. A type 3 sleep test measures several respiratory channels without EEG-based sleep staging.

B. Adult OSA is conventionally classified as mild at AHI 5–14.9/h, moderate at 15–29.9/h and severe at 30/h or more.

C. Patients with snoring still require physician assessment to exclude OSA before oral appliance treatment.

D. Shared includes written professional communication between GP, sleep physician and dentist.

E. Assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated.

CORRECT ANSWER A

A type 3 sleep test measures several respiratory channels without EEG-based sleep staging. Additional revision detail: Level 3 = ≥4 channels including airflow (≥2 channels of respiratory movement, or movement and airflow), heart rate/ECG, oxygen saturation. The other options describe different mechanisms, investigations or findings rather than portable monitor levels. Consult Integrated notebook, page 88, §4.6 for the full context.

REFERENCE Integrated notebook, page 88, §4.6

QUESTION 0410 · §4.7 · Clinical interpretation SBA

A dentist is reviewing the medical and dental history of a new oral appliance candidate. The clinical team records the following finding: SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences. Which concept best explains this observation?

A. SB + TMD

B. Wide reported range

C. Assessment grading

D. Sleep bruxism

E. Corollary

CORRECT ANSWER E

This finding most directly describes Corollary: SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences. Distractor distinctions: A refers instead to assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is; B refers instead to 6–91%, depending on assessment method; C refers instead to possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental ass; D refers instead to a masticatory muscle activity during sleep, characterised as rhythmic (phasic — grinding) or non-rhythmic (tonic — clenching).

REFERENCE Integrated notebook, page 89, §4.7

QUESTION 0411 · §4.7 · Applied clinical SBA

A 59-year-old with dental restorations and OSA asks about eligibility for mandibular advancement. Which of the following statements about corollary is most accurate?

A. SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences.

B. Sleep bruxism may be classified as possible, probable or definite according to assessment methods.

C. Assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated.

D. Shared clinical features includes both commoner supine; both involve oropharyngeal and masticatory muscle activation.

E. The protective-bruxism hypothesis proposes that jaw activity can follow obstruction and transiently restore airway tone.

CORRECT ANSWER A

SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences. The other options describe different mechanisms, investigations or findings rather than corollary. Consult Integrated notebook, page 89, §4.7 for the full context.

REFERENCE Integrated notebook, page 89, §4.7

QUESTION 0412 · §4.1 · Clinical interpretation SBA

A 59-year-old with dental restorations and OSA asks about eligibility for mandibular advancement. The clinical team records the following finding: Occlusion — examine and record the existing condition; this is your medicolegal baseline. Which concept best explains this observation?

A. Periodontal evaluation

B. Medical–dental history

C. Results vary, and some side effects

D. Teeth and restorations

E. Bruxism, orofacial pain, headaches

CORRECT ANSWER E

This finding most directly describes Bruxism, orofacial pain, headaches: Occlusion — examine and record the existing condition; this is your medicolegal baseline. Distractor distinctions: A refers instead to attachment loss, mobility, prognosis of the teeth that will bear the appliance; B refers instead to age, weight, height; neck and belt circumference as indicated; C refers instead to dental changes, discomfort — may occur; D refers instead to number of functional teeth, mobility, spacing, curve of Spee, heavily restored teeth, anticipated restorative needs.

REFERENCE Integrated notebook, page 86, §4.1

QUESTION 0413 · §4.1 · Applied clinical SBA

A patient with facial pain and unrefreshing sleep presents for multidisciplinary assessment. Which of the following statements about bruxism, orofacial pain, headaches is most accurate?

A. Attachment loss, mobility, prognosis of the teeth that will bear the appliance.

B. Relevant findings include CPAP, surgery, positional therapy.

C. Dental suitability assessment considers functional teeth, mobility, periodontal status and restorative needs.

D. Occlusion — examine and record the existing condition; this is your medicolegal baseline.

E. A screening assessment should document age, weight, height and relevant circumferences.

CORRECT ANSWER D

Occlusion — examine and record the existing condition; this is your medicolegal baseline. The other options describe different mechanisms, investigations or findings rather than bruxism, orofacial pain, headaches. Consult Integrated notebook, page 86, §4.1 for the full context.

REFERENCE Integrated notebook, page 86, §4.1

QUESTION 0414 · §4.2 · Focused knowledge SBA

A patient with tooth wear, jaw clicking and disruptive snoring attends an initial dental assessment. Which of the following statements about smoking is most accurate?

A. Obesity increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure.

B. Alcohol can impair pharyngeal muscle tone and worsen obstruction, including during appliance use.

C. Rhinosinusitis, allergic rhinitis and obstructive lung disease may complicate OSA treatment.

D. Drugs and medications includes anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function.

E. Smoking contributes to upper-airway inflammation and may increase sleep-disordered breathing risk.

CORRECT ANSWER E

Smoking contributes to upper-airway inflammation and may increase sleep-disordered breathing risk. Additional revision detail: chronic inflammation, upper airway pathology, susceptibility to infection and cancer, higher morbidity and mortality. The other options describe different mechanisms, investigations or findings rather than smoking. Consult Integrated notebook, page 87, §4.2 for the full context.

REFERENCE Integrated notebook, page 87, §4.2

QUESTION 0415 · §4.2 · Applied clinical SBA

A dentist is reviewing the medical and dental history of a new oral appliance candidate. Which of the following statements about smoking is most accurate?

A. Drugs and medications includes anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function.

B. Obesity increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure.

C. Rhinosinusitis, allergic rhinitis and obstructive lung disease may complicate OSA treatment.

D. Smoking contributes to upper-airway inflammation and may increase sleep-disordered breathing risk.

E. Alcohol can impair pharyngeal muscle tone and worsen obstruction, including during appliance use.

CORRECT ANSWER D

Smoking contributes to upper-airway inflammation and may increase sleep-disordered breathing risk. Additional revision detail: chronic inflammation, upper airway pathology, susceptibility to infection and cancer, higher morbidity and mortality. The other options describe different mechanisms, investigations or findings rather than smoking. Consult Integrated notebook, page 87, §4.2 for the full context.

REFERENCE Integrated notebook, page 87, §4.2

QUESTION 0416 · §4.3 · Clinical interpretation SBA

A patient with facial pain and unrefreshing sleep presents for multidisciplinary assessment. The clinical team records the following finding: tiredness, fatigue, sleepiness, poor motivation and poor concentration — complicating the diagnosis and treatment of each. Which concept best explains this observation?

A. Symptoms of OSA and depression overlap

B. RLS and PLMD

C. CPAP's effect on depression is not conclusive

D. The relationship appears mutual rather than cause-and-effect

E. Insufficient sleep and shift work disorder

CORRECT ANSWER A

This finding most directly describes Symptoms of OSA and depression overlap: tiredness, fatigue, sleepiness, poor motivation and poor concentration — complicating the diagnosis and treatment of each. Distractor distinctions: B refers instead to Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute; C refers instead to evidence is limited; some studies report benefit for both sleepiness and depression, others no effect. 4.4 Medical comorbidit; D refers instead to The pathophysiological relationship is not fully understood: it may be coincidental, or due to sleep fragmentation or repeate; E refers instead to SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are.

REFERENCE Integrated notebook, page 95, §4.3

QUESTION 0417 · §4.3 · Applied clinical SBA

A 46-year-old with OSA is referred for dental suitability assessment before oral appliance therapy. Which of the following statements about symptoms of OSA and depression overlap is most accurate?

A. Sleep-disordered breathing and limb-movement disorders may coexist and compound sleep fragmentation.

B. Coexisting sleep disorders can compound daytime sleepiness and complicate clinical diagnosis.

C. The evidence for improved depression after CPAP is mixed and does not establish universal benefit.

D. Comorbid insomnia and OSA can compound sleep fragmentation, sleepiness and reduced quality of life.

E. OSA and depression share fatigue, impaired concentration and sleep-related symptoms.

CORRECT ANSWER E

OSA and depression share fatigue, impaired concentration and sleep-related symptoms. Additional revision detail: tiredness, fatigue, sleepiness, poor motivation and poor concentration — complicating the diagnosis and treatment of each. The other options describe different mechanisms, investigations or findings rather than symptoms of osa and depression overlap. Consult Integrated notebook, page 95, §4.3 for the full context.

REFERENCE Integrated notebook, page 95, §4.3

QUESTION 0418 · §4.6 · Focused knowledge SBA

A patient with facial pain and unrefreshing sleep presents for multidisciplinary assessment. Which of the following statements about shared is most accurate?

A. Patients with snoring still require physician assessment to exclude OSA before oral appliance treatment.

B. Shared includes written professional communication between GP, sleep physician and dentist.

C. Adult OSA is conventionally classified as mild at AHI 5–14.9/h, moderate at 15–29.9/h and severe at 30/h or more.

D. Attachment loss, mobility, prognosis of the teeth that will bear the appliance.

E. A type 3 sleep test measures several respiratory channels without EEG-based sleep staging.

CORRECT ANSWER B

Shared includes written professional communication between GP, sleep physician and dentist. Additional revision detail: written professional communication between GP, sleep physician and dentist. The other options describe different mechanisms, investigations or findings rather than shared. Consult Integrated notebook, page 88, §4.6 for the full context.

REFERENCE Integrated notebook, page 88, §4.6

QUESTION 0419 · §4.6 · Applied clinical SBA

A 46-year-old with OSA is referred for dental suitability assessment before oral appliance therapy. Which of the following statements about shared is most accurate?

A. Attachment loss, mobility, prognosis of the teeth that will bear the appliance.

B. Adult OSA is conventionally classified as mild at AHI 5–14.9/h, moderate at 15–29.9/h and severe at 30/h or more.

C. A type 3 sleep test measures several respiratory channels without EEG-based sleep staging.

D. Shared includes written professional communication between GP, sleep physician and dentist.

E. Patients with snoring still require physician assessment to exclude OSA before oral appliance treatment.

CORRECT ANSWER D

Shared includes written professional communication between GP, sleep physician and dentist. Additional revision detail: written professional communication between GP, sleep physician and dentist. The other options describe different mechanisms, investigations or findings rather than shared. Consult Integrated notebook, page 88, §4.6 for the full context.

REFERENCE Integrated notebook, page 88, §4.6

QUESTION 0420 · §4.7 · Clinical interpretation SBA

A dentist is reviewing the medical and dental history of a new oral appliance candidate. The clinical team records the following finding: a masticatory muscle activity during sleep, characterised as rhythmic (phasic — grinding) or non-rhythmic (tonic — clenching), and not a movement disorder or a sleep disorder in otherwise healthy individuals. Which concept best explains this observation?

A. Assessment grading

B. Self-reported sleep bruxism

C. Corollary

D. SB + TMD

E. Sleep bruxism

CORRECT ANSWER E

This finding most directly describes Sleep bruxism: a masticatory muscle activity during sleep, characterised as rhythmic (phasic — grinding) or non-rhythmic (tonic — clenching), and not a movement disorder or a sleep disorder in otherwise healthy individuals. Distractor distinctions: A refers instead to possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental ass; B refers instead to ~8% of the adult population; C refers instead to SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences; D refers instead to assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is.

REFERENCE Integrated notebook, page 89, §4.7

QUESTION 0421 · §4.7 · Applied clinical SBA

A 59-year-old with dental restorations and OSA asks about eligibility for mandibular advancement. Which of the following statements about sleep bruxism is most accurate?

A. SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences.

B. Sleep bruxism includes a masticatory muscle activity during sleep, characterised as rhythmic (phasic — grinding) or non-rhythmic.

C. Sleep bruxism may be classified as possible, probable or definite according to assessment methods.

D. The protective-bruxism hypothesis proposes that jaw activity can follow obstruction and transiently restore airway tone.

E. Assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated.

CORRECT ANSWER B

Sleep bruxism includes a masticatory muscle activity during sleep, characterised as rhythmic (phasic — grinding) or non-rhythmic. Additional revision detail: a masticatory muscle activity during sleep, characterised as rhythmic (phasic — grinding) or non-rhythmic (tonic — clenching), and not a movement disorder or a sleep disorder in otherwise healthy individuals. The other options describe different mechanisms, investigations or findings rather than sleep bruxism. Consult Integrated notebook, page 89, §4.7 for the full context.

REFERENCE Integrated notebook, page 89, §4.7

QUESTION 0422 · §4.8 · Clinical interpretation SBA

A 46-year-old with OSA is referred for dental suitability assessment before oral appliance therapy. The clinical team records the following finding: hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation. Which concept best explains this observation?

A. SB prevalence

B. Implications

C. Conversely

D. Mechanisms

E. Successful outcome

CORRECT ANSWER C

This finding most directly describes Conversely: hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation. Distractor distinctions: A refers instead to reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients; B refers instead to chronic pain management must address sleep simultaneously, usually multidisciplinary; psychological input for the mood and an; D refers instead to dysregulated serotonin, noradrenaline and dopamine (regulating both pain and sleep); pro-inflammatory TNF-α and IL-6 in both; E refers instead to AHI/RDI/REI 50% reduction.

REFERENCE Integrated notebook, page 90, §4.8

QUESTION 0423 · §4.8 · Applied clinical SBA

A patient with tooth wear, jaw clicking and disruptive snoring attends an initial dental assessment. Which of the following statements about conversely is most accurate?

A. Hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation.

B. Successful outcome includes aHI/RDI/REI 50% reduction.

C. Fibromyalgia (acr 1990) includes pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%.

D. Published estimates of sleep bruxism prevalence vary widely because diagnostic definitions differ.

E. Mechanisms includes dysregulated serotonin, noradrenaline and dopamine (regulating both pain and sleep).

CORRECT ANSWER A

Hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation. The other options describe different mechanisms, investigations or findings rather than conversely. Consult Integrated notebook, page 90, §4.8 for the full context.

REFERENCE Integrated notebook, page 90, §4.8

QUESTION 0424 · §4.1 · Focused knowledge SBA

A 46-year-old with OSA is referred for dental suitability assessment before oral appliance therapy. Which of the following statements about imaging is most accurate?

A. Dental suitability assessment considers functional teeth, mobility, periodontal status and restorative needs.

B. Occlusion — examine and record the existing condition; this is your medicolegal baseline.

C. Dental radiographs should be justified by clinical indication rather than ordered routinely without purpose.

D. Reason for attendance includes snoring, suspected or diagnosed OSA, sleep bruxism, or orofacial pain related to sleep disorders.

E. Examination should document Mallampati class, tonsil size, tongue dimensions, palatal shape and nasal patency.

CORRECT ANSWER C

Dental radiographs should be justified by clinical indication rather than ordered routinely without purpose. Additional revision detail: initial dental radiographs as indicated; panoramic and lateral cephalometric as a pre-treatment record. CBCT and other CT are not indicated at this stage. The other options describe different mechanisms, investigations or findings rather than imaging. Consult Integrated notebook, page 86, §4.1 for the full context.

REFERENCE Integrated notebook, page 86, §4.1

QUESTION 0425 · §4.1 · Applied clinical SBA

A patient with tooth wear, jaw clicking and disruptive snoring attends an initial dental assessment. Which of the following statements about imaging is most accurate?

A. Occlusion — examine and record the existing condition; this is your medicolegal baseline.

B. Reason for attendance includes snoring, suspected or diagnosed OSA, sleep bruxism, or orofacial pain related to sleep disorders.

C. Examination should document Mallampati class, tonsil size, tongue dimensions, palatal shape and nasal patency.

D. Dental radiographs should be justified by clinical indication rather than ordered routinely without purpose.

E. Dental suitability assessment considers functional teeth, mobility, periodontal status and restorative needs.

CORRECT ANSWER D

Dental radiographs should be justified by clinical indication rather than ordered routinely without purpose. Additional revision detail: initial dental radiographs as indicated; panoramic and lateral cephalometric as a pre-treatment record. CBCT and other CT are not indicated at this stage. The other options describe different mechanisms, investigations or findings rather than imaging. Consult Integrated notebook, page 86, §4.1 for the full context.

REFERENCE Integrated notebook, page 86, §4.1

QUESTION 0426 · §4.2 · Focused knowledge SBA

A 59-year-old with dental restorations and OSA asks about eligibility for mandibular advancement. Which of the following statements about alcohol is most accurate?

A. Alcohol can impair pharyngeal muscle tone and worsen obstruction, including during appliance use.

B. Rhinosinusitis, allergic rhinitis and obstructive lung disease may complicate OSA treatment.

C. Obesity increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure.

D. Smoking contributes to upper-airway inflammation and may increase sleep-disordered breathing risk.

E. Drugs and medications includes anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function.

CORRECT ANSWER A

Alcohol can impair pharyngeal muscle tone and worsen obstruction, including during appliance use. Additional revision detail: impairs airway muscle tone and increases obstruction risk, especially with sedatives; even moderate intake may cause transient desaturation despite appliance use. The other options describe different mechanisms, investigations or findings rather than alcohol. Consult Integrated notebook, page 87, §4.2 for the full context.

REFERENCE Integrated notebook, page 87, §4.2

QUESTION 0427 · §4.2 · Applied clinical SBA

A patient with facial pain and unrefreshing sleep presents for multidisciplinary assessment. Which of the following statements about alcohol is most accurate?

A. Rhinosinusitis, allergic rhinitis and obstructive lung disease may complicate OSA treatment.

B. Smoking contributes to upper-airway inflammation and may increase sleep-disordered breathing risk.

C. Drugs and medications includes anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function.

D. Obesity increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure.

E. Alcohol can impair pharyngeal muscle tone and worsen obstruction, including during appliance use.

CORRECT ANSWER E

Alcohol can impair pharyngeal muscle tone and worsen obstruction, including during appliance use. Additional revision detail: impairs airway muscle tone and increases obstruction risk, especially with sedatives; even moderate intake may cause transient desaturation despite appliance use. The other options describe different mechanisms, investigations or findings rather than alcohol. Consult Integrated notebook, page 87, §4.2 for the full context.

REFERENCE Integrated notebook, page 87, §4.2

QUESTION 0428 · §4.3 · Clinical interpretation SBA

A patient with tooth wear, jaw clicking and disruptive snoring attends an initial dental assessment. The clinical team records the following finding: COMISA. Highly prevalent in SDB; the overlap produces more severe fragmentation, increased sleepiness and poorer quality of life. Which concept best explains this observation?

A. Insomnia

B. Narcolepsy

C. RLS and PLMD

D. Symptoms of OSA and depression overlap

E. Mood disorders

CORRECT ANSWER A

This finding most directly describes Insomnia: COMISA. Highly prevalent in SDB; the overlap produces more severe fragmentation, increased sleepiness and poorer quality of life. Distractor distinctions: B refers instead to May coexist, compounding sleepiness and cognitive impairment; symptom overlap makes clinical differentiation unreliable, so c; C refers instead to Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute; D refers instead to tiredness, fatigue, sleepiness, poor motivation and poor concentration — complicating the diagnosis and treatment of each; E refers instead to SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia, creating a cyc.

REFERENCE Integrated notebook, page 87, §4.3

QUESTION 0429 · §4.3 · Applied clinical SBA

A dentist is reviewing the medical and dental history of a new oral appliance candidate. Which of the following statements about insomnia is most accurate?

A. Coexisting sleep disorders can compound daytime sleepiness and complicate clinical diagnosis.

B. Comorbid insomnia and OSA can compound sleep fragmentation, sleepiness and reduced quality of life.

C. Sleep-disordered breathing and limb-movement disorders may coexist and compound sleep fragmentation.

D. OSA and depression share fatigue, impaired concentration and sleep-related symptoms.

E. The evidence for improved depression after CPAP is mixed and does not establish universal benefit.

CORRECT ANSWER B

Comorbid insomnia and OSA can compound sleep fragmentation, sleepiness and reduced quality of life. Additional revision detail: COMISA. Highly prevalent in SDB; the overlap produces more severe fragmentation, increased sleepiness and poorer quality of life. The other options describe different mechanisms, investigations or findings rather than insomnia. Consult Integrated notebook, page 87, §4.3 for the full context.

REFERENCE Integrated notebook, page 87, §4.3

QUESTION 0430 · §4.6 · Clinical interpretation SBA

A patient with tooth wear, jaw clicking and disruptive snoring attends an initial dental assessment. The clinical team records the following finding: Primary snoring without sleepiness → still refer to exclude OSA before providing an appliance. Which concept best explains this observation?

A. Shared

B. Portable monitor levels

C. Borderline cases

D. Severity and outcome in the shared-care setting

E. Assessment grading

CORRECT ANSWER C

This finding most directly describes Borderline cases: Primary snoring without sleepiness → still refer to exclude OSA before providing an appliance. Distractor distinctions: A refers instead to written professional communication between GP, sleep physician and dentist; B refers instead to Level 3 = ≥4 channels including airflow (≥2 channels of respiratory movement, or movement and airflow), heart rate/ECG, oxyge; D refers instead to Adult mild 5–14 · moderate 15–30 · severe >30; E refers instead to possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental ass.

REFERENCE Integrated notebook, page 88, §4.6

QUESTION 0431 · §4.6 · Applied clinical SBA

A dentist is reviewing the medical and dental history of a new oral appliance candidate. Which of the following statements about borderline cases is most accurate?

A. Adult OSA is conventionally classified as mild at AHI 5–14.9/h, moderate at 15–29.9/h and severe at 30/h or more.

B. Hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation.

C. Shared includes written professional communication between GP, sleep physician and dentist.

D. Patients with snoring still require physician assessment to exclude OSA before oral appliance treatment.

E. A type 3 sleep test measures several respiratory channels without EEG-based sleep staging.

CORRECT ANSWER D

Patients with snoring still require physician assessment to exclude OSA before oral appliance treatment. Additional revision detail: Primary snoring without sleepiness → still refer to exclude OSA before providing an appliance. The other options describe different mechanisms, investigations or findings rather than borderline cases. Consult Integrated notebook, page 88, §4.6 for the full context.

REFERENCE Integrated notebook, page 88, §4.6

QUESTION 0432 · §4.7 · Clinical interpretation SBA

A patient with facial pain and unrefreshing sleep presents for multidisciplinary assessment. The clinical team records the following finding: Sequence: muscle tone falls → obstruction → more negative intraluminal pressure → bruxism triggered, increasing tone and restoring patency → less negative pressure. Which concept best explains this observation?

A. Wide reported range

B. Shared clinical features

C. The "reactive–protective" hypothesis

D. Assessment grading

E. Self-reported sleep bruxism

CORRECT ANSWER C

This finding most directly describes The "reactive–protective" hypothesis: Sequence: muscle tone falls → obstruction → more negative intraluminal pressure → bruxism triggered, increasing tone and restoring patency → less negative pressure. Distractor distinctions: A refers instead to 6–91%, depending on assessment method; B refers instead to both commoner supine; both involve oropharyngeal and masticatory muscle activation; D refers instead to possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental ass; E refers instead to ~8% of the adult population.

REFERENCE Integrated notebook, page 89, §4.7

QUESTION 0433 · §4.7 · Applied clinical SBA

A 46-year-old with OSA is referred for dental suitability assessment before oral appliance therapy. Which of the following statements about the "reactive–protective" hypothesis is most accurate?

A. Sleep bruxism may be classified as possible, probable or definite according to assessment methods.

B. SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences.

C. Sleep bruxism includes a masticatory muscle activity during sleep, characterised as rhythmic (phasic — grinding) or non-rhythmic.

D. Assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated.

E. The protective-bruxism hypothesis proposes that jaw activity can follow obstruction and transiently restore airway tone.

CORRECT ANSWER E

The protective-bruxism hypothesis proposes that jaw activity can follow obstruction and transiently restore airway tone. Additional revision detail: Sequence: muscle tone falls → obstruction → more negative intraluminal pressure → bruxism triggered, increasing tone and restoring patency → less negative pressure. The other options describe different mechanisms, investigations or findings rather than the "reactive–protective" hypothesis. Consult Integrated notebook, page 89, §4.7 for the full context.

REFERENCE Integrated notebook, page 89, §4.7

QUESTION 0434 · §4.8 · Clinical interpretation SBA

A dentist is reviewing the medical and dental history of a new oral appliance candidate. The clinical team records the following finding: chronic pain management must address sleep simultaneously, usually multidisciplinary; psychological input for the mood and anxiety components; improving sleep improves pain tolerance and quality of life. Which concept best explains this observation?

A. Conversely

B. Successful outcome

C. SB prevalence

D. Mechanisms

E. Implications

CORRECT ANSWER E

This finding most directly describes Implications: chronic pain management must address sleep simultaneously, usually multidisciplinary; psychological input for the mood and anxiety components; improving sleep improves pain tolerance and quality of life. Distractor distinctions: A refers instead to hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation; B refers instead to AHI/RDI/REI 50% reduction; C refers instead to reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients; D refers instead to dysregulated serotonin, noradrenaline and dopamine (regulating both pain and sleep); pro-inflammatory TNF-α and IL-6 in both.

REFERENCE Integrated notebook, page 90, §4.8

QUESTION 0435 · §4.8 · Applied clinical SBA

A 59-year-old with dental restorations and OSA asks about eligibility for mandibular advancement. Which of the following statements about implications is most accurate?

A. Fibromyalgia (acr 1990) includes pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%.

B. Mechanisms includes dysregulated serotonin, noradrenaline and dopamine (regulating both pain and sleep).

C. Hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation.

D. Published estimates of sleep bruxism prevalence vary widely because diagnostic definitions differ.

E. Chronic pain management must address sleep simultaneously, usually multidisciplinary.

CORRECT ANSWER E

Chronic pain management must address sleep simultaneously, usually multidisciplinary. Additional revision detail: chronic pain management must address sleep simultaneously, usually multidisciplinary; psychological input for the mood and anxiety components; improving sleep improves pain tolerance and quality of life. The other options describe different mechanisms, investigations or findings rather than implications. Consult Integrated notebook, page 90, §4.8 for the full context.

REFERENCE Integrated notebook, page 90, §4.8

QUESTION 0436 · §4.1 · Clinical interpretation SBA

A dentist is reviewing the medical and dental history of a new oral appliance candidate. The clinical team records the following finding: dental changes, discomfort — may occur. Which concept best explains this observation?

A. Medical–dental history

B. Teeth and restorations

C. Bruxism, orofacial pain, headaches

D. Results vary, and some side effects

E. Explain the type of device being considered and all alternatives

CORRECT ANSWER D

This finding most directly describes Results vary, and some side effects: dental changes, discomfort — may occur. Distractor distinctions: A refers instead to age, weight, height; neck and belt circumference as indicated; B refers instead to number of functional teeth, mobility, spacing, curve of Spee, heavily restored teeth, anticipated restorative needs; C refers instead to Occlusion — examine and record the existing condition; this is your medicolegal baseline; E refers instead to CPAP, surgery, positional therapy.

REFERENCE Integrated notebook, page 92, §4.1

QUESTION 0437 · §4.1 · Applied clinical SBA

A 59-year-old with dental restorations and OSA asks about eligibility for mandibular advancement. Which of the following statements about results vary, and some side effects is most accurate?

A. Attachment loss, mobility, prognosis of the teeth that will bear the appliance.

B. Relevant findings include CPAP, surgery, positional therapy.

C. Occlusion — examine and record the existing condition; this is your medicolegal baseline.

D. Dental changes, discomfort — may occur.

E. A screening assessment should document age, weight, height and relevant circumferences.

CORRECT ANSWER D

Dental changes, discomfort — may occur. The other options describe different mechanisms, investigations or findings rather than results vary, and some side effects. Consult Integrated notebook, page 92, §4.1 for the full context.

REFERENCE Integrated notebook, page 92, §4.1

QUESTION 0438 · §4.2 · Clinical interpretation SBA

A 46-year-old with OSA is referred for dental suitability assessment before oral appliance therapy. The clinical team records the following finding: chronic rhinosinusitis, allergic rhinitis, asthma or COPD worsen airway function and complicate management. Which concept best explains this observation?

A. Other sleep disorders and comorbidities

B. Obesity

C. Smoking

D. Drugs and medications

E. Alcohol

CORRECT ANSWER A

This finding most directly describes Other sleep disorders and comorbidities: chronic rhinosinusitis, allergic rhinitis, asthma or COPD worsen airway function and complicate management. Distractor distinctions: B refers instead to increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure; requires ta; C refers instead to chronic inflammation, upper airway pathology, susceptibility to infection and cancer, higher morbidity and mortality; D refers instead to anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function, increasing obstruction and respiratory f; E refers instead to impairs airway muscle tone and increases obstruction risk, especially with sedatives; even moderate intake may cause transien.

REFERENCE Integrated notebook, page 87, §4.2

QUESTION 0439 · §4.2 · Applied clinical SBA

A patient with tooth wear, jaw clicking and disruptive snoring attends an initial dental assessment. Which of the following statements about other sleep disorders and comorbidities is most accurate?

A. Smoking contributes to upper-airway inflammation and may increase sleep-disordered breathing risk.

B. Alcohol can impair pharyngeal muscle tone and worsen obstruction, including during appliance use.

C. Obesity increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure.

D. Drugs and medications includes anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function.

E. Rhinosinusitis, allergic rhinitis and obstructive lung disease may complicate OSA treatment.

CORRECT ANSWER E

Rhinosinusitis, allergic rhinitis and obstructive lung disease may complicate OSA treatment. Additional revision detail: chronic rhinosinusitis, allergic rhinitis, asthma or COPD worsen airway function and complicate management. The other options describe different mechanisms, investigations or findings rather than other sleep disorders and comorbidities. Consult Integrated notebook, page 87, §4.2 for the full context.

REFERENCE Integrated notebook, page 87, §4.2

QUESTION 0440 · §4.3 · Clinical interpretation SBA

A 59-year-old with dental restorations and OSA asks about eligibility for mandibular advancement. The clinical team records the following finding: SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are at higher risk of sleepiness and accidents. Which concept best explains this observation?

A. The relationship appears mutual rather than cause-and-effect

B. Insufficient sleep and shift work disorder

C. RLS and PLMD

D. Symptoms of OSA and depression overlap

E. CPAP's effect on depression is not conclusive

CORRECT ANSWER B

This finding most directly describes Insufficient sleep and shift work disorder: SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are at higher risk of sleepiness and accidents. Distractor distinctions: A refers instead to The pathophysiological relationship is not fully understood: it may be coincidental, or due to sleep fragmentation or repeate; C refers instead to Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute; D refers instead to tiredness, fatigue, sleepiness, poor motivation and poor concentration — complicating the diagnosis and treatment of each; E refers instead to evidence is limited; some studies report benefit for both sleepiness and depression, others no effect. 4.4 Medical comorbidit.

REFERENCE Integrated notebook, page 87, §4.3

QUESTION 0441 · §4.3 · Applied clinical SBA

A patient with facial pain and unrefreshing sleep presents for multidisciplinary assessment. Which of the following statements about insufficient sleep and shift work disorder is most accurate?

A. OSA and depression share fatigue, impaired concentration and sleep-related symptoms.

B. SDB can exacerbate daytime dysfunction in people with insufficient sleep or shift-work disorder.

C. Comorbid insomnia and OSA can compound sleep fragmentation, sleepiness and reduced quality of life.

D. Sleep-disordered breathing and limb-movement disorders may coexist and compound sleep fragmentation.

E. Coexisting sleep disorders can compound daytime sleepiness and complicate clinical diagnosis.

CORRECT ANSWER B

SDB can exacerbate daytime dysfunction in people with insufficient sleep or shift-work disorder. Additional revision detail: SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are at higher risk of sleepiness and accidents. The other options describe different mechanisms, investigations or findings rather than insufficient sleep and shift work disorder. Consult Integrated notebook, page 87, §4.3 for the full context.

REFERENCE Integrated notebook, page 87, §4.3

QUESTION 0442 · §4.7 · Clinical interpretation SBA

A patient with facial pain and unrefreshing sleep presents for multidisciplinary assessment. The clinical team records the following finding: possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental assessment, notably PSG with EMG). Which concept best explains this observation?

A. Assessment grading

B. Sleep bruxism

C. Wide reported range

D. Shared clinical features

E. Corollary

CORRECT ANSWER A

This finding most directly describes Assessment grading: possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental assessment, notably PSG with EMG). Distractor distinctions: B refers instead to a masticatory muscle activity during sleep, characterised as rhythmic (phasic — grinding) or non-rhythmic (tonic — clenching); C refers instead to 6–91%, depending on assessment method; D refers instead to both commoner supine; both involve oropharyngeal and masticatory muscle activation; E refers instead to SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences.

REFERENCE Integrated notebook, page 89, §4.7

QUESTION 0443 · §4.7 · Applied clinical SBA

A 46-year-old with OSA is referred for dental suitability assessment before oral appliance therapy. Which of the following statements about assessment grading is most accurate?

A. Sleep bruxism may be classified as possible, probable or definite according to assessment methods.

B. Sleep bruxism includes a masticatory muscle activity during sleep, characterised as rhythmic (phasic — grinding) or non-rhythmic.

C. The protective-bruxism hypothesis proposes that jaw activity can follow obstruction and transiently restore airway tone.

D. SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences.

E. Assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated.

CORRECT ANSWER A

Sleep bruxism may be classified as possible, probable or definite according to assessment methods. Additional revision detail: possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental assessment, notably PSG with EMG). The other options describe different mechanisms, investigations or findings rather than assessment grading. Consult Integrated notebook, page 89, §4.7 for the full context.

REFERENCE Integrated notebook, page 89, §4.7

QUESTION 0444 · §4.8 · Clinical interpretation SBA

A dentist is reviewing the medical and dental history of a new oral appliance candidate. The clinical team records the following finding: dysregulated serotonin, noradrenaline and dopamine (regulating both pain and sleep); pro-inflammatory TNF-α and IL-6 in both pathways; HPA-axis dysfunction. Which concept best explains this observation?

A. Mechanisms

B. SB prevalence

C. Successful outcome

D. Implications

E. Conversely

CORRECT ANSWER A

This finding most directly describes Mechanisms: dysregulated serotonin, noradrenaline and dopamine (regulating both pain and sleep); pro-inflammatory TNF-α and IL-6 in both pathways; HPA-axis dysfunction. Distractor distinctions: B refers instead to reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients; C refers instead to AHI/RDI/REI 50% reduction; D refers instead to chronic pain management must address sleep simultaneously, usually multidisciplinary; psychological input for the mood and an; E refers instead to hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation.

REFERENCE Integrated notebook, page 90, §4.8

QUESTION 0445 · §4.8 · Applied clinical SBA

A 59-year-old with dental restorations and OSA asks about eligibility for mandibular advancement. Which of the following statements about mechanisms is most accurate?

A. Chronic pain management must address sleep simultaneously, usually multidisciplinary.

B. Hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation.

C. Mechanisms includes dysregulated serotonin, noradrenaline and dopamine (regulating both pain and sleep).

D. Published estimates of sleep bruxism prevalence vary widely because diagnostic definitions differ.

E. Fibromyalgia (acr 1990) includes pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%.

CORRECT ANSWER C

Mechanisms includes dysregulated serotonin, noradrenaline and dopamine (regulating both pain and sleep). Additional revision detail: dysregulated serotonin, noradrenaline and dopamine (regulating both pain and sleep); pro-inflammatory TNF-α and IL-6 in both pathways; HPA-axis dysfunction. The other options describe different mechanisms, investigations or findings rather than mechanisms. Consult Integrated notebook, page 90, §4.8 for the full context.

REFERENCE Integrated notebook, page 90, §4.8

QUESTION 0446 · §4.1 · Clinical interpretation SBA

A dentist is reviewing the medical and dental history of a new oral appliance candidate. The clinical team records the following finding: snoring, suspected or diagnosed OSA, sleep bruxism, or orofacial pain related to sleep disorders. Ask about sleep symptoms, previous diagnoses, comorbidities, and impact on daily life and well-being. Which concept best explains this observation?

A. Soft tissue/intraoral

B. Reason for attendance

C. Periodontal evaluation

D. Teeth and restorations

E. Medical–dental history

CORRECT ANSWER B

This finding most directly describes Reason for attendance: snoring, suspected or diagnosed OSA, sleep bruxism, or orofacial pain related to sleep disorders. Ask about sleep symptoms, previous diagnoses, comorbidities, and impact on daily life and well-being. Distractor distinctions: A refers instead to modified Mallampati, tonsil grade, tongue size and scalloping, palatal vault, posterior pharyngeal crowding, signs of mouth b; C refers instead to attachment loss, mobility, prognosis of the teeth that will bear the appliance; D refers instead to number of functional teeth, mobility, spacing, curve of Spee, heavily restored teeth, anticipated restorative needs; E refers instead to age, weight, height; neck and belt circumference as indicated.

REFERENCE Integrated notebook, page 86, §4.1

QUESTION 0447 · §4.1 · Applied clinical SBA

A 59-year-old with dental restorations and OSA asks about eligibility for mandibular advancement. Which of the following statements about reason for attendance is most accurate?

A. Occlusion — examine and record the existing condition; this is your medicolegal baseline.

B. Dental radiographs should be justified by clinical indication rather than ordered routinely without purpose.

C. Dental suitability assessment considers functional teeth, mobility, periodontal status and restorative needs.

D. Reason for attendance includes snoring, suspected or diagnosed OSA, sleep bruxism, or orofacial pain related to sleep disorders.

E. Examination should document Mallampati class, tonsil size, tongue dimensions, palatal shape and nasal patency.

CORRECT ANSWER D

Reason for attendance includes snoring, suspected or diagnosed OSA, sleep bruxism, or orofacial pain related to sleep disorders. Additional revision detail: snoring, suspected or diagnosed OSA, sleep bruxism, or orofacial pain related to sleep disorders. Ask about sleep symptoms, previous diagnoses, comorbidities, and impact on daily life and well-being. The other options describe different mechanisms, investigations or findings rather than reason for attendance. Consult Integrated notebook, page 86, §4.1 for the full context.

REFERENCE Integrated notebook, page 86, §4.1

QUESTION 0448 · §4.3 · Clinical interpretation SBA

A patient with tooth wear, jaw clicking and disruptive snoring attends an initial dental assessment. The clinical team records the following finding: SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia, creating a cycle of worsening sleep and mental health. OSA and depression — the nuance. Which concept best explains this observation?

A. Symptoms of OSA and depression overlap

B. RLS and PLMD

C. Narcolepsy

D. Insomnia

E. Mood disorders

CORRECT ANSWER E

This finding most directly describes Mood disorders: SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia, creating a cycle of worsening sleep and mental health. OSA and depression — the nuance. Distractor distinctions: A refers instead to tiredness, fatigue, sleepiness, poor motivation and poor concentration — complicating the diagnosis and treatment of each; B refers instead to Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute; C refers instead to May coexist, compounding sleepiness and cognitive impairment; symptom overlap makes clinical differentiation unreliable, so c; D refers instead to COMISA. Highly prevalent in SDB; the overlap produces more severe fragmentation, increased sleepiness and poorer quality of l.

REFERENCE Integrated notebook, page 87, §4.3

QUESTION 0449 · §4.3 · Applied clinical SBA

A dentist is reviewing the medical and dental history of a new oral appliance candidate. Which of the following statements about mood disorders is most accurate?

A. SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia.

B. The evidence for improved depression after CPAP is mixed and does not establish universal benefit.

C. Sleep-disordered breathing and limb-movement disorders may coexist and compound sleep fragmentation.

D. Coexisting sleep disorders can compound daytime sleepiness and complicate clinical diagnosis.

E. SDB can exacerbate daytime dysfunction in people with insufficient sleep or shift-work disorder.

CORRECT ANSWER A

SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia. Additional revision detail: SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia, creating a cycle of worsening sleep and mental health. OSA and depression — the nuance. The other options describe different mechanisms, investigations or findings rather than mood disorders. Consult Integrated notebook, page 87, §4.3 for the full context.

REFERENCE Integrated notebook, page 87, §4.3

QUESTION 0450 · §4.7 · Clinical interpretation SBA

A dentist is reviewing the medical and dental history of a new oral appliance candidate. The clinical team records the following finding: assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated. Which concept best explains this observation?

A. Corollary

B. SB + TMD

C. Sleep bruxism

D. Assessment grading

E. Wide reported range

CORRECT ANSWER B

This finding most directly describes SB + TMD: assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated. Distractor distinctions: A refers instead to SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences; C refers instead to a masticatory muscle activity during sleep, characterised as rhythmic (phasic — grinding) or non-rhythmic (tonic — clenching); D refers instead to possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental ass; E refers instead to 6–91%, depending on assessment method.

REFERENCE Integrated notebook, page 90, §4.7

QUESTION 0451 · §4.7 · Applied clinical SBA

A 59-year-old with dental restorations and OSA asks about eligibility for mandibular advancement. Which of the following statements about sb + TMD is most accurate?

A. Assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated.

B. Shared clinical features includes both commoner supine; both involve oropharyngeal and masticatory muscle activation.

C. SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences.

D. The protective-bruxism hypothesis proposes that jaw activity can follow obstruction and transiently restore airway tone.

E. Sleep bruxism may be classified as possible, probable or definite according to assessment methods.

CORRECT ANSWER A

Assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated. The other options describe different mechanisms, investigations or findings rather than sb + tmd. Consult Integrated notebook, page 90, §4.7 for the full context.

REFERENCE Integrated notebook, page 90, §4.7

QUESTION 0452 · §4.8 · Clinical interpretation SBA

A 46-year-old with OSA is referred for dental suitability assessment before oral appliance therapy. The clinical team records the following finding: reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients. Which concept best explains this observation?

A. Mechanisms

B. Conversely

C. Implications

D. Successful outcome

E. SB prevalence

CORRECT ANSWER E

This finding most directly describes SB prevalence: reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients. Distractor distinctions: A refers instead to dysregulated serotonin, noradrenaline and dopamine (regulating both pain and sleep); pro-inflammatory TNF-α and IL-6 in both; B refers instead to hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation; C refers instead to chronic pain management must address sleep simultaneously, usually multidisciplinary; psychological input for the mood and an; D refers instead to AHI/RDI/REI 50% reduction.

REFERENCE Integrated notebook, page 103, §4.8

QUESTION 0453 · §4.8 · Applied clinical SBA

A patient with tooth wear, jaw clicking and disruptive snoring attends an initial dental assessment. Which of the following statements about sb prevalence is most accurate?

A. Fibromyalgia (acr 1990) includes pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%.

B. Mechanisms includes dysregulated serotonin, noradrenaline and dopamine (regulating both pain and sleep).

C. Hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation.

D. Published estimates of sleep bruxism prevalence vary widely because diagnostic definitions differ.

E. Successful outcome includes aHI/RDI/REI 50% reduction.

CORRECT ANSWER D

Published estimates of sleep bruxism prevalence vary widely because diagnostic definitions differ. Additional revision detail: reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients. The other options describe different mechanisms, investigations or findings rather than sb prevalence. Consult Integrated notebook, page 103, §4.8 for the full context.

REFERENCE Integrated notebook, page 103, §4.8

QUESTION 0454 · §4.1 · Clinical interpretation SBA

A 46-year-old with OSA is referred for dental suitability assessment before oral appliance therapy. The clinical team records the following finding: modified Mallampati, tonsil grade, tongue size and scalloping, palatal vault, posterior pharyngeal crowding, signs of mouth breathing, signs of GORD such as enamel erosion, nasal patency. Which concept best explains this observation?

A. Teeth and restorations

B. Soft tissue/intraoral

C. Reason for attendance

D. Periodontal evaluation

E. Medical–dental history

CORRECT ANSWER B

This finding most directly describes Soft tissue/intraoral: modified Mallampati, tonsil grade, tongue size and scalloping, palatal vault, posterior pharyngeal crowding, signs of mouth breathing, signs of GORD such as enamel erosion, nasal patency. Distractor distinctions: A refers instead to number of functional teeth, mobility, spacing, curve of Spee, heavily restored teeth, anticipated restorative needs; C refers instead to snoring, suspected or diagnosed OSA, sleep bruxism, or orofacial pain related to sleep disorders. Ask about sleep symptoms, p; D refers instead to attachment loss, mobility, prognosis of the teeth that will bear the appliance; E refers instead to age, weight, height; neck and belt circumference as indicated.

REFERENCE Integrated notebook, page 86, §4.1

QUESTION 0455 · §4.1 · Applied clinical SBA

A patient with tooth wear, jaw clicking and disruptive snoring attends an initial dental assessment. Which of the following statements about soft tissue/intraoral is most accurate?

A. Reason for attendance includes snoring, suspected or diagnosed OSA, sleep bruxism, or orofacial pain related to sleep disorders.

B. Dental radiographs should be justified by clinical indication rather than ordered routinely without purpose.

C. Occlusion — examine and record the existing condition; this is your medicolegal baseline.

D. Dental suitability assessment considers functional teeth, mobility, periodontal status and restorative needs.

E. Examination should document Mallampati class, tonsil size, tongue dimensions, palatal shape and nasal patency.

CORRECT ANSWER E

Examination should document Mallampati class, tonsil size, tongue dimensions, palatal shape and nasal patency. Additional revision detail: modified Mallampati, tonsil grade, tongue size and scalloping, palatal vault, posterior pharyngeal crowding, signs of mouth breathing, signs of GORD such as enamel erosion, nasal patency. The other options describe different mechanisms, investigations or findings rather than soft tissue/intraoral. Consult Integrated notebook, page 86, §4.1 for the full context.

REFERENCE Integrated notebook, page 86, §4.1

QUESTION 0456 · §4.3 · Clinical interpretation SBA

A patient with facial pain and unrefreshing sleep presents for multidisciplinary assessment. The clinical team records the following finding: Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness. Which concept best explains this observation?

A. Mood disorders

B. Symptoms of OSA and depression overlap

C. The relationship appears mutual rather than cause-and-effect

D. RLS and PLMD

E. Insufficient sleep and shift work disorder

CORRECT ANSWER D

This finding most directly describes RLS and PLMD: Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness. Distractor distinctions: A refers instead to SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia, creating a cyc; B refers instead to tiredness, fatigue, sleepiness, poor motivation and poor concentration — complicating the diagnosis and treatment of each; C refers instead to The pathophysiological relationship is not fully understood: it may be coincidental, or due to sleep fragmentation or repeate; E refers instead to SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are.

REFERENCE Integrated notebook, page 87, §4.3

QUESTION 0457 · §4.3 · Applied clinical SBA

A 46-year-old with OSA is referred for dental suitability assessment before oral appliance therapy. Which of the following statements about rls and plmd is most accurate?

A. The evidence for improved depression after CPAP is mixed and does not establish universal benefit.

B. Comorbid insomnia and OSA can compound sleep fragmentation, sleepiness and reduced quality of life.

C. Coexisting sleep disorders can compound daytime sleepiness and complicate clinical diagnosis.

D. SDB can exacerbate daytime dysfunction in people with insufficient sleep or shift-work disorder.

E. Sleep-disordered breathing and limb-movement disorders may coexist and compound sleep fragmentation.

CORRECT ANSWER E

Sleep-disordered breathing and limb-movement disorders may coexist and compound sleep fragmentation. Additional revision detail: Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness. The other options describe different mechanisms, investigations or findings rather than rls and plmd. Consult Integrated notebook, page 87, §4.3 for the full context.

REFERENCE Integrated notebook, page 87, §4.3

QUESTION 0458 · §4.7 · Clinical interpretation SBA

A 46-year-old with OSA is referred for dental suitability assessment before oral appliance therapy. The clinical team records the following finding: ~8% of the adult population. Which concept best explains this observation?

A. Sleep bruxism

B. Self-reported sleep bruxism

C. Shared clinical features

D. Assessment grading

E. Wide reported range

CORRECT ANSWER B

This finding most directly describes Self-reported sleep bruxism: ~8% of the adult population. Distractor distinctions: A refers instead to a masticatory muscle activity during sleep, characterised as rhythmic (phasic — grinding) or non-rhythmic (tonic — clenching); C refers instead to both commoner supine; both involve oropharyngeal and masticatory muscle activation; D refers instead to possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental ass; E refers instead to 6–91%, depending on assessment method.

REFERENCE Integrated notebook, page 100, §4.7

QUESTION 0459 · §4.7 · Applied clinical SBA

A patient with tooth wear, jaw clicking and disruptive snoring attends an initial dental assessment. Which of the following statements about self-reported sleep bruxism is most accurate?

A. Shared clinical features includes both commoner supine; both involve oropharyngeal and masticatory muscle activation.

B. Assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated.

C. Self-reported sleep bruxism: ~8% of the adult population.

D. SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences.

E. Wide reported range: 6–91%, depending on assessment method.

CORRECT ANSWER C

Self-reported sleep bruxism: ~8% of the adult population. Additional revision detail: ~8% of the adult population. The other options describe different mechanisms, investigations or findings rather than self-reported sleep bruxism. Consult Integrated notebook, page 100, §4.7 for the full context.

REFERENCE Integrated notebook, page 100, §4.7

QUESTION 0460 · §4.8 · Clinical interpretation SBA

A 59-year-old with dental restorations and OSA asks about eligibility for mandibular advancement. The clinical team records the following finding: AHI/RDI/REI 50% reduction. Which concept best explains this observation?

A. Implications

B. SB prevalence

C. Successful treatment outcome

D. Successful outcome

E. Fibromyalgia (ACR 1990)

CORRECT ANSWER C

This finding most directly describes Successful treatment outcome: AHI/RDI/REI 50% reduction. Distractor distinctions: A refers instead to chronic pain management must address sleep simultaneously, usually multidisciplinary; psychological input for the mood and an; B refers instead to reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients; D refers instead to AHI/RDI/REI 50% reduction; E refers instead to pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%.

REFERENCE Integrated notebook, page 103, §4.8

QUESTION 0461 · §4.8 · Applied clinical SBA

A patient with facial pain and unrefreshing sleep presents for multidisciplinary assessment. Which of the following statements about successful treatment outcome is most accurate?

A. Published estimates of sleep bruxism prevalence vary widely because diagnostic definitions differ.

B. Fibromyalgia (acr 1990) includes pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%.

C. Hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation.

D. Mechanisms includes dysregulated serotonin, noradrenaline and dopamine (regulating both pain and sleep).

E. Successful outcome includes aHI/RDI/REI 50% reduction.

CORRECT ANSWER E

Successful outcome includes aHI/RDI/REI 50% reduction. Additional revision detail: AHI/RDI/REI 50% reduction. The other options describe different mechanisms, investigations or findings rather than successful outcome. Consult Integrated notebook, page 103, §4.8 for the full context.

REFERENCE Integrated notebook, page 103, §4.8

QUESTION 0462 · §4.1 · Clinical interpretation SBA

A 59-year-old with dental restorations and OSA asks about eligibility for mandibular advancement. The clinical team records the following finding: CPAP, surgery, positional therapy. Which concept best explains this observation?

A. Medical–dental history

B. Explain the type of device being considered and all alternatives

C. Results vary, and some side effects

D. Teeth and restorations

E. Bruxism, orofacial pain, headaches

CORRECT ANSWER B

This finding most directly describes Explain the type of device being considered and all alternatives: CPAP, surgery, positional therapy. Distractor distinctions: A refers instead to age, weight, height; neck and belt circumference as indicated; C refers instead to dental changes, discomfort — may occur; D refers instead to number of functional teeth, mobility, spacing, curve of Spee, heavily restored teeth, anticipated restorative needs; E refers instead to Occlusion — examine and record the existing condition; this is your medicolegal baseline.

REFERENCE Integrated notebook, page 93, §4.1

QUESTION 0463 · §4.1 · Applied clinical SBA

A patient with facial pain and unrefreshing sleep presents for multidisciplinary assessment. Which of the following statements about explain the type of device being considered and all alternatives is most accurate?

A. Dental changes, discomfort — may occur.

B. Relevant findings include CPAP, surgery, positional therapy.

C. A screening assessment should document age, weight, height and relevant circumferences.

D. Attachment loss, mobility, prognosis of the teeth that will bear the appliance.

E. Occlusion — examine and record the existing condition; this is your medicolegal baseline.

CORRECT ANSWER B

Relevant findings include CPAP, surgery, positional therapy. Additional revision detail: CPAP, surgery, positional therapy. The other options describe different mechanisms, investigations or findings rather than explain the type of device being considered and all alternatives. Consult Integrated notebook, page 93, §4.1 for the full context.

REFERENCE Integrated notebook, page 93, §4.1

QUESTION 0464 · §4.3 · Clinical interpretation SBA

A dentist is reviewing the medical and dental history of a new oral appliance candidate. The clinical team records the following finding: evidence is limited; some studies report benefit for both sleepiness and depression, others no effect. 4.4 Medical comorbidities. Which concept best explains this observation?

A. The relationship appears mutual rather than cause-and-effect

B. CPAP's effect on depression is not conclusive

C. Insufficient sleep and shift work disorder

D. Symptoms of OSA and depression overlap

E. Mood disorders

CORRECT ANSWER B

This finding most directly describes CPAP's effect on depression is not conclusive: evidence is limited; some studies report benefit for both sleepiness and depression, others no effect. 4.4 Medical comorbidities. Distractor distinctions: A refers instead to The pathophysiological relationship is not fully understood: it may be coincidental, or due to sleep fragmentation or repeate; C refers instead to SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are; D refers instead to tiredness, fatigue, sleepiness, poor motivation and poor concentration — complicating the diagnosis and treatment of each; E refers instead to SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia, creating a cyc.

REFERENCE Integrated notebook, page 87, §4.3

QUESTION 0465 · §4.3 · Applied clinical SBA

A 59-year-old with dental restorations and OSA asks about eligibility for mandibular advancement. Which of the following statements about CPAP's effect on depression is not conclusive is most accurate?

A. OSA and depression share fatigue, impaired concentration and sleep-related symptoms.

B. Comorbid insomnia and OSA can compound sleep fragmentation, sleepiness and reduced quality of life.

C. Sleep-disordered breathing and limb-movement disorders may coexist and compound sleep fragmentation.

D. The evidence for improved depression after CPAP is mixed and does not establish universal benefit.

E. Coexisting sleep disorders can compound daytime sleepiness and complicate clinical diagnosis.

CORRECT ANSWER D

The evidence for improved depression after CPAP is mixed and does not establish universal benefit. Additional revision detail: evidence is limited; some studies report benefit for both sleepiness and depression, others no effect. 4.4 Medical comorbidities. The other options describe different mechanisms, investigations or findings rather than cpap's effect on depression is not conclusive. Consult Integrated notebook, page 87, §4.3 for the full context.

REFERENCE Integrated notebook, page 87, §4.3

QUESTION 0466 · §4.7 · Clinical interpretation SBA

A 59-year-old with dental restorations and OSA asks about eligibility for mandibular advancement. The clinical team records the following finding: both commoner supine; both involve oropharyngeal and masticatory muscle activation. Which concept best explains this observation?

A. Wide reported range

B. Assessment grading

C. Sleep bruxism

D. Shared clinical features

E. Self-reported sleep bruxism

CORRECT ANSWER D

This finding most directly describes Shared clinical features: both commoner supine; both involve oropharyngeal and masticatory muscle activation. Distractor distinctions: A refers instead to 6–91%, depending on assessment method; B refers instead to possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental ass; C refers instead to a masticatory muscle activity during sleep, characterised as rhythmic (phasic — grinding) or non-rhythmic (tonic — clenching); E refers instead to ~8% of the adult population.

REFERENCE Integrated notebook, page 89, §4.7

QUESTION 0467 · §4.7 · Applied clinical SBA

A patient with facial pain and unrefreshing sleep presents for multidisciplinary assessment. Which of the following statements about shared clinical features is most accurate?

A. Wide reported range: 6–91%, depending on assessment method.

B. Self-reported sleep bruxism: ~8% of the adult population.

C. Assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated.

D. Shared clinical features includes both commoner supine; both involve oropharyngeal and masticatory muscle activation.

E. SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences.

CORRECT ANSWER D

Shared clinical features includes both commoner supine; both involve oropharyngeal and masticatory muscle activation. Additional revision detail: both commoner supine; both involve oropharyngeal and masticatory muscle activation. The other options describe different mechanisms, investigations or findings rather than shared clinical features. Consult Integrated notebook, page 89, §4.7 for the full context.

REFERENCE Integrated notebook, page 89, §4.7

QUESTION 0468 · §4.8 · Clinical interpretation SBA

A patient with tooth wear, jaw clicking and disruptive snoring attends an initial dental assessment. The clinical team records the following finding: AHI/RDI/REI 50% reduction. Which concept best explains this observation?

A. Fibromyalgia (ACR 1990)

B. Implications

C. Successful treatment outcome

D. SB prevalence

E. Successful outcome

CORRECT ANSWER E

This finding most directly describes Successful outcome: AHI/RDI/REI 50% reduction. Distractor distinctions: A refers instead to pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%; B refers instead to chronic pain management must address sleep simultaneously, usually multidisciplinary; psychological input for the mood and an; C refers instead to AHI/RDI/REI 50% reduction; D refers instead to reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients.

REFERENCE Integrated notebook, page 91, §4.8

QUESTION 0469 · §4.8 · Applied clinical SBA

A dentist is reviewing the medical and dental history of a new oral appliance candidate. Which of the following statements about successful outcome is most accurate?

A. Successful outcome includes aHI/RDI/REI 50% reduction.

B. Published estimates of sleep bruxism prevalence vary widely because diagnostic definitions differ.

C. Hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation.

D. Mechanisms includes dysregulated serotonin, noradrenaline and dopamine (regulating both pain and sleep).

E. Fibromyalgia (acr 1990) includes pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%.

CORRECT ANSWER A

Successful outcome includes aHI/RDI/REI 50% reduction. Additional revision detail: AHI/RDI/REI 50% reduction. The other options describe different mechanisms, investigations or findings rather than successful outcome. Consult Integrated notebook, page 91, §4.8 for the full context.

REFERENCE Integrated notebook, page 91, §4.8

QUESTION 0470 · §4.1 · Clinical interpretation SBA

A patient with tooth wear, jaw clicking and disruptive snoring attends an initial dental assessment. The clinical team records the following finding: age, weight, height; neck and belt circumference as indicated. Which concept best explains this observation?

A. Teeth and restorations

B. Periodontal evaluation

C. Medical–dental history

D. Soft tissue/intraoral

E. Reason for attendance

CORRECT ANSWER C

This finding most directly describes Medical–dental history: age, weight, height; neck and belt circumference as indicated. Distractor distinctions: A refers instead to number of functional teeth, mobility, spacing, curve of Spee, heavily restored teeth, anticipated restorative needs; B refers instead to attachment loss, mobility, prognosis of the teeth that will bear the appliance; D refers instead to modified Mallampati, tonsil grade, tongue size and scalloping, palatal vault, posterior pharyngeal crowding, signs of mouth b; E refers instead to snoring, suspected or diagnosed OSA, sleep bruxism, or orofacial pain related to sleep disorders. Ask about sleep symptoms, p.

REFERENCE Integrated notebook, page 86, §4.1

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05 Appliance design, registration, manufacture and fitting

QUESTION 0471 · §5.7 · Clinical decision-making SBA

A lab says the impression tore at a retentive molar undercut. Which material property is most relevant?

A. No need for stable working models

B. Avoiding periodontal assessment

C. Higher AHI before treatment

D. Adequate tear strength and recovery without distortion

E. Greater maximum mandibular protrusion

CORRECT ANSWER D

Impression materials must reproduce fine detail and tolerate removal without tearing or permanent deformation.

REFERENCE Integrated notebook, Topic 5, §5.7

QUESTION 0472 · §5.6 · Clinical decision-making SBA

A patient has a maximum protrusive range of 10 mm and is being registered for a custom titratable MAD. Which initial registration is the most defensible?

A. At 100% of maximum protrusion

B. Approximately 50–70% of maximum, with minimal necessary vertical opening

C. In maximal retrusion and maximal vertical opening

D. At an arbitrary position chosen by the laboratory

E. A fixed 1 mm advancement regardless of protrusive range

CORRECT ANSWER B

An initial setting around 50–70% of maximum protrusive range is described in the source, with minimal vertical opening. Later titration is governed by response and comfort. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 5, §5.6

QUESTION 0473 · §5.4 · Clinical decision-making SBA

A patient with a history of dental restorations requires a mandibular advancement device that can be titrated gradually. Which appliance property is most important?

A. Irreversible fixed mandibular advancement

B. Stable retention with small, measurable, reversible increments

C. The largest possible vertical opening

D. Construction without records or fit verification

E. A purely decorative upper arch component

CORRECT ANSWER B

The notebook specifies titratability in small steps, a useful range, reversibility, verifiable settings and stable retention. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 5, §5.4

QUESTION 0474 · §5.10 · Clinical decision-making SBA

A new MAD user asks about daily cleaning and storage. Which advice is most appropriate?

A. Never inspect the appliance for cracks

B. Follow the manufacturer's material-specific cleaning instructions and attend reviews for wear and hygiene

C. Store the appliance near direct heat to sterilise it

D. Use abrasive toothpaste irrespective of material

E. Avoid any cleaning until the annual appointment

CORRECT ANSWER B

Appliance longevity and safe use depend on appropriate material-specific care, hygiene, inspection and regular follow-up. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 5, §5.10

QUESTION 0475 · §5.9 · Clinical decision-making SBA

A patient attends the delivery appointment for a new custom mandibular advancement appliance. Which is the most important initial clinical check?

A. Record the retail brand of the device only

B. Assume fit is satisfactory if the device clicks into place

C. Verify retention, comfort, protrusive setting, occlusion and patient insertion/removal

D. Advance to the mechanical end-stop before patient education

E. Defer discussion of adverse effects until the first complaint

CORRECT ANSWER C

Safe delivery includes verification of fit, retention, advancement, vertical opening and oral comfort, with patient education and documentation. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 5, §5.9

QUESTION 0476 · §5.8 · Clinical decision-making SBA

A laboratory produces a device at an arbitrary mandibular advancement because the prescription lacked a construction bite. What error occurred?

A. An unavoidable sleep-study scoring error

B. Failure to communicate the measured horizontal and vertical bite relationship

C. A confirmed medical diagnosis error

D. Proof that every device must be non-titratable

E. A fault corrected solely by prescribing more sleep hours

CORRECT ANSWER B

Laboratory requests should specify verified records, protrusion, vertical opening and chosen design so clinical treatment can be reproduced.

REFERENCE Integrated notebook, Topic 5, §5.8

QUESTION 0477 · §5.2 · Applied clinical SBA

A follow-up patient reports poor retention of a recently supplied oral appliance. Which of the following statements about mild OSA suits the mechanism better is most accurate?

A. Patient traits includes younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI.

B. Endotype includes less collapsible airway; lower loop gain; higher arousal threshold; lower response to arousal.

C. Polysomnographic includes supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than.

D. Less collapsible airways, and the pharynx increases more along its whole length than in severe disease.

E. Clinical and anthropometric predictors alone do not reliably identify oral appliance responders.

CORRECT ANSWER D

Less collapsible airways, and the pharynx increases more along its whole length than in severe disease. The other options describe different mechanisms, investigations or findings rather than mild osa suits the mechanism better. Consult Integrated notebook, page 107, §5.2 for the full context.

REFERENCE Integrated notebook, page 107, §5.2

QUESTION 0478 · §5.2 · Clinical interpretation SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. The clinical team records the following finding: less collapsible airways, and the pharynx increases more along its whole length than in severe disease. Which concept best explains this observation?

A. The airway enlarges

B. Dose dependence extends beyond anatomy

C. Polysomnographic

D. Essential caveat

E. Mild OSA suits the mechanism better

CORRECT ANSWER E

This finding most directly describes Mild OSA suits the mechanism better: less collapsible airways, and the pharynx increases more along its whole length than in severe disease. Distractor distinctions: A refers instead to particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly; velopharyngeal, n; B refers instead to there is dose-dependent enlargement of the upper airway, augmentation of upper airway muscle activity and improvement in mini; C refers instead to supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant; D refers instead to patient factors alone are not a good predictor, and demographic, anthropometric and polysomnographic data only weakly inform.

REFERENCE Integrated notebook, page 107, §5.2

QUESTION 0479 · §5.1 · Clinical decision-making SBA

A patient seeks an appliance for 'primary snoring' without any medical assessment. What is required first?

A. AHI can be inferred from ESS

B. A physician must exclude OSA before the label and treatment pathway are confirmed

C. A dental photograph confirms the absence of OSA

D. The dentist may diagnose primary snoring from sound alone

E. A stock mouthguard is an adequate diagnostic test

CORRECT ANSWER B

The notebook requires a physician diagnosis or exclusion of OSA before treating even apparently simple habitual snoring.

REFERENCE Integrated notebook, Topic 5, §5.1

QUESTION 0480 · §5.3 · Applied clinical SBA

A patient with several crowns is being considered for mandibular advancement therapy. Which of the following statements about severe OSA is a relative contraindication only is most accurate?

A. Appliance impressions should reproduce teeth and adjacent gingiva accurately.

B. Mono/twin block includes overlays fused at the occlusal surface (mono) or mated by interlocking blocks (twin).

C. Adequate retention often requires at least six sound teeth per arch, although clinical requirements vary.

D. MAD treatment is an option for mild-to-moderate OSA and selected patients who cannot tolerate CPAP.

E. CPAP should be trialled first, but an appliance is appropriate if CPAP fails.

CORRECT ANSWER E

CPAP should be trialled first, but an appliance is appropriate if CPAP fails. The other options describe different mechanisms, investigations or findings rather than severe osa is a relative contraindication only. Consult Integrated notebook, page 119, §5.3 for the full context.

REFERENCE Integrated notebook, page 119, §5.3

QUESTION 0481 · §5.3 · Clinical interpretation SBA

A follow-up patient reports poor retention of a recently supplied oral appliance. The clinical team records the following finding: CPAP should be trialled first, but an appliance is appropriate if CPAP fails. Which concept best explains this observation?

A. Severe OSA is a relative contraindication only

B. Indications

C. Severe OSA should have an initial trial of nasal CPAP

D. Mild OSA suits the mechanism better

E. Use a custom, titratable appliance over non-custom devices

CORRECT ANSWER A

This finding most directly describes Severe OSA is a relative contraindication only: CPAP should be trialled first, but an appliance is appropriate if CPAP fails. Distractor distinctions: B refers instead to mild–moderate OSA as a primary option; moderate–severe OSA where CPAP cannot be tolerated or adhered to; C refers instead to greater effectiveness demonstrated. Upper airway surgery (tonsillectomy and adenoidectomy, craniofacial operations, tracheost; D refers instead to less collapsible airways, and the pharynx increases more along its whole length than in severe disease; E refers instead to Custom titratable devices reduce AHI, arousal index and ODI and increase oxygen saturation to a greater extent.

REFERENCE Integrated notebook, page 119, §5.3

QUESTION 0482 · §5.3 · Clinical decision-making SBA

A patient has advanced active periodontal disease with mobile anchor teeth. Which appliance decision is most appropriate?

A. Use maximal protrusion to splint teeth

B. Stabilise dental disease and reassess suitability before a tooth-retained MAD

C. Ignore anchorage because the appliance is removable

D. Treat as an ideal candidate without modification

E. Use no maintenance programme

CORRECT ANSWER B

Adequate periodontal support and retention are prerequisites for a predictable device; tooth mobility increases risk.

REFERENCE Integrated notebook, Topic 5, §5.3

QUESTION 0483 · §5.4 · Applied clinical SBA

A dentist and laboratory technician discuss the design of a custom titratable oral appliance. Which of the following statements about selection factors is most accurate?

A. Acrylic offers established mechanical performance, repairability and clinical retention.

B. Mono/twin block includes overlays fused at the occlusal surface (mono) or mated by interlocking blocks (twin).

C. Selection factors includes number of teeth; arch relationship (class II vs III); bruxism; TMD; gagging.

D. Traditional includes dental casts and bite registrations used to create custom appliances. Well established, allowing precise.

E. Lateral pull includes bilateral elastomeric or nylon straps pulling the mandibular overlay forward.

CORRECT ANSWER C

Selection factors includes number of teeth; arch relationship (class II vs III); bruxism; TMD; gagging. Additional revision detail: number of teeth; arch relationship (class II vs III); bruxism; TMD; gagging. The other options describe different mechanisms, investigations or findings rather than selection factors. Consult Integrated notebook, page 110, §5.4 for the full context.

REFERENCE Integrated notebook, page 110, §5.4

QUESTION 0484 · §5.5 · Clinical interpretation SBA

A follow-up patient reports poor retention of a recently supplied oral appliance. The clinical team records the following finding: assess and treat first. It does not itself treat the OSA but improves the treatment outcome. Which concept best explains this observation?

A. Arch relationship

B. Nasal patency

C. Dental factors

D. Mouth and tongue

E. Skeletal pattern

CORRECT ANSWER B

This finding most directly describes Nasal patency: assess and treat first. It does not itself treat the OSA but improves the treatment outcome. Distractor distinctions: A refers instead to class II vs III alters mechanics and the direction of unwanted movement. An anterior open bite precludes a MAD — no posterior; C refers instead to sufficient healthy teeth and a suitable bite are crucial; D refers instead to a TRD is an option for missing teeth or where jaw alignment precludes a MAD; E refers instead to severe maxillomandibular deformity may need specialised treatment; retrognathia favours an appliance and is a specific proble.

REFERENCE Integrated notebook, page 111, §5.5

QUESTION 0485 · §5.6 · Clinical decision-making SBA

Maximum measured protrusive range is 10 mm. A cautious initial construction bite is planned at 50% of range. What advancement is registered?

A. 10 mm

B. 0 mm regardless of symptoms

C. 15 mm

D. 2 mm

E. 5 mm from the reference position

CORRECT ANSWER E

The notebook recommends a commonly used initial target around 50–70% of measured maximum rather than starting at the end range.

REFERENCE Integrated notebook, Topic 5, §5.6

QUESTION 0486 · §5.5 · Applied clinical SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. Which of the following statements about nasal patency is most accurate?

A. Severity and positional dependence; adjustable MADs have higher response rates than fixed devices.

B. Existing dental or periodontal disease should be stabilised before appliance therapy.

C. An anterior adjustment mechanism may be easier to use for a patient with limited dexterity.

D. A TRD is an option for missing teeth or where jaw alignment precludes a MAD.

E. Sufficient healthy teeth and a suitable bite are crucial.

CORRECT ANSWER B

Existing dental or periodontal disease should be stabilised before appliance therapy. Additional revision detail: assess and treat first. It does not itself treat the OSA but improves the treatment outcome. The other options describe different mechanisms, investigations or findings rather than nasal patency. Consult Integrated notebook, page 111, §5.5 for the full context.

REFERENCE Integrated notebook, page 111, §5.5

QUESTION 0487 · §5.6 · Clinical interpretation SBA

A patient with several crowns is being considered for mandibular advancement therapy. The clinical team records the following finding: uses speech (phonation of particular sounds) to find a position that keeps the airway open without excessive protrusion or opening. Which concept best explains this observation?

A. Phonetic

B. Classical practice

C. Neuromuscular (TENS)

D. Horizontal (protrusion)

E. Lateral (midline)

CORRECT ANSWER A

This finding most directly describes Phonetic: uses speech (phonation of particular sounds) to find a position that keeps the airway open without excessive protrusion or opening. Distractor distinctions: B refers instead to occlusal registration at 60% of maximum mandibular protrusion with 2 mm of vertical sep- aration of the upper and lower incis; C refers instead to transcutaneous electrical nerve stimulation relaxes the muscles, allowing a relaxed physiological mandibular position before; D refers instead to the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comf; E refers instead to accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identi.

REFERENCE Integrated notebook, page 112, §5.6

QUESTION 0488 · §5.6 · Clinical decision-making SBA

A patient develops discomfort when the construction bite is held at near-maximum protrusion. What adjustment is best justified?

A. Abandon all treatment without discussing alternatives

B. Reduce toward a comfortable submaximal initial setting

C. Increase vertical opening without limit

D. Do not measure the range

E. Increase beyond maximum protrusion

CORRECT ANSWER B

Higher protrusion may improve airway patency but increases dental and joint loading; titration balances efficacy and tolerance.

REFERENCE Integrated notebook, Topic 5, §5.6

QUESTION 0489 · §5.6 · Applied clinical SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. Which of the following statements about phonetic is most accurate?

A. Phonetic registration estimates jaw position by observing speech sounds, but is not a standard MAD titration endpoint.

B. A construction bite should record a reproducible mandibular midline even when it deviates during protrusion.

C. A construction bite may be recorded at approximately 60% of maximum protrusion with about 2 mm vertical separation.

D. Clinical measurement and simulation tools includes bite gauges and mandibular positioning simulators for reproducibility.

E. Construction-bite vertical opening should allow appliance clearance without excessive muscular strain.

CORRECT ANSWER A

Phonetic registration estimates jaw position by observing speech sounds, but is not a standard MAD titration endpoint. Additional revision detail: uses speech (phonation of particular sounds) to find a position that keeps the airway open without excessive protrusion or opening. The other options describe different mechanisms, investigations or findings rather than phonetic. Consult Integrated notebook, page 112, §5.6 for the full context.

REFERENCE Integrated notebook, page 112, §5.6

QUESTION 0490 · §5.7 · Focused knowledge SBA

A dentist and laboratory technician discuss the design of a custom titratable oral appliance. Which of the following statements about requirements is most accurate?

A. Sufficient healthy teeth and a suitable bite are crucial.

B. Selection factors includes number of teeth; arch relationship (class II vs III); bruxism; TMD; gagging.

C. The acceptable range for an individual is not known.

D. Appliance impressions should reproduce teeth and adjacent gingiva accurately.

E. The clinical finding includes accuracy and detail reproduction (some materials to 20–25 µm).

CORRECT ANSWER E

The clinical finding includes accuracy and detail reproduction (some materials to 20–25 µm). Additional revision detail: accuracy and detail reproduction (some materials to 20–25 µm). The other options describe different mechanisms, investigations or findings rather than requirements. Consult Integrated notebook, page 112, §5.7 for the full context.

REFERENCE Integrated notebook, page 112, §5.7

QUESTION 0491 · §5.6 · Clinical decision-making SBA

Two otherwise equivalent devices differ mainly in vertical opening. Which choice aligns with the notebook?

A. Use an open-mouth appliance by default

B. Use the least vertical opening compatible with adequate function and fit

C. A 10 mm opening is routinely required

D. Vertical opening has no airway effect

E. Prefer maximal vertical opening in everyone

CORRECT ANSWER B

Increasing vertical dimension may worsen upper-airway mechanics and mouth breathing; minimal necessary opening is favoured.

REFERENCE Integrated notebook, Topic 5, §5.6

QUESTION 0492 · §5.7 · Applied clinical SBA

A follow-up patient reports poor retention of a recently supplied oral appliance. Which of the following statements about requirements is most accurate?

A. Appliance impressions should reproduce teeth and adjacent gingiva accurately.

B. The acceptable range for an individual is not known.

C. Sufficient healthy teeth and a suitable bite are crucial.

D. The clinical finding includes accuracy and detail reproduction (some materials to 20–25 µm).

E. Selection factors includes number of teeth; arch relationship (class II vs III); bruxism; TMD; gagging.

CORRECT ANSWER D

The clinical finding includes accuracy and detail reproduction (some materials to 20–25 µm). Additional revision detail: accuracy and detail reproduction (some materials to 20–25 µm). The other options describe different mechanisms, investigations or findings rather than requirements. Consult Integrated notebook, page 112, §5.7 for the full context.

REFERENCE Integrated notebook, page 112, §5.7

QUESTION 0493 · §5.8 · Clinical interpretation SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. The clinical team records the following finding: Specify the chosen design, impressions or scans, bite registration, advancement and vertical settings. Which concept best explains this observation?

A. Laboratory prescription contents

B. Nylon / polyamide (PA12), CAD-CAM

C. 3D printing / additive manufacturing

D. Mild OSA suits the mechanism better

E. Dose dependence extends beyond anatomy

CORRECT ANSWER A

This finding most directly describes Laboratory prescription contents: Specify the chosen design, impressions or scans, bite registration, advancement and vertical settings. Distractor distinctions: B refers instead to lighter, smaller, robust, less breakage so better for bruxers; retention by flexion into undercuts; needs only ~3 mm clearanc; C refers instead to digital models built layer by layer. High customisation, potentially reduced production time and cost, and in-house printing; D refers instead to less collapsible airways, and the pharynx increases more along its whole length than in severe disease; E refers instead to there is dose-dependent enlargement of the upper airway, augmentation of upper airway muscle activity and improvement in mini.

REFERENCE Integrated notebook, page 111, §5.8

QUESTION 0494 · §5.4 · Clinical decision-making SBA

An appliance cannot be adjusted after manufacture and has no verifiable setting. What is the key problem?

A. It removes the need for testing

B. It does not meet the preferred custom titratable design criteria

C. It automatically gives higher efficacy

D. It is inherently safer for every TMJ

E. It is the preferred guideline device for moderate OSA

CORRECT ANSWER B

Guidelines favour custom titratable devices capable of gradual measurable reversal and adjustment.

REFERENCE Integrated notebook, Topic 5, §5.4

QUESTION 0495 · §5.8 · Applied clinical SBA

A patient with several crowns is being considered for mandibular advancement therapy. Which of the following statements about laboratory prescription contents is most accurate?

A. Less collapsible airways, and the pharynx increases more along its whole length than in severe disease.

B. The clinician should assess complete seating, patient-removable retention, stability, comfort and demonstrated insertion/removal.

C. A laboratory prescription should specify the device design, construction bite and advancement settings.

D. MAD treatment is an option for mild-to-moderate OSA and selected patients who cannot tolerate CPAP.

E. Polysomnographic includes supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than.

CORRECT ANSWER C

A laboratory prescription should specify the device design, construction bite and advancement settings. Additional revision detail: Specify the chosen design, impressions or scans, bite registration, advancement and vertical settings. The other options describe different mechanisms, investigations or findings rather than laboratory prescription contents. Consult Integrated notebook, page 111, §5.8 for the full context.

REFERENCE Integrated notebook, page 111, §5.8

QUESTION 0496 · §5.9 · Clinical interpretation SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. The clinical team records the following finding: Assess complete seating, patient-removable retention, stability, comfort and demonstrated insertion/removal. Which concept best explains this observation?

A. Delivery check

B. Acrylic (PMMA)

C. Patient traits

D. Dental factors

E. Device instruction at fit

CORRECT ANSWER A

This finding most directly describes Delivery check: Assess complete seating, patient-removable retention, stability, comfort and demonstrated insertion/removal. Distractor distinctions: B refers instead to good mechanical properties, well established; retention by frictional resistance; can be relined and repaired; used in DLP an; C refers instead to younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI; D refers instead to sufficient healthy teeth and a suitable bite are crucial; E refers instead to Teach insertion, removal, cleaning, adjustment and the need for further appointments.

REFERENCE Integrated notebook, page 112, §5.9

QUESTION 0497 · §5.8 · Clinical decision-making SBA

A dental laboratory receives an appliance order without the protrusive registration, design or material. What is the best corrective action?

A. Proceed without a verified bite

B. Select an arbitrary vertical dimension

C. Ask the technician to guess maximum protrusion

D. Send a complete laboratory prescription with records, design, bite setting and material instructions

E. Avoid documenting allergies or dental limitations

CORRECT ANSWER D

Laboratory communication must specify material, design, adequate records and construction bite so the fabricated device matches clinical planning.

REFERENCE Integrated notebook, Topic 5, §5.8

QUESTION 0498 · §5.9 · Applied clinical SBA

A dentist and laboratory technician discuss the design of a custom titratable oral appliance. Which of the following statements about delivery check is most accurate?

A. Device instruction at fit includes teach insertion, removal, cleaning, adjustment and the need for further appointments.

B. Polysomnographic includes supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than.

C. A laboratory prescription should specify the device design, construction bite and advancement settings.

D. Custom titratable appliances provide better respiratory outcomes than non-custom oral devices.

E. The clinician should assess complete seating, patient-removable retention, stability, comfort and demonstrated insertion/removal.

CORRECT ANSWER E

The clinician should assess complete seating, patient-removable retention, stability, comfort and demonstrated insertion/removal. Additional revision detail: Assess complete seating, patient-removable retention, stability, comfort and demonstrated insertion/removal. The other options describe different mechanisms, investigations or findings rather than delivery check. Consult Integrated notebook, page 112, §5.9 for the full context.

REFERENCE Integrated notebook, page 112, §5.9

QUESTION 0499 · §5.10 · Clinical interpretation SBA

A follow-up patient reports poor retention of a recently supplied oral appliance. The clinical team records the following finding: 2 mm grey George Gauge fork (5 mm white fork usually too much); interincisal increase 2–4 mm; posterior clearance nylon ~3 mm, acrylic ~5 mm, some designs 6 mm. Which concept best explains this observation?

A. Cleaning

B. Follow-up

C. Impression

D. Dentition

E. Vertical

CORRECT ANSWER E

This finding most directly describes Vertical: 2 mm grey George Gauge fork (5 mm white fork usually too much); interincisal increase 2–4 mm; posterior clearance nylon ~3 mm, acrylic ~5 mm, some designs 6 mm. Distractor distinctions: A refers instead to Rinse immediately after removal. Clean each morning with a soft-bristled brush and mild soap or denture cleaner, using cool o; B refers instead to every 6 months in year 1, then at least annually; C refers instead to dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick; D refers instead to ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support).

REFERENCE Integrated notebook, page 128, §5.10

QUESTION 0500 · §5.9 · Clinical decision-making SBA

At delivery a mandibular device dislodges easily when the patient opens. What should the dentist assess first?

A. Refer only after several years

B. Tell the patient to bite harder overnight

C. Assume the device works during sleep

D. Increase protrusion automatically

E. Seating, retention, stability and whether adjustment/remake is needed

CORRECT ANSWER E

A therapeutic device must remain stable and retentive; fit is evaluated at delivery before efficacy titration.

REFERENCE Integrated notebook, Topic 5, §5.9

QUESTION 0501 · §5.10 · Applied clinical SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. Which of the following statements about vertical is most accurate?

A. Cleaning includes rinse immediately after removal.

B. Vertical: 2 mm grey George Gauge fork (5 mm white fork usually too much).

C. Adequate retention often requires at least six sound teeth per arch, although clinical requirements vary.

D. Appliance impressions should reproduce teeth and adjacent gingiva accurately.

E. Handling and storage includes remove over a soft surface or a basin of water.

CORRECT ANSWER B

Vertical: 2 mm grey George Gauge fork (5 mm white fork usually too much). Additional revision detail: 2 mm grey George Gauge fork (5 mm white fork usually too much); interincisal increase 2–4 mm; posterior clearance nylon ~3 mm, acrylic ~5 mm, some designs 6 mm. The other options describe different mechanisms, investigations or findings rather than vertical. Consult Integrated notebook, page 128, §5.10 for the full context.

REFERENCE Integrated notebook, page 128, §5.10

QUESTION 0502 · §5.1 · Clinical interpretation SBA

A patient with several crowns is being considered for mandibular advancement therapy. The clinical team records the following finding: greater effectiveness demonstrated. Upper airway surgery (tonsillectomy and adenoidectomy, craniofacial operations, tracheostomy) may also supersede appliance use where predicted to be highly effective. Which concept best explains this observation?

A. Nasopharyngeal assessment

B. Severe OSA should have an initial trial of nasal CPAP

C. Use a custom, titratable appliance over non-custom devices

D. Treatment objectives

E. Diagnosis first

CORRECT ANSWER B

This finding most directly describes Severe OSA should have an initial trial of nasal CPAP: greater effectiveness demonstrated. Upper airway surgery (tonsillectomy and adenoidectomy, craniofacial operations, tracheostomy) may also supersede appliance use where predicted to be highly effective. Distractor distinctions: A refers instead to nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oro- and laryngopharynx, with jaw thrust and Mülle; C refers instead to Custom titratable devices reduce AHI, arousal index and ODI and increase oxygen saturation to a greater extent; D refers instead to Primary snoring without OSA or UARS: reduce snoring to a subjectively acceptable level; E refers instead to Presence or absence of OSA must be determined before initiating treatment — to identify patients at risk from complications,.

REFERENCE Integrated notebook, page 106, §5.1

QUESTION 0503 · §5.10 · Clinical decision-making SBA

A patient reports cleaning a nylon appliance with hot water and abrasive paste. What advice is appropriate?

A. Use abrasive grit to improve retention

B. Store it permanently in direct sun

C. Use the device-specific approved cleaning and storage protocol, avoiding heat-related distortion

D. Boil it each morning regardless of material

E. Never inspect for wear or biofilm

CORRECT ANSWER C

Home care should protect material stability, hygiene and device fit; heat may deform thermoplastic components.

REFERENCE Integrated notebook, Topic 5, §5.10

QUESTION 0504 · §5.1 · Applied clinical SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. Which of the following statements about severe OSA should have an initial trial of nasal CPAP is most accurate?

A. OSA must be assessed before treating apparently simple snoring with an oral appliance.

B. Custom titratable appliances provide better respiratory outcomes than non-custom oral devices.

C. Nasolaryngoscopy assesses pharyngeal stenosis and the level of upper-airway obstruction.

D. Adequate retention often requires at least six sound teeth per arch, although clinical requirements vary.

E. CPAP is generally more efficacious than oral appliances for severe OSA and should be considered first.

CORRECT ANSWER E

CPAP is generally more efficacious than oral appliances for severe OSA and should be considered first. Additional revision detail: greater effectiveness demonstrated. Upper airway surgery (tonsillectomy and adenoidectomy, craniofacial operations, tracheostomy) may also supersede appliance use where predicted to be highly effective. The other options describe different mechanisms, investigations or findings rather than severe osa should have an initial trial of nasal cpap. Consult Integrated notebook, page 106, §5.1 for the full context.

REFERENCE Integrated notebook, page 106, §5.1

QUESTION 0505 · §5.2 · Clinical interpretation SBA

A dentist and laboratory technician discuss the design of a custom titratable oral appliance. The clinical team records the following finding: sufficient AHI reduction on an overnight test with a remotely controlled mandibular positioner — the best predictor available ("excellent"), against clinical/craniofacial prediction ("poor") and nasopharyngoscopy ("fair to…. Which concept best explains this observation?

A. Patient traits

B. Cephalometric

C. Endotype

D. Trial-based

E. Adverse

CORRECT ANSWER D

This finding most directly describes Trial-based: sufficient AHI reduction on an overnight test with a remotely controlled mandibular positioner — the best predictor available ("excellent"), against clinical/craniofacial prediction ("poor") and nasopharyngoscopy ("fair to…. Distractor distinctions: A refers instead to younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI; B refers instead to shorter soft palate; longer maxilla; lower hyoid and decreased hyoid-to-mandibular-plane distance; greater angle between cran; C refers instead to less collapsible airway; lower loop gain; higher arousal threshold; lower response to arousal; E refers instead to weight increase during treatment relates to failure. Differences between patients in the ability to advance the mandible modi.

REFERENCE Integrated notebook, page 108, §5.2

QUESTION 0506 · §5.2 · Applied clinical SBA

A patient with several crowns is being considered for mandibular advancement therapy. Which of the following statements about trial-based is most accurate?

A. Weight increase during treatment relates to failure.

B. Particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly.

C. A remotely controlled mandibular positioner during PSG may help predict MAD response.

D. Advancement reduces collapsibility dosedependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population.

E. Cephalometric includes shorter soft palate; longer maxilla.

CORRECT ANSWER C

A remotely controlled mandibular positioner during PSG may help predict MAD response. Additional revision detail: response to mandibular advancement during an overnight study using a remotely controlled mandibular positioner may help predict the response to subsequent oral appliance therapy. The other options describe different mechanisms, investigations or findings rather than trial-based. Consult Integrated notebook, page 108, §5.2 for the full context.

REFERENCE Integrated notebook, page 108, §5.2

QUESTION 0507 · §5.3 · Focused knowledge SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. Which of the following statements about indications is most accurate?

A. MAD treatment is an option for mild-to-moderate OSA and selected patients who cannot tolerate CPAP.

B. Less collapsible airways, and the pharynx increases more along its whole length than in severe disease.

C. A laboratory prescription should specify the device design, construction bite and advancement settings.

D. CPAP should be trialled first, but an appliance is appropriate if CPAP fails.

E. Polysomnographic includes supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than.

CORRECT ANSWER A

MAD treatment is an option for mild-to-moderate OSA and selected patients who cannot tolerate CPAP. Additional revision detail: mild–moderate OSA as a primary option; moderate–severe OSA where CPAP cannot be tolerated or adhered to. The other options describe different mechanisms, investigations or findings rather than indications. Consult Integrated notebook, page 108, §5.3 for the full context.

REFERENCE Integrated notebook, page 108, §5.3

QUESTION 0508 · §5.3 · Applied clinical SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. Which of the following statements about indications is most accurate?

A. MAD treatment is an option for mild-to-moderate OSA and selected patients who cannot tolerate CPAP.

B. CPAP should be trialled first, but an appliance is appropriate if CPAP fails.

C. A laboratory prescription should specify the device design, construction bite and advancement settings.

D. Less collapsible airways, and the pharynx increases more along its whole length than in severe disease.

E. Polysomnographic includes supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than.

CORRECT ANSWER A

MAD treatment is an option for mild-to-moderate OSA and selected patients who cannot tolerate CPAP. Additional revision detail: mild–moderate OSA as a primary option; moderate–severe OSA where CPAP cannot be tolerated or adhered to. The other options describe different mechanisms, investigations or findings rather than indications. Consult Integrated notebook, page 108, §5.3 for the full context.

REFERENCE Integrated notebook, page 108, §5.3

QUESTION 0509 · §5.5 · Clinical interpretation SBA

A follow-up patient reports poor retention of a recently supplied oral appliance. The clinical team records the following finding: class II vs III alters mechanics and the direction of unwanted movement. An anterior open bite precludes a MAD — no posterior clearance at all. Which concept best explains this observation?

A. Mouth and tongue

B. Patient preference

C. Arch relationship

D. Dental factors

E. Skeletal pattern

CORRECT ANSWER C

This finding most directly describes Arch relationship: class II vs III alters mechanics and the direction of unwanted movement. An anterior open bite precludes a MAD — no posterior clearance at all. Distractor distinctions: A refers instead to a TRD is an option for missing teeth or where jaw alignment precludes a MAD; B refers instead to perceived comfort, ease of use, durability, cost; D refers instead to sufficient healthy teeth and a suitable bite are crucial; E refers instead to severe maxillomandibular deformity may need specialised treatment; retrognathia favours an appliance and is a specific proble.

REFERENCE Integrated notebook, page 111, §5.5

QUESTION 0510 · §5.4 · Applied clinical SBA

A follow-up patient reports poor retention of a recently supplied oral appliance. Which of the following statements about mono/twin block is most accurate?

A. Mono/twin block includes overlays fused at the occlusal surface (mono) or mated by interlocking blocks (twin).

B. Selection factors includes number of teeth; arch relationship (class II vs III); bruxism; TMD; gagging.

C. Acrylic offers established mechanical performance, repairability and clinical retention.

D. Traditional includes dental casts and bite registrations used to create custom appliances. Well established, allowing precise.

E. Lateral pull includes bilateral elastomeric or nylon straps pulling the mandibular overlay forward.

CORRECT ANSWER A

Mono/twin block includes overlays fused at the occlusal surface (mono) or mated by interlocking blocks (twin). Additional revision detail: overlays fused at the occlusal surface (mono) or mated by interlocking blocks (twin). The other options describe different mechanisms, investigations or findings rather than mono/twin block. Consult Integrated notebook, page 109, §5.4 for the full context.

REFERENCE Integrated notebook, page 109, §5.4

QUESTION 0511 · §5.6 · Clinical interpretation SBA

A dentist and laboratory technician discuss the design of a custom titratable oral appliance. The clinical team records the following finding: the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comfort and therapeutic range. Do not exceed ~75% initially. Which concept best explains this observation?

A. Vertical (bite opening)

B. Horizontal (protrusion)

C. Construction bite devices

D. Why not maximum protrusion

E. Neuromuscular (TENS)

CORRECT ANSWER B

This finding most directly describes Horizontal (protrusion): the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comfort and therapeutic range. Do not exceed ~75% initially. Distractor distinctions: A refers instead to provides interocclusal space without discomfort or fatigue. ~2 mm interincisal (moderate 1–3 mm increase typical); C refers instead to mechanical devices giving precise control and measurement in three dimensions, with guides and rulers documenting advancement; D refers instead to The acceptable range for an individual is not known; E refers instead to transcutaneous electrical nerve stimulation relaxes the muscles, allowing a relaxed physiological mandibular position before.

REFERENCE Integrated notebook, page 111, §5.6

QUESTION 0512 · §5.5 · Applied clinical SBA

A dentist and laboratory technician discuss the design of a custom titratable oral appliance. Which of the following statements about arch relationship is most accurate?

A. Severe maxillomandibular deformity may need specialised treatment.

B. Existing dental or periodontal disease should be stabilised before appliance therapy.

C. An anterior adjustment mechanism may be easier to use for a patient with limited dexterity.

D. Severity and positional dependence; adjustable MADs have higher response rates than fixed devices.

E. Sagittal occlusal relationships influence the mechanics and retention of an oral appliance.

CORRECT ANSWER E

Sagittal occlusal relationships influence the mechanics and retention of an oral appliance. Additional revision detail: class II vs III alters mechanics and the direction of unwanted movement. An anterior open bite precludes a MAD — no posterior clearance at all. The other options describe different mechanisms, investigations or findings rather than arch relationship. Consult Integrated notebook, page 111, §5.5 for the full context.

REFERENCE Integrated notebook, page 111, §5.5

QUESTION 0513 · §5.9 · Clinical interpretation SBA

A dentist and laboratory technician discuss the design of a custom titratable oral appliance. The clinical team records the following finding: Teach insertion, removal, cleaning, adjustment and the need for further appointments. Which concept best explains this observation?

A. Delivery check

B. Between-device differences are clinically small in symptoms or AHI

C. Device instruction at fit

D. Construction bite devices

E. Nasopharyngeal assessment

CORRECT ANSWER C

This finding most directly describes Device instruction at fit: Teach insertion, removal, cleaning, adjustment and the need for further appointments. Distractor distinctions: A refers instead to Assess complete seating, patient-removable retention, stability, comfort and demonstrated insertion/removal; B refers instead to The influence of design on efficacy and side effects is almost unknown, and side effects are a product of protruding the mand; D refers instead to mechanical devices giving precise control and measurement in three dimensions, with guides and rulers documenting advancement; E refers instead to nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oro- and laryngopharynx, with jaw thrust and Mülle.

REFERENCE Integrated notebook, page 113, §5.9

QUESTION 0514 · §5.6 · Applied clinical SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. Which of the following statements about horizontal (protrusion) is most accurate?

A. A construction bite may be recorded at approximately 60% of maximum protrusion with about 2 mm vertical separation.

B. Construction-bite gauges provide reproducible measurements of mandibular protrusion and opening.

C. Transcutaneous electrical nerve stimulation can relax masticatory muscles before bite registration.

D. Mandibular protrusion is the primary adjustable variable in oral appliance therapy.

E. A construction bite should record a reproducible mandibular midline even when it deviates during protrusion.

CORRECT ANSWER D

Mandibular protrusion is the primary adjustable variable in oral appliance therapy. Additional revision detail: the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comfort and therapeutic range. Do not exceed ~75% initially. The other options describe different mechanisms, investigations or findings rather than horizontal (protrusion). Consult Integrated notebook, page 111, §5.6 for the full context.

REFERENCE Integrated notebook, page 111, §5.6

QUESTION 0515 · §5.10 · Clinical interpretation SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. The clinical team records the following finding: Brush and floss regularly. Morning routine: gentle jaw exercises — resting the chin on a fist, curling the tongue back to find the retruded position — and the morning occlusal guide for 15–20 minutes if…. Which concept best explains this observation?

A. Follow-up

B. Cleaning

C. Oral health

D. Dentition

E. Impression

CORRECT ANSWER C

This finding most directly describes Oral health: Brush and floss regularly. Morning routine: gentle jaw exercises — resting the chin on a fist, curling the tongue back to find the retruded position — and the morning occlusal guide for 15–20 minutes if…. Distractor distinctions: A refers instead to every 6 months in year 1, then at least annually; B refers instead to Rinse immediately after removal. Clean each morning with a soft-bristled brush and mild soap or denture cleaner, using cool o; D refers instead to ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support); E refers instead to dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick.

REFERENCE Integrated notebook, page 113, §5.10

QUESTION 0516 · §5.9 · Applied clinical SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. Which of the following statements about device instruction at fit is most accurate?

A. An anterior adjustment mechanism may be easier to use for a patient with limited dexterity.

B. Mono/twin block includes overlays fused at the occlusal surface (mono) or mated by interlocking blocks (twin).

C. Severity and positional dependence; adjustable MADs have higher response rates than fixed devices.

D. Device instruction at fit includes teach insertion, removal, cleaning, adjustment and the need for further appointments.

E. The clinician should assess complete seating, patient-removable retention, stability, comfort and demonstrated insertion/removal.

CORRECT ANSWER D

Device instruction at fit includes teach insertion, removal, cleaning, adjustment and the need for further appointments. Additional revision detail: Teach insertion, removal, cleaning, adjustment and the need for further appointments. The other options describe different mechanisms, investigations or findings rather than device instruction at fit. Consult Integrated notebook, page 113, §5.9 for the full context.

REFERENCE Integrated notebook, page 113, §5.9

QUESTION 0517 · §5.1 · Clinical interpretation SBA

A patient with several crowns is being considered for mandibular advancement therapy. The clinical team records the following finding: generally ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support); assess mobility, spacing, curve of Spee, restorative status. Which concept best explains this observation?

A. Diagnosis first

B. Nasopharyngeal assessment

C. Treatment objectives

D. Severe OSA should have an initial trial of nasal CPAP

E. Dentition

CORRECT ANSWER E

This finding most directly describes Dentition: generally ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support); assess mobility, spacing, curve of Spee, restorative status. Distractor distinctions: A refers instead to Presence or absence of OSA must be determined before initiating treatment — to identify patients at risk from complications,; B refers instead to nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oro- and laryngopharynx, with jaw thrust and Mülle; C refers instead to Primary snoring without OSA or UARS: reduce snoring to a subjectively acceptable level; D refers instead to greater effectiveness demonstrated. Upper airway surgery (tonsillectomy and adenoidectomy, craniofacial operations, tracheost.

REFERENCE Integrated notebook, page 107, §5.1

QUESTION 0518 · §5.10 · Applied clinical SBA

A patient with several crowns is being considered for mandibular advancement therapy. Which of the following statements about oral health is most accurate?

A. An initial mandibular advancement around 50% of maximum lies within the commonly advised starting range.

B. Handling and storage includes remove over a soft surface or a basin of water.

C. Oral health includes regular oral hygiene and the use of appropriate morning jaw exercises or a custom occlusal guide, when.

D. Vertical: 2 mm grey George Gauge fork (5 mm white fork usually too much).

E. Cleaning includes rinse immediately after removal.

CORRECT ANSWER C

Oral health includes regular oral hygiene and the use of appropriate morning jaw exercises or a custom occlusal guide, when. Additional revision detail: regular oral hygiene and the use of appropriate morning jaw exercises or a custom occlusal guide, when indicated, help support safe long-term appliance use. The other options describe different mechanisms, investigations or findings rather than oral health. Consult Integrated notebook, page 113, §5.10 for the full context.

REFERENCE Integrated notebook, page 113, §5.10

QUESTION 0519 · §5.2 · Focused knowledge SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. Which of the following statements about dose dependence extends beyond anatomy is most accurate?

A. Particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly.

B. Weight increase during treatment relates to failure.

C. Advancement reduces collapsibility dosedependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population.

D. Cephalometric includes shorter soft palate; longer maxilla.

E. Greater comfortable mandibular protrusion can improve airway patency but may increase adverse effects.

CORRECT ANSWER E

Greater comfortable mandibular protrusion can improve airway patency but may increase adverse effects. Additional revision detail: greater comfortable mandibular advancement can reduce upper-airway collapsibility and may improve oxygenation, but the balance with comfort and adverse effects must be monitored. The other options describe different mechanisms, investigations or findings rather than dose dependence extends beyond anatomy. Consult Integrated notebook, page 118, §5.2 for the full context.

REFERENCE Integrated notebook, page 118, §5.2

QUESTION 0520 · §5.1 · Applied clinical SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. Which of the following statements about dentition is most accurate?

A. OSA must be assessed before treating apparently simple snoring with an oral appliance.

B. Adequate retention often requires at least six sound teeth per arch, although clinical requirements vary.

C. CPAP is generally more efficacious than oral appliances for severe OSA and should be considered first.

D. Treatment objectives includes primary snoring without OSA or UARS: reduce snoring to a subjectively acceptable level.

E. Nasolaryngoscopy assesses pharyngeal stenosis and the level of upper-airway obstruction.

CORRECT ANSWER B

Adequate retention often requires at least six sound teeth per arch, although clinical requirements vary. Additional revision detail: generally ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support); assess mobility, spacing, curve of Spee, restorative status. The other options describe different mechanisms, investigations or findings rather than dentition. Consult Integrated notebook, page 107, §5.1 for the full context.

REFERENCE Integrated notebook, page 107, §5.1

QUESTION 0521 · §5.4 · Clinical interpretation SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. The clinical team records the following finding: dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible. Titrated by swapping an overlay for one with a different setting (e.g. +1 mm). Which concept best explains this observation?

A. Precision Post

B. Not recommended

C. Mono/Twin Block

D. Acrylic (PMMA)

E. Lateral Pull

CORRECT ANSWER A

This finding most directly describes Precision Post: dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible. Titrated by swapping an overlay for one with a different setting (e.g. +1 mm). Distractor distinctions: B refers instead to designs marketed on theories of airway and bone remodelling, promoted by manufacturers, with no peer- reviewed publication, n; C refers instead to overlays fused at the occlusal surface (mono) or mated by interlocking blocks (twin); D refers instead to good mechanical properties, well established; retention by frictional resistance; can be relined and repaired; used in DLP an; E refers instead to bilateral elastomeric or nylon straps pulling the mandibular overlay forward.

REFERENCE Integrated notebook, page 109, §5.4

QUESTION 0522 · §5.2 · Applied clinical SBA

A follow-up patient reports poor retention of a recently supplied oral appliance. Which of the following statements about dose dependence extends beyond anatomy is most accurate?

A. Greater comfortable mandibular protrusion can improve airway patency but may increase adverse effects.

B. Particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly.

C. Cephalometric includes shorter soft palate; longer maxilla.

D. Weight increase during treatment relates to failure.

E. Advancement reduces collapsibility dosedependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population.

CORRECT ANSWER A

Greater comfortable mandibular protrusion can improve airway patency but may increase adverse effects. Additional revision detail: greater comfortable mandibular advancement can reduce upper-airway collapsibility and may improve oxygenation, but the balance with comfort and adverse effects must be monitored. The other options describe different mechanisms, investigations or findings rather than dose dependence extends beyond anatomy. Consult Integrated notebook, page 118, §5.2 for the full context.

REFERENCE Integrated notebook, page 118, §5.2

QUESTION 0523 · §5.5 · Clinical interpretation SBA

A patient with several crowns is being considered for mandibular advancement therapy. The clinical team records the following finding: sufficient healthy teeth and a suitable bite are crucial. Which concept best explains this observation?

A. Nasal patency

B. Dental factors

C. Mouth and tongue

D. Skeletal pattern

E. Arch relationship

CORRECT ANSWER B

This finding most directly describes Dental factors: sufficient healthy teeth and a suitable bite are crucial. Distractor distinctions: A refers instead to assess and treat first. It does not itself treat the OSA but improves the treatment outcome; C refers instead to a TRD is an option for missing teeth or where jaw alignment precludes a MAD; D refers instead to severe maxillomandibular deformity may need specialised treatment; retrognathia favours an appliance and is a specific proble; E refers instead to class II vs III alters mechanics and the direction of unwanted movement. An anterior open bite precludes a MAD — no posterior.

REFERENCE Integrated notebook, page 111, §5.5

QUESTION 0524 · §5.4 · Applied clinical SBA

A patient with several crowns is being considered for mandibular advancement therapy. Which of the following statements about precision post is most accurate?

A. Interlocking dorsal includes angled dorsal posts from the mandibular overlay interlocking with maxillary blocks.

B. Evidence comparing oral appliance designs for relative efficacy and long-term adverse effects is limited.

C. Nylon / polyamide (pa12), cad-cam includes lighter, smaller, robust, less breakage so better for bruxers.

D. Additive manufacturing builds appliances layer by layer from digital models.

E. Precision post includes dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible.

CORRECT ANSWER E

Precision post includes dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible. Additional revision detail: dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible. Titrated by swapping an overlay for one with a different setting (e.g. +1 mm). The other options describe different mechanisms, investigations or findings rather than precision post. Consult Integrated notebook, page 109, §5.4 for the full context.

REFERENCE Integrated notebook, page 109, §5.4

QUESTION 0525 · §5.6 · Clinical interpretation SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. The clinical team records the following finding: bite gauges and mandibular positioning simulators for reproducibility. Digital/ virtual — scanning and CAD, potentially outperforming traditional methods in accuracy and efficiency. Which concept best explains this observation?

A. Why not maximum protrusion

B. Construction bite devices

C. Vertical (bite opening)

D. Clinical measurement and simulation tools

E. Horizontal (protrusion)

CORRECT ANSWER D

This finding most directly describes Clinical measurement and simulation tools: bite gauges and mandibular positioning simulators for reproducibility. Digital/ virtual — scanning and CAD, potentially outperforming traditional methods in accuracy and efficiency. Distractor distinctions: A refers instead to The acceptable range for an individual is not known; B refers instead to mechanical devices giving precise control and measurement in three dimensions, with guides and rulers documenting advancement; C refers instead to provides interocclusal space without discomfort or fatigue. ~2 mm interincisal (moderate 1–3 mm increase typical); E refers instead to the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comf.

REFERENCE Integrated notebook, page 112, §5.6

QUESTION 0526 · §5.5 · Applied clinical SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. Which of the following statements about dental factors is most accurate?

A. Sufficient healthy teeth and a suitable bite are crucial.

B. Severity and positional dependence; adjustable MADs have higher response rates than fixed devices.

C. An anterior adjustment mechanism may be easier to use for a patient with limited dexterity.

D. Patient preference includes perceived comfort, ease of use, durability, cost.

E. A TRD is an option for missing teeth or where jaw alignment precludes a MAD.

CORRECT ANSWER A

Sufficient healthy teeth and a suitable bite are crucial. The other options describe different mechanisms, investigations or findings rather than dental factors. Consult Integrated notebook, page 111, §5.5 for the full context.

REFERENCE Integrated notebook, page 111, §5.5

QUESTION 0527 · §5.10 · Clinical interpretation SBA

A dentist and laboratory technician discuss the design of a custom titratable oral appliance. The clinical team records the following finding: Remove over a soft surface or a basin of water. Air dry before placing in the ventilated case. Store dry in the case — never in tissue or loose in a pocket. Keep away from sunlight, heat, pets and children. Which concept best explains this observation?

A. Follow-up

B. Oral health

C. Impression

D. Handling and storage

E. Dentition

CORRECT ANSWER D

This finding most directly describes Handling and storage: Remove over a soft surface or a basin of water. Air dry before placing in the ventilated case. Store dry in the case — never in tissue or loose in a pocket. Keep away from sunlight, heat, pets and children. Distractor distinctions: A refers instead to every 6 months in year 1, then at least annually; B refers instead to Brush and floss regularly. Morning routine: gentle jaw exercises — resting the chin on a fist, curling the tongue back to fin; C refers instead to dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick; E refers instead to ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support).

REFERENCE Integrated notebook, page 113, §5.10

QUESTION 0528 · §5.6 · Applied clinical SBA

A follow-up patient reports poor retention of a recently supplied oral appliance. Which of the following statements about clinical measurement and simulation tools is most accurate?

A. Mandibular protrusion is the primary adjustable variable in oral appliance therapy.

B. Clinical measurement and simulation tools includes bite gauges and mandibular positioning simulators for reproducibility.

C. Construction-bite gauges provide reproducible measurements of mandibular protrusion and opening.

D. A construction bite should record a reproducible mandibular midline even when it deviates during protrusion.

E. Transcutaneous electrical nerve stimulation can relax masticatory muscles before bite registration.

CORRECT ANSWER B

Clinical measurement and simulation tools includes bite gauges and mandibular positioning simulators for reproducibility. Additional revision detail: bite gauges and mandibular positioning simulators for reproducibility. Digital/ virtual — scanning and CAD, potentially outperforming traditional methods in accuracy and efficiency. The other options describe different mechanisms, investigations or findings rather than clinical measurement and simulation tools. Consult Integrated notebook, page 112, §5.6 for the full context.

REFERENCE Integrated notebook, page 112, §5.6

QUESTION 0529 · §5.1 · Clinical interpretation SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. The clinical team records the following finding: Primary snoring without OSA or UARS: reduce snoring to a subjectively acceptable level. Which concept best explains this observation?

A. Treatment objectives

B. Nasopharyngeal assessment

C. Severe OSA should have an initial trial of nasal CPAP

D. Diagnosis first

E. Dentition

CORRECT ANSWER A

This finding most directly describes Treatment objectives: Primary snoring without OSA or UARS: reduce snoring to a subjectively acceptable level. Distractor distinctions: B refers instead to nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oro- and laryngopharynx, with jaw thrust and Mülle; C refers instead to greater effectiveness demonstrated. Upper airway surgery (tonsillectomy and adenoidectomy, craniofacial operations, tracheost; D refers instead to Presence or absence of OSA must be determined before initiating treatment — to identify patients at risk from complications,; E refers instead to ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support).

REFERENCE Integrated notebook, page 106, §5.1

QUESTION 0530 · §5.10 · Applied clinical SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. Which of the following statements about handling and storage is most accurate?

A. Cleaning includes rinse immediately after removal.

B. Vertical: 2 mm grey George Gauge fork (5 mm white fork usually too much).

C. Handling and storage includes remove over a soft surface or a basin of water.

D. Adequate retention often requires at least six sound teeth per arch, although clinical requirements vary.

E. An initial mandibular advancement around 50% of maximum lies within the commonly advised starting range.

CORRECT ANSWER C

Handling and storage includes remove over a soft surface or a basin of water. Additional revision detail: Remove over a soft surface or a basin of water. Air dry before placing in the ventilated case. Store dry in the case — never in tissue or loose in a pocket. Keep away from sunlight, heat, pets and children. The other options describe different mechanisms, investigations or findings rather than handling and storage. Consult Integrated notebook, page 113, §5.10 for the full context.

REFERENCE Integrated notebook, page 113, §5.10

QUESTION 0531 · §5.4 · Clinical interpretation SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. The clinical team records the following finding: The influence of design on efficacy and side effects is almost unknown, and side effects are a product of protruding the mandible to achieve a therapeutic effect rather than of specific design. Which concept best explains this observation?

A. Selection factors

B. Between-device differences are clinically small in symptoms or AHI

C. 3D printing / additive manufacturing

D. Nylon / polyamide (PA12), CAD-CAM

E. Interlocking Dorsal

CORRECT ANSWER B

This finding most directly describes Between-device differences are clinically small in symptoms or AHI: The influence of design on efficacy and side effects is almost unknown, and side effects are a product of protruding the mandible to achieve a therapeutic effect rather than of specific design. Distractor distinctions: A refers instead to number of teeth; arch relationship (class II vs III); bruxism; TMD; gagging; C refers instead to digital models built layer by layer. High customisation, potentially reduced production time and cost, and in-house printing; D refers instead to lighter, smaller, robust, less breakage so better for bruxers; retention by flexion into undercuts; needs only ~3 mm clearanc; E refers instead to angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10.

REFERENCE Integrated notebook, page 110, §5.4

QUESTION 0532 · §5.1 · Applied clinical SBA

A patient with several crowns is being considered for mandibular advancement therapy. Which of the following statements about treatment objectives is most accurate?

A. Custom titratable appliances provide better respiratory outcomes than non-custom oral devices.

B. Adequate retention often requires at least six sound teeth per arch, although clinical requirements vary.

C. OSA must be assessed before treating apparently simple snoring with an oral appliance.

D. Treatment objectives includes primary snoring without OSA or UARS: reduce snoring to a subjectively acceptable level.

E. Nasolaryngoscopy assesses pharyngeal stenosis and the level of upper-airway obstruction.

CORRECT ANSWER D

Treatment objectives includes primary snoring without OSA or UARS: reduce snoring to a subjectively acceptable level. Additional revision detail: Primary snoring without OSA or UARS: reduce snoring to a subjectively acceptable level. The other options describe different mechanisms, investigations or findings rather than treatment objectives. Consult Integrated notebook, page 106, §5.1 for the full context.

REFERENCE Integrated notebook, page 106, §5.1

QUESTION 0533 · §5.5 · Clinical interpretation SBA

A patient with several crowns is being considered for mandibular advancement therapy. The clinical team records the following finding: perceived comfort, ease of use, durability, cost. Which concept best explains this observation?

A. Patient preference

B. Sleep study findings

C. Arch relationship

D. Skeletal pattern

E. Mouth and tongue

CORRECT ANSWER A

This finding most directly describes Patient preference: perceived comfort, ease of use, durability, cost. Distractor distinctions: B refers instead to severity and positional dependence; adjustable MADs have higher response rates than fixed devices; C refers instead to class II vs III alters mechanics and the direction of unwanted movement. An anterior open bite precludes a MAD — no posterior; D refers instead to severe maxillomandibular deformity may need specialised treatment; retrognathia favours an appliance and is a specific proble; E refers instead to a TRD is an option for missing teeth or where jaw alignment precludes a MAD.

REFERENCE Integrated notebook, page 111, §5.5

QUESTION 0534 · §5.4 · Applied clinical SBA

A patient with several crowns is being considered for mandibular advancement therapy. Which of the following statements about between-device differences are clinically small in symptoms or AHI is most accurate?

A. Evidence comparing oral appliance designs for relative efficacy and long-term adverse effects is limited.

B. Devices lacking peer-reviewed efficacy evidence should not displace established custom titratable appliances.

C. Precision post includes dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible.

D. Interlocking dorsal includes angled dorsal posts from the mandibular overlay interlocking with maxillary blocks.

E. Traditional includes dental casts and bite registrations used to create custom appliances. Well established, allowing precise.

CORRECT ANSWER A

Evidence comparing oral appliance designs for relative efficacy and long-term adverse effects is limited. Additional revision detail: The influence of design on efficacy and side effects is almost unknown, and side effects are a product of protruding the mandible to achieve a therapeutic effect rather than of specific design. The other options describe different mechanisms, investigations or findings rather than between-device differences are clinically small in symptoms or ahi. Consult Integrated notebook, page 110, §5.4 for the full context.

REFERENCE Integrated notebook, page 110, §5.4

QUESTION 0535 · §5.6 · Clinical interpretation SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. The clinical team records the following finding: transcutaneous electrical nerve stimulation relaxes the muscles, allowing a relaxed physiological mandibular position before recording, reducing nociceptive input and potentially improving accuracy. Which concept best explains this observation?

A. Vertical (bite opening)

B. Horizontal (protrusion)

C. Classical practice

D. Neuromuscular (TENS)

E. Lateral (midline)

CORRECT ANSWER D

This finding most directly describes Neuromuscular (TENS): transcutaneous electrical nerve stimulation relaxes the muscles, allowing a relaxed physiological mandibular position before recording, reducing nociceptive input and potentially improving accuracy. Distractor distinctions: A refers instead to provides interocclusal space without discomfort or fatigue. ~2 mm interincisal (moderate 1–3 mm increase typical); B refers instead to the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comf; C refers instead to occlusal registration at 60% of maximum mandibular protrusion with 2 mm of vertical sep- aration of the upper and lower incis; E refers instead to accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identi.

REFERENCE Integrated notebook, page 112, §5.6

QUESTION 0536 · §5.5 · Applied clinical SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. Which of the following statements about patient preference is most accurate?

A. An anterior adjustment mechanism may be easier to use for a patient with limited dexterity.

B. Sufficient healthy teeth and a suitable bite are crucial.

C. Patient preference includes perceived comfort, ease of use, durability, cost.

D. A TRD is an option for missing teeth or where jaw alignment precludes a MAD.

E. Existing dental or periodontal disease should be stabilised before appliance therapy.

CORRECT ANSWER C

Patient preference includes perceived comfort, ease of use, durability, cost. Additional revision detail: perceived comfort, ease of use, durability, cost. The other options describe different mechanisms, investigations or findings rather than patient preference. Consult Integrated notebook, page 111, §5.5 for the full context.

REFERENCE Integrated notebook, page 111, §5.5

QUESTION 0537 · §5.10 · Clinical interpretation SBA

A dentist and laboratory technician discuss the design of a custom titratable oral appliance. The clinical team records the following finding: dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick. Which concept best explains this observation?

A. Follow-up

B. Impression

C. Oral health

D. Cleaning

E. Dentition

CORRECT ANSWER B

This finding most directly describes Impression: dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick. Distractor distinctions: A refers instead to every 6 months in year 1, then at least annually; C refers instead to Brush and floss regularly. Morning routine: gentle jaw exercises — resting the chin on a fist, curling the tongue back to fin; D refers instead to Rinse immediately after removal. Clean each morning with a soft-bristled brush and mild soap or denture cleaner, using cool o; E refers instead to ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support).

REFERENCE Integrated notebook, page 113, §5.10

QUESTION 0538 · §5.6 · Applied clinical SBA

A follow-up patient reports poor retention of a recently supplied oral appliance. Which of the following statements about neuromuscular (tens) is most accurate?

A. Mandibular protrusion is the primary adjustable variable in oral appliance therapy.

B. Clinical measurement and simulation tools includes bite gauges and mandibular positioning simulators for reproducibility.

C. Construction-bite gauges provide reproducible measurements of mandibular protrusion and opening.

D. A construction bite should record a reproducible mandibular midline even when it deviates during protrusion.

E. Transcutaneous electrical nerve stimulation can relax masticatory muscles before bite registration.

CORRECT ANSWER E

Transcutaneous electrical nerve stimulation can relax masticatory muscles before bite registration. Additional revision detail: transcutaneous electrical nerve stimulation relaxes the muscles, allowing a relaxed physiological mandibular position before recording, reducing nociceptive input and potentially improving accuracy. The other options describe different mechanisms, investigations or findings rather than neuromuscular (tens). Consult Integrated notebook, page 112, §5.6 for the full context.

REFERENCE Integrated notebook, page 112, §5.6

QUESTION 0539 · §5.1 · Clinical interpretation SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. The clinical team records the following finding: nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oroand laryngopharynx, with jaw thrust and Müller manoeuvre. Which concept best explains this observation?

A. Diagnosis first

B. Dentition

C. Nasopharyngeal assessment

D. Severe OSA should have an initial trial of nasal CPAP

E. Treatment objectives

CORRECT ANSWER C

This finding most directly describes Nasopharyngeal assessment: nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oro- and laryngopharynx, with jaw thrust and Müller manoeuvre. Distractor distinctions: A refers instead to Presence or absence of OSA must be determined before initiating treatment — to identify patients at risk from complications,; B refers instead to ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support); D refers instead to greater effectiveness demonstrated. Upper airway surgery (tonsillectomy and adenoidectomy, craniofacial operations, tracheost; E refers instead to Primary snoring without OSA or UARS: reduce snoring to a subjectively acceptable level.

REFERENCE Integrated notebook, page 107, §5.1

QUESTION 0540 · §5.10 · Applied clinical SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. Which of the following statements about impression is most accurate?

A. Pterygomandibular raphe present in ~64% of people includes absence predicts better response.

B. Adequate retention often requires at least six sound teeth per arch, although clinical requirements vary.

C. An initial mandibular advancement around 50% of maximum lies within the commonly advised starting range.

D. Long-term oral appliance care requires at least annual dental review after the initial follow-up period.

E. Appliance impressions should reproduce teeth and adjacent gingiva accurately.

CORRECT ANSWER E

Appliance impressions should reproduce teeth and adjacent gingiva accurately. Additional revision detail: dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick. The other options describe different mechanisms, investigations or findings rather than impression. Consult Integrated notebook, page 113, §5.10 for the full context.

REFERENCE Integrated notebook, page 113, §5.10

QUESTION 0541 · §5.2 · Clinical interpretation SBA

A patient with several crowns is being considered for mandibular advancement therapy. The clinical team records the following finding: supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant. Which concept best explains this observation?

A. Polysomnographic

B. Patient traits

C. Cephalometric

D. Essential caveat

E. The airway enlarges

CORRECT ANSWER A

This finding most directly describes Polysomnographic: supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant. Distractor distinctions: B refers instead to younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI; C refers instead to shorter soft palate; longer maxilla; lower hyoid and decreased hyoid-to-mandibular-plane distance; greater angle between cran; D refers instead to patient factors alone are not a good predictor, and demographic, anthropometric and polysomnographic data only weakly inform; E refers instead to particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly; velopharyngeal, n.

REFERENCE Integrated notebook, page 108, §5.2

QUESTION 0542 · §5.1 · Applied clinical SBA

A patient with several crowns is being considered for mandibular advancement therapy. Which of the following statements about nasopharyngeal assessment is most accurate?

A. OSA must be assessed before treating apparently simple snoring with an oral appliance.

B. Nasolaryngoscopy assesses pharyngeal stenosis and the level of upper-airway obstruction.

C. Custom titratable appliances provide better respiratory outcomes than non-custom oral devices.

D. Treatment objectives includes primary snoring without OSA or UARS: reduce snoring to a subjectively acceptable level.

E. Adequate retention often requires at least six sound teeth per arch, although clinical requirements vary.

CORRECT ANSWER B

Nasolaryngoscopy assesses pharyngeal stenosis and the level of upper-airway obstruction. Additional revision detail: nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oroand laryngopharynx, with jaw thrust and Müller manoeuvre. The other options describe different mechanisms, investigations or findings rather than nasopharyngeal assessment. Consult Integrated notebook, page 107, §5.1 for the full context.

REFERENCE Integrated notebook, page 107, §5.1

QUESTION 0543 · §5.4 · Focused knowledge SBA

A dentist and laboratory technician discuss the design of a custom titratable oral appliance. Which of the following statements about not recommended is most accurate?

A. Interlocking dorsal includes angled dorsal posts from the mandibular overlay interlocking with maxillary blocks.

B. Devices lacking peer-reviewed efficacy evidence should not displace established custom titratable appliances.

C. Nylon / polyamide (pa12), cad-cam includes lighter, smaller, robust, less breakage so better for bruxers.

D. Precision post includes dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible.

E. Evidence comparing oral appliance designs for relative efficacy and long-term adverse effects is limited.

CORRECT ANSWER B

Devices lacking peer-reviewed efficacy evidence should not displace established custom titratable appliances. Additional revision detail: designs marketed on theories of airway and bone remodelling, promoted by manufacturers, with no peerreviewed publication, no demonstrated superiority, and non-adherence to AADSM guidance — typically bulky and not titratable. The other options describe different mechanisms, investigations or findings rather than not recommended. Consult Integrated notebook, page 110, §5.4 for the full context.

REFERENCE Integrated notebook, page 110, §5.4

QUESTION 0544 · §5.2 · Applied clinical SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. Which of the following statements about polysomnographic is most accurate?

A. Endotype includes less collapsible airway; lower loop gain; higher arousal threshold; lower response to arousal.

B. Polysomnographic includes supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than.

C. Patient traits includes younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI.

D. Clinical and anthropometric predictors alone do not reliably identify oral appliance responders.

E. Less collapsible airways, and the pharynx increases more along its whole length than in severe disease.

CORRECT ANSWER B

Polysomnographic includes supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than. Additional revision detail: supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant. The other options describe different mechanisms, investigations or findings rather than polysomnographic. Consult Integrated notebook, page 108, §5.2 for the full context.

REFERENCE Integrated notebook, page 108, §5.2

QUESTION 0545 · §5.5 · Clinical interpretation SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. The clinical team records the following finding: severe maxillomandibular deformity may need specialised treatment; retrognathia favours an appliance and is a specific problem for CPAP because of mask leakage. Which concept best explains this observation?

A. Arch relationship

B. Mouth and tongue

C. Skeletal pattern

D. Patient preference

E. Dental factors

CORRECT ANSWER C

This finding most directly describes Skeletal pattern: severe maxillomandibular deformity may need specialised treatment; retrognathia favours an appliance and is a specific problem for CPAP because of mask leakage. Distractor distinctions: A refers instead to class II vs III alters mechanics and the direction of unwanted movement. An anterior open bite precludes a MAD — no posterior; B refers instead to a TRD is an option for missing teeth or where jaw alignment precludes a MAD; D refers instead to perceived comfort, ease of use, durability, cost; E refers instead to sufficient healthy teeth and a suitable bite are crucial.

REFERENCE Integrated notebook, page 111, §5.5

QUESTION 0546 · §5.4 · Applied clinical SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. Which of the following statements about not recommended is most accurate?

A. Precision post includes dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible.

B. Evidence comparing oral appliance designs for relative efficacy and long-term adverse effects is limited.

C. Devices lacking peer-reviewed efficacy evidence should not displace established custom titratable appliances.

D. Nylon / polyamide (pa12), cad-cam includes lighter, smaller, robust, less breakage so better for bruxers.

E. Interlocking dorsal includes angled dorsal posts from the mandibular overlay interlocking with maxillary blocks.

CORRECT ANSWER C

Devices lacking peer-reviewed efficacy evidence should not displace established custom titratable appliances. Additional revision detail: designs marketed on theories of airway and bone remodelling, promoted by manufacturers, with no peerreviewed publication, no demonstrated superiority, and non-adherence to AADSM guidance — typically bulky and not titratable. The other options describe different mechanisms, investigations or findings rather than not recommended. Consult Integrated notebook, page 110, §5.4 for the full context.

REFERENCE Integrated notebook, page 110, §5.4

QUESTION 0547 · §5.6 · Clinical interpretation SBA

A patient with several crowns is being considered for mandibular advancement therapy. The clinical team records the following finding: accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible. Which concept best explains this observation?

A. Horizontal (protrusion)

B. Vertical (bite opening)

C. Lateral (midline)

D. Neuromuscular (TENS)

E. Classical practice

CORRECT ANSWER C

This finding most directly describes Lateral (midline): accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible. Distractor distinctions: A refers instead to the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comf; B refers instead to provides interocclusal space without discomfort or fatigue. ~2 mm interincisal (moderate 1–3 mm increase typical); D refers instead to transcutaneous electrical nerve stimulation relaxes the muscles, allowing a relaxed physiological mandibular position before; E refers instead to occlusal registration at 60% of maximum mandibular protrusion with 2 mm of vertical sep- aration of the upper and lower incis.

REFERENCE Integrated notebook, page 111, §5.6

QUESTION 0548 · §5.5 · Applied clinical SBA

A patient with several crowns is being considered for mandibular advancement therapy. Which of the following statements about skeletal pattern is most accurate?

A. An anterior adjustment mechanism may be easier to use for a patient with limited dexterity.

B. Existing dental or periodontal disease should be stabilised before appliance therapy.

C. Severe maxillomandibular deformity may need specialised treatment.

D. Sagittal occlusal relationships influence the mechanics and retention of an oral appliance.

E. Severity and positional dependence; adjustable MADs have higher response rates than fixed devices.

CORRECT ANSWER C

Severe maxillomandibular deformity may need specialised treatment. Additional revision detail: severe maxillomandibular deformity may need specialised treatment; retrognathia favours an appliance and is a specific problem for CPAP because of mask leakage. The other options describe different mechanisms, investigations or findings rather than skeletal pattern. Consult Integrated notebook, page 111, §5.5 for the full context.

REFERENCE Integrated notebook, page 111, §5.5

QUESTION 0549 · §5.1 · Clinical interpretation SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. The clinical team records the following finding: Custom titratable devices reduce AHI, arousal index and ODI and increase oxygen saturation to a greater extent. Which concept best explains this observation?

A. Diagnosis first

B. Treatment objectives

C. Nasopharyngeal assessment

D. Severe OSA should have an initial trial of nasal CPAP

E. Use a custom, titratable appliance over non-custom devices

CORRECT ANSWER E

This finding most directly describes Use a custom, titratable appliance over non-custom devices: Custom titratable devices reduce AHI, arousal index and ODI and increase oxygen saturation to a greater extent. Distractor distinctions: A refers instead to Presence or absence of OSA must be determined before initiating treatment — to identify patients at risk from complications,; B refers instead to Primary snoring without OSA or UARS: reduce snoring to a subjectively acceptable level; C refers instead to nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oro- and laryngopharynx, with jaw thrust and Mülle; D refers instead to greater effectiveness demonstrated. Upper airway surgery (tonsillectomy and adenoidectomy, craniofacial operations, tracheost.

REFERENCE Integrated notebook, page 106, §5.1

QUESTION 0550 · §5.6 · Applied clinical SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. Which of the following statements about lateral (midline) is most accurate?

A. Clinical measurement and simulation tools includes bite gauges and mandibular positioning simulators for reproducibility.

B. Mandibular protrusion is the primary adjustable variable in oral appliance therapy.

C. Phonetic registration estimates jaw position by observing speech sounds, but is not a standard MAD titration endpoint.

D. A construction bite may be recorded at approximately 60% of maximum protrusion with about 2 mm vertical separation.

E. A construction bite should record a reproducible mandibular midline even when it deviates during protrusion.

CORRECT ANSWER E

A construction bite should record a reproducible mandibular midline even when it deviates during protrusion. Additional revision detail: accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible. The other options describe different mechanisms, investigations or findings rather than lateral (midline). Consult Integrated notebook, page 111, §5.6 for the full context.

REFERENCE Integrated notebook, page 111, §5.6

QUESTION 0551 · §5.2 · Clinical interpretation SBA

A patient with several crowns is being considered for mandibular advancement therapy. The clinical team records the following finding: particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly; velopharyngeal, nasopharyngeal and anteroposterior dimensions all increase. Which concept best explains this observation?

A. Patient traits

B. Mild OSA suits the mechanism better

C. The airway enlarges

D. Polysomnographic

E. Essential caveat

CORRECT ANSWER C

This finding most directly describes The airway enlarges: particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly; velopharyngeal, nasopharyngeal and anteroposterior dimensions all increase. Distractor distinctions: A refers instead to younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI; B refers instead to less collapsible airways, and the pharynx increases more along its whole length than in severe disease; D refers instead to supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant; E refers instead to patient factors alone are not a good predictor, and demographic, anthropometric and polysomnographic data only weakly inform.

REFERENCE Integrated notebook, page 107, §5.2

QUESTION 0552 · §5.1 · Applied clinical SBA

A patient with several crowns is being considered for mandibular advancement therapy. Which of the following statements about use a custom, titratable appliance over non-custom devices is most accurate?

A. OSA must be assessed before treating apparently simple snoring with an oral appliance.

B. Custom titratable appliances provide better respiratory outcomes than non-custom oral devices.

C. Nasolaryngoscopy assesses pharyngeal stenosis and the level of upper-airway obstruction.

D. Treatment objectives includes primary snoring without OSA or UARS: reduce snoring to a subjectively acceptable level.

E. Adequate retention often requires at least six sound teeth per arch, although clinical requirements vary.

CORRECT ANSWER B

Custom titratable appliances provide better respiratory outcomes than non-custom oral devices. Additional revision detail: Custom titratable devices reduce AHI, arousal index and ODI and increase oxygen saturation to a greater extent. The other options describe different mechanisms, investigations or findings rather than use a custom, titratable appliance over non-custom devices. Consult Integrated notebook, page 106, §5.1 for the full context.

REFERENCE Integrated notebook, page 106, §5.1

QUESTION 0553 · §5.4 · Clinical interpretation SBA

A dentist and laboratory technician discuss the design of a custom titratable oral appliance. The clinical team records the following finding: good mechanical properties, well established; retention by frictional resistance; can be relined and repaired; used in DLP and SLA printing. Which concept best explains this observation?

A. Lateral Pull

B. Not recommended

C. Acrylic (PMMA)

D. Mono/Twin Block

E. Precision Post

CORRECT ANSWER C

This finding most directly describes Acrylic (PMMA): good mechanical properties, well established; retention by frictional resistance; can be relined and repaired; used in DLP and SLA printing. Distractor distinctions: A refers instead to bilateral elastomeric or nylon straps pulling the mandibular overlay forward; B refers instead to designs marketed on theories of airway and bone remodelling, promoted by manufacturers, with no peer- reviewed publication, n; D refers instead to overlays fused at the occlusal surface (mono) or mated by interlocking blocks (twin); E refers instead to dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible. Titrated by swapp.

REFERENCE Integrated notebook, page 110, §5.4

QUESTION 0554 · §5.2 · Applied clinical SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. Which of the following statements about the airway enlarges is most accurate?

A. Weight increase during treatment relates to failure.

B. Clinical and anthropometric predictors alone do not reliably identify oral appliance responders.

C. Cephalometric includes shorter soft palate; longer maxilla.

D. Particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly.

E. Advancement reduces collapsibility dosedependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population.

CORRECT ANSWER D

Particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly. Additional revision detail: particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly; velopharyngeal, nasopharyngeal and anteroposterior dimensions all increase. The other options describe different mechanisms, investigations or findings rather than the airway enlarges. Consult Integrated notebook, page 107, §5.2 for the full context.

REFERENCE Integrated notebook, page 107, §5.2

QUESTION 0555 · §5.5 · Clinical interpretation SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. The clinical team records the following finding: a TRD is an option for missing teeth or where jaw alignment precludes a MAD. Which concept best explains this observation?

A. Mouth and tongue

B. Arch relationship

C. Patient preference

D. Dental factors

E. Skeletal pattern

CORRECT ANSWER A

This finding most directly describes Mouth and tongue: a TRD is an option for missing teeth or where jaw alignment precludes a MAD. Distractor distinctions: B refers instead to class II vs III alters mechanics and the direction of unwanted movement. An anterior open bite precludes a MAD — no posterior; C refers instead to perceived comfort, ease of use, durability, cost; D refers instead to sufficient healthy teeth and a suitable bite are crucial; E refers instead to severe maxillomandibular deformity may need specialised treatment; retrognathia favours an appliance and is a specific proble.

REFERENCE Integrated notebook, page 111, §5.5

QUESTION 0556 · §5.4 · Applied clinical SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. Which of the following statements about acrylic (pmma) is most accurate?

A. Precision post includes dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible.

B. Acrylic offers established mechanical performance, repairability and clinical retention.

C. Additive manufacturing builds appliances layer by layer from digital models.

D. Traditional includes dental casts and bite registrations used to create custom appliances. Well established, allowing precise.

E. Nylon / polyamide (pa12), cad-cam includes lighter, smaller, robust, less breakage so better for bruxers.

CORRECT ANSWER B

Acrylic offers established mechanical performance, repairability and clinical retention. Additional revision detail: good mechanical properties, well established; retention by frictional resistance; can be relined and repaired; used in DLP and SLA printing. The other options describe different mechanisms, investigations or findings rather than acrylic (pmma). Consult Integrated notebook, page 110, §5.4 for the full context.

REFERENCE Integrated notebook, page 110, §5.4

QUESTION 0557 · §5.6 · Clinical interpretation SBA

A patient with several crowns is being considered for mandibular advancement therapy. The clinical team records the following finding: mechanical devices giving precise control and measurement in three dimensions, with guides and rulers documenting advancement and vertical opening numerically. The George Gauge is the archetype. Which concept best explains this observation?

A. Neuromuscular (TENS)

B. Why not maximum protrusion

C. Vertical (bite opening)

D. Construction bite devices

E. Horizontal (protrusion)

CORRECT ANSWER D

This finding most directly describes Construction bite devices: mechanical devices giving precise control and measurement in three dimensions, with guides and rulers documenting advancement and vertical opening numerically. The George Gauge is the archetype. Distractor distinctions: A refers instead to transcutaneous electrical nerve stimulation relaxes the muscles, allowing a relaxed physiological mandibular position before; B refers instead to The acceptable range for an individual is not known; C refers instead to provides interocclusal space without discomfort or fatigue. ~2 mm interincisal (moderate 1–3 mm increase typical); E refers instead to the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comf.

REFERENCE Integrated notebook, page 112, §5.6

QUESTION 0558 · §5.5 · Applied clinical SBA

A patient with several crowns is being considered for mandibular advancement therapy. Which of the following statements about mouth and tongue is most accurate?

A. A TRD is an option for missing teeth or where jaw alignment precludes a MAD.

B. Existing dental or periodontal disease should be stabilised before appliance therapy.

C. Severity and positional dependence; adjustable MADs have higher response rates than fixed devices.

D. An anterior adjustment mechanism may be easier to use for a patient with limited dexterity.

E. Sufficient healthy teeth and a suitable bite are crucial.

CORRECT ANSWER A

A TRD is an option for missing teeth or where jaw alignment precludes a MAD. The other options describe different mechanisms, investigations or findings rather than mouth and tongue. Consult Integrated notebook, page 111, §5.5 for the full context.

REFERENCE Integrated notebook, page 111, §5.5

QUESTION 0559 · §5.2 · Clinical interpretation SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. The clinical team records the following finding: younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI. Which concept best explains this observation?

A. Essential caveat

B. Patient traits

C. Cephalometric

D. Polysomnographic

E. Trial-based

CORRECT ANSWER B

This finding most directly describes Patient traits: younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI. Distractor distinctions: A refers instead to patient factors alone are not a good predictor, and demographic, anthropometric and polysomnographic data only weakly inform; C refers instead to shorter soft palate; longer maxilla; lower hyoid and decreased hyoid-to-mandibular-plane distance; greater angle between cran; D refers instead to supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant; E refers instead to sufficient AHI reduction on an overnight test with a remotely controlled mandibular positioner — the best predictor available.

REFERENCE Integrated notebook, page 108, §5.2

QUESTION 0560 · §5.6 · Applied clinical SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. Which of the following statements about construction bite devices is most accurate?

A. Transcutaneous electrical nerve stimulation can relax masticatory muscles before bite registration.

B. Construction-bite gauges provide reproducible measurements of mandibular protrusion and opening.

C. Mandibular protrusion is the primary adjustable variable in oral appliance therapy.

D. A construction bite should record a reproducible mandibular midline even when it deviates during protrusion.

E. A construction bite may be recorded at approximately 60% of maximum protrusion with about 2 mm vertical separation.

CORRECT ANSWER B

Construction-bite gauges provide reproducible measurements of mandibular protrusion and opening. Additional revision detail: mechanical devices giving precise control and measurement in three dimensions, with guides and rulers documenting advancement and vertical opening numerically. The George Gauge is the archetype. The other options describe different mechanisms, investigations or findings rather than construction bite devices. Consult Integrated notebook, page 112, §5.6 for the full context.

REFERENCE Integrated notebook, page 112, §5.6

QUESTION 0561 · §5.4 · Clinical interpretation SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. The clinical team records the following finding: angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10 turns = +1 mm). An unattached mechanism. Which concept best explains this observation?

A. Acrylic (PMMA)

B. Not recommended

C. Interlocking Dorsal

D. Selection factors

E. Mono/Twin Block

CORRECT ANSWER C

This finding most directly describes Interlocking Dorsal: angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10 turns = +1 mm). An unattached mechanism. Distractor distinctions: A refers instead to good mechanical properties, well established; retention by frictional resistance; can be relined and repaired; used in DLP an; B refers instead to designs marketed on theories of airway and bone remodelling, promoted by manufacturers, with no peer- reviewed publication, n; D refers instead to number of teeth; arch relationship (class II vs III); bruxism; TMD; gagging; E refers instead to overlays fused at the occlusal surface (mono) or mated by interlocking blocks (twin).

REFERENCE Integrated notebook, page 109, §5.4

QUESTION 0562 · §5.2 · Applied clinical SBA

A follow-up patient reports poor retention of a recently supplied oral appliance. Which of the following statements about patient traits is most accurate?

A. Clinical and anthropometric predictors alone do not reliably identify oral appliance responders.

B. Polysomnographic includes supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than.

C. Endotype includes less collapsible airway; lower loop gain; higher arousal threshold; lower response to arousal.

D. Patient traits includes younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI.

E. Less collapsible airways, and the pharynx increases more along its whole length than in severe disease.

CORRECT ANSWER D

Patient traits includes younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI. Additional revision detail: younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI. The other options describe different mechanisms, investigations or findings rather than patient traits. Consult Integrated notebook, page 108, §5.2 for the full context.

REFERENCE Integrated notebook, page 108, §5.2

QUESTION 0563 · §5.5 · Clinical interpretation SBA

A patient with several crowns is being considered for mandibular advancement therapy. The clinical team records the following finding: an anterior adjustment mechanism suits poor dexterity, accepting anterior force concentration. Which concept best explains this observation?

A. Dexterity, cognition, visual acuity, range of motion

B. Sleep study findings

C. Mouth and tongue

D. Arch relationship

E. Patient preference

CORRECT ANSWER A

This finding most directly describes Dexterity, cognition, visual acuity, range of motion: an anterior adjustment mechanism suits poor dexterity, accepting anterior force concentration. Distractor distinctions: B refers instead to severity and positional dependence; adjustable MADs have higher response rates than fixed devices; C refers instead to a TRD is an option for missing teeth or where jaw alignment precludes a MAD; D refers instead to class II vs III alters mechanics and the direction of unwanted movement. An anterior open bite precludes a MAD — no posterior; E refers instead to perceived comfort, ease of use, durability, cost.

REFERENCE Integrated notebook, page 111, §5.5

QUESTION 0564 · §5.4 · Applied clinical SBA

A patient with several crowns is being considered for mandibular advancement therapy. Which of the following statements about interlocking dorsal is most accurate?

A. Interlocking dorsal includes angled dorsal posts from the mandibular overlay interlocking with maxillary blocks.

B. Nylon / polyamide (pa12), cad-cam includes lighter, smaller, robust, less breakage so better for bruxers.

C. Evidence comparing oral appliance designs for relative efficacy and long-term adverse effects is limited.

D. Additive manufacturing builds appliances layer by layer from digital models.

E. Precision post includes dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible.

CORRECT ANSWER A

Interlocking dorsal includes angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Additional revision detail: angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10 turns = +1 mm). An unattached mechanism. The other options describe different mechanisms, investigations or findings rather than interlocking dorsal. Consult Integrated notebook, page 109, §5.4 for the full context.

REFERENCE Integrated notebook, page 109, §5.4

QUESTION 0565 · §5.6 · Clinical interpretation SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. The clinical team records the following finding: provides interocclusal space without discomfort or fatigue. ~2 mm interincisal (moderate 1–3 mm increase typical). Which concept best explains this observation?

A. Horizontal (protrusion)

B. Why not maximum protrusion

C. Neuromuscular (TENS)

D. Construction bite devices

E. Vertical (bite opening)

CORRECT ANSWER E

This finding most directly describes Vertical (bite opening): provides interocclusal space without discomfort or fatigue. ~2 mm interincisal (moderate 1–3 mm increase typical). Distractor distinctions: A refers instead to the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comf; B refers instead to The acceptable range for an individual is not known; C refers instead to transcutaneous electrical nerve stimulation relaxes the muscles, allowing a relaxed physiological mandibular position before; D refers instead to mechanical devices giving precise control and measurement in three dimensions, with guides and rulers documenting advancement.

REFERENCE Integrated notebook, page 111, §5.6

QUESTION 0566 · §5.5 · Applied clinical SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. Which of the following statements about dexterity, cognition, visual acuity, range of motion is most accurate?

A. Existing dental or periodontal disease should be stabilised before appliance therapy.

B. Sufficient healthy teeth and a suitable bite are crucial.

C. Severity and positional dependence; adjustable MADs have higher response rates than fixed devices.

D. An anterior adjustment mechanism may be easier to use for a patient with limited dexterity.

E. A TRD is an option for missing teeth or where jaw alignment precludes a MAD.

CORRECT ANSWER D

An anterior adjustment mechanism may be easier to use for a patient with limited dexterity. Additional revision detail: an anterior adjustment mechanism suits poor dexterity, accepting anterior force concentration. The other options describe different mechanisms, investigations or findings rather than dexterity, cognition, visual acuity, range of motion. Consult Integrated notebook, page 111, §5.5 for the full context.

REFERENCE Integrated notebook, page 111, §5.5

QUESTION 0567 · §5.10 · Clinical interpretation SBA

A dentist and laboratory technician discuss the design of a custom titratable oral appliance. The clinical team records the following finding: every 6 months in year 1, then at least annually. Which concept best explains this observation?

A. Impression

B. Follow-up

C. Vertical

D. Dentition

E. Cleaning

CORRECT ANSWER B

This finding most directly describes Follow-up: every 6 months in year 1, then at least annually. Distractor distinctions: A refers instead to dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick; C refers instead to 2 mm grey George Gauge fork (5 mm white fork usually too much); interincisal increase 2–4 mm; posterior clearance nylon ~3 mm; D refers instead to ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support); E refers instead to Rinse immediately after removal. Clean each morning with a soft-bristled brush and mild soap or denture cleaner, using cool o.

REFERENCE Integrated notebook, page 128, §5.10

QUESTION 0568 · §5.6 · Applied clinical SBA

A follow-up patient reports poor retention of a recently supplied oral appliance. Which of the following statements about vertical (bite opening) is most accurate?

A. Phonetic registration estimates jaw position by observing speech sounds, but is not a standard MAD titration endpoint.

B. Clinical measurement and simulation tools includes bite gauges and mandibular positioning simulators for reproducibility.

C. A construction bite may be recorded at approximately 60% of maximum protrusion with about 2 mm vertical separation.

D. Construction-bite vertical opening should allow appliance clearance without excessive muscular strain.

E. A construction bite should record a reproducible mandibular midline even when it deviates during protrusion.

CORRECT ANSWER D

Construction-bite vertical opening should allow appliance clearance without excessive muscular strain. Additional revision detail: provides interocclusal space without discomfort or fatigue. ~2 mm interincisal (moderate 1–3 mm increase typical). The other options describe different mechanisms, investigations or findings rather than vertical (bite opening). Consult Integrated notebook, page 111, §5.6 for the full context.

REFERENCE Integrated notebook, page 111, §5.6

QUESTION 0569 · §5.2 · Clinical interpretation SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. The clinical team records the following finding: less collapsible airway; lower loop gain; higher arousal threshold; lower response to arousal. Which concept best explains this observation?

A. Trial-based

B. Patient traits

C. Adverse

D. Endotype

E. Cephalometric

CORRECT ANSWER D

This finding most directly describes Endotype: less collapsible airway; lower loop gain; higher arousal threshold; lower response to arousal. Distractor distinctions: A refers instead to sufficient AHI reduction on an overnight test with a remotely controlled mandibular positioner — the best predictor available; B refers instead to younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI; C refers instead to weight increase during treatment relates to failure. Differences between patients in the ability to advance the mandible modi; E refers instead to shorter soft palate; longer maxilla; lower hyoid and decreased hyoid-to-mandibular-plane distance; greater angle between cran.

REFERENCE Integrated notebook, page 108, §5.2

QUESTION 0570 · §5.10 · Applied clinical SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. Which of the following statements about follow-up is most accurate?

A. Adequate retention often requires at least six sound teeth per arch, although clinical requirements vary.

B. Appliance impressions should reproduce teeth and adjacent gingiva accurately.

C. Long-term oral appliance care requires at least annual dental review after the initial follow-up period.

D. Pterygomandibular raphe present in ~64% of people includes absence predicts better response.

E. An initial mandibular advancement around 50% of maximum lies within the commonly advised starting range.

CORRECT ANSWER C

Long-term oral appliance care requires at least annual dental review after the initial follow-up period. Additional revision detail: every 6 months in year 1, then at least annually. The other options describe different mechanisms, investigations or findings rather than follow-up. Consult Integrated notebook, page 128, §5.10 for the full context.

REFERENCE Integrated notebook, page 128, §5.10

QUESTION 0571 · §5.4 · Clinical interpretation SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. The clinical team records the following finding: dental casts and bite registrations used to create custom appliances. Well established, allowing precise custom fitting to the patient's anatomy. Which concept best explains this observation?

A. Precision Post

B. Mono/Twin Block

C. Acrylic (PMMA)

D. Lateral Pull

E. Traditional

CORRECT ANSWER E

This finding most directly describes Traditional: dental casts and bite registrations used to create custom appliances. Well established, allowing precise custom fitting to the patient's anatomy. Distractor distinctions: A refers instead to dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible. Titrated by swapp; B refers instead to overlays fused at the occlusal surface (mono) or mated by interlocking blocks (twin); C refers instead to good mechanical properties, well established; retention by frictional resistance; can be relined and repaired; used in DLP an; D refers instead to bilateral elastomeric or nylon straps pulling the mandibular overlay forward.

REFERENCE Integrated notebook, page 123, §5.4

QUESTION 0572 · §5.2 · Applied clinical SBA

A follow-up patient reports poor retention of a recently supplied oral appliance. Which of the following statements about endotype is most accurate?

A. Clinical and anthropometric predictors alone do not reliably identify oral appliance responders.

B. Less collapsible airways, and the pharynx increases more along its whole length than in severe disease.

C. Endotype includes less collapsible airway; lower loop gain; higher arousal threshold; lower response to arousal.

D. Patient traits includes younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI.

E. Polysomnographic includes supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than.

CORRECT ANSWER C

Endotype includes less collapsible airway; lower loop gain; higher arousal threshold; lower response to arousal. Additional revision detail: less collapsible airway; lower loop gain; higher arousal threshold; lower response to arousal. The other options describe different mechanisms, investigations or findings rather than endotype. Consult Integrated notebook, page 108, §5.2 for the full context.

REFERENCE Integrated notebook, page 108, §5.2

QUESTION 0573 · §5.5 · Clinical interpretation SBA

A patient with several crowns is being considered for mandibular advancement therapy. The clinical team records the following finding: severity and positional dependence; adjustable MADs have higher response rates than fixed devices. Which concept best explains this observation?

A. Arch relationship

B. Skeletal pattern

C. Mouth and tongue

D. Patient preference

E. Sleep study findings

CORRECT ANSWER E

This finding most directly describes Sleep study findings: severity and positional dependence; adjustable MADs have higher response rates than fixed devices. Distractor distinctions: A refers instead to class II vs III alters mechanics and the direction of unwanted movement. An anterior open bite precludes a MAD — no posterior; B refers instead to severe maxillomandibular deformity may need specialised treatment; retrognathia favours an appliance and is a specific proble; C refers instead to a TRD is an option for missing teeth or where jaw alignment precludes a MAD; D refers instead to perceived comfort, ease of use, durability, cost.

REFERENCE Integrated notebook, page 111, §5.5

QUESTION 0574 · §5.4 · Applied clinical SBA

A patient with several crowns is being considered for mandibular advancement therapy. Which of the following statements about traditional is most accurate?

A. Nylon / polyamide (pa12), cad-cam includes lighter, smaller, robust, less breakage so better for bruxers.

B. Additive manufacturing builds appliances layer by layer from digital models.

C. Acrylic offers established mechanical performance, repairability and clinical retention.

D. Traditional includes dental casts and bite registrations used to create custom appliances. Well established, allowing precise.

E. Interlocking dorsal includes angled dorsal posts from the mandibular overlay interlocking with maxillary blocks.

CORRECT ANSWER D

Traditional includes dental casts and bite registrations used to create custom appliances. Well established, allowing precise. Additional revision detail: dental casts and bite registrations used to create custom appliances. Well established, allowing precise custom fitting to the patient's anatomy. The other options describe different mechanisms, investigations or findings rather than traditional. Consult Integrated notebook, page 123, §5.4 for the full context.

REFERENCE Integrated notebook, page 123, §5.4

QUESTION 0575 · §5.6 · Clinical interpretation SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. The clinical team records the following finding: The acceptable range for an individual is not known. Which concept best explains this observation?

A. Construction bite devices

B. Vertical (bite opening)

C. Why not maximum protrusion

D. Horizontal (protrusion)

E. Neuromuscular (TENS)

CORRECT ANSWER C

This finding most directly describes Why not maximum protrusion: The acceptable range for an individual is not known. Distractor distinctions: A refers instead to mechanical devices giving precise control and measurement in three dimensions, with guides and rulers documenting advancement; B refers instead to provides interocclusal space without discomfort or fatigue. ~2 mm interincisal (moderate 1–3 mm increase typical); D refers instead to the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comf; E refers instead to transcutaneous electrical nerve stimulation relaxes the muscles, allowing a relaxed physiological mandibular position before.

REFERENCE Integrated notebook, page 111, §5.6

QUESTION 0576 · §5.5 · Applied clinical SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. Which of the following statements about sleep study findings is most accurate?

A. An anterior adjustment mechanism may be easier to use for a patient with limited dexterity.

B. A TRD is an option for missing teeth or where jaw alignment precludes a MAD.

C. Sufficient healthy teeth and a suitable bite are crucial.

D. Severity and positional dependence; adjustable MADs have higher response rates than fixed devices.

E. Existing dental or periodontal disease should be stabilised before appliance therapy.

CORRECT ANSWER D

Severity and positional dependence; adjustable MADs have higher response rates than fixed devices. The other options describe different mechanisms, investigations or findings rather than sleep study findings. Consult Integrated notebook, page 111, §5.5 for the full context.

REFERENCE Integrated notebook, page 111, §5.5

QUESTION 0577 · §5.10 · Clinical interpretation SBA

A dentist and laboratory technician discuss the design of a custom titratable oral appliance. The clinical team records the following finding: ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support). Which concept best explains this observation?

A. Follow-up

B. Cleaning

C. Vertical

D. Dentition

E. Impression

CORRECT ANSWER D

This finding most directly describes Dentition: ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support). Distractor distinctions: A refers instead to every 6 months in year 1, then at least annually; B refers instead to Rinse immediately after removal. Clean each morning with a soft-bristled brush and mild soap or denture cleaner, using cool o; C refers instead to 2 mm grey George Gauge fork (5 mm white fork usually too much); interincisal increase 2–4 mm; posterior clearance nylon ~3 mm; E refers instead to dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick.

REFERENCE Integrated notebook, page 128, §5.10

QUESTION 0578 · §5.6 · Applied clinical SBA

A follow-up patient reports poor retention of a recently supplied oral appliance. Which of the following statements about why not maximum protrusion is most accurate?

A. A construction bite should record a reproducible mandibular midline even when it deviates during protrusion.

B. A construction bite may be recorded at approximately 60% of maximum protrusion with about 2 mm vertical separation.

C. The acceptable range for an individual is not known.

D. Phonetic registration estimates jaw position by observing speech sounds, but is not a standard MAD titration endpoint.

E. Construction-bite vertical opening should allow appliance clearance without excessive muscular strain.

CORRECT ANSWER C

The acceptable range for an individual is not known. The other options describe different mechanisms, investigations or findings rather than why not maximum protrusion. Consult Integrated notebook, page 111, §5.6 for the full context.

REFERENCE Integrated notebook, page 111, §5.6

QUESTION 0579 · §5.2 · Focused knowledge SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. Which of the following statements about adverse is most accurate?

A. Polysomnographic includes supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than.

B. Clinical and anthropometric predictors alone do not reliably identify oral appliance responders.

C. Cephalometric includes shorter soft palate; longer maxilla.

D. Advancement reduces collapsibility dosedependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population.

E. Weight increase during treatment relates to failure.

CORRECT ANSWER E

Weight increase during treatment relates to failure. Additional revision detail: weight increase during treatment relates to failure. Differences between patients in the ability to advance the mandible modify the potential to eliminate events. The other options describe different mechanisms, investigations or findings rather than adverse. Consult Integrated notebook, page 108, §5.2 for the full context.

REFERENCE Integrated notebook, page 108, §5.2

QUESTION 0580 · §5.10 · Applied clinical SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. Which of the following statements about dentition is most accurate?

A. Pterygomandibular raphe present in ~64% of people includes absence predicts better response.

B. An initial mandibular advancement around 50% of maximum lies within the commonly advised starting range.

C. Appliance impressions should reproduce teeth and adjacent gingiva accurately.

D. Adequate retention often requires at least six sound teeth per arch, although clinical requirements vary.

E. Long-term oral appliance care requires at least annual dental review after the initial follow-up period.

CORRECT ANSWER D

Adequate retention often requires at least six sound teeth per arch, although clinical requirements vary. Additional revision detail: ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support). The other options describe different mechanisms, investigations or findings rather than dentition. Consult Integrated notebook, page 128, §5.10 for the full context.

REFERENCE Integrated notebook, page 128, §5.10

QUESTION 0581 · §5.4 · Clinical interpretation SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. The clinical team records the following finding: digital models built layer by layer. High customisation, potentially reduced production time and cost, and in-house printing offering fast replacement. Which concept best explains this observation?

A. 3D printing / additive manufacturing

B. Mono/Twin Block

C. Nylon / polyamide (PA12), CAD-CAM

D. Selection factors

E. Interlocking Dorsal

CORRECT ANSWER A

This finding most directly describes 3D printing / additive manufacturing: digital models built layer by layer. High customisation, potentially reduced production time and cost, and in-house printing offering fast replacement. Distractor distinctions: B refers instead to overlays fused at the occlusal surface (mono) or mated by interlocking blocks (twin); C refers instead to lighter, smaller, robust, less breakage so better for bruxers; retention by flexion into undercuts; needs only ~3 mm clearanc; D refers instead to number of teeth; arch relationship (class II vs III); bruxism; TMD; gagging; E refers instead to angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10.

REFERENCE Integrated notebook, page 123, §5.4

QUESTION 0582 · §5.2 · Applied clinical SBA

A follow-up patient reports poor retention of a recently supplied oral appliance. Which of the following statements about adverse is most accurate?

A. Weight increase during treatment relates to failure.

B. Clinical and anthropometric predictors alone do not reliably identify oral appliance responders.

C. Advancement reduces collapsibility dosedependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population.

D. Cephalometric includes shorter soft palate; longer maxilla.

E. Polysomnographic includes supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than.

CORRECT ANSWER A

Weight increase during treatment relates to failure. Additional revision detail: weight increase during treatment relates to failure. Differences between patients in the ability to advance the mandible modify the potential to eliminate events. The other options describe different mechanisms, investigations or findings rather than adverse. Consult Integrated notebook, page 108, §5.2 for the full context.

REFERENCE Integrated notebook, page 108, §5.2

QUESTION 0583 · §5.6 · Clinical interpretation SBA

A follow-up patient reports poor retention of a recently supplied oral appliance. The clinical team records the following finding: occlusal registration at 60% of maximum mandibular protrusion with 2 mm of vertical separation of the upper and lower incisors. Which concept best explains this observation?

A. Neuromuscular (TENS)

B. Classical practice

C. Vertical (bite opening)

D. Horizontal (protrusion)

E. Lateral (midline)

CORRECT ANSWER B

This finding most directly describes Classical practice: occlusal registration at 60% of maximum mandibular protrusion with 2 mm of vertical sep- aration of the upper and lower incisors. Distractor distinctions: A refers instead to transcutaneous electrical nerve stimulation relaxes the muscles, allowing a relaxed physiological mandibular position before; C refers instead to provides interocclusal space without discomfort or fatigue. ~2 mm interincisal (moderate 1–3 mm increase typical); D refers instead to the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comf; E refers instead to accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identi.

REFERENCE Integrated notebook, page 124, §5.6

QUESTION 0584 · §5.4 · Applied clinical SBA

A patient with several crowns is being considered for mandibular advancement therapy. Which of the following statements about 3d printing / additive manufacturing is most accurate?

A. Nylon / polyamide (pa12), cad-cam includes lighter, smaller, robust, less breakage so better for bruxers.

B. Traditional includes dental casts and bite registrations used to create custom appliances. Well established, allowing precise.

C. Additive manufacturing builds appliances layer by layer from digital models.

D. Interlocking dorsal includes angled dorsal posts from the mandibular overlay interlocking with maxillary blocks.

E. Acrylic offers established mechanical performance, repairability and clinical retention.

CORRECT ANSWER C

Additive manufacturing builds appliances layer by layer from digital models. Additional revision detail: digital models built layer by layer. High customisation, potentially reduced production time and cost, and in-house printing offering fast replacement. The other options describe different mechanisms, investigations or findings rather than 3d printing / additive manufacturing. Consult Integrated notebook, page 123, §5.4 for the full context.

REFERENCE Integrated notebook, page 123, §5.4

QUESTION 0585 · §5.10 · Clinical interpretation SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. The clinical team records the following finding: 25–75% acceptable); start at 50%, do not exceed 75%; the challenged "70% threshold" concept. Which concept best explains this observation?

A. Initial protrusion 50–70% of maximum (AADSM 2025

B. Impression

C. Handling and storage

D. Oral health

E. Pterygomandibular raphe present in ~64% of people

CORRECT ANSWER A

This finding most directly describes Initial protrusion 50–70% of maximum (AADSM 2025: 25–75% acceptable); start at 50%, do not exceed 75%; the challenged "70% threshold" concept. Distractor distinctions: B refers instead to dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick; C refers instead to Remove over a soft surface or a basin of water. Air dry before placing in the ventilated case. Store dry in the case — never; D refers instead to Brush and floss regularly. Morning routine: gentle jaw exercises — resting the chin on a fist, curling the tongue back to fin; E refers instead to absence predicts better response.

REFERENCE Integrated notebook, page 128, §5.10

QUESTION 0586 · §5.6 · Applied clinical SBA

A dentist and laboratory technician discuss the design of a custom titratable oral appliance. Which of the following statements about classical practice is most accurate?

A. Phonetic registration estimates jaw position by observing speech sounds, but is not a standard MAD titration endpoint.

B. Mandibular protrusion is the primary adjustable variable in oral appliance therapy.

C. A construction bite should record a reproducible mandibular midline even when it deviates during protrusion.

D. Construction-bite vertical opening should allow appliance clearance without excessive muscular strain.

E. A construction bite may be recorded at approximately 60% of maximum protrusion with about 2 mm vertical separation.

CORRECT ANSWER E

A construction bite may be recorded at approximately 60% of maximum protrusion with about 2 mm vertical separation. Additional revision detail: occlusal registration at 60% of maximum mandibular protrusion with 2 mm of vertical separation of the upper and lower incisors. The other options describe different mechanisms, investigations or findings rather than classical practice. Consult Integrated notebook, page 124, §5.6 for the full context.

REFERENCE Integrated notebook, page 124, §5.6

QUESTION 0587 · §5.2 · Clinical interpretation SBA

A follow-up patient reports poor retention of a recently supplied oral appliance. The clinical team records the following finding: Advancement reduces collapsibility dosedependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population. Which concept best explains this observation?

A. Primary mode of action is improvement in passive pharyngeal anatomy

B. Mild OSA suits the mechanism better

C. The airway enlarges

D. Essential caveat

E. Dose dependence extends beyond anatomy

CORRECT ANSWER A

This finding most directly describes Primary mode of action is improvement in passive pharyngeal anatomy: Advancement reduces collapsibility dose- dependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population. Distractor distinctions: B refers instead to less collapsible airways, and the pharynx increases more along its whole length than in severe disease; C refers instead to particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly; velopharyngeal, n; D refers instead to patient factors alone are not a good predictor, and demographic, anthropometric and polysomnographic data only weakly inform; E refers instead to there is dose-dependent enlargement of the upper airway, augmentation of upper airway muscle activity and improvement in mini.

REFERENCE Integrated notebook, page 107, §5.2

QUESTION 0588 · §5.10 · Applied clinical SBA

A patient with several crowns is being considered for mandibular advancement therapy. Which of the following statements about initial protrusion 50–70% of maximum (aadsm 2025 is most accurate?

A. Adequate retention often requires at least six sound teeth per arch, although clinical requirements vary.

B. Appliance impressions should reproduce teeth and adjacent gingiva accurately.

C. An initial mandibular advancement around 50% of maximum lies within the commonly advised starting range.

D. Pterygomandibular raphe present in ~64% of people includes absence predicts better response.

E. Long-term oral appliance care requires at least annual dental review after the initial follow-up period.

CORRECT ANSWER C

An initial mandibular advancement around 50% of maximum lies within the commonly advised starting range. Additional revision detail: 25–75% acceptable); start at 50%, do not exceed 75%; the challenged "70% threshold" concept. The other options describe different mechanisms, investigations or findings rather than initial protrusion 50–70% of maximum (aadsm 2025. Consult Integrated notebook, page 128, §5.10 for the full context.

REFERENCE Integrated notebook, page 128, §5.10

QUESTION 0589 · §5.4 · Clinical interpretation SBA

A patient with several crowns is being considered for mandibular advancement therapy. The clinical team records the following finding: lighter, smaller, robust, less breakage so better for bruxers; retention by flexion into undercuts; needs only ~3 mm clearance; titration scale visible on the device. Which concept best explains this observation?

A. Nylon / polyamide (PA12), CAD-CAM

B. Selection factors

C. Interlocking Dorsal

D. Mono/Twin Block

E. 3D printing / additive manufacturing

CORRECT ANSWER A

This finding most directly describes Nylon / polyamide (PA12), CAD-CAM: lighter, smaller, robust, less breakage so better for bruxers; retention by flexion into undercuts; needs only ~3 mm clearance; titration scale visible on the device. Distractor distinctions: B refers instead to number of teeth; arch relationship (class II vs III); bruxism; TMD; gagging; C refers instead to angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10; D refers instead to overlays fused at the occlusal surface (mono) or mated by interlocking blocks (twin); E refers instead to digital models built layer by layer. High customisation, potentially reduced production time and cost, and in-house printing.

REFERENCE Integrated notebook, page 110, §5.4

QUESTION 0590 · §5.2 · Applied clinical SBA

A dentist and laboratory technician discuss the design of a custom titratable oral appliance. Which of the following statements about primary mode of action is improvement in passive pharyngeal anatomy is most accurate?

A. Less collapsible airways, and the pharynx increases more along its whole length than in severe disease.

B. Weight increase during treatment relates to failure.

C. Clinical and anthropometric predictors alone do not reliably identify oral appliance responders.

D. Advancement reduces collapsibility dosedependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population.

E. Polysomnographic includes supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than.

CORRECT ANSWER D

Advancement reduces collapsibility dosedependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population. The other options describe different mechanisms, investigations or findings rather than primary mode of action is improvement in passive pharyngeal anatomy. Consult Integrated notebook, page 107, §5.2 for the full context.

REFERENCE Integrated notebook, page 107, §5.2

QUESTION 0591 · §5.10 · Clinical interpretation SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. The clinical team records the following finding: absence predicts better response. Which concept best explains this observation?

A. Oral health

B. Impression

C. Initial protrusion 50–70% of maximum (AADSM 2025

D. Handling and storage

E. Pterygomandibular raphe present in ~64% of people

CORRECT ANSWER E

This finding most directly describes Pterygomandibular raphe present in ~64% of people: absence predicts better response. Distractor distinctions: A refers instead to Brush and floss regularly. Morning routine: gentle jaw exercises — resting the chin on a fist, curling the tongue back to fin; B refers instead to dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick; C refers instead to 25–75% acceptable); start at 50%, do not exceed 75%; the challenged "70% threshold" concept; D refers instead to Remove over a soft surface or a basin of water. Air dry before placing in the ventilated case. Store dry in the case — never.

REFERENCE Integrated notebook, page 128, §5.10

QUESTION 0592 · §5.4 · Applied clinical SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. Which of the following statements about nylon / polyamide (pa12), cad-cam is most accurate?

A. Interlocking dorsal includes angled dorsal posts from the mandibular overlay interlocking with maxillary blocks.

B. Nylon / polyamide (pa12), cad-cam includes lighter, smaller, robust, less breakage so better for bruxers.

C. Acrylic offers established mechanical performance, repairability and clinical retention.

D. Additive manufacturing builds appliances layer by layer from digital models.

E. Traditional includes dental casts and bite registrations used to create custom appliances. Well established, allowing precise.

CORRECT ANSWER B

Nylon / polyamide (pa12), cad-cam includes lighter, smaller, robust, less breakage so better for bruxers. Additional revision detail: lighter, smaller, robust, less breakage so better for bruxers; retention by flexion into undercuts; needs only ~3 mm clearance; titration scale visible on the device. The other options describe different mechanisms, investigations or findings rather than nylon / polyamide (pa12), cad-cam. Consult Integrated notebook, page 110, §5.4 for the full context.

REFERENCE Integrated notebook, page 110, §5.4

QUESTION 0593 · §5.2 · Clinical interpretation SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. The clinical team records the following finding: shorter soft palate; longer maxilla; lower hyoid and decreased hyoid-to-mandibular-plane distance; greater angle between cranial base and mandibular plane; retrognathic mandible with narrow SNB and wider SNA. Which concept best explains this observation?

A. Polysomnographic

B. Trial-based

C. Patient traits

D. Cephalometric

E. Essential caveat

CORRECT ANSWER D

This finding most directly describes Cephalometric: shorter soft palate; longer maxilla; lower hyoid and decreased hyoid-to-mandibular-plane distance; greater angle between cranial base and mandibular plane; retrognathic mandible with narrow SNB and wider SNA. Distractor distinctions: A refers instead to supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant; B refers instead to sufficient AHI reduction on an overnight test with a remotely controlled mandibular positioner — the best predictor available; C refers instead to younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI; E refers instead to patient factors alone are not a good predictor, and demographic, anthropometric and polysomnographic data only weakly inform.

REFERENCE Integrated notebook, page 108, §5.2

QUESTION 0594 · §5.10 · Applied clinical SBA

A follow-up patient reports poor retention of a recently supplied oral appliance. Which of the following statements about pterygomandibular raphe present in ~64% of people is most accurate?

A. An initial mandibular advancement around 50% of maximum lies within the commonly advised starting range.

B. Long-term oral appliance care requires at least annual dental review after the initial follow-up period.

C. Appliance impressions should reproduce teeth and adjacent gingiva accurately.

D. Pterygomandibular raphe present in ~64% of people includes absence predicts better response.

E. Adequate retention often requires at least six sound teeth per arch, although clinical requirements vary.

CORRECT ANSWER D

Pterygomandibular raphe present in ~64% of people includes absence predicts better response. Additional revision detail: absence predicts better response. The other options describe different mechanisms, investigations or findings rather than pterygomandibular raphe present in ~64% of people. Consult Integrated notebook, page 128, §5.10 for the full context.

REFERENCE Integrated notebook, page 128, §5.10

QUESTION 0595 · §5.4 · Clinical interpretation SBA

A follow-up patient reports poor retention of a recently supplied oral appliance. The clinical team records the following finding: bilateral elastomeric or nylon straps pulling the mandibular overlay forward. Which concept best explains this observation?

A. Acrylic (PMMA)

B. Mono/Twin Block

C. Precision Post

D. Lateral Pull

E. Traditional

CORRECT ANSWER D

This finding most directly describes Lateral Pull: bilateral elastomeric or nylon straps pulling the mandibular overlay forward. Distractor distinctions: A refers instead to good mechanical properties, well established; retention by frictional resistance; can be relined and repaired; used in DLP an; B refers instead to overlays fused at the occlusal surface (mono) or mated by interlocking blocks (twin); C refers instead to dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible. Titrated by swapp; E refers instead to dental casts and bite registrations used to create custom appliances. Well established, allowing precise custom fitting to th.

REFERENCE Integrated notebook, page 109, §5.4

QUESTION 0596 · §5.2 · Applied clinical SBA

A patient with several crowns is being considered for mandibular advancement therapy. Which of the following statements about cephalometric is most accurate?

A. Cephalometric includes shorter soft palate; longer maxilla.

B. Particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly.

C. Clinical and anthropometric predictors alone do not reliably identify oral appliance responders.

D. Weight increase during treatment relates to failure.

E. Advancement reduces collapsibility dosedependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population.

CORRECT ANSWER A

Cephalometric includes shorter soft palate; longer maxilla. Additional revision detail: shorter soft palate; longer maxilla; lower hyoid and decreased hyoid-to-mandibular-plane distance; greater angle between cranial base and mandibular plane; retrognathic mandible with narrow SNB and wider SNA. The other options describe different mechanisms, investigations or findings rather than cephalometric. Consult Integrated notebook, page 108, §5.2 for the full context.

REFERENCE Integrated notebook, page 108, §5.2

QUESTION 0597 · §5.10 · Clinical interpretation SBA

A dentist and laboratory technician discuss the design of a custom titratable oral appliance. The clinical team records the following finding: Rinse immediately after removal. Clean each morning with a soft-bristled brush and mild soap or denture cleaner, using cool or warm — never hot — water (hot water warps the device). Which concept best explains this observation?

A. Vertical

B. Impression

C. Dentition

D. Cleaning

E. Follow-up

CORRECT ANSWER D

This finding most directly describes Cleaning: Rinse immediately after removal. Clean each morning with a soft-bristled brush and mild soap or denture cleaner, using cool or warm — never hot — water (hot water warps the device). Distractor distinctions: A refers instead to 2 mm grey George Gauge fork (5 mm white fork usually too much); interincisal increase 2–4 mm; posterior clearance nylon ~3 mm; B refers instead to dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick; C refers instead to ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support); E refers instead to every 6 months in year 1, then at least annually.

REFERENCE Integrated notebook, page 113, §5.10

QUESTION 0598 · §5.4 · Applied clinical SBA

A dentist and laboratory technician discuss the design of a custom titratable oral appliance. Which of the following statements about lateral pull is most accurate?

A. Selection factors includes number of teeth; arch relationship (class II vs III); bruxism; TMD; gagging.

B. Traditional includes dental casts and bite registrations used to create custom appliances. Well established, allowing precise.

C. Acrylic offers established mechanical performance, repairability and clinical retention.

D. Lateral pull includes bilateral elastomeric or nylon straps pulling the mandibular overlay forward.

E. Mono/twin block includes overlays fused at the occlusal surface (mono) or mated by interlocking blocks (twin).

CORRECT ANSWER D

Lateral pull includes bilateral elastomeric or nylon straps pulling the mandibular overlay forward. Additional revision detail: bilateral elastomeric or nylon straps pulling the mandibular overlay forward. The other options describe different mechanisms, investigations or findings rather than lateral pull. Consult Integrated notebook, page 109, §5.4 for the full context.

REFERENCE Integrated notebook, page 109, §5.4

QUESTION 0599 · §5.2 · Clinical interpretation SBA

At appliance delivery, a patient reports a comfortable initial protrusive setting. The clinical team records the following finding: patient factors alone are not a good predictor, and demographic, anthropometric and polysomnographic data only weakly inform efficacy. Which concept best explains this observation?

A. Patient traits

B. The airway enlarges

C. Cephalometric

D. Polysomnographic

E. Essential caveat

CORRECT ANSWER E

This finding most directly describes Essential caveat: patient factors alone are not a good predictor, and demographic, anthropometric and polysomnographic data only weakly inform efficacy. Distractor distinctions: A refers instead to younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI; B refers instead to particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly; velopharyngeal, n; C refers instead to shorter soft palate; longer maxilla; lower hyoid and decreased hyoid-to-mandibular-plane distance; greater angle between cran; D refers instead to supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant.

REFERENCE Integrated notebook, page 108, §5.2

QUESTION 0600 · §5.10 · Applied clinical SBA

A 52-year-old with diagnosed OSA attends the construction-bite appointment for a custom MAD. Which of the following statements about cleaning is most accurate?

A. An initial mandibular advancement around 50% of maximum lies within the commonly advised starting range.

B. Adequate retention often requires at least six sound teeth per arch, although clinical requirements vary.

C. Handling and storage includes remove over a soft surface or a basin of water.

D. Vertical: 2 mm grey George Gauge fork (5 mm white fork usually too much).

E. Cleaning includes rinse immediately after removal.

CORRECT ANSWER E

Cleaning includes rinse immediately after removal. Additional revision detail: Rinse immediately after removal. Clean each morning with a soft-bristled brush and mild soap or denture cleaner, using cool or warm — never hot — water (hot water warps the device). The other options describe different mechanisms, investigations or findings rather than cleaning. Consult Integrated notebook, page 113, §5.10 for the full context.

REFERENCE Integrated notebook, page 113, §5.10

06 Titration, treatment effectiveness and adherence

QUESTION 0601 · §6.1 · Clinical decision-making SBA

A patient reports less snoring two weeks after MAD fitting but has worsening bilateral jaw soreness. What is the best titration decision?

A. Classify complete OSA cure from snoring alone

B. Advance to maximum immediately

C. Reassess comfort and consider pausing or reducing advancement before further titration

D. Disregard jaw findings

E. Stop physician follow-up permanently

CORRECT ANSWER C

Titration must balance symptomatic gain with tolerability; persistent symptoms and adverse effects require adjustment and monitoring.

REFERENCE Integrated notebook, Topic 6, §6.1

QUESTION 0602 · §6.3 · Clinical decision-making SBA

After three months using a MAD, a patient reports much less snoring but remains sleepy. What is the best way to establish whether OSA is adequately controlled?

A. Accept the snoring history as proof of cure

B. Increase protrusion without regard to symptoms or pain

C. Record adherence and clinical findings and arrange physician-directed objective reassessment

D. Stop treatment because snoring improved

E. Use a repeat STOP-BANG score as definitive efficacy testing

CORRECT ANSWER C

Subjective improvement is not proof of normalised AHI. Reassess fit, adherence and symptoms, then refer for an objective sleep study under physician supervision. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 6, §6.3

QUESTION 0603 · §6.5 · Clinical decision-making SBA

A patient has a large relative AHI improvement after OAT but the residual AHI remains above the normal range. Which is the best conclusion?

A. Treatment is necessarily a complete response

B. Residual disease may persist despite a substantial percentage reduction

C. The patient cannot benefit symptomatically

D. OAT should always be stopped on the basis of one number

E. The dentist may certify a cure without medical follow-up

CORRECT ANSWER B

A substantial percentage improvement does not automatically mean normalization; both residual AHI and symptoms matter for complete-versus-partial response. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 6, §6.5

QUESTION 0604 · §6.6 · Clinical decision-making SBA

A patient reports wearing a MAD nightly for eight hours, but the objective wear-time sensor shows much shorter use. Which conclusion is most defensible?

A. Ignore the objective data

B. Use objective adherence data alongside the patient's account to guide counselling and assessment

C. Count all reported time as proven device use

D. Diagnose new sleep apnoea from adherence alone

E. Change the AHI definition to match the diary

CORRECT ANSWER B

Objective adherence measurement can reveal discrepancies between recalled and actual appliance use and helps explain effectiveness in the real world. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 6, §6.6

QUESTION 0605 · §6.4 · Clinical decision-making SBA

A patient is comfortable at 5 mm MAD advancement but develops persistent jaw pain at 7 mm. Which is the best next step?

A. Increase immediately to 9 mm

B. Reduce to a comfortable setting, manage pain and reassess efficacy

C. Continue at 7 mm regardless of persistent symptoms

D. Replace the custom device with an identical fixed appliance immediately

E. Use maximum protrusion as the only measure of success

CORRECT ANSWER B

The titration goal is maximum comfortable and effective advancement, not maximum possible protrusion. Persistent pain requires review and adjustment. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 6, §6.4

QUESTION 0606 · §6.3 · Clinical decision-making SBA

A patient feels dramatically better using a MAD; baseline AHI was 38. How should treatment success be confirmed?

A. Refer to the physician for appropriate objective efficacy testing

B. Conclude residual AHI is zero

C. Declare cure solely from ESS improvement

D. Avoid further testing because snoring decreased

E. Use the dentist's portable monitor as definitive diagnosis

CORRECT ANSWER A

Symptom improvement alone does not prove normalisation; the medical provider interprets objective follow-up testing.

REFERENCE Integrated notebook, Topic 6, §6.3

QUESTION 0607 · §6.2 · Applied clinical SBA

A sleep specialist receives an update on an oral appliance candidate during titration. Which of the following statements about dental discomfort and bite change is most accurate?

A. Appliance condition includes cracks, warping, wear affecting fit and efficacy; confirm retention and stability.

B. Hypersalivation and dry mouth can occur early in MAD therapy and often diminish during adaptation.

C. Appliance-related oral discomfort may include tooth pain, gingival irritation or a transient change in bite.

D. Comfort with nightly use, and whether side effects are reducing use.

E. Jaw and tmj includes discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need.

CORRECT ANSWER C

Appliance-related oral discomfort may include tooth pain, gingival irritation or a transient change in bite. Additional revision detail: tooth pain, gingival irritation, soreness, the bite feeling "off" or teeth not fitting as before; long-term use decreases overbite and overjet — monitor and record. The other options describe different mechanisms, investigations or findings rather than dental discomfort and bite change. Consult Integrated notebook, page 132, §6.2 for the full context.

REFERENCE Integrated notebook, page 132, §6.2

QUESTION 0608 · §6.2 · Clinical interpretation SBA

A patient returns after several weeks of MAD use and says snoring has improved, but fatigue persists. The clinical team records the following finding: tooth pain, gingival irritation, soreness, the bite feeling "off" or teeth not fitting as before; long-term use decreases overbite and overjet — monitor and record. Which concept best explains this observation?

A. Unexpected persistent snoring

B. Jaw and TMJ

C. Appliance condition

D. Tolerance and adherence

E. Dental discomfort and bite change

CORRECT ANSWER E

This finding most directly describes Dental discomfort and bite change: tooth pain, gingival irritation, soreness, the bite feeling "off" or teeth not fitting as before; long-term use decreases overbite and overjet — monitor and record. Distractor distinctions: A refers instead to Reassess adherence, appliance stability, symptom change and residual OSA; B refers instead to discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need management or adjust; C refers instead to cracks, warping, wear affecting fit and efficacy; confirm retention and stability; D refers instead to comfort with nightly use, and whether side effects are reducing use.

REFERENCE Integrated notebook, page 132, §6.2

QUESTION 0609 · §6.4 · Clinical decision-making SBA

A mandibular positioner identifies an effective setting during an overnight study. What is the next appropriate step?

A. Translate the setting into the device and verify final clinical efficacy through shared care

B. Assume the research device's setting proves permanent cure

C. Skip appliance fit examination

D. Use the result to diagnose OSA de novo

E. Ignore adverse effects

CORRECT ANSWER A

An instrumented titration can guide protrusion but final device performance, tolerance and physician-led outcome assessment remain necessary.

REFERENCE Integrated notebook, Topic 6, §6.4

QUESTION 0610 · §6.3 · Applied clinical SBA

A patient with previously moderate OSA has reached the maximally comfortable protrusion. Which of the following statements about not indicated is most accurate?

A. Partial treatment response includes improved symptoms with at least a 50% AHI fall but residual AHI at least five.

B. Dentist-operated portable data is Used only to guide titration within local scope rules.

C. Not indicated includes primary snoring, unless symptoms worsen or fail to resolve.

D. Persistent symptoms and inadequate objective AHI improvement indicate an inadequate treatment response.

E. The treating physician using appropriate objective sleep testing.

CORRECT ANSWER C

Not indicated includes primary snoring, unless symptoms worsen or fail to resolve. Additional revision detail: primary snoring, unless symptoms worsen or fail to resolve. The other options describe different mechanisms, investigations or findings rather than not indicated. Consult Integrated notebook, page 133, §6.3 for the full context.

REFERENCE Integrated notebook, page 133, §6.3

QUESTION 0611 · §6.3 · Clinical interpretation SBA

A sleep specialist receives an update on an oral appliance candidate during titration. The clinical team records the following finding: primary snoring, unless symptoms worsen or fail to resolve. Which concept best explains this observation?

A. Jaw and TMJ

B. Dentist-operated portable data

C. Who verifies efficacy

D. AADSM compliance

E. Not indicated

CORRECT ANSWER E

This finding most directly describes Not indicated: primary snoring, unless symptoms worsen or fail to resolve. Distractor distinctions: A refers instead to discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need management or adjust; B refers instead to Used only to guide titration within local scope rules; C refers instead to The treating physician using appropriate objective sleep testing; D refers instead to ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night.

REFERENCE Integrated notebook, page 133, §6.3

QUESTION 0612 · §6.5 · Clinical decision-making SBA

A patient's snoring resolves but follow-up AHI remains 24. Which statement is most accurate?

A. The dentist should independently diagnose central apnoea

B. An appliance is effective whenever it is worn

C. Subjective improvement does not establish adequate control of OSA

D. A high residual AHI requires no action

E. Snoring resolution always implies AHI <5

CORRECT ANSWER C

Residual objective disease requires review of adherence, fit, advancement and medical options.

REFERENCE Integrated notebook, Topic 6, §6.5

QUESTION 0613 · §6.4 · Applied clinical SBA

A dentist is titrating a custom appliance while monitoring discomfort and adherence. Which of the following statements about multimethod positioning is most accurate?

A. Improved symptoms after initial advancement still require objective confirmation of efficacy.

B. A remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation.

C. Instrumented overnight mandibular positioning can identify an effective advancement during PSG.

D. Long-term oral appliance care requires at least annual dental review after the initial follow-up period.

E. Clinical titration guides adjustment, but efficacy requires physician-directed objective reassessment.

CORRECT ANSWER E

Clinical titration guides adjustment, but efficacy requires physician-directed objective reassessment. Additional revision detail: clinical titration and symptom monitoring guide adjustment; final efficacy verification requires appropriate physician-directed objective reassessment. The other options describe different mechanisms, investigations or findings rather than multimethod positioning. Consult Integrated notebook, page 133, §6.4 for the full context.

REFERENCE Integrated notebook, page 133, §6.4

QUESTION 0614 · §6.4 · Clinical interpretation SBA

A patient with previously moderate OSA has reached the maximally comfortable protrusion. The clinical team records the following finding: initial positioning → resolution or improvement of signs and symptoms → pulse oximetry, highresolution oximetry or HSAT administered by the qualified dentist, and/or HSAT or PSG ordered by the physician → verification of…. Which concept best explains this observation?

A. Remotely controlled mandibular positioner (RCMP)

B. Signs and symptoms only

C. Drug-induced sleep endoscopy

D. Multimethod positioning

E. Both end in the same place

CORRECT ANSWER D

This finding most directly describes Multimethod positioning: initial positioning → resolution or improvement of signs and symptoms → pulse oximetry, high- resolution oximetry or HSAT administered by the qualified dentist, and/or HSAT or PSG ordered by the physician → verification of…. Distractor distinctions: A refers instead to allows objective, single-night titration during polysom- nography to identify the optimal therapeutic advancement; B refers instead to initial mandibular positioning → resolution or improvement of signs and symptoms → verification of efficacy by the physician; C refers instead to a remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation; E refers instead to The dentist's objective data supplement the titration decision; they never replace physician verification.

REFERENCE Integrated notebook, page 133, §6.4

QUESTION 0615 · §6.6 · Clinical decision-making SBA

An oral appliance appears ineffective but a temperature microsensor shows it is worn two hours nightly. What should be addressed?

A. Call the therapy a confirmed physiological failure

B. Assume objective adherence never matters

C. Increase protrusion by 10 mm without review

D. Replace medical efficacy testing with the adherence record

E. Poor adherence before concluding that the device itself cannot work

CORRECT ANSWER E

Effectiveness depends on both physiological efficacy and actual use; objective adherence helps distinguish an ineffective device from an unused one.

REFERENCE Integrated notebook, Topic 6, §6.6

QUESTION 0616 · §6.5 · Applied clinical SBA

A 55-year-old feels better after appliance adjustment and asks whether treatment is now effective. Which of the following statements about persistent snoring, no other symptoms is most accurate?

A. Complete treatment response includes resolution of symptoms and AHI less than five per hour on treatment.

B. Partial treatment response includes improved symptoms with at least a 50% AHI fall but residual AHI at least five.

C. Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint.

D. Persistent symptoms and inadequate objective AHI improvement indicate an inadequate treatment response.

E. Persistent snoring, no other symptoms may be acceptable to the patient.

CORRECT ANSWER E

Persistent snoring, no other symptoms may be acceptable to the patient. Additional revision detail: may be acceptable to the patient; but persistent snoring on treatment relates to insufficient apnoea control and poor adherence, so verify objectively before accepting it. The other options describe different mechanisms, investigations or findings rather than persistent snoring, no other symptoms. Consult Integrated notebook, page 134, §6.5 for the full context.

REFERENCE Integrated notebook, page 134, §6.5

QUESTION 0617 · §6.5 · Clinical interpretation SBA

A dentist is titrating a custom appliance while monitoring discomfort and adherence. The clinical team records the following finding: may be acceptable to the patient; but persistent snoring on treatment relates to insufficient apnoea control and poor adherence, so verify objectively before accepting it. Which concept best explains this observation?

A. Partial treatment response

B. Persistent snoring, no other symptoms

C. Ongoing clinical failure

D. Long-term effectiveness

E. Complete treatment response

CORRECT ANSWER B

This finding most directly describes Persistent snoring, no other symptoms: may be acceptable to the patient; but persistent snoring on treatment relates to insufficient apnoea control and poor adherence, so verify objectively before accepting it. Distractor distinctions: A refers instead to Improved symptoms with at least a 50% AHI fall but residual AHI at least five; C refers instead to Persistent symptoms with insufficient objective improvement; D refers instead to Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint; applian; E refers instead to Resolution of symptoms and AHI less than five per hour on treatment.

REFERENCE Integrated notebook, page 134, §6.5

QUESTION 0618 · §6.6 · Clinical decision-making SBA

A patient wears a MAD for 6 hours on three nights a week. Does this meet the notebook's AADSM adherence definition?

A. Yes, because the 4-hour CPAP convention applies

B. Yes, if snoring improves on those nights

C. No, because a MAD must be used for 24 hours

D. Yes, because any three nights suffice

E. No, because five nights per week are specified

CORRECT ANSWER E

The notebook defines satisfactory use as at least 80% of the sleep period on at least five nights per week; CPAP research conventions are different.

REFERENCE Integrated notebook, Topic 6, §6.6

QUESTION 0619 · §6.6 · Applied clinical SBA

A patient returns after several weeks of MAD use and says snoring has improved, but fatigue persists. Which of the following statements about it distinguishes device inefficacy from poor compliance is most accurate?

A. Titration includes start at 50% of maximum; 2-week acclimatisation, then 6-week self-adjustment.

B. Oral appliances achieved a treated AHI below 5/h in approximately 37% in the studies summarised.

C. AADSM compliance: ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night.

D. Adherence measurements help distinguish inadequate appliance use from inadequate physiological efficacy.

E. A device-embedded temperature sensor can objectively estimate nightly wear.

CORRECT ANSWER D

Adherence measurements help distinguish inadequate appliance use from inadequate physiological efficacy. Additional revision detail: the single most important diagnostic function of adherence data. Without it you cannot know whether to advance the appliance or address behaviour. The other options describe different mechanisms, investigations or findings rather than it distinguishes device inefficacy from poor compliance. Consult Integrated notebook, page 142, §6.6 for the full context.

REFERENCE Integrated notebook, page 142, §6.6

QUESTION 0620 · §6.6 · Clinical interpretation SBA

A 55-year-old feels better after appliance adjustment and asks whether treatment is now effective. The clinical team records the following finding: the single most important diagnostic function of adherence data. Without it you cannot know whether to advance the appliance or address behaviour. Which concept best explains this observation?

A. Titration

B. Recorded adherence technology

C. It distinguishes device inefficacy from poor compliance

D. AADSM compliance

E. Efficacy

CORRECT ANSWER C

This finding most directly describes It distinguishes device inefficacy from poor compliance: the single most important diagnostic function of adherence data. Without it you cannot know whether to advance the appliance or address behaviour. Distractor distinctions: A refers instead to start at 50% of maximum; 2-week acclimatisation, then 6-week self-adjustment; increments 0.2–0.4 mm per night; total titratio; B refers instead to A device-embedded temperature sensor can objectively estimate nightly wear; D refers instead to ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night; E refers instead to AHI <5 ~37% · AHI <10 ~52% · ≥50% reduction ~64% · ⅔ meaningful benefit, ⅓ none.

REFERENCE Integrated notebook, page 142, §6.6

QUESTION 0621 · §6.2 · Clinical decision-making SBA

A patient says the bite feels 'off' for an hour after removing the splint. Which question is most useful?

A. Should the patient never remove the appliance?

B. Has the bite returned to baseline after morning exercises or alignment?

C. Is the appliance blue?

D. Is snoring audible in the afternoon?

E. Has the patient stopped eating breakfast?

CORRECT ANSWER B

Morning occlusal changes can occur after protrusion; assess their persistence, function and management rather than assuming harmlessness.

REFERENCE Integrated notebook, Topic 6, §6.2

QUESTION 0622 · §6.1 · Applied clinical SBA

A patient with previously moderate OSA has reached the maximally comfortable protrusion. Which of the following statements about initial protrusive range is most accurate?

A. Patients may titrate an adjustable appliance gradually over several weeks after acclimatisation.

B. Early appointment timing includes reassess within 30 days of insertion.

C. An initial acclimatisation period of roughly two weeks is often used before active titration.

D. Initial protrusive range includes start commonly around 50% of the measured maximum advancement.

E. Why revert includes to resolve unacceptable side effects; to balance efficacy against tolerance.

CORRECT ANSWER D

Initial protrusive range includes start commonly around 50% of the measured maximum advancement. Additional revision detail: Start commonly around 50% of the measured maximum advancement. The other options describe different mechanisms, investigations or findings rather than initial protrusive range. Consult Integrated notebook, page 132, §6.1 for the full context.

REFERENCE Integrated notebook, page 132, §6.1

QUESTION 0623 · §6.1 · Clinical interpretation SBA

A sleep specialist receives an update on an oral appliance candidate during titration. The clinical team records the following finding: Start commonly around 50% of the measured maximum advancement. Which concept best explains this observation?

A. Typical overall titration time

B. Patient-led adjustment window

C. Acclimatisation interval

D. Early appointment timing

E. Initial protrusive range

CORRECT ANSWER E

This finding most directly describes Initial protrusive range: Start commonly around 50% of the measured maximum advancement. Distractor distinctions: A refers instead to Approximately two to five months; B refers instead to About six weeks of gradual self-adjustment may follow acclimatisation; C refers instead to Allow approximately two weeks at the initial setting; D refers instead to Reassess within 30 days of insertion.

REFERENCE Integrated notebook, page 132, §6.1

QUESTION 0624 · §6.4 · Clinical decision-making SBA

After gradual protrusion the patient develops severe unilateral TMJ pain. What is best done?

A. Diagnose OSA severity from pain

B. Increase advancement until pain stops

C. Label it proof of therapeutic success

D. Ignore a unilateral change

E. Stop further advancement and evaluate the joint and device, with modification or referral as needed

CORRECT ANSWER E

Severe or persistent TMD symptoms require a safety response, not automatic further titration.

REFERENCE Integrated notebook, Topic 6, §6.4

QUESTION 0625 · §6.2 · Applied clinical SBA

A dentist is titrating a custom appliance while monitoring discomfort and adherence. Which of the following statements about unexpected persistent snoring is most accurate?

A. Unexpected persistent snoring includes reassess adherence, appliance stability, symptom change and residual OSA.

B. Appliance condition includes cracks, warping, wear affecting fit and efficacy; confirm retention and stability.

C. Jaw and tmj includes discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need.

D. Hypersalivation and dry mouth can occur early in MAD therapy and often diminish during adaptation.

E. Comfort with nightly use, and whether side effects are reducing use.

CORRECT ANSWER A

Unexpected persistent snoring includes reassess adherence, appliance stability, symptom change and residual OSA. Additional revision detail: Reassess adherence, appliance stability, symptom change and residual OSA. The other options describe different mechanisms, investigations or findings rather than unexpected persistent snoring. Consult Integrated notebook, page 133, §6.2 for the full context.

REFERENCE Integrated notebook, page 133, §6.2

QUESTION 0626 · §6.2 · Clinical interpretation SBA

A patient with previously moderate OSA has reached the maximally comfortable protrusion. The clinical team records the following finding: Reassess adherence, appliance stability, symptom change and residual OSA. Which concept best explains this observation?

A. Dental discomfort and bite change

B. Jaw and TMJ

C. Unexpected persistent snoring

D. Tolerance and adherence

E. Appliance condition

CORRECT ANSWER C

This finding most directly describes Unexpected persistent snoring: Reassess adherence, appliance stability, symptom change and residual OSA. Distractor distinctions: A refers instead to tooth pain, gingival irritation, soreness, the bite feeling "off" or teeth not fitting as before; long-term use decreases ove; B refers instead to discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need management or adjust; D refers instead to comfort with nightly use, and whether side effects are reducing use; E refers instead to cracks, warping, wear affecting fit and efficacy; confirm retention and stability.

REFERENCE Integrated notebook, page 133, §6.2

QUESTION 0627 · §6.1 · Clinical decision-making SBA

A baseline ESS was 16 but the dentist changes to a different questionnaire at follow-up. What is lost?

A. The ability to inspect an appliance

B. A directly comparable longitudinal symptom measure

C. The right to ask about comfort

D. All ability to discuss sleepiness

E. The diagnostic PSG's validity

CORRECT ANSWER B

Using the same validated measure at baseline and follow-up enables interpretation of change over time.

REFERENCE Integrated notebook, Topic 6, §6.1

QUESTION 0628 · §6.3 · Applied clinical SBA

A 55-year-old feels better after appliance adjustment and asks whether treatment is now effective. Which of the following statements about who verifies efficacy is most accurate?

A. A device-embedded temperature sensor can objectively estimate nightly wear.

B. Persistent symptoms and inadequate objective AHI improvement indicate an inadequate treatment response.

C. Not indicated includes primary snoring, unless symptoms worsen or fail to resolve.

D. The treating physician using appropriate objective sleep testing.

E. Dentist-operated portable data is Used only to guide titration within local scope rules.

CORRECT ANSWER D

The treating physician using appropriate objective sleep testing. The other options describe different mechanisms, investigations or findings rather than who verifies efficacy. Consult Integrated notebook, page 133, §6.3 for the full context.

REFERENCE Integrated notebook, page 133, §6.3

QUESTION 0629 · §6.3 · Clinical interpretation SBA

A dentist is titrating a custom appliance while monitoring discomfort and adherence. The clinical team records the following finding: The treating physician using appropriate objective sleep testing. Which concept best explains this observation?

A. Efficacy

B. Who verifies efficacy

C. Not indicated

D. Appliance condition

E. Dentist-operated portable data

CORRECT ANSWER B

This finding most directly describes Who verifies efficacy: The treating physician using appropriate objective sleep testing. Distractor distinctions: A refers instead to AHI <5 ~37% · AHI <10 ~52% · ≥50% reduction ~64% · ⅔ meaningful benefit, ⅓ none; C refers instead to primary snoring, unless symptoms worsen or fail to resolve; D refers instead to cracks, warping, wear affecting fit and efficacy; confirm retention and stability; E refers instead to Used only to guide titration within local scope rules.

REFERENCE Integrated notebook, page 133, §6.3

QUESTION 0630 · §6.5 · Clinical decision-making SBA

A patient has 50% AHI reduction from 40 to 20 but unchanged daytime symptoms. Which conclusion is most defensible?

A. The patient has no residual disease

B. Numerical improvement is insufficient to label a complete or clearly satisfactory response

C. It is a complete response because 50% fell

D. Snoring must be absent

E. A 20/h result is normal

CORRECT ANSWER B

The notebook requires symptom improvement plus objective reduction for partial response; residual AHI 20 and persisting symptoms demand review.

REFERENCE Integrated notebook, Topic 6, §6.5

QUESTION 0631 · §6.4 · Applied clinical SBA

A patient returns after several weeks of MAD use and says snoring has improved, but fatigue persists. Which of the following statements about further titration, modification, redesign or remake as required is most accurate?

A. The dentist's objective data supplement the titration decision; they never replace physician verification.

B. Long-term oral appliance care requires at least annual dental review after the initial follow-up period.

C. Improved symptoms after initial advancement still require objective confirmation of efficacy.

D. A remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation.

E. Instrumented overnight mandibular positioning can identify an effective advancement during PSG.

CORRECT ANSWER B

Long-term oral appliance care requires at least annual dental review after the initial follow-up period. Additional revision detail: Monitor monthly for the first six months and at least annually thereafter for as long as the appliance is worn. 6.5 Efficacy, long-term effectiveness, limitations. The other options describe different mechanisms, investigations or findings rather than further titration, modification, redesign or remake as required. Consult Integrated notebook, page 133, §6.4 for the full context.

REFERENCE Integrated notebook, page 133, §6.4

QUESTION 0632 · §6.4 · Clinical interpretation SBA

A 55-year-old feels better after appliance adjustment and asks whether treatment is now effective. The clinical team records the following finding: Monitor monthly for the first six months and at least annually thereafter for as long as the appliance is worn. 6.5 Efficacy, long-term effectiveness, limitations. Which concept best explains this observation?

A. Multimethod positioning

B. Further titration, modification, redesign or remake as required

C. Both end in the same place

D. Drug-induced sleep endoscopy

E. Remotely controlled mandibular positioner (RCMP)

CORRECT ANSWER B

This finding most directly describes Further titration, modification, redesign or remake as required: Monitor monthly for the first six months and at least annually thereafter for as long as the appliance is worn. 6.5 Efficacy, long-term effectiveness, limitations. Distractor distinctions: A refers instead to initial positioning → resolution or improvement of signs and symptoms → pulse oximetry, high- resolution oximetry or HSAT adm; C refers instead to The dentist's objective data supplement the titration decision; they never replace physician verification; D refers instead to a remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation; E refers instead to allows objective, single-night titration during polysom- nography to identify the optimal therapeutic advancement.

REFERENCE Integrated notebook, page 133, §6.4

QUESTION 0633 · §6.3 · Clinical decision-making SBA

A patient has residual AHI 17 after comfortable titration. Who should determine whether the remaining disease is acceptably controlled?

A. The appliance manufacturer

B. The patient's bed partner alone

C. The dentist alone without testing

D. An insurer acting as clinical decision-maker

E. The sleep physician in discussion with the patient and dentist

CORRECT ANSWER E

Ongoing abnormal sleep indices require medically supervised evaluation; the dentist supplies fit, tolerance and adherence information.

REFERENCE Integrated notebook, Topic 6, §6.3

QUESTION 0634 · §6.5 · Applied clinical SBA

A sleep specialist receives an update on an oral appliance candidate during titration. Which of the following statements about partial treatment response is most accurate?

A. Persistent symptoms and inadequate objective AHI improvement indicate an inadequate treatment response.

B. Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint.

C. Complete treatment response includes resolution of symptoms and AHI less than five per hour on treatment.

D. Partial treatment response includes improved symptoms with at least a 50% AHI fall but residual AHI at least five.

E. Persistent snoring, no other symptoms may be acceptable to the patient.

CORRECT ANSWER D

Partial treatment response includes improved symptoms with at least a 50% AHI fall but residual AHI at least five. Additional revision detail: Improved symptoms with at least a 50% AHI fall but residual AHI at least five. The other options describe different mechanisms, investigations or findings rather than partial treatment response. Consult Integrated notebook, page 134, §6.5 for the full context.

REFERENCE Integrated notebook, page 134, §6.5

QUESTION 0635 · §6.5 · Clinical interpretation SBA

A patient returns after several weeks of MAD use and says snoring has improved, but fatigue persists. The clinical team records the following finding: Improved symptoms with at least a 50% AHI fall but residual AHI at least five. Which concept best explains this observation?

A. Partial treatment response

B. Persistent snoring, no other symptoms

C. Ongoing clinical failure

D. Both end in the same place

E. Long-term effectiveness

CORRECT ANSWER A

This finding most directly describes Partial treatment response: Improved symptoms with at least a 50% AHI fall but residual AHI at least five. Distractor distinctions: B refers instead to may be acceptable to the patient; but persistent snoring on treatment relates to insufficient apnoea control and poor adheren; C refers instead to Persistent symptoms with insufficient objective improvement; D refers instead to The dentist's objective data supplement the titration decision; they never replace physician verification; E refers instead to Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint; applian.

REFERENCE Integrated notebook, page 134, §6.5

QUESTION 0636 · §6.4 · Clinical decision-making SBA

A 55-year-old is comfortable at 5 mm advancement but develops jaw pain at 7 mm. What is the most appropriate therapeutic target?

A. A fixed 10 mm in all adults

B. 7 mm regardless of pain

C. No protrusion assessment

D. The largest measurable anatomical protrusion

E. The maximally comfortable setting with verified clinical efficacy

CORRECT ANSWER E

Optimal titration seeks the effective dose the patient can tolerate; an excessive protrusive position increases adverse effects.

REFERENCE Integrated notebook, Topic 6, §6.4

QUESTION 0637 · §6.6 · Applied clinical SBA

A patient with previously moderate OSA has reached the maximally comfortable protrusion. Which of the following statements about titration is most accurate?

A. Oral appliances achieved a treated AHI below 5/h in approximately 37% in the studies summarised.

B. Titration includes start at 50% of maximum; 2-week acclimatisation, then 6-week self-adjustment.

C. A device-embedded temperature sensor can objectively estimate nightly wear.

D. Adherence measurements help distinguish inadequate appliance use from inadequate physiological efficacy.

E. AADSM compliance: ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night.

CORRECT ANSWER B

Titration includes start at 50% of maximum; 2-week acclimatisation, then 6-week self-adjustment. Additional revision detail: start at 50% of maximum; 2-week acclimatisation, then 6-week self-adjustment; increments 0.2–0.4 mm per night; total titration ~2–5 months; vertical dimension held constant. The other options describe different mechanisms, investigations or findings rather than titration. Consult Integrated notebook, page 143, §6.6 for the full context.

REFERENCE Integrated notebook, page 143, §6.6

QUESTION 0638 · §6.6 · Clinical interpretation SBA

A sleep specialist receives an update on an oral appliance candidate during titration. The clinical team records the following finding: start at 50% of maximum; 2-week acclimatisation, then 6-week self-adjustment; increments 0.2–0.4 mm per night; total titration ~2–5 months; vertical dimension held constant. Which concept best explains this observation?

A. Recorded adherence technology

B. Titration

C. AADSM compliance

D. Efficacy

E. It distinguishes device inefficacy from poor compliance

CORRECT ANSWER B

This finding most directly describes Titration: start at 50% of maximum; 2-week acclimatisation, then 6-week self-adjustment; increments 0.2–0.4 mm per night; total titration ~2–5 months; vertical dimension held constant. Distractor distinctions: A refers instead to A device-embedded temperature sensor can objectively estimate nightly wear; C refers instead to ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night; D refers instead to AHI <5 ~37% · AHI <10 ~52% · ≥50% reduction ~64% · ⅔ meaningful benefit, ⅓ none; E refers instead to the single most important diagnostic function of adherence data. Without it you cannot know whether to advance the appliance.

REFERENCE Integrated notebook, page 143, §6.6

QUESTION 0639 · §6.6 · Clinical decision-making SBA

A patient reports wearing the appliance every night, but sensor data show use only twice a week. What is the key clinical consequence?

A. A low ESS is sufficient to prove control

B. The physician study becomes irrelevant

C. The appliance is necessarily mismanufactured

D. Low actual use limits effectiveness despite potential physiological efficacy

E. Reported use always outranks objective data

CORRECT ANSWER D

Objective adherence data identify suboptimal usage that can explain inadequate real-world benefit.

REFERENCE Integrated notebook, Topic 6, §6.6

QUESTION 0640 · §6.1 · Applied clinical SBA

A 55-year-old feels better after appliance adjustment and asks whether treatment is now effective. Which of the following statements about early appointment timing is most accurate?

A. Early appointment timing includes reassess within 30 days of insertion.

B. Oral appliance titration commonly requires approximately two to five months.

C. An initial acclimatisation period of roughly two weeks is often used before active titration.

D. Patients may titrate an adjustable appliance gradually over several weeks after acclimatisation.

E. Initial protrusive range includes start commonly around 50% of the measured maximum advancement.

CORRECT ANSWER A

Early appointment timing includes reassess within 30 days of insertion. Additional revision detail: Reassess within 30 days of insertion. The other options describe different mechanisms, investigations or findings rather than early appointment timing. Consult Integrated notebook, page 132, §6.1 for the full context.

REFERENCE Integrated notebook, page 132, §6.1

QUESTION 0641 · §6.1 · Clinical interpretation SBA

A dentist is titrating a custom appliance while monitoring discomfort and adherence. The clinical team records the following finding: Reassess within 30 days of insertion. Which concept best explains this observation?

A. Initial protrusive range

B. Typical overall titration time

C. Patient-led adjustment window

D. Acclimatisation interval

E. Early appointment timing

CORRECT ANSWER E

This finding most directly describes Early appointment timing: Reassess within 30 days of insertion. Distractor distinctions: A refers instead to Start commonly around 50% of the measured maximum advancement; B refers instead to Approximately two to five months; C refers instead to About six weeks of gradual self-adjustment may follow acclimatisation; D refers instead to Allow approximately two weeks at the initial setting.

REFERENCE Integrated notebook, page 132, §6.1

QUESTION 0642 · §6.5 · Clinical decision-making SBA

A patient reaches AHI 4 but continues to report major sleepiness. How should the dentist respond?

A. Seek medical assessment for other sleepiness causes while reviewing OAT use

B. Abandon documentation

C. Declare all sleep symptoms cured

D. Diagnose narcolepsy independently

E. Increase the appliance beyond comfortable range automatically

CORRECT ANSWER A

The OSA index is normalised, but persistent sleepiness may reflect other disorders, inadequate sleep or medication effects.

REFERENCE Integrated notebook, Topic 6, §6.5

QUESTION 0643 · §6.2 · Applied clinical SBA

A patient returns after several weeks of MAD use and says snoring has improved, but fatigue persists. Which of the following statements about jaw and tmj is most accurate?

A. Jaw and tmj includes discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need.

B. Unexpected persistent snoring includes reassess adherence, appliance stability, symptom change and residual OSA.

C. Hypersalivation and dry mouth can occur early in MAD therapy and often diminish during adaptation.

D. Appliance-related oral discomfort may include tooth pain, gingival irritation or a transient change in bite.

E. Appliance condition includes cracks, warping, wear affecting fit and efficacy; confirm retention and stability.

CORRECT ANSWER A

Jaw and tmj includes discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need. Additional revision detail: discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need management or adjustment. The other options describe different mechanisms, investigations or findings rather than jaw and tmj. Consult Integrated notebook, page 132, §6.2 for the full context.

REFERENCE Integrated notebook, page 132, §6.2

QUESTION 0644 · §6.2 · Clinical interpretation SBA

A 55-year-old feels better after appliance adjustment and asks whether treatment is now effective. The clinical team records the following finding: discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need management or adjustment. Which concept best explains this observation?

A. Tolerance and adherence

B. Jaw and TMJ

C. Dental discomfort and bite change

D. Salivation

E. Appliance condition

CORRECT ANSWER B

This finding most directly describes Jaw and TMJ: discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need management or adjustment. Distractor distinctions: A refers instead to comfort with nightly use, and whether side effects are reducing use; C refers instead to tooth pain, gingival irritation, soreness, the bite feeling "off" or teeth not fitting as before; long-term use decreases ove; D refers instead to excessive salivation or dry mouth, common initially and often transient (salivation typically settles after a few weeks) but; E refers instead to cracks, warping, wear affecting fit and efficacy; confirm retention and stability.

REFERENCE Integrated notebook, page 132, §6.2

QUESTION 0645 · §6.1 · Clinical decision-making SBA

At three weeks a MAD user asks how much to advance per turn. Which principle best limits early discomfort?

A. Gradual small increments with symptom- and TMJ-guided reassessment

B. Full end-range advancement in one night

C. Increasing vertical dimension to maximum

D. No follow-up until year five

E. Using pain as the treatment target

CORRECT ANSWER A

The source describes small measurable increments during monitored titration rather than abrupt maximal protrusion.

REFERENCE Integrated notebook, Topic 6, §6.1

QUESTION 0646 · §6.3 · Applied clinical SBA

A sleep specialist receives an update on an oral appliance candidate during titration. Which of the following statements about dentist-operated portable data is most accurate?

A. Dentist-operated portable data is Used only to guide titration within local scope rules.

B. An initial acclimatisation period of roughly two weeks is often used before active titration.

C. Not indicated includes primary snoring, unless symptoms worsen or fail to resolve.

D. The treating physician using appropriate objective sleep testing.

E. Persistent symptoms and inadequate objective AHI improvement indicate an inadequate treatment response.

CORRECT ANSWER A

Dentist-operated portable data is Used only to guide titration within local scope rules. Additional revision detail: Used only to guide titration within local scope rules. The other options describe different mechanisms, investigations or findings rather than dentist-operated portable data. Consult Integrated notebook, page 133, §6.3 for the full context.

REFERENCE Integrated notebook, page 133, §6.3

QUESTION 0647 · §6.3 · Clinical interpretation SBA

A patient returns after several weeks of MAD use and says snoring has improved, but fatigue persists. The clinical team records the following finding: Used only to guide titration within local scope rules. Which concept best explains this observation?

A. Dentist-operated portable data

B. Not indicated

C. Typical overall titration time

D. Who verifies efficacy

E. Patient-led adjustment window

CORRECT ANSWER A

This finding most directly describes Dentist-operated portable data: Used only to guide titration within local scope rules. Distractor distinctions: B refers instead to primary snoring, unless symptoms worsen or fail to resolve; C refers instead to Approximately two to five months; D refers instead to The treating physician using appropriate objective sleep testing; E refers instead to About six weeks of gradual self-adjustment may follow acclimatisation.

REFERENCE Integrated notebook, page 133, §6.3

QUESTION 0648 · §6.4 · Clinical decision-making SBA

On a treatment study, the patient removes the appliance midway through recording because of pain. What should the report include?

A. Assumption that the device was worn continuously

B. An AHI classified as a definitive appliance failure without use context

C. Actual device use and treatment position, with the limitation on interpreting effectiveness

D. Only total recording time

E. No adverse effect history

CORRECT ANSWER C

Interpreting efficacy requires knowing whether the device was present and at what position; incomplete use limits inference.

REFERENCE Integrated notebook, Topic 6, §6.4

QUESTION 0649 · §6.4 · Applied clinical SBA

A patient with previously moderate OSA has reached the maximally comfortable protrusion. Which of the following statements about both end in the same place is most accurate?

A. The dentist's objective data supplement the titration decision; they never replace physician verification.

B. Instrumented overnight mandibular positioning can identify an effective advancement during PSG.

C. A remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation.

D. Long-term oral appliance care requires at least annual dental review after the initial follow-up period.

E. Improved symptoms after initial advancement still require objective confirmation of efficacy.

CORRECT ANSWER A

The dentist's objective data supplement the titration decision; they never replace physician verification. The other options describe different mechanisms, investigations or findings rather than both end in the same place. Consult Integrated notebook, page 133, §6.4 for the full context.

REFERENCE Integrated notebook, page 133, §6.4

QUESTION 0650 · §6.4 · Clinical interpretation SBA

A sleep specialist receives an update on an oral appliance candidate during titration. The clinical team records the following finding: The dentist's objective data supplement the titration decision; they never replace physician verification. Which concept best explains this observation?

A. Multimethod positioning

B. Remotely controlled mandibular positioner (RCMP)

C. Both end in the same place

D. Signs and symptoms only

E. Drug-induced sleep endoscopy

CORRECT ANSWER C

This finding most directly describes Both end in the same place: The dentist's objective data supplement the titration decision; they never replace physician verification. Distractor distinctions: A refers instead to initial positioning → resolution or improvement of signs and symptoms → pulse oximetry, high- resolution oximetry or HSAT adm; B refers instead to allows objective, single-night titration during polysom- nography to identify the optimal therapeutic advancement; D refers instead to initial mandibular positioning → resolution or improvement of signs and symptoms → verification of efficacy by the physician; E refers instead to a remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation.

REFERENCE Integrated notebook, page 133, §6.4

QUESTION 0651 · §6.2 · Clinical decision-making SBA

A patient develops a sore gingival margin without recurrent snoring. What is the immediate device-focused assessment?

A. Prescribe greater mandibular advancement

B. Examine appliance edges, fit, retention and hygiene

C. Assume no problem because AHI was improved

D. Ignore tissue injury until the annual review

E. Use only a STOP-BANG score

CORRECT ANSWER B

Local adverse effects can stem from fit or material contact and should be managed while protecting treatment adherence.

REFERENCE Integrated notebook, Topic 6, §6.2

QUESTION 0652 · §6.5 · Applied clinical SBA

A dentist is titrating a custom appliance while monitoring discomfort and adherence. Which of the following statements about complete treatment response is most accurate?

A. Partial treatment response includes improved symptoms with at least a 50% AHI fall but residual AHI at least five.

B. Persistent symptoms and inadequate objective AHI improvement indicate an inadequate treatment response.

C. Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint.

D. Persistent snoring, no other symptoms may be acceptable to the patient.

E. Complete treatment response includes resolution of symptoms and AHI less than five per hour on treatment.

CORRECT ANSWER E

Complete treatment response includes resolution of symptoms and AHI less than five per hour on treatment. Additional revision detail: Resolution of symptoms and AHI less than five per hour on treatment. The other options describe different mechanisms, investigations or findings rather than complete treatment response. Consult Integrated notebook, page 134, §6.5 for the full context.

REFERENCE Integrated notebook, page 134, §6.5

QUESTION 0653 · §6.5 · Clinical interpretation SBA

A patient with previously moderate OSA has reached the maximally comfortable protrusion. The clinical team records the following finding: Resolution of symptoms and AHI less than five per hour on treatment. Which concept best explains this observation?

A. Complete treatment response

B. Both end in the same place

C. Persistent snoring, no other symptoms

D. Ongoing clinical failure

E. Long-term effectiveness

CORRECT ANSWER A

This finding most directly describes Complete treatment response: Resolution of symptoms and AHI less than five per hour on treatment. Distractor distinctions: B refers instead to The dentist's objective data supplement the titration decision; they never replace physician verification; C refers instead to may be acceptable to the patient; but persistent snoring on treatment relates to insufficient apnoea control and poor adheren; D refers instead to Persistent symptoms with insufficient objective improvement; E refers instead to Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint; applian.

REFERENCE Integrated notebook, page 134, §6.5

QUESTION 0654 · §6.6 · Applied clinical SBA

A patient returns after several weeks of MAD use and says snoring has improved, but fatigue persists. Which of the following statements about aadsm compliance is most accurate?

A. Oral appliances achieved a treated AHI below 5/h in approximately 37% in the studies summarised.

B. AADSM compliance: ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night.

C. A device-embedded temperature sensor can objectively estimate nightly wear.

D. Adherence measurements help distinguish inadequate appliance use from inadequate physiological efficacy.

E. Titration includes start at 50% of maximum; 2-week acclimatisation, then 6-week self-adjustment.

CORRECT ANSWER B

AADSM compliance: ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night. Additional revision detail: ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night. The other options describe different mechanisms, investigations or findings rather than aadsm compliance. Consult Integrated notebook, page 143, §6.6 for the full context.

REFERENCE Integrated notebook, page 143, §6.6

QUESTION 0655 · §6.6 · Clinical interpretation SBA

A 55-year-old feels better after appliance adjustment and asks whether treatment is now effective. The clinical team records the following finding: ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night. Which concept best explains this observation?

A. AADSM compliance

B. Titration

C. Recorded adherence technology

D. Efficacy

E. It distinguishes device inefficacy from poor compliance

CORRECT ANSWER A

This finding most directly describes AADSM compliance: ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night. Distractor distinctions: B refers instead to start at 50% of maximum; 2-week acclimatisation, then 6-week self-adjustment; increments 0.2–0.4 mm per night; total titratio; C refers instead to A device-embedded temperature sensor can objectively estimate nightly wear; D refers instead to AHI <5 ~37% · AHI <10 ~52% · ≥50% reduction ~64% · ⅔ meaningful benefit, ⅓ none; E refers instead to the single most important diagnostic function of adherence data. Without it you cannot know whether to advance the appliance.

REFERENCE Integrated notebook, page 143, §6.6

QUESTION 0656 · §6.1 · Applied clinical SBA

A dentist is titrating a custom appliance while monitoring discomfort and adherence. Which of the following statements about why revert is most accurate?

A. Why revert includes to resolve unacceptable side effects; to balance efficacy against tolerance.

B. Initial protrusive range includes start commonly around 50% of the measured maximum advancement.

C. An initial acclimatisation period of roughly two weeks is often used before active titration.

D. Patients may titrate an adjustable appliance gradually over several weeks after acclimatisation.

E. Early appointment timing includes reassess within 30 days of insertion.

CORRECT ANSWER A

Why revert includes to resolve unacceptable side effects; to balance efficacy against tolerance. Additional revision detail: to resolve unacceptable side effects; to balance efficacy against tolerance. The other options describe different mechanisms, investigations or findings rather than why revert. Consult Integrated notebook, page 132, §6.1 for the full context.

REFERENCE Integrated notebook, page 132, §6.1

QUESTION 0657 · §6.1 · Clinical interpretation SBA

A patient with previously moderate OSA has reached the maximally comfortable protrusion. The clinical team records the following finding: to resolve unacceptable side effects; to balance efficacy against tolerance. Which concept best explains this observation?

A. Initial protrusive range

B. Patient-led adjustment window

C. Early appointment timing

D. Why revert

E. Acclimatisation interval

CORRECT ANSWER D

This finding most directly describes Why revert: to resolve unacceptable side effects; to balance efficacy against tolerance. Distractor distinctions: A refers instead to Start commonly around 50% of the measured maximum advancement; B refers instead to About six weeks of gradual self-adjustment may follow acclimatisation; C refers instead to Reassess within 30 days of insertion; E refers instead to Allow approximately two weeks at the initial setting.

REFERENCE Integrated notebook, page 132, §6.1

QUESTION 0658 · §6.2 · Applied clinical SBA

A patient returns after several weeks of MAD use and says snoring has improved, but fatigue persists. Which of the following statements about appliance condition is most accurate?

A. Comfort with nightly use, and whether side effects are reducing use.

B. Jaw and tmj includes discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need.

C. Hypersalivation and dry mouth can occur early in MAD therapy and often diminish during adaptation.

D. Unexpected persistent snoring includes reassess adherence, appliance stability, symptom change and residual OSA.

E. Appliance condition includes cracks, warping, wear affecting fit and efficacy; confirm retention and stability.

CORRECT ANSWER E

Appliance condition includes cracks, warping, wear affecting fit and efficacy; confirm retention and stability. Additional revision detail: cracks, warping, wear affecting fit and efficacy; confirm retention and stability. The other options describe different mechanisms, investigations or findings rather than appliance condition. Consult Integrated notebook, page 132, §6.2 for the full context.

REFERENCE Integrated notebook, page 132, §6.2

QUESTION 0659 · §6.2 · Clinical interpretation SBA

A 55-year-old feels better after appliance adjustment and asks whether treatment is now effective. The clinical team records the following finding: cracks, warping, wear affecting fit and efficacy; confirm retention and stability. Which concept best explains this observation?

A. Jaw and TMJ

B. Salivation

C. Tolerance and adherence

D. Unexpected persistent snoring

E. Appliance condition

CORRECT ANSWER E

This finding most directly describes Appliance condition: cracks, warping, wear affecting fit and efficacy; confirm retention and stability. Distractor distinctions: A refers instead to discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need management or adjust; B refers instead to excessive salivation or dry mouth, common initially and often transient (salivation typically settles after a few weeks) but; C refers instead to comfort with nightly use, and whether side effects are reducing use; D refers instead to Reassess adherence, appliance stability, symptom change and residual OSA.

REFERENCE Integrated notebook, page 132, §6.2

QUESTION 0660 · §6.4 · Applied clinical SBA

A sleep specialist receives an update on an oral appliance candidate during titration. Which of the following statements about signs and symptoms only is most accurate?

A. The dentist's objective data supplement the titration decision; they never replace physician verification.

B. A remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation.

C. Instrumented overnight mandibular positioning can identify an effective advancement during PSG.

D. Improved symptoms after initial advancement still require objective confirmation of efficacy.

E. Long-term oral appliance care requires at least annual dental review after the initial follow-up period.

CORRECT ANSWER D

Improved symptoms after initial advancement still require objective confirmation of efficacy. Additional revision detail: initial mandibular positioning → resolution or improvement of signs and symptoms → verification of efficacy by the physician. The other options describe different mechanisms, investigations or findings rather than signs and symptoms only. Consult Integrated notebook, page 133, §6.4 for the full context.

REFERENCE Integrated notebook, page 133, §6.4

QUESTION 0661 · §6.4 · Clinical interpretation SBA

A patient returns after several weeks of MAD use and says snoring has improved, but fatigue persists. The clinical team records the following finding: initial mandibular positioning → resolution or improvement of signs and symptoms → verification of efficacy by the physician. Which concept best explains this observation?

A. Signs and symptoms only

B. Multimethod positioning

C. Remotely controlled mandibular positioner (RCMP)

D. Drug-induced sleep endoscopy

E. Both end in the same place

CORRECT ANSWER A

This finding most directly describes Signs and symptoms only: initial mandibular positioning → resolution or improvement of signs and symptoms → verification of efficacy by the physician. Distractor distinctions: B refers instead to initial positioning → resolution or improvement of signs and symptoms → pulse oximetry, high- resolution oximetry or HSAT adm; C refers instead to allows objective, single-night titration during polysom- nography to identify the optimal therapeutic advancement; D refers instead to a remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation; E refers instead to The dentist's objective data supplement the titration decision; they never replace physician verification.

REFERENCE Integrated notebook, page 133, §6.4

QUESTION 0662 · §6.5 · Applied clinical SBA

A dentist is titrating a custom appliance while monitoring discomfort and adherence. Which of the following statements about long-term effectiveness is most accurate?

A. Complete treatment response includes resolution of symptoms and AHI less than five per hour on treatment.

B. Persistent symptoms and inadequate objective AHI improvement indicate an inadequate treatment response.

C. Persistent snoring, no other symptoms may be acceptable to the patient.

D. Partial treatment response includes improved symptoms with at least a 50% AHI fall but residual AHI at least five.

E. Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint.

CORRECT ANSWER E

Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint. Additional revision detail: Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint; appliances also benefit blood pressure, endothelial function and left ventricular function. The other options describe different mechanisms, investigations or findings rather than long-term effectiveness. Consult Integrated notebook, page 133, §6.5 for the full context.

REFERENCE Integrated notebook, page 133, §6.5

QUESTION 0663 · §6.5 · Clinical interpretation SBA

A patient with previously moderate OSA has reached the maximally comfortable protrusion. The clinical team records the following finding: Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint; appliances also benefit blood pressure, endothelial function and left ventricular function. Which concept best explains this observation?

A. Persistent snoring, no other symptoms

B. Ongoing clinical failure

C. Partial treatment response

D. Complete treatment response

E. Long-term effectiveness

CORRECT ANSWER E

This finding most directly describes Long-term effectiveness: Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint; appliances also benefit blood pressure, endothelial function and left ventricular function. Distractor distinctions: A refers instead to may be acceptable to the patient; but persistent snoring on treatment relates to insufficient apnoea control and poor adheren; B refers instead to Persistent symptoms with insufficient objective improvement; C refers instead to Improved symptoms with at least a 50% AHI fall but residual AHI at least five; D refers instead to Resolution of symptoms and AHI less than five per hour on treatment.

REFERENCE Integrated notebook, page 133, §6.5

QUESTION 0664 · §6.6 · Applied clinical SBA

A patient returns after several weeks of MAD use and says snoring has improved, but fatigue persists. Which of the following statements about efficacy is most accurate?

A. Adherence measurements help distinguish inadequate appliance use from inadequate physiological efficacy.

B. Titration includes start at 50% of maximum; 2-week acclimatisation, then 6-week self-adjustment.

C. A device-embedded temperature sensor can objectively estimate nightly wear.

D. Oral appliances achieved a treated AHI below 5/h in approximately 37% in the studies summarised.

E. AADSM compliance: ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night.

CORRECT ANSWER D

Oral appliances achieved a treated AHI below 5/h in approximately 37% in the studies summarised. Additional revision detail: AHI <5 ~37% · AHI <10 ~52% · ≥50% reduction ~64% · ⅔ meaningful benefit, ⅓ none. The other options describe different mechanisms, investigations or findings rather than efficacy. Consult Integrated notebook, page 143, §6.6 for the full context.

REFERENCE Integrated notebook, page 143, §6.6

QUESTION 0665 · §6.6 · Focused knowledge SBA

A 55-year-old feels better after appliance adjustment and asks whether treatment is now effective. Which of the following statements about efficacy is most accurate?

A. Adherence measurements help distinguish inadequate appliance use from inadequate physiological efficacy.

B. AADSM compliance: ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night.

C. Oral appliances achieved a treated AHI below 5/h in approximately 37% in the studies summarised.

D. A device-embedded temperature sensor can objectively estimate nightly wear.

E. Titration includes start at 50% of maximum; 2-week acclimatisation, then 6-week self-adjustment.

CORRECT ANSWER C

Oral appliances achieved a treated AHI below 5/h in approximately 37% in the studies summarised. Additional revision detail: AHI <5 ~37% · AHI <10 ~52% · ≥50% reduction ~64% · ⅔ meaningful benefit, ⅓ none. The other options describe different mechanisms, investigations or findings rather than efficacy. Consult Integrated notebook, page 143, §6.6 for the full context.

REFERENCE Integrated notebook, page 143, §6.6

QUESTION 0666 · §6.1 · Applied clinical SBA

A dentist is titrating a custom appliance while monitoring discomfort and adherence. Which of the following statements about patient-led adjustment window is most accurate?

A. An initial acclimatisation period of roughly two weeks is often used before active titration.

B. Why revert includes to resolve unacceptable side effects; to balance efficacy against tolerance.

C. Early appointment timing includes reassess within 30 days of insertion.

D. Patients may titrate an adjustable appliance gradually over several weeks after acclimatisation.

E. Initial protrusive range includes start commonly around 50% of the measured maximum advancement.

CORRECT ANSWER D

Patients may titrate an adjustable appliance gradually over several weeks after acclimatisation. Additional revision detail: About six weeks of gradual self-adjustment may follow acclimatisation. The other options describe different mechanisms, investigations or findings rather than patient-led adjustment window. Consult Integrated notebook, page 132, §6.1 for the full context.

REFERENCE Integrated notebook, page 132, §6.1

QUESTION 0667 · §6.1 · Clinical interpretation SBA

A patient with previously moderate OSA has reached the maximally comfortable protrusion. The clinical team records the following finding: About six weeks of gradual self-adjustment may follow acclimatisation. Which concept best explains this observation?

A. Acclimatisation interval

B. Patient-led adjustment window

C. Typical overall titration time

D. Initial protrusive range

E. Early appointment timing

CORRECT ANSWER B

This finding most directly describes Patient-led adjustment window: About six weeks of gradual self-adjustment may follow acclimatisation. Distractor distinctions: A refers instead to Allow approximately two weeks at the initial setting; C refers instead to Approximately two to five months; D refers instead to Start commonly around 50% of the measured maximum advancement; E refers instead to Reassess within 30 days of insertion.

REFERENCE Integrated notebook, page 132, §6.1

QUESTION 0668 · §6.2 · Applied clinical SBA

A patient returns after several weeks of MAD use and says snoring has improved, but fatigue persists. Which of the following statements about salivation is most accurate?

A. Appliance-related oral discomfort may include tooth pain, gingival irritation or a transient change in bite.

B. Jaw and tmj includes discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need.

C. Hypersalivation and dry mouth can occur early in MAD therapy and often diminish during adaptation.

D. Unexpected persistent snoring includes reassess adherence, appliance stability, symptom change and residual OSA.

E. Comfort with nightly use, and whether side effects are reducing use.

CORRECT ANSWER C

Hypersalivation and dry mouth can occur early in MAD therapy and often diminish during adaptation. Additional revision detail: excessive salivation or dry mouth, common initially and often transient (salivation typically settles after a few weeks) but document and manage. The other options describe different mechanisms, investigations or findings rather than salivation. Consult Integrated notebook, page 132, §6.2 for the full context.

REFERENCE Integrated notebook, page 132, §6.2

QUESTION 0669 · §6.2 · Clinical interpretation SBA

A 55-year-old feels better after appliance adjustment and asks whether treatment is now effective. The clinical team records the following finding: excessive salivation or dry mouth, common initially and often transient (salivation typically settles after a few weeks) but document and manage. Which concept best explains this observation?

A. Salivation

B. Appliance condition

C. Tolerance and adherence

D. Jaw and TMJ

E. Unexpected persistent snoring

CORRECT ANSWER A

This finding most directly describes Salivation: excessive salivation or dry mouth, common initially and often transient (salivation typically settles after a few weeks) but document and manage. Distractor distinctions: B refers instead to cracks, warping, wear affecting fit and efficacy; confirm retention and stability; C refers instead to comfort with nightly use, and whether side effects are reducing use; D refers instead to discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need management or adjust; E refers instead to Reassess adherence, appliance stability, symptom change and residual OSA.

REFERENCE Integrated notebook, page 132, §6.2

QUESTION 0670 · §6.4 · Applied clinical SBA

A sleep specialist receives an update on an oral appliance candidate during titration. Which of the following statements about drug-induced sleep endoscopy is most accurate?

A. Improved symptoms after initial advancement still require objective confirmation of efficacy.

B. The dentist's objective data supplement the titration decision; they never replace physician verification.

C. Instrumented overnight mandibular positioning can identify an effective advancement during PSG.

D. A remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation.

E. Long-term oral appliance care requires at least annual dental review after the initial follow-up period.

CORRECT ANSWER D

A remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation. The other options describe different mechanisms, investigations or findings rather than drug-induced sleep endoscopy. Consult Integrated notebook, page 139, §6.4 for the full context.

REFERENCE Integrated notebook, page 139, §6.4

QUESTION 0671 · §6.4 · Clinical interpretation SBA

A patient returns after several weeks of MAD use and says snoring has improved, but fatigue persists. The clinical team records the following finding: a remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation. Which concept best explains this observation?

A. Signs and symptoms only

B. Drug-induced sleep endoscopy

C. Remotely controlled mandibular positioner (RCMP)

D. Multimethod positioning

E. Both end in the same place

CORRECT ANSWER B

This finding most directly describes Drug-induced sleep endoscopy: a remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation. Distractor distinctions: A refers instead to initial mandibular positioning → resolution or improvement of signs and symptoms → verification of efficacy by the physician; C refers instead to allows objective, single-night titration during polysom- nography to identify the optimal therapeutic advancement; D refers instead to initial positioning → resolution or improvement of signs and symptoms → pulse oximetry, high- resolution oximetry or HSAT adm; E refers instead to The dentist's objective data supplement the titration decision; they never replace physician verification.

REFERENCE Integrated notebook, page 139, §6.4

QUESTION 0672 · §6.5 · Applied clinical SBA

A dentist is titrating a custom appliance while monitoring discomfort and adherence. Which of the following statements about ongoing clinical failure is most accurate?

A. Persistent symptoms and inadequate objective AHI improvement indicate an inadequate treatment response.

B. Persistent snoring, no other symptoms may be acceptable to the patient.

C. Partial treatment response includes improved symptoms with at least a 50% AHI fall but residual AHI at least five.

D. Complete treatment response includes resolution of symptoms and AHI less than five per hour on treatment.

E. Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint.

CORRECT ANSWER A

Persistent symptoms and inadequate objective AHI improvement indicate an inadequate treatment response. Additional revision detail: Persistent symptoms with insufficient objective improvement. The other options describe different mechanisms, investigations or findings rather than ongoing clinical failure. Consult Integrated notebook, page 134, §6.5 for the full context.

REFERENCE Integrated notebook, page 134, §6.5

QUESTION 0673 · §6.5 · Clinical interpretation SBA

A patient with previously moderate OSA has reached the maximally comfortable protrusion. The clinical team records the following finding: Persistent symptoms with insufficient objective improvement. Which concept best explains this observation?

A. Persistent snoring, no other symptoms

B. Complete treatment response

C. Partial treatment response

D. Ongoing clinical failure

E. Long-term effectiveness

CORRECT ANSWER D

This finding most directly describes Ongoing clinical failure: Persistent symptoms with insufficient objective improvement. Distractor distinctions: A refers instead to may be acceptable to the patient; but persistent snoring on treatment relates to insufficient apnoea control and poor adheren; B refers instead to Resolution of symptoms and AHI less than five per hour on treatment; C refers instead to Improved symptoms with at least a 50% AHI fall but residual AHI at least five; E refers instead to Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint; applian.

REFERENCE Integrated notebook, page 134, §6.5

QUESTION 0674 · §6.6 · Applied clinical SBA

A patient returns after several weeks of MAD use and says snoring has improved, but fatigue persists. Which of the following statements about recorded adherence technology is most accurate?

A. A device-embedded temperature sensor can objectively estimate nightly wear.

B. Titration includes start at 50% of maximum; 2-week acclimatisation, then 6-week self-adjustment.

C. AADSM compliance: ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night.

D. Adherence measurements help distinguish inadequate appliance use from inadequate physiological efficacy.

E. Oral appliances achieved a treated AHI below 5/h in approximately 37% in the studies summarised.

CORRECT ANSWER A

A device-embedded temperature sensor can objectively estimate nightly wear. The other options describe different mechanisms, investigations or findings rather than recorded adherence technology. Consult Integrated notebook, page 135, §6.6 for the full context.

REFERENCE Integrated notebook, page 135, §6.6

QUESTION 0675 · §6.6 · Clinical interpretation SBA

A 55-year-old feels better after appliance adjustment and asks whether treatment is now effective. The clinical team records the following finding: A device-embedded temperature sensor can objectively estimate nightly wear. Which concept best explains this observation?

A. Recorded adherence technology

B. Efficacy

C. It distinguishes device inefficacy from poor compliance

D. Titration

E. AADSM compliance

CORRECT ANSWER A

This finding most directly describes Recorded adherence technology: A device-embedded temperature sensor can objectively estimate nightly wear. Distractor distinctions: B refers instead to AHI <5 ~37% · AHI <10 ~52% · ≥50% reduction ~64% · ⅔ meaningful benefit, ⅓ none; C refers instead to the single most important diagnostic function of adherence data. Without it you cannot know whether to advance the appliance; D refers instead to start at 50% of maximum; 2-week acclimatisation, then 6-week self-adjustment; increments 0.2–0.4 mm per night; total titratio; E refers instead to ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night.

REFERENCE Integrated notebook, page 135, §6.6

QUESTION 0676 · §6.1 · Applied clinical SBA

A dentist is titrating a custom appliance while monitoring discomfort and adherence. Which of the following statements about typical overall titration time is most accurate?

A. Oral appliance titration commonly requires approximately two to five months.

B. Initial protrusive range includes start commonly around 50% of the measured maximum advancement.

C. Early appointment timing includes reassess within 30 days of insertion.

D. An initial acclimatisation period of roughly two weeks is often used before active titration.

E. Patients may titrate an adjustable appliance gradually over several weeks after acclimatisation.

CORRECT ANSWER A

Oral appliance titration commonly requires approximately two to five months. Additional revision detail: Approximately two to five months. The other options describe different mechanisms, investigations or findings rather than typical overall titration time. Consult Integrated notebook, page 132, §6.1 for the full context.

REFERENCE Integrated notebook, page 132, §6.1

QUESTION 0677 · §6.1 · Clinical interpretation SBA

A patient with previously moderate OSA has reached the maximally comfortable protrusion. The clinical team records the following finding: Approximately two to five months. Which concept best explains this observation?

A. Patient-led adjustment window

B. Early appointment timing

C. Acclimatisation interval

D. Initial protrusive range

E. Typical overall titration time

CORRECT ANSWER E

This finding most directly describes Typical overall titration time: Approximately two to five months. Distractor distinctions: A refers instead to About six weeks of gradual self-adjustment may follow acclimatisation; B refers instead to Reassess within 30 days of insertion; C refers instead to Allow approximately two weeks at the initial setting; D refers instead to Start commonly around 50% of the measured maximum advancement.

REFERENCE Integrated notebook, page 132, §6.1

QUESTION 0678 · §6.2 · Applied clinical SBA

A patient returns after several weeks of MAD use and says snoring has improved, but fatigue persists. Which of the following statements about tolerance and adherence is most accurate?

A. Unexpected persistent snoring includes reassess adherence, appliance stability, symptom change and residual OSA.

B. Hypersalivation and dry mouth can occur early in MAD therapy and often diminish during adaptation.

C. Comfort with nightly use, and whether side effects are reducing use.

D. Jaw and tmj includes discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need.

E. Appliance condition includes cracks, warping, wear affecting fit and efficacy; confirm retention and stability.

CORRECT ANSWER C

Comfort with nightly use, and whether side effects are reducing use. The other options describe different mechanisms, investigations or findings rather than tolerance and adherence. Consult Integrated notebook, page 132, §6.2 for the full context.

REFERENCE Integrated notebook, page 132, §6.2

QUESTION 0679 · §6.2 · Clinical interpretation SBA

A 55-year-old feels better after appliance adjustment and asks whether treatment is now effective. The clinical team records the following finding: comfort with nightly use, and whether side effects are reducing use. Which concept best explains this observation?

A. Dental discomfort and bite change

B. Unexpected persistent snoring

C. Jaw and TMJ

D. Appliance condition

E. Tolerance and adherence

CORRECT ANSWER E

This finding most directly describes Tolerance and adherence: comfort with nightly use, and whether side effects are reducing use. Distractor distinctions: A refers instead to tooth pain, gingival irritation, soreness, the bite feeling "off" or teeth not fitting as before; long-term use decreases ove; B refers instead to Reassess adherence, appliance stability, symptom change and residual OSA; C refers instead to discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need management or adjust; D refers instead to cracks, warping, wear affecting fit and efficacy; confirm retention and stability.

REFERENCE Integrated notebook, page 132, §6.2

QUESTION 0680 · §6.4 · Clinical interpretation SBA

A sleep specialist receives an update on an oral appliance candidate during titration. The clinical team records the following finding: allows objective, single-night titration during polysomnography to identify the optimal therapeutic advancement. Which concept best explains this observation?

A. Drug-induced sleep endoscopy

B. Both end in the same place

C. Multimethod positioning

D. Remotely controlled mandibular positioner (RCMP)

E. Further titration, modification, redesign or remake as required

CORRECT ANSWER D

This finding most directly describes Remotely controlled mandibular positioner (RCMP): allows objective, single-night titration during polysom- nography to identify the optimal therapeutic advancement. Distractor distinctions: A refers instead to a remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation; B refers instead to The dentist's objective data supplement the titration decision; they never replace physician verification; C refers instead to initial positioning → resolution or improvement of signs and symptoms → pulse oximetry, high- resolution oximetry or HSAT adm; E refers instead to Monitor monthly for the first six months and at least annually thereafter for as long as the appliance is worn. 6.5 Efficacy,.

REFERENCE Integrated notebook, page 139, §6.4

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07 Long-term oral appliance follow-up

QUESTION 0681 · §7.1 · Clinical decision-making SBA

Three years after fitting, a patient gains 12 kg and develops recurrent snoring despite unchanged appliance setting. What should be assessed?

A. Possible disease progression with weight-related increased airway burden

B. Diagnose the new severity by BMI alone

C. Stop all follow-up after the first year

D. Assume the appliance still normalises AHI

E. Replace the device without a medical review

CORRECT ANSWER A

Weight gain is a major modifier of OSA and may reduce an appliance's effectiveness, prompting symptom, device and physician review.

REFERENCE Integrated notebook, Topic 7, §7.1

QUESTION 0682 · §7.3 · Clinical decision-making SBA

A long-term MAD user previously achieved good OSA control. Following a 12% weight increase, the partner again notices apnoeic pauses. What is the most appropriate management?

A. Assume the old result is still valid

B. Arrange re-evaluation of symptoms, device adherence and physician-led sleep testing

C. Replace the appliance without any reassessment

D. Discharge the patient permanently from follow-up

E. Increase the device to maximum protrusion without examination

CORRECT ANSWER B

Weight change and recurrent symptoms may indicate worsening disease. Long-term surveillance should trigger clinical and objective reassessment rather than reliance on old efficacy results. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 7, §7.3

QUESTION 0683 · §7.1 · Clinical decision-making SBA

A long-term MAD user starts taking sedative medication and reports renewed snoring. Which factor is most pertinent to investigate?

A. Whether sedatives can change airway tone and arousal responses

B. The colour of the aligner case

C. Whether the device has an older trademark

D. The brand of toothbrush used

E. Whether the patient now sleeps on a different pillow only

CORRECT ANSWER A

Medication changes, particularly sedatives and opioids, may alter sleep-related breathing and should be reviewed alongside weight, alcohol and positional factors. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 7, §7.1

QUESTION 0684 · §7.2 · Clinical decision-making SBA

At baseline a patient recorded ESS and partner-rated snoring. Which approach best supports longitudinal comparison after years of OAT?

A. Use a different unvalidated questionnaire at each visit

B. Repeat comparable baseline measures and document changes alongside objective reassessment when needed

C. Rely only on patient memory of the first visit

D. Record no data if the device fits

E. Use overjet alone as a surrogate for OSA severity

CORRECT ANSWER B

Standardised repeated measures support longitudinal clinical assessment, although objective medical evaluation remains important when efficacy is uncertain. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 7, §7.2

QUESTION 0685 · §7.4 · Clinical decision-making SBA

At an annual review, a patient reports good nightly MAD use but has progressive overbite reduction. Which approach is best?

A. Ignore bite changes because they are unrelated to OAT

B. Check and document occlusion against baseline, discuss the change and review mitigation options

C. Stop all follow-up until jaw pain develops

D. Perform immediate irreversible bite correction

E. Treat good adherence as proof there are no adverse effects

CORRECT ANSWER B

Long-term follow-up includes dental wear, retention, occlusal change, symptoms and adherence; document objective changes and discuss clinical trade-offs. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 7, §7.4

QUESTION 0686 · §7.1 · Clinical decision-making SBA

A previously successful patient begins using a sedating muscle relaxant at night. What should be documented?

A. A promise that adjustment will correct every drug effect

B. An assumption that medication cannot affect OSA

C. Only the device's colour

D. Only the patient's dental shade

E. Medication change as a possible contributor to recurrent airway collapse

CORRECT ANSWER E

Sedatives and muscle relaxants may worsen respiratory control or muscle tone; coordinate with the prescribing clinician.

REFERENCE Integrated notebook, Topic 7, §7.1

QUESTION 0687 · §7.2 · Applied clinical SBA

An established MAD user reports a change in alcohol intake and daytime symptoms. Which of the following statements about evolution tracking is most accurate?

A. Longitudinal care should document symptoms with repeated questionnaires and structured review.

B. Partner reports can document snoring, witnessed apnoeas and restless sleep.

C. Medication change includes drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids.

D. Before sleep it exacerbates collapse and increases severity, undermining benefit.

E. Subjective review uses patient-reported sleepiness, symptoms, sleep diaries and treatment experience.

CORRECT ANSWER A

Longitudinal care should document symptoms with repeated questionnaires and structured review. Additional revision detail: regular follow-ups, app-based tracking, and repeated administration of the same baseline questionnaires. Objective data integration — compliance-monitor data, CPAP usage logs, polysomnography, oximetry. The other options describe different mechanisms, investigations or findings rather than evolution tracking. Consult Integrated notebook, page 146, §7.2 for the full context.

REFERENCE Integrated notebook, page 146, §7.2

QUESTION 0688 · §7.2 · Clinical interpretation SBA

A 61-year-old who has used a MAD for four years attends an annual dental review. The clinical team records the following finding: regular follow-ups, app-based tracking, and repeated administration of the same baseline questionnaires. Objective data integration — compliance-monitor data, CPAP usage logs, polysomnography, oximetry. Which concept best explains this observation?

A. Evolution tracking

B. Self-reported symptoms

C. Medication change

D. Sleep-observer reports

E. Key considerations

CORRECT ANSWER A

This finding most directly describes Evolution tracking: regular follow-ups, app-based tracking, and repeated administration of the same baseline questionnaires. Objective data integration — compliance-monitor data, CPAP usage logs, polysomnography, oximetry. Distractor distinctions: B refers instead to the patient's perception of severity and therapy effect: ESS, sleep diaries, mobile applications; C refers instead to drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter a; D refers instead to the bed partner or family on snoring intensity, apnoeas and restless sleep, offering "objective-seeming" data that complement; E refers instead to the optimal position is unique to each patient and determined by careful repeated titration.

REFERENCE Integrated notebook, page 146, §7.2

QUESTION 0689 · §7.2 · Clinical decision-making SBA

The patient says there is no snoring, but a partner witnesses pauses and choking again. What is the best response?

A. Assume absence of self-reported symptoms proves efficacy

B. Set the device at maximal protrusion immediately

C. Ignore partner reports

D. Diagnose new events without testing

E. Consider observer evidence and seek medical reassessment where indicated

CORRECT ANSWER E

Long-term review combines self-report, bed-partner observations and objective testing when deterioration is suspected.

REFERENCE Integrated notebook, Topic 7, §7.2

QUESTION 0690 · §7.3 · Applied clinical SBA

A patient returns after several years of nightly OAT with a new occlusal complaint. Which of the following statements about key considerations is most accurate?

A. Subjective review uses patient-reported sleepiness, symptoms, sleep diaries and treatment experience.

B. Sleep apnoea worsens over time, and the optimal advancement needed to keep the airway open may increase.

C. The optimal position is unique to each patient and determined by careful repeated titration.

D. Prolonged MAD use can reduce overjet and overbite through dental movements.

E. Ensure the appliance does not interfere with the bite or cause premature contacts.

CORRECT ANSWER C

The optimal position is unique to each patient and determined by careful repeated titration. The other options describe different mechanisms, investigations or findings rather than key considerations. Consult Integrated notebook, page 147, §7.3 for the full context.

REFERENCE Integrated notebook, page 147, §7.3

QUESTION 0691 · §7.3 · Clinical interpretation SBA

An established MAD user reports a change in alcohol intake and daytime symptoms. The clinical team records the following finding: the optimal position is unique to each patient and determined by careful repeated titration. Which concept best explains this observation?

A. Evolution tracking

B. Key considerations

C. Long-term dental change

D. Medication change

E. Progression of the disease

CORRECT ANSWER B

This finding most directly describes Key considerations: the optimal position is unique to each patient and determined by careful repeated titration. Distractor distinctions: A refers instead to regular follow-ups, app-based tracking, and repeated administration of the same baseline questionnaires. Objective data integ; C refers instead to Prolonged appliance use produces dentoskeletal change — reduced overbite and overjet, altered incisor position; D refers instead to drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter a; E refers instead to Sleep apnoea worsens over time, and the optimal advancement needed to keep the airway open may increase.

REFERENCE Integrated notebook, page 147, §7.3

QUESTION 0692 · §7.4 · Clinical decision-making SBA

At an annual review, the device looks intact but the advancement mechanism slips during sleep. What is the priority?

A. Assume appearance proves function

B. Repair or remake the mechanism and verify the retained protrusive setting

C. Stop all cleaning instructions

D. Disregard slipping if the patient is happy

E. Document no changes

CORRECT ANSWER B

Hardware functionality and stable settings are essential; visible condition alone cannot prove continued delivery of therapy.

REFERENCE Integrated notebook, Topic 7, §7.4

QUESTION 0693 · §7.4 · Applied clinical SBA

A long-term appliance user asks whether routine reviews are still necessary. Which of the following statements about better response is most accurate?

A. Ensure the appliance does not interfere with the bite or cause premature contacts.

B. Better response includes younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant.

C. Poorer response includes supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment.

D. Long-term oral appliance care requires at least annual dental review after the initial follow-up period.

E. Retention includes gently tug to verify the device resists displacement during normal oral function.

CORRECT ANSWER B

Better response includes younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant. Additional revision detail: younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant. The other options describe different mechanisms, investigations or findings rather than better response. Consult Integrated notebook, page 147, §7.4 for the full context.

REFERENCE Integrated notebook, page 147, §7.4

QUESTION 0694 · §7.4 · Clinical interpretation SBA

A patient returns after several years of nightly OAT with a new occlusal complaint. The clinical team records the following finding: younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant. Which concept best explains this observation?

A. Material wear

B. Better response

C. Occlusal contacts

D. Seating accuracy

E. Poorer response

CORRECT ANSWER B

This finding most directly describes Better response: younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant. Distractor distinctions: A refers instead to note discolouration, thinning or deformation of the acrylic or thermoplastic; C refers instead to ensure the appliance does not interfere with the bite or cause premature contacts; D refers instead to confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking; E refers instead to supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment.

REFERENCE Integrated notebook, page 147, §7.4

QUESTION 0695 · §7.4 · Clinical decision-making SBA

An old appliance has accumulated deposits, loss of retention and tooth migration. What should the review include?

A. No dental examination because OSA is medical

B. Hygiene, device stability, occlusion and treatment redesign if needed

C. An automatic prescription for CPAP without discussion

D. Only a snoring recording

E. Only an ESS score

CORRECT ANSWER B

Yearly dental review examines wear, hygiene, fitting, occlusal change, adherence and evolving medical risk.

REFERENCE Integrated notebook, Topic 7, §7.4

QUESTION 0696 · §7.1 · Applied clinical SBA

A long-term appliance user asks whether routine reviews are still necessary. Which of the following statements about weight change is most accurate?

A. Gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes.

B. Medication change includes drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids.

C. Before sleep it exacerbates collapse and increases severity, undermining benefit.

D. Better long-term response includes younger; less obese with smaller neck circumference (men); lower baseline AHI.

E. Adequate sleep opportunity and appropriate sleep habits can improve overall symptom management.

CORRECT ANSWER A

Gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes. The other options describe different mechanisms, investigations or findings rather than weight change. Consult Integrated notebook, page 146, §7.1 for the full context.

REFERENCE Integrated notebook, page 146, §7.1

QUESTION 0697 · §7.1 · Clinical interpretation SBA

A patient returns after several years of nightly OAT with a new occlusal complaint. The clinical team records the following finding: gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes. Which concept best explains this observation?

A. Weight change

B. Alcohol

C. Better long-term response

D. Sleep hygiene and quantity

E. Medication change

CORRECT ANSWER A

This finding most directly describes Weight change: gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes. Distractor distinctions: B refers instead to before sleep it exacerbates collapse and increases severity, undermining benefit; even moderate intake may cause transient de; C refers instead to younger; less obese with smaller neck circumference (men); lower baseline AHI; D refers instead to good hygiene and adequate duration support success by stabilising architecture and reducing factors that worsen apnoea; E refers instead to drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter a.

REFERENCE Integrated notebook, page 146, §7.1

QUESTION 0698 · §7.3 · Clinical decision-making SBA

A patient with worsening OSA cannot tolerate additional protrusion. What is the best plan?

A. Discuss multidisciplinary alternatives or combination therapy rather than force further advancement

B. Declare long-term follow-up unnecessary

C. Increase beyond comfort indefinitely

D. Avoid discussing PAP

E. Assume positional measures cannot help anyone

CORRECT ANSWER A

Progressive disease may outstrip tolerable appliance settings; the team may consider PAP, positional measures, weight strategies, redesign or alternatives.

REFERENCE Integrated notebook, Topic 7, §7.3

QUESTION 0699 · §7.2 · Applied clinical SBA

A previously stable OSA patient reports recent weight gain and renewed snoring. Which of the following statements about self-reported symptoms is most accurate?

A. Subjective review uses patient-reported sleepiness, symptoms, sleep diaries and treatment experience.

B. Sleep apnoea worsens over time, and the optimal advancement needed to keep the airway open may increase.

C. The optimal position is unique to each patient and determined by careful repeated titration.

D. Partner reports can document snoring, witnessed apnoeas and restless sleep.

E. Longitudinal care should document symptoms with repeated questionnaires and structured review.

CORRECT ANSWER A

Subjective review uses patient-reported sleepiness, symptoms, sleep diaries and treatment experience. Additional revision detail: the patient's perception of severity and therapy effect: ESS, sleep diaries, mobile applications. The other options describe different mechanisms, investigations or findings rather than self-reported symptoms. Consult Integrated notebook, page 146, §7.2 for the full context.

REFERENCE Integrated notebook, page 146, §7.2

QUESTION 0700 · §7.2 · Clinical interpretation SBA

A long-term appliance user asks whether routine reviews are still necessary. The clinical team records the following finding: the patient's perception of severity and therapy effect: ESS, sleep diaries, mobile applications. Which concept best explains this observation?

A. Long-term dental change

B. Hardware functionality

C. Sleep-observer reports

D. Evolution tracking

E. Self-reported symptoms

CORRECT ANSWER E

This finding most directly describes Self-reported symptoms: the patient's perception of severity and therapy effect: ESS, sleep diaries, mobile applications. Distractor distinctions: A refers instead to Prolonged appliance use produces dentoskeletal change — reduced overbite and overjet, altered incisor position; B refers instead to where metal components are present (clasps, hinges, screws), ensure they are free of corrosion and move smoothly without bind; C refers instead to the bed partner or family on snoring intensity, apnoeas and restless sleep, offering "objective-seeming" data that complement; D refers instead to regular follow-ups, app-based tracking, and repeated administration of the same baseline questionnaires. Objective data integ.

REFERENCE Integrated notebook, page 146, §7.2

QUESTION 0701 · §7.1 · Clinical decision-making SBA

A patient now drinks substantial alcohol immediately before bed. What impact is most likely?

A. No effect on oral-appliance outcomes

B. Guaranteed reduction in AHI

C. Increased airway collapsibility and reduced control of sleep-disordered breathing

D. Prevention of REM-related events

E. Improved genioglossus responsiveness

CORRECT ANSWER C

Evening alcohol can worsen upper-airway collapse, undermining established therapy; counselling is part of follow-up.

REFERENCE Integrated notebook, Topic 7, §7.1

QUESTION 0702 · §7.3 · Applied clinical SBA

A 61-year-old who has used a MAD for four years attends an annual dental review. Which of the following statements about progression of the disease is most accurate?

A. The optimal position is unique to each patient and determined by careful repeated titration.

B. Sleep apnoea worsens over time, and the optimal advancement needed to keep the airway open may increase.

C. Subjective review uses patient-reported sleepiness, symptoms, sleep diaries and treatment experience.

D. Gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes.

E. Prolonged MAD use can reduce overjet and overbite through dental movements.

CORRECT ANSWER B

Sleep apnoea worsens over time, and the optimal advancement needed to keep the airway open may increase. The other options describe different mechanisms, investigations or findings rather than progression of the disease. Consult Integrated notebook, page 151, §7.3 for the full context.

REFERENCE Integrated notebook, page 151, §7.3

QUESTION 0703 · §7.3 · Clinical interpretation SBA

A previously stable OSA patient reports recent weight gain and renewed snoring. The clinical team records the following finding: Sleep apnoea worsens over time, and the optimal advancement needed to keep the airway open may increase. Which concept best explains this observation?

A. Long-term dental change

B. Progression of the disease

C. Better long-term response

D. Key considerations

E. Sleep hygiene and quantity

CORRECT ANSWER B

This finding most directly describes Progression of the disease: Sleep apnoea worsens over time, and the optimal advancement needed to keep the airway open may increase. Distractor distinctions: A refers instead to Prolonged appliance use produces dentoskeletal change — reduced overbite and overjet, altered incisor position; C refers instead to younger; less obese with smaller neck circumference (men); lower baseline AHI; D refers instead to the optimal position is unique to each patient and determined by careful repeated titration; E refers instead to good hygiene and adequate duration support success by stabilising architecture and reducing factors that worsen apnoea.

REFERENCE Integrated notebook, page 151, §7.3

QUESTION 0704 · §7.2 · Clinical decision-making SBA

Baseline symptoms were recorded verbally only; no instrument was completed. How can future monitoring be improved?

A. Stop recording dental changes

B. Avoid written communication with the physician

C. Begin consistent validated symptom and adherence records and track them serially

D. Use a different tool every appointment

E. Rely solely on the technician's memory

CORRECT ANSWER C

A reproducible longitudinal record supports recognition of recurrence and evaluates treatment modification.

REFERENCE Integrated notebook, Topic 7, §7.2

QUESTION 0705 · §7.4 · Applied clinical SBA

An established MAD user reports a change in alcohol intake and daytime symptoms. Which of the following statements about handling is most accurate?

A. Where metal components are present (clasps, hinges, screws), ensure they are free of corrosion and move smoothly without binding.

B. Better response includes younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant.

C. Storage includes store dry in the ventilated case when not in use — never wrapped in tissue or loose in a pocket; keep away.

D. Long-term oral appliance care requires at least annual dental review after the initial follow-up period.

E. Handling includes always remove over a soft surface or a basin filled with water to prevent accidental drop damage.

CORRECT ANSWER E

Handling includes always remove over a soft surface or a basin filled with water to prevent accidental drop damage. Additional revision detail: always remove over a soft surface or a basin filled with water to prevent accidental drop damage; avoid hot water and direct sunlight, which warp the appliance. The other options describe different mechanisms, investigations or findings rather than handling. Consult Integrated notebook, page 153, §7.4 for the full context.

REFERENCE Integrated notebook, page 153, §7.4

QUESTION 0706 · §7.4 · Clinical interpretation SBA

A 61-year-old who has used a MAD for four years attends an annual dental review. The clinical team records the following finding: always remove over a soft surface or a basin filled with water to prevent accidental drop damage; avoid hot water and direct sunlight, which warp the appliance. Which concept best explains this observation?

A. Follow-up

B. Retention

C. Storage

D. Cleaning

E. Handling

CORRECT ANSWER E

This finding most directly describes Handling: always remove over a soft surface or a basin filled with water to prevent accidental drop damage; avoid hot water and direct sunlight, which warp the appliance. Distractor distinctions: A refers instead to every 6 months in year 1, at least annually thereafter; physician reassessment approximately every 5 years; B refers instead to gently tug to verify the device resists displacement during normal oral function; C refers instead to store dry in the ventilated case when not in use — never wrapped in tissue or loose in a pocket; keep away from pets, childre; D refers instead to rinse immediately after removal to wash away saliva and debris.

REFERENCE Integrated notebook, page 153, §7.4

QUESTION 0707 · §7.4 · Clinical decision-making SBA

The patient asks why a dental appliance needs review once the AHI has been controlled. What is the best explanation?

A. OSA and oral structures change over time; both disease control and the appliance need ongoing surveillance

B. Only CPAP needs maintenance

C. The dentist may replace medical assessment entirely

D. Changes in occlusion never affect fit

E. Once fitted, the setting is lifelong without exception

CORRECT ANSWER A

OSA is often progressive while dentition, periodontal support, retention and materials change; annual dental and periodic medical follow-up are needed.

REFERENCE Integrated notebook, Topic 7, §7.4

QUESTION 0708 · §7.1 · Applied clinical SBA

An established MAD user reports a change in alcohol intake and daytime symptoms. Which of the following statements about sleep hygiene and quantity is most accurate?

A. Adequate sleep opportunity and appropriate sleep habits can improve overall symptom management.

B. Medication change includes drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids.

C. Gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes.

D. Before sleep it exacerbates collapse and increases severity, undermining benefit.

E. Better long-term response includes younger; less obese with smaller neck circumference (men); lower baseline AHI.

CORRECT ANSWER A

Adequate sleep opportunity and appropriate sleep habits can improve overall symptom management. Additional revision detail: good hygiene and adequate duration support success by stabilising architecture and reducing factors that worsen apnoea. The other options describe different mechanisms, investigations or findings rather than sleep hygiene and quantity. Consult Integrated notebook, page 146, §7.1 for the full context.

REFERENCE Integrated notebook, page 146, §7.1

QUESTION 0709 · §7.1 · Clinical interpretation SBA

A 61-year-old who has used a MAD for four years attends an annual dental review. The clinical team records the following finding: good hygiene and adequate duration support success by stabilising architecture and reducing factors that worsen apnoea. Which concept best explains this observation?

A. Weight change

B. Alcohol

C. Better long-term response

D. Sleep hygiene and quantity

E. Medication change

CORRECT ANSWER D

This finding most directly describes Sleep hygiene and quantity: good hygiene and adequate duration support success by stabilising architecture and reducing factors that worsen apnoea. Distractor distinctions: A refers instead to gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes; B refers instead to before sleep it exacerbates collapse and increases severity, undermining benefit; even moderate intake may cause transient de; C refers instead to younger; less obese with smaller neck circumference (men); lower baseline AHI; E refers instead to drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter a.

REFERENCE Integrated notebook, page 146, §7.1

QUESTION 0710 · §7.3 · Clinical decision-making SBA

A patient's current treatment AHI is markedly higher than the historical value despite good nightly use. What does this most strongly justify?

A. Ignore the change because use is high

B. Use only a morning aligner to treat residual AHI

C. Define snoring reduction as an objective cure

D. Physician-led repeat assessment of efficacy and evolving disease

E. Conclude that adherence is the cause

CORRECT ANSWER D

Adequate use with worsening objective indices points toward disease progression, device failure or phenotype change requiring reassessment.

REFERENCE Integrated notebook, Topic 7, §7.3

QUESTION 0711 · §7.2 · Applied clinical SBA

A patient returns after several years of nightly OAT with a new occlusal complaint. Which of the following statements about sleep-observer reports is most accurate?

A. Handling includes always remove over a soft surface or a basin filled with water to prevent accidental drop damage.

B. Subjective review uses patient-reported sleepiness, symptoms, sleep diaries and treatment experience.

C. Before sleep it exacerbates collapse and increases severity, undermining benefit.

D. Partner reports can document snoring, witnessed apnoeas and restless sleep.

E. Longitudinal care should document symptoms with repeated questionnaires and structured review.

CORRECT ANSWER D

Partner reports can document snoring, witnessed apnoeas and restless sleep. Additional revision detail: the bed partner or family on snoring intensity, apnoeas and restless sleep, offering "objective-seeming" data that complement self-report: partner observations, STOP-BANG items. The other options describe different mechanisms, investigations or findings rather than sleep-observer reports. Consult Integrated notebook, page 146, §7.2 for the full context.

REFERENCE Integrated notebook, page 146, §7.2

QUESTION 0712 · §7.2 · Clinical interpretation SBA

An established MAD user reports a change in alcohol intake and daytime symptoms. The clinical team records the following finding: the bed partner or family on snoring intensity, apnoeas and restless sleep, offering "objective-seeming" data that complement self-report: partner observations, STOP-BANG items. Which concept best explains this observation?

A. Sleep hygiene and quantity

B. Sleep-observer reports

C. Evolution tracking

D. Self-reported symptoms

E. Hardware functionality

CORRECT ANSWER B

This finding most directly describes Sleep-observer reports: the bed partner or family on snoring intensity, apnoeas and restless sleep, offering "objective-seeming" data that complement self-report: partner observations, STOP-BANG items. Distractor distinctions: A refers instead to good hygiene and adequate duration support success by stabilising architecture and reducing factors that worsen apnoea; C refers instead to regular follow-ups, app-based tracking, and repeated administration of the same baseline questionnaires. Objective data integ; D refers instead to the patient's perception of severity and therapy effect: ESS, sleep diaries, mobile applications; E refers instead to where metal components are present (clasps, hinges, screws), ensure they are free of corrosion and move smoothly without bind.

REFERENCE Integrated notebook, page 146, §7.2

QUESTION 0713 · §7.3 · Applied clinical SBA

A previously stable OSA patient reports recent weight gain and renewed snoring. Which of the following statements about long-term dental change is most accurate?

A. Gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes.

B. A seated oral appliance should engage its intended retention points without rocking or dislodgment.

C. Prolonged MAD use can reduce overjet and overbite through dental movements.

D. Sleep apnoea worsens over time, and the optimal advancement needed to keep the airway open may increase.

E. The optimal position is unique to each patient and determined by careful repeated titration.

CORRECT ANSWER C

Prolonged MAD use can reduce overjet and overbite through dental movements. Additional revision detail: Prolonged appliance use produces dentoskeletal change — reduced overbite and overjet, altered incisor position. The other options describe different mechanisms, investigations or findings rather than long-term dental change. Consult Integrated notebook, page 151, §7.3 for the full context.

REFERENCE Integrated notebook, page 151, §7.3

QUESTION 0714 · §7.3 · Clinical interpretation SBA

A long-term appliance user asks whether routine reviews are still necessary. The clinical team records the following finding: Prolonged appliance use produces dentoskeletal change — reduced overbite and overjet, altered incisor position. Which concept best explains this observation?

A. Long-term dental change

B. Key considerations

C. Better long-term response

D. Medication change

E. Progression of the disease

CORRECT ANSWER A

This finding most directly describes Long-term dental change: Prolonged appliance use produces dentoskeletal change — reduced overbite and overjet, altered incisor position. Distractor distinctions: B refers instead to the optimal position is unique to each patient and determined by careful repeated titration; C refers instead to younger; less obese with smaller neck circumference (men); lower baseline AHI; D refers instead to drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter a; E refers instead to Sleep apnoea worsens over time, and the optimal advancement needed to keep the airway open may increase.

REFERENCE Integrated notebook, page 151, §7.3

QUESTION 0715 · §7.4 · Applied clinical SBA

An established MAD user reports a change in alcohol intake and daytime symptoms. Which of the following statements about poorer response is most accurate?

A. Retention includes gently tug to verify the device resists displacement during normal oral function.

B. The clinician should note discolouration, thinning or deformation of the acrylic or thermoplastic.

C. Ensure the appliance does not interfere with the bite or cause premature contacts.

D. Better response includes younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant.

E. Poorer response includes supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment.

CORRECT ANSWER E

Poorer response includes supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment. Additional revision detail: supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment. The other options describe different mechanisms, investigations or findings rather than poorer response. Consult Integrated notebook, page 153, §7.4 for the full context.

REFERENCE Integrated notebook, page 153, §7.4

QUESTION 0716 · §7.4 · Clinical interpretation SBA

A 61-year-old who has used a MAD for four years attends an annual dental review. The clinical team records the following finding: supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment. Which concept best explains this observation?

A. Poorer response

B. Occlusal contacts

C. Material wear

D. Better response

E. Seating accuracy

CORRECT ANSWER A

This finding most directly describes Poorer response: supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment. Distractor distinctions: B refers instead to ensure the appliance does not interfere with the bite or cause premature contacts; C refers instead to note discolouration, thinning or deformation of the acrylic or thermoplastic; D refers instead to younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant; E refers instead to confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking.

REFERENCE Integrated notebook, page 153, §7.4

QUESTION 0717 · §7.1 · Applied clinical SBA

A patient returns after several years of nightly OAT with a new occlusal complaint. Which of the following statements about medication change is most accurate?

A. Better long-term response includes younger; less obese with smaller neck circumference (men); lower baseline AHI.

B. Gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes.

C. Adequate sleep opportunity and appropriate sleep habits can improve overall symptom management.

D. Medication change includes drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids.

E. Before sleep it exacerbates collapse and increases severity, undermining benefit.

CORRECT ANSWER D

Medication change includes drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids. Additional revision detail: drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter architecture, potentially reducing efficacy. The other options describe different mechanisms, investigations or findings rather than medication change. Consult Integrated notebook, page 146, §7.1 for the full context.

REFERENCE Integrated notebook, page 146, §7.1

QUESTION 0718 · §7.1 · Clinical interpretation SBA

An established MAD user reports a change in alcohol intake and daytime symptoms. The clinical team records the following finding: drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter architecture, potentially reducing efficacy. Which concept best explains this observation?

A. Alcohol

B. Weight change

C. Sleep hygiene and quantity

D. Better long-term response

E. Medication change

CORRECT ANSWER E

This finding most directly describes Medication change: drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter architecture, potentially reducing efficacy. Distractor distinctions: A refers instead to before sleep it exacerbates collapse and increases severity, undermining benefit; even moderate intake may cause transient de; B refers instead to gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes; C refers instead to good hygiene and adequate duration support success by stabilising architecture and reducing factors that worsen apnoea; D refers instead to younger; less obese with smaller neck circumference (men); lower baseline AHI.

REFERENCE Integrated notebook, page 146, §7.1

QUESTION 0719 · §7.4 · Applied clinical SBA

A patient returns after several years of nightly OAT with a new occlusal complaint. Which of the following statements about occlusal contacts is most accurate?

A. The clinician should note discolouration, thinning or deformation of the acrylic or thermoplastic.

B. Better response includes younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant.

C. The clinician should check for cracks, crazing or rough spots that could irritate the oral tissues.

D. Retention includes gently tug to verify the device resists displacement during normal oral function.

E. Ensure the appliance does not interfere with the bite or cause premature contacts.

CORRECT ANSWER E

Ensure the appliance does not interfere with the bite or cause premature contacts. The other options describe different mechanisms, investigations or findings rather than occlusal contacts. Consult Integrated notebook, page 152, §7.4 for the full context.

REFERENCE Integrated notebook, page 152, §7.4

QUESTION 0720 · §7.4 · Clinical interpretation SBA

An established MAD user reports a change in alcohol intake and daytime symptoms. The clinical team records the following finding: ensure the appliance does not interfere with the bite or cause premature contacts. Which concept best explains this observation?

A. Occlusal contacts

B. Surface integrity

C. Poorer response

D. Better response

E. Seating accuracy

CORRECT ANSWER A

This finding most directly describes Occlusal contacts: ensure the appliance does not interfere with the bite or cause premature contacts. Distractor distinctions: B refers instead to check for cracks, crazing or rough spots that could irritate the oral tissues; C refers instead to supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment; D refers instead to younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant; E refers instead to confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking.

REFERENCE Integrated notebook, page 152, §7.4

QUESTION 0721 · §7.1 · Applied clinical SBA

A long-term appliance user asks whether routine reviews are still necessary. Which of the following statements about better long-term response is most accurate?

A. Gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes.

B. Medication change includes drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids.

C. Before sleep it exacerbates collapse and increases severity, undermining benefit.

D. Adequate sleep opportunity and appropriate sleep habits can improve overall symptom management.

E. Better long-term response includes younger; less obese with smaller neck circumference (men); lower baseline AHI.

CORRECT ANSWER E

Better long-term response includes younger; less obese with smaller neck circumference (men); lower baseline AHI. Additional revision detail: younger; less obese with smaller neck circumference (men); lower baseline AHI. The other options describe different mechanisms, investigations or findings rather than better long-term response. Consult Integrated notebook, page 146, §7.1 for the full context.

REFERENCE Integrated notebook, page 146, §7.1

QUESTION 0722 · §7.1 · Clinical interpretation SBA

A patient returns after several years of nightly OAT with a new occlusal complaint. The clinical team records the following finding: younger; less obese with smaller neck circumference (men); lower baseline AHI. Which concept best explains this observation?

A. Alcohol

B. Medication change

C. Better long-term response

D. Weight change

E. Sleep hygiene and quantity

CORRECT ANSWER C

This finding most directly describes Better long-term response: younger; less obese with smaller neck circumference (men); lower baseline AHI. Distractor distinctions: A refers instead to before sleep it exacerbates collapse and increases severity, undermining benefit; even moderate intake may cause transient de; B refers instead to drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter a; D refers instead to gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes; E refers instead to good hygiene and adequate duration support success by stabilising architecture and reducing factors that worsen apnoea.

REFERENCE Integrated notebook, page 146, §7.1

QUESTION 0723 · §7.4 · Applied clinical SBA

A long-term appliance user asks whether routine reviews are still necessary. Which of the following statements about weight is most accurate?

A. Cleaning includes rinse immediately after removal to wash away saliva and debris.

B. The clinician should check for cracks, crazing or rough spots that could irritate the oral tissues.

C. A 10% weight gain is associated with an approximately 32% rise in AHI.

D. The clinician should note discolouration, thinning or deformation of the acrylic or thermoplastic.

E. Ensure the appliance does not interfere with the bite or cause premature contacts.

CORRECT ANSWER C

A 10% weight gain is associated with an approximately 32% rise in AHI. Additional revision detail: 10% gain → +32% AHI; 10% loss → −26% AHI. The other options describe different mechanisms, investigations or findings rather than weight. Consult Integrated notebook, page 147, §7.4 for the full context.

REFERENCE Integrated notebook, page 147, §7.4

QUESTION 0724 · §7.4 · Focused knowledge SBA

A patient returns after several years of nightly OAT with a new occlusal complaint. Which of the following statements about weight is most accurate?

A. Cleaning includes rinse immediately after removal to wash away saliva and debris.

B. Ensure the appliance does not interfere with the bite or cause premature contacts.

C. A 10% weight gain is associated with an approximately 32% rise in AHI.

D. The clinician should note discolouration, thinning or deformation of the acrylic or thermoplastic.

E. The clinician should check for cracks, crazing or rough spots that could irritate the oral tissues.

CORRECT ANSWER C

A 10% weight gain is associated with an approximately 32% rise in AHI. Additional revision detail: 10% gain → +32% AHI; 10% loss → −26% AHI. The other options describe different mechanisms, investigations or findings rather than weight. Consult Integrated notebook, page 147, §7.4 for the full context.

REFERENCE Integrated notebook, page 147, §7.4

QUESTION 0725 · §7.4 · Applied clinical SBA

An established MAD user reports a change in alcohol intake and daytime symptoms. Which of the following statements about follow-up is most accurate?

A. Retention includes gently tug to verify the device resists displacement during normal oral function.

B. A seated oral appliance should engage its intended retention points without rocking or dislodgment.

C. Poorer response includes supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment.

D. Long-term oral appliance care requires at least annual dental review after the initial follow-up period.

E. Better response includes younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant.

CORRECT ANSWER D

Long-term oral appliance care requires at least annual dental review after the initial follow-up period. Additional revision detail: every 6 months in year 1, at least annually thereafter; physician reassessment approximately every 5 years. The other options describe different mechanisms, investigations or findings rather than follow-up. Consult Integrated notebook, page 153, §7.4 for the full context.

REFERENCE Integrated notebook, page 153, §7.4

QUESTION 0726 · §7.4 · Clinical interpretation SBA

A 61-year-old who has used a MAD for four years attends an annual dental review. The clinical team records the following finding: every 6 months in year 1, at least annually thereafter; physician reassessment approximately every 5 years. Which concept best explains this observation?

A. Cleaning

B. Handling

C. Storage

D. Retention

E. Follow-up

CORRECT ANSWER E

This finding most directly describes Follow-up: every 6 months in year 1, at least annually thereafter; physician reassessment approximately every 5 years. Distractor distinctions: A refers instead to rinse immediately after removal to wash away saliva and debris; B refers instead to always remove over a soft surface or a basin filled with water to prevent accidental drop damage; avoid hot water and direct; C refers instead to store dry in the ventilated case when not in use — never wrapped in tissue or loose in a pocket; keep away from pets, childre; D refers instead to gently tug to verify the device resists displacement during normal oral function.

REFERENCE Integrated notebook, page 153, §7.4

QUESTION 0727 · §7.4 · Applied clinical SBA

A previously stable OSA patient reports recent weight gain and renewed snoring. Which of the following statements about surface integrity is most accurate?

A. The clinician should check for cracks, crazing or rough spots that could irritate the oral tissues.

B. Ensure the appliance does not interfere with the bite or cause premature contacts.

C. The clinician should note discolouration, thinning or deformation of the acrylic or thermoplastic.

D. Retention includes gently tug to verify the device resists displacement during normal oral function.

E. Better response includes younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant.

CORRECT ANSWER A

The clinician should check for cracks, crazing or rough spots that could irritate the oral tissues. Additional revision detail: check for cracks, crazing or rough spots that could irritate the oral tissues. The other options describe different mechanisms, investigations or findings rather than surface integrity. Consult Integrated notebook, page 152, §7.4 for the full context.

REFERENCE Integrated notebook, page 152, §7.4

QUESTION 0728 · §7.4 · Clinical interpretation SBA

A long-term appliance user asks whether routine reviews are still necessary. The clinical team records the following finding: check for cracks, crazing or rough spots that could irritate the oral tissues. Which concept best explains this observation?

A. Poorer response

B. Surface integrity

C. Better response

D. Seating accuracy

E. Occlusal contacts

CORRECT ANSWER B

This finding most directly describes Surface integrity: check for cracks, crazing or rough spots that could irritate the oral tissues. Distractor distinctions: A refers instead to supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment; C refers instead to younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant; D refers instead to confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking; E refers instead to ensure the appliance does not interfere with the bite or cause premature contacts.

REFERENCE Integrated notebook, page 152, §7.4

QUESTION 0729 · §7.4 · Applied clinical SBA

A patient returns after several years of nightly OAT with a new occlusal complaint. Which of the following statements about hardware functionality is most accurate?

A. A seated oral appliance should engage its intended retention points without rocking or dislodgment.

B. Poorer response includes supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment.

C. Storage includes store dry in the ventilated case when not in use — never wrapped in tissue or loose in a pocket; keep away.

D. Long-term oral appliance care requires at least annual dental review after the initial follow-up period.

E. Where metal components are present (clasps, hinges, screws), ensure they are free of corrosion and move smoothly without binding.

CORRECT ANSWER E

Where metal components are present (clasps, hinges, screws), ensure they are free of corrosion and move smoothly without binding. The other options describe different mechanisms, investigations or findings rather than hardware functionality. Consult Integrated notebook, page 152, §7.4 for the full context.

REFERENCE Integrated notebook, page 152, §7.4

QUESTION 0730 · §7.4 · Clinical interpretation SBA

An established MAD user reports a change in alcohol intake and daytime symptoms. The clinical team records the following finding: where metal components are present (clasps, hinges, screws), ensure they are free of corrosion and move smoothly without binding. Which concept best explains this observation?

A. Hardware functionality

B. Better response

C. Seating accuracy

D. Surface integrity

E. Occlusal contacts

CORRECT ANSWER A

This finding most directly describes Hardware functionality: where metal components are present (clasps, hinges, screws), ensure they are free of corrosion and move smoothly without binding. Distractor distinctions: B refers instead to younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant; C refers instead to confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking; D refers instead to check for cracks, crazing or rough spots that could irritate the oral tissues; E refers instead to ensure the appliance does not interfere with the bite or cause premature contacts.

REFERENCE Integrated notebook, page 152, §7.4

QUESTION 0731 · §7.4 · Applied clinical SBA

A 61-year-old who has used a MAD for four years attends an annual dental review. Which of the following statements about material wear is most accurate?

A. Retention includes gently tug to verify the device resists displacement during normal oral function.

B. Poorer response includes supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment.

C. The clinician should check for cracks, crazing or rough spots that could irritate the oral tissues.

D. The clinician should note discolouration, thinning or deformation of the acrylic or thermoplastic.

E. Ensure the appliance does not interfere with the bite or cause premature contacts.

CORRECT ANSWER D

The clinician should note discolouration, thinning or deformation of the acrylic or thermoplastic. Additional revision detail: note discolouration, thinning or deformation of the acrylic or thermoplastic. The other options describe different mechanisms, investigations or findings rather than material wear. Consult Integrated notebook, page 152, §7.4 for the full context.

REFERENCE Integrated notebook, page 152, §7.4

QUESTION 0732 · §7.4 · Clinical interpretation SBA

A previously stable OSA patient reports recent weight gain and renewed snoring. The clinical team records the following finding: note discolouration, thinning or deformation of the acrylic or thermoplastic. Which concept best explains this observation?

A. Follow-up

B. Poorer response

C. Seating accuracy

D. Better response

E. Material wear

CORRECT ANSWER E

This finding most directly describes Material wear: note discolouration, thinning or deformation of the acrylic or thermoplastic. Distractor distinctions: A refers instead to every 6 months in year 1, at least annually thereafter; physician reassessment approximately every 5 years; B refers instead to supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment; C refers instead to confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking; D refers instead to younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant.

REFERENCE Integrated notebook, page 152, §7.4

QUESTION 0733 · §7.4 · Applied clinical SBA

A long-term appliance user asks whether routine reviews are still necessary. Which of the following statements about retention is most accurate?

A. The clinician should check for cracks, crazing or rough spots that could irritate the oral tissues.

B. Poorer response includes supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment.

C. Retention includes gently tug to verify the device resists displacement during normal oral function.

D. Ensure the appliance does not interfere with the bite or cause premature contacts.

E. The clinician should note discolouration, thinning or deformation of the acrylic or thermoplastic.

CORRECT ANSWER C

Retention includes gently tug to verify the device resists displacement during normal oral function. Additional revision detail: gently tug to verify the device resists displacement during normal oral function. The other options describe different mechanisms, investigations or findings rather than retention. Consult Integrated notebook, page 152, §7.4 for the full context.

REFERENCE Integrated notebook, page 152, §7.4

QUESTION 0734 · §7.4 · Clinical interpretation SBA

A patient returns after several years of nightly OAT with a new occlusal complaint. The clinical team records the following finding: gently tug to verify the device resists displacement during normal oral function. Which concept best explains this observation?

A. Retention

B. Cleaning

C. Handling

D. Follow-up

E. Storage

CORRECT ANSWER A

This finding most directly describes Retention: gently tug to verify the device resists displacement during normal oral function. Distractor distinctions: B refers instead to rinse immediately after removal to wash away saliva and debris; C refers instead to always remove over a soft surface or a basin filled with water to prevent accidental drop damage; avoid hot water and direct; D refers instead to every 6 months in year 1, at least annually thereafter; physician reassessment approximately every 5 years; E refers instead to store dry in the ventilated case when not in use — never wrapped in tissue or loose in a pocket; keep away from pets, childre.

REFERENCE Integrated notebook, page 152, §7.4

QUESTION 0735 · §7.4 · Applied clinical SBA

An established MAD user reports a change in alcohol intake and daytime symptoms. Which of the following statements about seating accuracy is most accurate?

A. Where metal components are present (clasps, hinges, screws), ensure they are free of corrosion and move smoothly without binding.

B. Ensure the appliance does not interfere with the bite or cause premature contacts.

C. The clinician should check for cracks, crazing or rough spots that could irritate the oral tissues.

D. Long-term oral appliance care requires at least annual dental review after the initial follow-up period.

E. A seated oral appliance should engage its intended retention points without rocking or dislodgment.

CORRECT ANSWER E

A seated oral appliance should engage its intended retention points without rocking or dislodgment. Additional revision detail: confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking. The other options describe different mechanisms, investigations or findings rather than seating accuracy. Consult Integrated notebook, page 152, §7.4 for the full context.

REFERENCE Integrated notebook, page 152, §7.4

QUESTION 0736 · §7.4 · Clinical interpretation SBA

A 61-year-old who has used a MAD for four years attends an annual dental review. The clinical team records the following finding: confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking. Which concept best explains this observation?

A. Occlusal contacts

B. Poorer response

C. Better response

D. Surface integrity

E. Seating accuracy

CORRECT ANSWER E

This finding most directly describes Seating accuracy: confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking. Distractor distinctions: A refers instead to ensure the appliance does not interfere with the bite or cause premature contacts; B refers instead to supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment; C refers instead to younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant; D refers instead to check for cracks, crazing or rough spots that could irritate the oral tissues.

REFERENCE Integrated notebook, page 152, §7.4

QUESTION 0737 · §7.4 · Applied clinical SBA

A previously stable OSA patient reports recent weight gain and renewed snoring. Which of the following statements about cleaning is most accurate?

A. The clinician should note discolouration, thinning or deformation of the acrylic or thermoplastic.

B. The clinician should check for cracks, crazing or rough spots that could irritate the oral tissues.

C. Retention includes gently tug to verify the device resists displacement during normal oral function.

D. Ensure the appliance does not interfere with the bite or cause premature contacts.

E. Cleaning includes rinse immediately after removal to wash away saliva and debris.

CORRECT ANSWER E

Cleaning includes rinse immediately after removal to wash away saliva and debris. Additional revision detail: rinse immediately after removal to wash away saliva and debris. The other options describe different mechanisms, investigations or findings rather than cleaning. Consult Integrated notebook, page 153, §7.4 for the full context.

REFERENCE Integrated notebook, page 153, §7.4

QUESTION 0738 · §7.4 · Clinical interpretation SBA

A long-term appliance user asks whether routine reviews are still necessary. The clinical team records the following finding: rinse immediately after removal to wash away saliva and debris. Which concept best explains this observation?

A. Follow-up

B. Handling

C. Cleaning

D. Storage

E. Retention

CORRECT ANSWER C

This finding most directly describes Cleaning: rinse immediately after removal to wash away saliva and debris. Distractor distinctions: A refers instead to every 6 months in year 1, at least annually thereafter; physician reassessment approximately every 5 years; B refers instead to always remove over a soft surface or a basin filled with water to prevent accidental drop damage; avoid hot water and direct; D refers instead to store dry in the ventilated case when not in use — never wrapped in tissue or loose in a pocket; keep away from pets, childre; E refers instead to gently tug to verify the device resists displacement during normal oral function.

REFERENCE Integrated notebook, page 153, §7.4

QUESTION 0739 · §7.4 · Applied clinical SBA

A patient returns after several years of nightly OAT with a new occlusal complaint. Which of the following statements about storage is most accurate?

A. A seated oral appliance should engage its intended retention points without rocking or dislodgment.

B. Ensure the appliance does not interfere with the bite or cause premature contacts.

C. Handling includes always remove over a soft surface or a basin filled with water to prevent accidental drop damage.

D. Storage includes store dry in the ventilated case when not in use — never wrapped in tissue or loose in a pocket; keep away.

E. Where metal components are present (clasps, hinges, screws), ensure they are free of corrosion and move smoothly without binding.

CORRECT ANSWER D

Storage includes store dry in the ventilated case when not in use — never wrapped in tissue or loose in a pocket; keep away. Additional revision detail: store dry in the ventilated case when not in use — never wrapped in tissue or loose in a pocket; keep away from pets, children and heat sources. The other options describe different mechanisms, investigations or findings rather than storage. Consult Integrated notebook, page 153, §7.4 for the full context.

REFERENCE Integrated notebook, page 153, §7.4

QUESTION 0740 · §7.4 · Focused knowledge SBA

An established MAD user reports a change in alcohol intake and daytime symptoms. Which of the following statements about storage is most accurate?

A. A seated oral appliance should engage its intended retention points without rocking or dislodgment.

B. Storage includes store dry in the ventilated case when not in use — never wrapped in tissue or loose in a pocket; keep away.

C. Ensure the appliance does not interfere with the bite or cause premature contacts.

D. Where metal components are present (clasps, hinges, screws), ensure they are free of corrosion and move smoothly without binding.

E. Handling includes always remove over a soft surface or a basin filled with water to prevent accidental drop damage.

CORRECT ANSWER B

Storage includes store dry in the ventilated case when not in use — never wrapped in tissue or loose in a pocket; keep away. Additional revision detail: store dry in the ventilated case when not in use — never wrapped in tissue or loose in a pocket; keep away from pets, children and heat sources. The other options describe different mechanisms, investigations or findings rather than storage. Consult Integrated notebook, page 153, §7.4 for the full context.

REFERENCE Integrated notebook, page 153, §7.4

08 Paediatric sleep-disordered breathing

QUESTION 0741 · §8.3 · Clinical decision-making SBA

A child with paediatric OSA is irritable and underperforming in school but not sleepy by adult measures. Why is the presentation plausible?

A. Only adults can experience OSA

B. Tonsil size predicts the exact cognitive score

C. Normal ESS in children rules out OSA

D. Sleep disruption cannot affect learning

E. Paediatric SDB often presents with behavioural and cognitive disturbance

CORRECT ANSWER E

Children may manifest sleep fragmentation through hyperactivity, inattention, learning and behavioural problems.

REFERENCE Integrated notebook, Topic 8, §8.3

QUESTION 0742 · §8.3 · Clinical decision-making SBA

Parents bring a 7-year-old with habitual snoring, mouth breathing and disruptive behaviour at school. Which statement is most appropriate?

A. Children with OSA must report daytime sleepiness

B. The symptoms warrant evaluation for paediatric sleep-disordered breathing

C. School behaviour excludes a sleep disorder

D. Adult AHI severity thresholds always apply to this child

E. A dental impression alone can establish the diagnosis

CORRECT ANSWER B

Children often manifest sleep-disordered breathing through hyperactivity, inattention and learning difficulties rather than adult-pattern sleepiness. Specialist evaluation is indicated. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 8, §8.3

QUESTION 0743 · §8.2 · Clinical decision-making SBA

A 5-year-old snores loudly, breathes through the mouth and has enlarged palatine tonsils. Which anatomical factor is most commonly important at this age?

A. Age-related loss of tooth enamel

B. Adenotonsillar hypertrophy

C. Age-related tongue base muscle degeneration

D. Large mandibular tori in every child

E. Menopausal fat redistribution

CORRECT ANSWER B

Adenotonsillar hypertrophy is an important driver of obstructive sleep-disordered breathing during preschool and early school ages. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 8, §8.2

QUESTION 0744 · §8.6 · Clinical decision-making SBA

Parents ask whether rapid maxillary expansion will reliably cure their child's PSG-confirmed OSA. Which counselling statement is best?

A. It always normalises the AHI

B. It may have an adjunctive role in selected children, but benefit is variable and medical follow-up remains necessary

C. It replaces physician diagnosis

D. No objective follow-up is required after expansion

E. It is equivalent to adenotonsillectomy in every child

CORRECT ANSWER B

Expansion may modify nasal and maxillary anatomy in selected patients, but response is variable and it should not be presented as guaranteed OSA treatment. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 8, §8.6

QUESTION 0745 · §8.9 · Clinical decision-making SBA

A 4-year-old child with Down syndrome has no parent-reported sleep complaints. What is the most appropriate assessment strategy?

A. No sleep testing unless daytime sleepiness develops

B. Watch until adulthood because snoring is rare

C. Recommend physician-led PSG screening according to relevant paediatric guidance

D. Start a mandibular advancement appliance without diagnosis

E. Use dental crowding as the sole screening test

CORRECT ANSWER C

Down syndrome is associated with a high prevalence of often unrecognised paediatric OSA. Guidance in the notebook supports PSG screening by age four even without reported symptoms. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 8, §8.9

QUESTION 0746 · §8.2 · Clinical decision-making SBA

A preschool child develops obstructive breathing during sleep despite no obesity. The tonsils are markedly enlarged. Which aetiology is most typical?

A. A malformed dental retainer as the only explanation

B. Adult-type smoking exposure as the necessary cause

C. Fixed central sleep apnoea in all children

D. Isolated lower incisor crowding as the diagnosis

E. Adenotonsillar hypertrophy narrowing the pharyngeal airway

CORRECT ANSWER E

Adenotonsillar enlargement is the leading cause in younger children; aetiology changes with growth and comorbidity.

REFERENCE Integrated notebook, Topic 8, §8.2

QUESTION 0747 · §8.2 · Applied clinical SBA

A 12-year-old with behavioural changes and habitual snoring is referred for assessment. Which of the following statements about common preschool cause is most accurate?

A. Adenotonsillar hypertrophy is the predominant anatomical factor.

B. Partial tonsillectomy includes leaves tissue covering the tonsillar fossa musculature.

C. Attention, hyperactivity and behavioural or learning difficulties may predominate.

D. The clinical finding includes OSA 1–5% (typically 2–3%); habitual snoring 3–12% (some cite 10–20%).

E. Paediatric OSA is influenced by craniofacial anatomy, adenotonsillar size and ventilatory control.

CORRECT ANSWER A

Adenotonsillar hypertrophy is the predominant anatomical factor. The other options describe different mechanisms, investigations or findings rather than common preschool cause. Consult Integrated notebook, page 156, §8.2 for the full context.

REFERENCE Integrated notebook, page 156, §8.2

QUESTION 0748 · §8.2 · Clinical interpretation SBA

Parents report that their 6-year-old snores and struggles with classroom attention. The clinical team records the following finding: Adenotonsillar hypertrophy is the predominant anatomical factor. Which concept best explains this observation?

A. Mental health problems

B. Circadian phase delay

C. Sleep physician or ENT

D. Common preschool cause

E. Sleep–wake homeostasis

CORRECT ANSWER D

This finding most directly describes Common preschool cause: Adenotonsillar hypertrophy is the predominant anatomical factor. Distractor distinctions: A refers instead to anxiety, stress sleep, leading to residual daytime sleepiness; B refers instead to Early adolescent development and pubertal onset are associated with a more evening-type circadian phase preference — the pref; C refers instead to for children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging. F; E refers instead to Sleep pressure builds more slowly with time awake in adolescents.

REFERENCE Integrated notebook, page 156, §8.2

QUESTION 0749 · §8.3 · Clinical decision-making SBA

A six-year-old snores heavily and has hyperactivity and poor school concentration without daytime sleepiness. What should the dentist consider?

A. Adult ESS alone establishes severity

B. OSA is excluded because the child is hyperactive

C. Possible paediatric OSA with behavioural presentation

D. Treat with a mandibular splint without referral

E. Only depression can explain such behaviour

CORRECT ANSWER C

Children often manifest SDB through hyperactivity, inattention, behavioural problems or growth concerns rather than classic adult sleepiness.

REFERENCE Integrated notebook, Topic 8, §8.3

QUESTION 0750 · §8.3 · Applied clinical SBA

A child with Down syndrome attends a routine dental appointment and has no reported sleepiness. Which of the following statements about paediatric daytime presentation is most accurate?

A. Adenotonsillar hypertrophy is the predominant anatomical factor.

B. Submucous cleft palate is relative — a partial adenotonsillectomy may be performed.

C. Attention, hyperactivity and behavioural or learning difficulties may predominate.

D. In children, obstructive AHI of 1/h or more is abnormal, with severe disease generally at 10/h or more.

E. The clinical finding includes OSA 1–5% (typically 2–3%); habitual snoring 3–12% (some cite 10–20%).

CORRECT ANSWER C

Attention, hyperactivity and behavioural or learning difficulties may predominate. The other options describe different mechanisms, investigations or findings rather than paediatric daytime presentation. Consult Integrated notebook, page 157, §8.3 for the full context.

REFERENCE Integrated notebook, page 157, §8.3

QUESTION 0751 · §8.3 · Clinical interpretation SBA

A 12-year-old with behavioural changes and habitual snoring is referred for assessment. The clinical team records the following finding: Attention, hyperactivity and behavioural or learning difficulties may predominate. Which concept best explains this observation?

A. Paediatric AHI

B. Adenotonsillectomy failure risk

C. Paediatric daytime presentation

D. General prevalence of paediatric OSA

E. Presentation

CORRECT ANSWER C

This finding most directly describes Paediatric daytime presentation: Attention, hyperactivity and behavioural or learning difficulties may predominate. Distractor distinctions: A refers instead to normal <1 · mild 1–5 · moderate 5–10 · severe ≥10. Obstructive AHI ≥1 is abnormal; B refers instead to age <3 or ≥7, obesity, high pre-op AHI, nasal/craniofacial abnormality, family history, asthma. Overnight admission if SpO2 n; D refers instead to Approximately 1–5% of children, with estimates near 2–3%; E refers instead to Children: snoring or noisy breathing; restless sleep or unusual positions; frequent bed-wetting; daytime fatigue and attentio.

REFERENCE Integrated notebook, page 157, §8.3

QUESTION 0752 · §8.8 · Clinical decision-making SBA

A child has an obstructive AHI of 6 per hour. Which severity category follows the notebook?

A. Unclassifiable because adults use AHI 6 as mild

B. Severe only above 30/h

C. Mild

D. Moderate paediatric OSA

E. Normal

CORRECT ANSWER D

The child thresholds differ from adult thresholds: 1–<5 mild, 5–<10 moderate, and at least 10 severe.

REFERENCE Integrated notebook, Topic 8, §8.8

QUESTION 0753 · §8.4 · Applied clinical SBA

Parents request advice for a child with large tonsils and nocturnal gasping. Which of the following statements about recommended duration is most accurate?

A. Recommended sleep duration varies with age, from 12–16 h/day in infants aged 4–12 months to 8–10 h in teenagers.

B. Less than recommended includes attention, behaviour and learning problems.

C. Tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development.

D. Adolescents generally need 8–10 hours of sleep per night; younger children require longer sleep.

E. Obesity and higher preoperative AHI increase the risk of residual paediatric OSA after surgery.

CORRECT ANSWER D

Adolescents generally need 8–10 hours of sleep per night; younger children require longer sleep. Additional revision detail: <4 months not specified (variation too large) · 4–12 months 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h. The other options describe different mechanisms, investigations or findings rather than recommended duration. Consult Integrated notebook, page 156, §8.4 for the full context.

REFERENCE Integrated notebook, page 156, §8.4

QUESTION 0754 · §8.4 · Clinical interpretation SBA

A child with Down syndrome attends a routine dental appointment and has no reported sleepiness. The clinical team records the following finding: <4 months not specified (variation too large) · 4–12 months 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h. Which concept best explains this observation?

A. Sleep duration

B. Predisposing factors

C. Positional and other

D. Less than recommended

E. Recommended duration

CORRECT ANSWER E

This finding most directly describes Recommended duration: <4 months not specified (variation too large) · 4–12 months 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h. Distractor distinctions: A refers instead to 4–12 mo 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h. <8 h in adolescence → ~3× suicide-attempt r; B refers instead to macroglossia, midface hypoplasia, overweight, poor muscle tone. Moderate–severe disease is more prevalent at younger ages; C refers instead to AHI is increased supine and lowest prone in children — but position is hard to maintain overnight; D refers instead to attention, behaviour and learning problems; increased risk of accidents, injuries, hypertension, obesity, diabetes and depres.

REFERENCE Integrated notebook, page 156, §8.4

QUESTION 0755 · §8.7 · Clinical decision-making SBA

A five-year-old with large obstructive tonsils has PSG-confirmed OSA. What is generally first-line treatment?

A. ENT assessment for adenotonsillectomy

B. A panoramic radiograph as definitive management

C. Nasal EPAP alone without assessment

D. No treatment until adulthood

E. A dentist-issued mandibular advancement splint as standard first-line

CORRECT ANSWER A

Adenotonsillar hypertrophy is common in younger children; adenotonsillectomy is typically the first-line intervention when appropriate.

REFERENCE Integrated notebook, Topic 8, §8.7

QUESTION 0756 · §8.5 · Applied clinical SBA

An 8-year-old with a narrow palate and restless sleep is seen for an orthodontic assessment. Which of the following statements about mental health problems is most accurate?

A. Evening screen use can delay bedtime and interfere with adolescent sleep regulation.

B. Anxiety and stress may disturb sleep independently of obstructive respiratory events.

C. Other causes includes chronic medical illness; anxiety and stress; prescribed psychotropics.

D. Early school starts can worsen insufficient sleep in adolescents with delayed circadian timing.

E. Sleep–wake homeostasis includes sleep pressure builds more slowly with time awake in adolescents.

CORRECT ANSWER B

Anxiety and stress may disturb sleep independently of obstructive respiratory events. Additional revision detail: anxiety, stress sleep, leading to residual daytime sleepiness. The other options describe different mechanisms, investigations or findings rather than mental health problems. Consult Integrated notebook, page 167, §8.5 for the full context.

REFERENCE Integrated notebook, page 167, §8.5

QUESTION 0757 · §8.6 · Clinical interpretation SBA

A child with Down syndrome attends a routine dental appointment and has no reported sleepiness. The clinical team records the following finding: retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillary length. Which concept best explains this observation?

A. Palate and arches

B. Tonsils and adenoids

C. Medical, dental and family history

D. Tongue posture and function

E. Jaw size and position

CORRECT ANSWER E

This finding most directly describes Jaw size and position: retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillary length. Distractor distinctions: A refers instead to high-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing; B refers instead to use the modified Mallampati, Friedman tongue classification, or the Brodsky scale. Hypertrophy obstructing >50% of the airway; C refers instead to chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematur; D refers instead to tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial develo.

REFERENCE Integrated notebook, page 158, §8.6

QUESTION 0758 · §8.9 · Clinical decision-making SBA

A child with Down syndrome appears symptom-free but is approaching four years of age. Which plan accords with the notebook?

A. Use adult AHI severity cutoffs exclusively

B. Assume a questionnaire rules out OSA

C. Exclude OSA due to absent snoring

D. Wait for school failure before evaluation

E. Arrange physician-led PSG assessment even if symptoms are not reported

CORRECT ANSWER E

OSA is common and may be clinically silent in Down syndrome; PSG screening by age four is highlighted in the notes.

REFERENCE Integrated notebook, Topic 8, §8.9

QUESTION 0759 · §8.6 · Applied clinical SBA

Parents report that their 6-year-old snores and struggles with classroom attention. Which of the following statements about jaw size and position is most accurate?

A. High-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing.

B. Children with SDB may show inattention, irritability or hyperactivity instead of prominent sleepiness.

C. Tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development.

D. Paediatric evaluation should elicit chronic allergies, asthma, recurrent ENT infections and a family history of OSA.

E. Retrognathia or micrognathia can contribute to a narrower pharyngeal airway.

CORRECT ANSWER E

Retrognathia or micrognathia can contribute to a narrower pharyngeal airway. Additional revision detail: retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillary length. The other options describe different mechanisms, investigations or findings rather than jaw size and position. Consult Integrated notebook, page 158, §8.6 for the full context.

REFERENCE Integrated notebook, page 158, §8.6

QUESTION 0760 · §8.8 · Clinical interpretation SBA

A child with Down syndrome attends a routine dental appointment and has no reported sleepiness. The clinical team records the following finding: Children: thorough clinical evaluation of behaviour and sleep patterns; PSG; imaging (radiographs, endoscopy) may identify anatomical obstruction; end-tidal CO2 is a standard PSG measurement. Which concept best explains this observation?

A. Treatment

B. Presentation

C. Causes

D. Thresholds

E. Diagnosis

CORRECT ANSWER E

This finding most directly describes Diagnosis: Children: thorough clinical evaluation of behaviour and sleep patterns; PSG; imaging (radiographs, endoscopy) may identify anatomical obstruction; end-tidal CO2 is a standard PSG measurement. Distractor distinctions: A refers instead to Children: surgical removal of enlarged tonsils and adenoids first; weight management; PAP where surgery is ineffective; behav; B refers instead to Children: snoring or noisy breathing; restless sleep or unusual positions; frequent bed-wetting; daytime fatigue and attentio; C refers instead to Children: enlarged tonsils and adenoids lead; genetic predisposition; obesity in a minority but increasingly in adolescents;; D refers instead to Children: AHI ≥1 abnormal; mild 1–5, moderate 5–10, severe ≥10. Adults: AHI ≥5 with symptoms or ≥15 regardless; mild 5–15, mo.

REFERENCE Integrated notebook, page 160, §8.8

QUESTION 0761 · §8.7 · Clinical decision-making SBA

A parent requests rapid maxillary expansion specifically to cure their child's OSA. What is an evidence-consistent response?

A. All children worsen following expansion

B. Expansion cures every child

C. Expansion may help selected maxillary constriction but is not a reliable substitute for medical treatment or follow-up PSG

D. Physician assessment can be omitted

E. An orthodontic scan alone diagnoses OSA

CORRECT ANSWER C

Published improvements are variable and not definitive; RME may be adjunctive in selected cases under coordinated medical supervision.

REFERENCE Integrated notebook, Topic 8, §8.7

QUESTION 0762 · §8.7 · Applied clinical SBA

A 12-year-old with behavioural changes and habitual snoring is referred for assessment. Which of the following statements about indications is most accurate?

A. AHI is increased supine and lowest prone in children — but position is hard to maintain overnight.

B. Managing adverse effects and providing behavioural support can improve appliance adherence.

C. Children with persistent OSA after surgery may require PAP or additional specialist management.

D. Obesity and higher preoperative AHI increase the risk of residual paediatric OSA after surgery.

E. Submucous cleft palate is relative — a partial adenotonsillectomy may be performed.

CORRECT ANSWER C

Children with persistent OSA after surgery may require PAP or additional specialist management. Additional revision detail: children who do not respond adequately to surgery, or in whom surgery is contraindicated — which can include patient and family preference, for example religious belief against surgery or blood transfusion. The other options describe different mechanisms, investigations or findings rather than indications. Consult Integrated notebook, page 176, §8.7 for the full context.

REFERENCE Integrated notebook, page 176, §8.7

QUESTION 0763 · §8.9 · Clinical interpretation SBA

Parents request advice for a child with large tonsils and nocturnal gasping. The clinical team records the following finding: only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of children with OSA had no symptoms. Which concept best explains this observation?

A. Genetic syndromes

B. Medical conditions

C. Clinical symptoms are unreliable

D. Predisposing factors

E. OSA is highly prevalent

CORRECT ANSWER C

This finding most directly describes Clinical symptoms are unreliable: only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of children with OSA had no symptoms. Distractor distinctions: A refers instead to Down, Prader–Willi, Crouzon and cleft lip and palate often involve craniofacial abnormalities — midfacial hypoplasia, macrogl; B refers instead to obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction, inflammation or decreased; D refers instead to macroglossia, midface hypoplasia, overweight, poor muscle tone. Moderate–severe disease is more prevalent at younger ages; E refers instead to Prevalence by AHI threshold: >1 in 69%; >1.5 in 76%; >2 in 75%; >5 (moderate–severe) in 50%; >10 in 34%. Compare 1–5% in the.

REFERENCE Integrated notebook, page 178, §8.9

QUESTION 0764 · §8.4 · Clinical decision-making SBA

A 14-year-old sleeps 6.5 hours on school nights and catches up at weekends. Which issue should be assessed first?

A. Weekend sleep always cures school-night debt

B. Normal recommended sleep duration is six hours

C. Chronic insufficient sleep in the setting of adolescent circadian delay

D. A mandibular device is the first-line response

E. The pattern proves OSA

CORRECT ANSWER C

Teenagers generally require 8–10 hours; adolescent delayed phase and early school starts frequently create chronic sleep restriction.

REFERENCE Integrated notebook, Topic 8, §8.4

QUESTION 0765 · §8.8 · Applied clinical SBA

A child with Down syndrome attends a routine dental appointment and has no reported sleepiness. Which of the following statements about diagnosis is most accurate?

A. In children, an obstructive AHI of at least 1 event/h is abnormal; thresholds are lower than in adults.

B. Paediatric sleep apnoea requires clinical assessment and physician-directed sleep testing.

C. Children with OSA may have snoring, behavioural changes and attention problems rather than obvious daytime sleepiness.

D. Adenotonsillar enlargement is a major cause of paediatric obstructive sleep apnoea.

E. Adenotonsillectomy is commonly first-line for paediatric OSA with adenotonsillar hypertrophy.

CORRECT ANSWER B

Paediatric sleep apnoea requires clinical assessment and physician-directed sleep testing. Additional revision detail: Children: thorough clinical evaluation of behaviour and sleep patterns; PSG; imaging (radiographs, endoscopy) may identify anatomical obstruction; end-tidal CO2 is a standard PSG measurement. The other options describe different mechanisms, investigations or findings rather than diagnosis. Consult Integrated notebook, page 160, §8.8 for the full context.

REFERENCE Integrated notebook, page 160, §8.8

QUESTION 0766 · §8.10 · Clinical interpretation SBA

An 8-year-old with a narrow palate and restless sleep is seen for an orthodontic assessment. The clinical team records the following finding: age <3 or ≥7, obesity, high pre-op AHI, nasal/craniofacial abnormality, family history, asthma. Overnight admission if SpO2 nadir <80%, AHI ≥24/h or peak PCO2 ≥60 mmHg. Partial tonsillectomy regrowth 0.5– 16%. Which concept best explains this observation?

A. Indications for PSG

B. Adenotonsillectomy failure risk

C. PSQ threshold >0.33 (>8 of 22 items); FAIREST-6

D. Paediatric AHI

E. Sleep physician or ENT

CORRECT ANSWER B

This finding most directly describes Adenotonsillectomy failure risk: age <3 or ≥7, obesity, high pre-op AHI, nasal/craniofacial abnormality, family history, asthma. Overnight admission if SpO2 nadir <80%, AHI ≥24/h or peak PCO2 ≥60 mmHg. Partial tonsillectomy regrowth 0.5– 16%. Distractor distinctions: A refers instead to clinical symptoms of SDB, especially before adenotonsillectomy in children under 2 years, in obesity, in craniofacial or neur; C refers instead to 2 mild / 4 moderate / 6 severe; paediatric ESS ≥10; tonsillar obstruction >50% warrants evaluation; D refers instead to normal <1 · mild 1–5 · moderate 5–10 · severe ≥10. Obstructive AHI ≥1 is abnormal; E refers instead to for children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging. F.

REFERENCE Integrated notebook, page 162, §8.10

QUESTION 0767 · §8.6 · Clinical decision-making SBA

A 7-year-old has loud snoring, mouth breathing and tonsils occupying more than half of the oropharyngeal space. What is the most useful action?

A. Infer AHI from tonsil grade alone

B. Start orthodontic expansion before medical evaluation

C. Dismiss the symptoms as normal growing pains

D. Document tonsil grade and refer for paediatric airway evaluation

E. Fit an adult CPAP setting independently

CORRECT ANSWER D

Brodsky tonsil grading documents obstruction but does not itself diagnose OSA; symptoms plus hypertrophy justify specialist evaluation.

REFERENCE Integrated notebook, Topic 8, §8.6

QUESTION 0768 · §8.9 · Applied clinical SBA

Parents request advice for a child with large tonsils and nocturnal gasping. Which of the following statements about clinical symptoms are unreliable is most accurate?

A. Macroglossia, midface hypoplasia, overweight, poor muscle tone. Moderate–severe disease is more prevalent at younger ages.

B. Down, Prader–Willi and craniofacial syndromes increase the likelihood of paediatric OSA.

C. Only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of children with OSA had no symptoms.

D. Medical conditions includes obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction.

E. Down syndrome is associated with a high prevalence of paediatric OSA, even without reported symptoms.

CORRECT ANSWER C

Only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of children with OSA had no symptoms. The other options describe different mechanisms, investigations or findings rather than clinical symptoms are unreliable. Consult Integrated notebook, page 178, §8.9 for the full context.

REFERENCE Integrated notebook, page 178, §8.9

QUESTION 0769 · §8.1 · Clinical interpretation SBA

Parents report that their 6-year-old snores and struggles with classroom attention. The clinical team records the following finding: craniofacial morphology, tonsillar hypertrophy, altered neuromuscular tone, ventilatory control. Which concept best explains this observation?

A. Clinical symptoms are unreliable

B. The four contributing mechanisms

C. Habitual snoring

D. Prevalence

E. General prevalence of paediatric OSA

CORRECT ANSWER B

This finding most directly describes The four contributing mechanisms: craniofacial morphology, tonsillar hypertrophy, altered neuromuscular tone, ventilatory control. Distractor distinctions: A refers instead to only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of child; C refers instead to 3–12% (some sources cite 10–20%); D refers instead to OSA 1–5% (typically 2–3%); habitual snoring 3–12% (some cite 10–20%); E refers instead to Approximately 1–5% of children, with estimates near 2–3%.

REFERENCE Integrated notebook, page 156, §8.1

QUESTION 0770 · §8.9 · Clinical decision-making SBA

A child with obesity and craniofacial syndrome remains symptomatic after adenotonsillectomy. What is the next principle?

A. Reassessment for residual OSA with a multidisciplinary team

B. Use adult screening exclusively

C. Treat only the bite

D. Ignore obesity and neuromuscular factors

E. Assume the surgery always cures disease

CORRECT ANSWER A

Residual OSA risk is higher with obesity, severe baseline OSA and syndromic or craniofacial factors; objective reassessment is important.

REFERENCE Integrated notebook, Topic 8, §8.9

QUESTION 0771 · §8.10 · Applied clinical SBA

An 8-year-old with a narrow palate and restless sleep is seen for an orthodontic assessment. Which of the following statements about adenotonsillectomy failure risk is most accurate?

A. Recommended sleep duration varies with age, from 12–16 h/day in infants aged 4–12 months to 8–10 h in teenagers.

B. Paediatric PSG is particularly indicated when significant comorbidities or uncertain clinical findings complicate suspected OSA.

C. For children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging.

D. Residual paediatric OSA after adenotonsillectomy is more likely with obesity or high preoperative AHI.

E. PSQ threshold >0.33 (>8 of 22 items); FAIREST-6: 2 mild / 4 moderate / 6 severe; paediatric ESS ≥10; tonsillar obstruction >50% warrants evaluation.

CORRECT ANSWER D

Residual paediatric OSA after adenotonsillectomy is more likely with obesity or high preoperative AHI. Additional revision detail: age <3 or ≥7, obesity, high pre-op AHI, nasal/craniofacial abnormality, family history, asthma. Overnight admission if SpO2 nadir <80%, AHI ≥24/h or peak PCO2 ≥60 mmHg. Partial tonsillectomy regrowth 0.5– 16%. The other options describe different mechanisms, investigations or findings rather than adenotonsillectomy failure risk. Consult Integrated notebook, page 162, §8.10 for the full context.

REFERENCE Integrated notebook, page 162, §8.10

QUESTION 0772 · §8.4 · Clinical interpretation SBA

An 8-year-old with a narrow palate and restless sleep is seen for an orthodontic assessment. The clinical team records the following finding: attention, behaviour and learning problems; increased risk of accidents, injuries, hypertension, obesity, diabetes and depression; in teenagers, increased self-harm, suicidal thoughts and suicide attempts. Which concept best explains this observation?

A. Circadian phase delay

B. Recommended duration

C. Jaw size and position

D. Partial tonsillectomy

E. Less than recommended

CORRECT ANSWER E

This finding most directly describes Less than recommended: attention, behaviour and learning problems; increased risk of accidents, injuries, hypertension, obesity, diabetes and depression; in teenagers, increased self-harm, suicidal thoughts and suicide attempts. Distractor distinctions: A refers instead to Early adolescent development and pubertal onset are associated with a more evening-type circadian phase preference — the pref; B refers instead to <4 months not specified (variation too large) · 4–12 months 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y; C refers instead to retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillar; D refers instead to leaves tissue covering the tonsillar fossa musculature.

REFERENCE Integrated notebook, page 157, §8.4

QUESTION 0773 · §8.10 · Clinical decision-making SBA

A parent reports habitual snoring in a preschool child and asks whether a dental examination diagnoses OSA. What is correct?

A. A flat-plane nightguard is mandatory before PSG

B. Dentists may declare OSA from snoring alone

C. A PSQ above threshold establishes definitive AHI

D. Clinical screening triggers referral; overnight PSG is the diagnostic gold standard

E. A Mallampati score diagnoses paediatric OSA

CORRECT ANSWER D

Paediatric screening supports suspicion, while diagnosis and severity require appropriate physician-led sleep evaluation.

REFERENCE Integrated notebook, Topic 8, §8.10

QUESTION 0774 · §8.1 · Applied clinical SBA

Parents report that their 6-year-old snores and struggles with classroom attention. Which of the following statements about the four contributing mechanisms is most accurate?

A. PSQ threshold >0.33 (>8 of 22 items); FAIREST-6: 2 mild / 4 moderate / 6 severe; paediatric ESS ≥10; tonsillar obstruction >50% warrants evaluation.

B. The clinical finding includes OSA 1–5% (typically 2–3%); habitual snoring 3–12% (some cite 10–20%).

C. Paediatric OSA is influenced by craniofacial anatomy, adenotonsillar size and ventilatory control.

D. Paediatric OSA is estimated to affect approximately 1–5% of children.

E. Habitual snoring: 3–12% (some sources cite 10–20%).

CORRECT ANSWER C

Paediatric OSA is influenced by craniofacial anatomy, adenotonsillar size and ventilatory control. Additional revision detail: craniofacial morphology, tonsillar hypertrophy, altered neuromuscular tone, ventilatory control. The other options describe different mechanisms, investigations or findings rather than the four contributing mechanisms. Consult Integrated notebook, page 156, §8.1 for the full context.

REFERENCE Integrated notebook, page 156, §8.1

QUESTION 0775 · §8.5 · Clinical interpretation SBA

Parents report that their 6-year-old snores and struggles with classroom attention. The clinical team records the following finding: Sleep pressure builds more slowly with time awake in adolescents. Which concept best explains this observation?

A. Sleep–wake homeostasis

B. School start times

C. Circadian phase delay

D. Electronic media

E. Mental health problems

CORRECT ANSWER A

This finding most directly describes Sleep–wake homeostasis: Sleep pressure builds more slowly with time awake in adolescents. Distractor distinctions: B refers instead to early starts conflict with the delayed rhythm, producing chronic insufficient sleep; C refers instead to Early adolescent development and pubertal onset are associated with a more evening-type circadian phase preference — the pref; D refers instead to four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night; E refers instead to anxiety, stress sleep, leading to residual daytime sleepiness.

REFERENCE Integrated notebook, page 166, §8.5

QUESTION 0776 · §8.5 · Clinical decision-making SBA

An adolescent has late-night device use, delayed sleep onset and poor decisions during daytime. What is the most appropriate explanatory model?

A. Poor decisions are unrelated to sleep

B. An oral appliance fixes all adolescent sleep debt

C. Delayed phase means OSA is impossible

D. Media displacement, circadian phase delay and sleep debt can interact

E. Screen time guarantees central apnoea

CORRECT ANSWER D

Adolescent SDB and insufficient sleep need separate assessment; media use and phase delay can impair cognition and impulse control.

REFERENCE Integrated notebook, Topic 8, §8.5

QUESTION 0777 · §8.4 · Applied clinical SBA

An 8-year-old with a narrow palate and restless sleep is seen for an orthodontic assessment. Which of the following statements about less than recommended is most accurate?

A. Less than recommended includes attention, behaviour and learning problems.

B. Adolescents generally need 8–10 hours of sleep per night; younger children require longer sleep.

C. Paediatric PSG is particularly indicated when significant comorbidities or uncertain clinical findings complicate suspected OSA.

D. The clinician should use the modified Mallampati, Friedman tongue classification, or the Brodsky scale.

E. Paediatric sleep apnoea requires clinical assessment and physician-directed sleep testing.

CORRECT ANSWER A

Less than recommended includes attention, behaviour and learning problems. Additional revision detail: attention, behaviour and learning problems; increased risk of accidents, injuries, hypertension, obesity, diabetes and depression; in teenagers, increased self-harm, suicidal thoughts and suicide attempts. The other options describe different mechanisms, investigations or findings rather than less than recommended. Consult Integrated notebook, page 157, §8.4 for the full context.

REFERENCE Integrated notebook, page 157, §8.4

QUESTION 0778 · §8.6 · Clinical interpretation SBA

A 12-year-old with behavioural changes and habitual snoring is referred for assessment. The clinical team records the following finding: use the modified Mallampati, Friedman tongue classification, or the Brodsky scale. Hypertrophy obstructing >50% of the airway warrants further evaluation — but tonsil size does not correlate with severity. Which concept best explains this observation?

A. Palate and arches

B. Tongue posture and function

C. Jaw size and position

D. Medical, dental and family history

E. Tonsils and adenoids

CORRECT ANSWER E

This finding most directly describes Tonsils and adenoids: use the modified Mallampati, Friedman tongue classification, or the Brodsky scale. Hypertrophy obstructing >50% of the airway warrants further evaluation — but tonsil size does not correlate with severity. Distractor distinctions: A refers instead to high-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing; B refers instead to tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial develo; C refers instead to retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillar; D refers instead to chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematur.

REFERENCE Integrated notebook, page 158, §8.6

QUESTION 0779 · §8.5 · Applied clinical SBA

A 12-year-old with behavioural changes and habitual snoring is referred for assessment. Which of the following statements about sleep–wake homeostasis is most accurate?

A. Anxiety and stress may disturb sleep independently of obstructive respiratory events.

B. Other causes includes chronic medical illness; anxiety and stress; prescribed psychotropics.

C. Early school starts can worsen insufficient sleep in adolescents with delayed circadian timing.

D. Evening screen use can delay bedtime and interfere with adolescent sleep regulation.

E. Sleep–wake homeostasis includes sleep pressure builds more slowly with time awake in adolescents.

CORRECT ANSWER E

Sleep–wake homeostasis includes sleep pressure builds more slowly with time awake in adolescents. Additional revision detail: Sleep pressure builds more slowly with time awake in adolescents. The other options describe different mechanisms, investigations or findings rather than sleep–wake homeostasis. Consult Integrated notebook, page 166, §8.5 for the full context.

REFERENCE Integrated notebook, page 166, §8.5

QUESTION 0780 · §8.7 · Clinical interpretation SBA

Parents request advice for a child with large tonsils and nocturnal gasping. The clinical team records the following finding: Attempt to improve adherence by addressing adverse effects and using behaviour modification techniques; if the child remains non-compliant, use an alternative treatment. Which concept best explains this observation?

A. Positional and other

B. Contraindication

C. Risk factors for failure

D. Requires objective assessment of adherence

E. Partial tonsillectomy

CORRECT ANSWER D

This finding most directly describes Requires objective assessment of adherence: Attempt to improve adherence by addressing adverse effects and using behaviour modification techniques; if the child remains non-compliant, use an alternative treatment. Distractor distinctions: A refers instead to AHI is increased supine and lowest prone in children — but position is hard to maintain overnight; B refers instead to submucous cleft palate is relative — a partial adenotonsillectomy may be performed; C refers instead to obesity; high pre-operative AHI; age <3 or ≥7; nasal abnormality or craniofacial disproportion; family history of OSA; asthma; E refers instead to leaves tissue covering the tonsillar fossa musculature.

REFERENCE Integrated notebook, page 176, §8.7

QUESTION 0781 · §8.6 · Applied clinical SBA

Parents request advice for a child with large tonsils and nocturnal gasping. Which of the following statements about tonsils and adenoids is most accurate?

A. Paediatric evaluation should elicit chronic allergies, asthma, recurrent ENT infections and a family history of OSA.

B. The clinician should use the modified Mallampati, Friedman tongue classification, or the Brodsky scale.

C. High-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing.

D. Tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development.

E. Retrognathia or micrognathia can contribute to a narrower pharyngeal airway.

CORRECT ANSWER B

The clinician should use the modified Mallampati, Friedman tongue classification, or the Brodsky scale. Additional revision detail: use the modified Mallampati, Friedman tongue classification, or the Brodsky scale. Hypertrophy obstructing >50% of the airway warrants further evaluation — but tonsil size does not correlate with severity. The other options describe different mechanisms, investigations or findings rather than tonsils and adenoids. Consult Integrated notebook, page 158, §8.6 for the full context.

REFERENCE Integrated notebook, page 158, §8.6

QUESTION 0782 · §8.8 · Clinical interpretation SBA

Parents report that their 6-year-old snores and struggles with classroom attention. The clinical team records the following finding: Children: AHI ≥1 abnormal; mild 1–5, moderate 5–10, severe ≥10. Adults: AHI ≥5 with symptoms or ≥15 regardless; mild 5–15, moderate 15–30, severe ≥30. Age boundary 12 years. Which concept best explains this observation?

A. Treatment

B. Thresholds

C. Causes

D. Presentation

E. Diagnosis

CORRECT ANSWER B

This finding most directly describes Thresholds: Children: AHI ≥1 abnormal; mild 1–5, moderate 5–10, severe ≥10. Adults: AHI ≥5 with symptoms or ≥15 regardless; mild 5–15, moderate 15–30, severe ≥30. Age boundary 12 years. Distractor distinctions: A refers instead to Children: surgical removal of enlarged tonsils and adenoids first; weight management; PAP where surgery is ineffective; behav; C refers instead to Children: enlarged tonsils and adenoids lead; genetic predisposition; obesity in a minority but increasingly in adolescents;; D refers instead to Children: snoring or noisy breathing; restless sleep or unusual positions; frequent bed-wetting; daytime fatigue and attentio; E refers instead to Children: thorough clinical evaluation of behaviour and sleep patterns; PSG; imaging (radiographs, endoscopy) may identify an.

REFERENCE Integrated notebook, page 160, §8.8

QUESTION 0783 · §8.7 · Applied clinical SBA

Parents report that their 6-year-old snores and struggles with classroom attention. Which of the following statements about requires objective assessment of adherence is most accurate?

A. Obesity and higher preoperative AHI increase the risk of residual paediatric OSA after surgery.

B. AHI is increased supine and lowest prone in children — but position is hard to maintain overnight.

C. Children with persistent OSA after surgery may require PAP or additional specialist management.

D. Medication includes intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe.

E. Managing adverse effects and providing behavioural support can improve appliance adherence.

CORRECT ANSWER E

Managing adverse effects and providing behavioural support can improve appliance adherence. Additional revision detail: Attempt to improve adherence by addressing adverse effects and using behaviour modification techniques; if the child remains non-compliant, use an alternative treatment. The other options describe different mechanisms, investigations or findings rather than requires objective assessment of adherence. Consult Integrated notebook, page 176, §8.7 for the full context.

REFERENCE Integrated notebook, page 176, §8.7

QUESTION 0784 · §8.9 · Clinical interpretation SBA

A child with Down syndrome attends a routine dental appointment and has no reported sleepiness. The clinical team records the following finding: Prevalence by AHI threshold: >1 in 69%; >1.5 in 76%; >2 in 75%; >5 (moderate–severe) in 50%; >10 in 34%. Compare 1–5% in the general paediatric population. Which concept best explains this observation?

A. Clinical symptoms are unreliable

B. Medical conditions

C. Genetic syndromes

D. Predisposing factors

E. OSA is highly prevalent

CORRECT ANSWER E

This finding most directly describes OSA is highly prevalent: Prevalence by AHI threshold: >1 in 69%; >1.5 in 76%; >2 in 75%; >5 (moderate–severe) in 50%; >10 in 34%. Compare 1–5% in the general paediatric population. Distractor distinctions: A refers instead to only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of child; B refers instead to obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction, inflammation or decreased; C refers instead to Down, Prader–Willi, Crouzon and cleft lip and palate often involve craniofacial abnormalities — midfacial hypoplasia, macrogl; D refers instead to macroglossia, midface hypoplasia, overweight, poor muscle tone. Moderate–severe disease is more prevalent at younger ages.

REFERENCE Integrated notebook, page 178, §8.9

QUESTION 0785 · §8.8 · Applied clinical SBA

A child with Down syndrome attends a routine dental appointment and has no reported sleepiness. Which of the following statements about thresholds is most accurate?

A. In children, an obstructive AHI of at least 1 event/h is abnormal; thresholds are lower than in adults.

B. Children with OSA may have snoring, behavioural changes and attention problems rather than obvious daytime sleepiness.

C. Adenotonsillectomy is commonly first-line for paediatric OSA with adenotonsillar hypertrophy.

D. Paediatric sleep apnoea requires clinical assessment and physician-directed sleep testing.

E. Adenotonsillar enlargement is a major cause of paediatric obstructive sleep apnoea.

CORRECT ANSWER A

In children, an obstructive AHI of at least 1 event/h is abnormal; thresholds are lower than in adults. Additional revision detail: Children: AHI ≥1 abnormal; mild 1–5, moderate 5–10, severe ≥10. Adults: AHI ≥5 with symptoms or ≥15 regardless; mild 5–15, moderate 15–30, severe ≥30. Age boundary 12 years. The other options describe different mechanisms, investigations or findings rather than thresholds. Consult Integrated notebook, page 160, §8.8 for the full context.

REFERENCE Integrated notebook, page 160, §8.8

QUESTION 0786 · §8.10 · Clinical interpretation SBA

An 8-year-old with a narrow palate and restless sleep is seen for an orthodontic assessment. The clinical team records the following finding: 4–12 mo 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h. <8 h in adolescence → ~3× suicide-attempt risk; caffeine users 2× more likely to have difficulty sleeping. Which concept best explains this observation?

A. Indications for PSG

B. Sleep duration

C. Sleep physician or ENT

D. Paediatric AHI

E. Adenotonsillectomy failure risk

CORRECT ANSWER B

This finding most directly describes Sleep duration: 4–12 mo 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h. <8 h in adolescence → ~3× suicide-attempt risk; caffeine users 2× more likely to have difficulty sleeping. Distractor distinctions: A refers instead to clinical symptoms of SDB, especially before adenotonsillectomy in children under 2 years, in obesity, in craniofacial or neur; C refers instead to for children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging. F; D refers instead to normal <1 · mild 1–5 · moderate 5–10 · severe ≥10. Obstructive AHI ≥1 is abnormal; E refers instead to age <3 or ≥7, obesity, high pre-op AHI, nasal/craniofacial abnormality, family history, asthma. Overnight admission if SpO2 n.

REFERENCE Integrated notebook, page 162, §8.10

QUESTION 0787 · §8.9 · Applied clinical SBA

An 8-year-old with a narrow palate and restless sleep is seen for an orthodontic assessment. Which of the following statements about OSA is highly prevalent is most accurate?

A. Down syndrome is associated with a high prevalence of paediatric OSA, even without reported symptoms.

B. Medical conditions includes obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction.

C. Macroglossia, midface hypoplasia, overweight, poor muscle tone. Moderate–severe disease is more prevalent at younger ages.

D. Down, Prader–Willi and craniofacial syndromes increase the likelihood of paediatric OSA.

E. Only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of children with OSA had no symptoms.

CORRECT ANSWER A

Down syndrome is associated with a high prevalence of paediatric OSA, even without reported symptoms. Additional revision detail: Prevalence by AHI threshold: >1 in 69%; >1.5 in 76%; >2 in 75%; >5 (moderate–severe) in 50%; >10 in 34%. Compare 1–5% in the general paediatric population. The other options describe different mechanisms, investigations or findings rather than osa is highly prevalent. Consult Integrated notebook, page 178, §8.9 for the full context.

REFERENCE Integrated notebook, page 178, §8.9

QUESTION 0788 · §8.1 · Clinical interpretation SBA

A 12-year-old with behavioural changes and habitual snoring is referred for assessment. The clinical team records the following finding: 3–12% (some sources cite 10–20%). Which concept best explains this observation?

A. Contraindication

B. The four contributing mechanisms

C. General prevalence of paediatric OSA

D. Prevalence

E. Habitual snoring

CORRECT ANSWER E

This finding most directly describes Habitual snoring: 3–12% (some sources cite 10–20%). Distractor distinctions: A refers instead to submucous cleft palate is relative — a partial adenotonsillectomy may be performed; B refers instead to craniofacial morphology, tonsillar hypertrophy, altered neuromuscular tone, ventilatory control; C refers instead to Approximately 1–5% of children, with estimates near 2–3%; D refers instead to OSA 1–5% (typically 2–3%); habitual snoring 3–12% (some cite 10–20%).

REFERENCE Integrated notebook, page 163, §8.1

QUESTION 0789 · §8.10 · Applied clinical SBA

A 12-year-old with behavioural changes and habitual snoring is referred for assessment. Which of the following statements about sleep duration is most accurate?

A. Residual paediatric OSA after adenotonsillectomy is more likely with obesity or high preoperative AHI.

B. Paediatric PSG is particularly indicated when significant comorbidities or uncertain clinical findings complicate suspected OSA.

C. For children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging.

D. In children, obstructive AHI of 1/h or more is abnormal, with severe disease generally at 10/h or more.

E. Recommended sleep duration varies with age, from 12–16 h/day in infants aged 4–12 months to 8–10 h in teenagers.

CORRECT ANSWER E

Recommended sleep duration varies with age, from 12–16 h/day in infants aged 4–12 months to 8–10 h in teenagers. Additional revision detail: 4–12 mo 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h. <8 h in adolescence → ~3× suicide-attempt risk; caffeine users 2× more likely to have difficulty sleeping. The other options describe different mechanisms, investigations or findings rather than sleep duration. Consult Integrated notebook, page 162, §8.10 for the full context.

REFERENCE Integrated notebook, page 162, §8.10

QUESTION 0790 · §8.5 · Clinical interpretation SBA

Parents report that their 6-year-old snores and struggles with classroom attention. The clinical team records the following finding: four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night. Which concept best explains this observation?

A. Electronic media

B. Weekend catch-up

C. School start times

D. Other causes

E. Circadian phase delay

CORRECT ANSWER A

This finding most directly describes Electronic media: four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night. Distractor distinctions: B refers instead to Going to bed later and waking later at weekends reflects circadian biology and is also a response to insufficient weekday sle; C refers instead to early starts conflict with the delayed rhythm, producing chronic insufficient sleep; D refers instead to chronic medical illness; anxiety and stress; prescribed psychotropics; E refers instead to Early adolescent development and pubertal onset are associated with a more evening-type circadian phase preference — the pref.

REFERENCE Integrated notebook, page 157, §8.5

QUESTION 0791 · §8.1 · Applied clinical SBA

Parents request advice for a child with large tonsils and nocturnal gasping. Which of the following statements about habitual snoring is most accurate?

A. Partial tonsillectomy includes leaves tissue covering the tonsillar fossa musculature.

B. Habitual snoring: 3–12% (some sources cite 10–20%).

C. Paediatric OSA is influenced by craniofacial anatomy, adenotonsillar size and ventilatory control.

D. Paediatric OSA is estimated to affect approximately 1–5% of children.

E. The clinical finding includes OSA 1–5% (typically 2–3%); habitual snoring 3–12% (some cite 10–20%).

CORRECT ANSWER B

Habitual snoring: 3–12% (some sources cite 10–20%). Additional revision detail: 3–12% (some sources cite 10–20%). The other options describe different mechanisms, investigations or findings rather than habitual snoring. Consult Integrated notebook, page 163, §8.1 for the full context.

REFERENCE Integrated notebook, page 163, §8.1

QUESTION 0792 · §8.6 · Clinical interpretation SBA

A child with Down syndrome attends a routine dental appointment and has no reported sleepiness. The clinical team records the following finding: chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematurity and syndromic conditions increase risk. Which concept best explains this observation?

A. Tonsils and adenoids

B. Medical, dental and family history

C. Tongue posture and function

D. Jaw size and position

E. Cognitive, behavioural and developmental assessment

CORRECT ANSWER B

This finding most directly describes Medical, dental and family history: chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematurity and syndromic conditions increase risk. Distractor distinctions: A refers instead to use the modified Mallampati, Friedman tongue classification, or the Brodsky scale. Hypertrophy obstructing >50% of the airway; C refers instead to tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial develo; D refers instead to retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillar; E refers instead to children with SDB often display hyperactivity, irritability and difficulty concentrating, closely resembling ADHD.

REFERENCE Integrated notebook, page 168, §8.6

QUESTION 0793 · §8.5 · Applied clinical SBA

A child with Down syndrome attends a routine dental appointment and has no reported sleepiness. Which of the following statements about electronic media is most accurate?

A. Anxiety and stress may disturb sleep independently of obstructive respiratory events.

B. Sleep–wake homeostasis includes sleep pressure builds more slowly with time awake in adolescents.

C. Evening screen use can delay bedtime and interfere with adolescent sleep regulation.

D. Other causes includes chronic medical illness; anxiety and stress; prescribed psychotropics.

E. Early school starts can worsen insufficient sleep in adolescents with delayed circadian timing.

CORRECT ANSWER C

Evening screen use can delay bedtime and interfere with adolescent sleep regulation. Additional revision detail: four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night. The other options describe different mechanisms, investigations or findings rather than electronic media. Consult Integrated notebook, page 157, §8.5 for the full context.

REFERENCE Integrated notebook, page 157, §8.5

QUESTION 0794 · §8.7 · Clinical interpretation SBA

An 8-year-old with a narrow palate and restless sleep is seen for an orthodontic assessment. The clinical team records the following finding: AHI is increased supine and lowest prone in children — but position is hard to maintain overnight. Which concept best explains this observation?

A. Contraindication

B. Risk factors for failure

C. Indications

D. Partial tonsillectomy

E. Positional and other

CORRECT ANSWER E

This finding most directly describes Positional and other: AHI is increased supine and lowest prone in children — but position is hard to maintain overnight. Distractor distinctions: A refers instead to submucous cleft palate is relative — a partial adenotonsillectomy may be performed; B refers instead to obesity; high pre-operative AHI; age <3 or ≥7; nasal abnormality or craniofacial disproportion; family history of OSA; asthma; C refers instead to children who do not respond adequately to surgery, or in whom surgery is contraindicated — which can include patient and fami; D refers instead to leaves tissue covering the tonsillar fossa musculature.

REFERENCE Integrated notebook, page 160, §8.7

QUESTION 0795 · §8.6 · Applied clinical SBA

An 8-year-old with a narrow palate and restless sleep is seen for an orthodontic assessment. Which of the following statements about medical, dental and family history is most accurate?

A. The clinician should use the modified Mallampati, Friedman tongue classification, or the Brodsky scale.

B. Tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development.

C. Retrognathia or micrognathia can contribute to a narrower pharyngeal airway.

D. Children with SDB may show inattention, irritability or hyperactivity instead of prominent sleepiness.

E. Paediatric evaluation should elicit chronic allergies, asthma, recurrent ENT infections and a family history of OSA.

CORRECT ANSWER E

Paediatric evaluation should elicit chronic allergies, asthma, recurrent ENT infections and a family history of OSA. Additional revision detail: chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematurity and syndromic conditions increase risk. The other options describe different mechanisms, investigations or findings rather than medical, dental and family history. Consult Integrated notebook, page 168, §8.6 for the full context.

REFERENCE Integrated notebook, page 168, §8.6

QUESTION 0796 · §8.8 · Focused knowledge SBA

A 12-year-old with behavioural changes and habitual snoring is referred for assessment. Which of the following statements about causes is most accurate?

A. Paediatric sleep apnoea requires clinical assessment and physician-directed sleep testing.

B. Adenotonsillar enlargement is a major cause of paediatric obstructive sleep apnoea.

C. In children, an obstructive AHI of at least 1 event/h is abnormal; thresholds are lower than in adults.

D. Children with OSA may have snoring, behavioural changes and attention problems rather than obvious daytime sleepiness.

E. Adenotonsillectomy is commonly first-line for paediatric OSA with adenotonsillar hypertrophy.

CORRECT ANSWER B

Adenotonsillar enlargement is a major cause of paediatric obstructive sleep apnoea. Additional revision detail: Children: enlarged tonsils and adenoids lead; genetic predisposition; obesity in a minority but increasingly in adolescents; commonest at 3–6 y. The other options describe different mechanisms, investigations or findings rather than causes. Consult Integrated notebook, page 160, §8.8 for the full context.

REFERENCE Integrated notebook, page 160, §8.8

QUESTION 0797 · §8.7 · Applied clinical SBA

A 12-year-old with behavioural changes and habitual snoring is referred for assessment. Which of the following statements about positional and other is most accurate?

A. Medication includes intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe.

B. AHI is increased supine and lowest prone in children — but position is hard to maintain overnight.

C. Obesity and higher preoperative AHI increase the risk of residual paediatric OSA after surgery.

D. Submucous cleft palate is relative — a partial adenotonsillectomy may be performed.

E. Partial tonsillectomy includes leaves tissue covering the tonsillar fossa musculature.

CORRECT ANSWER B

AHI is increased supine and lowest prone in children — but position is hard to maintain overnight. The other options describe different mechanisms, investigations or findings rather than positional and other. Consult Integrated notebook, page 160, §8.7 for the full context.

REFERENCE Integrated notebook, page 160, §8.7

QUESTION 0798 · §8.9 · Clinical interpretation SBA

Parents request advice for a child with large tonsils and nocturnal gasping. The clinical team records the following finding: obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction, inflammation or decreased airway muscle tone. Which concept best explains this observation?

A. Clinical symptoms are unreliable

B. OSA is highly prevalent

C. Predisposing factors

D. Genetic syndromes

E. Medical conditions

CORRECT ANSWER E

This finding most directly describes Medical conditions: obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction, inflammation or decreased airway muscle tone. Distractor distinctions: A refers instead to only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of child; B refers instead to Prevalence by AHI threshold: >1 in 69%; >1.5 in 76%; >2 in 75%; >5 (moderate–severe) in 50%; >10 in 34%. Compare 1–5% in the; C refers instead to macroglossia, midface hypoplasia, overweight, poor muscle tone. Moderate–severe disease is more prevalent at younger ages; D refers instead to Down, Prader–Willi, Crouzon and cleft lip and palate often involve craniofacial abnormalities — midfacial hypoplasia, macrogl.

REFERENCE Integrated notebook, page 160, §8.9

QUESTION 0799 · §8.8 · Applied clinical SBA

Parents request advice for a child with large tonsils and nocturnal gasping. Which of the following statements about causes is most accurate?

A. In children, an obstructive AHI of at least 1 event/h is abnormal; thresholds are lower than in adults.

B. Adenotonsillectomy is commonly first-line for paediatric OSA with adenotonsillar hypertrophy.

C. Children with OSA may have snoring, behavioural changes and attention problems rather than obvious daytime sleepiness.

D. Adenotonsillar enlargement is a major cause of paediatric obstructive sleep apnoea.

E. Paediatric sleep apnoea requires clinical assessment and physician-directed sleep testing.

CORRECT ANSWER D

Adenotonsillar enlargement is a major cause of paediatric obstructive sleep apnoea. Additional revision detail: Children: enlarged tonsils and adenoids lead; genetic predisposition; obesity in a minority but increasingly in adolescents; commonest at 3–6 y. The other options describe different mechanisms, investigations or findings rather than causes. Consult Integrated notebook, page 160, §8.8 for the full context.

REFERENCE Integrated notebook, page 160, §8.8

QUESTION 0800 · §8.10 · Clinical interpretation SBA

Parents report that their 6-year-old snores and struggles with classroom attention. The clinical team records the following finding: for children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging. For enlarged adenoids and tonsils, surgical removal is first-line. Which concept best explains this observation?

A. Indications for PSG

B. Adenotonsillectomy failure risk

C. Paediatric AHI

D. Sleep physician or ENT

E. Sleep duration

CORRECT ANSWER D

This finding most directly describes Sleep physician or ENT: for children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging. For enlarged adenoids and tonsils, surgical removal is first-line. Distractor distinctions: A refers instead to clinical symptoms of SDB, especially before adenotonsillectomy in children under 2 years, in obesity, in craniofacial or neur; B refers instead to age <3 or ≥7, obesity, high pre-op AHI, nasal/craniofacial abnormality, family history, asthma. Overnight admission if SpO2 n; C refers instead to normal <1 · mild 1–5 · moderate 5–10 · severe ≥10. Obstructive AHI ≥1 is abnormal; E refers instead to 4–12 mo 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h. <8 h in adolescence → ~3× suicide-attempt r.

REFERENCE Integrated notebook, page 161, §8.10

QUESTION 0801 · §8.9 · Applied clinical SBA

Parents report that their 6-year-old snores and struggles with classroom attention. Which of the following statements about medical conditions is most accurate?

A. Down syndrome is associated with a high prevalence of paediatric OSA, even without reported symptoms.

B. Medical conditions includes obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction.

C. Down, Prader–Willi and craniofacial syndromes increase the likelihood of paediatric OSA.

D. Only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of children with OSA had no symptoms.

E. Macroglossia, midface hypoplasia, overweight, poor muscle tone. Moderate–severe disease is more prevalent at younger ages.

CORRECT ANSWER B

Medical conditions includes obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction. Additional revision detail: obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction, inflammation or decreased airway muscle tone. The other options describe different mechanisms, investigations or findings rather than medical conditions. Consult Integrated notebook, page 160, §8.9 for the full context.

REFERENCE Integrated notebook, page 160, §8.9

QUESTION 0802 · §8.1 · Clinical interpretation SBA

A child with Down syndrome attends a routine dental appointment and has no reported sleepiness. The clinical team records the following finding: OSA 1–5% (typically 2–3%); habitual snoring 3–12% (some cite 10–20%). Which concept best explains this observation?

A. Medication

B. General prevalence of paediatric OSA

C. Prevalence

D. Habitual snoring

E. The four contributing mechanisms

CORRECT ANSWER C

This finding most directly describes Prevalence: OSA 1–5% (typically 2–3%); habitual snoring 3–12% (some cite 10–20%). Distractor distinctions: A refers instead to Intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe; B refers instead to Approximately 1–5% of children, with estimates near 2–3%; D refers instead to 3–12% (some sources cite 10–20%); E refers instead to craniofacial morphology, tonsillar hypertrophy, altered neuromuscular tone, ventilatory control.

REFERENCE Integrated notebook, page 156, §8.1

QUESTION 0803 · §8.10 · Applied clinical SBA

A child with Down syndrome attends a routine dental appointment and has no reported sleepiness. Which of the following statements about sleep physician or ent is most accurate?

A. PSQ threshold >0.33 (>8 of 22 items); FAIREST-6: 2 mild / 4 moderate / 6 severe; paediatric ESS ≥10; tonsillar obstruction >50% warrants evaluation.

B. Paediatric PSG is particularly indicated when significant comorbidities or uncertain clinical findings complicate suspected OSA.

C. Residual paediatric OSA after adenotonsillectomy is more likely with obesity or high preoperative AHI.

D. For children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging.

E. Recommended sleep duration varies with age, from 12–16 h/day in infants aged 4–12 months to 8–10 h in teenagers.

CORRECT ANSWER D

For children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging. Additional revision detail: for children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging. For enlarged adenoids and tonsils, surgical removal is first-line. The other options describe different mechanisms, investigations or findings rather than sleep physician or ent. Consult Integrated notebook, page 161, §8.10 for the full context.

REFERENCE Integrated notebook, page 161, §8.10

QUESTION 0804 · §8.5 · Focused knowledge SBA

A 12-year-old with behavioural changes and habitual snoring is referred for assessment. Which of the following statements about weekend catch-up is most accurate?

A. Evening screen use can delay bedtime and interfere with adolescent sleep regulation.

B. Anxiety and stress may disturb sleep independently of obstructive respiratory events.

C. Early school starts can worsen insufficient sleep in adolescents with delayed circadian timing.

D. Adolescence is associated with a physiological delay in preferred sleep and wake times.

E. Late weekend sleep schedules can reinforce delayed sleep phase in adolescents.

CORRECT ANSWER E

Late weekend sleep schedules can reinforce delayed sleep phase in adolescents. Additional revision detail: late weekend bedtimes and delayed waking can reinforce an adolescent circadian phase delay and make school-day sleep timing more difficult. The other options describe different mechanisms, investigations or findings rather than weekend catch-up. Consult Integrated notebook, page 166, §8.5 for the full context.

REFERENCE Integrated notebook, page 166, §8.5

QUESTION 0805 · §8.1 · Applied clinical SBA

An 8-year-old with a narrow palate and restless sleep is seen for an orthodontic assessment. Which of the following statements about Prevalence is most accurate?

A. The clinical finding includes OSA 1–5% (typically 2–3%); habitual snoring 3–12% (some cite 10–20%).

B. Habitual snoring: 3–12% (some sources cite 10–20%).

C. Other causes includes chronic medical illness; anxiety and stress; prescribed psychotropics.

D. Paediatric OSA is influenced by craniofacial anatomy, adenotonsillar size and ventilatory control.

E. Paediatric OSA is estimated to affect approximately 1–5% of children.

CORRECT ANSWER A

The clinical finding includes OSA 1–5% (typically 2–3%); habitual snoring 3–12% (some cite 10–20%). Additional revision detail: OSA 1–5% (typically 2–3%); habitual snoring 3–12% (some cite 10–20%). The other options describe different mechanisms, investigations or findings rather than prevalence. Consult Integrated notebook, page 156, §8.1 for the full context.

REFERENCE Integrated notebook, page 156, §8.1

QUESTION 0806 · §8.6 · Clinical interpretation SBA

Parents request advice for a child with large tonsils and nocturnal gasping. The clinical team records the following finding: children with SDB often display hyperactivity, irritability and difficulty concentrating, closely resembling ADHD. Which concept best explains this observation?

A. Medical, dental and family history

B. Tongue posture and function

C. Tonsils and adenoids

D. Jaw size and position

E. Cognitive, behavioural and developmental assessment

CORRECT ANSWER E

This finding most directly describes Cognitive, behavioural and developmental assessment: children with SDB often display hyperactivity, irritability and difficulty concentrating, closely resembling ADHD. Distractor distinctions: A refers instead to chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematur; B refers instead to tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial develo; C refers instead to use the modified Mallampati, Friedman tongue classification, or the Brodsky scale. Hypertrophy obstructing >50% of the airway; D refers instead to retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillar.

REFERENCE Integrated notebook, page 168, §8.6

QUESTION 0807 · §8.5 · Applied clinical SBA

Parents request advice for a child with large tonsils and nocturnal gasping. Which of the following statements about weekend catch-up is most accurate?

A. Adolescence is associated with a physiological delay in preferred sleep and wake times.

B. Late weekend sleep schedules can reinforce delayed sleep phase in adolescents.

C. Evening screen use can delay bedtime and interfere with adolescent sleep regulation.

D. Anxiety and stress may disturb sleep independently of obstructive respiratory events.

E. Early school starts can worsen insufficient sleep in adolescents with delayed circadian timing.

CORRECT ANSWER B

Late weekend sleep schedules can reinforce delayed sleep phase in adolescents. Additional revision detail: late weekend bedtimes and delayed waking can reinforce an adolescent circadian phase delay and make school-day sleep timing more difficult. The other options describe different mechanisms, investigations or findings rather than weekend catch-up. Consult Integrated notebook, page 166, §8.5 for the full context.

REFERENCE Integrated notebook, page 166, §8.5

QUESTION 0808 · §8.7 · Clinical interpretation SBA

Parents report that their 6-year-old snores and struggles with classroom attention. The clinical team records the following finding: obesity; high pre-operative AHI; age <3 or ≥7; nasal abnormality or craniofacial disproportion; family history of OSA; asthma. Which concept best explains this observation?

A. Indications

B. Risk factors for failure

C. Partial tonsillectomy

D. Contraindication

E. Positional and other

CORRECT ANSWER B

This finding most directly describes Risk factors for failure: obesity; high pre-operative AHI; age <3 or ≥7; nasal abnormality or craniofacial disproportion; family history of OSA; asthma. Distractor distinctions: A refers instead to children who do not respond adequately to surgery, or in whom surgery is contraindicated — which can include patient and fami; C refers instead to leaves tissue covering the tonsillar fossa musculature; D refers instead to submucous cleft palate is relative — a partial adenotonsillectomy may be performed; E refers instead to AHI is increased supine and lowest prone in children — but position is hard to maintain overnight.

REFERENCE Integrated notebook, page 159, §8.7

QUESTION 0809 · §8.6 · Applied clinical SBA

Parents report that their 6-year-old snores and struggles with classroom attention. Which of the following statements about cognitive, behavioural and developmental assessment is most accurate?

A. Retrognathia or micrognathia can contribute to a narrower pharyngeal airway.

B. Paediatric evaluation should elicit chronic allergies, asthma, recurrent ENT infections and a family history of OSA.

C. Children with SDB may show inattention, irritability or hyperactivity instead of prominent sleepiness.

D. High-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing.

E. Tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development.

CORRECT ANSWER C

Children with SDB may show inattention, irritability or hyperactivity instead of prominent sleepiness. Additional revision detail: children with SDB often display hyperactivity, irritability and difficulty concentrating, closely resembling ADHD. The other options describe different mechanisms, investigations or findings rather than cognitive, behavioural and developmental assessment. Consult Integrated notebook, page 168, §8.6 for the full context.

REFERENCE Integrated notebook, page 168, §8.6

QUESTION 0810 · §8.8 · Clinical interpretation SBA

A child with Down syndrome attends a routine dental appointment and has no reported sleepiness. The clinical team records the following finding: Children: snoring or noisy breathing; restless sleep or unusual positions; frequent bed-wetting; daytime fatigue and attention difficulties; behavioural problems that may mimic ADHD; poor academic performance. Which concept best explains this observation?

A. Presentation

B. Thresholds

C. Diagnosis

D. Treatment

E. Causes

CORRECT ANSWER A

This finding most directly describes Presentation: Children: snoring or noisy breathing; restless sleep or unusual positions; frequent bed-wetting; daytime fatigue and attention difficulties; behavioural problems that may mimic ADHD; poor academic performance. Distractor distinctions: B refers instead to Children: AHI ≥1 abnormal; mild 1–5, moderate 5–10, severe ≥10. Adults: AHI ≥5 with symptoms or ≥15 regardless; mild 5–15, mo; C refers instead to Children: thorough clinical evaluation of behaviour and sleep patterns; PSG; imaging (radiographs, endoscopy) may identify an; D refers instead to Children: surgical removal of enlarged tonsils and adenoids first; weight management; PAP where surgery is ineffective; behav; E refers instead to Children: enlarged tonsils and adenoids lead; genetic predisposition; obesity in a minority but increasingly in adolescents;.

REFERENCE Integrated notebook, page 160, §8.8

QUESTION 0811 · §8.7 · Applied clinical SBA

A child with Down syndrome attends a routine dental appointment and has no reported sleepiness. Which of the following statements about risk factors for failure is most accurate?

A. Submucous cleft palate is relative — a partial adenotonsillectomy may be performed.

B. Managing adverse effects and providing behavioural support can improve appliance adherence.

C. Medication includes intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe.

D. AHI is increased supine and lowest prone in children — but position is hard to maintain overnight.

E. Obesity and higher preoperative AHI increase the risk of residual paediatric OSA after surgery.

CORRECT ANSWER E

Obesity and higher preoperative AHI increase the risk of residual paediatric OSA after surgery. Additional revision detail: obesity; high pre-operative AHI; age <3 or ≥7; nasal abnormality or craniofacial disproportion; family history of OSA; asthma. The other options describe different mechanisms, investigations or findings rather than risk factors for failure. Consult Integrated notebook, page 159, §8.7 for the full context.

REFERENCE Integrated notebook, page 159, §8.7

QUESTION 0812 · §8.9 · Clinical interpretation SBA

An 8-year-old with a narrow palate and restless sleep is seen for an orthodontic assessment. The clinical team records the following finding: Down, Prader–Willi, Crouzon and cleft lip and palate often involve craniofacial abnormalities — midfacial hypoplasia, macroglossia, retrognathia — narrowing the airway. Which concept best explains this observation?

A. Predisposing factors

B. Genetic syndromes

C. Clinical symptoms are unreliable

D. OSA is highly prevalent

E. Medical conditions

CORRECT ANSWER B

This finding most directly describes Genetic syndromes: Down, Prader–Willi, Crouzon and cleft lip and palate often involve craniofacial abnormalities — midfacial hypoplasia, macroglossia, retrognathia — narrowing the airway. Distractor distinctions: A refers instead to macroglossia, midface hypoplasia, overweight, poor muscle tone. Moderate–severe disease is more prevalent at younger ages; C refers instead to only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of child; D refers instead to Prevalence by AHI threshold: >1 in 69%; >1.5 in 76%; >2 in 75%; >5 (moderate–severe) in 50%; >10 in 34%. Compare 1–5% in the; E refers instead to obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction, inflammation or decreased.

REFERENCE Integrated notebook, page 161, §8.9

QUESTION 0813 · §8.8 · Applied clinical SBA

An 8-year-old with a narrow palate and restless sleep is seen for an orthodontic assessment. Which of the following statements about presentation is most accurate?

A. Paediatric sleep apnoea requires clinical assessment and physician-directed sleep testing.

B. Adenotonsillectomy is commonly first-line for paediatric OSA with adenotonsillar hypertrophy.

C. Adenotonsillar enlargement is a major cause of paediatric obstructive sleep apnoea.

D. Children with OSA may have snoring, behavioural changes and attention problems rather than obvious daytime sleepiness.

E. In children, an obstructive AHI of at least 1 event/h is abnormal; thresholds are lower than in adults.

CORRECT ANSWER D

Children with OSA may have snoring, behavioural changes and attention problems rather than obvious daytime sleepiness. Additional revision detail: Children: snoring or noisy breathing; restless sleep or unusual positions; frequent bed-wetting; daytime fatigue and attention difficulties; behavioural problems that may mimic ADHD; poor academic performance. The other options describe different mechanisms, investigations or findings rather than presentation. Consult Integrated notebook, page 160, §8.8 for the full context.

REFERENCE Integrated notebook, page 160, §8.8

QUESTION 0814 · §8.10 · Clinical interpretation SBA

A 12-year-old with behavioural changes and habitual snoring is referred for assessment. The clinical team records the following finding: normal <1 · mild 1–5 · moderate 5–10 · severe ≥10. Obstructive AHI ≥1 is abnormal. Which concept best explains this observation?

A. Paediatric AHI

B. Adenotonsillectomy failure risk

C. Sleep physician or ENT

D. Sleep duration

E. Indications for PSG

CORRECT ANSWER A

This finding most directly describes Paediatric AHI: normal <1 · mild 1–5 · moderate 5–10 · severe ≥10. Obstructive AHI ≥1 is abnormal. Distractor distinctions: B refers instead to age <3 or ≥7, obesity, high pre-op AHI, nasal/craniofacial abnormality, family history, asthma. Overnight admission if SpO2 n; C refers instead to for children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging. F; D refers instead to 4–12 mo 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h. <8 h in adolescence → ~3× suicide-attempt r; E refers instead to clinical symptoms of SDB, especially before adenotonsillectomy in children under 2 years, in obesity, in craniofacial or neur.

REFERENCE Integrated notebook, page 180, §8.10

QUESTION 0815 · §8.9 · Applied clinical SBA

A 12-year-old with behavioural changes and habitual snoring is referred for assessment. Which of the following statements about genetic syndromes is most accurate?

A. Down, Prader–Willi and craniofacial syndromes increase the likelihood of paediatric OSA.

B. Only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of children with OSA had no symptoms.

C. Down syndrome is associated with a high prevalence of paediatric OSA, even without reported symptoms.

D. Macroglossia, midface hypoplasia, overweight, poor muscle tone. Moderate–severe disease is more prevalent at younger ages.

E. Medical conditions includes obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction.

CORRECT ANSWER A

Down, Prader–Willi and craniofacial syndromes increase the likelihood of paediatric OSA. Additional revision detail: Down, Prader–Willi, Crouzon and cleft lip and palate often involve craniofacial abnormalities — midfacial hypoplasia, macroglossia, retrognathia — narrowing the airway. The other options describe different mechanisms, investigations or findings rather than genetic syndromes. Consult Integrated notebook, page 161, §8.9 for the full context.

REFERENCE Integrated notebook, page 161, §8.9

QUESTION 0816 · §8.5 · Clinical interpretation SBA

An 8-year-old with a narrow palate and restless sleep is seen for an orthodontic assessment. The clinical team records the following finding: Early adolescent development and pubertal onset are associated with a more evening-type circadian phase preference — the preferred timing of sleep, wake and daytime activity shifts later. Which concept best explains this observation?

A. Circadian phase delay

B. School start times

C. Mental health problems

D. Sleep–wake homeostasis

E. Electronic media

CORRECT ANSWER A

This finding most directly describes Circadian phase delay: Early adolescent development and pubertal onset are associated with a more evening-type circadian phase preference — the preferred timing of sleep, wake and daytime activity shifts later. Distractor distinctions: B refers instead to early starts conflict with the delayed rhythm, producing chronic insufficient sleep; C refers instead to anxiety, stress sleep, leading to residual daytime sleepiness; D refers instead to Sleep pressure builds more slowly with time awake in adolescents; E refers instead to four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night.

REFERENCE Integrated notebook, page 166, §8.5

QUESTION 0817 · §8.10 · Applied clinical SBA

Parents request advice for a child with large tonsils and nocturnal gasping. Which of the following statements about paediatric AHI is most accurate?

A. In children, obstructive AHI of 1/h or more is abnormal, with severe disease generally at 10/h or more.

B. For children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging.

C. PSQ threshold >0.33 (>8 of 22 items); FAIREST-6: 2 mild / 4 moderate / 6 severe; paediatric ESS ≥10; tonsillar obstruction >50% warrants evaluation.

D. Recommended sleep duration varies with age, from 12–16 h/day in infants aged 4–12 months to 8–10 h in teenagers.

E. Paediatric PSG is particularly indicated when significant comorbidities or uncertain clinical findings complicate suspected OSA.

CORRECT ANSWER A

In children, obstructive AHI of 1/h or more is abnormal, with severe disease generally at 10/h or more. Additional revision detail: normal <1 · mild 1–5 · moderate 5–10 · severe ≥10. Obstructive AHI ≥1 is abnormal. The other options describe different mechanisms, investigations or findings rather than paediatric ahi. Consult Integrated notebook, page 180, §8.10 for the full context.

REFERENCE Integrated notebook, page 180, §8.10

QUESTION 0818 · §8.6 · Clinical interpretation SBA

A 12-year-old with behavioural changes and habitual snoring is referred for assessment. The clinical team records the following finding: high-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing. Which concept best explains this observation?

A. Jaw size and position

B. Tongue posture and function

C. Tonsils and adenoids

D. Palate and arches

E. Medical, dental and family history

CORRECT ANSWER D

This finding most directly describes Palate and arches: high-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing. Distractor distinctions: A refers instead to retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillar; B refers instead to tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial develo; C refers instead to use the modified Mallampati, Friedman tongue classification, or the Brodsky scale. Hypertrophy obstructing >50% of the airway; E refers instead to chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematur.

REFERENCE Integrated notebook, page 158, §8.6

QUESTION 0819 · §8.5 · Applied clinical SBA

A 12-year-old with behavioural changes and habitual snoring is referred for assessment. Which of the following statements about circadian phase delay is most accurate?

A. Other causes includes chronic medical illness; anxiety and stress; prescribed psychotropics.

B. Anxiety and stress may disturb sleep independently of obstructive respiratory events.

C. Early school starts can worsen insufficient sleep in adolescents with delayed circadian timing.

D. Adolescence is associated with a physiological delay in preferred sleep and wake times.

E. Evening screen use can delay bedtime and interfere with adolescent sleep regulation.

CORRECT ANSWER D

Adolescence is associated with a physiological delay in preferred sleep and wake times. Additional revision detail: Early adolescent development and pubertal onset are associated with a more evening-type circadian phase preference — the preferred timing of sleep, wake and daytime activity shifts later. The other options describe different mechanisms, investigations or findings rather than circadian phase delay. Consult Integrated notebook, page 166, §8.5 for the full context.

REFERENCE Integrated notebook, page 166, §8.5

QUESTION 0820 · §8.7 · Clinical interpretation SBA

Parents request advice for a child with large tonsils and nocturnal gasping. The clinical team records the following finding: submucous cleft palate is relative — a partial adenotonsillectomy may be performed. Which concept best explains this observation?

A. Positional and other

B. Partial tonsillectomy

C. Contraindication

D. Medication

E. Indications

CORRECT ANSWER C

This finding most directly describes Contraindication: submucous cleft palate is relative — a partial adenotonsillectomy may be performed. Distractor distinctions: A refers instead to AHI is increased supine and lowest prone in children — but position is hard to maintain overnight; B refers instead to leaves tissue covering the tonsillar fossa musculature; D refers instead to Intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe; E refers instead to children who do not respond adequately to surgery, or in whom surgery is contraindicated — which can include patient and fami.

REFERENCE Integrated notebook, page 159, §8.7

QUESTION 0821 · §8.6 · Applied clinical SBA

Parents request advice for a child with large tonsils and nocturnal gasping. Which of the following statements about palate and arches is most accurate?

A. Retrognathia or micrognathia can contribute to a narrower pharyngeal airway.

B. High-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing.

C. Children with SDB may show inattention, irritability or hyperactivity instead of prominent sleepiness.

D. Tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development.

E. Paediatric evaluation should elicit chronic allergies, asthma, recurrent ENT infections and a family history of OSA.

CORRECT ANSWER B

High-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing. The other options describe different mechanisms, investigations or findings rather than palate and arches. Consult Integrated notebook, page 158, §8.6 for the full context.

REFERENCE Integrated notebook, page 158, §8.6

QUESTION 0822 · §8.8 · Focused knowledge SBA

Parents report that their 6-year-old snores and struggles with classroom attention. Which of the following statements about treatment is most accurate?

A. In children, an obstructive AHI of at least 1 event/h is abnormal; thresholds are lower than in adults.

B. Adenotonsillectomy is commonly first-line for paediatric OSA with adenotonsillar hypertrophy.

C. Children with OSA may have snoring, behavioural changes and attention problems rather than obvious daytime sleepiness.

D. Paediatric sleep apnoea requires clinical assessment and physician-directed sleep testing.

E. Adenotonsillar enlargement is a major cause of paediatric obstructive sleep apnoea.

CORRECT ANSWER B

Adenotonsillectomy is commonly first-line for paediatric OSA with adenotonsillar hypertrophy. Additional revision detail: Children: surgical removal of enlarged tonsils and adenoids first; weight management; PAP where surgery is ineffective; behaviour-focused therapy for sleep hygiene; orthodontics in selected cases. The other options describe different mechanisms, investigations or findings rather than treatment. Consult Integrated notebook, page 160, §8.8 for the full context.

REFERENCE Integrated notebook, page 160, §8.8

QUESTION 0823 · §8.7 · Applied clinical SBA

Parents report that their 6-year-old snores and struggles with classroom attention. Which of the following statements about contraindication is most accurate?

A. Partial tonsillectomy includes leaves tissue covering the tonsillar fossa musculature.

B. Obesity and higher preoperative AHI increase the risk of residual paediatric OSA after surgery.

C. Medication includes intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe.

D. AHI is increased supine and lowest prone in children — but position is hard to maintain overnight.

E. Submucous cleft palate is relative — a partial adenotonsillectomy may be performed.

CORRECT ANSWER E

Submucous cleft palate is relative — a partial adenotonsillectomy may be performed. The other options describe different mechanisms, investigations or findings rather than contraindication. Consult Integrated notebook, page 159, §8.7 for the full context.

REFERENCE Integrated notebook, page 159, §8.7

QUESTION 0824 · §8.9 · Clinical interpretation SBA

A child with Down syndrome attends a routine dental appointment and has no reported sleepiness. The clinical team records the following finding: macroglossia, midface hypoplasia, overweight, poor muscle tone. Moderate–severe disease is more prevalent at younger ages. Which concept best explains this observation?

A. Medical conditions

B. OSA is highly prevalent

C. Predisposing factors

D. Genetic syndromes

E. Clinical symptoms are unreliable

CORRECT ANSWER C

This finding most directly describes Predisposing factors: macroglossia, midface hypoplasia, overweight, poor muscle tone. Moderate–severe disease is more prevalent at younger ages. Distractor distinctions: A refers instead to obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction, inflammation or decreased; B refers instead to Prevalence by AHI threshold: >1 in 69%; >1.5 in 76%; >2 in 75%; >5 (moderate–severe) in 50%; >10 in 34%. Compare 1–5% in the; D refers instead to Down, Prader–Willi, Crouzon and cleft lip and palate often involve craniofacial abnormalities — midfacial hypoplasia, macrogl; E refers instead to only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of child.

REFERENCE Integrated notebook, page 161, §8.9

QUESTION 0825 · §8.8 · Applied clinical SBA

A child with Down syndrome attends a routine dental appointment and has no reported sleepiness. Which of the following statements about treatment is most accurate?

A. Paediatric sleep apnoea requires clinical assessment and physician-directed sleep testing.

B. Adenotonsillectomy is commonly first-line for paediatric OSA with adenotonsillar hypertrophy.

C. Children with OSA may have snoring, behavioural changes and attention problems rather than obvious daytime sleepiness.

D. In children, an obstructive AHI of at least 1 event/h is abnormal; thresholds are lower than in adults.

E. Adenotonsillar enlargement is a major cause of paediatric obstructive sleep apnoea.

CORRECT ANSWER B

Adenotonsillectomy is commonly first-line for paediatric OSA with adenotonsillar hypertrophy. Additional revision detail: Children: surgical removal of enlarged tonsils and adenoids first; weight management; PAP where surgery is ineffective; behaviour-focused therapy for sleep hygiene; orthodontics in selected cases. The other options describe different mechanisms, investigations or findings rather than treatment. Consult Integrated notebook, page 160, §8.8 for the full context.

REFERENCE Integrated notebook, page 160, §8.8

QUESTION 0826 · §8.10 · Clinical interpretation SBA

An 8-year-old with a narrow palate and restless sleep is seen for an orthodontic assessment. The clinical team records the following finding: 2 mild / 4 moderate / 6 severe; paediatric ESS ≥10; tonsillar obstruction >50% warrants evaluation. Which concept best explains this observation?

A. Adenotonsillectomy failure risk

B. PSQ threshold >0.33 (>8 of 22 items); FAIREST-6

C. Indications for PSG

D. Paediatric AHI

E. Sleep physician or ENT

CORRECT ANSWER B

This finding most directly describes PSQ threshold >0.33 (>8 of 22 items); FAIREST-6: 2 mild / 4 moderate / 6 severe; paediatric ESS ≥10; tonsillar obstruction >50% warrants evaluation. Distractor distinctions: A refers instead to age <3 or ≥7, obesity, high pre-op AHI, nasal/craniofacial abnormality, family history, asthma. Overnight admission if SpO2 n; C refers instead to clinical symptoms of SDB, especially before adenotonsillectomy in children under 2 years, in obesity, in craniofacial or neur; D refers instead to normal <1 · mild 1–5 · moderate 5–10 · severe ≥10. Obstructive AHI ≥1 is abnormal; E refers instead to for children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging. F.

REFERENCE Integrated notebook, page 180, §8.10

QUESTION 0827 · §8.9 · Applied clinical SBA

An 8-year-old with a narrow palate and restless sleep is seen for an orthodontic assessment. Which of the following statements about predisposing factors is most accurate?

A. Medical conditions includes obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction.

B. Only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of children with OSA had no symptoms.

C. Down syndrome is associated with a high prevalence of paediatric OSA, even without reported symptoms.

D. Macroglossia, midface hypoplasia, overweight, poor muscle tone. Moderate–severe disease is more prevalent at younger ages.

E. Down, Prader–Willi and craniofacial syndromes increase the likelihood of paediatric OSA.

CORRECT ANSWER D

Macroglossia, midface hypoplasia, overweight, poor muscle tone. Moderate–severe disease is more prevalent at younger ages. The other options describe different mechanisms, investigations or findings rather than predisposing factors. Consult Integrated notebook, page 161, §8.9 for the full context.

REFERENCE Integrated notebook, page 161, §8.9

QUESTION 0828 · §8.5 · Clinical interpretation SBA

A child with Down syndrome attends a routine dental appointment and has no reported sleepiness. The clinical team records the following finding: early starts conflict with the delayed rhythm, producing chronic insufficient sleep. Which concept best explains this observation?

A. Mental health problems

B. Weekend catch-up

C. School start times

D. Circadian phase delay

E. Electronic media

CORRECT ANSWER C

This finding most directly describes School start times: early starts conflict with the delayed rhythm, producing chronic insufficient sleep. Distractor distinctions: A refers instead to anxiety, stress sleep, leading to residual daytime sleepiness; B refers instead to Going to bed later and waking later at weekends reflects circadian biology and is also a response to insufficient weekday sle; D refers instead to Early adolescent development and pubertal onset are associated with a more evening-type circadian phase preference — the pref; E refers instead to four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night.

REFERENCE Integrated notebook, page 157, §8.5

QUESTION 0829 · §8.10 · Applied clinical SBA

A 12-year-old with behavioural changes and habitual snoring is referred for assessment. Which of the following statements about psq threshold >0.33 (>8 of 22 items); fairest-6 is most accurate?

A. Recommended sleep duration varies with age, from 12–16 h/day in infants aged 4–12 months to 8–10 h in teenagers.

B. Residual paediatric OSA after adenotonsillectomy is more likely with obesity or high preoperative AHI.

C. In children, obstructive AHI of 1/h or more is abnormal, with severe disease generally at 10/h or more.

D. For children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging.

E. PSQ threshold >0.33 (>8 of 22 items); FAIREST-6: 2 mild / 4 moderate / 6 severe; paediatric ESS ≥10; tonsillar obstruction >50% warrants evaluation.

CORRECT ANSWER E

PSQ threshold >0.33 (>8 of 22 items); FAIREST-6: 2 mild / 4 moderate / 6 severe; paediatric ESS ≥10; tonsillar obstruction >50% warrants evaluation. Additional revision detail: 2 mild / 4 moderate / 6 severe; paediatric ESS ≥10; tonsillar obstruction >50% warrants evaluation. The other options describe different mechanisms, investigations or findings rather than psq threshold >0.33 (>8 of 22 items); fairest-6. Consult Integrated notebook, page 180, §8.10 for the full context.

REFERENCE Integrated notebook, page 180, §8.10

QUESTION 0830 · §8.6 · Clinical interpretation SBA

An 8-year-old with a narrow palate and restless sleep is seen for an orthodontic assessment. The clinical team records the following finding: tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development. Which concept best explains this observation?

A. Tongue posture and function

B. Medical, dental and family history

C. Jaw size and position

D. Palate and arches

E. Tonsils and adenoids

CORRECT ANSWER A

This finding most directly describes Tongue posture and function: tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development. Distractor distinctions: B refers instead to chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematur; C refers instead to retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillar; D refers instead to high-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing; E refers instead to use the modified Mallampati, Friedman tongue classification, or the Brodsky scale. Hypertrophy obstructing >50% of the airway.

REFERENCE Integrated notebook, page 158, §8.6

QUESTION 0831 · §8.5 · Applied clinical SBA

An 8-year-old with a narrow palate and restless sleep is seen for an orthodontic assessment. Which of the following statements about school start times is most accurate?

A. Sleep–wake homeostasis includes sleep pressure builds more slowly with time awake in adolescents.

B. Evening screen use can delay bedtime and interfere with adolescent sleep regulation.

C. Early school starts can worsen insufficient sleep in adolescents with delayed circadian timing.

D. Anxiety and stress may disturb sleep independently of obstructive respiratory events.

E. Other causes includes chronic medical illness; anxiety and stress; prescribed psychotropics.

CORRECT ANSWER C

Early school starts can worsen insufficient sleep in adolescents with delayed circadian timing. Additional revision detail: early starts conflict with the delayed rhythm, producing chronic insufficient sleep. The other options describe different mechanisms, investigations or findings rather than school start times. Consult Integrated notebook, page 157, §8.5 for the full context.

REFERENCE Integrated notebook, page 157, §8.5

QUESTION 0832 · §8.7 · Clinical interpretation SBA

A 12-year-old with behavioural changes and habitual snoring is referred for assessment. The clinical team records the following finding: leaves tissue covering the tonsillar fossa musculature. Which concept best explains this observation?

A. Risk factors for failure

B. Indications

C. Contraindication

D. Positional and other

E. Partial tonsillectomy

CORRECT ANSWER E

This finding most directly describes Partial tonsillectomy: leaves tissue covering the tonsillar fossa musculature. Distractor distinctions: A refers instead to obesity; high pre-operative AHI; age <3 or ≥7; nasal abnormality or craniofacial disproportion; family history of OSA; asthma; B refers instead to children who do not respond adequately to surgery, or in whom surgery is contraindicated — which can include patient and fami; C refers instead to submucous cleft palate is relative — a partial adenotonsillectomy may be performed; D refers instead to AHI is increased supine and lowest prone in children — but position is hard to maintain overnight.

REFERENCE Integrated notebook, page 159, §8.7

QUESTION 0833 · §8.6 · Applied clinical SBA

A 12-year-old with behavioural changes and habitual snoring is referred for assessment. Which of the following statements about tongue posture and function is most accurate?

A. Retrognathia or micrognathia can contribute to a narrower pharyngeal airway.

B. Paediatric evaluation should elicit chronic allergies, asthma, recurrent ENT infections and a family history of OSA.

C. Children with SDB may show inattention, irritability or hyperactivity instead of prominent sleepiness.

D. Tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development.

E. High-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing.

CORRECT ANSWER D

Tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development. The other options describe different mechanisms, investigations or findings rather than tongue posture and function. Consult Integrated notebook, page 158, §8.6 for the full context.

REFERENCE Integrated notebook, page 158, §8.6

QUESTION 0834 · §8.10 · Clinical interpretation SBA

A 12-year-old with behavioural changes and habitual snoring is referred for assessment. The clinical team records the following finding: clinical symptoms of SDB, especially before adenotonsillectomy in children under 2 years, in obesity, in craniofacial or neuromuscular disorders, in Down syndrome, or where symptoms persist after surgery. Which concept best explains this observation?

A. Sleep duration

B. Adenotonsillectomy failure risk

C. Sleep physician or ENT

D. Paediatric AHI

E. Indications for PSG

CORRECT ANSWER E

This finding most directly describes Indications for PSG: clinical symptoms of SDB, especially before adenotonsillectomy in children under 2 years, in obesity, in craniofacial or neuromuscular disorders, in Down syndrome, or where symptoms persist after surgery. Distractor distinctions: A refers instead to 4–12 mo 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h. <8 h in adolescence → ~3× suicide-attempt r; B refers instead to age <3 or ≥7, obesity, high pre-op AHI, nasal/craniofacial abnormality, family history, asthma. Overnight admission if SpO2 n; C refers instead to for children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging. F; D refers instead to normal <1 · mild 1–5 · moderate 5–10 · severe ≥10. Obstructive AHI ≥1 is abnormal.

REFERENCE Integrated notebook, page 180, §8.10

QUESTION 0835 · §8.7 · Applied clinical SBA

Parents request advice for a child with large tonsils and nocturnal gasping. Which of the following statements about partial tonsillectomy is most accurate?

A. Submucous cleft palate is relative — a partial adenotonsillectomy may be performed.

B. Obesity and higher preoperative AHI increase the risk of residual paediatric OSA after surgery.

C. AHI is increased supine and lowest prone in children — but position is hard to maintain overnight.

D. Medication includes intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe.

E. Partial tonsillectomy includes leaves tissue covering the tonsillar fossa musculature.

CORRECT ANSWER E

Partial tonsillectomy includes leaves tissue covering the tonsillar fossa musculature. Additional revision detail: leaves tissue covering the tonsillar fossa musculature. The other options describe different mechanisms, investigations or findings rather than partial tonsillectomy. Consult Integrated notebook, page 159, §8.7 for the full context.

REFERENCE Integrated notebook, page 159, §8.7

QUESTION 0836 · §8.5 · Clinical interpretation SBA

An 8-year-old with a narrow palate and restless sleep is seen for an orthodontic assessment. The clinical team records the following finding: chronic medical illness; anxiety and stress; prescribed psychotropics. Which concept best explains this observation?

A. Electronic media

B. Weekend catch-up

C. Other causes

D. School start times

E. Circadian phase delay

CORRECT ANSWER C

This finding most directly describes Other causes: chronic medical illness; anxiety and stress; prescribed psychotropics. Distractor distinctions: A refers instead to four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night; B refers instead to Going to bed later and waking later at weekends reflects circadian biology and is also a response to insufficient weekday sle; D refers instead to early starts conflict with the delayed rhythm, producing chronic insufficient sleep; E refers instead to Early adolescent development and pubertal onset are associated with a more evening-type circadian phase preference — the pref.

REFERENCE Integrated notebook, page 157, §8.5

QUESTION 0837 · §8.10 · Applied clinical SBA

Parents request advice for a child with large tonsils and nocturnal gasping. Which of the following statements about indications for PSG is most accurate?

A. Recommended sleep duration varies with age, from 12–16 h/day in infants aged 4–12 months to 8–10 h in teenagers.

B. Paediatric PSG is particularly indicated when significant comorbidities or uncertain clinical findings complicate suspected OSA.

C. PSQ threshold >0.33 (>8 of 22 items); FAIREST-6: 2 mild / 4 moderate / 6 severe; paediatric ESS ≥10; tonsillar obstruction >50% warrants evaluation.

D. Residual paediatric OSA after adenotonsillectomy is more likely with obesity or high preoperative AHI.

E. For children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging.

CORRECT ANSWER B

Paediatric PSG is particularly indicated when significant comorbidities or uncertain clinical findings complicate suspected OSA. Additional revision detail: clinical symptoms of SDB, especially before adenotonsillectomy in children under 2 years, in obesity, in craniofacial or neuromuscular disorders, in Down syndrome, or where symptoms persist after surgery. The other options describe different mechanisms, investigations or findings rather than indications for psg. Consult Integrated notebook, page 180, §8.10 for the full context.

REFERENCE Integrated notebook, page 180, §8.10

QUESTION 0838 · §8.7 · Clinical interpretation SBA

Parents report that their 6-year-old snores and struggles with classroom attention. The clinical team records the following finding: Intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe. Which concept best explains this observation?

A. Contraindication

B. Indications

C. Partial tonsillectomy

D. Medication

E. Positional and other

CORRECT ANSWER D

This finding most directly describes Medication: Intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe. Distractor distinctions: A refers instead to submucous cleft palate is relative — a partial adenotonsillectomy may be performed; B refers instead to children who do not respond adequately to surgery, or in whom surgery is contraindicated — which can include patient and fami; C refers instead to leaves tissue covering the tonsillar fossa musculature; E refers instead to AHI is increased supine and lowest prone in children — but position is hard to maintain overnight.

REFERENCE Integrated notebook, page 160, §8.7

QUESTION 0839 · §8.5 · Applied clinical SBA

A 12-year-old with behavioural changes and habitual snoring is referred for assessment. Which of the following statements about other causes is most accurate?

A. Sleep–wake homeostasis includes sleep pressure builds more slowly with time awake in adolescents.

B. Evening screen use can delay bedtime and interfere with adolescent sleep regulation.

C. Anxiety and stress may disturb sleep independently of obstructive respiratory events.

D. Early school starts can worsen insufficient sleep in adolescents with delayed circadian timing.

E. Other causes includes chronic medical illness; anxiety and stress; prescribed psychotropics.

CORRECT ANSWER E

Other causes includes chronic medical illness; anxiety and stress; prescribed psychotropics. Additional revision detail: chronic medical illness; anxiety and stress; prescribed psychotropics. The other options describe different mechanisms, investigations or findings rather than other causes. Consult Integrated notebook, page 157, §8.5 for the full context.

REFERENCE Integrated notebook, page 157, §8.5

QUESTION 0840 · §8.7 · Applied clinical SBA

Parents request advice for a child with large tonsils and nocturnal gasping. Which of the following statements about medication is most accurate?

A. Submucous cleft palate is relative — a partial adenotonsillectomy may be performed.

B. AHI is increased supine and lowest prone in children — but position is hard to maintain overnight.

C. Medication includes intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe.

D. Obesity and higher preoperative AHI increase the risk of residual paediatric OSA after surgery.

E. Partial tonsillectomy includes leaves tissue covering the tonsillar fossa musculature.

CORRECT ANSWER C

Medication includes intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe. Additional revision detail: Intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe. The other options describe different mechanisms, investigations or findings rather than medication. Consult Integrated notebook, page 160, §8.7 for the full context.

REFERENCE Integrated notebook, page 160, §8.7

09 Best-practice, documentation and ethical responsibilities

QUESTION 0841 · §9.1 · Clinical decision-making SBA

A patient asks whether a dentist can declare 'no OSA' after a home oximetry test. What is the best scope-of-practice response?

A. The device manufacturer establishes severity

B. The patient may diagnose based on snoring

C. A dentist may diagnose from oximetry alone

D. The diagnosis or exclusion of OSA belongs to an appropriately qualified physician

E. A normal ESS rules out OSA

CORRECT ANSWER D

The notebook repeatedly distinguishes dental screening and titration from physician-led diagnosis and formal objective efficacy verification.

REFERENCE Integrated notebook, Topic 9, §9.1

QUESTION 0842 · §9.1 · Clinical decision-making SBA

An adult has an elevated STOP-BANG score and wants a dentist to certify a diagnosis of OSA. Which is the best response?

A. Provide an OSA diagnosis because STOP-BANG is highly sensitive

B. Explain that screening identifies risk and refer for appropriate physician-led diagnostic assessment

C. Supply a custom MAD as diagnostic confirmation

D. Use BMI alone to determine AHI

E. Record snoring as equivalent to severe OSA

CORRECT ANSWER B

STOP-BANG estimates risk but does not establish OSA. The dentist screens and refers while the physician diagnoses and verifies effectiveness according to the notebook's shared-care model. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 9, §9.1

QUESTION 0843 · §9.2 · Clinical decision-making SBA

A patient disputes a course of OAT because no baseline dental records were stored. Which omission most weakens the clinic's ability to assess treatment-related bite change?

A. The absence of serial occlusal records and baseline models or scans

B. Missing supplier marketing flyers

C. Not recording the patient's preferred clinic music

D. No brand photograph

E. The patient's email font preference

CORRECT ANSWER A

Baseline examination, occlusal observations and study models/scans permit meaningful comparison and safe monitoring of long-term appliance side effects. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 9, §9.2

QUESTION 0844 · §9.3 · Clinical decision-making SBA

A dentist practising across two countries is asked to use a particular reimbursement item number for OAT. What is the safest response?

A. Use the same code worldwide

B. Confirm the current billing rules in the relevant jurisdiction before claiming

C. Treat a clinical AHI threshold as a billing code

D. Use an old code without verifying eligibility

E. Assume private insurers use identical rules

CORRECT ANSWER B

Administrative and reimbursement pathways are jurisdiction-specific and change; clinical principles do not validate a particular claim code. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 9, §9.3

QUESTION 0845 · §9.4 · Clinical decision-making SBA

A clinic is offered a financial incentive for exclusively fitting one oral appliance brand, regardless of individual patient needs. Which ethical principle is most directly at risk?

A. Justice in reimbursement only

B. Appropriate management of conflicts of interest and professional integrity

C. Maximising vertical opening

D. Interpretation of hypopnoea rules

E. Polysomnographic sleep staging

CORRECT ANSWER B

Commercial interests must not overrule patient-centred choice or evidence-based device selection. Transparency and professional independence are essential. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 9, §9.4

QUESTION 0846 · §9.2 · Clinical decision-making SBA

An oral appliance was fitted without recording pretreatment overjet, overbite or models. What is the key risk?

A. The clinician cannot reliably quantify later appliance-related dental change

B. Occlusal records are irrelevant once AHI improves

C. A signed bill substitutes for baseline measurements

D. Later changes become impossible

E. The physician alone is responsible for dental records

CORRECT ANSWER A

Baseline records are essential for monitoring predictable occlusal effects and resolving medicolegal disputes.

REFERENCE Integrated notebook, Topic 9, §9.2

QUESTION 0847 · §9.2 · Applied clinical SBA

A clinic considers an industry incentive linked to its choice of oral appliance. Which of the following statements about treatment planning is most accurate?

A. Interprofessional records should document coordinated decisions between the dentist and sleep physician.

B. Longitudinal notes should record symptoms, adherence, adjustments and patient response.

C. The clinical treatment plan should document appliance selection, titration, education and shared decisions.

D. Appliance delivery records should include home-care instructions, patient acknowledgement and warranty information.

E. Baseline sleep-study records should retain respiratory severity, sleep architecture and arousal measures.

CORRECT ANSWER C

The clinical treatment plan should document appliance selection, titration, education and shared decisions. Additional revision detail: comprehensive plans setting out rationale, appliance choice, adjustments, titration and patient education, with any shared decision-making discussions about options and intolerances recorded. The other options describe different mechanisms, investigations or findings rather than treatment planning. Consult Integrated notebook, page 185, §9.2 for the full context.

REFERENCE Integrated notebook, page 185, §9.2

QUESTION 0848 · §9.2 · Clinical interpretation SBA

A dental clinic is auditing its written records for an oral appliance therapy service. The clinical team records the following finding: comprehensive plans setting out rationale, appliance choice, adjustments, titration and patient education, with any shared decision-making discussions about options and intolerances recorded. Which concept best explains this observation?

A. Delivery documentation

B. Treatment planning

C. Informed consent

D. Ongoing clinical notes

E. Baseline sleep study data

CORRECT ANSWER B

This finding most directly describes Treatment planning: comprehensive plans setting out rationale, appliance choice, adjustments, titration and patient education, with any shared decision-making discussions about options and intolerances recorded. Distractor distinctions: A refers instead to written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the pati; C refers instead to complete signed forms explaining purpose, risks, benefits, alternatives and patient rights; D refers instead to patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians; E refers instead to the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment re.

REFERENCE Integrated notebook, page 185, §9.2

QUESTION 0849 · §9.2 · Clinical decision-making SBA

A dentist changes advancement settings but does not inform the sleep physician. Which best-practice principle is breached?

A. Only billing codes need updating

B. A dental assistant may provide the sleep diagnosis

C. Patient preferences are not relevant

D. Treatment never requires physician input after baseline

E. Documented interdisciplinary communication and shared care

CORRECT ANSWER E

OAT care requires communication of fitting, adjustment, response and referrals, with physician participation in outcome verification.

REFERENCE Integrated notebook, Topic 9, §9.2

QUESTION 0850 · §9.3 · Applied clinical SBA

A patient asks a dentist to provide a diagnosis on the basis of an overnight oximetry report. Which of the following statements about consult authoritative sources is most accurate?

A. Public, private and veterans' health schemes may use different billing codes and eligibility rules.

B. Current reimbursement codes and eligibility requirements should be checked against the relevant official payer.

C. Claims across different insurers must separate unrelated conditions and meet each payer's requirements.

D. The clinician should identify codes specific to the service and the insurer; maintain clear, separate consultation notes.

E. The clinician should use item codes and descriptions specific to the service provided and to the patient's in- surer.

CORRECT ANSWER B

Current reimbursement codes and eligibility requirements should be checked against the relevant official payer. Additional revision detail: the relevant government health department or insurer website — for current guidance rather than relying on memory or on colleagues. The other options describe different mechanisms, investigations or findings rather than consult authoritative sources. Consult Integrated notebook, page 192, §9.3 for the full context.

REFERENCE Integrated notebook, page 192, §9.3

QUESTION 0851 · §9.3 · Clinical interpretation SBA

A clinic considers an industry incentive linked to its choice of oral appliance. The clinical team records the following finding: the relevant government health department or insurer website — for current guidance rather than relying on memory or on colleagues. Which concept best explains this observation?

A. Identify the correct codes

B. Ethical dimension

C. Separate conditions

D. Consult authoritative sources

E. Use DVA-specific codes

CORRECT ANSWER D

This finding most directly describes Consult authoritative sources: the relevant government health department or insurer website — for current guidance rather than relying on memory or on colleagues. Distractor distinctions: A refers instead to Use item codes and descriptions specific to the service provided and to the patient's in- surer; B refers instead to billing and coding fall explicitly within the professionalism competencies of a qualified dentist. Appliance selection must n; C refers instead to For veterans with both DVA and other insurance, clearly separate discussion and documentation of unrelated conditions when bi; E refers instead to Veterans' affairs schemes maintain their own set of codes, which can differ from those used for the general public health ins.

REFERENCE Integrated notebook, page 192, §9.3

QUESTION 0852 · §9.3 · Clinical decision-making SBA

A practice sees patients funded through a veterans' scheme and proposes using private insurance codes interchangeably. What is appropriate?

A. Check the current payer-specific coding rules and document the service accurately

B. Use any code that increases reimbursement

C. Bill for sleep studies the dentist did not perform

D. Use unsigned clinical notes

E. Assume all jurisdictions use the same codes

CORRECT ANSWER A

Billing codes, including DVA arrangements, vary by jurisdiction and payer; ethical coding requires current authoritative guidance.

REFERENCE Integrated notebook, Topic 9, §9.3

QUESTION 0853 · §9.4 · Applied clinical SBA

A dentist and sleep physician are defining responsibilities for an OAT referral pathway. Which of the following statements about pm minimum is most accurate?

A. Beneficence includes promote well-being and provide high-quality care improving oral and overall health.

B. The clinician should use evidence-based practices, stay updated, and provide treatment only within established competence.

C. Pm minimum includes airflow + respiratory effort + blood oxygenation.

D. The clinician should treat only within your qualifications and expertise; deliver care within the qualified dentist's competence.

E. Professional conduct includes accurate billing, accountability and competent team-based care.

CORRECT ANSWER C

Pm minimum includes airflow + respiratory effort + blood oxygenation. Additional revision detail: airflow + respiratory effort + blood oxygenation. The other options describe different mechanisms, investigations or findings rather than pm minimum. Consult Integrated notebook, page 187, §9.4 for the full context.

REFERENCE Integrated notebook, page 187, §9.4

QUESTION 0854 · §9.4 · Clinical interpretation SBA

A patient asks a dentist to provide a diagnosis on the basis of an overnight oximetry report. The clinical team records the following finding: airflow + respiratory effort + blood oxygenation. Which concept best explains this observation?

A. PM minimum

B. Scope of competence

C. Professionalism

D. Autonomy

E. Beneficence

CORRECT ANSWER A

This finding most directly describes PM minimum: airflow + respiratory effort + blood oxygenation. Distractor distinctions: B refers instead to treat only within your qualifications and expertise; deliver care within the qualified dentist's competence; C refers instead to ethical principles including billing and coding; accountability and team training with adherence to applicable laws; D refers instead to patients have the right to make informed decisions; in practice, genuine informed consent requiring clear understanding of pu; E refers instead to promote well-being and provide high-quality care improving oral and overall health.

REFERENCE Integrated notebook, page 187, §9.4

QUESTION 0855 · §9.4 · Clinical decision-making SBA

A dentist with a commercial relationship to an appliance supplier recommends it without comparison. What ethical principle is at stake?

A. Patients need not know alternatives

B. The choice is ethical if most profitable

C. Commercial sponsorship proves superior outcomes

D. Conflict-of-interest management and evidence-based patient choice

E. Competence is irrelevant when devices are custom

CORRECT ANSWER D

Device selection must not be dictated by financial arrangements; integrity and patient-centred informed choice are required.

REFERENCE Integrated notebook, Topic 9, §9.4

QUESTION 0856 · §9.1 · Applied clinical SBA

A dentist and sleep physician are defining responsibilities for an OAT referral pathway. Which of the following statements about special indications is most accurate?

A. Relevant findings include but expensive and technically complex.

B. Where in-laboratory PSG is impossible because of immobility, safety or critical illness.

C. Indication may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA.

D. Attended laboratory polysomnography provides detailed, reliable sleep and breathing measurements.

E. Less expensive and quicker, and can be as accurate as PSG in selected populations.

CORRECT ANSWER B

Where in-laboratory PSG is impossible because of immobility, safety or critical illness. The other options describe different mechanisms, investigations or findings rather than special indications. Consult Integrated notebook, page 185, §9.1 for the full context.

REFERENCE Integrated notebook, page 185, §9.1

QUESTION 0857 · §9.1 · Clinical interpretation SBA

A patient asks a dentist to provide a diagnosis on the basis of an overnight oximetry report. The clinical team records the following finding: where in-laboratory PSG is impossible because of immobility, safety or critical illness. Which concept best explains this observation?

A. Supervision

B. Portable monitoring

C. In-laboratory PSG

D. Special indications

E. Indication

CORRECT ANSWER D

This finding most directly describes Special indications: where in-laboratory PSG is impossible because of immobility, safety or critical illness. Distractor distinctions: A refers instead to performed only in conjunction with a comprehensive sleep evaluation, by a board-certified sleep specialist or an individual m; B refers instead to less expensive and quicker, and can be as accurate as PSG in selected populations; C refers instead to accurate with a low failure rate, attended by technical staff; expensive and technically complex. Also the standard for narco; E refers instead to may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA.

REFERENCE Integrated notebook, page 185, §9.1

QUESTION 0858 · §9.4 · Clinical decision-making SBA

A patient declines OAT after learning about permanent bite changes. What should the clinician do?

A. Dismiss bite changes as impossible

B. Apply the device without consent

C. Treat refusal as a dental emergency

D. Conceal available options

E. Respect the informed refusal, document it and discuss alternatives

CORRECT ANSWER E

Autonomy includes the right to refuse treatment after understanding benefits, burdens and alternatives.

REFERENCE Integrated notebook, Topic 9, §9.4

QUESTION 0859 · §9.2 · Applied clinical SBA

An adult arrives with an elevated STOP-BANG score but no physician diagnosis of OSA. Which of the following statements about informed consent is most accurate?

A. Longitudinal notes should record symptoms, adherence, adjustments and patient response.

B. The clinical treatment plan should document appliance selection, titration, education and shared decisions.

C. Appliance delivery records should include home-care instructions, patient acknowledgement and warranty information.

D. Baseline sleep-study records should retain respiratory severity, sleep architecture and arousal measures.

E. Complete signed forms explaining purpose, risks, benefits, alternatives and patient rights.

CORRECT ANSWER E

Complete signed forms explaining purpose, risks, benefits, alternatives and patient rights. The other options describe different mechanisms, investigations or findings rather than informed consent. Consult Integrated notebook, page 185, §9.2 for the full context.

REFERENCE Integrated notebook, page 185, §9.2

QUESTION 0860 · §9.2 · Clinical interpretation SBA

A dentist and sleep physician are defining responsibilities for an OAT referral pathway. The clinical team records the following finding: complete signed forms explaining purpose, risks, benefits, alternatives and patient rights. Which concept best explains this observation?

A. Baseline sleep study data

B. Delivery documentation

C. Treatment planning

D. Ongoing clinical notes

E. Informed consent

CORRECT ANSWER E

This finding most directly describes Informed consent: complete signed forms explaining purpose, risks, benefits, alternatives and patient rights. Distractor distinctions: A refers instead to the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment re; B refers instead to written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the pati; C refers instead to comprehensive plans setting out rationale, appliance choice, adjustments, titration and patient education, with any shared de; D refers instead to patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians.

REFERENCE Integrated notebook, page 185, §9.2

QUESTION 0861 · §9.1 · Clinical decision-making SBA

A clinician uses an HSAT in a patient with suspected narcolepsy and significant heart failure. Why is this problematic?

A. A dentist may replace PSG with a dental scan

B. HSAT always records sleep stages

C. A positive snoring questionnaire confirms narcolepsy

D. Cardiac disease improves device accuracy

E. Portable monitoring may miss other disorders and is inappropriate in important comorbidity

CORRECT ANSWER E

The notebook reserves portable testing for appropriately selected high-risk uncomplicated patients; suspected complex disorders favour PSG.

REFERENCE Integrated notebook, Topic 9, §9.1

QUESTION 0862 · §9.3 · Applied clinical SBA

A dental clinic is auditing its written records for an oral appliance therapy service. Which of the following statements about separate conditions is most accurate?

A. The clinician should identify codes specific to the service and the insurer; maintain clear, separate consultation notes.

B. Claims across different insurers must separate unrelated conditions and meet each payer’s requirements.

C. Current reimbursement codes and eligibility requirements should be checked against the relevant official payer.

D. Ethical dimension includes billing and coding fall explicitly within the professionalism competencies of a qualified dentist.

E. Public, private and veterans’ health schemes may use different billing codes and eligibility rules.

CORRECT ANSWER B

Claims across different insurers must separate unrelated conditions and meet each payer’s requirements. Additional revision detail: For veterans with both DVA and other insurance, clearly separate discussion and documentation of unrelated conditions when billing multiple insurers. Best practice for both. The other options describe different mechanisms, investigations or findings rather than separate conditions. Consult Integrated notebook, page 192, §9.3 for the full context.

REFERENCE Integrated notebook, page 192, §9.3

QUESTION 0863 · §9.3 · Clinical interpretation SBA

An adult arrives with an elevated STOP-BANG score but no physician diagnosis of OSA. The clinical team records the following finding: For veterans with both DVA and other insurance, clearly separate discussion and documentation of unrelated conditions when billing multiple insurers. Best practice for both. Which concept best explains this observation?

A. Identify the correct codes

B. Separate conditions

C. Use DVA-specific codes

D. Private billing

E. Ethical dimension

CORRECT ANSWER B

This finding most directly describes Separate conditions: For veterans with both DVA and other insurance, clearly separate discussion and documentation of unrelated conditions when billing multiple insurers. Best practice for both. Distractor distinctions: A refers instead to Use item codes and descriptions specific to the service provided and to the patient's in- surer; C refers instead to Veterans' affairs schemes maintain their own set of codes, which can differ from those used for the general public health ins; D refers instead to identify codes specific to the service and the insurer; maintain clear, separate consultation notes differentiating insurers; E refers instead to billing and coding fall explicitly within the professionalism competencies of a qualified dentist. Appliance selection must n.

REFERENCE Integrated notebook, page 192, §9.3

QUESTION 0864 · §9.2 · Clinical decision-making SBA

The patient disputes whether appliance-related tooth movement was discussed. Which record most directly supports a proper process?

A. Only an SMS appointment reminder

B. A laboratory invoice alone

C. The appliance's colour

D. Dated signed consent plus documented discussion, baseline findings and follow-up

E. A verbal claim without notes

CORRECT ANSWER D

A signed form alone is not the entire consent process; contemporaneous records of discussion and baseline dental status are essential.

REFERENCE Integrated notebook, Topic 9, §9.2

QUESTION 0865 · §9.4 · Applied clinical SBA

A clinic considers an industry incentive linked to its choice of oral appliance. Which of the following statements about beneficence is most accurate?

A. The clinician should treat only within your qualifications and expertise; deliver care within the qualified dentist's competence.

B. Beneficence includes promote well-being and provide high-quality care improving oral and overall health.

C. Pm minimum includes airflow + respiratory effort + blood oxygenation.

D. The clinician should use evidence-based practices, stay updated, and provide treatment only within established competence.

E. Professional conduct includes accurate billing, accountability and competent team-based care.

CORRECT ANSWER B

Beneficence includes promote well-being and provide high-quality care improving oral and overall health. Additional revision detail: promote well-being and provide high-quality care improving oral and overall health. The other options describe different mechanisms, investigations or findings rather than beneficence. Consult Integrated notebook, page 186, §9.4 for the full context.

REFERENCE Integrated notebook, page 186, §9.4

QUESTION 0866 · §9.4 · Clinical interpretation SBA

A dental clinic is auditing its written records for an oral appliance therapy service. The clinical team records the following finding: promote well-being and provide high-quality care improving oral and overall health. Which concept best explains this observation?

A. Beneficence

B. Scope of competence

C. Autonomy

D. Professionalism

E. PM minimum

CORRECT ANSWER A

This finding most directly describes Beneficence: promote well-being and provide high-quality care improving oral and overall health. Distractor distinctions: B refers instead to treat only within your qualifications and expertise; deliver care within the qualified dentist's competence; C refers instead to patients have the right to make informed decisions; in practice, genuine informed consent requiring clear understanding of pu; D refers instead to ethical principles including billing and coding; accountability and team training with adherence to applicable laws; E refers instead to airflow + respiratory effort + blood oxygenation.

REFERENCE Integrated notebook, page 186, §9.4

QUESTION 0867 · §9.4 · Clinical decision-making SBA

A patient with financial barriers cannot access a custom appliance. Which ethical principle requires attention to equitable access?

A. Non-maleficence alone

B. Veracity only

C. Technician independence

D. Autonomy alone

E. Justice

CORRECT ANSWER E

Justice addresses fairness and equity in access to appropriate care, balanced with clinical safety and scope rules.

REFERENCE Integrated notebook, Topic 9, §9.4

QUESTION 0868 · §9.1 · Applied clinical SBA

A clinic considers an industry incentive linked to its choice of oral appliance. Which of the following statements about accurate, with a low failure rate, and attended by technical staff is most accurate?

A. Where in-laboratory PSG is impossible because of immobility, safety or critical illness.

B. Relevant findings include but expensive and technically complex.

C. Less expensive and quicker, and can be as accurate as PSG in selected populations.

D. Indication may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA.

E. Attended laboratory polysomnography provides detailed, reliable sleep and breathing measurements.

CORRECT ANSWER B

Relevant findings include but expensive and technically complex. Additional revision detail: but expensive and technically complex. The other options describe different mechanisms, investigations or findings rather than accurate, with a low failure rate, and attended by technical staff. Consult Integrated notebook, page 188, §9.1 for the full context.

REFERENCE Integrated notebook, page 188, §9.1

QUESTION 0869 · §9.1 · Clinical interpretation SBA

A dental clinic is auditing its written records for an oral appliance therapy service. The clinical team records the following finding: but expensive and technically complex. Which concept best explains this observation?

A. Portable monitoring

B. Accurate, with a low failure rate, and attended by technical staff

C. In-laboratory PSG

D. Special indications

E. Supervision

CORRECT ANSWER B

This finding most directly describes Accurate, with a low failure rate, and attended by technical staff: but expensive and technically complex. Distractor distinctions: A refers instead to less expensive and quicker, and can be as accurate as PSG in selected populations; C refers instead to accurate with a low failure rate, attended by technical staff; expensive and technically complex. Also the standard for narco; D refers instead to where in-laboratory PSG is impossible because of immobility, safety or critical illness; E refers instead to performed only in conjunction with a comprehensive sleep evaluation, by a board-certified sleep specialist or an individual m.

REFERENCE Integrated notebook, page 188, §9.1

QUESTION 0870 · §9.4 · Clinical decision-making SBA

A dentist accurately discloses uncertainty about the appliance's expected success and limitations of research. Which principle is best illustrated?

A. Marketing loyalty

B. Unjustified paternalism

C. Avoidance of consent

D. Excessive treatment

E. Veracity

CORRECT ANSWER E

Veracity is truthful communication about evidence, realistic response rates, side effects and uncertainty.

REFERENCE Integrated notebook, Topic 9, §9.4

QUESTION 0871 · §9.2 · Applied clinical SBA

A patient asks a dentist to provide a diagnosis on the basis of an overnight oximetry report. Which of the following statements about ongoing clinical notes is most accurate?

A. The clinical treatment plan should document appliance selection, titration, education and shared decisions.

B. Appliance delivery records should include home-care instructions, patient acknowledgement and warranty information.

C. Interprofessional records should document coordinated decisions between the dentist and sleep physician.

D. Longitudinal notes should record symptoms, adherence, adjustments and patient response.

E. Baseline sleep-study records should retain respiratory severity, sleep architecture and arousal measures.

CORRECT ANSWER D

Longitudinal notes should record symptoms, adherence, adjustments and patient response. Additional revision detail: patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians and qualified dentists. The other options describe different mechanisms, investigations or findings rather than ongoing clinical notes. Consult Integrated notebook, page 185, §9.2 for the full context.

REFERENCE Integrated notebook, page 185, §9.2

QUESTION 0872 · §9.2 · Clinical interpretation SBA

A clinic considers an industry incentive linked to its choice of oral appliance. The clinical team records the following finding: patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians and qualified dentists. Which concept best explains this observation?

A. Delivery documentation

B. Baseline sleep study data

C. Informed consent

D. Ongoing clinical notes

E. Treatment planning

CORRECT ANSWER D

This finding most directly describes Ongoing clinical notes: patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians and qualified dentists. Distractor distinctions: A refers instead to written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the pati; B refers instead to the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment re; C refers instead to complete signed forms explaining purpose, risks, benefits, alternatives and patient rights; E refers instead to comprehensive plans setting out rationale, appliance choice, adjustments, titration and patient education, with any shared de.

REFERENCE Integrated notebook, page 185, §9.2

QUESTION 0873 · §9.3 · Applied clinical SBA

An adult arrives with an elevated STOP-BANG score but no physician diagnosis of OSA. Which of the following statements about identify the correct codes is most accurate?

A. The clinician should use item codes and descriptions specific to the service provided and to the patient's in- surer.

B. Current reimbursement codes and eligibility requirements should be checked against the relevant official payer.

C. Public, private and veterans' health schemes may use different billing codes and eligibility rules.

D. The clinician should identify codes specific to the service and the insurer; maintain clear, separate consultation notes.

E. Claims across different insurers must separate unrelated conditions and meet each payer's requirements.

CORRECT ANSWER A

The clinician should use item codes and descriptions specific to the service provided and to the patient's in- surer. Additional revision detail: Use item codes and descriptions specific to the service provided and to the patient's in- surer. The other options describe different mechanisms, investigations or findings rather than identify the correct codes. Consult Integrated notebook, page 191, §9.3 for the full context.

REFERENCE Integrated notebook, page 191, §9.3

QUESTION 0874 · §9.3 · Clinical interpretation SBA

A dentist and sleep physician are defining responsibilities for an OAT referral pathway. The clinical team records the following finding: Use item codes and descriptions specific to the service provided and to the patient's in- surer. Which concept best explains this observation?

A. Consult authoritative sources

B. Identify the correct codes

C. Use DVA-specific codes

D. Separate conditions

E. Ethical dimension

CORRECT ANSWER B

This finding most directly describes Identify the correct codes: Use item codes and descriptions specific to the service provided and to the patient's in- surer. Distractor distinctions: A refers instead to the relevant government health department or insurer website — for current guidance rather than relying on memory or on colle; C refers instead to Veterans' affairs schemes maintain their own set of codes, which can differ from those used for the general public health ins; D refers instead to For veterans with both DVA and other insurance, clearly separate discussion and documentation of unrelated conditions when bi; E refers instead to billing and coding fall explicitly within the professionalism competencies of a qualified dentist. Appliance selection must n.

REFERENCE Integrated notebook, page 191, §9.3

QUESTION 0875 · §9.4 · Applied clinical SBA

A clinic considers an industry incentive linked to its choice of oral appliance. Which of the following statements about scope of competence is most accurate?

A. The clinician should use evidence-based practices, stay updated, and provide treatment only within established competence.

B. Beneficence includes promote well-being and provide high-quality care improving oral and overall health.

C. The clinician should treat only within your qualifications and expertise; deliver care within the qualified dentist's competence.

D. Pm minimum includes airflow + respiratory effort + blood oxygenation.

E. Professional conduct includes accurate billing, accountability and competent team-based care.

CORRECT ANSWER C

The clinician should treat only within your qualifications and expertise; deliver care within the qualified dentist's competence. Additional revision detail: treat only within your qualifications and expertise; deliver care within the qualified dentist's competence. The other options describe different mechanisms, investigations or findings rather than scope of competence. Consult Integrated notebook, page 186, §9.4 for the full context.

REFERENCE Integrated notebook, page 186, §9.4

QUESTION 0876 · §9.4 · Clinical interpretation SBA

A dental clinic is auditing its written records for an oral appliance therapy service. The clinical team records the following finding: treat only within your qualifications and expertise; deliver care within the qualified dentist's competence. Which concept best explains this observation?

A. Non-maleficence (do no harm)

B. PM minimum

C. Professionalism

D. Scope of competence

E. Beneficence

CORRECT ANSWER D

This finding most directly describes Scope of competence: treat only within your qualifications and expertise; deliver care within the qualified dentist's competence. Distractor distinctions: A refers instead to use evidence-based practices, stay updated, and provide treatment only within established competence; B refers instead to airflow + respiratory effort + blood oxygenation; C refers instead to ethical principles including billing and coding; accountability and team training with adherence to applicable laws; E refers instead to promote well-being and provide high-quality care improving oral and overall health.

REFERENCE Integrated notebook, page 186, §9.4

QUESTION 0877 · §9.1 · Applied clinical SBA

A patient asks a dentist to provide a diagnosis on the basis of an overnight oximetry report. Which of the following statements about portable monitoring is most accurate?

A. Where in-laboratory PSG is impossible because of immobility, safety or critical illness.

B. Indication may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA.

C. Attended laboratory polysomnography provides detailed, reliable sleep and breathing measurements.

D. Less expensive and quicker, and can be as accurate as PSG in selected populations.

E. Relevant findings include but expensive and technically complex.

CORRECT ANSWER D

Less expensive and quicker, and can be as accurate as PSG in selected populations. The other options describe different mechanisms, investigations or findings rather than portable monitoring. Consult Integrated notebook, page 183, §9.1 for the full context.

REFERENCE Integrated notebook, page 183, §9.1

QUESTION 0878 · §9.1 · Clinical interpretation SBA

A clinic considers an industry incentive linked to its choice of oral appliance. The clinical team records the following finding: less expensive and quicker, and can be as accurate as PSG in selected populations. Which concept best explains this observation?

A. Special indications

B. Indication

C. Supervision

D. Portable monitoring

E. In-laboratory PSG

CORRECT ANSWER D

This finding most directly describes Portable monitoring: less expensive and quicker, and can be as accurate as PSG in selected populations. Distractor distinctions: A refers instead to where in-laboratory PSG is impossible because of immobility, safety or critical illness; B refers instead to may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA; C refers instead to performed only in conjunction with a comprehensive sleep evaluation, by a board-certified sleep specialist or an individual m; E refers instead to accurate with a low failure rate, attended by technical staff; expensive and technically complex. Also the standard for narco.

REFERENCE Integrated notebook, page 183, §9.1

QUESTION 0879 · §9.2 · Applied clinical SBA

An adult arrives with an elevated STOP-BANG score but no physician diagnosis of OSA. Which of the following statements about baseline sleep study data is most accurate?

A. Interprofessional records should document coordinated decisions between the dentist and sleep physician.

B. Baseline sleep-study records should retain respiratory severity, sleep architecture and arousal measures.

C. Longitudinal notes should record symptoms, adherence, adjustments and patient response.

D. The clinical treatment plan should document appliance selection, titration, education and shared decisions.

E. Appliance delivery records should include home-care instructions, patient acknowledgement and warranty information.

CORRECT ANSWER B

Baseline sleep-study records should retain respiratory severity, sleep architecture and arousal measures. Additional revision detail: the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment reference and follow-up comparison. The other options describe different mechanisms, investigations or findings rather than baseline sleep study data. Consult Integrated notebook, page 185, §9.2 for the full context.

REFERENCE Integrated notebook, page 185, §9.2

QUESTION 0880 · §9.2 · Clinical interpretation SBA

A dentist and sleep physician are defining responsibilities for an OAT referral pathway. The clinical team records the following finding: the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment reference and follow-up comparison. Which concept best explains this observation?

A. Interprofessional communication

B. Delivery documentation

C. Treatment planning

D. Ongoing clinical notes

E. Baseline sleep study data

CORRECT ANSWER E

This finding most directly describes Baseline sleep study data: the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment reference and follow-up comparison. Distractor distinctions: A refers instead to records of communication between sleep physicians, dentists and others, ensuring coordinated care with shared assessment, adj; B refers instead to written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the pati; C refers instead to comprehensive plans setting out rationale, appliance choice, adjustments, titration and patient education, with any shared de; D refers instead to patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians.

REFERENCE Integrated notebook, page 185, §9.2

QUESTION 0881 · §9.3 · Applied clinical SBA

A clinic considers an industry incentive linked to its choice of oral appliance. Which of the following statements about use dva-specific codes is most accurate?

A. Current reimbursement codes and eligibility requirements should be checked against the relevant official payer.

B. Public, private and veterans’ health schemes may use different billing codes and eligibility rules.

C. The clinician should identify codes specific to the service and the insurer; maintain clear, separate consultation notes.

D. Claims across different insurers must separate unrelated conditions and meet each payer’s requirements.

E. Ethical dimension includes billing and coding fall explicitly within the professionalism competencies of a qualified dentist.

CORRECT ANSWER B

Public, private and veterans’ health schemes may use different billing codes and eligibility rules. Additional revision detail: Veterans' affairs schemes maintain their own set of codes, which can differ from those used for the general public health insurer or for private insurance. The other options describe different mechanisms, investigations or findings rather than use dva-specific codes. Consult Integrated notebook, page 192, §9.3 for the full context.

REFERENCE Integrated notebook, page 192, §9.3

QUESTION 0882 · §9.3 · Clinical interpretation SBA

A dental clinic is auditing its written records for an oral appliance therapy service. The clinical team records the following finding: Veterans' affairs schemes maintain their own set of codes, which can differ from those used for the general public health insurer or for private insurance. Which concept best explains this observation?

A. Identify the correct codes

B. Separate conditions

C. Consult authoritative sources

D. Ethical dimension

E. Use DVA-specific codes

CORRECT ANSWER E

This finding most directly describes Use DVA-specific codes: Veterans' affairs schemes maintain their own set of codes, which can differ from those used for the general public health insurer or for private insurance. Distractor distinctions: A refers instead to Use item codes and descriptions specific to the service provided and to the patient's in- surer; B refers instead to For veterans with both DVA and other insurance, clearly separate discussion and documentation of unrelated conditions when bi; C refers instead to the relevant government health department or insurer website — for current guidance rather than relying on memory or on colle; D refers instead to billing and coding fall explicitly within the professionalism competencies of a qualified dentist. Appliance selection must n.

REFERENCE Integrated notebook, page 192, §9.3

QUESTION 0883 · §9.4 · Applied clinical SBA

A dentist and sleep physician are defining responsibilities for an OAT referral pathway. Which of the following statements about autonomy is most accurate?

A. The clinician should use evidence-based practices, stay updated, and provide treatment only within established competence.

B. Respect for autonomy requires informed patient decisions about benefits, risks and alternatives.

C. The clinician should treat only within your qualifications and expertise; deliver care within the qualified dentist's competence.

D. Professional conduct includes accurate billing, accountability and competent team-based care.

E. Beneficence includes promote well-being and provide high-quality care improving oral and overall health.

CORRECT ANSWER B

Respect for autonomy requires informed patient decisions about benefits, risks and alternatives. Additional revision detail: patients have the right to make informed decisions; in practice, genuine informed consent requiring clear understanding of purpose, risks, benefits and alternatives. The other options describe different mechanisms, investigations or findings rather than autonomy. Consult Integrated notebook, page 186, §9.4 for the full context.

REFERENCE Integrated notebook, page 186, §9.4

QUESTION 0884 · §9.4 · Clinical interpretation SBA

A patient asks a dentist to provide a diagnosis on the basis of an overnight oximetry report. The clinical team records the following finding: patients have the right to make informed decisions; in practice, genuine informed consent requiring clear understanding of purpose, risks, benefits and alternatives. Which concept best explains this observation?

A. Scope of competence

B. Beneficence

C. Autonomy

D. PM minimum

E. Professionalism

CORRECT ANSWER C

This finding most directly describes Autonomy: patients have the right to make informed decisions; in practice, genuine informed consent requiring clear understanding of purpose, risks, benefits and alternatives. Distractor distinctions: A refers instead to treat only within your qualifications and expertise; deliver care within the qualified dentist's competence; B refers instead to promote well-being and provide high-quality care improving oral and overall health; D refers instead to airflow + respiratory effort + blood oxygenation; E refers instead to ethical principles including billing and coding; accountability and team training with adherence to applicable laws.

REFERENCE Integrated notebook, page 186, §9.4

QUESTION 0885 · §9.1 · Applied clinical SBA

An adult arrives with an elevated STOP-BANG score but no physician diagnosis of OSA. Which of the following statements about in-laboratory PSG is most accurate?

A. Attended laboratory polysomnography provides detailed, reliable sleep and breathing measurements.

B. Indication may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA.

C. Where in-laboratory PSG is impossible because of immobility, safety or critical illness.

D. Less expensive and quicker, and can be as accurate as PSG in selected populations.

E. Portable monitoring should occur within a comprehensive sleep evaluation supervised by a qualified sleep physician.

CORRECT ANSWER A

Attended laboratory polysomnography provides detailed, reliable sleep and breathing measurements. Additional revision detail: accurate with a low failure rate, attended by technical staff; expensive and technically complex. Also the standard for narcolepsy and central sleep apnoea. The other options describe different mechanisms, investigations or findings rather than in-laboratory psg. Consult Integrated notebook, page 183, §9.1 for the full context.

REFERENCE Integrated notebook, page 183, §9.1

QUESTION 0886 · §9.1 · Clinical interpretation SBA

A dentist and sleep physician are defining responsibilities for an OAT referral pathway. The clinical team records the following finding: accurate with a low failure rate, attended by technical staff; expensive and technically complex. Also the standard for narcolepsy and central sleep apnoea. Which concept best explains this observation?

A. Special indications

B. Supervision

C. Indication

D. Portable monitoring

E. In-laboratory PSG

CORRECT ANSWER E

This finding most directly describes In-laboratory PSG: accurate with a low failure rate, attended by technical staff; expensive and technically complex. Also the standard for narcolepsy and central sleep apnoea. Distractor distinctions: A refers instead to where in-laboratory PSG is impossible because of immobility, safety or critical illness; B refers instead to performed only in conjunction with a comprehensive sleep evaluation, by a board-certified sleep specialist or an individual m; C refers instead to may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA; D refers instead to less expensive and quicker, and can be as accurate as PSG in selected populations.

REFERENCE Integrated notebook, page 183, §9.1

QUESTION 0887 · §9.2 · Applied clinical SBA

A clinic considers an industry incentive linked to its choice of oral appliance. Which of the following statements about interprofessional communication is most accurate?

A. The clinical treatment plan should document appliance selection, titration, education and shared decisions.

B. Baseline sleep-study records should retain respiratory severity, sleep architecture and arousal measures.

C. Longitudinal notes should record symptoms, adherence, adjustments and patient response.

D. Appliance delivery records should include home-care instructions, patient acknowledgement and warranty information.

E. Interprofessional records should document coordinated decisions between the dentist and sleep physician.

CORRECT ANSWER E

Interprofessional records should document coordinated decisions between the dentist and sleep physician. Additional revision detail: records of communication between sleep physicians, dentists and others, ensuring coordinated care with shared assessment, adjustments and follow-up, documented with patient permission. The other options describe different mechanisms, investigations or findings rather than interprofessional communication. Consult Integrated notebook, page 185, §9.2 for the full context.

REFERENCE Integrated notebook, page 185, §9.2

QUESTION 0888 · §9.2 · Clinical interpretation SBA

A dental clinic is auditing its written records for an oral appliance therapy service. The clinical team records the following finding: records of communication between sleep physicians, dentists and others, ensuring coordinated care with shared assessment, adjustments and follow-up, documented with patient permission. Which concept best explains this observation?

A. Delivery documentation

B. Interprofessional communication

C. Ongoing clinical notes

D. Treatment planning

E. Baseline sleep study data

CORRECT ANSWER B

This finding most directly describes Interprofessional communication: records of communication between sleep physicians, dentists and others, ensuring coordinated care with shared assessment, adjustments and follow-up, documented with patient permission. Distractor distinctions: A refers instead to written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the pati; C refers instead to patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians; D refers instead to comprehensive plans setting out rationale, appliance choice, adjustments, titration and patient education, with any shared de; E refers instead to the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment re.

REFERENCE Integrated notebook, page 185, §9.2

QUESTION 0889 · §9.3 · Applied clinical SBA

A dentist and sleep physician are defining responsibilities for an OAT referral pathway. Which of the following statements about ethical dimension is most accurate?

A. Public, private and veterans' health schemes may use different billing codes and eligibility rules.

B. Ethical dimension includes billing and coding fall explicitly within the professionalism competencies of a qualified dentist.

C. Claims across different insurers must separate unrelated conditions and meet each payer's requirements.

D. Current reimbursement codes and eligibility requirements should be checked against the relevant official payer.

E. The clinician should identify codes specific to the service and the insurer; maintain clear, separate consultation notes.

CORRECT ANSWER B

Ethical dimension includes billing and coding fall explicitly within the professionalism competencies of a qualified dentist. Additional revision detail: billing and coding fall explicitly within the professionalism competencies of a qualified dentist. Appliance selection must never be driven by what an insurer will reimburse. 9.4 Ethics. The other options describe different mechanisms, investigations or findings rather than ethical dimension. Consult Integrated notebook, page 186, §9.3 for the full context.

REFERENCE Integrated notebook, page 186, §9.3

QUESTION 0890 · §9.3 · Clinical interpretation SBA

A patient asks a dentist to provide a diagnosis on the basis of an overnight oximetry report. The clinical team records the following finding: billing and coding fall explicitly within the professionalism competencies of a qualified dentist. Appliance selection must never be driven by what an insurer will reimburse. 9.4 Ethics. Which concept best explains this observation?

A. Use DVA-specific codes

B. Private billing

C. Separate conditions

D. Ethical dimension

E. Identify the correct codes

CORRECT ANSWER D

This finding most directly describes Ethical dimension: billing and coding fall explicitly within the professionalism competencies of a qualified dentist. Appliance selection must never be driven by what an insurer will reimburse. 9.4 Ethics. Distractor distinctions: A refers instead to Veterans' affairs schemes maintain their own set of codes, which can differ from those used for the general public health ins; B refers instead to identify codes specific to the service and the insurer; maintain clear, separate consultation notes differentiating insurers; C refers instead to For veterans with both DVA and other insurance, clearly separate discussion and documentation of unrelated conditions when bi; E refers instead to Use item codes and descriptions specific to the service provided and to the patient's in- surer.

REFERENCE Integrated notebook, page 186, §9.3

QUESTION 0891 · §9.4 · Applied clinical SBA

A dental clinic is auditing its written records for an oral appliance therapy service. Which of the following statements about professionalism is most accurate?

A. Beneficence includes promote well-being and provide high-quality care improving oral and overall health.

B. Respect for autonomy requires informed patient decisions about benefits, risks and alternatives.

C. The clinician should use evidence-based practices, stay updated, and provide treatment only within established competence.

D. The clinician should treat only within your qualifications and expertise; deliver care within the qualified dentist's competence.

E. Professional conduct includes accurate billing, accountability and competent team-based care.

CORRECT ANSWER E

Professional conduct includes accurate billing, accountability and competent team-based care. Additional revision detail: ethical principles including billing and coding; accountability and team training with adherence to applicable laws. The other options describe different mechanisms, investigations or findings rather than professionalism. Consult Integrated notebook, page 187, §9.4 for the full context.

REFERENCE Integrated notebook, page 187, §9.4

QUESTION 0892 · §9.4 · Clinical interpretation SBA

An adult arrives with an elevated STOP-BANG score but no physician diagnosis of OSA. The clinical team records the following finding: ethical principles including billing and coding; accountability and team training with adherence to applicable laws. Which concept best explains this observation?

A. Beneficence

B. PM minimum

C. Scope of competence

D. Autonomy

E. Professionalism

CORRECT ANSWER E

This finding most directly describes Professionalism: ethical principles including billing and coding; accountability and team training with adherence to applicable laws. Distractor distinctions: A refers instead to promote well-being and provide high-quality care improving oral and overall health; B refers instead to airflow + respiratory effort + blood oxygenation; C refers instead to treat only within your qualifications and expertise; deliver care within the qualified dentist's competence; D refers instead to patients have the right to make informed decisions; in practice, genuine informed consent requiring clear understanding of pu.

REFERENCE Integrated notebook, page 187, §9.4

QUESTION 0893 · §9.1 · Applied clinical SBA

A clinic considers an industry incentive linked to its choice of oral appliance. Which of the following statements about indication is most accurate?

A. Less expensive and quicker, and can be as accurate as PSG in selected populations.

B. Indication may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA.

C. Where in-laboratory PSG is impossible because of immobility, safety or critical illness.

D. Relevant findings include but expensive and technically complex.

E. Attended laboratory polysomnography provides detailed, reliable sleep and breathing measurements.

CORRECT ANSWER B

Indication may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA. Additional revision detail: may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA. The other options describe different mechanisms, investigations or findings rather than indication. Consult Integrated notebook, page 184, §9.1 for the full context.

REFERENCE Integrated notebook, page 184, §9.1

QUESTION 0894 · §9.1 · Clinical interpretation SBA

A dental clinic is auditing its written records for an oral appliance therapy service. The clinical team records the following finding: may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA. Which concept best explains this observation?

A. Supervision

B. Indication

C. In-laboratory PSG

D. Portable monitoring

E. Special indications

CORRECT ANSWER B

This finding most directly describes Indication: may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA. Distractor distinctions: A refers instead to performed only in conjunction with a comprehensive sleep evaluation, by a board-certified sleep specialist or an individual m; C refers instead to accurate with a low failure rate, attended by technical staff; expensive and technically complex. Also the standard for narco; D refers instead to less expensive and quicker, and can be as accurate as PSG in selected populations; E refers instead to where in-laboratory PSG is impossible because of immobility, safety or critical illness.

REFERENCE Integrated notebook, page 184, §9.1

QUESTION 0895 · §9.2 · Applied clinical SBA

A dentist and sleep physician are defining responsibilities for an OAT referral pathway. Which of the following statements about delivery documentation is most accurate?

A. Baseline sleep-study records should retain respiratory severity, sleep architecture and arousal measures.

B. Appliance delivery records should include home-care instructions, patient acknowledgement and warranty information.

C. The clinical treatment plan should document appliance selection, titration, education and shared decisions.

D. Longitudinal notes should record symptoms, adherence, adjustments and patient response.

E. Interprofessional records should document coordinated decisions between the dentist and sleep physician.

CORRECT ANSWER B

Appliance delivery records should include home-care instructions, patient acknowledgement and warranty information. Additional revision detail: written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the patient and one retained. The other options describe different mechanisms, investigations or findings rather than delivery documentation. Consult Integrated notebook, page 185, §9.2 for the full context.

REFERENCE Integrated notebook, page 185, §9.2

QUESTION 0896 · §9.2 · Clinical interpretation SBA

A patient asks a dentist to provide a diagnosis on the basis of an overnight oximetry report. The clinical team records the following finding: written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the patient and one retained. Which concept best explains this observation?

A. Baseline sleep study data

B. Informed consent

C. Ongoing clinical notes

D. Delivery documentation

E. Treatment planning

CORRECT ANSWER D

This finding most directly describes Delivery documentation: written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the patient and one retained. Distractor distinctions: A refers instead to the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment re; B refers instead to complete signed forms explaining purpose, risks, benefits, alternatives and patient rights; C refers instead to patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians; E refers instead to comprehensive plans setting out rationale, appliance choice, adjustments, titration and patient education, with any shared de.

REFERENCE Integrated notebook, page 185, §9.2

QUESTION 0897 · §9.3 · Applied clinical SBA

A dental clinic is auditing its written records for an oral appliance therapy service. Which of the following statements about private billing is most accurate?

A. The clinician should identify codes specific to the service and the insurer; maintain clear, separate consultation notes.

B. The clinician should use item codes and descriptions specific to the service provided and to the patient's in- surer.

C. Public, private and veterans’ health schemes may use different billing codes and eligibility rules.

D. Current reimbursement codes and eligibility requirements should be checked against the relevant official payer.

E. Claims across different insurers must separate unrelated conditions and meet each payer’s requirements.

CORRECT ANSWER A

The clinician should identify codes specific to the service and the insurer; maintain clear, separate consultation notes. Additional revision detail: identify codes specific to the service and the insurer; maintain clear, separate consultation notes differentiating insurers and conditions. The other options describe different mechanisms, investigations or findings rather than private billing. Consult Integrated notebook, page 186, §9.3 for the full context.

REFERENCE Integrated notebook, page 186, §9.3

QUESTION 0898 · §9.3 · Clinical interpretation SBA

An adult arrives with an elevated STOP-BANG score but no physician diagnosis of OSA. The clinical team records the following finding: identify codes specific to the service and the insurer; maintain clear, separate consultation notes differentiating insurers and conditions. Which concept best explains this observation?

A. Private billing

B. Identify the correct codes

C. Separate conditions

D. Ethical dimension

E. Use DVA-specific codes

CORRECT ANSWER A

This finding most directly describes Private billing: identify codes specific to the service and the insurer; maintain clear, separate consultation notes differentiating insurers and conditions. Distractor distinctions: B refers instead to Use item codes and descriptions specific to the service provided and to the patient's in- surer; C refers instead to For veterans with both DVA and other insurance, clearly separate discussion and documentation of unrelated conditions when bi; D refers instead to billing and coding fall explicitly within the professionalism competencies of a qualified dentist. Appliance selection must n; E refers instead to Veterans' affairs schemes maintain their own set of codes, which can differ from those used for the general public health ins.

REFERENCE Integrated notebook, page 186, §9.3

QUESTION 0899 · §9.4 · Applied clinical SBA

A patient asks a dentist to provide a diagnosis on the basis of an overnight oximetry report. Which of the following statements about non-maleficence (do no harm) is most accurate?

A. Pm minimum includes airflow + respiratory effort + blood oxygenation.

B. The clinician should treat only within your qualifications and expertise; deliver care within the qualified dentist's competence.

C. Beneficence includes promote well-being and provide high-quality care improving oral and overall health.

D. The clinician should use evidence-based practices, stay updated, and provide treatment only within established competence.

E. Professional conduct includes accurate billing, accountability and competent team-based care.

CORRECT ANSWER D

The clinician should use evidence-based practices, stay updated, and provide treatment only within established competence. Additional revision detail: use evidence-based practices, stay updated, and provide treatment only within established competence. The other options describe different mechanisms, investigations or findings rather than non-maleficence (do no harm). Consult Integrated notebook, page 186, §9.4 for the full context.

REFERENCE Integrated notebook, page 186, §9.4

QUESTION 0900 · §9.4 · Clinical interpretation SBA

A clinic considers an industry incentive linked to its choice of oral appliance. The clinical team records the following finding: use evidence-based practices, stay updated, and provide treatment only within established competence. Which concept best explains this observation?

A. Beneficence

B. PM minimum

C. Non-maleficence (do no harm)

D. Professionalism

E. Scope of competence

CORRECT ANSWER C

This finding most directly describes Non-maleficence (do no harm): use evidence-based practices, stay updated, and provide treatment only within established competence. Distractor distinctions: A refers instead to promote well-being and provide high-quality care improving oral and overall health; B refers instead to airflow + respiratory effort + blood oxygenation; D refers instead to ethical principles including billing and coding; accountability and team training with adherence to applicable laws; E refers instead to treat only within your qualifications and expertise; deliver care within the qualified dentist's competence.

REFERENCE Integrated notebook, page 186, §9.4

10 Informed consent and management of adverse effects

QUESTION 0901 · §10.1 · Clinical decision-making SBA

Before a MAD is fitted, a patient signs a form but cannot explain any long-term side effects. What is the best response?

A. Proceed without further discussion

B. The signature proves valid comprehension

C. Repeat the explanation and verify understanding rather than treating a signature as sufficient

D. Avoid mentioning alternatives

E. Ask the technician to consent for the patient

CORRECT ANSWER C

Informed consent requires capacity, voluntariness and comprehension; teach-back helps verify meaningful understanding.

REFERENCE Integrated notebook, Topic 10, §10.1

QUESTION 0902 · §10.1 · Clinical decision-making SBA

Before starting OAT, a patient signs a form but says that long-term bite changes and alternative treatments were never explained. Which is the best conclusion?

A. A signature alone establishes fully informed consent

B. The consent process is inadequate until material risks, benefits and reasonable alternatives are discussed and understood

C. Consent can always be postponed until after symptoms arise

D. Bite change is too uncommon to disclose

E. Only reimbursement information needs discussion

CORRECT ANSWER B

Informed consent is a process, not a signature. Dental changes, possible TMJ symptoms, reasonable alternatives and treatment limitations are material to decision-making. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 10, §10.1

QUESTION 0903 · §10.3 · Clinical decision-making SBA

After years of nightly mandibular advancement, a patient has reduced overjet and fewer posterior contacts compared with baseline. Which is the most likely contributor?

A. Documented long-term dentoalveolar and occlusal effects of OAT

B. A normal change that proves OSA cure

C. Instant irreversible skeletal surgery caused by the device

D. A change that can never occur with OAT

E. CPAP mask pressure on the nasal bridge only

CORRECT ANSWER A

Gradual tooth movement and occlusal adaptation are recognised potential long-term consequences of sustained mandibular advancement. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 10, §10.3

QUESTION 0904 · §10.4 · Clinical decision-making SBA

A MAD user describes a transient altered bite every morning. Which adjunctive approach may be recommended after examination?

A. No morning assessment because bite changes are irrelevant

B. A properly recorded morning occlusal guide and gentle exercises when appropriate

C. Permanent mechanical opening of the jaw throughout the day

D. Irreversible equilibration without informed consent

E. Unlimited overnight protrusion

CORRECT ANSWER B

A morning repositioning guide and exercises can assist restoration of habitual occlusion after nightly advancement; documentation and individualisation are important. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 10, §10.4

QUESTION 0905 · §10.5 · Clinical decision-making SBA

After several weeks of MAD use, a patient develops persistent TMJ pain and restricted opening. Which action is best?

A. Continue increasing protrusion despite the new limitation

B. Assess the joint, review the appliance setting and temporarily reduce or suspend use if indicated

C. Reassure without examination because pain is always transient

D. Perform irreversible occlusal equilibration at once

E. Treat pain only by increasing vertical opening

CORRECT ANSWER B

Persistent painful restriction warrants reassessment of joint function and the appliance, modification of advancement and, when appropriate, temporary suspension or referral. The alternatives either confuse screening with diagnosis, misapply the treatment indication, or omit a necessary clinical safeguard.

REFERENCE Integrated notebook, Topic 10, §10.5

QUESTION 0906 · §10.2 · Clinical decision-making SBA

A person develops reduced overjet after five years of nightly MAD use. How should this be framed?

A. An impossible adverse effect

B. An isolated event unrelated to mandibular protrusion

C. A definitive marker that OSA is cured

D. A recognised dose- and time-dependent occlusal effect that should have been disclosed and monitored

E. Proof the patient fabricated the complaint

CORRECT ANSWER D

Posterior forces on upper teeth and anterior forces on lower teeth predict changes in overjet, overbite and tooth position over time.

REFERENCE Integrated notebook, Topic 10, §10.2

QUESTION 0907 · §10.2 · Applied clinical SBA

A patient using a MAD reports persistent morning occlusal changes and reduced comfort. Which of the following statements about Mechanism is most accurate?

A. Appliance-associated bite changes can cause food impaction, altered speech or chewing difficulty.

B. Failure to restore habitual mandibular position after nightly appliance use may contribute to occlusal changes.

C. Many appliance users have small occlusal changes that they do not notice or find troublesome.

D. Long-term nightly mandibular advancement frequently produces detectable occlusal changes.

E. Mandibular advancement can shift maxillary teeth distally and mandibular teeth mesially over time.

CORRECT ANSWER B

Failure to restore habitual mandibular position after nightly appliance use may contribute to occlusal changes. Additional revision detail: occlusal changes occur because of a failure to reposition the mandible during the day following use of the device, rather than as a result of immutable changes caused by the device overnight. The other options describe different mechanisms, investigations or findings rather than mechanism. Consult Integrated notebook, page 207, §10.2 for the full context.

REFERENCE Integrated notebook, page 207, §10.2

QUESTION 0908 · §10.2 · Focused knowledge SBA

A patient starting long-term MAD therapy asks about bite change and jaw discomfort. Which of the following statements about Mechanism is most accurate?

A. Many appliance users have small occlusal changes that they do not notice or find troublesome.

B. Long-term nightly mandibular advancement frequently produces detectable occlusal changes.

C. Appliance-associated bite changes can cause food impaction, altered speech or chewing difficulty.

D. Mandibular advancement can shift maxillary teeth distally and mandibular teeth mesially over time.

E. Failure to restore habitual mandibular position after nightly appliance use may contribute to occlusal changes.

CORRECT ANSWER E

Failure to restore habitual mandibular position after nightly appliance use may contribute to occlusal changes. Additional revision detail: occlusal changes occur because of a failure to reposition the mandible during the day following use of the device, rather than as a result of immutable changes caused by the device overnight. The other options describe different mechanisms, investigations or findings rather than mechanism. Consult Integrated notebook, page 207, §10.2 for the full context.

REFERENCE Integrated notebook, page 207, §10.2

QUESTION 0909 · §10.4 · Clinical decision-making SBA

A MAD user wakes with a temporary posterior open bite. Which preventive strategy appears in the notes?

A. No baseline models

B. Prolonged overnight mouth opening

C. Maximal extra protrusion on waking

D. A custom morning occlusal guide used for about 15–20 minutes

E. Immediate irreversible grinding of the bite

CORRECT ANSWER D

A morning guide and jaw exercises may help return the mandible to habitual occlusion and document progression.

REFERENCE Integrated notebook, Topic 10, §10.4

QUESTION 0910 · §10.3 · Applied clinical SBA

A dentist is discussing material risks and reasonable alternatives before appliance delivery. Which of the following statements about soft tissues is most accurate?

A. Hypersalivation and dry mouth can occur during initial adaptation to oral appliances.

B. Long-term nightly mandibular advancement frequently produces detectable occlusal changes.

C. A smaller protrusive dose may be sufficient in some mild-to-moderate OSA cases.

D. A morning occlusal guide helps the mandible return toward habitual intercuspation after appliance removal.

E. Risk increases with greater advancement, with levels exceeding 50% of maximum often correlating with significantly higher risk.

CORRECT ANSWER A

Hypersalivation and dry mouth can occur during initial adaptation to oral appliances. Additional revision detail: hypersalivation or dry mouth; the intended airway enlargement by advancing the mandible and repositioning the tongue. The other options describe different mechanisms, investigations or findings rather than soft tissues. Consult Integrated notebook, page 199, §10.3 for the full context.

REFERENCE Integrated notebook, page 199, §10.3

QUESTION 0911 · §10.3 · Clinical interpretation SBA

A patient using a MAD reports persistent morning occlusal changes and reduced comfort. The clinical team records the following finding: hypersalivation or dry mouth; the intended airway enlargement by advancing the mandible and repositioning the tongue. Which concept best explains this observation?

A. Soft tissue irritation

B. Soft tissues

C. How it works

D. Factors influencing severity

E. Corollary

CORRECT ANSWER B

This finding most directly describes Soft tissues: hypersalivation or dry mouth; the intended airway enlargement by advancing the mandible and repositioning the tongue. Distractor distinctions: A refers instead to screw mechanism or rough surface rubbing tongue or cheek; cheek or lip biting from bite change. → polish; discontinue until h; C refers instead to realigns the bite toward original maximum intercuspation; exercises and stretches the muscles of mastication, tight after a n; D refers instead to risk increases with greater advancement, with levels exceeding 50% of maximum often correlating with significantly higher ris; E refers instead to an appliance is already effective at a less protrusive position in mild and moderate OSA, and therefore causes less severe de.

REFERENCE Integrated notebook, page 199, §10.3

QUESTION 0912 · §10.5 · Clinical decision-making SBA

A patient develops mild transient salivation soon after appliance insertion. What is the usual first response?

A. Automatic permanent cessation

B. Maximal advancement

C. Diagnose central apnoea

D. Surgical removal of salivary glands

E. Reassure, assess device fit and monitor while giving practical advice

CORRECT ANSWER E

Early excess saliva is a common transient side effect; persistent or severe symptoms need assessment and modification.

REFERENCE Integrated notebook, Topic 10, §10.5

QUESTION 0913 · §10.4 · Applied clinical SBA

An adult with new TMJ discomfort returns after increasing mandibular protrusion. Which of the following statements about exercises increase contact between the teeth is most accurate?

A. Exercises increase contact between the teeth includes good in the short term, with limited long-term evidence.

B. A morning occlusal guide helps the mandible return toward habitual intercuspation after appliance removal.

C. Wear for 15–20 minutes in the morning after waking includes some protocols specify around 10 minutes.

D. Gentle morning jaw exercises may relieve post-appliance muscular stiffness.

E. Morning occlusal exercises may help recover habitual mandibular position after overnight advancement.

CORRECT ANSWER A

Exercises increase contact between the teeth includes good in the short term, with limited long-term evidence. Additional revision detail: good in the short term, with limited long-term evidence. The other options describe different mechanisms, investigations or findings rather than exercises increase contact between the teeth. Consult Integrated notebook, page 210, §10.4 for the full context.

REFERENCE Integrated notebook, page 210, §10.4

QUESTION 0914 · §10.4 · Clinical interpretation SBA

A dentist is discussing material risks and reasonable alternatives before appliance delivery. The clinical team records the following finding: good in the short term, with limited long-term evidence. Which concept best explains this observation?

A. Wear for 15–20 minutes in the morning after waking

B. Positions the condyles

C. Exercises increase contact between the teeth

D. Exercises the jaw muscles

E. Realigns the bite

CORRECT ANSWER C

This finding most directly describes Exercises increase contact between the teeth: good in the short term, with limited long-term evidence. Distractor distinctions: A refers instead to some protocols specify around 10 minutes; B refers instead to assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort; D refers instead to applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear; E refers instead to custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation.

REFERENCE Integrated notebook, page 210, §10.4

QUESTION 0915 · §10.5 · Clinical decision-making SBA

Persistent severe unilateral jaw pain develops during titration. What action is most appropriate?

A. Increase protrusion to maximum

B. Ignore it because TMD never matters

C. Classify the patient as cured

D. Suspend advancement, evaluate the TMJ and consider appliance adjustment or temporary cessation

E. Avoid any clinical examination

CORRECT ANSWER D

Ongoing severe pain is a reason to modify or pause therapy, potentially with specialist referral, rather than continuing automatic advancement.

REFERENCE Integrated notebook, Topic 10, §10.5

QUESTION 0916 · §10.5 · Applied clinical SBA

At an annual OAT review, a patient notices a reduction in overjet and altered occlusal contacts. Which of the following statements about minor bite change is most accurate?

A. Unilateral jaw pain includes prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines.

B. A qualified dentist is essential for monitoring side effects and making necessary adjustments to the appliance or plan.

C. Transient morning jaw discomfort may result from overnight mandibular advancement.

D. Soft tissue irritation includes screw mechanism or rough surface rubbing tongue or cheek.

E. Progressive occlusal changes arise from sustained dental forces during long-term MAD use.

CORRECT ANSWER E

Progressive occlusal changes arise from sustained dental forces during long-term MAD use. Additional revision detail: mechanical forces causing gradual shifts. → jaw exercises, passive exercises, morning occlusal guide; adjust the appliance or settings. The other options describe different mechanisms, investigations or findings rather than minor bite change. Consult Integrated notebook, page 200, §10.5 for the full context.

REFERENCE Integrated notebook, page 200, §10.5

QUESTION 0917 · §10.5 · Clinical interpretation SBA

An adult with new TMJ discomfort returns after increasing mandibular protrusion. The clinical team records the following finding: mechanical forces causing gradual shifts. → jaw exercises, passive exercises, morning occlusal guide; adjust the appliance or settings. Which concept best explains this observation?

A. Palliative care

B. Minor bite change

C. Bilateral jaw pain

D. Regular follow-up

E. Patient education

CORRECT ANSWER B

This finding most directly describes Minor bite change: mechanical forces causing gradual shifts. → jaw exercises, passive exercises, morning occlusal guide; adjust the appliance or settings. Distractor distinctions: A refers instead to supportive care to manage discomfort, especially during the initial adaptation period; C refers instead to advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed); D refers instead to monitoring by qualified dentists is necessary to survey for dental side effects and occlusal change, and is essential for max; E refers instead to reassure that symptoms are likely temporary and manageable.

REFERENCE Integrated notebook, page 200, §10.5

QUESTION 0918 · §10.6 · Clinical decision-making SBA

A patient cannot tolerate further protrusion but retains substantial residual OSA. What should be discussed?

A. Device wear without adequate benefit indefinitely

B. Alternative or combination therapy through the sleep physician while prioritising safety

C. Unilateral tooth movement as the sole treatment

D. No further care needed

E. Immediate orthodontics without consent

CORRECT ANSWER B

If therapeutic benefit is inadequate at a tolerable setting, multidisciplinary alternatives and informed decisions about stopping or changing treatment are required.

REFERENCE Integrated notebook, Topic 10, §10.6

QUESTION 0919 · §10.6 · Applied clinical SBA

A patient starting long-term MAD therapy asks about bite change and jaw discomfort. Which of the following statements about patient-specific factors is most accurate?

A. Patient-specific factors includes overall health, tolerance for side effects and commitment to therapy.

B. Some early appliance-associated bite changes improve after adjustment or cessation with monitored care.

C. Qualified dentists should review oral appliance therapy regularly for adherence, effectiveness and side effects.

D. Regular reviews is considered at least 6-monthly for the first year, then annually.

E. The conversation is only protective if documented.

CORRECT ANSWER A

Patient-specific factors includes overall health, tolerance for side effects and commitment to therapy. Additional revision detail: overall health, tolerance for side effects and commitment to therapy. The other options describe different mechanisms, investigations or findings rather than patient-specific factors. Consult Integrated notebook, page 212, §10.6 for the full context.

REFERENCE Integrated notebook, page 212, §10.6

QUESTION 0920 · §10.6 · Clinical interpretation SBA

At an annual OAT review, a patient notices a reduction in overjet and altered occlusal contacts. The clinical team records the following finding: overall health, tolerance for side effects and commitment to therapy. Which concept best explains this observation?

A. Patient-specific factors

B. Where treatment stops

C. Discuss with the patient and record the discussion

D. Occlusal change the patient finds unacceptable

E. Remember the rule from 10.5

CORRECT ANSWER A

This finding most directly describes Patient-specific factors: overall health, tolerance for side effects and commitment to therapy. Distractor distinctions: B refers instead to notify the medical provider, and refer non-compliant patients back to consider alternatives, since untreated or subtherapeuti; C refers instead to the conversation is only protective if documented; D refers instead to treatment should be discontinued only if the patient accepts another form of therapy from their attending physician, or under; E refers instead to definitive correction should be undertaken only once appliance therapy has been discontinued and the patient is on another OS.

REFERENCE Integrated notebook, page 212, §10.6

QUESTION 0921 · §10.3 · Clinical decision-making SBA

A patient with minimal baseline overjet develops an anterior crossbite during prolonged therapy. Why were they vulnerable?

A. Only the lower molars move under MAD therapy

B. Small starting overjet leaves little margin for expected incisor movement

C. Overjet generally increases with MAD use

D. Crossbite proves no night-time use

E. Baseline occlusion has no predictive value

CORRECT ANSWER B

Initial edge-to-edge relations create vulnerability because the appliance tends to reduce overjet and overbite over time.

REFERENCE Integrated notebook, Topic 10, §10.3

QUESTION 0922 · §10.1 · Applied clinical SBA

A dentist is discussing material risks and reasonable alternatives before appliance delivery. Which of the following statements about three tests of valid consent is most accurate?

A. Valid consent requires decision-making capacity, voluntariness and comprehension of material information.

B. A smaller protrusive dose may be sufficient in some mild-to-moderate OSA cases.

C. Appliance-associated bite changes can cause food impaction, altered speech or chewing difficulty.

D. Efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be preferable to abandonment.

E. Poor appliance adherence warrants communication with the treating physician about alternatives.

CORRECT ANSWER A

Valid consent requires decision-making capacity, voluntariness and comprehension of material information. Additional revision detail: capacity, voluntariness (free of undue influence) and comprehension — verified, not assumed, for example by asking patients to repeat back what they heard. The other options describe different mechanisms, investigations or findings rather than three tests of valid consent. Consult Integrated notebook, page 196, §10.1 for the full context.

REFERENCE Integrated notebook, page 196, §10.1

QUESTION 0923 · §10.1 · Clinical interpretation SBA

A patient using a MAD reports persistent morning occlusal changes and reduced comfort. The clinical team records the following finding: capacity, voluntariness (free of undue influence) and comprehension — verified, not assumed, for example by asking patients to repeat back what they heard. Which concept best explains this observation?

A. Where noticed, symptoms of bite change are

B. Direction of force

C. Possible dental side effects, before starting therapy

D. Position of professional bodies

E. Three tests of valid consent

CORRECT ANSWER E

This finding most directly describes Three tests of valid consent: capacity, voluntariness (free of undue influence) and comprehension — verified, not assumed, for example by asking patients to repeat back what they heard. Distractor distinctions: A refers instead to gaps and food impaction; occasional lisping when talking; and — rarely — difficulty chewing on the back teeth or tearing food; B refers instead to appliances produce distally directed forces on the upper dentition and forwardly directed forces on the lower teeth; C refers instead to at minimum the chance of reduction in overjet and over- bite leading to a suboptimal occlusion, with fewer occlusal contact p; D refers instead to for most people the change in bite is not annoying, is not a risk to oral health, and does not require further dental treatme.

REFERENCE Integrated notebook, page 196, §10.1

QUESTION 0924 · §10.6 · Clinical decision-making SBA

After an appliance is discontinued, minor occlusal change is detected. What is the preferred initial approach in the notebook?

A. Immediate full-mouth reconstruction for every small change

B. Restart extreme protrusion

C. Ignore patient preference

D. No record of the issue

E. Review, allow time and consider conservative measures before irreversible correction

CORRECT ANSWER E

The source advises that some changes may settle after discontinuation and exercises; irreversible intervention should not be rushed.

REFERENCE Integrated notebook, Topic 10, §10.6

QUESTION 0925 · §10.2 · Applied clinical SBA

An adult with new TMJ discomfort returns after increasing mandibular protrusion. Which of the following statements about CPAP is not innocent is most accurate?

A. Long-term PAP interfaces and oral appliances can both affect occlusion through different forces.

B. Long-term mandibular advancement causes mainly dental rather than substantial skeletal positional changes.

C. Failure to restore habitual mandibular position after nightly appliance use may contribute to occlusal changes.

D. Mandibular advancement can shift maxillary teeth distally and mandibular teeth mesially over time.

E. Many appliance users have small occlusal changes that they do not notice or find troublesome.

CORRECT ANSWER A

Long-term PAP interfaces and oral appliances can both affect occlusion through different forces. Additional revision detail: both long-term PAP mask pressure and mandibular advancement therapy can influence dental or occlusal relationships, but through different mechanical mechanisms. The other options describe different mechanisms, investigations or findings rather than cpap is not innocent. Consult Integrated notebook, page 207, §10.2 for the full context.

REFERENCE Integrated notebook, page 207, §10.2

QUESTION 0926 · §10.2 · Clinical interpretation SBA

A dentist is discussing material risks and reasonable alternatives before appliance delivery. The clinical team records the following finding: CPAP also causes retroclination of the upper incisors, from posteriorly directed forces on the anterior maxilla by a tightly fitted nasal mask, and long-term use of both CPAP and appliances produces significant dental…. Which concept best explains this observation?

A. Position of professional bodies

B. Frequency

C. CPAP is not innocent

D. Direction of force

E. Mechanism

CORRECT ANSWER C

This finding most directly describes CPAP is not innocent: CPAP also causes retroclination of the upper incisors, from posteriorly directed forces on the anterior maxilla by a tightly fitted nasal mask, and long-term use of both CPAP and appliances produces significant dental…. Distractor distinctions: A refers instead to for most people the change in bite is not annoying, is not a risk to oral health, and does not require further dental treatme; B refers instead to ~80% of nightly users over 7 years experience change; 85% detectable on models and cephalograms (some bites do not change); D refers instead to appliances produce distally directed forces on the upper dentition and forwardly directed forces on the lower teeth; E refers instead to occlusal changes occur because of a failure to reposition the mandible during the day following use of the device, rather tha.

REFERENCE Integrated notebook, page 207, §10.2

QUESTION 0927 · §10.1 · Clinical decision-making SBA

A patient has capacity and freely consents but does not know that CPAP and no treatment are alternatives. What is missing?

A. An ESS score

B. A physical examination

C. Voluntariness

D. Capacity

E. Adequate disclosure of relevant alternatives and their consequences

CORRECT ANSWER E

An informed decision requires disclosure and understanding of the condition, proposed care, alternatives including no treatment, and benefits/risks.

REFERENCE Integrated notebook, Topic 10, §10.1

QUESTION 0928 · §10.3 · Applied clinical SBA

At an annual OAT review, a patient notices a reduction in overjet and altered occlusal contacts. Which of the following statements about corollary is most accurate?

A. Hypersalivation and dry mouth can occur during initial adaptation to oral appliances.

B. A morning occlusal guide supports recovery toward the patient's original bite.

C. Risk increases with greater advancement, with levels exceeding 50% of maximum often correlating with significantly higher risk.

D. A smaller protrusive dose may be sufficient in some mild-to-moderate OSA cases.

E. Many patients tolerate gradual occlusal change, but the risk still requires informed consent and monitoring.

CORRECT ANSWER D

A smaller protrusive dose may be sufficient in some mild-to-moderate OSA cases. Additional revision detail: an appliance is already effective at a less protrusive position in mild and moderate OSA, and therefore causes less severe dental side effects. The other options describe different mechanisms, investigations or findings rather than corollary. Consult Integrated notebook, page 199, §10.3 for the full context.

REFERENCE Integrated notebook, page 199, §10.3

QUESTION 0929 · §10.3 · Clinical interpretation SBA

An adult with new TMJ discomfort returns after increasing mandibular protrusion. The clinical team records the following finding: an appliance is already effective at a less protrusive position in mild and moderate OSA, and therefore causes less severe dental side effects. Which concept best explains this observation?

A. Frequency

B. Dry mouth

C. Corollary

D. Soft tissues

E. Factors influencing severity

CORRECT ANSWER C

This finding most directly describes Corollary: an appliance is already effective at a less protrusive position in mild and moderate OSA, and therefore causes less severe dental side effects. Distractor distinctions: A refers instead to ~80% of nightly users over 7 years experience change; 85% detectable on models and cephalograms (some bites do not change); B refers instead to poor nasal breathing → mouth breathing; vertical dimension too large making lip seal difficult; D refers instead to hypersalivation or dry mouth; the intended airway enlargement by advancing the mandible and repositioning the tongue; E refers instead to risk increases with greater advancement, with levels exceeding 50% of maximum often correlating with significantly higher ris.

REFERENCE Integrated notebook, page 199, §10.3

QUESTION 0930 · §10.5 · Clinical decision-making SBA

A device repeatedly traumatizes the gingiva while the patient's AHI is improved. What is appropriate?

A. Remove all retention permanently

B. Assume dry mouth is the only cause

C. Relieve the source of trauma and reassess fit rather than accepting tissue injury as a necessary cost

D. Ignore trauma whenever AHI falls

E. Increase wear without adjustment

CORRECT ANSWER C

Soft tissue irritation is addressed by correcting appliance contours or fit, hygiene and follow-up while preserving therapeutic function.

REFERENCE Integrated notebook, Topic 10, §10.5

QUESTION 0931 · §10.4 · Applied clinical SBA

A patient starting long-term MAD therapy asks about bite change and jaw discomfort. Which of the following statements about perform slowly and gently is most accurate?

A. Thermoplastic appliances may be fabricated using beads or heat-moulded prefabricated sheets.

B. A morning occlusal guide supports recovery toward the patient's original bite.

C. Gentle morning jaw exercises may relieve post-appliance muscular stiffness.

D. Patients should use their morning occlusal guide daily and report persistent misfit.

E. Gentle opening and lateral jaw movements can help restore comfort after overnight appliance use.

CORRECT ANSWER E

Gentle opening and lateral jaw movements can help restore comfort after overnight appliance use. Additional revision detail: opening the mouth wide, moving the jaw left and right, resting the chin on a fist to let the jaw settle, and curling the tongue back to find the retruded position. The other options describe different mechanisms, investigations or findings rather than perform slowly and gently. Consult Integrated notebook, page 210, §10.4 for the full context.

REFERENCE Integrated notebook, page 210, §10.4

QUESTION 0932 · §10.4 · Clinical interpretation SBA

At an annual OAT review, a patient notices a reduction in overjet and altered occlusal contacts. The clinical team records the following finding: opening the mouth wide, moving the jaw left and right, resting the chin on a fist to let the jaw settle, and curling the tongue back to find the retruded position. Which concept best explains this observation?

A. Monitors progress

B. Two fabrication types

C. Exercises the jaw muscles

D. Perform slowly and gently

E. Positions the condyles

CORRECT ANSWER D

This finding most directly describes Perform slowly and gently: opening the mouth wide, moving the jaw left and right, resting the chin on a fist to let the jaw settle, and curling the tongue back to find the retruded position. Distractor distinctions: A refers instead to the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical poin; B refers instead to those using thermoacrylic beads, and those using a prefabricated sheet (AM aligner) placed in hot water, moulded around the t; C refers instead to applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear; E refers instead to assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort.

REFERENCE Integrated notebook, page 210, §10.4

QUESTION 0933 · §10.5 · Applied clinical SBA

A dentist is discussing material risks and reasonable alternatives before appliance delivery. Which of the following statements about regular follow-up is most accurate?

A. Regular dental reviews detect and manage oral appliance side effects and occlusal changes.

B. A qualified dentist is essential for monitoring side effects and making necessary adjustments to the appliance or plan.

C. Soft tissue irritation includes screw mechanism or rough surface rubbing tongue or cheek.

D. Nasal obstruction and excessive vertical opening may promote mouth breathing with a MAD.

E. Palliative care includes supportive care to manage discomfort, especially during the initial adaptation period.

CORRECT ANSWER A

Regular dental reviews detect and manage oral appliance side effects and occlusal changes. Additional revision detail: every 6 months to a year, to evaluate for and manage side effects appropriately, with documentation of presence, management and resolution, and communication to the general dentist and medical provider. The other options describe different mechanisms, investigations or findings rather than regular follow-up. Consult Integrated notebook, page 211, §10.5 for the full context.

REFERENCE Integrated notebook, page 211, §10.5

QUESTION 0934 · §10.5 · Clinical interpretation SBA

A patient using a MAD reports persistent morning occlusal changes and reduced comfort. The clinical team records the following finding: every 6 months to a year, to evaluate for and manage side effects appropriately, with documentation of presence, management and resolution, and communication to the general dentist and medical provider. Which concept best explains this observation?

A. Regular follow-up

B. Minor bite change

C. Palliative care

D. Bilateral jaw pain

E. Patient education

CORRECT ANSWER A

This finding most directly describes Regular follow-up: every 6 months to a year, to evaluate for and manage side effects appropriately, with documentation of presence, management and resolution, and communication to the general dentist and medical pro- vider. Distractor distinctions: B refers instead to mechanical forces causing gradual shifts. → jaw exercises, passive exercises, morning occlusal guide; adjust the appliance or; C refers instead to supportive care to manage discomfort, especially during the initial adaptation period; D refers instead to advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed); E refers instead to reassure that symptoms are likely temporary and manageable.

REFERENCE Integrated notebook, page 211, §10.5

QUESTION 0935 · §10.6 · Applied clinical SBA

At an annual OAT review, a patient notices a reduction in overjet and altered occlusal contacts. Which of the following statements about device and settings is most accurate?

A. Poor appliance adherence warrants communication with the treating physician about alternatives.

B. Qualified dentists should review oral appliance therapy regularly for adherence, effectiveness and side effects.

C. Definitive correction of occlusal changes is usually deferred until appliance therapy is stopped or altered.

D. Efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be preferable to abandonment.

E. If appliance therapy is discontinued, the physician should arrange alternative OSA treatment or discuss the risks.

CORRECT ANSWER D

Efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be preferable to abandonment. The other options describe different mechanisms, investigations or findings rather than device and settings. Consult Integrated notebook, page 212, §10.6 for the full context.

REFERENCE Integrated notebook, page 212, §10.6

QUESTION 0936 · §10.6 · Clinical interpretation SBA

An adult with new TMJ discomfort returns after increasing mandibular protrusion. The clinical team records the following finding: efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be preferable to abandonment. Which concept best explains this observation?

A. Device and settings

B. Where treatment stops

C. Occlusal change

D. Discuss with the patient and record the discussion

E. Regular follow-up

CORRECT ANSWER A

This finding most directly describes Device and settings: efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be preferable to abandonment. Distractor distinctions: B refers instead to notify the medical provider, and refer non-compliant patients back to consider alternatives, since untreated or subtherapeuti; C refers instead to OJ ~1.9 mm, OB ~2.3 mm (average ~2.5 mm); ~⅓ exceed 1 mm; at 17 y median −1.1 / −1.6 mm; ~80% of 7-year nightly users; 85% de; D refers instead to the conversation is only protective if documented; E refers instead to monitoring by qualified dentists is necessary to survey for dental side effects and occlusal change, and is essential for max.

REFERENCE Integrated notebook, page 212, §10.6

QUESTION 0937 · §10.2 · Applied clinical SBA

A patient using a MAD reports persistent morning occlusal changes and reduced comfort. Which of the following statements about direction of force is most accurate?

A. Appliance-associated bite changes can cause food impaction, altered speech or chewing difficulty.

B. Many patients tolerate gradual occlusal change, but the risk still requires informed consent and monitoring.

C. Occlusal change is a recognised long-term risk of mandibular advancement therapy.

D. Mandibular advancement can shift maxillary teeth distally and mandibular teeth mesially over time.

E. Long-term nightly mandibular advancement frequently produces detectable occlusal changes.

CORRECT ANSWER D

Mandibular advancement can shift maxillary teeth distally and mandibular teeth mesially over time. Additional revision detail: appliances produce distally directed forces on the upper dentition and forwardly directed forces on the lower teeth. The other options describe different mechanisms, investigations or findings rather than direction of force. Consult Integrated notebook, page 207, §10.2 for the full context.

REFERENCE Integrated notebook, page 207, §10.2

QUESTION 0938 · §10.2 · Clinical interpretation SBA

A patient starting long-term MAD therapy asks about bite change and jaw discomfort. The clinical team records the following finding: appliances produce distally directed forces on the upper dentition and forwardly directed forces on the lower teeth. Which concept best explains this observation?

A. Where noticed, symptoms of bite change are

B. Frequency

C. Direction of force

D. CPAP is not innocent

E. Position of professional bodies

CORRECT ANSWER C

This finding most directly describes Direction of force: appliances produce distally directed forces on the upper dentition and forwardly directed forces on the lower teeth. Distractor distinctions: A refers instead to gaps and food impaction; occasional lisping when talking; and — rarely — difficulty chewing on the back teeth or tearing food; B refers instead to ~80% of nightly users over 7 years experience change; 85% detectable on models and cephalograms (some bites do not change); D refers instead to CPAP also causes retroclination of the upper incisors, from posteriorly directed forces on the anterior maxilla by a tightly; E refers instead to for most people the change in bite is not annoying, is not a risk to oral health, and does not require further dental treatme.

REFERENCE Integrated notebook, page 207, §10.2

QUESTION 0939 · §10.3 · Applied clinical SBA

An adult with new TMJ discomfort returns after increasing mandibular protrusion. Which of the following statements about factors influencing severity is most accurate?

A. Risk increases with greater advancement, with levels exceeding 50% of maximum often correlating with significantly higher risk.

B. Hypersalivation and dry mouth can occur during initial adaptation to oral appliances.

C. A smaller protrusive dose may be sufficient in some mild-to-moderate OSA cases.

D. Patients should use their morning occlusal guide daily and report persistent misfit.

E. A morning occlusal guide supports recovery toward the patient's original bite.

CORRECT ANSWER A

Risk increases with greater advancement, with levels exceeding 50% of maximum often correlating with significantly higher risk. The other options describe different mechanisms, investigations or findings rather than factors influencing severity. Consult Integrated notebook, page 199, §10.3 for the full context.

REFERENCE Integrated notebook, page 199, §10.3

QUESTION 0940 · §10.3 · Clinical interpretation SBA

A dentist is discussing material risks and reasonable alternatives before appliance delivery. The clinical team records the following finding: risk increases with greater advancement, with levels exceeding 50% of maximum often correlating with significantly higher risk. Which concept best explains this observation?

A. Factors influencing severity

B. Three tests of valid consent

C. Soft tissues

D. Corollary

E. Patient-specific factors

CORRECT ANSWER A

This finding most directly describes Factors influencing severity: risk increases with greater advancement, with levels exceeding 50% of maximum often correlating with significantly higher risk. Distractor distinctions: B refers instead to capacity, voluntariness (free of undue influence) and comprehension — verified, not assumed, for example by asking patients t; C refers instead to hypersalivation or dry mouth; the intended airway enlargement by advancing the mandible and repositioning the tongue; D refers instead to an appliance is already effective at a less protrusive position in mild and moderate OSA, and therefore causes less severe de; E refers instead to overall health, tolerance for side effects and commitment to therapy.

REFERENCE Integrated notebook, page 199, §10.3

QUESTION 0941 · §10.4 · Applied clinical SBA

A patient starting long-term MAD therapy asks about bite change and jaw discomfort. Which of the following statements about two fabrication types is most accurate?

A. A morning occlusal guide supports recovery toward the patient's original bite.

B. Gentle opening and lateral jaw movements can help restore comfort after overnight appliance use.

C. A morning occlusal guide helps the mandible return toward habitual intercuspation after appliance removal.

D. Thermoplastic appliances may be fabricated using beads or heat-moulded prefabricated sheets.

E. Morning occlusal exercises may help recover habitual mandibular position after overnight advancement.

CORRECT ANSWER D

Thermoplastic appliances may be fabricated using beads or heat-moulded prefabricated sheets. Additional revision detail: those using thermoacrylic beads, and those using a prefabricated sheet (AM aligner) placed in hot water, moulded around the teeth, with the patient asked to bite in centric occlusion. The other options describe different mechanisms, investigations or findings rather than two fabrication types. Consult Integrated notebook, page 209, §10.4 for the full context.

REFERENCE Integrated notebook, page 209, §10.4

QUESTION 0942 · §10.4 · Clinical interpretation SBA

At an annual OAT review, a patient notices a reduction in overjet and altered occlusal contacts. The clinical team records the following finding: those using thermoacrylic beads, and those using a prefabricated sheet (AM aligner) placed in hot water, moulded around the teeth, with the patient asked to bite in centric occlusion. Which concept best explains this observation?

A. Monitors progress

B. Exercises the jaw muscles

C. Two fabrication types

D. Perform slowly and gently

E. Positions the condyles

CORRECT ANSWER C

This finding most directly describes Two fabrication types: those using thermoacrylic beads, and those using a prefabricated sheet (AM aligner) placed in hot water, moulded around the teeth, with the patient asked to bite in centric occlusion. Distractor distinctions: A refers instead to the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical poin; B refers instead to applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear; D refers instead to opening the mouth wide, moving the jaw left and right, resting the chin on a fist to let the jaw settle, and curling the tong; E refers instead to assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort.

REFERENCE Integrated notebook, page 209, §10.4

QUESTION 0943 · §10.5 · Applied clinical SBA

A dentist is discussing material risks and reasonable alternatives before appliance delivery. Which of the following statements about transient morning jaw pain is most accurate?

A. Palliative care includes supportive care to manage discomfort, especially during the initial adaptation period.

B. Transient morning jaw discomfort may result from overnight mandibular advancement.

C. Bilateral jaw pain includes advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed).

D. Transient hypersalivation often improves during early appliance adaptation.

E. Progressive occlusal changes arise from sustained dental forces during long-term MAD use.

CORRECT ANSWER B

Transient morning jaw discomfort may result from overnight mandibular advancement. Additional revision detail: muscles adjusting to the forward mandible. → jaw exercises, massage, heat; morning occlusal guide. The other options describe different mechanisms, investigations or findings rather than transient morning jaw pain. Consult Integrated notebook, page 200, §10.5 for the full context.

REFERENCE Integrated notebook, page 200, §10.5

QUESTION 0944 · §10.5 · Clinical interpretation SBA

A patient using a MAD reports persistent morning occlusal changes and reduced comfort. The clinical team records the following finding: muscles adjusting to the forward mandible. → jaw exercises, massage, heat; morning occlusal guide. Which concept best explains this observation?

A. Soft tissue irritation

B. Transient morning jaw pain

C. Bilateral jaw pain

D. Unilateral jaw pain

E. Monitoring and adjustment

CORRECT ANSWER B

This finding most directly describes Transient morning jaw pain: muscles adjusting to the forward mandible. → jaw exercises, massage, heat; morning occlusal guide. Distractor distinctions: A refers instead to screw mechanism or rough surface rubbing tongue or cheek; cheek or lip biting from bite change. → polish; discontinue until h; C refers instead to advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed); D refers instead to prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines; E refers instead to a qualified dentist is essential for monitoring side effects and making necessary adjustments to the appliance or plan.

REFERENCE Integrated notebook, page 200, §10.5

QUESTION 0945 · §10.6 · Applied clinical SBA

At an annual OAT review, a patient notices a reduction in overjet and altered occlusal contacts. Which of the following statements about where treatment stops is most accurate?

A. Definitive correction of occlusal changes is usually deferred until appliance therapy is stopped or altered.

B. Qualified dentists should review oral appliance therapy regularly for adherence, effectiveness and side effects.

C. Some early appliance-associated bite changes improve after adjustment or cessation with monitored care.

D. Poor appliance adherence warrants communication with the treating physician about alternatives.

E. Efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be preferable to abandonment.

CORRECT ANSWER D

Poor appliance adherence warrants communication with the treating physician about alternatives. Additional revision detail: notify the medical provider, and refer non-compliant patients back to consider alternatives, since untreated or subtherapeutic OSA carries long-term risk. The other options describe different mechanisms, investigations or findings rather than where treatment stops. Consult Integrated notebook, page 200, §10.6 for the full context.

REFERENCE Integrated notebook, page 200, §10.6

QUESTION 0946 · §10.6 · Clinical interpretation SBA

An adult with new TMJ discomfort returns after increasing mandibular protrusion. The clinical team records the following finding: notify the medical provider, and refer non-compliant patients back to consider alternatives, since untreated or subtherapeutic OSA carries long-term risk. Which concept best explains this observation?

A. Patient-specific factors

B. Device and settings

C. Regular follow-up

D. Occlusal change

E. Where treatment stops

CORRECT ANSWER E

This finding most directly describes Where treatment stops: notify the medical provider, and refer non-compliant patients back to consider alternatives, since untreated or subtherapeutic OSA carries long-term risk. Distractor distinctions: A refers instead to overall health, tolerance for side effects and commitment to therapy; B refers instead to efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be pr; C refers instead to monitoring by qualified dentists is necessary to survey for dental side effects and occlusal change, and is essential for max; D refers instead to OJ ~1.9 mm, OB ~2.3 mm (average ~2.5 mm); ~⅓ exceed 1 mm; at 17 y median −1.1 / −1.6 mm; ~80% of 7-year nightly users; 85% de.

REFERENCE Integrated notebook, page 200, §10.6

QUESTION 0947 · §10.2 · Applied clinical SBA

A patient using a MAD reports persistent morning occlusal changes and reduced comfort. Which of the following statements about where noticed, symptoms of bite change are is most accurate?

A. Many patients tolerate gradual occlusal change, but the risk still requires informed consent and monitoring.

B. Appliance-associated bite changes can cause food impaction, altered speech or chewing difficulty.

C. Mandibular advancement can shift maxillary teeth distally and mandibular teeth mesially over time.

D. Long-term mandibular advancement causes mainly dental rather than substantial skeletal positional changes.

E. Long-term nightly mandibular advancement frequently produces detectable occlusal changes.

CORRECT ANSWER B

Appliance-associated bite changes can cause food impaction, altered speech or chewing difficulty. Additional revision detail: gaps and food impaction; occasional lisping when talking; and — rarely — difficulty chewing on the back teeth or tearing food with the front teeth. The other options describe different mechanisms, investigations or findings rather than where noticed, symptoms of bite change are. Consult Integrated notebook, page 208, §10.2 for the full context.

REFERENCE Integrated notebook, page 208, §10.2

QUESTION 0948 · §10.2 · Clinical interpretation SBA

A patient starting long-term MAD therapy asks about bite change and jaw discomfort. The clinical team records the following finding: gaps and food impaction; occasional lisping when talking; and — rarely — difficulty chewing on the back teeth or tearing food with the front teeth. Which concept best explains this observation?

A. Position of professional bodies

B. What patients actually experience

C. Direction of force

D. Where noticed, symptoms of bite change are

E. Long term (moderate–severe, ongoing)

CORRECT ANSWER D

This finding most directly describes Where noticed, symptoms of bite change are: gaps and food impaction; occasional lisping when talking; and — rarely — difficulty chewing on the back teeth or tearing food with the front teeth. Distractor distinctions: A refers instead to for most people the change in bite is not annoying, is not a risk to oral health, and does not require further dental treatme; B refers instead to Mostly changes are minor and unnoticed; only a minority notice. Most are unaware, untroubled, and do not want to stop. Where; C refers instead to appliances produce distally directed forces on the upper dentition and forwardly directed forces on the lower teeth; E refers instead to jaw pain ~34%; TMJ restriction or noise; bite change 14–26%; damage to teeth or dental work; appliance issues.

REFERENCE Integrated notebook, page 208, §10.2

QUESTION 0949 · §10.4 · Applied clinical SBA

A dentist is discussing material risks and reasonable alternatives before appliance delivery. Which of the following statements about exercises the jaw muscles is most accurate?

A. Gentle morning jaw exercises may relieve post-appliance muscular stiffness.

B. A morning occlusal guide supports recovery toward the patient's original bite.

C. Patients should use their morning occlusal guide daily and report persistent misfit.

D. Morning occlusal exercises may help recover habitual mandibular position after overnight advancement.

E. A morning occlusal guide helps the mandible return toward habitual intercuspation after appliance removal.

CORRECT ANSWER A

Gentle morning jaw exercises may relieve post-appliance muscular stiffness. Additional revision detail: applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear. The other options describe different mechanisms, investigations or findings rather than exercises the jaw muscles. Consult Integrated notebook, page 209, §10.4 for the full context.

REFERENCE Integrated notebook, page 209, §10.4

QUESTION 0950 · §10.4 · Clinical interpretation SBA

A patient using a MAD reports persistent morning occlusal changes and reduced comfort. The clinical team records the following finding: applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear. Which concept best explains this observation?

A. Realigns the bite

B. Wear for 15–20 minutes in the morning after waking

C. Exercises the jaw muscles

D. Exercises increase contact between the teeth

E. Positions the condyles

CORRECT ANSWER C

This finding most directly describes Exercises the jaw muscles: applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear. Distractor distinctions: A refers instead to custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation; B refers instead to some protocols specify around 10 minutes; D refers instead to good in the short term, with limited long-term evidence; E refers instead to assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort.

REFERENCE Integrated notebook, page 209, §10.4

QUESTION 0951 · §10.5 · Applied clinical SBA

At an annual OAT review, a patient notices a reduction in overjet and altered occlusal contacts. Which of the following statements about palliative care is most accurate?

A. Transient hypersalivation often improves during early appliance adaptation.

B. Palliative care includes supportive care to manage discomfort, especially during the initial adaptation period.

C. Nasal obstruction and excessive vertical opening may promote mouth breathing with a MAD.

D. Bilateral jaw pain includes advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed).

E. Unilateral jaw pain includes prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines.

CORRECT ANSWER B

Palliative care includes supportive care to manage discomfort, especially during the initial adaptation period. Additional revision detail: supportive care to manage discomfort, especially during the initial adaptation period. The other options describe different mechanisms, investigations or findings rather than palliative care. Consult Integrated notebook, page 211, §10.5 for the full context.

REFERENCE Integrated notebook, page 211, §10.5

QUESTION 0952 · §10.5 · Clinical interpretation SBA

An adult with new TMJ discomfort returns after increasing mandibular protrusion. The clinical team records the following finding: supportive care to manage discomfort, especially during the initial adaptation period. Which concept best explains this observation?

A. Patient education

B. Minor bite change

C. Regular follow-up

D. Palliative care

E. Bilateral jaw pain

CORRECT ANSWER D

This finding most directly describes Palliative care: supportive care to manage discomfort, especially during the initial adaptation period. Distractor distinctions: A refers instead to reassure that symptoms are likely temporary and manageable; B refers instead to mechanical forces causing gradual shifts. → jaw exercises, passive exercises, morning occlusal guide; adjust the appliance or; C refers instead to monitoring by qualified dentists is necessary to survey for dental side effects and occlusal change, and is essential for max; E refers instead to advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed).

REFERENCE Integrated notebook, page 211, §10.5

QUESTION 0953 · §10.6 · Applied clinical SBA

A patient using a MAD reports persistent morning occlusal changes and reduced comfort. Which of the following statements about discuss with the patient and record the discussion is most accurate?

A. Patient-specific factors includes overall health, tolerance for side effects and commitment to therapy.

B. The conversation is only protective if documented.

C. Regular reviews is considered at least 6-monthly for the first year, then annually.

D. Some early appliance-associated bite changes improve after adjustment or cessation with monitored care.

E. Definitive correction of occlusal changes is usually deferred until appliance therapy is stopped or altered.

CORRECT ANSWER B

The conversation is only protective if documented. The other options describe different mechanisms, investigations or findings rather than discuss with the patient and record the discussion. Consult Integrated notebook, page 201, §10.6 for the full context.

REFERENCE Integrated notebook, page 201, §10.6

QUESTION 0954 · §10.6 · Clinical interpretation SBA

A patient starting long-term MAD therapy asks about bite change and jaw discomfort. The clinical team records the following finding: the conversation is only protective if documented. Which concept best explains this observation?

A. Occlusal change the patient finds unacceptable

B. Discuss with the patient and record the discussion

C. Leave it alone within the first 12–18 months

D. Remember the rule from 10.5

E. Patient-specific factors

CORRECT ANSWER B

This finding most directly describes Discuss with the patient and record the discussion: the conversation is only protective if documented. Distractor distinctions: A refers instead to treatment should be discontinued only if the patient accepts another form of therapy from their attending physician, or under; C refers instead to most will self-correct. This is why it is important to review vulnerable patients early, so that this option remains availabl; D refers instead to definitive correction should be undertaken only once appliance therapy has been discontinued and the patient is on another OS; E refers instead to overall health, tolerance for side effects and commitment to therapy.

REFERENCE Integrated notebook, page 201, §10.6

QUESTION 0955 · §10.4 · Applied clinical SBA

A patient using a MAD reports persistent morning occlusal changes and reduced comfort. Which of the following statements about realigns the bite is most accurate?

A. Gentle morning jaw exercises may relieve post-appliance muscular stiffness.

B. A morning occlusal guide supports recovery toward the patient's original bite.

C. Morning occlusal exercises may help recover habitual mandibular position after overnight advancement.

D. Patients should use their morning occlusal guide daily and report persistent misfit.

E. A morning occlusal guide helps the mandible return toward habitual intercuspation after appliance removal.

CORRECT ANSWER E

A morning occlusal guide helps the mandible return toward habitual intercuspation after appliance removal. Additional revision detail: custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation. The other options describe different mechanisms, investigations or findings rather than realigns the bite. Consult Integrated notebook, page 209, §10.4 for the full context.

REFERENCE Integrated notebook, page 209, §10.4

QUESTION 0956 · §10.4 · Clinical interpretation SBA

A patient starting long-term MAD therapy asks about bite change and jaw discomfort. The clinical team records the following finding: custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation. Which concept best explains this observation?

A. Exercises the jaw muscles

B. Realigns the bite

C. Exercises increase contact between the teeth

D. Wear for 15–20 minutes in the morning after waking

E. Positions the condyles

CORRECT ANSWER B

This finding most directly describes Realigns the bite: custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation. Distractor distinctions: A refers instead to applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear; C refers instead to good in the short term, with limited long-term evidence; D refers instead to some protocols specify around 10 minutes; E refers instead to assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort.

REFERENCE Integrated notebook, page 209, §10.4

QUESTION 0957 · §10.5 · Applied clinical SBA

An adult with new TMJ discomfort returns after increasing mandibular protrusion. Which of the following statements about excessive salivation is most accurate?

A. Transient hypersalivation often improves during early appliance adaptation.

B. Bilateral jaw pain includes advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed).

C. Unilateral jaw pain includes prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines.

D. Transient morning jaw discomfort may result from overnight mandibular advancement.

E. Palliative care includes supportive care to manage discomfort, especially during the initial adaptation period.

CORRECT ANSWER A

Transient hypersalivation often improves during early appliance adaptation. Additional revision detail: the appliance itself. → towel on the pillow; reassurance, resolving in 2–3 weeks. The other options describe different mechanisms, investigations or findings rather than excessive salivation. Consult Integrated notebook, page 200, §10.5 for the full context.

REFERENCE Integrated notebook, page 200, §10.5

QUESTION 0958 · §10.5 · Clinical interpretation SBA

A dentist is discussing material risks and reasonable alternatives before appliance delivery. The clinical team records the following finding: the appliance itself. → towel on the pillow; reassurance, resolving in 2–3 weeks. Which concept best explains this observation?

A. Soft tissue irritation

B. Bilateral jaw pain

C. Minor bite change

D. Unilateral jaw pain

E. Excessive salivation

CORRECT ANSWER E

This finding most directly describes Excessive salivation: the appliance itself. → towel on the pillow; reassurance, resolving in 2–3 weeks. Distractor distinctions: A refers instead to screw mechanism or rough surface rubbing tongue or cheek; cheek or lip biting from bite change. → polish; discontinue until h; B refers instead to advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed); C refers instead to mechanical forces causing gradual shifts. → jaw exercises, passive exercises, morning occlusal guide; adjust the appliance or; D refers instead to prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines.

REFERENCE Integrated notebook, page 200, §10.5

QUESTION 0959 · §10.6 · Applied clinical SBA

A patient starting long-term MAD therapy asks about bite change and jaw discomfort. Which of the following statements about regular follow-up is most accurate?

A. Qualified dentists should review oral appliance therapy regularly for adherence, effectiveness and side effects.

B. Poor appliance adherence warrants communication with the treating physician about alternatives.

C. If appliance therapy is discontinued, the physician should arrange alternative OSA treatment or discuss the risks.

D. Efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be preferable to abandonment.

E. Definitive correction of occlusal changes is usually deferred until appliance therapy is stopped or altered.

CORRECT ANSWER A

Qualified dentists should review oral appliance therapy regularly for adherence, effectiveness and side effects. Additional revision detail: monitoring by qualified dentists is necessary to survey for dental side effects and occlusal change, and is essential for maximising adherence and effectiveness. The other options describe different mechanisms, investigations or findings rather than regular follow-up. Consult Integrated notebook, page 212, §10.6 for the full context.

REFERENCE Integrated notebook, page 212, §10.6

QUESTION 0960 · §10.6 · Clinical interpretation SBA

At an annual OAT review, a patient notices a reduction in overjet and altered occlusal contacts. The clinical team records the following finding: monitoring by qualified dentists is necessary to survey for dental side effects and occlusal change, and is essential for maximising adherence and effectiveness. Which concept best explains this observation?

A. Regular follow-up

B. Occlusal change

C. Regular reviews

D. Where treatment stops

E. Device and settings

CORRECT ANSWER A

This finding most directly describes Regular follow-up: monitoring by qualified dentists is necessary to survey for dental side effects and occlusal change, and is essential for maximising adherence and effectiveness. Distractor distinctions: B refers instead to OJ ~1.9 mm, OB ~2.3 mm (average ~2.5 mm); ~⅓ exceed 1 mm; at 17 y median −1.1 / −1.6 mm; ~80% of 7-year nightly users; 85% de; C refers instead to at least 6-monthly for the first year, then annually; D refers instead to notify the medical provider, and refer non-compliant patients back to consider alternatives, since untreated or subtherapeuti; E refers instead to efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be pr.

REFERENCE Integrated notebook, page 212, §10.6

QUESTION 0961 · §10.2 · Applied clinical SBA

A dentist is discussing material risks and reasonable alternatives before appliance delivery. Which of the following statements about position of professional bodies is most accurate?

A. Mandibular advancement can shift maxillary teeth distally and mandibular teeth mesially over time.

B. Many patients tolerate gradual occlusal change, but the risk still requires informed consent and monitoring.

C. Long-term mandibular advancement causes mainly dental rather than substantial skeletal positional changes.

D. Occlusal change is a recognised long-term risk of mandibular advancement therapy.

E. Long-term nightly mandibular advancement frequently produces detectable occlusal changes.

CORRECT ANSWER B

Many patients tolerate gradual occlusal change, but the risk still requires informed consent and monitoring. Additional revision detail: for most people the change in bite is not annoying, is not a risk to oral health, and does not require further dental treatment. The other options describe different mechanisms, investigations or findings rather than position of professional bodies. Consult Integrated notebook, page 208, §10.2 for the full context.

REFERENCE Integrated notebook, page 208, §10.2

QUESTION 0962 · §10.2 · Clinical interpretation SBA

A patient using a MAD reports persistent morning occlusal changes and reduced comfort. The clinical team records the following finding: for most people the change in bite is not annoying, is not a risk to oral health, and does not require further dental treatment. Which concept best explains this observation?

A. Direction of force

B. Long term (moderate–severe, ongoing)

C. Position of professional bodies

D. What patients actually experience

E. Where noticed, symptoms of bite change are

CORRECT ANSWER C

This finding most directly describes Position of professional bodies: for most people the change in bite is not annoying, is not a risk to oral health, and does not require further dental treatment. Distractor distinctions: A refers instead to appliances produce distally directed forces on the upper dentition and forwardly directed forces on the lower teeth; B refers instead to jaw pain ~34%; TMJ restriction or noise; bite change 14–26%; damage to teeth or dental work; appliance issues; D refers instead to Mostly changes are minor and unnoticed; only a minority notice. Most are unaware, untroubled, and do not want to stop. Where; E refers instead to gaps and food impaction; occasional lisping when talking; and — rarely — difficulty chewing on the back teeth or tearing food.

REFERENCE Integrated notebook, page 208, §10.2

QUESTION 0963 · §10.4 · Applied clinical SBA

An adult with new TMJ discomfort returns after increasing mandibular protrusion. Which of the following statements about monitors progress is most accurate?

A. Gentle morning jaw exercises may relieve post-appliance muscular stiffness.

B. Gentle opening and lateral jaw movements can help restore comfort after overnight appliance use.

C. Patients should use their morning occlusal guide daily and report persistent misfit.

D. A morning occlusal guide helps the mandible return toward habitual intercuspation after appliance removal.

E. Morning occlusal exercises may help recover habitual mandibular position after overnight advancement.

CORRECT ANSWER C

Patients should use their morning occlusal guide daily and report persistent misfit. Additional revision detail: the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. The other options describe different mechanisms, investigations or findings rather than monitors progress. Consult Integrated notebook, page 209, §10.4 for the full context.

REFERENCE Integrated notebook, page 209, §10.4

QUESTION 0964 · §10.4 · Clinical interpretation SBA

A dentist is discussing material risks and reasonable alternatives before appliance delivery. The clinical team records the following finding: the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Which concept best explains this observation?

A. Positions the condyles

B. Monitors progress

C. Two fabrication types

D. How it works

E. Realigns the bite

CORRECT ANSWER B

This finding most directly describes Monitors progress: the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical points. Distractor distinctions: A refers instead to assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort; C refers instead to those using thermoacrylic beads, and those using a prefabricated sheet (AM aligner) placed in hot water, moulded around the t; D refers instead to realigns the bite toward original maximum intercuspation; exercises and stretches the muscles of mastication, tight after a n; E refers instead to custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation.

REFERENCE Integrated notebook, page 209, §10.4

QUESTION 0965 · §10.5 · Applied clinical SBA

A patient starting long-term MAD therapy asks about bite change and jaw discomfort. Which of the following statements about patient education is most accurate?

A. Reassure that symptoms are likely temporary and manageable.

B. Palliative care includes supportive care to manage discomfort, especially during the initial adaptation period.

C. Nasal obstruction and excessive vertical opening may promote mouth breathing with a MAD.

D. Unilateral jaw pain includes prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines.

E. Transient hypersalivation often improves during early appliance adaptation.

CORRECT ANSWER A

Reassure that symptoms are likely temporary and manageable. The other options describe different mechanisms, investigations or findings rather than patient education. Consult Integrated notebook, page 211, §10.5 for the full context.

REFERENCE Integrated notebook, page 211, §10.5

QUESTION 0966 · §10.5 · Clinical interpretation SBA

At an annual OAT review, a patient notices a reduction in overjet and altered occlusal contacts. The clinical team records the following finding: reassure that symptoms are likely temporary and manageable. Which concept best explains this observation?

A. Minor bite change

B. Regular follow-up

C. Bilateral jaw pain

D. Palliative care

E. Patient education

CORRECT ANSWER E

This finding most directly describes Patient education: reassure that symptoms are likely temporary and manageable. Distractor distinctions: A refers instead to mechanical forces causing gradual shifts. → jaw exercises, passive exercises, morning occlusal guide; adjust the appliance or; B refers instead to monitoring by qualified dentists is necessary to survey for dental side effects and occlusal change, and is essential for max; C refers instead to advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed); D refers instead to supportive care to manage discomfort, especially during the initial adaptation period.

REFERENCE Integrated notebook, page 211, §10.5

QUESTION 0967 · §10.6 · Applied clinical SBA

A dentist is discussing material risks and reasonable alternatives before appliance delivery. Which of the following statements about regular reviews is most accurate?

A. The conversation is only protective if documented.

B. If appliance therapy is discontinued, the physician should arrange alternative OSA treatment or discuss the risks.

C. Regular reviews is considered at least 6-monthly for the first year, then annually.

D. Patient-specific factors includes overall health, tolerance for side effects and commitment to therapy.

E. Some early appliance-associated bite changes improve after adjustment or cessation with monitored care.

CORRECT ANSWER C

Regular reviews is considered at least 6-monthly for the first year, then annually. Additional revision detail: at least 6-monthly for the first year, then annually. The other options describe different mechanisms, investigations or findings rather than regular reviews. Consult Integrated notebook, page 201, §10.6 for the full context.

REFERENCE Integrated notebook, page 201, §10.6

QUESTION 0968 · §10.6 · Clinical interpretation SBA

A patient using a MAD reports persistent morning occlusal changes and reduced comfort. The clinical team records the following finding: at least 6-monthly for the first year, then annually. Which concept best explains this observation?

A. Regular reviews

B. Device and settings

C. Occlusal change

D. Where treatment stops

E. Regular follow-up

CORRECT ANSWER A

This finding most directly describes Regular reviews: at least 6-monthly for the first year, then annually. Distractor distinctions: B refers instead to efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be pr; C refers instead to OJ ~1.9 mm, OB ~2.3 mm (average ~2.5 mm); ~⅓ exceed 1 mm; at 17 y median −1.1 / −1.6 mm; ~80% of 7-year nightly users; 85% de; D refers instead to notify the medical provider, and refer non-compliant patients back to consider alternatives, since untreated or subtherapeuti; E refers instead to monitoring by qualified dentists is necessary to survey for dental side effects and occlusal change, and is essential for max.

REFERENCE Integrated notebook, page 201, §10.6

QUESTION 0969 · §10.4 · Applied clinical SBA

A dentist is discussing material risks and reasonable alternatives before appliance delivery. Which of the following statements about positions the condyles is most accurate?

A. Gentle morning jaw exercises may relieve post-appliance muscular stiffness.

B. Patients should use their morning occlusal guide daily and report persistent misfit.

C. A morning occlusal guide helps the mandible return toward habitual intercuspation after appliance removal.

D. A morning occlusal guide supports recovery toward the patient's original bite.

E. Morning occlusal exercises may help recover habitual mandibular position after overnight advancement.

CORRECT ANSWER E

Morning occlusal exercises may help recover habitual mandibular position after overnight advancement. Additional revision detail: assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort. The other options describe different mechanisms, investigations or findings rather than positions the condyles. Consult Integrated notebook, page 209, §10.4 for the full context.

REFERENCE Integrated notebook, page 209, §10.4

QUESTION 0970 · §10.4 · Clinical interpretation SBA

A patient using a MAD reports persistent morning occlusal changes and reduced comfort. The clinical team records the following finding: assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort. Which concept best explains this observation?

A. Exercises increase contact between the teeth

B. Exercises the jaw muscles

C. Positions the condyles

D. Wear for 15–20 minutes in the morning after waking

E. Realigns the bite

CORRECT ANSWER C

This finding most directly describes Positions the condyles: assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort. Distractor distinctions: A refers instead to good in the short term, with limited long-term evidence; B refers instead to applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear; D refers instead to some protocols specify around 10 minutes; E refers instead to custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation.

REFERENCE Integrated notebook, page 209, §10.4

QUESTION 0971 · §10.5 · Applied clinical SBA

At an annual OAT review, a patient notices a reduction in overjet and altered occlusal contacts. Which of the following statements about soft tissue irritation is most accurate?

A. A qualified dentist is essential for monitoring side effects and making necessary adjustments to the appliance or plan.

B. Progressive occlusal changes arise from sustained dental forces during long-term MAD use.

C. Soft tissue irritation includes screw mechanism or rough surface rubbing tongue or cheek.

D. Regular dental reviews detect and manage oral appliance side effects and occlusal changes.

E. Transient morning jaw discomfort may result from overnight mandibular advancement.

CORRECT ANSWER C

Soft tissue irritation includes screw mechanism or rough surface rubbing tongue or cheek. Additional revision detail: screw mechanism or rough surface rubbing tongue or cheek; cheek or lip biting from bite change. → polish; discontinue until healed; change appliance; orthodontic wax. The other options describe different mechanisms, investigations or findings rather than soft tissue irritation. Consult Integrated notebook, page 200, §10.5 for the full context.

REFERENCE Integrated notebook, page 200, §10.5

QUESTION 0972 · §10.5 · Clinical interpretation SBA

An adult with new TMJ discomfort returns after increasing mandibular protrusion. The clinical team records the following finding: screw mechanism or rough surface rubbing tongue or cheek; cheek or lip biting from bite change. → polish; discontinue until healed; change appliance; orthodontic wax. Which concept best explains this observation?

A. Monitoring and adjustment

B. Soft tissue irritation

C. Bilateral jaw pain

D. Unilateral jaw pain

E. Excessive salivation

CORRECT ANSWER B

This finding most directly describes Soft tissue irritation: screw mechanism or rough surface rubbing tongue or cheek; cheek or lip biting from bite change. → polish; discontinue until healed; change appliance; orthodontic wax. Distractor distinctions: A refers instead to a qualified dentist is essential for monitoring side effects and making necessary adjustments to the appliance or plan; C refers instead to advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed); D refers instead to prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines; E refers instead to the appliance itself. → towel on the pillow; reassurance, resolving in 2–3 weeks.

REFERENCE Integrated notebook, page 200, §10.5

QUESTION 0973 · §10.6 · Applied clinical SBA

A patient using a MAD reports persistent morning occlusal changes and reduced comfort. Which of the following statements about occlusal change the patient finds unacceptable is most accurate?

A. If appliance therapy is discontinued, the physician should arrange alternative OSA treatment or discuss the risks.

B. Definitive correction of occlusal changes is usually deferred until appliance therapy is stopped or altered.

C. Occlusal change includes oJ ~1.9 mm, OB ~2.3 mm (average ~2.5 mm).

D. Poor appliance adherence warrants communication with the treating physician about alternatives.

E. Efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be preferable to abandonment.

CORRECT ANSWER A

If appliance therapy is discontinued, the physician should arrange alternative OSA treatment or discuss the risks. Additional revision detail: treatment should be discontinued only if the patient accepts another form of therapy from their attending physician, or understands the risks of discontinuation and their physician is informed. The other options describe different mechanisms, investigations or findings rather than occlusal change the patient finds unacceptable. Consult Integrated notebook, page 200, §10.6 for the full context.

REFERENCE Integrated notebook, page 200, §10.6

QUESTION 0974 · §10.6 · Clinical interpretation SBA

A patient starting long-term MAD therapy asks about bite change and jaw discomfort. The clinical team records the following finding: treatment should be discontinued only if the patient accepts another form of therapy from their attending physician, or understands the risks of discontinuation and their physician is informed. Which concept best explains this observation?

A. Leave it alone within the first 12–18 months

B. Occlusal change

C. Discuss with the patient and record the discussion

D. Occlusal change the patient finds unacceptable

E. Remember the rule from 10.5

CORRECT ANSWER D

This finding most directly describes Occlusal change the patient finds unacceptable: treatment should be discontinued only if the patient accepts another form of therapy from their attending physician, or understands the risks of discontinuation and their physician is informed. Distractor distinctions: A refers instead to most will self-correct. This is why it is important to review vulnerable patients early, so that this option remains availabl; B refers instead to OJ ~1.9 mm, OB ~2.3 mm (average ~2.5 mm); ~⅓ exceed 1 mm; at 17 y median −1.1 / −1.6 mm; ~80% of 7-year nightly users; 85% de; C refers instead to the conversation is only protective if documented; E refers instead to definitive correction should be undertaken only once appliance therapy has been discontinued and the patient is on another OS.

REFERENCE Integrated notebook, page 200, §10.6

QUESTION 0975 · §10.4 · Applied clinical SBA

A patient using a MAD reports persistent morning occlusal changes and reduced comfort. Which of the following statements about wear for 15–20 minutes in the morning after waking is most accurate?

A. Gentle morning jaw exercises may relieve post-appliance muscular stiffness.

B. Wear for 15–20 minutes in the morning after waking includes some protocols specify around 10 minutes.

C. A morning occlusal guide helps the mandible return toward habitual intercuspation after appliance removal.

D. Morning occlusal exercises may help recover habitual mandibular position after overnight advancement.

E. Exercises increase contact between the teeth includes good in the short term, with limited long-term evidence.

CORRECT ANSWER B

Wear for 15–20 minutes in the morning after waking includes some protocols specify around 10 minutes. Additional revision detail: some protocols specify around 10 minutes. The other options describe different mechanisms, investigations or findings rather than wear for 15–20 minutes in the morning after waking. Consult Integrated notebook, page 209, §10.4 for the full context.

REFERENCE Integrated notebook, page 209, §10.4

QUESTION 0976 · §10.4 · Clinical interpretation SBA

A patient starting long-term MAD therapy asks about bite change and jaw discomfort. The clinical team records the following finding: some protocols specify around 10 minutes. Which concept best explains this observation?

A. Wear for 15–20 minutes in the morning after waking

B. Exercises the jaw muscles

C. Positions the condyles

D. Exercises increase contact between the teeth

E. Realigns the bite

CORRECT ANSWER A

This finding most directly describes Wear for 15–20 minutes in the morning after waking: some protocols specify around 10 minutes. Distractor distinctions: B refers instead to applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear; C refers instead to assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort; D refers instead to good in the short term, with limited long-term evidence; E refers instead to custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation.

REFERENCE Integrated notebook, page 209, §10.4

QUESTION 0977 · §10.5 · Applied clinical SBA

An adult with new TMJ discomfort returns after increasing mandibular protrusion. Which of the following statements about monitoring and adjustment is most accurate?

A. Unilateral jaw pain includes prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines.

B. Nasal obstruction and excessive vertical opening may promote mouth breathing with a MAD.

C. A qualified dentist is essential for monitoring side effects and making necessary adjustments to the appliance or plan.

D. Transient morning jaw discomfort may result from overnight mandibular advancement.

E. Progressive occlusal changes arise from sustained dental forces during long-term MAD use.

CORRECT ANSWER C

A qualified dentist is essential for monitoring side effects and making necessary adjustments to the appliance or plan. The other options describe different mechanisms, investigations or findings rather than monitoring and adjustment. Consult Integrated notebook, page 211, §10.5 for the full context.

REFERENCE Integrated notebook, page 211, §10.5

QUESTION 0978 · §10.5 · Clinical interpretation SBA

A dentist is discussing material risks and reasonable alternatives before appliance delivery. The clinical team records the following finding: a qualified dentist is essential for monitoring side effects and making necessary adjustments to the appliance or plan. Which concept best explains this observation?

A. Excessive salivation

B. Soft tissue irritation

C. Unilateral jaw pain

D. Monitoring and adjustment

E. Transient morning jaw pain

CORRECT ANSWER D

This finding most directly describes Monitoring and adjustment: a qualified dentist is essential for monitoring side effects and making necessary adjustments to the appliance or plan. Distractor distinctions: A refers instead to the appliance itself. → towel on the pillow; reassurance, resolving in 2–3 weeks; B refers instead to screw mechanism or rough surface rubbing tongue or cheek; cheek or lip biting from bite change. → polish; discontinue until h; C refers instead to prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines; E refers instead to muscles adjusting to the forward mandible. → jaw exercises, massage, heat; morning occlusal guide.

REFERENCE Integrated notebook, page 211, §10.5

QUESTION 0979 · §10.6 · Applied clinical SBA

A patient starting long-term MAD therapy asks about bite change and jaw discomfort. Which of the following statements about leave it alone within the first 12–18 months is most accurate?

A. Poor appliance adherence warrants communication with the treating physician about alternatives.

B. Qualified dentists should review oral appliance therapy regularly for adherence, effectiveness and side effects.

C. Patient-specific factors includes overall health, tolerance for side effects and commitment to therapy.

D. Some early appliance-associated bite changes improve after adjustment or cessation with monitored care.

E. Efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be preferable to abandonment.

CORRECT ANSWER D

Some early appliance-associated bite changes improve after adjustment or cessation with monitored care. Additional revision detail: most will self-correct. This is why it is important to review vulnerable patients early, so that this option remains available. The other options describe different mechanisms, investigations or findings rather than leave it alone within the first 12–18 months. Consult Integrated notebook, page 212, §10.6 for the full context.

REFERENCE Integrated notebook, page 212, §10.6

QUESTION 0980 · §10.6 · Clinical interpretation SBA

At an annual OAT review, a patient notices a reduction in overjet and altered occlusal contacts. The clinical team records the following finding: most will self-correct. This is why it is important to review vulnerable patients early, so that this option remains available. Which concept best explains this observation?

A. Discuss with the patient and record the discussion

B. Leave it alone within the first 12–18 months

C. Patient-specific factors

D. Occlusal change the patient finds unacceptable

E. Remember the rule from 10.5

CORRECT ANSWER B

This finding most directly describes Leave it alone within the first 12–18 months: most will self-correct. This is why it is important to review vulnerable patients early, so that this option remains available. Distractor distinctions: A refers instead to the conversation is only protective if documented; C refers instead to overall health, tolerance for side effects and commitment to therapy; D refers instead to treatment should be discontinued only if the patient accepts another form of therapy from their attending physician, or under; E refers instead to definitive correction should be undertaken only once appliance therapy has been discontinued and the patient is on another OS.

REFERENCE Integrated notebook, page 212, §10.6

QUESTION 0981 · §10.2 · Applied clinical SBA

A dentist is discussing material risks and reasonable alternatives before appliance delivery. Which of the following statements about what patients actually experience is most accurate?

A. Many appliance users have small occlusal changes that they do not notice or find troublesome.

B. Long-term mandibular advancement causes mainly dental rather than substantial skeletal positional changes.

C. Many patients tolerate gradual occlusal change, but the risk still requires informed consent and monitoring.

D. Appliance-associated bite changes can cause food impaction, altered speech or chewing difficulty.

E. Failure to restore habitual mandibular position after nightly appliance use may contribute to occlusal changes.

CORRECT ANSWER A

Many appliance users have small occlusal changes that they do not notice or find troublesome. Additional revision detail: Mostly changes are minor and unnoticed; only a minority notice. Most are unaware, untroubled, and do not want to stop. Where noticed: gaps and food impaction; occasional lisping; rarely difficulty chewing on. The other options describe different mechanisms, investigations or findings rather than what patients actually experience. Consult Integrated notebook, page 198, §10.2 for the full context.

REFERENCE Integrated notebook, page 198, §10.2

QUESTION 0982 · §10.2 · Clinical interpretation SBA

A patient using a MAD reports persistent morning occlusal changes and reduced comfort. The clinical team records the following finding: Mostly changes are minor and unnoticed; only a minority notice. Most are unaware, untroubled, and do not want to stop. Where noticed: gaps and food impaction; occasional lisping; rarely difficulty chewing on. Which concept best explains this observation?

A. What patients actually experience

B. Where noticed, symptoms of bite change are

C. CPAP is not innocent

D. Position of professional bodies

E. Long term (moderate–severe, ongoing)

CORRECT ANSWER A

This finding most directly describes What patients actually experience: Mostly changes are minor and unnoticed; only a minority notice. Most are unaware, untroubled, and do not want to stop. Where noticed: gaps and food impaction; occasional lisping; rarely difficulty chewing on. Distractor distinctions: B refers instead to gaps and food impaction; occasional lisping when talking; and — rarely — difficulty chewing on the back teeth or tearing food; C refers instead to CPAP also causes retroclination of the upper incisors, from posteriorly directed forces on the anterior maxilla by a tightly; D refers instead to for most people the change in bite is not annoying, is not a risk to oral health, and does not require further dental treatme; E refers instead to jaw pain ~34%; TMJ restriction or noise; bite change 14–26%; damage to teeth or dental work; appliance issues.

REFERENCE Integrated notebook, page 198, §10.2

QUESTION 0983 · §10.4 · Applied clinical SBA

An adult with new TMJ discomfort returns after increasing mandibular protrusion. Which of the following statements about how it works is most accurate?

A. Morning occlusal exercises may help recover habitual mandibular position after overnight advancement.

B. Gentle morning jaw exercises may relieve post-appliance muscular stiffness.

C. Patients should use their morning occlusal guide daily and report persistent misfit.

D. A morning occlusal guide helps the mandible return toward habitual intercuspation after appliance removal.

E. A morning occlusal guide supports recovery toward the patient's original bite.

CORRECT ANSWER E

A morning occlusal guide supports recovery toward the patient's original bite. Additional revision detail: realigns the bite toward original maximum intercuspation; exercises and stretches the muscles of mastication, tight after a night of wear. The other options describe different mechanisms, investigations or findings rather than how it works. Consult Integrated notebook, page 199, §10.4 for the full context.

REFERENCE Integrated notebook, page 199, §10.4

QUESTION 0984 · §10.4 · Clinical interpretation SBA

A dentist is discussing material risks and reasonable alternatives before appliance delivery. The clinical team records the following finding: realigns the bite toward original maximum intercuspation; exercises and stretches the muscles of mastication, tight after a night of wear. Which concept best explains this observation?

A. Monitors progress

B. How it works

C. Positions the condyles

D. Two fabrication types

E. Realigns the bite

CORRECT ANSWER B

This finding most directly describes How it works: realigns the bite toward original maximum intercuspation; exercises and stretches the muscles of mastication, tight after a night of wear. Distractor distinctions: A refers instead to the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical poin; C refers instead to assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort; D refers instead to those using thermoacrylic beads, and those using a prefabricated sheet (AM aligner) placed in hot water, moulded around the t; E refers instead to custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation.

REFERENCE Integrated notebook, page 199, §10.4

QUESTION 0985 · §10.5 · Applied clinical SBA

A patient starting long-term MAD therapy asks about bite change and jaw discomfort. Which of the following statements about unilateral jaw pain is most accurate?

A. Nasal obstruction and excessive vertical opening may promote mouth breathing with a MAD.

B. Bilateral jaw pain includes advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed).

C. Unilateral jaw pain includes prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines.

D. Palliative care includes supportive care to manage discomfort, especially during the initial adaptation period.

E. Transient hypersalivation often improves during early appliance adaptation.

CORRECT ANSWER C

Unilateral jaw pain includes prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines. Additional revision detail: prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines. The other options describe different mechanisms, investigations or findings rather than unilateral jaw pain. Consult Integrated notebook, page 200, §10.5 for the full context.

REFERENCE Integrated notebook, page 200, §10.5

QUESTION 0986 · §10.5 · Clinical interpretation SBA

At an annual OAT review, a patient notices a reduction in overjet and altered occlusal contacts. The clinical team records the following finding: prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines. Which concept best explains this observation?

A. Patient education

B. Transient morning jaw pain

C. Unilateral jaw pain

D. Minor bite change

E. Excessive salivation

CORRECT ANSWER C

This finding most directly describes Unilateral jaw pain: prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines. Distractor distinctions: A refers instead to reassure that symptoms are likely temporary and manageable; B refers instead to muscles adjusting to the forward mandible. → jaw exercises, massage, heat; morning occlusal guide; D refers instead to mechanical forces causing gradual shifts. → jaw exercises, passive exercises, morning occlusal guide; adjust the appliance or; E refers instead to the appliance itself. → towel on the pillow; reassurance, resolving in 2–3 weeks.

REFERENCE Integrated notebook, page 200, §10.5

QUESTION 0987 · §10.6 · Applied clinical SBA

A dentist is discussing material risks and reasonable alternatives before appliance delivery. Which of the following statements about occlusal change is most accurate?

A. Poor appliance adherence warrants communication with the treating physician about alternatives.

B. If appliance therapy is discontinued, the physician should arrange alternative OSA treatment or discuss the risks.

C. Occlusal change includes oJ ~1.9 mm, OB ~2.3 mm (average ~2.5 mm).

D. Definitive correction of occlusal changes is usually deferred until appliance therapy is stopped or altered.

E. Qualified dentists should review oral appliance therapy regularly for adherence, effectiveness and side effects.

CORRECT ANSWER C

Occlusal change includes oJ ~1.9 mm, OB ~2.3 mm (average ~2.5 mm). Additional revision detail: OJ ~1.9 mm, OB ~2.3 mm (average ~2.5 mm); ~⅓ exceed 1 mm; at 17 y median −1.1 / −1.6 mm; ~80% of 7-year nightly users; 85% detectable on models and cephalograms; 13.3% by 2 years then constant. The other options describe different mechanisms, investigations or findings rather than occlusal change. Consult Integrated notebook, page 201, §10.6 for the full context.

REFERENCE Integrated notebook, page 201, §10.6

QUESTION 0988 · §10.6 · Clinical interpretation SBA

A patient using a MAD reports persistent morning occlusal changes and reduced comfort. The clinical team records the following finding: OJ ~1.9 mm, OB ~2.3 mm (average ~2.5 mm); ~⅓ exceed 1 mm; at 17 y median −1.1 / −1.6 mm; ~80% of 7-year nightly users; 85% detectable on models and cephalograms; 13.3% by 2 years then constant. Which concept best explains this observation?

A. Occlusal change

B. Regular follow-up

C. Occlusal change the patient finds unacceptable

D. Regular reviews

E. Device and settings

CORRECT ANSWER A

This finding most directly describes Occlusal change: OJ ~1.9 mm, OB ~2.3 mm (average ~2.5 mm); ~⅓ exceed 1 mm; at 17 y median −1.1 / −1.6 mm; ~80% of 7-year nightly users; 85% detectable on models and cephalograms; 13.3% by 2 years then constant. Distractor distinctions: B refers instead to monitoring by qualified dentists is necessary to survey for dental side effects and occlusal change, and is essential for max; C refers instead to treatment should be discontinued only if the patient accepts another form of therapy from their attending physician, or under; D refers instead to at least 6-monthly for the first year, then annually; E refers instead to efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be pr.

REFERENCE Integrated notebook, page 201, §10.6

QUESTION 0989 · §10.2 · Applied clinical SBA

At an annual OAT review, a patient notices a reduction in overjet and altered occlusal contacts. Which of the following statements about causes is most accurate?

A. Many appliance users have small occlusal changes that they do not notice or find troublesome.

B. Appliance-associated bite changes can cause food impaction, altered speech or chewing difficulty.

C. Long-term mandibular advancement causes mainly dental rather than substantial skeletal positional changes.

D. Many patients tolerate gradual occlusal change, but the risk still requires informed consent and monitoring.

E. Occlusal change is a recognised long-term risk of mandibular advancement therapy.

CORRECT ANSWER C

Long-term mandibular advancement causes mainly dental rather than substantial skeletal positional changes. Additional revision detail: what it is not. Change in mandibular length: unlikely. Change in skeletal pattern: unlikely — appliances cause tooth movement, not craniofacial skeletal change. The other options describe different mechanisms, investigations or findings rather than causes. Consult Integrated notebook, page 198, §10.2 for the full context.

REFERENCE Integrated notebook, page 198, §10.2

QUESTION 0990 · §10.2 · Focused knowledge SBA

An adult with new TMJ discomfort returns after increasing mandibular protrusion. Which of the following statements about causes is most accurate?

A. Appliance-associated bite changes can cause food impaction, altered speech or chewing difficulty.

B. Occlusal change is a recognised long-term risk of mandibular advancement therapy.

C. Many appliance users have small occlusal changes that they do not notice or find troublesome.

D. Many patients tolerate gradual occlusal change, but the risk still requires informed consent and monitoring.

E. Long-term mandibular advancement causes mainly dental rather than substantial skeletal positional changes.

CORRECT ANSWER E

Long-term mandibular advancement causes mainly dental rather than substantial skeletal positional changes. Additional revision detail: what it is not. Change in mandibular length: unlikely. Change in skeletal pattern: unlikely — appliances cause tooth movement, not craniofacial skeletal change. The other options describe different mechanisms, investigations or findings rather than causes. Consult Integrated notebook, page 198, §10.2 for the full context.

REFERENCE Integrated notebook, page 198, §10.2

QUESTION 0991 · §10.5 · Applied clinical SBA

At an annual OAT review, a patient notices a reduction in overjet and altered occlusal contacts. Which of the following statements about dry mouth is most accurate?

A. Transient hypersalivation often improves during early appliance adaptation.

B. Nasal obstruction and excessive vertical opening may promote mouth breathing with a MAD.

C. Palliative care includes supportive care to manage discomfort, especially during the initial adaptation period.

D. Bilateral jaw pain includes advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed).

E. Transient morning jaw discomfort may result from overnight mandibular advancement.

CORRECT ANSWER B

Nasal obstruction and excessive vertical opening may promote mouth breathing with a MAD. Additional revision detail: poor nasal breathing → mouth breathing; vertical dimension too large making lip seal difficult. The other options describe different mechanisms, investigations or findings rather than dry mouth. Consult Integrated notebook, page 200, §10.5 for the full context.

REFERENCE Integrated notebook, page 200, §10.5

QUESTION 0992 · §10.5 · Clinical interpretation SBA

An adult with new TMJ discomfort returns after increasing mandibular protrusion. The clinical team records the following finding: poor nasal breathing → mouth breathing; vertical dimension too large making lip seal difficult. Which concept best explains this observation?

A. Dry mouth

B. Minor bite change

C. Regular follow-up

D. Patient education

E. Palliative care

CORRECT ANSWER A

This finding most directly describes Dry mouth: poor nasal breathing → mouth breathing; vertical dimension too large making lip seal difficult. Distractor distinctions: B refers instead to mechanical forces causing gradual shifts. → jaw exercises, passive exercises, morning occlusal guide; adjust the appliance or; C refers instead to monitoring by qualified dentists is necessary to survey for dental side effects and occlusal change, and is essential for max; D refers instead to reassure that symptoms are likely temporary and manageable; E refers instead to supportive care to manage discomfort, especially during the initial adaptation period.

REFERENCE Integrated notebook, page 200, §10.5

QUESTION 0993 · §10.6 · Clinical decision-making SBA

A patient experiences a persistent new posterior open bite after several years of oral appliance therapy. The appliance remains effective for OSA and the patient wishes to continue. Which management principle is best?

A. Increase protrusion to reposition the posterior teeth

B. Discuss the trade-off, review function and comfort, document serial occlusion and consider preventive or corrective options with the patient

C. Perform irreversible occlusal adjustment immediately without reassessing therapy

D. Discontinue all OSA treatment before discussing dental options

E. Ignore the finding if the patient reports reduced snoring

CORRECT ANSWER B

The decision should balance continuing control of OSA with the functional and dental effects of therapy. Serial records, shared decision-making, reasonable mitigation and specialist input when needed are preferable to reflexive irreversible treatment.

REFERENCE Integrated notebook, Topic 10, §10.6

QUESTION 0994 · §10.6 · Clinical decision-making SBA

A patient has persistent severe jaw pain despite reduction of mandibular advancement, a temporary treatment pause and appropriate conservative measures. Which is the best next management decision?

A. Continue mandibular advancement at the previous painful setting

B. Increase vertical opening without examining the joint

C. Consider suspending or abandoning the current device and coordinate an alternative OSA therapy with the treating physician

D. Leave the patient untreated without contacting the sleep physician

E. Perform irreversible jaw repositioning without diagnosis

CORRECT ANSWER C

Failure to tolerate the device despite appropriate management may require suspension or discontinuation. The OSA still requires safe and effective treatment, so alternative management should be coordinated with the sleep physician.

REFERENCE Integrated notebook, Topic 10, §10.6

QUESTION 0995 · §10.2 · Applied clinical SBA

An adult with new TMJ discomfort returns after increasing mandibular protrusion. Which of the following statements about frequency is most accurate?

A. Occlusal change is a recognised long-term risk of mandibular advancement therapy.

B. Long-term nightly mandibular advancement frequently produces detectable occlusal changes.

C. Appliance-associated bite changes can cause food impaction, altered speech or chewing difficulty.

D. Many patients tolerate gradual occlusal change, but the risk still requires informed consent and monitoring.

E. Mandibular advancement can shift maxillary teeth distally and mandibular teeth mesially over time.

CORRECT ANSWER B

Long-term nightly mandibular advancement frequently produces detectable occlusal changes. Additional revision detail: ~80% of nightly users over 7 years experience change; 85% detectable on models and cephalograms (some bites do not change). The other options describe different mechanisms, investigations or findings rather than frequency. Consult Integrated notebook, page 198, §10.2 for the full context.

REFERENCE Integrated notebook, page 198, §10.2

QUESTION 0996 · §10.2 · Focused knowledge SBA

A dentist is discussing material risks and reasonable alternatives before appliance delivery. Which of the following statements about frequency is most accurate?

A. Appliance-associated bite changes can cause food impaction, altered speech or chewing difficulty.

B. Mandibular advancement can shift maxillary teeth distally and mandibular teeth mesially over time.

C. Long-term nightly mandibular advancement frequently produces detectable occlusal changes.

D. Many patients tolerate gradual occlusal change, but the risk still requires informed consent and monitoring.

E. Occlusal change is a recognised long-term risk of mandibular advancement therapy.

CORRECT ANSWER C

Long-term nightly mandibular advancement frequently produces detectable occlusal changes. Additional revision detail: ~80% of nightly users over 7 years experience change; 85% detectable on models and cephalograms (some bites do not change). The other options describe different mechanisms, investigations or findings rather than frequency. Consult Integrated notebook, page 198, §10.2 for the full context.

REFERENCE Integrated notebook, page 198, §10.2

QUESTION 0997 · §10.5 · Applied clinical SBA

An adult with new TMJ discomfort returns after increasing mandibular protrusion. Which of the following statements about bilateral jaw pain is most accurate?

A. Nasal obstruction and excessive vertical opening may promote mouth breathing with a MAD.

B. Bilateral jaw pain includes advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed).

C. Unilateral jaw pain includes prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines.

D. Palliative care includes supportive care to manage discomfort, especially during the initial adaptation period.

E. Transient hypersalivation often improves during early appliance adaptation.

CORRECT ANSWER B

Bilateral jaw pain includes advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed). Additional revision detail: advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed). The other options describe different mechanisms, investigations or findings rather than bilateral jaw pain. Consult Integrated notebook, page 200, §10.5 for the full context.

REFERENCE Integrated notebook, page 200, §10.5

QUESTION 0998 · §10.5 · Clinical decision-making SBA

A MAD user develops bilateral jaw pain and limited opening after the appliance is advanced beyond the previously comfortable position. Which factor is the most likely contributor?

A. The appliance's treatment of primary snoring itself

B. Loss of homeostatic sleep pressure

C. Excessive mandibular advancement beyond TMJ tolerance

D. A normal increase in airway volume in the absence of symptoms

E. A reduced Epworth Sleepiness Scale score

CORRECT ANSWER C

The temporal relationship to increased protrusion, bilateral pain and reduced opening points toward an excessive mechanical load on the TMJ or masticatory system. Review and reduce the setting before further titration.

REFERENCE Integrated notebook, Topic 10, §10.5

QUESTION 0999 · §10.2 · Clinical decision-making SBA

A 58-year-old has used a mandibular advancement appliance nightly for seven years. Compared with baseline, overjet has reduced from 3 mm to 1 mm and posterior contacts have altered. Which is the most likely explanation?

A. A change in the apnoea-hypopnoea index directly moves the incisors

B. Gradual dentoalveolar adaptation associated with nightly mandibular advancement

C. An isolated measurement error that cannot recur over serial records

D. Increased nasal airway resistance independently reduces overjet

E. The occlusal change confirms complete resolution of OSA

CORRECT ANSWER B

Mandibular advancement appliances exert forces on the dentition and may produce progressive overjet, overbite and posterior occlusal changes. The baseline comparison establishes the clinically relevant change but does not confirm therapeutic efficacy.

REFERENCE Integrated notebook, Topic 10, §10.2

QUESTION 1000 · §10.5 · Clinical decision-making SBA

A patient develops persistent TMJ tenderness and restricted opening after increasing mandibular protrusion from 5 mm to 7 mm. Snoring is improved, but oral function is now impaired. What is the most appropriate immediate management?

A. Advance further to maximize airway opening

B. Continue unchanged until the next annual review

C. Assess the TMJ and appliance, reduce to the last comfortable setting and review symptoms

D. Perform immediate irreversible occlusal correction

E. Treat the reduction in snoring as proof that the painful setting is optimal

CORRECT ANSWER C

Titration is limited by comfort and oral function. Persistent symptoms warrant assessment and modification of the appliance, with temporary suspension or further referral if symptoms do not settle; treatment efficacy requires separate objective verification.

REFERENCE Integrated notebook, Topic 10, §10.5